MSc Marriage and Family Therapy

The Lecture Library

Twenty-eight course books in one file

This single file contains every volume of the library, in programme order: eight courses in Semester One, seven in Semester Two, seven in Semester Three including the thesis, and six electives. Each course keeps its own cover, imprint, contents, running heads and folio, so the file can be printed whole or the individual volumes printed from the separate PDFs in `pdf/`.

Use the bookmark panel of your PDF reader to jump to any course or to any chapter within a course. Every volume is also available as a standalone file, and the same text is published as a browsable site in `site/`.

Courses in this library

Code	Course	Semester	Units
MFT 701	Fundamentals of Marriage and Family Therapy	One	1.71
MFT 702	Theories of Family Therapy	One	1.71
MFT 703	Human Development Across the Lifespan	One	1.71
MFT 704	Ethical and Legal Issues in Therapy	One	1.71
MFT 705	Research Methods in Marriage and Family Therapy	One	1.71
MFT 706	Clinical Practice and Techniques	One	1.71
MFT 707	Assessment and Diagnosis in Family Therapy	One	1.71
MFT 708	Cultural Competence in Therapy	One	1.71
MFT 709	Group Therapy Techniques	Two	1.71
MFT 710	Couples Therapy	Two	1.71
MFT 711	Child and Adolescent Therapy	Two	1.71
MFT 712	Crisis Intervention Strategies	Two	1.71
MFT 713	Substance Abuse and Family Dynamics	Two	1.71
MFT 714	Trauma-Informed Care	Two	1.71
MFT 715	Family Systems Theory	Two	1.71
MFT 716	Communication Skills in Therapy	Three	1.71
MFT 717	Psychopathology in Families	Three	1.71
MFT 718	Intimacy and Relationship Dynamics	Three	1.71
MFT 719	Family Life Cycle and Transitions	Three	1.71
MFT 720	Supervision and Consultation in Therapy	Three	1.71
MFT 721	Research Methodology	Three	1.71
MFT 722	Professional Development and Self-Care	Electives	1.71
MFT 723	Special Populations in Therapy	Electives	1.71
MFT 724	Family Law and Advocacy	Electives	1.71
MFT 725	Integrative Approaches in Therapy	Electives	1.71
MFT 726	Neurobiology of Relationships	Electives	1.71
MFT 727	Play Therapy Techniques	Electives	1.71
MFT 770	Master Thesis	Three	1.71

Contents of each volume: how to use the book; course description and rationale; learning outcomes; assessment; the fourteen-week plan; fourteen teaching weeks; an integration chapter; a glossary; a self-assessment paper with model answers; and further reading.

MSC MARRIAGE AND FAMILY THERAPY

LECTURE LIBRARY · VOLUME 1 OF 28

MFT 701

Fundamentals of Marriage and Family Therapy

14 teaching weeks · 32 practicals · 19 worked cases · 1 figure

SEMESTER ONE

1.71 units · Prerequisite: none

> “This is the course that turns a helping professional into a systemic one.”

One of 28 self-contained course books in the MSc Marriage and Family Therapy lecture library: a full taught semester per volume, with cases, practicals, assessment briefs, glossaries and model answers.

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MFT 701 — Fundamentals of Marriage and Family Therapy

MSc Marriage and Family Therapy · Semester One · 1.71 Units · 14 teaching weeks · Prerequisite: none

This volume is one of 28 course books that together cover the whole MSc Marriage and Family Therapy taught programme. Each book is written to carry a 14–16 week semester on its own: every week pairs a core idea with the practice that follows from it, a worked case, and a reflective task, and each book closes with an integration chapter, a glossary and a self-assessment paper with model answers.

The books are designed to be read straight through before the semester begins, revisited week by week, and used again in revision. Cross-references point to the other volumes where a topic is treated in more depth.

How to cite this volume

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A note on the case material

All families, couples and individuals in this book are fictional composites. They are written to teach pattern recognition, not to describe any real person. Nothing here replaces supervised clinical practice, professional registration, legal advice or medical care; legal content is orientation only and must be checked against the law of the relevant jurisdiction.

The lecture library

Twenty-eight self-contained course books carry the taught programme: eight in Semester One, seven in Semester Two, seven in Semester Three including the thesis, and six electives. The volume you are holding is marked in the margin.

Code	Course	Semester	Units
MFT 701	**Fundamentals of Marriage and Family Therapy**	One	1.71
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MFT 719	Family Life Cycle and Transitions	Three	1.71
MFT 720	Supervision and Consultation in Therapy	Three	1.71
MFT 721	Research Methodology	Three	1.71
MFT 722	Professional Development and Self-Care	Electives	1.71
MFT 723	Special Populations in Therapy	Electives	1.71
MFT 724	Family Law and Advocacy	Electives	1.71
MFT 725	Integrative Approaches in Therapy	Electives	1.71
MFT 726	Neurobiology of Relationships	Electives	1.71
MFT 727	Play Therapy Techniques	Electives	1.71
MFT 770	Master Thesis	Three	1.71

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Course leader's summary. This is the course that turns a helping professional into a systemic one. It covers the intellectual history of family therapy, the epistemology that separates it from individual work, the profession's identity and scope, and the basic architecture of a first session. Everything later in the programme — theory, technique, assessment, ethics — assumes the vocabulary built here.

How to use this book

Each week contains four things: **the idea** (what you must understand), **the practice** (what you must be able to do), **the case** (what it looks like in a room), and **a reflective task** (what it asks of you personally). Read the week before the lecture; do the practical in the lab; write the reflection within 48 hours, while it is still uncomfortable.

The book is written to be read straight through, but it is also designed for revision: the integration chapter at the end compresses the whole course into two pages, and the glossary defines every term the examination may use.

1. Course description and rationale

Most mental health training teaches us to look inside a person: their thoughts, their symptoms, their attachment history. Family therapy teaches us to look between people: the loop, the triangle, the rule that nobody wrote down. This is not a rejection of individual psychology. It is an additional discipline of attention, and it changes what counts as the problem, the cause, and the cure.

Consider three common presentations:

Presentation	Individual formulation	Systemic formulation
A 15-year-old refuses school	Anxiety disorder, school phobia	A child whose symptoms hold a parental conflict at bay; the school refusal is the family's most effective way of preventing an argument about the marriage
A wife is "depressed"	Major depressive episode	A woman in a triad with her husband and his mother, carrying the family's unspoken grief about a lost pregnancy
A man "works too much"	Workaholism, avoidance	The person who regulates the family's emotional temperature by being absent, in a system that rewards his absence

None of these systemic formulations is automatically more true. Each is a hypothesis with different implications for where you intervene. The mature clinician holds both — the individual and the relational — and chooses deliberately.

Why this course first. Because students who begin with technique learn to do something they cannot explain, and students who begin with theory learn to explain something they cannot do. MFT 701 begins with framing: what kind of problem is this, and therefore what kind of help is required?

2. Learning outcomes

By the end of the course, students will be able to:

#	Outcome	Assessed in
LO1	Explain the epistemological shift from linear to circular causality, and its clinical consequences	Exam, Week 2 log
LO2	Trace the historical development of family therapy and locate contemporary models within it	Exam
LO3	Describe the profession's identity, scope of practice, and relationship to allied professions	Assignment
LO4	Conduct a structured first session, including joining, problem definition, and contracting	Skills assessment
LO5	Draw a three-generation genogram and generate hypotheses from it	Assignment
LO6	Critically discuss the common-factors evidence and its implications for model choice	Exam essay
LO7	Identify their own family-of-origin patterns and describe their effect on clinical work	Reflective log

3. Assessment

Component	Weight	Due
Reflective practice log (weekly, 300–400 words)	20%	Fortnightly
Genogram and hypothesis assignment (2,000 words + diagram)	30%	Week 9
Two-hour written examination (seen case + essay + short answers)	50%	Week 15

Skills gate. Students must pass a 15-minute observed first session (Week 6) before proceeding to MFT 706. The gate is pass/fail with one resit.

4. The fourteen-week teaching plan

Week	Topic	Practical	Core reading
1	What is family therapy? The relational turn	Triad: "the problem as described"	Nichols ch. 1; Handbook ch. 1
2	Linear and circular causality; first- and second-order cybernetics	Observation of a recorded argument	Watzlawick, Pragmatics ch. 1
3	A short history: from schizophrenia research to evidence-based practice	Timeline construction	Nichols ch. 2–3
4	Systems thinking: boundaries, feedback, homeostasis, rules	Feedback-loop mapping exercise	Becvar & Becvar ch. 2

Week	Topic	Practical	Core reading
5	The profession: identity, scope, settings, teamwork	Role boundaries case discussion	AAMFT code, scope sections
6	The first session I: joining, and the referral question	**Observed skills gate**	Minuchin & Fishman ch. 1
7	The first session II: defining the problem in the family's language	Triad: problem definition	Haley, Leaving Home ch. 1
8	The genogram as a hypothesis machine	Genogram lab (own family)	McGoldrick, Genograms ch. 1–2
9	Formulating relationally: the 5-P grid applied to a family	Formulation workshop	Dallos & Draper ch. 4
10	Culture, class and power in the consulting room	Reflexivity exercise	McGoldrick, Ethnicity & Family Therapy
11	Evidence: what works, for whom, and the common factors	Appraisal of one RCT and one qualitative study	Wampold ch. 2
12	Ethics, risk and the limits of the systemic frame	Risk-screen rehearsal	MFT 704 pre-reading
13	Integration I: three models applied to one family	Live demonstration + discussion	Nichols ch. 10
14	Integration II: your emerging clinical stance	Position statement presentation	Course review

Week 1 — What is family therapy? The relational turn

1.1 The lecture in ninety minutes

Week 1 has one job: to make the relational frame feel inevitable rather than eccentric. Everything a student does later — the first session, the genogram, the formulation, the ethics of conjoint work — assumes they can see a relationship as a thing with a shape, a history and a set of rules. Students arrive able to see people. Very few arrive able to see between people.

The ninety minutes run like this.

Minutes	Segment	What the lecturer does	What students do
0–10	Two formulations	Read the Okonkwo referral twice: once as an individual case, once as a relational one	Note which version they would have written
10–30	Where the field came from	The three failures of the individual frame (1.2)	Timeline exercise: place five founding texts
30–50	Five claims of the relational frame	Build the list on the board, testing each against a live example	Vote: which claim is hardest to accept?
50–65	Systems as a way of thinking	Cybernetics, feedback, the Watzlawick axioms (1.5)	Map one family loop on paper

Minutes	Segment	What the lecturer does	What students do
65–80	Three openings in a first session	Annotated transcript, read aloud in parts	Mark where the therapist widened the field
80–90	Set the lab and the journal	Hand out the widening-question protocol (1.11)	Form triads, choose a real (non-clinical) difficulty

The lecture is deliberately front-loaded with history. Students who know the field's origins stop treating systemic thinking as a technique among techniques and start treating it as an epistemology — a claim about what kind of thing a problem is.

1.2 Why the field exists: three failures of the individual frame

Family therapy was not invented because someone preferred groups to individuals. It was invented because three well-funded, well-intentioned research programmes ran into the same wall: the individual frame kept losing information that mattered clinically.

The first failure: the patient kept relapsing into a family. In the 1950s, Gregory Bateson's research group at Palo Alto — Bateson, Don Jackson, Jay Haley and John Weakland — was studying communication in families where one member had been diagnosed with schizophrenia. Their 1956 paper, "Toward a theory of schizophrenia," proposed the **double bind**: a pattern in which a person receives two messages at different logical levels that contradict each other, cannot comment on the contradiction, and cannot leave the field. A mother who says "come here, dear" while stiffening when the child approaches; a father who asks for honesty and then punishes it; a family that requires affection and forbids the expression of need that would make affection possible.

The double-bind theory has not survived as an explanation of schizophrenia. Its status as a causal claim about psychosis is dead, and students should know that. But something else in that research survived and became the spine of the field: the observation that **sequences of behaviour between people carry information that is invisible when you describe the people separately.** The patient's behaviour was intelligible as a move in a game; the game was not in the patient.

The second failure: the symptom made sense at home. Independently, researchers studying families of people with serious mental illness — Theodore Lidz at Yale, Lyman Wynne at the National Institute of Mental Health — noticed that families organised themselves around the symptomatic member. Wynne described **pseudo-mutuality**: a family that maintains the appearance of harmony by prohibiting any relationship that would threaten it. Lidz described **marital schism** and **marital skew**: two patterns in which the couple's conflict or the couple's imbalance shaped the child's development. Again the aetiological claims are dated. Again the structural insight held: **a family can be organised by a symptom**, and the symptom can be doing work for the system.

The third failure: the child was the family's messenger. The child guidance movement of the 1920s–1950s treated the child, advised the mother, and sent both home to a household that had not changed. Clinicians began to notice that improvement in the child was routinely followed by a new symptom in a sibling, or in the mother, or by a marital crisis that had been dormant. The pattern is now so familiar that it has a name —

symptom substitution — and the observation that produced it is the most practically useful idea in this week: if the symptom is functional for the system, removing the symptom without changing the system is unstable.

Three research programmes, three different populations, one conclusion: the unit of assessment was wrong. Between 1950 and 1975 the field built an alternative. Nathan Ackerman founded the Family Institute in 1960 and, with Jackson, the journal Family Process in 1962. Murray Bowen developed his theory of differentiation and multigenerational transmission at the Menninger Clinic and later at Georgetown. Virginia Satir was teaching conjoint family interviewing in California. Salvador Minuchin, working with delinquent boys at the Wiltwyck School and then at the Philadelphia Child Guidance Clinic, built structural family therapy out of the observation that these families were not sick so much as **organised at the wrong level of hierarchy** — children in charge, parents disengaged or in coalition, boundaries too diffuse or too rigid. Jay Haley joined Minuchin in Philadelphia in 1967 and brought strategy, power and the deliberate use of directives.

By the late 1970s the field had a canon: the Palo Alto communication school, Bowen's multigenerational theory, Minuchin's structural therapy, Haley and Madanes's strategic therapy, the Milan group's systemic therapy with its rituals and positive connotation, Satir's human validation, and the beginnings of solution-focused work at Milwaukee and narrative therapy in Adelaide and Auckland. Students will meet each of these properly in MFT 702. For now, one thing matters: **every one of these schools is an answer to the question "what is the unit?"** and each answers it slightly differently — the sequence, the generations, the structure, the meaning, the solution, the story.

> **DEBATE**
> "If the aetiological claims of the 1950s were wrong, the field built on them is unsound." Ten minutes, two sides. The strongest argument against: a science is allowed to keep its methods after discarding its first theories — medicine kept auscultation after humoral theory fell. The strongest argument for: the field has never fully shed the habit of blaming families for individual distress.

1.3 What "relational" actually means: five claims

"Systemic" is a word students learn to say before they learn to mean. Here are the five claims that make the relational frame a frame, in ascending order of how much they cost you as a clinician.

Claim 1 — Behaviour is patterned. Any two people who spend time together generate repeating sequences. The pattern is not in either person; it is in the loop. A couple who report "we never talk" will, within twenty minutes of a first session, demonstrate precisely how the not-talking is produced: one asks a question that contains an accusation, the other answers with a fact, the first escalates the volume, the second goes quiet. Neither is doing it to the other. Both are in it.

Claim 2 — The pattern is maintained by its consequences, not by its causes. This is cybernetics in one sentence. The question "why did this start?" is usually unanswerable and clinically useless; the question "what keeps it going?" is answerable and clinically decisive. A father's withdrawal is maintained by his wife's criticism, which is maintained by his withdrawal. Ten years of history explain how they got into the loop; only the loop explains why they are still in it.

Claim 3 — Meaning is negotiated between people. The same act means different things to different family members, and those meanings are not private. A daughter's silence means "respect" to her father, "punishment" to her mother, and "safety" to herself. The clinical task is not to find the true meaning but to make the several meanings audible to everyone in the room.

Claim 4 — Symptoms can serve the system. Not always, not deliberately, and never in a way that makes the symptomatic person responsible. But frequently enough that the therapist must ask: what would change in this household if this symptom disappeared tomorrow? Sometimes the honest answer is "nothing." Sometimes it is "my marriage would have to be looked at," and that answer reorganises the whole treatment.

Claim 5 — The observer is inside the observed system. You are not a camera. Your presence changes what the family does, what they say, what they notice about themselves, and what they will risk in front of each other. The family that arrives furious will perform its fury for you; the family that arrives polite will perform its politeness. This is not contamination to be minimised — it is the instrument you have. Second- order cybernetics, which students meet formally in MFT 715, is simply the discipline of using that instrument knowingly (Figure 1.1).

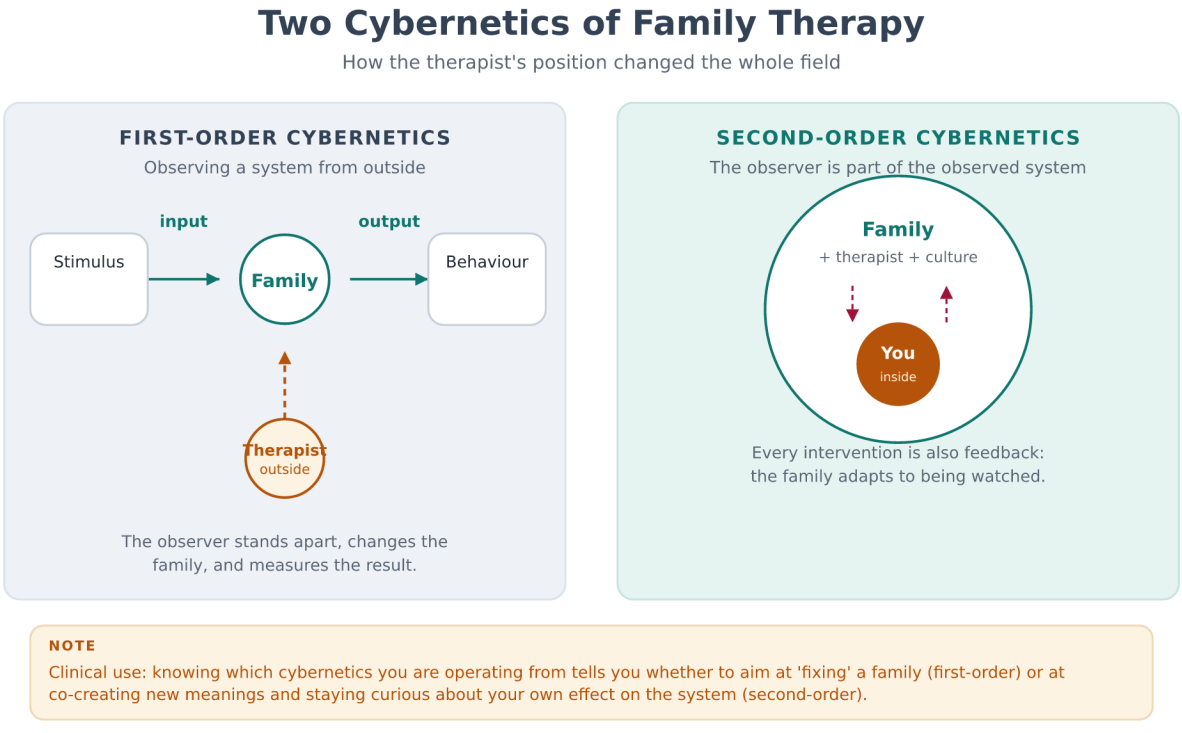


Figure 1.1 — First- and second-order cybernetics: where the therapist stands

A working definition. Marriage and family therapy is the assessment and treatment of relational systems — couples, families, and the wider networks in which they live — using interventions that target interaction patterns, meaning systems, and the emotional bonds between people. Read the definition carefully: assessment of relational systems means the unit of assessment is the relationship; interventions that target patterns means the unit of intervention is the loop, the rule, the structure or the story; emotional bonds means that a purely behavioural account is insufficient, because people are attached to each other and the attachment is what makes the pattern stubborn.

What it is not. Four clarifications, because students routinely collect the wrong ones.

1. **It is not "seeing everyone together" as a matter of policy.** Sometimes the right systemic move is an individual session inside family work. Conjoint work is a means, not a doctrine.
2. **It is not a denial that individuals exist, or that biology, trauma and temperament matter.** It is a disciplined focus on context, not a metaphysics.
3. **It is not the claim that families cause mental illness.** That claim was made in the 1950s and it was wrong, cruel, and expensive. Contemporary systemic practice holds families as the site where distress is expressed, maintained and relieved — not as the sole cause.
4. **It is not "we don't do diagnosis."** Systemic therapists diagnose, formulate and often work inside services that require a diagnosis; see MFT 707.

1.4 The four consequences, worked through

The four consequences in the original week note are worth more than a bullet list. Take them one at a time, each with what it changes on Monday morning.

Consequence 1 — The unit of assessment changes. You must hear from more than one person, and notice what happens between them. In practice this means the assessment cannot be completed in a single interview with one informant. It means disagreement between informants is data rather than noise: if the mother says the boy is defiant and the father says he is fine, the difference is the finding. It means you watch the room — who sits where, who answers for whom, who looks at whom before speaking, what happens to the volume when one person enters.

Consequence 2 — The unit of intervention changes. You intervene in sequences, rules, meanings and structures, not only in individual cognition. A cognitive intervention asks the depressed woman to examine the evidence for her belief that she is a burden. A systemic intervention asks what happens in the household in the forty-eight hours after she expresses that belief — and often finds that the expression triggers reassurance, that reassurance prevents any change in the division of labour, and that the division of labour is what is depressing her. Both are legitimate. The second is invisible from inside the first.

Consequence 3 — The definition of success changes. Success may be a family that argues well, not a family that never argues. This is the hardest consequence for students trained in symptom reduction, because it means the goal is not the absence of conflict but the presence of **repair**. A couple who can fight, rupture and come back together within an

hour is doing better than a couple who never fight and are quietly contemptuous. Outcome measures should reflect this: not only "has the symptom gone?" but "can this system handle disagreement without breaking?"

Consequence 4 — The therapist's position changes. You are not a technician outside the system; you are a participant who affects it. Three practical implications. First, you must account for your own effect: the family will behave differently because you are watching, and the difference is informative rather than annoying. Second, you cannot claim neutrality; you can only be transparent about your partiality. Third, your own family of origin is in the room with you — which is why every week of this book ends with a reflective task, and why personal therapy and supervision are professional requirements, not luxuries.

> **CASE**
> Mrs Okonkwo brings her 9-year-old son, Ebuka, for "stubbornness". In the first ten minutes she mentions, twice, that his father "is never home". The linear frame says: assess and treat Ebuka's behaviour. The systemic frame says: ask who else is affected by the behaviour, what happens after it, and what would change in the household if it stopped. The second question is not a rejection of the first; it is a widening of the field in which the first can be treated.

1.5 Systems as a way of thinking

Students often assume "system" is jargon for "family". It is not. A system is any set of elements whose interaction produces effects none of the elements produces alone. Ludwig von Bertalanffy's general system theory (1968), Norbert Wiener's cybernetics (1948) and the communications research at Palo Alto gave the field three tools, and they are still the three tools.

Feedback. A system regulates itself through loops. **Negative feedback** reduces deviation and restores the previous state — the family that pulls a member back whenever they move too far, the couple whose argument returns to the same sentence. **Positive feedback** amplifies deviation and produces change or escalation — the quarrel that grows, the affair that is discovered, the child who leaves. Note that the words are not evaluative: negative feedback is not bad, positive is not good. Most clinical problems are stuck negative-feedback loops with high cost; most crises are positive-feedback loops that have run away.

Homeostasis and morphostasis. Systems resist change in order to remain themselves. When a family comes to therapy, two processes are always running: the pressure to solve the problem and the pressure to keep the system intact. This is why families sometimes seem to sabotage their own treatment, and why the therapist who moves too fast loses the family. The technical answer is to pace: introduce change at a rate the system can absorb, and check that each change is stable before the next.

Rules and punctuation. Watzlawick, Beavin and Jackson's Pragmatics of Human Communication (1967) gave the field its most quoted propositions: one cannot not

communicate; every communication has a content and a relationship level; relationships are defined by punctuation of sequences; humans communicate digitally (words) and analogically (behaviour, tone, gesture). The idea of **punctuation** is the most clinically valuable. Each partner punctuates the same sequence differently: she says "he goes silent, so I get loud"; he says "she gets loud, so I go silent". Both are describing the same loop from different points on it, and each punctuation makes the other the cause. Therapy begins when the loop, rather than either partner, becomes the object of attention.

Structure. Minuchin added what communication theory lacked: an account of organisation. Families have subsystems (parental, sibling, couple), boundaries (clear, diffuse, rigid), hierarchies, alliances and coalitions. A child who is in a coalition with one parent against the other is not "manipulative"; the child is holding a structural position that the adults' conflict has created. Structure gives the therapist something to see in the seating plan and something to change in the session.

Meaning. The later schools — Milan, narrative, collaborative — pushed the field from pattern to interpretation. If the system is a set of interactions, it is also a set of stories about those interactions. A family that has decided "our son is the problem" has organised its attention, its conversations and its hopes around that sentence. Change the sentence and the interactions move; move the interactions and the sentence loses its evidence.

> **PRACTICAL**
> Choose an ordinary recurring household event: the school run, Sunday lunch, the nightly argument about homework. Draw it as a loop with four to six nodes, each node a behaviour, with arrows showing what follows what. Then mark: where is the negative feedback (what restores the previous state)? Where is the positive feedback (what escalates)? Where does each participant punctuate the loop? Bring the drawing to the lab; you will redraw it after hearing someone else's account of the same event.

1.11a What the evidence says, in one page

Students are often told either that family therapy "works" or that the evidence is contested. Both are unhelpful. Here is what a careful reading of the outcome literature supports, stated with the hedge each claim requires.

Family and couple therapy outperform no treatment, and generally match individual treatment for relational problems. Meta-analyses of couple and family therapy trials report effect sizes in the moderate range — roughly half a standard deviation to eight-tenths — against no-treatment and wait-list controls, with the larger effects for couple distress, child conduct problems, and adolescent substance use. Effects are smaller and less consistent against credible active comparators, which is what you would expect if the specific model matters less than the common structure of good therapy.

The model explains less of the variance than the alliance. The most robust finding in psychotherapy research, replicated across modalities, is that the therapeutic alliance predicts outcome better than the technical ingredients of any particular school. In couple

and family work the alliance is more complicated than in individual work, because there is more than one alliance to build and they can pull against each other: siding with one partner to engage them alienates the other, and a determined even-handedness can leave both feeling unheard. This is why Week 6 of this book spends two sessions on joining, and why your assessed skill in this course includes your ability to hold two alliances at once.

Common factors do a lot of the work. Client factors (severity, chronicity, social support, motivation), the alliance, the therapist's skill and the expectancy generated by a credible rationale account for a large share of change. The practical implication is not that technique is irrelevant but that technique is delivered through a relationship, and a technique delivered through a poor relationship is inert or harmful.

Dose and dropout matter more than most trainees think. A substantial proportion of families attend one session and never return; the first session is therefore an outcome event, not a preliminary. Retention is predicted by early alliance, by the fit between the therapist's explanation and the family's own understanding, and by practicalities — travel, cost, timing, childcare — that a therapist who ignores them will mistake for resistance.

Evidence is unevenly distributed across problems. Couple therapy for relationship distress, behavioural parent training for conduct problems, family-based treatment for adolescent eating disorders, multidimensional family therapy for adolescent substance use, and systemic work with chronic illness in children have reasonably strong support. Evidence for some popular ideas — mechanical use of paradox, unexamined "dysfunctional family" typologies, polyvagal explanations treated as established mechanism — is much weaker. Learning to distinguish these is part of professional formation, and MFT 705 and MFT 721 teach you to read the literature rather than the marketing.

> **CAUTION**
> Do not let the common-factors literature become an excuse for not learning a model. "The alliance is what matters" is true and is also the sentence most often used by trainees who have not yet learned to do anything. You need a model precise enough to be taught, supervised and evaluated, and a relationship good enough for the model to reach the family. Neither substitutes for the other.

1.6 First contact: three openings, annotated

The relational frame is not announced, it is enacted. Three openings show what it looks like when a therapist begins relationally in the first five minutes — before any genogram, any diagnosis, any technique.

Opening A — from the referred person to the referral system.

> **THERAPIST:**
> Before we start, tell me how this appointment came about. Who first said something needed to change, and who agreed? **Mrs Okonkwo:** The school called me. His teacher said he refuses to write anything. **Therapist:** And when you told your husband, what did he say? **Mrs Okonkwo:** (pause) He said I should handle it. He is not around. **Therapist:** So the school spoke to you, you spoke to your husband, and here you are with Ebuka. Who else in the family knows you are here today?

Note what the therapist has done. She has not asked about the symptom at all. She has mapped the **referral pathway**, which is itself a relational fact: who is worried, who is not, who carries the problem, who was told and who was not. She has also discovered, in four questions, the sentence that will organise the assessment: he said I should handle it. That sentence is not a complaint about a husband. It is a description of a household in which one adult holds all the worry, and a child's symptom sits in the space where the other adult would be.

Opening B — from the symptom to the sequence.

> **THERAPIST:**
> Tell me about the last time it happened. Not the worst time — the most recent. **Mrs Okonkwo:** Yesterday morning. He would not get out of bed. I called him three times, then I started shouting, then he got up and stood in the bathroom for twenty minutes. **Therapist:** And then? **Mrs Okonkwo:** Then I drove him late and I was angry all day. **Therapist:** What did Ebuka do when you were angry all day? **Mrs Okonkwo:** He was very quiet. He even made me tea.

The sequence is now visible: refusal, escalation, compliance, and then **repair initiated by the child**. That is the loop. The therapist did not need to interpret it. She asked for a specific episode and then asked "and then?" until the loop closed. This is the single most useful micro-skill in the first session: **episode, not opinion.** Opinions produce character descriptions ("he is stubborn"); episodes produce sequences.

Opening C — from the individual to the room.

> **THERAPIST:**
> (to Ebuka) Your mum says mornings are hard. If I asked her to describe a good morning, what would she say happens? **Ebuka:** (shrugs) She would be happy. **Therapist:** How would she know? What would she see? **Ebuka:** (long pause) My dad would be there.

Three questions, and the child has produced the formulation. The therapist did not ask "how do you feel about your father's absence?" — a question no nine-year-old answers truthfully. She asked a **behavioural question about a third person's perception**, which is easier to answer and much harder to fake. Involving the child as an informant, not a patient, is a structural act: it moves the child out of the identified- patient position and into the family's information system.

What all three openings have in common. Each one moves the unit of attention one step outward — from the person to the pathway, from the event to the sequence, from the individual to the room — and each does so with a question that is answerable. That is the whole craft of a first session in miniature. The student who can do these three moves in the first ten minutes will get a formulation. The student who cannot will get a history.

1.7 The Okonkwo case, fully worked

Referral. Ebuka, 9, referred by his school for "defiance and refusal to write". Six weeks of refusing to complete written work; two episodes of leaving the classroom. Class teacher describes him as "stubborn and rude". Mother reports he is "fine at home, except mornings". Father did not attend.

What the linear frame produces. A differential diagnosis: oppositional defiant disorder versus a specific learning difficulty (writing), versus anxiety, versus an attention problem. An assessment plan: psychometric testing, teacher rating scales, classroom observation, possibly a paediatric review. A treatment plan: behavioural contract, reward chart, parent training in commands and consequences. This is not wrong. It is incomplete in a way that will matter, because the most important information in this family is not about Ebuka's writing.

What the systemic frame adds. Four moves, in order.

Move 1 — the referral pathway. Who is worried? The school, then the mother. Who is not in the room? The father. Who decided that the mother should manage it? The father, according to the mother. The first structural fact: **the parental subsystem is not operating as a subsystem.** One adult holds the worry, the other holds the distance.

Move 2 — the sequence. Mornings: refusal, three calls, shouting, compliance, lateness, a day of maternal anger, evening reparation initiated by the child. The second structural fact: **the child regulates the mother's emotional state.** He absorbs her anger and then repairs it. A nine-year-old is doing the work of a partner.

Move 3 — the function of the symptom. What would change at home if Ebuka got up cheerfully and wrote fluently? The mornings would be calm; the mother's day would be easier; and the father's absence would become a topic rather than a background condition. There is no evidence that anyone wants the child to be symptomatic. There is good evidence that the symptom sits at the exact point where a marital conversation would otherwise occur.

Move 4 — the wider system. School, church, extended family. In this case the maternal grandmother is mentioned twice and the paternal aunt once. In many Nigerian households the grandmother is a co-parent and a court of appeal; leaving her out of the assessment is not neutrality, it is an omission.

The formulation, in one paragraph. Ebuka's refusal to write is maintained by a morning sequence in which escalation reliably produces compliance and in which the child then repairs his mother's distress. That sequence is itself maintained by a household in which one parent carries all the worry and the other is absent, and by a school system that has defined the problem as a defect in the child. The symptom is not caused by the

marriage, and nobody is to blame; but the symptom is located where the marriage would otherwise be discussed, and it will be difficult to resolve without that conversation becoming possible.

The plan that follows. Conjoint sessions including the father (with an explicit invitation framed as expertise rather than blame: "I need your view of what happens in the mornings; your wife and I are working from one account"). A behavioural component for the mornings, because it works and because success builds the parents' confidence in themselves as a team. A separate conversation with the parents about the marriage, held only when both are willing. Liaison with the school to reframe "defiance" as "a problem we are assessing", which reduces the pressure that feeds the loop.

> **TASK**
> Rewrite this case as you would present it in a multidisciplinary team meeting in forty seconds, without using the word "systemic" and without blaming either parent. Then rewrite it for the parents themselves. The two versions should differ in vocabulary, not in content.

1.8 Where the profession sits

Students enter training with a blurred map of the mental health professions. In Nigeria, as in most places, the practical differences matter for referral, for legal standing, and for what you are allowed to do.

Profession	Core unit	Typical training	What they usually do with a struggling family
Psychiatry	The patient, medically	Medical degree, then residency	Diagnose, prescribe, admit; refer for therapy
Clinical psychology	The individual	Doctorate or master's plus internship	Assess, formulate, deliver individual therapy
Counselling psychology	The individual in context	Master's or doctorate	Individual and group counselling, some couple work
Social work	The person in their environment	Degree plus registration	Case management, child protection, family support
Marriage and family therapy	The relationship	Master's or doctorate in MFT	Conjoint, family and couple therapy; relational formulation

The distinctive contribution is not a technique. Clinical psychologists do behavioural couple work; psychiatrists do family meetings; social workers do family support. The distinctive contribution is the **relational formulation** — the disciplined habit of locating the problem in the space between people, and the training in interventions that change that space. A therapist who can only see individuals will treat a family by treating its members one at a time and will be genuinely puzzled when the symptom returns.

Settings in which the work happens. Outpatient clinics and private practice; hospitals, where family meetings follow diagnoses that reorganise households; schools, where the referral pathway usually begins; NGOs and humanitarian settings, where family

separation, displacement and bereavement are the presenting context; faith settings, where pastors, imams and church counsellors are often the first and sometimes the only port of call; and courts and child protection services, where the therapist's opinion may be read by a judge. Each setting changes the ethics (see MFT 704), the assessment (MFT 707) and even the language you use.

Scope of practice. Three boundaries worth stating on the first day of training. First, family therapists do not prescribe; any question about medication is a referral question, and the referral should be made without implying that medication is a failure. Second, family therapists do not give legal advice; they provide clinical opinion and, where required, reports, and they tell families to obtain a lawyer (see MFT 724). Third, family therapists do not practise beyond their competence — and competence is specific: adult couple work is not child work, child work is not forensic work, and neither is trauma processing without specific training (MFT 711, MFT 714).

> **AUDIT**
> Write down, in one sentence each, what you would do and would not do if a family asked you to (a) advise whether the husband can legally take a second wife, (b) tell the court which parent should have custody, (c) prescribe something for the child's sleep. Bring the three sentences to the lab.

1.9 Culture, faith and class in the first meeting

The relational frame is not culturally neutral, and pretending otherwise is its own kind of arrogance. Three adjustments matter from the first session in most Nigerian consulting rooms.

Kinship is wider than the household. A genogram that stops at the nuclear family will miss the grandmother who raised the child for three years, the uncle who pays the school fees and therefore has standing, the aunt who lives with the family and disciplines the children, and the cousin whose presence has changed the sleeping arrangements. Ask who is involved, not who lives there. The distinction between household and family is one of the most practically useful in the whole programme.

Faith is a resource and a referral system, not a rival. Families commonly arrive having already seen a pastor, an imam, a traditional healer or all three. The therapist's task is not to adjudicate but to coordinate: what did the prayer, the deliverance, the fasting, the herbal treatment do? What was helpful? What was frightening? What does the family believe is happening spiritually? A family that believes the problem is spiritual and a therapist who believes it is relational can still work together if the therapist takes the belief seriously as a meaning in the system, which is exactly what it is.

Class shapes what is possible. A recommendation to reduce working hours is meaningless to a family with two jobs and no childcare. A recommendation to attend weekly sessions is meaningless to a family who travel four hours. A recommendation to "talk to your husband" is meaningless in a household where his mother is the decision-maker. Practical recommendations fail not because families are resistant but

because the therapist prescribed from a life the family does not have. The discipline is to ask, before prescribing: what would have to be true in your week for this to happen? The answer is usually the real treatment plan.

> **CAUTION**
> Do not treat cultural deference as consent. A wife who says "I will do whatever my husband decides" may be expressing a genuine value, or may be signalling that she cannot speak safely. The difference is discovered by seeing her alone at least once, and by asking about safety in the standard way — not by assuming either interpretation.

1.10 Six errors that begin in week one

These are the mistakes students actually make in their first role-plays, and each one is a failure of the relational frame rather than a failure of technique.

1. **The single-informant assessment.** You interview the mother, form a picture, and never test it. Fix: no relational formulation without at least two informants, and no family assessment without asking who is missing.
2. **The character formulation.** "He is oppositional." "She is enmeshed." These are descriptions of persons disguised as systemic language. Fix: every formulation must contain at least one sequence ("when X, then Y, then Z") and at least one context ("in the presence of…").
3. **Premature reassurance.** "I'm sure he loves you." It closes the exploration and hands the therapist's opinion to a family that needs its own. Fix: acknowledge the experience, ask for the evidence, keep the question open.
4. **The alliance slide.** You begin to like one member more than another and your questions begin to build the case for that person. Fix: track your questions across a session (record it) and count how many were addressed to each person and whose account each question elaborated.
5. **The technique-first session.** You arrive with a genogram to draw, a technique to demonstrate, a model to apply — and the family's actual concern is never heard. Fix: the first session belongs to the family's account, not to the therapist's plan.
6. **The safety omission.** You conduct a full conjoint assessment without ever asking about violence. Fix: every first session includes a private screen for harm to self, harm to others, abuse of children and intimate partner violence, conducted in a way that makes disclosure possible (see MFT 704 and MFT 712).

> **REFLECT**
> Which of the six are you most likely to commit? Not in theory: in your last conversation with someone who was struggling. Write the answer down. You will be asked to revisit it in Week 14.

1.11 Laboratory: the widening question

The week's practical is a triad drill in which the therapist may only ask questions that include another person. It looks simple and it is not: most students discover that their natural question is a question about an individual, and that widening feels artificial for the first twenty minutes.

Setup. Groups of three. Roles rotate every eight minutes, so each person is therapist once. One person brings a real but non-clinical difficulty — a disagreement with a flatmate, a difficult decision about a sibling, a strain with a colleague. No clinical material, no disclosure that would be regretted.

Rules for the therapist.

1. No statements, interpretations or advice — questions only.
2. Every question must include a third party or a relationship: "who notices…", "what does your sister do when…", "who would disagree…", "what happens between you after…".
3. One question at a time, with silence after each.
4. Eight minutes, then stop whether or not the conversation feels finished.

Observer's task. Two columns on one sheet: **W** (questions that widened the field) and **N** (questions that narrowed it back to the individual). Tally, then quote the single most useful widening question and the single most narrowing one.

Debrief questions. What did the client notice about their own description when it was widened? Which widening felt intrusive, and why? What did the therapist want to say but could not? What happened to the client's tone when a third person entered the story?

Marking rubric (used in the Week 6 skills gate).

Criterion	Weight	What full marks looks like
Proportion of widening questions	30%	More than 80% of questions include another person or a relationship
Question quality	25%	Open, specific, answerable; no double questions; no embedded advice
Pacing and silence	20%	Roughly one question per 30–45 seconds; client speaks more than therapist
Use of the client's words	15%	At least three questions reuse the client's own phrasing
Reflexivity in debrief	10%	Can name what was hard, what was tempting, what they avoided

> **DRILL**
> Record a five-minute role-play on your phone. Count your questions. Then count how many contain a third person. Most first attempts are around one in ten. The target is eight in ten, and it takes about three hours of deliberate practice to get there.

1.12 Reflective task

Write 500 words on the last significant conflict in your own family, then rewrite it from the point of view of the person you understood least. Note what changed in your body as you wrote. Keep both versions; you will return to them in Week 14, when you write your emerging clinical stance, and you will find that the second version contains the therapist you are becoming.

Two rules for this task. Do not show it to the person concerned, and do not bring the person's name into the seminar. The purpose is to develop the capacity to hold more than one punctuation of the same sequence at once — which is precisely what you will ask families to do.

1.13 Self-test

Six questions, answers below. If you cannot answer 4 and 5 without notes, reread 1.5.

1. Define negative and positive feedback, and give one clinical example of each.
2. Why is "punctuation" clinically useful?
3. State the double-bind hypothesis and its current scientific status.
4. What is symptom substitution, and what does it imply for treatment?
5. Name the five claims of the relational frame, in order.
6. What are the four consequences of treating the relationship as the client?

Answers. 1. Negative feedback reduces deviation and restores a prior state (a family pulling a member back from independence); positive feedback amplifies deviation (an argument that escalates through counter-complaint). 2. Because each participant's punctuation makes the other appear to be the cause; the therapist who makes punctuation visible turns blame into a loop both can act on. 3. A person receives contradictory messages at different logical levels, cannot comment on the contradiction and cannot leave the field; it is historically important and not a supported causal explanation of schizophrenia. 4. The observation that removing a symptom is often followed by a new symptom elsewhere in the system; it implies that treatment must change the system, not only the symptom. 5. Behaviour is patterned; patterns are maintained by consequences rather than causes; meaning is negotiated; symptoms can serve the system; the observer is inside the observed system. 6. The unit of assessment, the unit of intervention, the definition of success, and the therapist's position.

1.14 Readings for this week

Read the first two before the lecture; the rest are for the assignment.

- **Bateson, Jackson, Haley & Weakland (1956)**, "Toward a theory of schizophrenia", Behavioral Science. Read it as a historical document and notice how carefully the authors hedge.
- **Watzlawick, Beavin & Jackson (1967)**, Pragmatics of Human Communication, chapters 1–2. The axioms, in the original.

- **Nichols & Schwartz**, Family Therapy: Concepts and Methods, chapter 1. The best single overview of the field's history.
- **Becvar & Becvar**, Family Therapy: A Systemic Integration, chapters 3–5. On cybernetics and second-order thinking.
- **Hoffman (1985)**, "Beyond power and control", Family Systems Medicine. The turn to second-order cybernetics, written by a participant.
- **Minuchin (1974)**, Families and Family Therapy, chapter 2. Structure, in Minuchin's own voice.

1.6a An extended transcript, annotated

Role-plays teach faster than principles. Below is a twelve-turn excerpt from a first session with a couple who present with "we cannot communicate". The therapist is a trainee in her second term. Read the transcript, cover the annotations, and decide what you would have asked next. Then read the annotations.

> **T1 Therapist:** What would you like me to understand first?
> **T2 Folake:** He doesn't talk to me. He comes home, eats, watches television, sleeps. I am alone in this house.
> **T3 Therapist:** (to Dele) What do you think your wife means when she says she is alone?
> **T4 Dele:** I don't know. I work. I am tired. She is not alone, there are three children in the house.
> **T5 Therapist:** Folake, what do you do when he is watching television?
> **T6 Folake:** I stand there. Sometimes I ask him something and he says "mm".
> **T7 Therapist:** And what happens then?
> **T8 Folake:** I get angry. I say something sharp. Then he goes to bed.
> **T9 Therapist:** Dele, when she says something sharp, what goes through your mind?
> **T10 Dele:** That nothing I do is enough. So I stop.
> **T11 Therapist:** And when you stop, Folake, what happens to the anger?
> **T12 Folake:** It grows. And then I cannot sleep and I am tired the next day and the children see me crying.

Annotations.

T1 is a neutral opening that hands the agenda to the couple without inviting either partner to be the complainant. Compare "What is the problem?" — which usually produces one person's problem statement and positions the other as the problem.

T3 is the first systemic move: instead of asking Dele to defend himself, the therapist asks him to **interpret his wife**, which requires him to engage with her experience rather than his own. It also reveals whether he has a theory of her mind at all. He does not: "I don't know" is honest and important.

T5 is an **episode question** and it is doing three things at once. It bypasses the argument about whether she is alone (unwinnable), it specifies a moment (television, evening), and it puts the therapist physically in the room with them. Students who ask "why do you feel

alone?" get an explanation; students who ask "what do you do when he is watching television?" get the behaviour that maintains the loneliness.

T7, "and then?", is the single most productive two-word intervention in family therapy. It builds the sequence one link at a time and stops the therapist from doing the formulating.

T9 takes the same event and asks for the **other partner's internal sequence**, which turns two monologues into one loop. Note that the therapist does not challenge Dele's account ("three children in the house" is not an answer to loneliness, but arguing about it would lose him). She lets the sequence do the work.

T10 is the pivot of the session. "Nothing I do is enough, so I stop" is a complete formulation of Dele's side of the loop, offered by Dele. Nothing the therapist could have said would have been as convincing or as usable.

T11 closes the loop by returning to Folake with the consequence of his withdrawal. At T12 she supplies it: the anger grows, sleep is lost, the children see her crying. The loop is now on the table, described entirely in the couple's own words, and neither partner has been blamed.

What the therapist has **not** done in twelve turns is worth listing, because trainees over-value these moves: she has not asked about the history of the marriage, has not asked how they met, has not asked about sex, has not offered an interpretation, has not normalised, has not asked either partner to change anything, and has not used the word "communication". Every one of those is legitimate; none of them was needed to establish the relational frame in the first ten minutes.

> **PRACTICAL**
> In pairs, take turns being the therapist with this transcript. Cover T3 to T12 and improvise your own next question at each point. Then compare with the original. Where did you ask for an opinion rather than an episode? Where did you reassure? Where did you take a side without noticing?

1.10a Three punctuations of one quarrel

The same sequence can be punctuated at least three ways, and each punctuation licenses a different intervention. Take the quarrel the couple above actually have.

Punctuation 1 — the character story. "He is cold and withdrawn; I am affectionate and he rejects me." What it licenses: an appeal to Dele to change, a diagnosis of Dele (avoidant, depressive, possibly autistic), and a treatment that works on Dele. What it costs: Folake's contribution disappears, Dele becomes a project rather than a partner, and Dele — who already believes nothing he does is enough — experiences therapy as one more arena in which he fails.

Punctuation 2 — the mirror story. "She criticises; I withdraw; she criticises more." What it licenses: a circular formulation, a sequence map, and interventions aimed at the loop (interrupting the escalation, slowing the exchange, rehearsing a different response to

criticism). What it costs: the loop can become an abstraction that hides the asymmetry of power and the real losses — his hours, her sleeplessness, the children's exposure.

Punctuation 3 — the meaning story. "She is asking for evidence that she matters; he is protecting himself from evidence that he fails." What it licenses: work on attachment, on the meanings each assigns to the other's behaviour, and on the moment of turning toward or away. What it costs: meaning work without behaviour change can become insight that never reaches the household.

A competent therapist holds all three and chooses deliberately, usually sequencing them: the loop first (because it lowers blame), the meaning second (because it explains why the loop is so adhesive), and the character story never as a formulation, only as each partner's experience to be listened to. This pluralism — several punctuations available, one chosen for a reason — is what distinguishes a systemic clinician from someone who has memorised a model.

> **DEBATE**
> "Circular formulations are kinder but less true." Fifteen minutes. Note that the strongest argument for circularity is not moral but empirical: linear accounts of complex interaction are almost always wrong about causation, even when they are right about blame.

1.13a Examination question with model answer

Question (20 marks). "Family therapy is a way of seeing, not a set of techniques." Discuss, with reference to the historical development of the field and to at least one clinical example.

Model answer, in note form, with the markers' expectations visible.

Introduction (2 marks). State the claim in your own words: the relational frame is an epistemological commitment — a claim about the unit of analysis and the nature of causation — and techniques are downstream of it. Signal the structure of the answer: history, epistemology, clinical example, limits.

History (6 marks). Three research programmes converged on the same failure: Bateson's group and the double bind; Lidz and Wynne on family organisation around psychosis; the child guidance movement and symptom substitution. The aetiological claims of the 1950s were largely wrong and you should say so; what survived is the methodological insight that sequences carry information individuals do not. Trace the schools as answers to "what is the unit?" — Palo Alto (the sequence), Bowen (the generations), Minuchin (the structure), Milan (the meaning), Milwaukee (the solution), Adelaide (the story).

Epistemology (6 marks). Circular versus linear causality; feedback and homeostasis; punctuation; second-order cybernetics and the therapist's position. Use Watzlawick's axioms precisely — content and relationship levels, digital and analogic communication — and show what each buys clinically. Demonstrate that you understand the difference between an ecological claim (the system is the unit of assessment) and a causal claim (the system causes the symptom), and reject the second.

Clinical example (4 marks). The strongest answers use one case throughout rather than several in passing. The Okonkwo case works: a nine-year-old's school refusal, a morning sequence, a parental subsystem that is not operating, and a formulation that no single-informant assessment could have produced. Show what the relational frame let you see and what it let you do.

Limits (2 marks). Name them honestly: the frame can obscure individual biology, trauma and temperament; it can be used to distribute responsibility in ways that leave an abusive member unaccountable; it can become a language that describes loops without ever changing them. Note the feminist critique of neutrality and the evidence that the alliance — not the model — predicts much of the outcome.

Common ways to lose marks. Describing techniques without explaining the frame; treating "systemic" as a synonym for "family"; repeating the double-bind hypothesis as if it were current; failing to give a clinical example at all; writing about circular causality without ever defining feedback.

1.13b What "good" looks like at the end of week one

A checklist you can use on yourself before Week 2. Tick only what you can do without notes.

- ■ Explain, in ninety seconds, why the field moved from the individual to the relationship, using three historical examples and one honest caveat about the science of the 1950s.
- ■ Define negative and positive feedback and give a clinical example of each that you have actually seen.
- ■ Explain punctuation to a couple without using the word "punctuation".
- ■ Turn three opinion questions into episode questions, out loud, without hesitation.
- ■ State the four consequences of treating the relationship as the client, and give one practical implication of each for a first session.
- ■ Describe the difference between a linear and a circular formulation using the same case twice.
- ■ Name the three professional boundaries from 1.8 and what you would say to a family that crossed them.
- ■ Ask about safety in a first session without it sounding like an accusation or an afterthought.

Week 2 — Linear and circular causality

2.1 The lecture in ninety minutes

Last week asked who is the client. This week asks a harder question: **what kind of thing is a cause?** It is the single most consequential idea in the programme, because everything downstream — the formulation, the questions you ask, what you count as progress, how you write a report for a court — depends on whether you think in lines or in loops.

Students usually arrive believing they already understand this. They can define circular causality in a sentence: "each person's behaviour causes the other's." Then they conduct a role-play in which every question they ask is a search for who started it. The gap between being able to say it and being able to do it is the work of this week.

Minutes	Segment	What the lecturer does	What students do
0–10	Two sentences	Reads a case in a linear account, then in a circular account	Mark every causal verb in both
10–25	The philosophy nobody teaches	Aristotle's four causes, Hume, Mills, why "cause" is plural (2.2)	Sort five clinical explanations into cause types
25–45	Feedback, formally	Loop diagrams, delay, gain, thresholds (2.3–2.4)	Draw the same family as two different loops
45–60	Punctuation, again	The same quarrel punctuated three ways (2.5)	Identify their own punctuation habit
60–75	Circular questioning	Live demonstration, then triads (2.6)	Ask three circular questions each
75–90	Hypothesis discipline	How to hold a hypothesis without becoming its servant (2.7)	Write one hypothesis, then one disconfirming question

The lecture has a deliberate shape: philosophy first, engineering second, clinical craft third. Students who skip the philosophy conclude that circular causality is a nicer way of saying "we are all responsible". Students who skip the engineering cannot tell a loop from a list.

2.2 What a cause is: four traditions you will use without knowing

Family therapy inherited a muddle about causation and never fully cleaned it up. The muddle is worth untangling, because most of the field's disagreements about evidence, responsibility and blame are disagreements about what counts as a cause.

Aristotle's four causes. Aristotle distinguished the material cause (what something is made of), the efficient cause (what set it in motion), the formal cause (the pattern it realises) and the final cause (the purpose it serves). A wooden chair: wood, a carpenter, a design, the purpose of sitting. Now apply the scheme to a symptom. A boy's school refusal: his anxiety (material), a frightening event (efficient), a pattern of family accommodation (formal), and the function the refusal serves in the household (final). Most clinical arguments are people defending one of these four while denying the others. The systemic tradition is chiefly interested in the formal and final causes — the pattern and the function — which is exactly why it seems to slight the efficient cause (the event that started it) and infuriates colleagues who want to know what happened.

Hume and the regularity theory. A cause is an event constantly conjoined with another, temporally prior, and contiguous with it. This is the model implicit in most medical training and in most of what patients believe. It works well for infections and badly for families, because in families the "effect" is usually also the "cause", arriving before and after in alternation.

Mill's methods. Agreement, difference, concomitant variation. These underwrite the randomised trial, and they are why the trial remains the best tool for the question "does this treatment cause improvement?" They are much weaker for the question "what maintains this pattern in this family?", where the relevant variation cannot be manipulated.

Systems and constitutive causation. In a system, the state of each element is partly constituted by its relations to the others. A thermostat is not caused to switch by the temperature in the way a billiard ball is caused to move by another; the switching is what the system does. Family therapists use this sense when they say that the pursuit and the withdrawal "cause" each other: not that A pushes B and B pushes A in a chain, but that the two behaviours are what they are in virtue of the loop they form.

> **EXERCISE**
> Take the sentence "his drinking broke up the marriage". Rewrite it four times, once for each Aristotelian cause. Then decide which of your four sentences a court would want, which a wife would want, and which would actually help in session.

Two consequences follow, and students should write them down.

First, circularity is not a moral claim. "We are both in the loop" does not mean "we are equally responsible", and it certainly does not mean "the victim is complicit". A man who is violent and a woman who placates him are in a loop, and only one of them is doing the violence. Confusing description with responsibility is the most serious error in the systemic literature, and the feminist critique of the 1980s — developed by Hare-Mustin, Goldner, Bograd and others — was right about it. The corrective is not to abandon circularity but to pair it with an explicit account of power: who has the resources, who can leave, who is believed, whose account is treated as the account.

Second, circularity is a stance, not a finding. You cannot observe a loop from outside; you can only offer a description of a loop from a position inside it. That is second-order cybernetics in one line, and it means your circular formulation is a hypothesis with an author, not a fact with a discoverer.

2.3 Feedback, formally

The engineering vocabulary is not decoration. It gives you words for things you can otherwise only gesture at, and the words make the difference between a formulation that guides intervention and one that merely sounds sophisticated.

Loop, not line. A loop has a variable, a sensor, a comparison with a reference value, and an effector. In a family: the variable might be proximity between the couple; the sensor is each partner's attention to the other; the reference value is an implicit standard of how close they should be; the effector is the behaviour that moves them. This sounds mechanical and is not: the reference values are moral and historical, inherited from the families the partners grew up in.

Negative feedback. Deviation is detected and reduced. The system returns to its set point. Clinically: the family that pulls the adolescent back whenever she separates; the

couple whose argument always returns to the same unresolved sentence; the mother whose anxiety rises when the household is calm, and who finds a problem to restore the familiar tension. Note the crucial point: **negative feedback produces stability, and stability is not always health.**

Positive feedback. Deviation is amplified. Clinically: the argument that escalates because each response is experienced as an attack; the affair discovered, denied, discovered again; the child's school refusal that produces maternal anxiety that produces more refusal. Positive feedback produces change, and change is not always growth. Most crises are runaway positive loops; most chronic presentations are locked negative loops.

Delay. The most under-taught concept in the field. Systems respond to yesterday's state, not today's. A wife raises a complaint on Monday; the husband responds on Thursday with a small act of help; she has by then concluded he does not care and does not notice the help. Delay is why good interventions often look as though they failed: the change is in the pipeline, and the therapist who panics at session four and changes approach destroys the very feedback they needed to see. Rule of thumb: **give a structural intervention three sessions before judging it.**

Gain. How much a system responds to a small deviation. High-gain systems escalate fast and are fragile; low-gain systems are stable and can be rigid. A high-gain couple needs the therapist to slow the exchange and lower the volume; a low-gain family needs the therapist to introduce novelty, raise the emotional temperature slightly, and use enactment. This is the practical payoff of the vocabulary: gain tells you whether to accelerate or decelerate.

Thresholds and hysteresis. Systems do not change smoothly; they flip. A family tolerates a behaviour for years and then, at a threshold, reorganises around it — the daughter's disclosure that triggers a family schism, the debt that ends the pretence of solvency. Hysteresis means the threshold for going one way is not the threshold for coming back: it is easier to enter a crisis than to leave it, and families often need help to cross back.

> **PRACTICAL**
> Draw one family twice. First as a negative-feedback loop with a labelled set point: what state does this family return to, and how does it get there? Then as a positive-feedback loop: what is amplifying, and what would a threshold for change look like? Ten minutes, both drawings, no words allowed beyond labels.

2.4 Three ways of drawing the same family

The Adeyemi family: Amara, 16, has refused school for six weeks. Her mother has depression and is on medication. Her father was made redundant fourteen months ago and drinks in the evenings. There is a younger brother, 11.

Drawing one — the linear account. Redundancy → paternal drinking → marital conflict → maternal depression → Amara's caregiving and anxiety → school refusal. Every arrow points one way. This drawing is true as far as it goes, and it is what a referral letter

usually contains. It is also useless as a treatment plan, because every arrow points at a cause the therapist cannot move, and the only intervention it licenses is at Amara.

Drawing two — the circular account. Amara's refusal keeps her at home; her presence at home reduces the parents' opportunity to argue; the reduction in argument lowers the mother's distress; the mother's lowered distress makes Amara's presence feel necessary; the father's drinking continues in the space that the parental conflict has vacated. No single arrow is the cause; each arrow maintains the next. This drawing is also true, and it is actionable: the loop has four places where it could be interrupted.

Drawing three — the power account. Overlay resources and legitimacy: who earns, who decides, who is believed, who can leave, whose distress is taken seriously. The father's drinking is not neutralised by being "part of a loop": it is a behaviour with a cost, exercised by the person with the most structural power in the household. The mother's depression constrains her capacity to act. Amara has the least power and the most symptomatic expression. The overlay changes what the therapist says: the mother is not "enmeshed", she is exhausted and unsupported; the father is not "in a loop", he is a man who is drinking and needs to be asked to stop, with the consequences named.

What the three drawings teach. A formulation is a choice of drawing for a purpose. The linear drawing is good for history and for medico-legal chronology; the circular drawing is good for intervention design and for lowering blame; the power drawing is good for ethics, safety and responsibility. A competent clinician keeps all three and says which they are using and why. A weak clinician uses the circular drawing because it is the house style and then cannot understand why the wife in the violent marriage is not helped by a conversation about mutual escalation.

> **CAUTION**
> If there is violence, the circular drawing must never be used in the room as the primary formulation until safety has been established and responsibility assigned. Nothing in this week's material licenses the sentence "you are both keeping the cycle going" to a woman who has been hit. See MFT 704, MFT 712 and MFT 714 for the full protocol.

2.4a A worked loop, with numbers

Circular accounts are often criticised for being unfalsifiable. They need not be. Any loop can be given rough magnitudes, and doing so changes what you notice.

Take Loop B from the Adeyemi case: mother's low mood → Amara's caregiving → mother's temporary relief → Amara's increased responsibility → Amara's anxiety about school → school refusal → more time at home.

Assign a plausible scale, in the family's own terms, and mark how confident you are.

Link	Direction	Rough magnitude	Confidence	Evidence
Mother's mood → Amara's caregiving	↑ caregiving as mood falls	Large	High	Both describe it; it happens most days

Link	Direction	Rough magnitude	Confidence	Evidence
Caregiving → mother's relief	Relief for hours	Moderate	High	Mother: "she settles me"
Relief → Amara's load	Load grows, no reduction	Moderate	Medium	Described across six weeks
Load → anxiety about leaving	Anxiety rises	Large	Medium	Amara: "I can't settle at school"
Anxiety → refusal	Refusal	Large	High	Directly observed
Refusal → time at home	Six extra hours a day	Large	High	Arithmetic
Time at home → caregiving	Loop closes	Large	High	Session observation

Three things follow, and each is a clinical decision rather than a theoretical one.

First, the loop has a strongest link. The largest, best-evidenced arrows run through time at home. That is why the first intervention addressed the timetable — not because school attendance is a moral good, but because it is the highest-leverage node in the loop as drawn.

Second, magnitudes tell you what to expect from partial change. If caregiving fell by half but the mother's mood did not improve, the loop would reassert within days, because the relief arrow would still be live. This is why single interventions on complex loops fail and why therapists conclude, wrongly, that "the family is resistant". The family is not resisting; the arithmetic is against you.

Third, confidence is part of the formulation. Two of the seven links are medium-confidence. A good therapist names them in supervision and tests them with a question, rather than building a plan on them silently. "I am assuming your daughter's being at home makes your days easier; is that right, or is it more complicated than that?" is not a weak question — it is the question that protects the plan.

> **PRACTICAL**
> Take your own loop drawing from 2.3 and add two columns: magnitude (small/moderate/large) and confidence (low/medium/high). Then mark the single link you would test first in the next session, and write the question you would use to test it.

2.5 Punctuation, worked

Punctuation is the process by which each participant marks a beginning in a continuous sequence, thereby assigning cause. It is not a mistake to be corrected; it is the ordinary operation of a mind trying to make sense of a loop it is inside.

The four standard punctuations in couple work.

1. **The complainant's punctuation.** "I complain because he withdraws." The complaint is a response; the withdrawal is the cause. Intervention: ask the complainant what they do immediately before the withdrawal, in a specific episode.

2. **The withdrawer's punctuation.** "I withdraw because she criticises." The withdrawal is a response; the criticism is the cause. Intervention: ask what they notice in their body immediately before they go quiet, and what they are protecting.
3. **The reciprocity punctuation.** "We wind each other up." This sounds enlightened and often conceals the asymmetry: someone is doing more of the escalating, or someone is more afraid.
4. **The system punctuation.** "The loop between us has a life of its own." Useful therapeutically, but it can become a way of talking about the relationship at the exact moment when someone needs to be accountable for a specific act.

How punctuation is changed in the room. Not by argument. Telling a husband "actually you both do it" does not change his punctuation; it recruits him into defending it. Punctuation changes through **specific episode work**: slow the sequence down until both partners can see the moment where their own action occurred, and let them describe it themselves. In practice this means three moves, which students should rehearse until they are automatic:

Move 1 — narrow the time frame. "Not the usual argument. The last one. Tuesday evening. Start at the point where you were both still in the kitchen."

Move 2 — ask for the moment before. "What were you doing in the thirty seconds before he stood up?" This is the question that gives the complainant back their own agency without blaming them, because it is factual rather than evaluative.

Move 3 — ask what they noticed inside. "What went through your mind when she said that?" This converts a behavioural sequence into an emotional one, which is where change becomes possible.

Done well, the couple arrives at a sentence neither could have said at the start: "I go quiet because I think I have already lost, and then she gets louder because the quiet looks like contempt." That sentence is the treatment target for the rest of the work.

> **DRILL**
> Record a five-minute role-play in which one partner describes a recurring argument. Listen back and count how many times the therapist asks "why" (which invites justification) versus "what happened next" and "what did you notice" (which invite sequence). Targets: no more than one "why" in five minutes; at least six sequence questions.

2.6 Circular questioning

The Milan associates — Mara Selvini Palazzoli, Luigi Boscolo, Gianfranco Cecchin and Giuliana Prata — developed **circular questioning** in the late 1970s as the operational form of systemic thinking: questions designed to elicit differences and relationships between people, rather than facts about individuals. The technique is more than fifty years old and remains the most portable skill in the field, because it works inside any model.

The four families of circular question.

1. **Difference questions.** "Who is most upset by this? Who is least? Who would notice first if it changed?" These map the system's distribution of concern and give every member a position.
2. **Behavioural-effect questions.** "When she cries, what does your father do? And then what does your mother do?" These build the sequence in the room, from several points of view.
3. **Comparison questions.** "Is this worse than it was when your mother was alive? Better than last year?" These locate the problem in time and reveal the family's own baseline.
4. **Hypothetical or future questions.** "If this problem disappeared tomorrow, what would be the first thing each of you noticed? Who would miss it?" The second half of that question is the one that produces the information, and it must be asked gently.

Two more devices worth knowing. The **interviewing-the-interviewer** move — the therapist asks one member to comment on what the therapist has been asking, which reveals the observing system inside the family. And the **positive connotation** — describing the symptom as serving the family's cohesion — which was Milan's way of preventing the family from defending itself against the therapist. Both are powerful and both are easy to misuse: positive connotation applied to violence is a serious error, and interviewing-the-interviewer can feel like a parlour trick if it is not clearly in the service of the family's question.

What makes a circular question good. It is answerable; it concerns a third party; it asks for a difference; and it is asked without a preferred answer. The last criterion is the hardest. A question like "don't you think your mother worries more than your father?" is a statement with a question mark, and families can hear the difference even when students cannot.

> **PRACTICAL**
> In triads, one person describes a family difficulty. The "therapist" must ask eight questions, two from each family of circular question. The observer records which questions produced new information and which produced a repeat of information already given. The correlation with quality is brutal and instructive.

2.6a Reframing: the intervention that follows from circularity

: the intervention that follows from circularity

If the loop is the unit, then the fastest way to change a loop is to change the meaning each member assigns to the other's behaviour, because meaning is what selects the response. **Reframing** is the deliberate provision of a new frame that fits the same facts and supports a different response. It is the most portable intervention in the field and the easiest to do badly.

The structure of a reframe. Three parts: the behaviour as observed (unchanging, factual), the intention or function (new, plausible, benign), and the consequence for the

relationship (which now invites a different response). Example, with a father who works late:

> "You are the person who makes sure this family is provided for, and you do it by being away from it. That has worked for the family's security. What your wife is telling me is that it has also taught your son that a man's place is outside the house. Neither of you wants that lesson."

Note what has happened. The behaviour is unchanged: he works late. The intention is newly stated and creditable: he provides. The consequence is newly visible: the son is learning something. The father is not accused and not excused, and a different response is now available to him.

Five rules for a reframe that works.

1. **It must fit the facts.** A reframe the family experiences as false is worse than no reframe: it damages credibility and closes the topic. If you cannot find a genuinely plausible benign intention, do not invent one — use a difference question instead.
2. **It must be delivered as an offer, not a verdict.** "I wonder whether…" outperforms "What is really happening is…" in almost every family, because it leaves room for the family to correct you, which is itself engagement.
3. **It must be addressed to the person whose behaviour is being reframed, in their presence.** Reframing someone to their partner behind their back is gossip with a therapeutic vocabulary.
4. **It must not absolve harm.** You can reframe the function of a behaviour without reframing away its cost. "He drinks to manage a shame he cannot speak" is a reframe; it does not make the drinking acceptable, and the next sentence must say so.
5. **It must be followed by a task.** A reframe without a next step is a compliment. The loop changes when the family does something different, not when it understands something new.

Reframes that students overuse. "You are both doing the best you can" (true and inert). "Your son's behaviour is a message of love" (often false, and it places an impossible burden on the child). "This is a normal stage" (sometimes true, and sometimes a way of not assessing). Each of these is a sentence that makes the therapist feel kind and the family feel handled.

> **DRILL**
> Write three reframes for the Adeyemi case: one for the mother's depression, one for the father's drinking, one for Amara's refusal. For each, check the five rules explicitly, then read it aloud to a peer who plays the family member and tells you whether it landed as an offer or a verdict.

2.7 Hypotheses: how to hold one without being owned by it

A **systemic hypothesis** is a provisional explanation of how the presenting problem is maintained by the relationships in the system, stated so that it can be wrong. Milan's team formalised the discipline: the team forms a hypothesis before the session, tests it through circular questioning, and revises it, keeping several alive at once.

Three properties of a usable hypothesis.

1. **It is relational.** It mentions at least two people and at least one sequence: "when the father criticises, the mother intervenes, and the son then refuses to speak — which returns the couple to a familiar division of labour."
2. **It is testable in the room.** There is a question whose answer would make you doubt it. If no such question exists, you are holding a belief, not a hypothesis.
3. **It includes the function.** What work does the symptom do for the system? The answer may be "very little, and the family is simply suffering", which is a legitimate finding and should be said out loud.

The hypothesis-discipline drill. Write your hypothesis in one sentence. Then write three questions: one that would support it, one that would complicate it, and one that would refute it. Ask all three. Trainees routinely discover that they only ever ask the first. That habit is the beginning of confirmatory practice, and it is the most common failure of experienced therapists as well as novices.

Hypotheses in a team. Where a reflecting team or a live supervisor is available, the discipline is strongest: the team holds the hypothesis, the therapist stays curious, and the family is spared the weight of being interpreted. Where there is no team, the therapist must build the discipline internally — writing the hypothesis down before the session, and reviewing it after, is not bureaucracy; it is the substitute for a second mind.

> **TASK**
> Take the Adeyemi family. Write three competing hypotheses: one structural, one multigenerational, one strategic. For each, name the single question you would ask first and the observation that would make you abandon it. Bring all three to the lab; you will be asked to defend the one you would bet on, then switch.

2.8 The Adeyemi family, worked: from loop to plan

Referral. Amara, 16, out of school for six weeks. Referred by the school counsellor, who writes: "Amara presents with anxiety and possible depression; family reluctant to engage." Mother attends with Amara. Father "working" and unavailable.

Session one, what actually happens. The mother speaks for twenty minutes. Amara sits with her hood up and says four words. The therapist's temptation, named openly in supervision afterwards, was to get Amara talking — the standard move, and the wrong one, because it would have confirmed the family's map in which Amara is the patient and the mother is the escort. Instead the therapist asked three questions, in this order.

1. "Before Amara stopped going to school, what was happening in the house?" Mother: a long account of her husband's redundancy, her own illness, the money.
2. "Who else in the family knows how bad things have been?" Mother: nobody. Her sister "would only worry". Her husband "knows some of it".
3. "Amara, when your mother is having a bad day, what do you do?" Amara, first full sentence: "I stay with her."

The sequence is now visible, and it took three questions. The daughter's presence at home is not only a symptom; it is the household's arrangement for managing the mother's distress in the absence of another adult. The school refusal is doing work.

Formulation. Three loops, nested.

- **Loop A (parental).** Father's redundancy → shame and drinking → mother's depression → father's further withdrawal (he experiences the household as a place where he fails) → more drinking. This is the primary loop, and it is invisible while the child is the identified patient.
- **Loop B (mother–daughter).** Mother's low mood → Amara's caregiving → mother's temporary relief → Amara's increasing responsibility → Amara's anxiety about leaving → school refusal. This loop is maintained by love and by the absence of another adult, not by pathology.
- **Loop C (family–school).** School pressure → mother's shame → pressure on Amara → Amara's withdrawal → school escalation → more pressure. This loop is powered by an institution's timetable, which is why it is so hard to slow.

What the therapist did next, and why. The plan was sequenced deliberately, and the sequencing is the teachable part.

Step 1 — change the unit of the sessions before changing anything else. The therapist asked for a session with both parents, framed as expertise: "I need your husband's view of what the house is like now; I am working from one account and it is not fair to any of you." Framing the invitation as a request for information rather than a summons to be confronted is the difference between a father who attends and a father who refuses.

Step 2 — give the parents a job that is theirs. Not "spend more time together" (an instruction that will be experienced as another failure) but a specific, small, observable task: both parents to handle the next school letter together, before either of them speaks to Amara about it. The task is structural — it creates a parental subsystem in the presence of the therapist — and it is small enough to succeed.

Step 3 — separate the caregiving from the child. With the mother: who else could be with her on a bad day? The answer produced a sister, a neighbour and a church member. The therapist did not ask the mother to need less; she asked the system to supply more adults.

Step 4 — take the school out of the loop. A single call from the therapist to the school counsellor, reframing "anxiety and reluctance" as "a family in a difficult year, with a plan", reduced the institutional pressure that was feeding Loop C. Ten minutes of professional-to-professional contact changed the family's week.

Step 5 — attend to the father's drinking, on its own terms. Not as a symptom of the marriage. The therapist raised it directly, with the mother present and without moralising: how much, how often, what it does, what he thinks about it. He agreed to a referral for assessment. He did not stop drinking in the course of the work. The family still improved, which is a finding worth holding: improvement is not contingent on every problem being solved.

What happened by session eight. Amara returned to school part-time, with a staged timetable agreed between the parents, the school and the therapist. Her caregiving load was shared. The parents' arguments did not disappear; they became discussable in session, which is a modest and real outcome. The father attended six of eight sessions and was, by his own account, "not fixed, but in the conversation".

What a student should take from the case. Four things. The symptom was doing work, and naming that did not blame the child. The primary loop was the parents', and the presenting problem was the route to it, not a distraction from it. The interventions were sequenced, small, and structural. And not everything was solved — which is what real work looks like and what written case studies too rarely admit.

2.9 Annotated transcript: turning a linear account into a loop

The following excerpt is from the first session, minutes 18–26. The mother has been describing Amara's refusal. Read the annotation after each turn.

> **T1 Mother:** She just will not go. I have tried everything. Her father says I am too soft with her.
> **T2 Therapist:** So your husband thinks the answer is firmer handling. What do you think he would say if I asked him why?
> **T3 Mother:** (pause) He would say I let her wrap me round her finger. He thinks I need her at home.
> **T4 Therapist:** What makes you say that?
> **T5 Mother:** Because when she is at home she sits with me. And when she is at school I am on my own all day.
> **T6 Therapist:** So her being at home helps you. And it also worries you.
> **T7 Mother:** (crying) Both. I know it is wrong to say that.
> **T8 Therapist:** It is not wrong. It is the most honest thing said in this room today. Amara, when your mother is crying, what do you do?
> **T9 Amara:** I get her water.
> **T10 Therapist:** And what happens to your own day when you are getting her water?

Annotations.

T2 is the pivot of the whole session and it is worth studying closely. The therapist does not ask "are you too soft?" — which would put the mother on trial — nor does she defend the mother. She asks the mother to **report her husband's view and its reasons**, which does three things: it brings the absent father into the room as a voice rather than a criticism; it treats the mother as a reliable observer of her marriage; and it reveals that she already

knows what he thinks. Families usually do know. The therapist's job is to make the knowing speakable.

T3–T5 produce the formulation from the mother's own mouth. Note that the therapist asks only "what makes you say that?" — five words, no interpretation. The hypothesis (the daughter's presence is serving the mother's isolation) is now the mother's own statement, which is the only form in which it can be worked with. Had the therapist said it, the mother would have had to defend herself against it.

T6 is the **both-and** move: it holds two truths without resolving them. "It helps you and it worries you." This is the linguistic form of circularity, and it is a skill that can be practised: every time you are tempted by "but", try "and".

T7 is the moment of shame, and it must be handled with precision. What the mother fears is that she is being accused of wanting her child ill. T8 does not reassure her ("of course you don't want that") because reassurance would close the opening; it validates the honesty and moves immediately to the child, which distributes the emotional weight and prevents the mother from becoming the identified patient in turn.

T9–T10 bring the child in as an informant about her own behaviour in relation to her mother. T10 is the crucial one: "what happens to your own day when you are getting her water?" It is a question about consequence, addressed to the person with least power, and it invites her to describe her own experience without requiring her to blame her mother. Her answer — that she cannot settle, that she worries, that she cannot face school after a night of listening — completes the loop.

What the therapist avoided, deliberately. She did not ask about the marriage directly in this session. She did not name the father's drinking. She did not offer a diagnosis. She did not reassure the mother that she was a good mother, though she was. Each omission was a decision: the first session's job was to establish that the problem lived between people, and every one of those moves would have pulled it back into one person.

> **DRILL**
> Take turns reading T1–T5 aloud in pairs. The listener must replace T6 with their own "both-and" sentence. Then replace T8 with a sentence that validates without reassuring. Compare. Most attempts either reassure ("you're a good mother") or interpret ("so you see, you need her at home") — both of which close the opening that T7 created.

2.10 Laboratory: the same case, three drawings

The week's assessed practical. Ninety minutes, groups of four, one case per group drawn from the case bank (Appendix to this week).

Stage 1 (20 minutes) — linear account. Each group writes the case as a one-directional chain of at least six arrows, in the form "A → B → C". No loops permitted. This constraint is uncomfortable and instructive: students discover how natural the linear account is, and how much it leaves out.

Stage 2 (25 minutes) — circular account. Redraw as a loop. Rules: every node must be a behaviour or an observable event, not a trait; every arrow must be justifiable from something a family member said; at least one node must be something the therapist could actually change.

Stage 3 (20 minutes) — power overlay. On the same drawing, mark for each member: resources (income, housing, mobility), legitimacy (whose account is believed), and exit options. Then write one sentence stating what the overlay changes about the plan.

Stage 4 (25 minutes) — presenting. Each group presents all three drawings in four minutes. The audience scores on a single criterion: **does the third drawing change the plan?** If the power overlay leaves the intervention identical, the group has decorated rather than analysed.

Marking rubric.

Criterion	Weight	Full marks
Linear account fidelity	15%	Chain is accurate to the referral and contains no smuggled loops
Circular account quality	30%	Nodes behavioural, arrows evidenced, one actionable node identified
Power overlay	25%	Resources, legitimacy and exit named for each member; plan visibly changed
Integration and presentation	20%	Three drawings used together, with the purpose of each stated
Reflexivity	10%	Group can name the drawing they found hardest and why

2.11 Six common errors with circularity

1. **Circularity as absolution.** Using the loop to avoid naming a behaviour that must stop. Fix: pair every circular formulation with an explicit statement of responsibility where responsibility exists.
2. **The endless loop with no entry point.** A drawing with eight nodes and no plausible intervention. Fix: every formulation must nominate the node the therapist will work on first, and why that one.
3. **Loop-washing the history.** "It doesn't matter who started it" is sometimes true and sometimes a way of avoiding a trauma that needs to be named and mourned. Fix: ask the history question, then decide whether to work with it.
4. **Circular questions as interrogation.** Eight questions in a row with no acknowledgement, and a family that closes. Fix: one question, one answer, one acknowledgement; the ratio should feel slow to the therapist.
5. **The hypothesis that cannot lose.** Interpreting every response as confirmation. Fix: write down, before the session, what would count as refutation.
6. **Systems language as a way of sounding clever.** "The system is communicating its homeostatic imperative" is not a sentence that helps anybody. Fix: translate every formulation into language a 14-year-old in the room could follow, and check that you can.

AUDIT

Record one of your own sessions (role-play is fine). Transcribe three minutes. Mark every causal claim you made. Count how many were linear, how many circular, and how many attributed responsibility. If the third number is zero, you have a habit worth breaking.

2.12 Culture, faith and the limits of circularity

Two cautions, both learned the hard way in supervision.

The respectful-withdrawal pattern. In many families, a father's distance is not "coldness" but a culturally scripted division of labour: he is the provider, the mother is the emotional manager of the household, and his non-attendance at a clinic is not a symptom but a role. Reading this pattern as "disengagement" and intervening to increase his emotional involvement can be experienced as an accusation of inadequate masculinity — and it will fail. The better move is to reframe the invitation in terms of his existing role: "I need the person who understands the finances and the school fees, because decisions about Amara's school are partly financial." You are not asking him to become someone else; you are asking him to bring what he already does into the room.

The extended-family triangle. Circular accounts built on the nuclear household routinely miss the grandmother who is the actual decision-maker, the aunt who funds the school, or the uncle whose approval the father needs. When these figures are absent from the drawing, the loop looks like a couple problem and the intervention fails at the point where the real authorisation was required. Ask, always: who else would need to agree for this plan to happen? Then decide whether to invite them, telephone them with consent, or work with the family's account of them.

Faith as an explanation in the system. "This is a spiritual attack" is a causal account, and a serious one. It competes with your circular account, and arguing with it is both rude and ineffective. The systemic move is to treat it as part of the system's explanatory map: who in the family believes it, who does not, what the belief licenses (more prayer, less school), and what the family would accept as evidence either way. Very often the family does not need the therapist to agree; it needs the therapist to take the belief seriously enough to plan around it.

CAUTION

Do not use circularity to soften a disclosure. If a father says "she provoked me" about a physical assault, the correct response is not "so you both escalate". It is to name the act, state that it is not acceptable, and follow the safety protocol. Circular thinking is a tool for understanding, never a substitute for safeguarding.

2.13 Key terms for this week

Term	Definition	Why it matters clinically
Linear causality	A causes B in one direction	Good for history and legal chronology; poor for intervention
Circular causality	Each element both causes and is caused by the others	The basis of loop formulations and of blame reduction
Punctuation	The way each participant marks a beginning in a sequence, assigning cause	The mechanism by which blame is distributed; changeable through episode work
Feedback (negative)	Process that reduces deviation and restores a set point	Explains stability, including stable misery
Feedback (positive)	Process that amplifies deviation	Explains escalation, crisis and sudden change
Set point	The state a system returns to	Named in the family's own words whenever possible
Delay	Lag between an action and its effect in the system	The reason therapists abandon good interventions too early
Gain	Responsiveness of a system to small deviations	Tells you whether to slow down or speed up
Threshold / hysteresis	The point at which a system flips, and the asymmetry of flipping back	Explains why crises are easier to enter than to leave
Systemic hypothesis	A provisional relational explanation, stated so it can be refuted	Keeps the therapist curious rather than certain
Circular question	A question that elicits difference or relationship between people	The core skill of systemic interviewing
Difference question	Who is most/least affected; who notices first	Maps the distribution of concern
Behavioural-effect question	What one person does when another does X	Builds the sequence in the room
Positive connotation	Describing a symptom as serving the system's cohesion	Powerful, and dangerous if applied to violence
Second-order cybernetics	The observer is part of the observed system	Makes the therapist's own effect part of the data
Constitutive causation	Relations constitute the elements rather than pushing them	The philosophical basis of the loop

2.14 Evidence note: does circularity survive contact with the evidence?

Students are right to ask whether a conceptual scheme this old still earns its place. Three honest answers.

As a description of interaction, circularity is not controversial. Sequential analysis of couple interaction — coding who does what, in what order, and what follows — reliably shows reciprocity: negative affect from one partner raises the probability of negative affect from the other, and the strength of that reciprocity predicts relationship satisfaction. Gottman's cascade models and the demand–withdraw literature (Christensen

and colleagues) are, in effect, empirical confirmations that the loop is real. What the literature does not support is the stronger claim that the loop is the cause of the distress, or that both parties contribute equally. Asymmetry is the norm: demand–withdraw patterns are strongly gendered, with women more often in the demand role and men in the withdraw role, and that asymmetry is not an artefact of observation.

As a therapeutic stance, circularity has modest but real support. Interventions that externalise the problem and reduce mutual blame — reframing, positive connotation, circular questioning used to build shared understanding — are associated with improved alliance and reduced dropout, and the alliance is one of the strongest predictors of outcome. This is not the same as saying that circular formulations cure. It says that a formulation the whole family can accept is a precondition for the work, and circular formulations are better at earning acceptance than accusatory ones.

As a theory of causation, circularity is under-specified. The critique from outside the field is that "circular causality" often means merely "correlation over time", and the critique from inside is that the metaphor smuggles in a homeostatic assumption — that systems prefer the status quo — which is not always true and which can make therapists conservative in the face of change that families are ready for. The most defensible position is the one this week has taught: **circularity is a stance for generating hypotheses and questions, not a causal theory to be tested.** Use it to widen the field and lower blame; do not use it to explain the aetiology of anything.

> **DEBATE**
> "Circular causality is a metaphor that has done more good than harm, and it is time to retire it." Twenty minutes. Note that the strongest argument for retiring it is the violence literature; the strongest against is that no better vocabulary for interaction has been found.

2.15 Self-test

Eight multiple-choice items and three short answers. Answers follow.

1. Which statement best captures negative feedback? (a) A process that amplifies deviation (b) A process that reduces deviation and restores a set point (c) A criticism delivered calmly (d) A system's refusal to change
2. "He goes silent because I shout" and "I shout because he goes silent" are examples of: (a) Linear causality (b) Different punctuations of one sequence (c) Homeostasis (d) Positive connotation
3. Which is a difference question? (a) Why do you fight? (b) What do you do when she cries? (c) Who is most upset by this, and who is least? (d) Do you think he is depressed?
4. A couple escalate quickly and the session feels unmanageable. In systems terms the system has: (a) High gain (b) Low gain (c) High delay (d) Strong hysteresis
5. The strongest reason to give an intervention three sessions before judging it is: (a) Families are resistant (b) Delay in feedback loops (c) Therapist fatigue (d) Ethical guidelines

6. Which formulation error does "you are both keeping the cycle going" most clearly risk when there is violence? (a) Over-attributing responsibility (b) Absolving responsibility (c) Confusing content and relationship (d) Ignoring delay
7. A hypothesis that cannot be refuted is best described as: (a) Robust (b) Systemic (c) A belief (d) A difference question
8. Positive connotation should not be used when: (a) The family is angry (b) There is violence or abuse (c) The therapist is unsure (d) The symptom is mild

Answers. 1 b · 2 b · 3 c · 4 a · 5 b · 6 b · 7 c · 8 b.

Short answers (150 words each).

Explain the difference between a linear and a circular formulation, using one family. A strong answer names the direction of the arrows, states that the linear account is not false but incomplete, gives a specific sequence, and shows what each formulation licenses clinically — including that the linear account is the one a court or a school will usually want.

What does the power overlay add to a circular formulation? A strong answer names resources, legitimacy and exit options; explains that circularity describes interaction while power explains whose interaction counts; and gives an example in which the overlay changes the intervention (for instance, from "reduce mutual escalation" to "name the violence, safety-plan, and refer").

Why can a good hypothesis make a therapist worse? A strong answer describes confirmatory questioning, the loss of curiosity, the family's experience of being interpreted, and the discipline of writing down a refutation condition before the session.

2.16 Reflective task

Think of a recurring difficulty in your own family. Write two paragraphs. In the first, punctuate it the way you usually do — that is, with the other person's behaviour as the beginning. In the second, punctuate it from the other person's position, with your behaviour as the beginning. Do not try to be fair; try to be accurate about what each punctuation leaves out.

Then answer, in three sentences: what did you have to believe about yourself to write the second paragraph, and what did you have to give up? Most students find the answer uncomfortable, and that discomfort is the beginning of the capacity to hold a loop without needing to be innocent in it.

2.17 Readings for this week

Read the first two before the lecture.

- **Watzlawick, Beavin & Jackson (1967)**, Pragmatics of Human Communication, chapter 2 — punctuation, in the original. Notice how much of the argument is philosophical rather than empirical.

- **Selvini Palazzoli, Boscolo, Cecchin & Prata (1980)**, "Hypothesizing–circularity–neutrality: three guidelines for the conductor of the session", Family Process. The canonical statement of Milan technique.
- **Bateson (1972)**, Steps to an Ecology of Mind, part III. The cybernetic essays, which are better written than most of what followed.
- **Hoffman (1981)**, Foundations of Family Therapy, chapters 1–4. The best history of the shift from linear to circular thinking.
- **Goldner (1985)**, "Feminism and family therapy", Family Process, and **Hare-Mustin (1978)**, "A feminist approach to family therapy", Journal of Marital and Family Therapy. The critique that circularity without power is not neutral.
- **Christensen & Heavey (1990)** and the later demand–withdraw literature. Read one empirical paper on reciprocity and note what it does and does not establish.

Next: Week 3 — A short history of the field

Week 3 — A short history of the field

3.1 The lecture in ninety minutes

Why does a clinician need history? Three reasons, and they are not decorative. First, models are answers to questions, and you cannot evaluate an answer without the question. Second, the field's current fashions are usually old arguments in new clothes, and a clinician who knows the argument is harder to sell a rebranding. Third, your own professional identity is built from this material: what you call yourself, what you think you are doing, and what you refuse to do all come from a seventy-year conversation.

The lecture is a walk through five waves, with a hard question attached to each.

Minutes	Segment	What the lecturer does	What students do
0–10	Prehistory	The child guidance movement, marriage counselling, social casework	Note which problems each could and could not solve
10–30	Wave 1: communication	Palo Alto, Bateson, Jackson, Haley, Watzlawick	Timeline: place five key texts
30–45	Wave 2: structure and generations	Minuchin, Haley, Bowen, Satir	Draw the same family as each would see it
45–60	Wave 3: meaning	Milan, systemic therapy, positive connotation, circular questioning	Identify what Milan added and what it cost
60–72	Wave 4: solutions and stories	de Shazer, White, Epston, Andersen	Compare two session transcripts
72–85	Wave 5: evidence and integration	Common factors, attachment, neuroscience, feedback-informed work	Argue for what the sixth wave will be
85–90	Set the essay	The comparative assignment (3.14)	Choose two models to compare

3.2 Prehistory: three streams before "family therapy"

Nobody invented family therapy in a single place. Three professional streams were converging by 1950, each with its own methods and its own blind spots.

Child guidance. From the 1920s, clinics in the United States and Britain treated children referred for behavioural and emotional problems. The standard team was a psychiatrist, a psychologist and a social worker: the psychiatrist saw the child, the social worker saw the mother. Fathers were largely absent from the literature, and the mother was usually the object of investigation — a framing that did enormous damage and whose echoes are still audible in referrals today ("mother is over-involved"). The practical legacy is the multi-disciplinary team, which survives in child and adolescent mental health services.

Marriage counselling. From the 1930s, and especially after the Second World War, counsellors began seeing couples together — often in religious or voluntary settings, often with an explicitly moral vocabulary. The American Association of Marriage Counsellors was founded in 1942. Its legacy is the couple as a legitimate clinical unit and, less happily, a tendency to treat marital distress as a communication problem to be instructed rather than a system to be understood.

Social casework. Caseworkers had always worked with families, but as the environment of an individual client rather than as a system with its own properties. The legacy is attention to context — housing, income, kin, migration — that purely psychological models lack, and which this library's cultural chapters depend on.

What the three streams lacked. A theory of interaction. Each could describe a person in a family; none could describe the family as a thing with a structure. That arrived from an unexpected direction: engineering and biology.

3.3 Wave 1: communication (1952–1970)

The founding texts are not clinical. They are cybernetic and anthropological, and that origin still shapes how the field thinks.

Bateson's project. Gregory Bateson — anthropologist, cyberneticist, and a man whose influence is out of all proportion to the number of patients he treated — assembled a research group at Palo Alto from 1952 with Jay Haley, John Weakland and, from 1954, Don Jackson. They studied communication in families with a member diagnosed with schizophrenia, and produced the double-bind paper in 1956. Bateson's wider project was a theory of mind as a property of systems rather than brains: "the difference which makes a difference", information as a pattern in a circuit.

Jackson and homeostasis. Don Jackson, a psychiatrist, founded the Mental Research Institute (MRI) in 1959 and, with Ackerman, the journal Family Process in 1962. His contribution was the application of homeostasis to families: a family is a rule-governed system that maintains itself against change, and symptomatic behaviour can be part of that maintenance. He also introduced **family homeostasis** as a clinical observation: when one member improves, another may worsen, because the system's set point has not changed.

Haley and strategy. Jay Haley's Strategies of Psychotherapy (1963) and Problem-Solving Therapy (1976) treated symptoms as tactics in a power struggle, and therapy as a contest of influence to be won by the therapist's directives. Out of this came **strategic therapy**: prescribing the symptom, ordeals, paradoxical instructions, and a therapist who takes responsibility for change. Haley's later collaboration with Cloe Madanes produced a body of technique that is unfashionable and still effective in the right hands.

Watzlawick and the axioms. Pragmatics of Human Communication (1967) gave the field a vocabulary: one cannot not communicate; content and relationship levels; punctuation; digital and analogic codes; symmetrical and complementary relationships. Two of these are now so embedded in clinical language that students use them without knowing their source, which is exactly what a foundational vocabulary looks like.

What wave 1 got right. The unit is the sequence. Communication is not a skill laid on top of a relationship; it is the relationship. Change the sequence and the symptoms move.

What wave 1 got wrong. It was often manipulative (therapists withholding information, using paradox on unconsenting families), it was blind to power and gender, and its central aetiological claim about schizophrenia was false. Students should be able to say all three without either dismissing the wave or defending it.

3.4 Wave 2: structure and generations (1965–1980)

Wave 2 made the field clinical. It brought families into rooms, built training programmes, and produced the models still most widely taught.

Minuchin and structure. Salvador Minuchin, trained as a paediatrician and psychoanalyst, worked with delinquent boys at the Wiltwyck School and then led the Philadelphia Child Guidance Clinic from 1965. His observation was that these families were not deficient in insight or motivation; they were **organised** in ways that made the child's symptom functional — a weak parental subsystem, a child in coalition with one parent against the other, boundaries too diffuse or too rigid. Families and Family Therapy (1974) remains the clearest book in the field. His techniques — joining, enactment, unbalancing, restructuring, mapping — are the backbone of MFT 706.

Haley in Philadelphia. Haley joined Minuchin in 1967, and the exchange between structure and strategy produced much of what is now standard: the therapist as an active organiser of the session, the deliberate use of directives, the assumption that change in behaviour precedes change in understanding.

Bowen and the generations. Murray Bowen, at the Menninger Clinic and later Georgetown, built the most elaborate theory in the field: **differentiation of self** (the capacity to stay emotionally connected while thinking for oneself), **triangles** (the three-person unit as the molecule of emotional systems), **multigenerational transmission** (patterns passed down and intensified across generations), **family projection** (a child recruited to carry the family's undifferentiation), **emotional cutoff**, and **societal regression**. Bowen's therapy was explicitly about the therapist's own family of origin: trainees were required to work on their own genograms and to re-enter

relationships with their families of origin as the core of their training. That requirement is why this library asks you to do a reflective task every week.

Satir and the person. Virginia Satir brought warmth, communication training, family sculpture and an explicitly humanistic value system. Her categories — placator, blamer, computer, distractor, leveller — are crude and useful: students recognise them instantly, and so do families. She also insisted, against the cybernetic mainstream, that the therapist's own personhood is the instrument, a claim that the common-factors literature later supported with less charm.

The experiential wing. Carl Whitaker's symbolic-experiential therapy — deliberately unsettling, co-therapy with a live team, the therapist's own fantasies used in the room — represents a road the field mostly did not take, and its influence survives in the use of self and in the practice of working with the therapist's own reactions in the moment.

> **PRACTICAL**
> Take the Okonkwo family. Write four sentences describing the same session as Minuchin, Bowen, Satir and Whitaker would each have conducted it. Where do they differ: in the seating, the first question, the target of change, or the theory of what a symptom is?

3.5 Wave 3: meaning (1975–1990)

The Milan group — Selvini Palazzoli, Boscolo, Cecchin and Prata — began as strategic therapists and became something else. Their Paradox and Counterparadox (1978) described a team working behind a one-way mirror, long intervals between sessions, and interventions delivered as letters: rituals, positive connotation, and the **prescription of the symptom**. The style was hierarchical, elegant and, in retrospect, ethically awkward.

By the early 1980s the group had split, and the second phase — associated especially with Cecchin — turned from intervention to **meaning**. The key concepts: **hypothesising** (a provisional explanation, held lightly and revised), **circularity** (the interviewer's discipline of asking about differences and relationships), **neutrality** (curiosity about all positions rather than indifference), and **circular questioning** as the primary instrument. The therapist's job was less to change the system than to introduce information that the system could not have generated itself.

Second-order cybernetics entered here, through the work of Heinz von Foerster and the writing of Lynn Hoffman ("Beyond power and control", 1985). The observer is part of the observed system; there is no Archimedean point; therapy is a conversation in which the therapist's own descriptions are part of what is described. This is intellectually attractive and clinically dangerous, because a therapist who is only curious can leave a violent household undisturbed. The corrective came from the outside, in wave 5.

The reflecting team. Tom Andersen, in Tromsø, Norway, developed the reflecting team (1985–87): the team observes and then discusses in front of the family, with the family listening, in a tone of tentative curiosity. The move distributes expertise and gives the family the experience of being thought about rather than judged. It is one of the few techniques of this period that has survived intact and is used across models.

3.6 Wave 4: solutions and stories (1980–2000)

Two movements, both impatient with pathology, both popular with trainees because they feel respectful, and both vulnerable to the criticism that respect can shade into avoidance.

Solution-focused brief therapy. Steve de Shazer, Insoo Kim Berg and colleagues at the Milwaukee Brief Family Therapy Center built an approach around the question "what would be different if the problem were solved?" — the **miracle question** — plus **scaling**, **exceptions** (times the problem did not happen), and **coping questions**. The stance is minimal-intervention, client-as-expert, and oriented to the next small step. The strength: engagement, brevity, respect. The weakness: when there is trauma, violence or serious mental illness, a focus on solutions can leave the worst of the situation unaddressed, and the therapist may not notice, because the conversation is pleasant.

Narrative therapy. Michael White and David Epston, in Adelaide and Auckland, worked from the premise that people organise experience through stories, and that problems are maintained by **dominant stories** that narrow what a person can be. The techniques: **externalisation** ("the problem is the problem, the person is not the problem"), **unique outcomes** (moments that contradict the dominant story), **re-authoring**, **definitional ceremonies**, and the use of letters and documents. The political commitment is explicit: narrative therapy is designed to notice and resist the power of diagnosis, institutions and cultural norms — including, in White's later work, the norms that shape gender and family life.

Collaborative and dialogic practice. Harlene Anderson and Harry Goolishian's collaborative language systems approach, and the open-dialogue work of Jaakko Seikkula in Finnish Western Lapland, pushed further: therapy as a conversation among many voices, the therapist as a participant in a dialogue rather than an agent of change. Open dialogue's outcomes with first-episode psychosis have been widely discussed and are methodologically contested; students should read the primary papers rather than the summaries, which are usually more enthusiastic than the data.

> **DEBATE**
> "Solution-focused and narrative therapy are the most ethical models in the field, and the least adequate for serious risk." Twenty minutes. Both halves of the sentence need defending.

3.7 Wave 5: evidence, attachment and integration (1990–present)

Contemporary practice is not a single wave but a convergence, and students should be able to name its parts.

The evidence movement. Manualised treatments, randomised trials, meta-analysis, and the rise of evidence-based practice as a policy requirement. Its achievements are real: couple therapy for relationship distress, behavioural parent training, family-based treatment for anorexia, multidimensional family therapy for adolescent substance use, and systemic work in paediatric chronic illness all have credible support. Its costs are also real: manualisation can strip the flexibility that made systemic work effective; trials

under-represent the families who actually attend public clinics; and the hierarchy of evidence privileges questions about average effects over questions about mechanism and meaning.

The common-factors turn. Research on the alliance, on therapist effects and on client factors demonstrated that a large share of outcome variance is not attributable to the model. This did not make models irrelevant; it made the fit between model, therapist and family the central question. Feedback-informed treatment — the routine use of session-by-session outcome and alliance measures — is the practical descendant, and it is now a standard of good practice rather than an innovation.

Attachment. Emotionally focused therapy (Sue Johnson, Les Greenberg) rebuilt couple therapy on attachment theory and emotion science. It is the most influential couple model of the last thirty years, and its evidence base — including several randomised trials and meta-analytic support — is the strongest of any couple approach. Attachment thinking also reorganised work with children, fostering and adoption (see MFT 711).

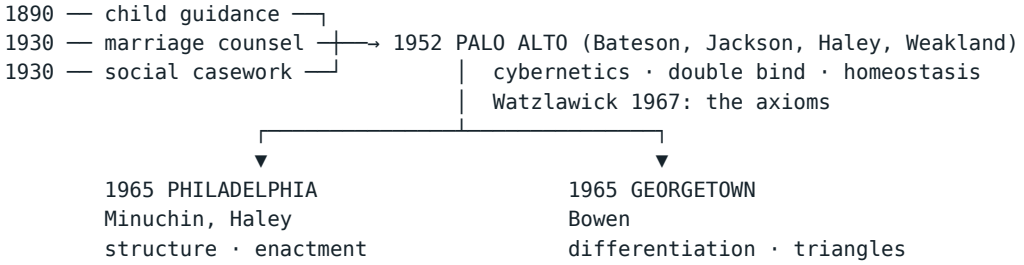
Neuroscience. Findings on the social brain, stress physiology, co-regulation and memory have enriched the field's language and some of its techniques (see MFT 726). The danger is **neuro-pseudo-profundity**: using brain vocabulary as though it explained what it merely redescribes. Polyvagal theory, in particular, is clinically generative and scientifically contested; a professional should be able to use its metaphors while saying honestly that the mechanism is not settled.

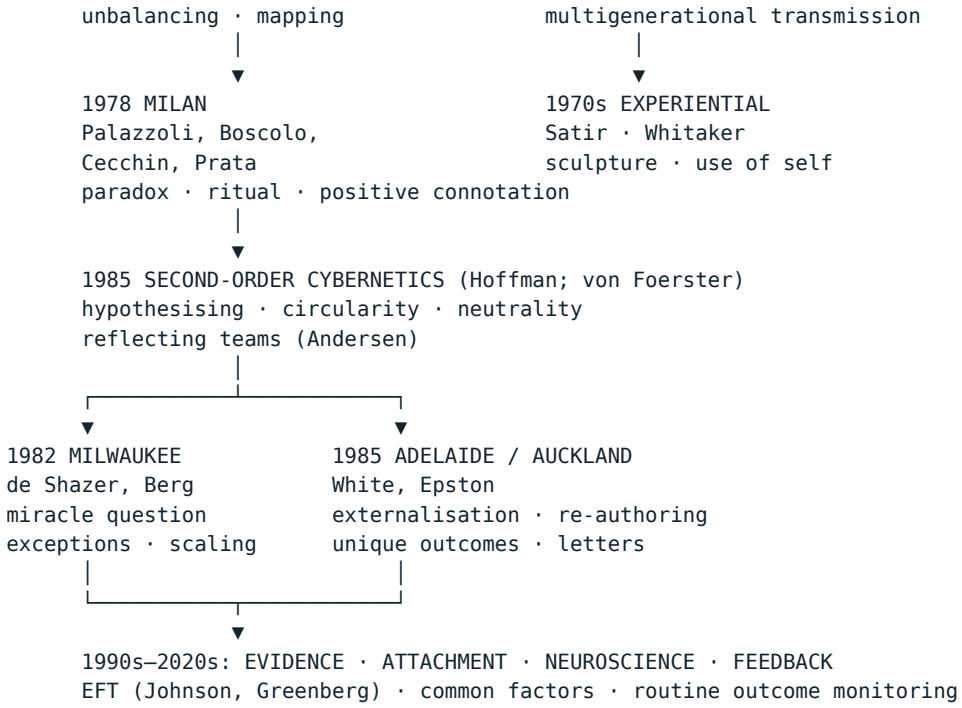
Integration and the common structure. Most practitioners now describe themselves as integrative. The substantive question is not whether to integrate but how: by theory (a superordinate framework), by technique (borrowing tools), by sequence (matching the model to the phase of work), or by feedback (letting outcome data choose). MFT 725 takes this up properly; here it is enough to note that "integrative" is a description of a problem, not a solution.

> **TASK**
> Write 300 words predicting the sixth wave. Defend it with two pieces of evidence from the last five years and one prediction that could be shown to be wrong within a decade. Keep it; you will reread it in Week 14.

3.8 The family tree as a diagram

The history is easier to hold as a picture than as a list. Draw it once, by hand, and you will not need to revise it.





Read the diagram three ways. **Vertically**, as descent: Milan comes out of Palo Alto, solution-focused and narrative come out of Milan's turn to meaning. **Horizontally**, as disagreement: Philadelphia and Georgetown were contemporaries answering the same question differently, and their disagreement about whether the target is structure or differentiation is still unresolved. **Diagonally**, as return: the contemporary interest in attachment is a return to wave 1's question about emotional bonds, with better instruments.

What the diagram leaves out, and why it matters. The feminist critique (Hare-Mustin, Goldner, Walters, Bograd, and later intersectional writers) is drawn as an arrow into the whole picture rather than a branch: it did not produce a school but it changed every school's self-understanding, by insisting that power, gender and violence are not background variables. The same is true of the cultural critique, which this library treats as constitutive rather than supplementary (MFT 708).

3.9 What each wave still gives you

Students often ask, reasonably, which model to learn. The honest answer is that each wave contributes something you will use whatever you call yourself, and that the contributions are not equally large.

Wave	Permanent contribution	What to leave behind
Communication	Sequence, punctuation, feedback vocabulary, the idea that communication is the relationship	Manipulation; treating paradox as a default; the double-bind aetiology
Structure and generations	Enactment, joining, mapping, subsystem and boundary language, the multigenerational lens, work on the therapist's own family	Rigid normative assumptions about "healthy" family form

Wave	Permanent contribution	What to leave behind
Meaning	Hypothesising, circular questioning, reflecting teams, curiosity as a discipline	Neutrality as an excuse for inaction in the face of danger
Solutions and stories	Engagement, brevity, externalisation, respect for the family's own language and values	Avoidance of trauma, risk and diagnosis dressed as respect
Evidence and integration	Outcome measurement, attachment science, feedback-informed practice, training in what demonstrably helps	Manual-worship; neuro-vocabulary substituting for mechanism

The test of a professional. Can you say, for a specific family, why you chose a particular stance and what you would have done differently if you had chosen another? The therapist who cannot answer that question is not eclectic; they are unskilled and lucky.

3.10 The Adeyemi family seen through five lenses

The same family, five ways. This exercise is the heart of the week: it demonstrates that the models are not competing truths but different instruments, each of which reveals something the others hide.

Communication lens (wave 1). The sequence: father criticises → mother defends Amara → father withdraws → Amara refuses school → mother appeals to father → father criticises. Punctuation: father says "she is out of control because you indulge her"; mother says "he is never here and then he interferes". The intervention would be to slow the sequence, mark the punctuation, and change one link — for example, agreeing that criticism happens only in a scheduled conversation. What this lens sees: the mechanics of escalation. What it misses: the father's grief about his job, which is the fuel.

Structural lens (wave 2). The parental subsystem is weak; Amara is in a coalition with her mother against her father; the boundary between mother and daughter is diffuse. The intervention: strengthen the parental subsystem by giving the parents a joint task, move Amara out of the executive position, and re-establish the sibling subsystem by giving the brother a role that is not "the easy one". What this lens sees: hierarchy and coalition, and it produces action immediately. What it misses: the meaning of the father's drinking and the mother's depression, which structure alone treats as noise.

Multigenerational lens (Bowen). The father's own father was a proud man who lost everything in a business failure and drank; the father's withdrawal is a repetition he has never named. The mother is the child who managed her own mother's depression and has recruited Amara to the same role — family projection in progress. The intervention: a genogram across three generations, work on the father's relationship with his own father, and coaching the mother to differentiate rather than to manage. What this lens sees: the timeline that explains why this family, now. What it misses: the immediate sequence, and the risk that a multigenerational story becomes an excuse ("it was passed down to us").

Meaning lens (wave 3). What does the family believe is happening? Mother: "I am failing as a mother." Father: "I am no longer the man of this house." Amara: "If I leave, something will happen." Brother: "Nobody notices me." The intervention: circular

questions that make these beliefs audible to one another; a reframe that the family is protecting each other rather than failing; a ritual — a weekly family conversation with a fixed structure — that lets new meanings be enacted. What this lens sees: the moral world of the family and its capacity for loyalty. What it misses: the practicalities of a household in financial strain.

Solution-focused lens (wave 4). Exceptions: what happens on the days Amara goes to school? (Her mother's sister visited; the father was at home in the morning.) Scaling: where are you today, 0 to 10, on "this family coping"? What would one point higher look like this week? The intervention: build on the exceptions, deliberately, one small step at a time. What this lens sees: competence, and it engages a family exhausted by being a problem. What it misses: the depression and the drinking, which are not solved by scaling.

The point of the exercise. A formulation is a choice of lens for a purpose, and the competent clinician uses at least two lenses and says which they are using. The incompetent clinician uses one lens, calls it "the evidence-based approach", and cannot explain what it hides.

> **PRACTICAL**
> In groups of five, each person takes one lens and prepares a five-minute opening for the family. Present in role. The observers score each opening on two criteria: does it fit the family's own language, and does it make the next step obvious?

3.10a Four primary sources, read closely

History is learned from secondary summaries at the student's peril. Below are four short extracts, each with what to notice. You do not need to agree with any of them; you need to be able to say what they claim and what they assume.

Bateson, Jackson, Haley and Weakland (1956), on the double bind. The authors describe a mother who, when her child approaches, stiffens; when the child withdraws, she asks why he does not love her. The child cannot win and cannot comment on the game. What to notice: the paper is hedged much more carefully than its reputation suggests — the authors call it a hypothesis and explicitly disclaim a simple causal claim; and the language of "victim" is used without any account of what the mother herself is experiencing. That absence is the seed of the feminist critique, twenty years early.

Watzlawick, Beavin and Jackson (1967), on punctuation. "The nature of a relationship is contingent upon the punctuation of the communicational sequences between the communicants." What to notice: this is a claim about description, not about causation. The authors do not say both parties are equally responsible; they say each behaves as though the other started it. Students who muddle those two claims produce circularity without accountability, which is the error discussed in Week 2.

Minuchin (1974), on structure. "The therapist joins the family in a position of leadership and then uses himself to change the organisation." What to notice: the therapist is inside, and directive. The sentence is unimaginable in the Milan writing that followed, and it is the ancestor of every modern discussion of the therapist's use of self.

White (1989), on externalisation. "The problem is the problem; the person is not the problem." What to notice: the sentence's power is grammatical. It separates a person from a description, which changes what the family can do; and its weakness is that some problems are also acts — violence is not merely a story, and a person who commits it is not only a narrator.

3.11 Annotated transcript: history in one sentence

Students often experience the history as inert because it is taught as a list of names. It comes alive when you see a clinician choose a tradition in a single sentence. Four openings, all defensible, all different.

> **OPENING A (STRUCTURAL):**
> "Before we talk about Amara, I would like to see how the two of you decide something together. Can you choose something small — the next school letter, say — and decide it now, in front of me?"

What this does. It creates the parental subsystem by asking it to act, and it gives the therapist live data rather than reported data. It comes straight out of Minuchin's enactment. What it costs. It is directive, and a family that feels managed will resist it; it presumes the parents can act together at all.

> **OPENING B (BOWENIAN):**
> "I want to understand where this pattern comes from. Could you tell me about your own parents' marriage — what happened in their house when money was short?"

What this does. It places the current difficulty in a multigenerational frame and begins the process of differentiation by asking an adult to think about their family rather than react within it. What it costs. It can feel like a history lesson when the family wants relief, and it can deflect from immediate risk.

> **OPENING C (MILAN):**
> "I am going to ask some unusual questions, and I want each of you to answer for someone else. Amara, who in this family is most worried about your mother? And who is least worried?"

What this does. It builds a map of the system's concern and immediately distributes the problem across relationships. What it costs. It can feel like a parlour game, particularly with a family in acute distress, and it can produce abstract answers that the therapist then over-interprets.

> **OPENING D (SOLUTION-FOCUSED):**
> "Tell me about a day in the last month when things went better than usual — even slightly. What was different about that morning?"

What this does. It starts from competence and gives the family an experience of itself as capable, which is powerful with a demoralised household. What it costs. If the honest answer is "there hasn't been one", the therapist is left holding an approach the family has just refuted; a skilled practitioner will have a bridge ready ("then tell me how you have kept going").

The lesson. Each opening is a hypothesis about what will help, expressed as a first move. Your training should leave you able to make any of the four deliberately, and able to explain afterwards why you chose the one you chose. That capacity — not loyalty to a school — is what "integrative" should mean.

> **DRILL**
> Deliver all four openings to the same peer, in role as the mother. Ask which one made her want to come back, and which one made her want to leave. The answers will surprise you and will differ by person, which is the point.

3.11a How the field's arguments became clinical habits

The history is not over; its disagreements are embedded in the language clinicians use, usually without attribution. Five examples.

"Let's not get into the past" comes from the strategic and solution-focused waves, which treated history as a distraction from the loop. It is right about the futility of archaeology and wrong when the past contains unmourned loss or untreated trauma.

"Everyone in the family is doing their best" comes from Satir and the humanistic wing. It is a useful corrective to pathologising and a dangerous substitute for accountability.

"What would it look like if the problem were solved?" comes from Milwaukee. It is the single most portable question in the field and it will fail with a family for whom the problem is a death.

"That's a boundary issue" comes from Minuchin, filtered through thirty years of training. It is often accurate and almost always unusable as a sentence uttered in the room.

"The system is resisting" comes from the cybernetic wave and is the field's most damaging inheritance. It converts the therapist's failure into the family's pathology. The corrective is a sentence this library repeats: if nothing is changing, check the formulation, the dose, the goal and the fit before you check the family's motivation.

> **AUDIT**
> List five sentences you habitually use in clinical conversation. For each, identify which wave it comes from and what it assumes. Then mark the ones you would be unable to defend if challenged by a colleague, a supervisor or a family member.

3.12 Six errors in using history

1. **The name-drop.** Citing Bateson to a family. Fix: history is for you, not for them; translate every idea into the family's language before it leaves your mouth.
2. **Nostalgia.** Treating the 1970s as a golden age. Fix: name what those clinicians did that would now be unethical, and say why.
3. **Fashion as evidence.** Adopting a model because it is current or because a training was convenient. Fix: ask what evidence supports it for this problem, and what it costs.
4. **Model loyalty as identity.** "I am a structural therapist" as a personality claim. Fix: describe what you do, when, and why; let others attach a label if they need one.
5. **The single-story training.** Learning one model deeply and calling it integration. Fix: the comparative assignment exists for this reason; do it honestly.
6. **Ignoring the critiques.** Teaching the canon without the feminist, anti-racist and survivor critiques. Fix: they are not add-ons; they are the reason the canon changed.

3.12a A supervision case: what happens when the model fails

A trainee presented this in supervision. She had been working with a family of four for six sessions using a structural approach — joining, mapping, enacting, unbalancing — and nothing had shifted. The parents performed the joint tasks in the room and abandoned them at home. The children were polite and blank. The trainee's formulation, reviewed in supervision, was accurate but incomplete: she had described the family's organisation and had not asked what the organisation was for.

Two questions changed the case.

"What does this family believe would happen if they did the task?" The answer, discovered through circular questions, was that both parents believed that acting together would expose the marriage as a pretence. The father's family had a saying: "a house that discusses its problems is a house with no roof." The mother had been raised to keep the peace at any price, and had watched her own mother absorb her father's rage for thirty years. The structural task was, from inside their world, a request to demolish the house.

"What has this family survived together?" A fire, a business collapse, a stillbirth, and a period of separation the children knew nothing about. This is a family with a history of endurance and a rule that endurance is done silently.

What the trainee did in the next session. She abandoned the enactment task for the moment and used a different instrument: she asked the parents, in front of the children, to describe one thing they were proud of having got through together. It took four minutes and produced more contact between them than six sessions of structure. She then reframed the joint parental task in the family's own terms — not "decide together" but "as a family that has survived worse, decide how to handle this one".

The lesson for this week. Models are instruments, and the skill is diagnostic: when an instrument is not working, the question is not "how do I do the technique better?" but "what does the family's world make this technique mean?" That question is historical in its logic — it asks which tradition of family life the family itself belongs to, and which of the

field's available languages can reach them. A therapist who has only one model has only one chance of finding a language the family can hear.

3.13 Culture, faith and the Nigerian lineage

The history above is mostly North American and European, and a student in Ibadan should ask what it has to do with them. Two answers.

First, the critiques from the global South are part of the field's history, not a local appendix. The assumption that the nuclear household is the unit, that individuation is the goal, and that the therapist is a neutral observer have all been contested from African, Asian and Latin American contexts for decades. The Nigerian literature — in counselling, psychology, social work and pastoral care — has its own lineage: community mental health work in the 1970s and 1980s, the integration of traditional healers into referral pathways, pastoral counselling in the churches and alfas and imams in the mosques, and a strong tradition of family-life education. Knowing that lineage is not decoration: it is what allows you to place imported models rather than being placed by them.

Second, the extended family is a theoretical asset, not a complication. Western family therapy spent thirty years discovering that the household is not the system — through work on multigenerational process, on non-resident fathers, on kinship care. A clinician trained in a context where grandmothers, aunts, uncles and cousins routinely co-parent starts with that insight rather than arriving at it. The task is to make it explicit and teachable, not to apologise for it.

A caution. Cultural specificity can become a way of exempting families from scrutiny. "In our culture, discipline means beating" is a claim that requires examination, not deference — the evidence on corporal punishment is clear, and the child's safety is not a Western import. The discipline is to take the family's frame seriously and to hold the professional's obligations, including the legal ones (see MFT 724).

> **REFLECT**
> Which of the five waves most resembles how your own family handles difficulty? Which one most resembles how you were taught to handle it professionally? Write 250 words on the gap between them.

3.13a The Nigerian and African lineage, in more detail

A student who learns only the North American canon will arrive at a professional identity that is borrowed. The local lineage is substantial and should be known.

Community psychiatry and the extension of care, 1960s–1980s. The Aro Mental Hospital in Abeokuta, and its community-based programme from the early 1960s under T. Adeoye Lambo, is a landmark in world psychiatry: it replaced custodial care with village-based care, involved families and traditional healers, and demonstrated outcomes that were, for their time, remarkable. Family involvement was not an added technique; it was the model. Any honest history of systemic practice has to include it.

Traditional and faith-based healing as part of the referral system. Herbalists, babalawos, pastors, alfas and imams are, for most Nigerian families, the first port of call and sometimes the only one. The professional task is not to compete but to understand, coordinate and — where a practice is dangerous — to say so clearly while keeping the family engaged. Research on collaboration between traditional healers and formal services has a long history in Nigeria and West Africa, and the practical lessons are consistent: respect, clarity about what each can offer, and an explicit safety floor.

Family life education and pastoral counselling. Churches and mosques run premarital counselling, marriage enrichment, and considerable informal couple and family work. Many of the referrals you will receive come through these channels, and many of the families you see will have had a pastoral conversation before they had a clinical one. Knowing what those conversations contain — and their limits — makes you a better collaborator and a less anxious professional.

The academic lineage. Departments of psychology, sociology, guidance and counselling, and social work in Nigerian universities have produced a substantial literature on marriage, family structure, child-rearing, migration and mental health. Students writing dissertations (MFT 721, MFT 770) should search it deliberately: the international databases are not neutral archives, and a literature review that cites only Northern journals will have missed the studies with the most relevant samples.

What to take from this. The relational frame is not an import that must be adapted to local reality; it is an insight that local practice arrived at independently, with different vocabulary and different institutional forms. Your task is to learn both lineages and to be able to say, for any particular technique, whether it travels, what it needs to travel, and what it might destroy on arrival.

3.14 The comparative essay: how to write it

Half of the marks for this course come from a comparative essay of 2,000 words on two models applied to one case. Students lose marks in predictable ways, and the essay is easier than it looks if you follow the structure.

The brief. Choose two models from different waves — for example, structural (wave 2) and narrative (wave 4), or Milan systemic (wave 3) and emotionally focused (wave 5). Choose one family from the case bank. Write an essay with the following sections, in this order.

1. **The presenting pattern** (250 words). The family as they arrive: who is worried, who is present, what has been tried, what the referral says. No theory yet. This section establishes that you can describe before you interpret — a skill examiners find conspicuously rare.
2. **Model one: how it sees** (450 words). The model's assumptions: what a symptom is, where pathology lives, what counts as the unit, what counts as change. Then the formulation in that model's terms, in its vocabulary, with the specific evidence from the case that supports it.

3. **Model one: what it would do** (250 words). Two or three concrete moves, in sequence, with the reasoning. Not "use enactment"; rather, "ask the parents to decide the school letter in front of Amara, because the parental subsystem is the target and doing it live gives me better data than asking about it".
4. **Model two: how it sees** (450 words). As above. Resist the temptation to make model two a foil; a comparison in which one model is obviously superior is a failed comparison.
5. **Model two: what it would do** (250 words). As above.
6. **The comparison** (250 words). Where do the two formulations conflict? Where do they merely differ in emphasis? What does each model see that the other cannot? Which would you choose for this family, and on what grounds — evidence, fit, the family's preferences, your own competence, or the setting's constraints? The grounds must be explicit.
7. **Conclusion** (100 words). What you have learned about the relationship between theory and practice.

The six ways students lose marks. Describing models without applying them to the case; applying both models to different aspects of the case so no comparison occurs; declaring one model "more holistic" without evidence; using the models' jargon without translating it; ignoring power, culture and risk; and failing to state a reasoned preference.

Model paragraph, for calibration. Here is a body paragraph at distinction level.

> Structural family therapy locates the problem in the organisation of the family rather than in the individual. Minuchin's claim is that symptomatic children are frequently occupying a position in the hierarchy that belongs to adults — in coalition with one parent against the other, or parentified by an absent adult. This is visible in the Adeyemi family in three places: Amara sits beside her mother and answers questions addressed to her; she describes managing her mother's low mood; and her father, when he attends, is treated by both as a visitor whose opinion arrives from outside the household. The structural formulation is therefore that the parental subsystem is not functioning as a subsystem, and that Amara's school refusal is a stabilising move within that structure. What this formulation gains is a target that can be acted on in the room: the therapist can create the parental subsystem by giving the parents a joint task and observing them attempt it. What it risks losing is the meaning of the father's absence to the father — his own redundancy, his own father's drinking — which structural language can describe as "distance" but cannot explain.

Note what the paragraph does: it names the model's claim, applies it to specific observable evidence, states what the formulation buys and what it costs. That last sentence is what separates a distinction from a competent pass.

3.15 Timeline for revision

Twenty dates, in the order a student should be able to produce them. The examination sometimes asks for a timeline, and always rewards precision about which decade a development belongs to.

Year	Event	Why it matters
1942	American Association of Marriage Counsellors founded	The couple as a clinical unit, before "family therapy" existed
1952	Bateson's group begins work at Palo Alto	Cybernetics enters the study of families
1956	"Toward a theory of schizophrenia" published	The double bind; the field's founding controversy
1959	Mental Research Institute founded (Jackson)	An institutional base for communication research
1960	Ackerman Family Institute, New York	Clinical training in family therapy begins
1962	Family Process founded (Ackerman and Jackson)	The field acquires a journal and a canon
1963	Haley, Strategies of Psychotherapy	Strategic therapy formalised
1965	Minuchin leads the Philadelphia Child Guidance Clinic	Structural family therapy develops
1966	Bowen, "The use of family theory in clinical practice"	Differentiation and triangles published
1967	Watzlawick, Beavin and Jackson, Pragmatics	The axioms; the field's vocabulary fixed
1972	Bateson, Steps to an Ecology of Mind	The cybernetic essays collected
1974	Minuchin, Families and Family Therapy	The clearest clinical text in the field
1978	Selvini Palazzoli et al., Paradox and Counterparadox	Milan systemic therapy; the team behind the mirror
1980	"Hypothesizing–circularity–neutrality" published	The technique of circular questioning formalised
1982	Milwaukee Brief Family Therapy Center founded	Solution-focused work begins
1985	Hoffman, "Beyond power and control"	Second-order cybernetics enters clinical writing
1985–87	Andersen's reflecting team papers	The team discusses in front of the family
1990	White and Epston, Narrative Means to Therapeutic Ends	Narrative therapy consolidated
1990s	EFT trials begin to appear (Johnson, Greenberg)	Attachment-based couple therapy acquires evidence
2000s–	Feedback-informed treatment; routine outcome monitoring	The model matters less than the fit and the feedback

3.16 Self-test

Multiple choice, then short answers.

1. The double-bind hypothesis was developed by: (a) Minuchin and Haley (b) Bateson, Jackson, Haley and Weakland (c) Bowen and Kerr (d) White and Epston
2. "The problem is the problem; the person is not the problem" is a principle of: (a) Structural therapy (b) Strategic therapy (c) Narrative therapy (d) Bowenian therapy

3. Which model is most associated with enactment and unbalancing? (a) Milan systemic (b) Structural (c) Solution-focused (d) Collaborative
4. Positive connotation and ritual are techniques of: (a) The Milan group (b) The Milwaukee group (c) The Philadelphia group (d) The Galveston group
5. Differentiation of self, triangles and multigenerational transmission belong to: (a) Satir (b) Whitaker (c) Bowen (d) de Shazer
6. Second-order cybernetics holds that: (a) Systems have two levels (b) The observer is part of the observed system (c) Feedback operates twice (d) Two therapists should work together
7. The reflecting team was developed by: (a) Andersen (b) Anderson and Goolishian (c) Seikkula (d) Cecchin
8. Which is the strongest evidence-supported couple therapy? (a) Milan systemic (b) Emotionally focused therapy (c) Strategic therapy (d) Collaborative therapy

Answers. 1 b · 2 c · 3 b · 4 a · 5 c · 6 b · 7 a · 8 b.

Short answers.

What did wave 3 add that wave 1 lacked, and what did it lose? A strong answer names meaning, hypothesis, circular questioning and the reflecting team; then names the loss — therapeutic ambition, responsibility for change, and attention to power and risk.

Why is "integrative" a description of a problem rather than a solution? A strong answer distinguishes integration by theory, technique, sequence and feedback, and shows that each requires a criterion for choosing; without a criterion, integration is incoherence with good manners.

3.16a What the examiners look for

A note on assessment for this week's material, since the examination question on history is one of the most predictable and the most frequently answered badly.

Three habits earn marks. Precision about dates and decades (not "in the seventies" when you mean 1978); attribution that is correct (positive connotation is Milan, not Minuchin); and evaluation — what each wave gained and lost, stated in the same paragraph as the description.

Three habits lose them. The list, in which each model is described in a sentence and nothing is compared; the hagiography, in which founders are praised rather than understood; and the anachronism, in which 1950s claims are judged by 2020s standards without noting that the standards themselves are a product of the intervening argument.

A model sentence for evaluation. "Milan's turn to meaning gave the field its most refined interview technique and deprived it, for a decade, of a theory of action; the reflecting team is best understood as an attempt to recover action without abandoning curiosity." A sentence like that demonstrates that you can hold a gain and a loss at once, which is the intellectual skill this course is trying to build.

3.17 Reflective task

Write 300 words on the model you find most attractive and the model you find most repellent. For each, name the reason — and then check the reason against your own family history. Students usually discover that the attractive model is the one that would have helped their family and the repellent one is the one their family was subjected to. That discovery is not a debunking; it is professional formation, and it is the reason supervision exists.

3.18 Readings for this week

- **Nichols & Schwartz**, Family Therapy: Concepts and Methods, chapters 1–4. The best overview; read it as a map, not as a verdict.
- **Hoffman (1981)**, Foundations of Family Therapy. The history written by someone who was there.
- **Minuchin (1974)**, Families and Family Therapy, chapters 1–3. Read for the clarity of the clinical reasoning, not for the model.
- **Bowen (1978)**, Family Therapy in Clinical Practice, chapters 1–5. Dense, idiosyncratic, foundational.
- **White & Epston (1990)**, Narrative Means to Therapeutic Ends, chapters 1–2. Externalisation, in the original.
- **de Shazer (1985)**, Keys to Solution in Brief Therapy. The miracle question and its rationale.
- **Hare-Mustin (1978)** and **Goldner (1985)**, as for Week 2. The critiques that changed the canon.

Next: Week 4 — Systems concepts you will actually use

3.16b Two worked formulations, same family, different waves

The clearest test of historical knowledge is the ability to formulate the same family in two vocabularies without mixing them. Here are two, both about the Adeyemis, both defensible, written to be read side by side.

Structural formulation. The household is organised around a weak parental subsystem. The father is peripherally involved: he is present, but he occupies a visiting position, offering opinions from outside the executive of the family. The mother–daughter boundary is diffuse; Amara functions as her mother's emotional companion and, in the mornings, as the person who absorbs and then repairs her mother's distress. A stable cross-generational coalition has formed. The school refusal is not a symptom that arrived from nowhere; it is the behaviour that keeps the coalition intact by keeping Amara at home. Target of change: strengthen the parental subsystem, move Amara out of the executive position, and re-establish the sibling subsystem so that the younger brother is not recruited into the space Amara vacates. First move: a joint parental task, attempted in the session, on a real decision.

Bowenian formulation. The family's level of differentiation is low in the marital dyad, and the anxiety that this generates has been triangulated through Amara, who is the child most attuned to her mother and therefore the most available for the family projection process. The father's emotional cutoff from his own family of origin — he has not spoken to his brother in nine years, and his father drank — has left him with no vocabulary for the shame of redundancy and no relationship in which to think about it. The mother's over-functioning is a repetition of her position in her family of origin, where she managed her own mother's depression. Target of change: increase differentiation — the capacity of each adult to stay connected while thinking for themselves — and reduce the triangulation of Amara. First move: a three-generation genogram, followed by coaching the mother to define a position to her own mother, and the father to re-open contact with his brother.

What to notice in comparing them. Structural language gives you an immediate, testable, in-session action; Bowenian language gives you a timeline and a theory of why these particular people, and it produces change slowly, through the adults' own development. Neither is a description of the family; both are descriptions of what the clinician has decided to notice. The mark of a trained clinician is that the choice between them is explicit and defensible rather than temperamental.

> **TASK**
> Write a third formulation, 300 words, in the vocabulary of narrative therapy. Then write one sentence stating what your narrative formulation can see that neither of the above can, and one sentence stating what it cannot see.

Week 4 — Systems concepts you will actually use

4.1 The lecture in ninety minutes

Weeks 1 to 3 built the frame. This week supplies the vocabulary you will use in every session for the rest of your career. It is a technical week, and students often find it the most useful of the first month, because for the first time they can describe what they are seeing rather than merely reacting to it.

The organising principle: a concept earns its place only if it changes what you do. Every term in this week is therefore taught with three things attached — what you can observe that tells you it is present, what it costs the family, and what you would do about it.

Minutes	Segment	What the lecturer does	What students do
0–10	The test of a concept	Two formulations, one with and one without usable concepts	Decide which they could act on
10–25	Boundaries and subsystems	Live mapping with a volunteer family of dolls	Map a family from a transcript
25–40	Triangles, coalitions, detouring	The three-person diagram, drawn and then applied	Identify the triangle in a case

Minutes	Segment	What the lecturer does	What students do
40–55	Rules, roles, myths, secrets	The invisible rule exercise	Write three rules from a family description
55–70	Communication concepts	Metacommunication, levels, symmetry and complementarity	Mark levels in a four-line transcript
70–85	Mapping the Adeyemis	Draw the map, defend every symbol	Draw and defend in pairs
85–90	Set the lab	The structural map assessment	Prepare a map for next week

4.2 What makes a concept usable

A concept is usable when it meets four criteria.

1. **Observable.** You can point to something in the session that would count as evidence. "Enmeshment" fails this test in most students' hands; "mother answers questions addressed to her daughter, three times in ten minutes" passes it.
2. **Relational.** It describes something between people, not inside one person. "Triangulation" passes; "manipulative" fails.
3. **Actionable.** It tells you where to intervene, or at least where to look next.
4. **Falsifiable.** You can be wrong. If no observation would make you withdraw the description, you are not describing the family; you are describing your own frame.

The discipline of this week is to translate every concept into observables before you use it in a report, a supervision or a session. It is the difference between a clinician and someone who has read about clinicians.

4.3 Boundaries

A **boundary** is the set of rules that governs who participates in what, and how close members may be. It is not a wall; it is a permeable membrane that regulates contact, and it exists between subsystems (parent/child, couple/siblings), between the family and the outside (school, kin, services), and inside individuals (self/other, thought/feeling).

Minuchin's three-way classification is still the most useful, provided you replace his normative tone with observational criteria.

Type	What you observe	Typical cost	Typical presentation
Clear	Adults decide adult matters; children are informed appropriately; each member has privacy and a voice	None inherent; flexibility is the point	Families who manage difficulty without losing function
Diffuse	Members answer for each other; privacy is treated as secrecy; distress in one member is felt by all and acted on by all	Chronic anxiety; difficulty individuating; a child recruited as confidant	"We are very close" families in which one adolescent cannot leave

Type	What you observe	Typical cost	Typical presentation
Rigid	Information is compartmentalised; emotion is not discussed; contact is formal or distant	Isolation; secrets; children who cannot read emotion	"We don't have problems" families who present late and in crisis

Observational markers of a diffuse boundary. One member finishes another's sentence; a parent answers a question put to a child; the family reports that everyone knows everything; a member's mood is described as the household's weather; there is no closed door in the house; alliances are assumed rather than chosen.

Observational markers of a rigid boundary. The family reports that a member "has never been one to talk about feelings"; there is a topic nobody names; the parents describe the marriage as "not the children's business" and also cannot discuss it with each other; siblings are estranged; contact with extended family has been cut after a dispute.

Two cautions. First, boundaries are culturally specified. A household in which three generations share sleeping space, income and child-rearing is not enmeshed by definition; the question is whether its members can think, disagree and move. Second, boundaries fluctuate with circumstance: after a death, a migration or an illness, a family will temporarily close ranks, and that is adaptive. Diagnose a boundary only when it is stable across contexts and costly to someone.

> **PRACTICAL**
> Listen to five minutes of any family conversation (a recorded role-play is fine). For each member, count: (a) times they answered for another person, (b) times they were interrupted, (c) times they were asked directly and (d) times they spoke without being asked. The four counts give you a boundary profile in numbers, which is worth more than an adjective.

4.4 Subsystems and hierarchy

Families are organised into **subsystems**: the couple, the parents, the siblings, each individual, and the extended kin who function as a subsystem even at a distance. Each subsystem has its own rules, its own language and its own legitimate authority.

Hierarchy is the arrangement of authority and responsibility. In families with children, the parental subsystem carries executive authority: decisions about money, schooling, discipline, safety and contact with the outside world. That authority is not a value judgement about who matters; it is a functional necessity, because someone has to decide, and children cannot.

Three common structural problems, with their observable signs.

Weak parental hierarchy. The child decides, negotiates or vetoes; parents ask the child's permission; a child is the family's spokesperson; discipline is inconsistent because it is negotiated. Cost: the child carries authority without the capacity for it, which produces anxiety, not confidence.

Cross-generational coalition. One parent and one child form a stable alliance against the other parent. Signs: the child knows about the marriage; the child takes one parent's side in arguments; the other parent is treated as an outsider in discipline. Cost: the child loses the parent they are allied with as a parent — the relationship becomes a partnership, and the child's own development is mortgaged.

Parentification. A child takes on practical or emotional care of a parent or siblings. Signs: the child cooks, organises medication, translates, manages the household's feelings. Cost: precocious competence alongside suppressed need; these children present in adolescence or adulthood with anxiety, resentment or a collapse when they finally cannot carry it. Note carefully: parentification is not always harmful. Many children in many households take real responsibility and thrive. The harm correlates with the absence of recognition and support, not with the task itself — a distinction that matters in cultures where children's contribution is normative.

4.5 Triangles, coalitions, detouring: three different things

Students conflate these constantly, and the conflation produces bad formulations. They are not synonyms.

A triangle (Bowen) is a three-person emotional configuration that is the smallest stable relationship system. Two people under tension recruit a third to manage it; the third may be a person, an activity, a substance or a symptom. Triangles are normal and universal; the question is how rigid and how costly they are. Observable signs: conversation about a third person whenever two are together; a member who is always discussed and never present; a symptom that appears when the couple's tension rises.

A coalition (Minuchin) is an alliance of two against a third that crosses generational lines. It is a structural fact, not an emotional process: parent and child against the other parent. Observable signs: seating; who looks at whom when a topic is raised; who defends whom; who is described as "the one who understands".

Detouring is a specific manoeuvre: the couple's conflict is displaced onto a third party — usually a child — so that the couple never has to fight. Two forms: **detouring-attacking** (the child is defined as the problem and both parents unite against them) and **detouring-supportive** (the child is defined as fragile and both parents unite to protect them). Observable signs: the couple's disagreements reliably become conversations about the child; the child's symptoms appear at moments of marital tension; the parents agree warmly about the child and coolly about everything else.

Why the distinction matters. A triangle is a process you interrupt with questions; a coalition is a structure you change with tasks and seating; a detour is a function you name, carefully, when the couple is ready to hear it. The same family can have all three, and the sequence in which you address them is a clinical judgement, not a doctrine.

> **DEBATE**
> "Detouring is a useful concept and a dangerous accusation." Fifteen minutes. Note that the danger is greatest when the child is present: any formulation the family hears as "your child's illness is your marriage" will mobilise defences and may harm the child's relationship with the therapist.

4.5a A triangle worked, carefully

Take a triangle that is present in the Adeyemi household and show how to establish it without asserting it.

The evidence. In the first session, four things happened. The mother described the father's drinking and then, within the same breath, described Amara's sensitivity: "she takes everything to heart, she is like me." When the therapist asked Amara a question about school, the mother answered first, twice. When the therapist asked about the marriage, the mother returned to Amara within one sentence. And when Amara was asked what she worried about, she said "that my mother will be alone".

The inference. The marital tension is being managed through the mother–daughter relationship. That is a triangle in Bowen's sense — a third party recruited to absorb what the dyad cannot hold — and it is also, at the structural level, a coalition, because the alliance is stable and the father is positioned outside it. Note that the two descriptions are of the same phenomenon seen with different instruments.

What you say in the room. Not "Amara is triangulated". The usable sentences are about behaviour and are addressed to the person who can act:

> "Mrs Adeyemi, I notice that when I ask about you and your husband, you tell me about Amara. I may be wrong, but I wonder whether Amara is the person you talk to about the hard things. Is that right?"

Then, to the daughter, without requiring her to betray anyone:

> "Amara, when your mother is upset, what do you do? And what happens to your own evening?"

What you do not do. You do not interpret the triangle to the child, you do not describe the father as "peripheral" in front of his family, and you do not invite the daughter to stop being close to her mother. Each of those moves would produce loyalty conflict in the person with least power, which is the clinical definition of harm in family work.

How you know it is working. Three signs, in order of appearance: the mother begins to answer questions about her marriage with information about her marriage; the daughter volunteers an opinion that differs from her mother's without checking her face; and the father, when present, is addressed by his wife before the daughter is consulted. None of these is dramatic. All of them are structural.

4.6 Rules, roles, myths and secrets

Four concepts that describe the invisible organisation of a family. Each is taught badly in most textbooks, because each is presented as a noun rather than as a procedure for noticing.

Rules. Families run on rules that nobody wrote down: who speaks first, who is consulted, who may be angry, what may be discussed at dinner, who apologises, how money is decided, what happens when someone cries. Most rules are functional and invisible, which is what makes them rules rather than decisions.

Metarules govern the rules: for example "we do not discuss the rules". A family whose metarule is "conflict must not be named" will produce a session in which nothing is argued and everything is tense, and it will describe itself as close.

How to surface a rule. Ask about a specific, recent, ordinary occasion: "What happens at dinner if someone is upset?" "Who decides when the television goes off?" "What would happen if you disagreed with your father in front of your mother?" The answers are rules, stated as customs — and customs can be examined.

Roles. Functional roles (the responsible one, the clown, the sick one, the peacemaker, the scapegoat) are distributed early and stabilise the system. The clinical significance is that a role can outlive its usefulness and imprison the person in it: the peacemaker who cannot express anger, the sick one who cannot be well without destabilising the system, the responsible one who cannot fail.

A crucial caution: roles are descriptions of positions in a system, not of personalities. A child in the scapegoat position is not a bad child. When a therapist slips into "he is the difficult one", the formulation has failed.

Myths. A family myth is a shared belief about the family that organises behaviour and resists disconfirming evidence: "we are a happy family", "we never needed anyone", "the men in this family are hot-tempered", "nobody in this family gets depressed, we just get on with it". Myths are not lies; they are the family's constitution, and they are usually protective. They become a problem when reality contradicts them and nobody is allowed to say so.

How to work with a myth. Never by confrontation. The usable route is through an exception or a cost: "You have described a family that never asks for help. What has that cost you?" Or, with a myth of invulnerability: "Which of you would find it hardest to say 'I am not coping'?" The myth is then available to be revised without anybody being humiliated.

Secrets. Not all secrets are damaging; some are protective and some are nobody else's business. Three categories are clinically important. Secrets about the past (a death, a prison term, an adoption, a previous marriage) that are known to some and not others, and that distort the family's explanatory map. Secrets inside the session (an affair disclosed by one partner, a plan to leave, a diagnosis withheld) that place the therapist in an impossible position; the ethical handling of these is treated in MFT 704 and revisited in

MFT 710. Secrets that protect harm (abuse, violence) which must be addressed under safeguarding protocols, whatever the family's wish.

The rule of thumb for secrets. Ask: who is protected by this secret, and who is protected from it? If the answer is the same person, the secret may be kind. If the person protected and the person kept in the dark are different, the secret is doing something else.

> **CASE**
> A 14-year-old is brought for "lying". The parents discovered she had been meeting a boy. In the second session the mother mentions, in passing, that the girl's father is not her biological father and that she does not know. The family myth is "we are completely honest with each other". The therapist's task is not to expose the myth but to establish that the girl's lying is a rational response to a household in which two people hold information the third is not allowed to have — and to work with the parents about whether, when and how the disclosure should happen, with the girl's interests, not the therapist's candour, as the criterion.

4.7 Communication concepts worth keeping

Week 2 gave you punctuation and feedback. Four more concepts complete the vocabulary.

Metacommunication. Communication about communication: "I don't mind you being late, it's that you don't text." Metacommunication is how relationships are negotiated, and families vary enormously in how much of it they do. A family with no metacommunication has no way to revise its rules except through crisis.

Levels: content and relationship. Every message carries information and a definition of the relationship. "How was your day?" can be an inquiry or a check on whereabouts. Most couple conflict is a relationship-level dispute conducted at content level — an argument about the dishes that is about respect.

Symmetrical and complementary. In symmetrical exchanges, both parties behave as equals (which can escalate into competition). In complementary exchanges, the behaviours are different and mutually fitting (parent/child, pursuer/withdrawer, teacher/student). Neither is healthy or unhealthy in itself; problems arise from rigidity. A couple stuck in complementary pursuit–withdrawal needs to build symmetry; a couple stuck in symmetrical escalation needs to accept asymmetry in some domains.

Double-level messages. Messages in which the content and the relationship level contradict each other ("Of course I want you to go out with your friends" said in a tone that forbids it). This is not the double bind — the double bind requires that the recipient cannot leave the field and cannot comment on the contradiction. Ordinary double-level messages are common, survivable and worth naming in session.

4.8 The genogram as a conceptual instrument

A **genogram** is a three-generation diagram of family structure, relationships and events. Students often treat it as a data-collection form. It is better understood as a hypothesis machine: its value is in what it makes visible, and in the family's reaction to seeing it.

For this week, three uses only. (Week 8 covers the technique in full.)

1. **To reveal structure.** Deaths, divorces, remarriages, migrations, births, adoptions — drawn to scale, across generations — make patterns visible that no interview would have produced.
2. **To reveal repetition.** "Your father left at 9, you left at 9, your son is 9." Nobody had said this sentence in the family before.
3. **To reveal the family's own version.** Ask the family to draw it, or to correct yours. The corrections are the data: who is included, who is omitted, which relationships are drawn close and which distant.

Symbols you must know. Male square, female circle, deceased crossed, marriage a horizontal line with the date, divorce two slashes, children descending in birth order, household membership a dotted rectangle, close relationship a double line, conflict a zigzag, cutoff a slash through the line. Learn to draw them small and neat; a genogram drawn badly is a message to the family about how carefully they are being taken.

4.9 The Adeyemi family, mapped

The map, described in words so that you can draw it.

Household. Father (48), mother (45), Amara (16), brother (11). A dotted rectangle around all four — with the brother's position marked and a note that he is "the easy one". The paternal grandmother lives 200 km away and is telephoned every day by the mother; draw a close line to the mother, a conflicted line to the father.

Relationships. Between the parents: conflict (zigzag), with a note "arguments about money and the father's drinking". Father to Amara: distant with moments of criticism; draw a dashed line and mark it "unpredictable". Mother to Amara: very close (double line) with an arrow from Amara to the mother marked "caretaking". Mother to brother: close but described as "neglected" by the mother herself. Brother to Amara: close, protective.

Events. Father's redundancy fourteen months ago (mark it on the timeline). Mother's first depressive episode six years ago, and the current one. The father's own father died of a stroke when the father was 19; mark deceased and note that the father "has never talked about it". A stillbirth between the two living children, never discussed.

What the map makes visible that the interview did not. Four things. The father's father died when he was Amara's age, which reframes his difficulty with her adolescence. The stillbirth sits between the two living children and is never mentioned, which suggests an unmourned loss in the mother's story. The grandmother is a daily presence and a structural authority. And the brother is described by everyone as fine, which in a family like this is a hypothesis to test rather than a fact to accept.

How to use the map in session. Show it, ask for corrections, and then ask the two questions the map makes possible: "Who is missing from this picture?" and "What would change on this drawing if therapy worked?"

PRACTICAL

Draw the Adeyemi genogram yourself, from the description above, without looking at the printed version in the appendix. Then compare. Most students omit the stillbirth, place the grandmother wrongly, or fail to mark the father's age at his own father's death — which is precisely the kind of omission that makes a formulation respectable and useless.

4.10 Annotated transcript: mapping in the room

T1 Therapist: I am going to draw your family on this paper while we talk. Tell me if I get it wrong.
T2 Mother: (laughs) You will get it wrong.
T3 Therapist: Good. Start me with your husband's family. Who were his parents?
T4 Mother: His father is dead. His mother is alive, she is in Ibadan. He does not talk to her much.
T5 Therapist: (draws) How long has he not talked to her much?
T6 Mother: Since his father died. That was a long time ago. He was still a boy.
T7 Therapist: How old was he?
T8 Mother: (pause) About nineteen. He had to leave school.
T9 Therapist: And Amara is how old?
T10 Mother: Sixteen.
T11 Therapist: (draws, then stops) Is there anything else on this paper that I should know about?

Annotations. T1 announces the task and immediately hands the family the right to correct the therapist. This is a structural move: it makes the map a shared object rather than the therapist's assessment. T2 is a test of the therapist's response to teasing; the answer — warmth and an invitation — establishes that the room can be informal.

T3 starts with the father's family, who are absent. This is deliberate: it prevents the session from becoming a conversation about the mother, and it surfaces the paternal cutoffs that no one has mentioned. Students almost always start with the presenting child.

T5–T8 produce the most important sentence in the session: the father was 19 when his own father died and he had to leave school. The therapist asks for the age and does not comment further. T9 is the juxtaposition — "and Amara is how old?" — which lets the mother make the connection herself. She does, in the pause before T10.

T11 is the invitation that produces secrets. "Is there anything else on this paper that I should know about?" is open, bounded by the drawing, and not threatening: it asks about the paper, not about the family's morality.

What the therapist did not do. She did not say "so your husband is repeating his own adolescence with his daughter" — which would have been an interpretation the mother could neither confirm nor use. She did not ask about the stillbirth directly, which would have been premature in a first mapping session. She did not draw conclusions from the map in front of the family; the map's job in session one is to gather and to invite.

4.10a Second transcript: naming the rule, not the person

> **T1 Father:** In this house we don't discuss feelings. That is not how I was raised.
> **T2 Therapist:** So the rule is that feelings are handled privately. Who taught you that rule?
> **T3 Father:** My father. He never said a word about anything. He just worked.
> **T4 Therapist:** And what did that give you?
> **T5 Father:** (long pause) It gave me a strong house. Nobody went hungry.
> **T6 Therapist:** And what did it cost?
> **T7 Father:** (longer pause) I don't know what my son is thinking. I don't know what my wife is thinking.
> **T8 Therapist:** Amara, in this family, if you were having a bad week, who would know?

Annotations. T2 is the whole technique of this week in one move: the father has stated a rule, and the therapist treats it as a rule — a thing with an author, a history and a cost — rather than as a fact about the family or a failing of the father. T4 asks for the gain before the cost, which is why T5 is not defensive; the therapist has protected the father's inheritance before examining it.

T6 is the pivot, and it only works because T4 came first. A therapist who leads with "what did it cost?" produces a defended father; a therapist who establishes what the rule achieved produces a father who can name its limits himself. T7 is the father doing the formulation.

T8 moves the question from the father, who has just disclosed something painful, to the daughter, in a way that lets her answer without contradicting him. Her answer — "nobody, unless my mother notices" — completes the picture: the rule is not just the father's; it is the household's, and the mother has been carrying the emotional work the rule leaves undone.

4.11 Laboratory: the structural map

The assessed practical. Groups of three. Each group receives a one-page case summary (case bank, appendix) and must produce a structural map, a genogram and a two-minute presentation.

Rules for the map. Every symbol must be justifiable from the text or from an inference you state aloud. Adjectives are not permitted on the drawing; only behaviours and events. Each map must mark at least one hypothesis with a question mark, so that the group declares what it does not know.

Marking rubric.

Criterion	Weight	Full marks
Genogram accuracy	20%	Three generations, correct symbols, dates and deaths, household marked
Structural reading	30%	Subsystems, boundaries and hierarchy named with observational evidence, not adjectives
Triangle/coalition/detour distinction	20%	The correct concept is applied, and the group can say why not the others

Criterion	Weight	Full marks
Hypothesis discipline	20%	At least two competing hypotheses, each with the question that would test it
Presentation	10%	Two minutes, no jargon the family could not follow

4.12 Six errors in using systems concepts

1. **Concept as insult.** "She is enmeshed" said about a mother who is doing what her culture and her circumstances require. Fix: describe the behaviour and its cost before you use any label.
2. **The static map.** Drawing a family once and treating the drawing as the family. Fix: redraw after every third session; families change, and so should your map.
3. **Triangle-spotting as a game.** Finding triangles everywhere, including where the evidence is a single sentence. Fix: a triangle requires evidence of a process — a third party recruited when tension rises — not merely three people in a room.
4. **Detour accusations.** Telling parents their child's illness is about their marriage. Fix: name the function only when the couple has enough containment to hear it, and never with the child present.
5. **Normative hierarchy.** Assuming that a "proper" family has a particular shape, and pathologising households that organise differently. Fix: ask what the organisation costs and whom, rather than whether it matches a template.
6. **The unmarked inference.** Presenting an inference as an observation, in a report or in a session. Fix: distinguish, out loud, between what you saw and what you think — "I noticed your husband answered for you twice; I may be wrong, but I wondered whether that happens often."

4.12a Writing a structural formulation for a report

Students can formulate in the room and freeze at the keyboard. A short template, with the rule that every sentence must survive a challenge from a colleague who has never met the family.

> Structure. The household consists of four members (father 48, mother 45, daughter 16, son 11) with a peripheral but present paternal role and a telephone-based involvement of the paternal grandmother. Hierarchy. Executive decisions are made by the mother; the father is consulted after the event, and his disagreement is registered as criticism rather than as participation. Boundaries. The mother–daughter relationship is highly involved and carries emotional confidences; the marriage is comparatively disengaged; the son is described by all members as "easy" and appears to be outside the organising conflict. Hypothesis. The daughter's school refusal is maintained by a sequence in which her presence at home reduces the mother's isolation and defers marital discussion. This is a hypothesis, not a conclusion: it is supported by three observations and would be disconfirmed if the refusal persisted during a period in which the parents' contact increased and the mother's support from others improved. Risk. No current indicators of violence or self-harm were disclosed on direct enquiry; the father's alcohol use was reported at 4–6 units on 5 nights per week and has been raised with him for assessment. Plan. Conjoint sessions including the father; a joint parental task in session; a staged school return negotiated with the school; support for the mother's own treatment; review at six sessions.

Three rules for this kind of writing. Separate observation from inference, in different sentences. State at least one thing that would prove you wrong. And never write a sentence you would not be willing to read aloud to the family in a feedback session — because in many settings, that is exactly what will happen.

4.13 Culture, faith and the boundary question

The concepts in this week are the ones most likely to do cultural damage, because they describe closeness, hierarchy and obligation — precisely the domains in which cultures differ most. Four corrections.

Closeness is not enmeshment. In households where three generations share care of children, sleep in the same room, pool income and take decisions collectively, a high level of involvement is functional. The diagnostic question is not "how close are they?" but "can each member think, disagree and move?" A young adult who consults her mother about a marriage proposal is not enmeshed; a young adult who cannot form an opinion without her mother's approval, and who experiences disagreement as betrayal, may be.

Hierarchy is not obedience. A family in which children do not question adults is not automatically dysfunctional. The functional question is whether the hierarchy carries responsibility as well as authority: do the adults protect, provide and decide, or do they merely command and then require the children to manage the consequences?

Filial duty is a resource. Expectations that adult children care for parents and siblings can be a source of meaning and support, and also of exhaustion. Both are true, and the therapist who treats obligation as pathology will lose the family. Ask instead: what does this obligation give you, and what does it cost, and who decided?

Respect for elders can mask abuse. The most difficult clinical situation in this domain is a household in which hierarchy is used to silence a victim: "you cannot speak against your uncle." The professional obligation is clear — child protection and adult safeguarding

override deference — but the skill is to act without humiliating the family, and to keep the door open for the members who need you. MFT 704 and MFT 724 give the legal and ethical framework; this week's contribution is the conceptual warning.

> **CAUTION**
> "In our culture, families are close" is sometimes a description and sometimes a prohibition on examining anything. The therapist's response is neither agreement nor challenge, but curiosity with boundaries: "Tell me what closeness means in your family, and what happens when someone needs distance."

4.13b A five-minute version for the corridor

Colleagues in hospitals and schools will ask you what you think, in a corridor, with thirty seconds to answer. A usable five-sentence formulation, in the order that survives interruption:

1. **Who is in the system** — "Four at home, plus a grandmother who makes the decisions by telephone."
2. **What is happening, behaviourally** — "The 16-year-old has been out of school six weeks; her mother answers for her; the father is present but outside the executive."
3. **What maintains it** — "Her presence at home reduces her mother's isolation and defers a marital conversation that nobody wants to have."
4. **What I am unsure about** — "Whether the father's drinking is a cause or a consequence; I have not assessed it yet."
5. **What I am doing next** — "Both parents in the room next week, a joint task on the school letter, and a separate alcohol assessment for the father."

Note the fourth sentence. Including what you do not know is not weakness in a corridor; it is how you prevent a colleague from acting on your guess as though it were an assessment. The habit also protects you: the clinician who always sounds certain is the one who is quoted in a serious case review.

4.13a When the concept is the problem: the misuse audit

A concept becomes dangerous at the moment it stops generating questions and starts closing them. Five signs, each with the question that reopens the inquiry.

Sign 1 — the label arrives before the evidence. You find yourself saying "this is an enmeshed family" in the first ten minutes. Reopening question: what exactly did I see, and what else could explain it?

Sign 2 — the family's own account is treated as resistance. A mother says "in our family we are close" and you hear denial. Reopening question: what would it mean if she were simply right?

Sign 3 — the concept explains everything. Every behaviour in the session becomes evidence of the same structure. Reopening question: name one observation from today that does not fit my formulation.

Sign 4 — the concept justifies a plan the family has not agreed to. You find yourself designing an intervention to change a boundary the family has not identified as a problem. Reopening question: whose goal is this, and did they ask for it?

Sign 5 — the concept travels into the report as fact. "The family is enmeshed" appears in a letter to a school or a court. Reopening question: can I rewrite this sentence as an observation with a date and a count?

> **AUDIT**
> Take your most recent formulation, written or spoken. Run the five signs against it. Then rewrite it so that every sentence could be defended in a case conference by a colleague who disagrees with you.

4.14 The concept table you will actually use

Twelve concepts, each with what to observe, what it usually costs, and what you would do. Learn this table and you can formulate almost any family in the language the profession shares. Copy it into the front of your clinical notebook.

Concept	What you observe	Usual cost	What you do
Diffuse boundary	Members answer for each other; no privacy; moods shared as weather	Chronic anxiety; difficulty leaving; confidences that a child cannot carry	Mark the behaviour in the room; build one small boundary with the family's consent
Rigid boundary	Topics that cannot be named; formality; information kept in compartments	Isolation; late presentation; secrets	Name the cost of the rule, not the rule; use a third-person question to open it
Weak parental hierarchy	Child decides or vetoes; parents negotiate with the child	Anxious, parentified or defiant child; inconsistent discipline	Give the parents a joint task in the session and coach them through it
Cross-generational coalition	Parent and child allied against the other parent; child knows the marriage	Child loses a parent as a parent; other parent discredited	Re-seat; give the couple a task; support the outsider without blaming the insider
Parentification	Child cooks, translates, manages a parent's feelings	Premature competence; suppressed need; later collapse or resentment	Name the competence and the cost; redistribute one task to an adult
Triangle	Third party discussed whenever two are together; symptom rises with tension	The third person carries what the pair cannot	Ask about the third person's view; bring them in; work the pair's tension directly
Detouring	Couple's conflict reliably becomes a conversation about a child	The child's symptom is the couple's containment	Name the function, cautiously, when the couple can hear it
Rule	Repeated pattern described as custom ("that's just how we are")	Rigidity; invisible obligations	Ask about a specific ordinary occasion; make the rule sayable

Concept	What you observe	Usual cost	What you do
Metarule	"We don't discuss that"; "we don't argue in this house"	The family cannot revise its own rules	Ask what would happen if it were discussed; work with the feared consequence
Role	Stable position (peacemaker, sick one, responsible one)	A person imprisoned in a function	Name the position, not the person; ask who else could occupy it
Myth	Shared belief resistant to evidence ("we never needed anyone")	Reality that cannot be spoken; delayed help-seeking	Work through exceptions and costs, never by confrontation
Secret	Information held by some and withheld from others	Distorted explanations; alliances; a member who "lies"	Ask who is protected, and from what; manage the disclosure ethically

How to use the table in supervision. Present the observation column first, with your evidence, and only then the concept. "The mother answered three questions addressed to the daughter, and the daughter looked at her mother before every answer, so I am working with a diffuse boundary — but I may be wrong, because the daughter also corrected her mother twice." A presentation in that order cannot be dismissed as labelling.

4.15 Two families, same concept, opposite interventions

The most important skill in this week is knowing that a concept does not dictate an intervention. Two cases illustrate the point.

Family A — the close household. A 20-year-old university student is brought by her parents because she is "withdrawn and rude". She has stopped attending lectures. The family is warm, loud, interdependent. The mother and daughter share a bed when the daughter is home; the father describes his wife and daughter as "best friends". The daughter says, in a private moment, that she has never made a decision without her mother.

Formulation: a diffuse boundary between mother and daughter, which was functional in adolescence and is now costly at the threshold of adulthood.

Intervention: not separation, and emphatically not a lecture on independence. First, name the closeness as a strength and get the family's agreement that it is. Second, create one small experiment: the daughter takes one decision a week without consulting her mother, and reports it afterwards. Third, work with the mother's own anxiety about the daughter's adulthood — what does the mother lose if the daughter becomes independent? Fourth, involve the father as a partner to the mother, so that the mother is not left with only the daughter's company.

Family B — the distant household. A 20-year-old university student is brought by her parents because she is "withdrawn and rude". She has stopped attending lectures. The family is formal; conversation is conducted in turn; nobody has raised their voice in the house for years. The father works abroad and telephones on Sundays. The daughter says, in a private moment, that she has never told her parents anything that mattered.

Formulation: a rigid boundary around emotional information, with an implicit rule that distress is not discussed. The withdrawal is not individuation; it is the family pattern applied to one member.

Intervention: the opposite of Family A. First, name the absence without accusing anyone: "In this family, people manage things privately — is that right?" Second, use a structured, low-risk disclosure task in the session (each person says one thing about the past month that the others did not know). Third, work with the father's distance explicitly, including the question of what he knows about his daughter's life. Fourth, expect the family to find the session uncomfortable and to report that it was "fine" — and use that report as the next topic.

The point. Diffuse and rigid boundaries need opposite moves. This is why a diagnosis-like use of concepts produces bad therapy: the same label, applied to two families, licenses interventions that would harm one and help the other. The concept tells you where to look; the family tells you which direction to move.

> **PRACTICAL**
> Read both cases again. Write the first three sentences you would say in each session. Then swap with a peer and check: could your sentences for Family A be said to Family B? If so, you are working from the concept rather than from the family.

4.16 Evidence note: which of these concepts have research support

Students should know how much weight each concept can bear outside the consulting room.

Boundaries and hierarchy. Structural family therapy has a reasonable evidence base for adolescent conduct and substance problems, and observational research on family organisation — for example the work on authoritative versus permissive versus authoritarian parenting — broadly supports the claim that clear hierarchy with warmth predicts better child outcomes. The specific vocabulary of "enmeshment" has weaker support: measures of family cohesion show curvilinear relationships with wellbeing, which is to say that both very low and very high cohesion can be problematic, and that the optimum depends on culture and stage.

Triangles. Bowen's triangles are theoretically influential and empirically under-tested as a mechanism. There is support for the broader claim that interparental conflict affects children's adjustment, and that children's involvement in parental conflict — especially that which makes them feel caught — predicts worse outcomes. This is not proof of triangles; it is consistent with them.

Parentification. There is good evidence that destructive parentification (role reversal without recognition or support) predicts internalising problems, and equally good evidence that adaptive or "filial responsibility" in supportive contexts predicts competence. The distinction is now standard in the literature and should be standard in your practice.

Rules, roles, myths. Mostly clinical concepts with qualitative support. They are useful and they are not measurable; say so when you write reports. Their value is descriptive and intervention-generating, not predictive.

Communication concepts. Demand–withdraw patterns, negative reciprocity and the balance of positive to negative affect have solid empirical support in couple research. The stronger claims of the early communication school — that specific communication deficits cause distress — have been revised: communication is as much a product of satisfaction as a cause of it, and the direction of effect is contested.

What this means for practice. Use the concepts; do not over-claim for them. A formulation can be clinically excellent and evidentially modest at the same time, and the professional habit is to be able to say which is which.

4.17 Key terms

Term	Definition
Boundary	The rules governing participation, closeness and privacy between subsystems
Subsystem	A unit within the family organised by generation, function or gender (couple, parents, siblings)
Hierarchy	The arrangement of authority and responsibility, especially across generations
Coalition	An alliance of two against a third that crosses generational lines
Triangle	A three-person emotional configuration recruited to manage tension between two
Detouring	Displacement of couple conflict onto a third party, usually a child
Parentification	A child taking on adult caregiving responsibilities for a parent or siblings
Enmeshment	Diffuse boundaries with excessive sharing and low autonomy (use with care)
Disengagement	Rigid boundaries with little emotional contact
Rule / metarule	A recurring pattern of what is permitted; a rule about rules
Family myth	A shared belief about the family that organises behaviour and resists disconfirmation
Metacommunication	Communication about the relationship or about communication itself
Symmetrical / complementary	Equal versus different-and-mutually-fitting exchanges
Genogram	A three-generation diagram of structure, relationships and events
Homeostasis	The system's tendency to maintain its existing organisation
Function of a symptom	The work a symptom does in maintaining the system's organisation

4.18 Self-test

1. Name the three boundary types and give one observational marker of each.
2. What distinguishes a coalition from a triangle?
3. What is the difference between detouring-attacking and detouring-supportive?

4. Why is "enmeshment" dangerous as a label in a collectivist household?
5. What is a metarule, and how do you surface one?
6. Which question helps you decide whether a secret is protective?
7. Give one concept from this week with strong empirical support and one with weak support.
8. Why do two families with the same boundary problem need opposite interventions?

Answers. 1. Clear (each member has voice and privacy; adults decide adult matters), diffuse (members answer for each other; moods shared as weather), rigid (topics that cannot be named; formality). 2. A coalition is a structural alliance across generations against a third; a triangle is an emotional process in which a third is recruited to manage tension between two. 3. In detouring-attacking the child is defined as the problem and the parents unite against them; in detouring-supportive the child is defined as fragile and the parents unite to protect them. 4. Because high involvement can be functional and normative; the diagnostic question is whether members can think, disagree and move, not how close they are. 5. A rule about what may be discussed; surface it by asking what would happen if the topic were raised. 6. Ask who is protected by the secret and who is protected from it. 7. Hierarchy/parenting style or demand–withdraw has reasonable support; "enmeshment" as a diagnosis or Bowen's triangles as a mechanism have weak support. 8. Because the same label can describe under-involvement or over-involvement, and the direction of the required change is opposite.

4.19 Reflective task

Draw your own family of origin as a structural map: subsystems, boundaries, hierarchy, one triangle, one rule that was never stated. Then answer three questions in 300 words. Which of the concepts was easiest to apply to your own family, and why might that be? Which was hardest, and what does the difficulty protect? And what would your family say if they saw the drawing?

Bring the drawing to the lab, but not the answers to the third question: that one stays private, and you will return to it in Week 14.

4.21 Appendix: the case bank for this week

The lab draws on four cases. Read all four; you will be assigned one.

Case 1 — the returning father. A father returns to the household after three years working in another city. The 13-year-old son refuses to obey him and the mother mediates every exchange. Concept focus: hierarchy, coalition, re-entry.

Case 2 — the two-household child. A 10-year-old moves between two households with different rules, and presents with "behaviour problems" that occur only after each transition. Concept focus: boundaries between systems, rules across contexts, the child as the carrier of a conflict between two subsystems.

Case 3 — the bereaved couple. A couple in their fifties lost their eldest son eighteen months ago. They describe no conflict and no intimacy. Their younger daughter has

become the household's organiser. Concept focus: rigid boundary around grief, parentification, the function of silence.

Case 4 — the church leader's family. A pastor's household in which the father is a public figure and the family's difficulties are treated as a threat to his ministry. The 17-year-old daughter has disclosed self-harm to a teacher. Concept focus: myth, secret, hierarchy, and the point at which risk overrides the family's preferred account.

What to produce for your assigned case. A genogram, a structural map, a one-paragraph formulation, two competing hypotheses with the question that would test each, and the first intervention with its rationale. Two pages maximum. The two-page limit is deliberate: a formulation you cannot compress is one you have not finished thinking about.

How the four cases are marked together. Cases 1 and 2 test hierarchy and cross-system boundaries; cases 3 and 4 test the concepts that carry the most risk of harm — parentification and secrecy respectively. Present your case to a group working on a different one, and spend five minutes on the question: which of your concepts would have been dangerous in the other group's family, and why? That question, asked honestly, is the best protection against the misuse discussed in section 4.13a.

4.20 Readings for this week

- **Minuchin (1974)**, Families and Family Therapy, chapters 2–4. Boundaries, subsystems, mapping.
- **Bowen (1978)**, Family Therapy in Clinical Practice, chapter 8 on triangles. Read for the mechanism, not the dogma.
- **Nichols & Schwartz**, chapters 5–6. The structural model, summarised fairly.
- **Byng-Hall (1995)**, "Creating a secure family base", Family Process. On caregiving roles and their costs — a bridge between structure and attachment.
- **Kerig (2005)**, on parentification. Read one empirical paper and notice the distinction between destructive and adaptive forms.
- **Palazzoli et al. (1978)**, Paradox and Counterparadox, on the referring person and the function of the symptom. Dated and illuminating.

Next: Week 5 — The profession: identity, scope and settings

Week 5 — The profession: identity, scope and settings

5.1 The lecture in ninety minutes

Students spend the first month learning to think relationally. This week asks a different question: who are you when you do it? Not in a philosophical sense, but in the practical one that governs your career — what you are licensed to do, where you may do it, what you must refuse, how you are paid, who you answer to, and how you explain yourself to a judge, a school principal, a church council or a hospital consultant.

The week has an uncomfortable edge, and it should. Family therapy is a young profession in most jurisdictions, practised by people with several different trainings, sometimes without a clear legal footing. Students who imagine that professional identity is conferred automatically by a certificate will be unprepared for the questions that arrive in practice: Can I see this child without the father's consent? Does my report carry weight in court? May I call myself a psychologist? What do I do when the imam who referred this family expects a report on their spiritual state?

Minutes	Segment	What the lecturer does	What students do
0–12	What makes an occupation a profession	Six criteria, applied to three occupations	Score MFT against the six
12–30	Scope of practice	The three circles: competence, authorisation, mandate	Place ten tasks inside or outside
30–45	Training and registration	Routes into the work; what each title may and may not claim	Map their own route and its limits
45–60	Settings	Six settings, each with its own contract	Identify the invisible contract in a case
60–75	Teams, referrals and boundaries	A live referral letter, criticised	Draft a referral in eight lines
75–90	Money, records and the long career	Fees, sliding scales, notes, supervision, burnout	Write their own three-year CPD plan

5.2 What makes an occupation a profession

The sociology of the professions gives six criteria. They are not a definition to be recited; they are a checklist for understanding the negotiations that shape your working life.

1. A specialised body of knowledge. Not merely skills, but a system of theory that practitioners must learn and that outsiders cannot easily acquire. Family therapy has this: systems theory, cybernetics, structural and attachment-based models, and a distinctive literature. The weakness is that the knowledge is dispersed across several disciplines, which makes the claim to exclusivity harder than it is for medicine.

2. Prolonged training and socialisation. A profession reproduces itself through education that changes who the student is, not only what they know. The reflective tasks in this library are part of that socialisation, not an indulgence.

3. A code of ethics with enforcement. Not a statement of values on a website, but a mechanism: a code, a complaints route, and sanctions including removal. Where the enforcement mechanism is weak, the public is protected mainly by individual conscience, which is not a system.

4. A registration or licensing mechanism. Some legal recognition of who may use the title and practise the activity. This is where the picture varies enormously across jurisdictions, and where students must know the facts for their own country rather than assume.

5. Autonomy and self-regulation. The profession, not the state or the market, sets its standards. Autonomy is always partial and always contested — by insurers, by employers, by government — and it has costs: professions that regulate themselves are slow to discipline their members.

6. A service orientation with a fiduciary duty. The client's interest comes before the practitioner's. This is the criterion most easily damaged by the conditions of practice: a therapist paid per session by an agency that requires a diagnosis has a conflict that must be managed openly, not denied.

Where family therapy sits. Strong on 1, 2 and 3; variable on 4 and 5 depending on jurisdiction; and permanently at risk on 6 wherever funding structures reward volume over outcome. A student who understands this will not be surprised by the political arguments that surround the field, and will know which of them they can influence and which they cannot.

> **AUDIT**
> Score marriage and family therapy, in your own jurisdiction, against the six criteria, 0–2 each. Where you score 0 or 1, write the name of the body — if any — that could change it. Most students discover that the weakest criterion is registration, and that the second weakest is enforcement.

5.3 Scope of practice: three circles, not one

The commonest professional error is to equate scope of practice with competence. It is narrower and more complicated. Think of three circles.

Circle 1 — Competence. What you can actually do well, given your training, your supervised experience and your current skills. This is a factual question, and the honest answer changes over a career: it grows with deliberate practice and shrinks with neglect.

Circle 2 — Authorisation. What you are permitted to do by law, by your registering body, and by your employer. A competent family therapist may be legally unable to write the report a court requires, or to see a child without a guardian's consent, or to assess for a diagnosis that only a clinical psychologist may make in that jurisdiction.

Circle 3 — Mandate. What this particular client, family or referrer has asked you to do, and what the setting expects. A school that refers a child wants an opinion about schooling; a court wants an opinion about custody; a family wants relief. These are not the same mandate, and the therapist who does not clarify it will eventually be experienced as either useless or intrusive.

The safe zone is the intersection. Competent, authorised and mandated. When one circle is missing, three legitimate options exist: refer, consult and work within limits, or obtain additional training and authorisation before proceeding. The illegitimate option — which is common, well-meant and career-ending — is to proceed because the family needs help and nobody else is available.

Situation	Circle missing	Correct response
Asked to assess a 6-year-old for ADHD and recommend medication	Authorisation (prescribing) and probably competence	Assess the family context, write to the paediatrician, do not recommend medication
Asked by a court to say which parent should have custody	Authorisation (in many jurisdictions) and competence (forensic)	Offer a clinical opinion on the child's needs; recommend a forensic assessment
Asked to see a couple where there is current violence	Competence and safety conditions	Screen, safety-plan, consider separate sessions, refer to a specialist service
Asked by a church to "deliver" a family from a spiritual problem	Mandate mismatch	Clarify roles; offer clinical work alongside, not instead of, pastoral care
Asked to work with a family in a language you speak poorly	Competence	Use a trained interpreter; do not use the family's child

5.4 Training, titles and what they may claim

Titles matter because they carry legal and public meaning. The details vary by country and change over time; what follows is orientation, and the rule is always: **verify the current requirements with the relevant council or association before you use a title or accept a referral.**

The routes into the work, in outline.

- **Marriage and family therapy as a distinct qualification** — a master's or doctoral programme in MFT, usually with supervised clinical hours, a placement, and a research component. This is the route this library supports.
- **Psychology** — clinical or counselling psychology, with its own professional registration, assessment privileges and, in many jurisdictions, a protected title.
- **Counselling** — a large and varied field, ranging from rigorous postgraduate training with supervised practice to short courses. The variance is the problem: a title that can mean anything protects nobody.
- **Psychiatry** — a medical route, with prescribing rights and, usually, authority over admission and diagnosis in institutional settings.
- **Social work** — with statutory powers in child protection in many jurisdictions, and a distinctive ecological training.
- **Pastoral and faith-based counselling** — substantial in practice, variable in training, and often the first service a family encounters.

Three things to be able to say about your own training. What you are qualified to do; what you are not; and what you are working towards. The third is what distinguishes a professional from a technician. Write the three sentences now, and revise them every year.

The title trap. Students sometimes use a title loosely — "psychotherapist", "counselling psychologist", "family therapist" — in a jurisdiction where the title is protected or where

an employer interprets it narrowly. The consequences range from embarrassment to prosecution, and they always damage the client's confidence. Use the title your registration permits, and describe the work rather than the label: "I work with couples and families using a systemic approach" is accurate, informative and safe.

> **CAUTION**
> Never describe yourself as supervising, diagnosing or assessing beyond your authorisation, even informally and even when asked by a senior colleague. The sentence "as the family therapist, I would say this child has..." has ended careers, and more importantly has harmed families who acted on an opinion that had no standing.

5.5 Six settings and their invisible contracts

Each setting imposes a contract that nobody writes down. The therapist who does not read it will be in conflict with the institution within a month.

Outpatient clinic. The contract: the family self-refers or is referred; attendance is voluntary; the work is time-limited by funding. What this affords: the best conditions for systemic work — a room, a series of sessions, a family that chose to come. What it demands: rapid engagement, because dropout is highest in the first three sessions. The characteristic failure: treating a voluntary contract as though it were compulsory, and losing the family by session two.

Hospital inpatient and liaison. The contract: the patient is admitted; the family is a visitor; the medical team owns the plan. What this affords: access at a moment of crisis, when a family is most open to change. What it demands: brevity, clarity about what you are and are not deciding, and tolerance of hierarchy. The characteristic failure: conducting a leisurely family assessment while a consultant waits for a one-paragraph opinion.

School. The contract: the school has a duty to the child's education and to other pupils; the parents have rights; the child has a voice that is easily lost. What this affords: early intervention, and access to the system in which the symptom is most visible. What it demands: an explicit agreement about what will be shared with whom, in writing, before the first session. The characteristic failure: becoming an instrument of institutional discipline, so that the child experiences therapy as another arm of the school.

NGO and humanitarian settings. The contract: a donor funds a defined service with defined indicators; the population is often displaced, bereaved and without legal documentation. What this affords: reach and flexibility — home visits, groups, community workers. What it demands: attention to the difference between what the donor counts and what the family needs, and a firm grip on boundaries in a context where the therapist may be the only professional the family sees. The characteristic failure: imposing a manualised model designed elsewhere, and recording outcomes that satisfy the indicator rather than describe the family.

Faith settings. The contract: the community's authority is spiritual; the therapist may be a guest, an employee or a member. What this affords: trust, access, and a language of

meaning that many families prefer to the language of pathology. What it demands: explicit role clarity — who is responsible for what — and willingness to name the limit when a spiritual explanation is being used to avoid medical or protective action. The characteristic failure: two authorities giving contradictory instructions to a family, with the family caught between them.

Court, statutory and forensic. The contract: the court is the client; the family is the subject; your opinion may be tested under adversarial conditions. What this affords: influence at the point where a family's life is decided. What it demands: forensic competence, meticulous records, and a clear statement of the limits of your assessment. The characteristic failure: the therapist who has been treating the family writing a custody opinion without the training, the mandate or the methodology.

> **PRACTICAL**
> Choose one setting you expect to work in. Write its invisible contract in five lines: who pays, who refers, who owns the plan, what is recorded, who can see the record. Then write the sentence you would say to a family in the first session to make that contract explicit.

5.6 A day in three settings

The abstractions become concrete in the ordinary texture of a working day. Three Thursdays, three settings.

Thursday in an outpatient clinic. Eight appointments, three of them new. The first family cancels; you use the hour to write a report that was due yesterday. The second is a couple in their forties, together eleven years, presenting with "we argue about everything"; by minute twenty you have the cycle, by minute forty they have agreed on one experiment, and you have agreed to see them fortnightly because they can only afford that. The third is a 15-year-old who comes alone because her mother works; you spend ten minutes on consent, and establish that the mother has agreed to the referral in writing. Between appointments you take a call from a school counsellor who wants to know whether a pupil's self-harm is "serious enough" to refer; you say you cannot discuss a pupil you have never met, and offer a consultation with the counsellor about the process. You finish twenty minutes late, which is normal and is a warning sign.

Thursday in a hospital. You are asked to see a family on the medical ward: a 62-year-old man after a stroke, his wife, two adult daughters, one of whom has travelled from abroad. The medical team wants to know whether the family "will manage at home". You have one hour and a ward round at three. You do four things: you find out what each person has understood of the diagnosis (they have four different understandings); you ask who makes decisions and who will do the caring (everyone assumes the daughter who lives nearby, who has not been asked); you name the disagreement between the sisters about nursing-home care, which nobody has said aloud; and you write six lines in the notes with a plan and a named person responsible for each step. The consultant reads it and says "useful", which in a hospital is high praise.

Thursday in a faith-based counselling centre. Three couples, all referred by their pastor. The first is straightforward couple work. The second involves a husband who has been violent twice; the pastoral expectation is reconciliation, your assessment is that the wife is at risk, and you spend the session on safety planning and on being explicit, in the pastor's presence, about what you will and will not do. The third is a family convinced that the daughter's seizures are spiritual; you establish that she has had a neurological workup, that the medication is being given irregularly because of the family's beliefs, and you negotiate — carefully, with the pastor as an ally rather than an obstacle — a plan in which prayer continues and the medication is given.

What the three days have in common. Every session involved a negotiation about role, mandate or confidentiality before any clinical work happened. The clinical content was, in each case, easier than the contract. This is what experienced practitioners mean when they say that most of the job is boundaries.

5.7 Working in a multidisciplinary team

Family therapists rarely work alone. The team is where the profession's identity is tested, because the team carries other professions' assumptions about what a family therapist does.

Four practical disciplines.

1. **Write your role into the record.** "Family meeting held; systemic assessment of the family's understanding of the diagnosis and capacity to care at home; recommendations below" tells the team what you did and did not do. "Family seen" tells them nothing and invites them to assume you did whatever they imagine.
2. **Separate information from opinion.** In team meetings, use two sentences: "What I observed was…" and "What I think it means is…". Colleagues will quote the first as fact and the second as your view, which is correct, and your practice should make the difference audible.
3. **Protect the family's confidences within the team.** The team needs the information necessary for care, not everything you know. A wife's disclosure of an affair is not team business unless it bears on safety; a child's disclosure of self-harm is team business immediately.
4. **Refuse the role of the team's conscience.** You will be invited to be the person who raises the emotional dimension that nobody else wants to discuss. Accept the task, not the identity: raise the issue, name the evidence, propose an action, and let the team decide.

Three conflicts that recur, and how to handle them.

The diagnosis conflict. A psychiatrist diagnoses the child and the team organises treatment around the child; you think the family is maintaining the problem. You are probably partly right and partly wrong, and the family needs both. Handle it by agreeing a shared formulation in writing, with each profession's contribution named, rather than by arguing about whose model is true.

The confidentiality conflict. The team wants information that the family gave you in confidence. Handle it by telling the family, at the start, exactly what you will share with the team and why — and by returning to them before you share anything outside that agreement.

The authority conflict. A senior colleague instructs you to do something outside your scope. Handle it by naming the scope, offering the alternative, and putting the exchange in writing, courteously. If the instruction is repeated, it becomes a supervision and, if necessary, a registration matter.

> **CASE**
> A paediatric team asks you to "sort out the mother" of a child with a chronic condition. The referral assumes she is the problem: she is "anxious, over-involved, not following the plan". You meet her. She is a woman managing a child's illness, two other children, a job and a husband who is away; she is following the plan as she understands it, and the plan was explained to her once, in English, at a bedside, while her child was crying. The team's formulation was not malicious; it was the natural product of a system that had never asked her what she had understood. Your intervention is not to treat her anxiety. It is to write a note that records what she understood, what she was asked to do, and what support she would need — and to ask the team to repeat the explanation with her, in her language, with a written summary.

5.8 Referral: the skill nobody teaches

A referral is a clinical act. Done well, it transfers care without losing the family; done badly, it loses them, or it transmits a formulation that follows the family for years.

The eight lines of a good referral. Every referral letter or call should contain, in this order:

1. **Who you are and what you are asking for.** "I am the family therapist seeing the Adeyemi family; I am asking for an alcohol assessment, not for treatment to begin without them."
2. **The presenting problem, in the family's language and in yours.** Both, labelled.
3. **What you have tried and what happened.**
4. **What you have assessed, and how.** Name the instruments and the sessions.
5. **Risk, stated explicitly.** Not "no concerns" — say what you asked about and what was reported.
6. **What the family has been told about the referral, and whether they agree.** Never refer behind a family's back except where safeguarding requires it, and then say so.
7. **What you are asking the recipient to do, and by when.**
8. **Your contact details and your availability for a conversation.**

What families experience. A referral is often experienced as rejection: "she has given up on us". Two sentences prevent this. First, name the referral as an addition rather than

an ending: "I am not stopping work with you; I am bringing in someone who knows more about this than I do." Second, be specific about what continues: "we will still meet on the 14th, and I want to hear how the appointment went."

Referral failures worth recognising.

- **The hot potato.** A family referred to four services in a year, each of which assessed them and passed them on. The pattern is usually a sign that the presenting problem is relational and each service is looking for an individual diagnosis. The systemic response is to convene a meeting of the involved professionals — with the family's consent — so that the network is treated as a system too.
- **The referral that transmits a verdict.** "Mother is enmeshed and resistant" in a referral letter becomes the next clinician's premise. Write observations, not verdicts.
- **The referral that abandons.** A family referred to a service with a two-year waiting list, and no interim plan. If the wait is long, the referral needs a holding plan, and the family needs to be told what to do if things get worse.
- **The referral to nowhere.** A service that does not exist, or does not take that kind of case. Check before you promise.

5.9 Money, contracts and the ordinary ethics of practice

Therapy is a service delivered in an economy. Students are often taught to be embarrassed by this, which produces two failure modes: therapists who cannot discuss money and therefore resent their clients, and therapists who optimise for income and call it efficiency.

Fees. Three principles. First, the fee is agreed before the work begins, in writing, including what happens to missed sessions. Second, the fee should be one you can charge without resentment: a therapist who feels underpaid becomes subtly punitive. Third, sliding scales and pro bono work are legitimate and should be structured rather than improvised — a fixed number of reduced-fee places, reviewed at intervals, protects both the client and the therapist.

Missed sessions. The policy should be simple, written, and applied consistently: for example, 48 hours' notice, otherwise the session is charged; exceptions for emergencies, at the therapist's discretion. The point is not the money; it is that an unclear policy produces resentment, and resentment produces bad therapy.

Records. Keep notes that are accurate, dated, and written as though the family and a court will read them — because both may. Record what was observed, what was said, what was decided, what was risk-assessed, and what the plan is. Do not record speculation as fact, do not record your irritation, and do not record information that is irrelevant to care. Retention periods and access rights are jurisdiction-specific: know yours, and tell families at the start.

Contracts with third parties. When an employer, a school, a court or a donor pays, the client is not the only party with a claim. The professional answer is not to pretend otherwise but to write it down: who the client is, what will be shared, what will not, and

what happens if the interests diverge. Families can understand this if it is said plainly in the first session; they cannot forgive discovering it later.

> **PRACTICAL**
> Write your own practice policy on one page: fees, missed sessions, confidentiality and its limits, records and retention, contact between sessions, and what you do in an emergency. Show it to a peer and ask them to find the three sentences a distressed family would misread. Rewrite those three.

5.10 The law, at the level a therapist needs

Legal detail belongs to MFT 704 and MFT 724. What belongs here is the shape of the therapist's legal exposure, because it determines how you practise in a setting.

Consent. Sessions with minors require the consent of a person with parental responsibility, except in specific statutory situations. Separated parents may each hold rights; a therapist who assumes the custodial parent speaks for both can find themselves in a dispute they did not anticipate. Establish, in writing, who has consented and who has been informed.

Confidentiality and its limits. Every family should be told, in the first session, in plain language, the circumstances in which you would share information without their consent: risk of serious harm to a child or vulnerable adult, risk of serious harm to self or others, and where a court orders disclosure. Then — and this is the part students forget — you must be able to say what you would do, not merely that there are limits.

Records and disclosure. Notes can be requested by courts, insurers, employers and the family themselves. Write accordingly. Where a request arrives, the correct response is rarely immediate compliance or immediate refusal: it is to seek advice, to inform the family where lawful, and to disclose only what is required.

Reports. A report written for a court or a school is a professional document with consequences. Three rules: state your qualifications and the limits of your assessment; separate observation from opinion; and never express an opinion outside your competence, however strongly you are pressed.

Duty to report. Many jurisdictions impose mandatory reporting of suspected child abuse. Know whether yours does, know the threshold, and know that a therapist's own judgement about whether a report will "help the family" is not a lawful basis for withholding one.

5.11 The long career: supervision, CPD and burnout

The profession's most common casualty is not malpractice but attrition. Therapists leave because they are exhausted, isolated, under-supported and under-paid, and the families who need experienced clinicians lose them.

Supervision. Not optional, not a sign of deficiency, and not the same as management. A supervisor is responsible for the quality and safety of your work; the relationship should

be contracted explicitly — frequency, cost, what is recorded, what happens in a disagreement, and how the supervisor would respond to a concern about your practice. Group supervision adds perspective and costs less, but individual supervision is necessary at points of difficulty. MFT 720 covers the practice in depth.

Continuing professional development. A three-part plan is more useful than a course list: one depth activity (a long training in a model you use), one breadth activity (something outside your model, to keep you honest), and one evidence activity (reading primary research in your area). Review annually. The trap is accumulating certificates in fashionable approaches while never deepening a single competence.

Burnout, and how it actually presents. Not as cynicism alone. Early signs: sessions feel longer; you stop writing notes properly; you notice you are avoiding a particular family; the families begin to blur; you are irritable at home; you feel relief when someone cancels. Later signs: dread before sessions, sleep disturbance, loss of empathy, intrusive thoughts about a case, and a private conviction that you are fraud.

What actually helps. Caseload limits you can defend; a peer group that meets reliably; supervision that is about the work rather than about your personality; attention to the practical (sleep, money, commuting); personal therapy when the material is touching your own history; and the professional courage to say "I cannot take this case" before you take it.

> **CAUTION**
> Vicarious trauma is an occupational hazard, not a character weakness, and the therapists most at risk are often the most conscientious. If you notice yourself avoiding a case, that is data. Take it to supervision before it becomes a decision you make alone.

5.12 Annotated transcript: the contract conversation

The first five minutes of a first session, in a faith-based setting, where the pastor has referred the family and is present at the start.

> **T1 Therapist:** Before I ask about the difficulty, I want to be clear about who I am and what happens to what you tell me. May I take five minutes on that?
> **T2 Father:** (to the pastor) She is the one you said would help.
> **T3 Therapist:** I am. And I want to be honest about my part. I am a family therapist; I am not a pastor, and I will not lead prayers. Pastor, you are here at the start because you made the referral, and my suggestion is that after today you are not in the room, unless the family wants you there for a particular session.
> **T4 Pastor:** That is fine. I only want to know that they are being helped.
> **T5 Therapist:** I will tell you whether they are attending and whether the plan is being followed. I will not tell you what they say about each other, and I will not give you a report. Is that acceptable to everyone?
> **T6 Mother:** (quietly) Yes.
> **T7 Therapist:** Two more things. If I believe a child is at risk of harm, I will have to act, and I will tell you what I am doing and why. And my notes are my records; you may ask to see what I have written about you.

Annotations. T1 asks permission for the contract conversation, which is both courteous and structural: it signals that the therapist has a procedure and is not purely reactive. T3 does three things at once — states the professional identity, declines the pastoral role explicitly (which respects the pastor rather than competing with him), and proposes a boundary for the pastor's presence. Note that the boundary is proposed, not imposed: "my suggestion is".

T5 is the most important sentence in the exchange. It distinguishes attendance and adherence (legitimately shared with a referrer) from content (not shared without consent). Most boundary failures with referrers are failures to make exactly this distinction.

T6 — the mother's quiet "yes" — should be noticed. The therapist does not pursue it in front of the pastor, but notes it and returns to it in the first individual moment available. The quietest voice in a contract conversation is often the one who will later disclose the most serious material.

T7 states the limits of confidentiality and records in two sentences, in plain language, before the family has said anything they might regret. Students often leave this to the end of the session, when the family is committed and the disclosure has already happened; that ordering is both less ethical and less effective.

5.13 Laboratory: the scope-of-practice tribunal

A ninety-minute exercise in which students practise refusing, referring and negotiating — the three skills nobody enjoys practising and every professional needs.

Setup. Groups of four. Three students receive a scenario card; one is the "registrar's assessor". Each scenario contains a plausible, urgent request that exceeds the therapist's scope, plus pressure to comply (a senior colleague, a distressed family, a deadline).

Scenario examples.

- A school principal asks you to confirm in writing that a pupil is "not a risk" so that he may return to school tomorrow, after an incident you did not witness and a family you have met once.
- A mother asks you to see her 14-year-old without telling the father, from whom she is separated.
- A judge's clerk asks you to send your case notes by Friday, including notes of individual sessions.
- A pastor asks you to tell him whether the wife in a couple you are seeing is "telling the truth" about her husband's violence.
- A hospital consultant asks you to "sign off" a family's capacity to care for a dying relative at home.

The task. The therapist must: identify which circle (competence, authorisation, mandate) is missing; say what they can offer; make one referral or consultation; and put the essential exchange into two written sentences. Six minutes per scenario, then the assessor gives a verdict.

Marking rubric.

Criterion	Weight	Full marks
Identification of the missing circle	25%	Names competence, authorisation or mandate precisely, not vaguely
Offering an alternative	25%	Offers something specific and useful; does not merely refuse
Boundary language	20%	Courteous, brief, no over-explanation, no moralising
Written record	20%	Two sentences that would protect the family and the therapist
Pressure handling	10%	Holds the boundary under at least two escalations without becoming defensive

5.14 Six errors in professional identity

1. **The competence-authorisation confusion.** Doing something you are good at but not permitted to do. Fix: know your three circles and check them before you accept a referral.
2. **The invisible contract.** Working in a setting without ever making its terms explicit to the family. Fix: the five-line contract conversation, in the first session, every time.
3. **The ambitious referral.** Promising a service you have not confirmed exists. Fix: check, then promise.
4. **The title stretch.** Using a protected or ambiguous title because it sounds more credible. Fix: describe the work, not the label.
5. **The solo practitioner.** Working without supervision, peer contact or a route to advice. Fix: build the network before you need it; the moment you need it is too late to build it.

6. **The unpaid martyr.** Accepting a caseload that cannot be sustained, then resenting the families. Fix: a written caseload policy and the discipline to say no.

5.15 Culture, faith and the professional's own position

Three observations that belong to this week rather than to the culture course, because they are about the profession's identity rather than the family's.

The imported profession. In many countries the structures of professional practice — registration, title protection, insurance, supervision contracts — were modelled on Northern institutions and do not map cleanly onto local realities of practice, funding and authority. A therapist working in a context where most families see a pastor first, where few can afford private fees, and where the state provides little, is practising a profession whose formal infrastructure is thin. The honest response is neither cynicism nor imitation: it is to build what is missing locally — peer supervision groups, referral networks, written policies, outcome measurement — rather than waiting for a regulator to arrive.

The class position of the therapist. You will usually be more educated and better paid than the families you see, and you will be consulting them in a room they had to travel to. This asymmetry is not dissolved by friendliness. It is managed by transparency: explaining what you are doing and why, offering choices, naming your own limits, and refusing to make decisions that belong to the family.

Faith and the therapist's own convictions. A therapist with strong religious commitments will meet families whose beliefs differ, and a therapist with none will meet families for whom faith is the organising fact of life. Both need the same discipline: the family's beliefs are part of the system you are assessing, not a position you are required to endorse or correct. If a family's belief leads to a child being denied medical care, your obligation is to act under the law and safeguarding framework — and to do so in a way that keeps the family in contact with help rather than driving them underground.

> **REFLECT**
> Write 300 words on the kind of professional you intend to be in five years: what you will be competent in, what you will refuse, who you will consult, and how you will know you are still learning. Keep it; you will compare it with reality at the end of the programme.

5.16 Evidence note: does professionalisation improve outcomes?

A question students rarely ask and should: does all this — registration, codes, supervision, CPD — actually make therapy better for families?

What the evidence suggests. Therapist effects are large: in most psychotherapy datasets, a small proportion of therapists account for a disproportionate share of outcomes, and the variance attributable to the therapist exceeds that attributable to the model. Training and deliberate practice predict skill acquisition; experience alone does not reliably predict better outcomes, and some therapists decline over time. Routine outcome monitoring, with feedback to the therapist, improves outcomes and reduces deterioration, particularly for therapists whose clients are deteriorating. Supervision is

mandated almost everywhere and is much less studied than its ubiquity would suggest; the available evidence links good supervisory alliance to better outcomes, and links supervision that is punitive or absent to worse ones.

What this implies for professional structures. The mechanisms most likely to protect the public are not titles but practices: measurement of outcomes, feedback loops, supervision with a genuine authority to intervene, and a culture in which a therapist can say "this case is not going well" without career damage. Registration without these practices protects the profession's status more than it protects families.

What students should demand of their own training. Regular outcome measurement in your placements; supervision that watches your actual work, not only your account of it; and honest discussion of your failures. If your training does not provide these, the professional habit to build is to provide them for yourself.

5.17 Key terms

Term	Definition
Profession	An occupation with specialised knowledge, prolonged training, a code, enforcement, and a service orientation
Registration / licensure	Legal recognition of who may use a title and practise an activity
Scope of practice	The intersection of competence, authorisation and mandate
Mandate	What the client, referrer or setting has asked you to do
Fiduciary duty	The obligation to put the client's interest before your own
Duty to report	A statutory obligation, in some jurisdictions, to report suspected abuse
Parental responsibility	The legal authority to consent on a child's behalf
Sliding scale	A structured reduced-fee arrangement
Vicarious trauma	Cumulative harm to the therapist from exposure to clients' trauma
Burnout	Exhaustion, detachment and reduced efficacy resulting from chronic occupational stress
Therapist effects	Variance in outcome attributable to the individual therapist rather than the model
Routine outcome monitoring	Session-by-session measurement of outcome and alliance, fed back to the therapist

5.18 Self-test

1. Name the six criteria of a profession and identify the two weakest for family therapy in most jurisdictions.
2. A family asks you to testify that a father is "safe" so that a court will allow unsupervised contact. Which circle is missing, and what do you offer?
3. What is the difference between competence and authorisation? Give an example of each.

4. What should a referral letter contain? Name five of the eight elements.
5. Why is a written missed-session policy a clinical matter and not merely an administrative one?
6. What is the strongest evidence-based argument for routine outcome monitoring?
7. What are the early signs of burnout, and what is the first thing you should do?
8. Why might registration alone not protect the public?

Answers. 1. Specialised knowledge; prolonged training; code with enforcement; registration; autonomy; service orientation. Weakest: registration and enforcement. 2. Authorisation (and forensic competence); offer a clinical opinion on the child's needs and recommend a forensic assessment. 3. Competence is what you can do well; authorisation is what you are permitted to do — for example, being skilled at assessment but not authorised to diagnose in that jurisdiction. 4. Any five of: who you are and the request; the presenting problem in both languages; what you tried; what you assessed and how; risk; what the family has been told; what you want and by when; contact details. 5. Because an unclear policy produces resentment, and resentment is enacted in the therapy. 6. Feedback improves outcomes and reduces deterioration, especially for therapists whose clients are worsening. 7. Sessions feel long, notes slip, avoidance of a particular family, families blurring, irritability, relief at cancellations; take it to supervision immediately. 8. Because titles do not change practice; outcome measurement, supervision with real authority, and a culture that tolerates admitting failure do.

5.19 Readings for this week

- **AAMFT Code of Ethics** (current edition). Read it as a document about power, not as a list of rules.
- **Corey, Corey & Callanan**, Issues and Ethics in the Helping Professions, chapters on competence and boundaries. The standard orientation text.
- **Strupp & Anderson (1997)**, on the therapist as a person, and the contemporary literature on therapist effects (for example the reviews by Wampold and by Barkham). Why who you are matters as much as what you do.
- **Lambert et al.**, on routine outcome monitoring and feedback. The strongest practical case for measurement.
- **Local professional association guidance** for your jurisdiction: registration requirements, codes, complaint procedures. Read the actual documents rather than the summaries.
- **Roberts (2005)**, Crisis Intervention Handbook, chapter on settings — useful on institutional contracts.
- **Your own jurisdiction's registration and complaints guidance.** Read the complaints procedure, not only the code of ethics: it tells you what the public can actually expect of you, what kind of failure is treated as serious, and how long a determination takes. Students are routinely surprised by both the scope and the pace of the process.

Next: Week 6 — The first session I: joining

5.5a Two more settings: private practice and telehealth

Two settings have grown fast enough to need their own note, because each changes the professional contract in ways that catch newly qualified therapists unprepared.

Private practice. The contract: the family pays directly, self-refers, and may leave at any time. What this affords: continuity, flexibility, and freedom from institutional definitions of the problem. What it demands: a business infrastructure — a written agreement, a fee policy, a records system, insurance, a referral network, and a plan for what happens when the therapist is ill or on leave. The characteristic failures: isolation (no team, no supervision, nobody to notice deterioration); over-acceptance (taking every referral because income is uncertain, and ending up with a caseload beyond competence); and the boundary erosion that comes from being reachable by WhatsApp at all hours. The professional answers are unglamorous and effective: a fixed number of clients, a supervision contract regardless of whether anyone requires it, an answering policy that states your response times, and a rule that clinical content is discussed in sessions and not in messages.

Telehealth. The contract: the same clinical relationship, delivered through a medium that changes what can be seen and who can be present. What this affords: access for families separated by distance or work, and a window into homes that a clinic never provides — you will see who walks through the room, who is listening off-camera, and what the household actually looks like. What it demands: explicit work on privacy (who else is in the house, who can overhear), on safety (you cannot physically intervene; you need a local contact and an address at the start of every session), on consent (recording, and the limits of confidentiality on a platform), and on the technical frame (a plan for disconnection). The characteristic failure: treating a video session as though it were a clinic session with a camera on it, and missing both the advantages and the risks.

A practical telehealth checklist for the first session. Confirm the full address of each participant; ask who else is in the house; agree a signal for "I cannot speak freely"; agree what happens if the connection drops; confirm that the family understands the limits of emergency response at a distance; and record the consent. Ten minutes of setup prevents the two most serious telehealth failures: a disclosure overheard by someone who should not have heard it, and a crisis in which the therapist does not know where the family is.

5.7a The network as a system: case management when several services are involved

Families with complex difficulties are usually known to several services at once: a school, a clinic, a social worker, a church, sometimes a court. This network behaves like a system, with the same properties you learned in Week 4 — boundaries, rules, triangles, and a tendency to nominate one member as the problem.

The observable signs that a network has become a system problem. Professionals criticise each other in front of the family; the family tells each professional a different story and each believes theirs; there are four assessments and no plan; the family is described by every service in the same negative terms ("non-engaged", "chaotic",

"resistant"), which is usually a description of the network's failure rather than the family's; and meetings are held about the family rather than with them.

The intervention. A network meeting, convened with the family present, with four agenda items and nothing else: what each service has observed; what each service is doing; what the family wants; and who will do what by when. Two rules make it work: no service may describe the family using adjectives it cannot evidence with an observation, and the family speaks first.

Why this belongs in a week about professional identity. Because the most common cause of a family being failed by services is not any single clinician's incompetence; it is the absence of anyone whose role includes the network. In many teams, that role is nobody's. The family therapist, trained to think in systems, is often the best-placed person to take it — provided they take it explicitly, rather than doing it invisibly and carrying the resentment that invisibility breeds.

5.11a Writing your first professional documents

New graduates are usually competent in the room and completely unprepared for the paperwork, which is where most complaints begin. Three documents, with the standard each should meet.

The client agreement (one page). What you offer, how long sessions last, the fee and cancellation policy, confidentiality and its limits, how records are kept and who may see them, how to reach you between sessions and what to do in an emergency, and a signature line for each adult and for the guardian of any minor. The test: a frightened person with no professional training should be able to read it once and know what to expect.

The case note (five lines per session). Date and attendees; presenting issue in the family's words; what you observed and did (including risk screening, explicitly); your formulation in one sentence; the plan and who is responsible. Written in the past tense, without speculation presented as fact, and without characterisations you would not want read aloud. The test: could a colleague who has never met this family pick up your file and continue safely?

The report (two pages). Who asked for it and what question it answers; who you saw, when, and with what consent; what you did and how; what you observed; your opinion, clearly labelled as opinion, with its limits; and recommendations that are specific, feasible and addressed to someone who can act. The test: would you be comfortable having every sentence read aloud in a hearing?

> **AUDIT**
> Take one page of any notes you have written, in a placement or a role-play, and apply the three tests above. Most students find at least one sentence that is speculation recorded as observation. Rewrite it.

5.12a The professional's first year: a survival map

The first year after qualification is the most dangerous for the practitioner and, indirectly, for clients. Five predictable pressures and the response that works.

Pressure 1 — the caseload creep. You accept a referral because you can help, then another, then a family you should have referred. Response: a written maximum, reviewed quarterly, and a rule that any exception is discussed with a supervisor before it is accepted.

Pressure 2 — the isolated practice. You are the only systemic clinician in your setting, so nobody can check your thinking. Response: buy or negotiate external supervision; join or start a peer group of three to five clinicians; commit to one case presentation a month.

Pressure 3 — the impossible mandate. Your employer wants throughput, the family needs depth. Response: negotiate in writing what the service can and cannot deliver in the time available, and record the consequence for families (waiting lists, brief work, referral elsewhere). The record protects the families who come after yours.

Pressure 4 — the case that follows you home. Response: a boundary ritual after work (a walk, a change of clothes, a specific moment of transition); a named person you can telephone; and the discipline of writing the note before you leave, which is both a legal and a psychological act.

Pressure 5 — the identity crisis. Six months in, you know more about what you cannot do than about what you can, and you begin to suspect the training was inadequate. Response: this is normal and is the beginning of real competence. Track your outcomes, review recordings of your own work with a supervisor, and notice that the families are still coming back. Competence is built in the second and third year, not the first.

Week 6 — The first session I: joining

6.1 The lecture in ninety minutes

This week is the assessed skills gate of the course, and it is also the week that separates students who understand family therapy from students who can do it. Joining is not a warm-up before the real work; it is the work, in its most compressed form. Everything that follows depends on whether this family will come back, and whether they will let you see what is actually happening.

The lecture has a simple structure: what joining is, why the first session is an outcome event, then four live demonstrations with the class in role.

Minutes	Segment	What the lecturer does	What students do
0–10	The evidence	Dropout figures; what predicts return	Predict which of four openings loses the family
10–25	What joining is	Minuchin's three operations: maintenance, tracking, mimesis	Mark them in a transcript

Minutes	Segment	What the lecturer does	What students do
25–40	The architecture	Pre-session, opening, contract, agenda, closing	Draft their own six-step opening
40–60	Four hard families	Reluctant father, silent adolescent, furious mother, polite family	Role-play one, in triads
60–75	Multi-alliance	Pinsof's dimensions; the balance problem	Diagnose an alliance failure on video
75–90	The gate	Rubric, what is assessed, what fails	Self-assess against the rubric

6.2 What joining is

Salvador Minuchin described joining as the therapist's effort to become part of the family system in a position from which change is possible. It has three technical operations, and students should be able to name them while doing them.

Maintenance. You affirm and support the family's existing structure and rules while you work. You do not challenge the father's authority in the first ten minutes; you do not correct the mother's description of her son; you do not require the family to reorganise itself in order to accommodate you. Maintenance sounds passive and is not: it is a deliberate choice to earn the right to disturb.

Tracking. You follow the family's own content, language and imagery rather than steering towards your agenda. If the mother talks about the kitchen, you ask about the kitchen. Tracking communicates that the family is being listened to by someone who can follow, which is rarer than students imagine and more persuasive than any technique.

Mimesis. You adopt the family's style — pace, formality, volume, vocabulary, dress, seating — enough to be experienced as familiar, not enough to be experienced as mockery. A loud family with a quiet therapist will either shout louder or conclude the therapist is judging them; a quiet family with a loud therapist will conclude the therapist is unsafe.

What joining is not. It is not agreement. It is not collusion with the family's account. It is not friendliness, though friendliness helps. It is not a phase you complete and leave behind: you re-join every session, and after every intervention that disturbs the system you re-join again. The rhythm of a well-conducted session is join — disturb — rejoin, repeated.

The test of successful joining. The family tells you something they did not plan to tell you, and they book another appointment. Those are the only two reliable indicators, and they typically occur within the first session or not at all.

6.3 Why the first session is an outcome event

Students often treat the first session as data collection and assume the real work starts later. The research does not support that model.

Dropout happens early. Across outpatient services, a substantial minority of families attend one session and never return; a larger group attends fewer than four. The single

strongest predictors of return are the early alliance and the fit between the therapist's explanation and the family's own understanding of the problem. Practical factors — distance, cost, timing, childcare, whether the father can get leave — matter enormously and are frequently ignored by trainees who interpret non-attendance as resistance.

The alliance is multi-dimensional. Pinsof's system for couple and family therapy distinguishes the individual alliance of each participant with the therapist, the subsystem alliance (for example, with the couple as a unit), and the whole-system alliance, across three domains: the content of the work, the goals and the tasks. A family can be high on one dimension and low on another: the mother feels understood (individual alliance high), the couple feels ganged up on (subsystem alliance low), and the family as a whole is uncertain that therapy can help (whole-system alliance low).

The practical consequence. You must monitor at least three alliances, and you cannot do it by intuition. Two habits make it possible. First, at the end of a first session, ask each person directly: "Was there anything about today that did not fit, or that you would want me to do differently?" Second, note, after the session, which member you spoke to most, which member you interrupted, and whose explanation you adopted — because the recording usually contradicts your memory.

> **CAUTION**
> The most common alliance failure in the first session is not hostility; it is over-agreement with the person who speaks first. The talkative member's formulation becomes the therapist's, and the quiet member concludes that the room belongs to someone else. Count your questions by addressee after every first session for a month; the numbers will surprise you.

6.4 Preparing the session

The first session begins before the family arrives. Six decisions, each of which changes what happens.

1. What you know, and what you refuse to conclude. Read the referral; note the referring person's formulation; write down two competing hypotheses before the session, and one question that would test each. This is the discipline that prevents the referral letter from becoming your assessment.

2. Who you invite. The people in the household, plus anyone with real authority over the problem — a grandmother who co-parents, an aunt who pays the fees, sometimes a pastor. The invitation should explain why: "I want to hear how it looks from everyone, because one person's view is never the whole picture." Where someone refuses to attend, ask the family how they would like that refusal handled, and consider whether the refusal is itself the central clinical fact.

3. The room. Chairs that can be moved; no desk between you and the family; tissues within reach; a clock you can see; something to write on that does not look like an assessment form. Seating matters more than students expect: a couple separated by a

child is already saying something, and where you sit either accepts that arrangement or quietly reconfigures it.

4. The interpreter. If a family needs one, use a trained interpreter, not a child, and brief them beforehand: first person, everything said, no side conversations, and the right to tell you when a phrase does not translate. See MFT 708 for the fuller protocol.

5. Your own state. Check your own distraction before the door opens. A therapist who is running late, hungry, or preoccupied with the previous session will join badly and will blame the family for it.

6. The paperwork. Consent, the practice agreement, the limits of confidentiality, and the recording consent — before the clinical work, not after. Twelve minutes of housekeeping at the start is twelve minutes that protects everyone in the room.

6.5 The opening ten minutes: an architecture

There is no single correct opening, but there is a shape that reliably works, and trainees are well served by practising it until it is automatic and can then be varied deliberately.

Step 1 — orient (1 minute). "I know a little from the referral, and I want to hear it from you. Before we start, is there anything you want to ask me?"

Step 2 — contract (3 minutes). Who you are; what you do; how long sessions last; how many sessions you expect to need before reviewing; what happens between sessions; confidentiality and its limits, stated concretely; and what you will do in an emergency.

Step 3 — agenda (2 minutes). "I would like to hear from each of you what you think is going on, and what you would want to be different. Then I will say something about how I understand it and what I would suggest. Is that all right?" The agenda is proposed, not imposed; the family may change it, and often does.

Step 4 — the account (25–35 minutes). Each person in turn, with the quieter members invited explicitly and early: mothers usually speak first and children usually last, and the reverse order is often more informative. Use episode questions, not opinion questions. Notice who answers for whom.

Step 5 — the sequence (10–15 minutes). Take one specific episode and slow it down until the loop is visible and the family recognises it. Do not interpret; ask until they describe it themselves.

Step 6 — the feedback (10 minutes). Offer your formulation as a hypothesis, in plain language, with an explicit invitation to correct it: "Here is what I think I am seeing; tell me what I have got wrong." Then ask for one small thing to be different this week, and book the next session.

Step 7 — the closing question (2 minutes). "Was there anything today that did not fit?" This is not politeness; it is alliance monitoring, and it produces information no questionnaire will.

6.5a Reading a family's style in seven minutes

Experienced therapists appear to join effortlessly because they are reading the family's style from the moment the door opens, and adjusting before they speak. The skill is teachable if you know what to look for. Seven dimensions, each with the adjustment that follows.

1. Pace. Fast, overlapping, interruptive — or slow, formal, with long silences. Adjustment: match it for five minutes, then slow the fast family by about a third and hold the slow family's pace without filling their silences. A fast family experiences a slow therapist as a barrier; a slow family experiences a fast therapist as an intruder.

2. Volume and affect. Loud and expressive, or quiet and restrained. Adjustment: join at their level and come down gradually. Do not begin quieter than the family, and do not begin louder: both read as judgement.

3. Formality. Titles, dress, posture, whether humour is permitted. Adjustment: err on the formal side for the first ten minutes, then follow the family's lead. It is easy to become less formal; it is almost impossible to recover from being too familiar too early.

4. Who speaks for whom. The mother who answers for the father; the father who answers for the child; the child who translates. Adjustment: interrupt the pattern once, politely, in the first fifteen minutes, by addressing the person directly: "I would like to hear it in your words." One interruption establishes that individual voices exist; repeated interruptions establish that the therapist is at war with the family's structure.

5. The family's theory of the problem. Medical ("it is a chemical imbalance"), moral ("he is stubborn"), spiritual ("there is a curse"), systemic ("we all wind each other up"), or fatalistic ("this is just how our family is"). Adjustment: adopt the family's language for the first session while keeping your own hypothesis private. A family whose theory is spiritual will not hear a neurochemical explanation in the first hour, and a family whose theory is medical will hear a relational explanation as blame.

6. The emotional temperature of the room. Who is crying, who is frozen, who is watching everyone else, who is trying to make the therapist laugh. Adjustment: address the most distressed person first and the most watchful person second; the watchful one is usually the family's emotional barometer and will tell you, in the car park, what everyone really thinks.

7. The relationship to help. Has the family been in therapy before? Was it useful? Were they blamed? Were they reported? Adjustment: if the family has been harmed by services, name it early and explicitly: "It sounds as though professionals have not been much use to you. I would like to know what went wrong, so that I do not repeat it." Nothing joins a previously harmed family faster than an accurate account of their previous experience.

A caution about reading style. Every one of these dimensions is culturally patterned, and the therapist's own style is not neutral. If you find yourself concluding that a family is "cold", check whether you are reading restraint as absence; if you conclude a family is "chaotic", check whether you are reading a different cultural norm of overlap as disorder. The dimensions are useful for adjustment, dangerous as diagnoses.

6.6 Joining each member

Each position in the family has its own way of being joined, and the same move that joins one member can alienate another.

The mother who carries the worry. Usually the referrer, usually the most motivated, usually the person whose account has already been adopted by everyone including the school. Joining her means taking her seriously and not adopting her formulation wholesale. The sentence that works: "You have been carrying this on your own for a long time, and I want to understand how you have managed. I also want to hear how it looks from the others, because I suspect there is more going on than any one person can see."

The father who is absent or reluctant. He arrives expecting to be blamed, because he usually has been. Joining him requires making him necessary rather than merely welcome: "I cannot do this without you, because you are the only person who can tell me what happens in this house when your wife is not in the room." Ask him about his work, his own father, his view of what has gone wrong — before you ask him to change anything. A father who is interviewed as an expert on his own family will often stay; a father who is interviewed as a defendant will not.

The adolescent. She has been brought, often against her will, and she has been discussed by professionals before. Joining her means giving her status and privacy: ask her permission before asking about her; offer the option of a session alone; ask what she thinks the adults have got wrong. The single most effective sentence with a reluctant adolescent is: "I am not here to make you talk about things you do not want to talk about. I would like to know what you think is going on, even if it is that this is a waste of time."

The child. Speak to the child directly, early, at their level, and about their own experience — not about the parents' problem. Ask what they notice, what they would change if they had a magic wand, and what they do when the house is tense. Children are reliable reporters of sequences and unreliable reporters of feelings under interrogation; ask for the sequence.

The grandparent or kin who holds authority. Join them as an elder, not as a complication. Ask for their view of the family's history, because they usually have information that nobody else has; ask what they think the younger generation has got wrong; and negotiate their role explicitly, since a grandmother who is present but not consulted will reassert her authority at the worst possible moment.

The person who is not there. The absent member is joined through questions: "What would your husband say if he were here?" "Who would disagree with that?" The answers keep the absent member in the system and stop the present members from speaking as though the family were complete.

> **PRACTICAL**
>
> In triads, practise joining a reluctant father. The father's opening line is: "I don't know why I'm here. She's the one with the problem." You have six minutes and three moves. Then swap. Observers record how many of your questions addressed him, and whether any of them positioned him as an expert.

6.7 Handling four hard families

The furious family. They arrive angry — with the referrer, with each other, with you for being the fourth professional to see them. The joining move is to name the anger and legitimise it before doing anything else: "Something has gone badly wrong for you before you got here, and I would like to know what it was." Do not defend the profession, the referrer or yourself. Give the anger somewhere to go — a structured account of what happened — and it usually converts into engagement within ten minutes.

The polite family. They present no problem, agree with everything, and give you nothing. Politeness is often a rule ("we do not argue in this family") rather than a personality. The joining move is to name the rule respectfully and test it: "You are all being very careful with each other. Is that how it is at home?" Then ask what would happen if someone disagreed in the room. The first disagreement is the clinical event.

The family that fights in the session. They continue the argument they were having in the car. Joining means structuring, not stopping: "I want to hear this argument, but slowly. Start again, and one of you speak at a time; I will tell you when to pause." Slowing the exchange is itself an intervention, and it converts a destructive argument into observable data.

The family in crisis. Something has just happened — a death, an arrest, a disclosure, an eviction. The structure of the session changes: safety and practicalities first, formulation later. The joining move is to acknowledge the acute situation and to ask what they need in the next forty-eight hours before asking about patterns. MFT 712 covers crisis work properly; here the point is that joining in a crisis means being useful before being insightful.

6.8 The Adeyemi first session, in full

The case has appeared in previous weeks; here it is as a first session, minute by minute, with the reasoning that a trainee would not see.

Before the session. Referral from the school counsellor. Two hypotheses written down: (a) the school refusal is maintaining a caregiving role for the mother; (b) the refusal is a protest against a parental conflict that the family will not discuss. One question to test each: (a) "What does Amara do when her mother is having a bad day?" (b) "What happens between the two of you when Amara is at school?"

Minutes 0–5. Mother and Amara arrive; the father has not come. Contract stated; both agree to proceed. The therapist notes the absence and asks a single question about it — "Is your husband aware you are here today?" — and does not pursue it. The answer ("he knows, he had work") is recorded as data, not challenged.

Minutes 5–20. The mother's account. The therapist tracks: asks about the school, the mornings, the mother's own health, and what she has tried. She uses maintenance: "You have been doing all of this on your own." She asks Amara twice, early: "Is that how it looks to you?" Amara says "mm" and looks at her mother. The therapist notes it and does not press.

Minutes 20–35. The pivot questions from Week 2: "Before Amara stopped going to school, what was happening in the house?" and then "Who else in the family knows how bad things have been?" The mother cries. The therapist acknowledges, offers tissues, waits. Then, to Amara: "When your mother is having a bad day, what do you do?" Answer: "I stay with her."

Minutes 35–50. The sequence is slowed: what happens in the morning, who says what, what Amara does, what her mother does, what her father does when he is there. The therapist draws nothing yet; she keeps the sequence in words so that the family can hear themselves describing it.

Minutes 50–60. Feedback as hypothesis. Three sentences: what she has seen; what she thinks it might mean; what she is less sure about. Then the invitation to correct: "Have I got that wrong?" The mother corrects one detail (the father is more involved than the therapist has assumed) and the therapist thanks her and writes it down — visibly, because visible correction teaches the family that the formulation is theirs to revise.

Minutes 60–70. The plan. A session with both parents, framed as a request for the father's expertise. A specific morning task, small and observable. A conversation with the school, with consent. A review in two weeks.

After the session. Notes written immediately: who attended, what was observed (two answers-for-another, one look-to-mother-before-answering), risk screening (asked about self-harm and violence; nothing disclosed), the formulation in one sentence, the plan. The therapist also writes one line for supervision: "I did not ask about the father's drinking today, and I need to decide when and how."

6.8a The Okonkwo first session, contrasted

The Adeyemi session above shows joining a family that arrives motivated and sad. The Okonkwo session shows joining a family that arrives defended, and the two together cover most first sessions a trainee will meet.

Referral. Ebuka, 9, "stubbornness". Mother attends; father does not. The mother opens with a complaint about the school, then about her son, then, in the fourth minute, about her husband, then apologises for complaining.

What is different. The mother arrives with a verdict rather than a question: the school says the child is the problem, she half-believes it, and she is braced to be told she is a bad mother. The therapist's first task is not to gather information but to lower the threat level.

Minutes 0–8. The therapist does three things before asking about Ebuka at all. She thanks the mother for coming, and mentions that coming to a clinic about a child is a decision most parents put off. She asks what the mother was told by the school in the school's words — "stubborn, refuses to write" — and asks what she thought when she heard it. The answer ("I thought they were saying I cannot control my own son") is the true opening of the session, and it took six minutes to reach.

Minutes 8–20. The therapist asks Ebuka, directly and first, what a good day at school looks like. He says "no writing". She asks what happens when the teacher gives him the

book; he demonstrates, physically, pushing it away. She asks what his teacher does then. He says she calls his mother. The sequence is now: worksheet → push away → teacher calls mother → mother is angry → evening reparation. The therapist has not mentioned stubbornness once.

Minutes 20–35. She asks the mother about the mornings, then about her week, then, gently, about her own support: who helps her? The answer names a sister, a neighbour, a church group, and, with a pause, "not my husband, he is working". The therapist does not pursue the husband yet. She asks: "When Ebuka is at school and writing is not happening, what is your day like?" The mother, unexpectedly, cries.

Minutes 35–50. The therapist works with the sequence rather than the diagnosis: what happens in the ten minutes before the worksheet comes out; what happens in the ten minutes after; what Ebuka does when his mother is upset. She draws nothing and interprets nothing. Twice she says, "I may be wrong about this — tell me", and twice the mother corrects her, which is exactly the outcome she wants.

Minutes 50–60. Feedback. Three sentences of hypothesis, one of which is about the marriage, offered carefully and framed as a question the family can decline. Then a plan with three parts: an assessment of whether there is a writing-specific difficulty (a legitimate referral, not a dismissal of the systemic view); a morning routine the mother can run without escalation; and an invitation to the father framed as expertise.

The contrast, summarised. With the Adeyemis, joining meant widening — bringing in the father, the school, the grandmother, and the marital conversation. With the Okonkwos, joining meant lowering the threat — taking the verdict off the child and the mother before any widening was possible. Same skill, opposite first move. Students who learn one template and apply it to both families will fail with one of them.

6.9 Annotated transcript: joining the father who did not come

The father attends the second session. Below are the first eight turns, with the reasoning.

> **T1 Therapist:** (standing, offering a hand) Mr Adeyemi. Thank you for coming. I know it was not easy to get away.
> **T2 Father:** The office allowed me. I do not know how much I can help.
> **T3 Therapist:** You can help more than anyone else in this room, and I will tell you why in a moment. First, sit where you like. Would you rather sit next to your wife or over here?
> **T4 Father:** (sits beside his wife, slightly apart) Here is fine.
> **T5 Therapist:** Before I ask you anything about Amara, I want to ask about you. What was your week like?
> **T6 Father:** (pause) Long. The work is not regular. I take what comes.
> **T7 Therapist:** And what does that do to you, by the end of a week?
> **T8 Father:** (longer pause) It makes me tired of being asked for things.

Annotations. T1 does two things at once: it greets him by name, which recognises him as a person rather than a category ("the father"), and it acknowledges the cost of attending — the first currency of joining with a reluctant man. T2 is a test: "I do not know how much

I can help" is an invitation to be told he is unnecessary. Many trainees respond with reassurance ("of course you can help!"), which is heard as hollow.

T3 does the opposite of reassurance: it asserts his necessity and defers the explanation, which creates a reason to stay. The seating question is not small talk; it is a structural intervention offered as a courtesy — he chooses whether to sit with his wife, and whatever he chooses is information, given without coercion.

T5 is the crux. Asking about the child first would have confirmed the referral's frame (he is here about Amara's problem). Asking about his week establishes that he is a subject in the room, not an exhibit. T6 gives the answer that matters clinically: precarious work. T7 follows the emotion, not the fact — and T8 produces the sentence that will organise the treatment: **tired of being asked for things.**

What the therapist does next, and why it matters. She says nothing for a moment, then:

> **T9 Therapist:** So you come home, and the house asks you for something too.
> **T10 Father:** (nods) Everybody wants something from me. I have nothing left to give.
> **T11 Therapist:** And when you have nothing left, what do you do?
> **T12 Father:** I sit in the car for a while before I come in. Sometimes I do not come in.

T9 is a reflection that stays with his metaphor — "asked for things" becomes "the house asks you for something" — which is tracking at the level of language. T11 is an episode question that produces the sequence, and T12 gives the therapist the behaviour that the family experiences as absence. No interpretation has been offered. No one has been blamed. And the father has, in four minutes, described his own side of the loop that the family has been living inside for fourteen months.

How the therapist then uses it. She does not say "so your absence is your way of coping". She turns to the mother and asks a genuine question: "Did you know about the car?" The mother did not. That single exchange — conducted without accusation, in the father's presence — does more for the couple's understanding than an hour of interpretation, and it is the reason the father returned for the remaining sessions.

6.10 Laboratory: the joining gate

This is the assessed skills gate for the course. Twenty minutes per student, in a triad with a trained role-player or a peer playing a scripted family, observed live and scored against the rubric below.

Task. Conduct the first twelve minutes of a first session with a family of three (two parents and a reluctant adolescent), in which the father is hostile, the mother is exhausted, and the adolescent says nothing. You must: state the contract; take an account from each person; join the hostile father; and offer one next step.

What is assessed.

Criterion	Weight	Full marks
Contract and consent	10%	Confidentiality limits and practicalities stated concretely, in plain language, checked for understanding
Agenda-setting	5%	Agenda proposed, not imposed; family's amendment accepted
Balance of speaking time	20%	Each member asked at least four substantive questions; the quietest member is not left until last
Joining the hostile member	25%	Positions him as necessary and expert; no reassurance, no defensiveness, no alliance against him
Episode questions	20%	At least three questions about a specific recent episode rather than opinions or traits
Session management	10%	Time held; interruptions managed; no attempt to cover everything
Closing and next step	10%	Hypothesis offered with an invitation to correct; one concrete next step agreed

What fails the gate, regardless of charm. Talking more than the family; adopting one member's formulation as fact; failing to state the limits of confidentiality; asking no episode questions in twelve minutes; and ignoring the silent member entirely.

Resit arrangements. A resit is available after two weeks of supervised practice, with a written reflection on what went wrong. The gate is not a punishment; it is a check that you can be trusted with a family's first hour, which is the hour that determines whether there is a second.

6.10a The feedback sequence, scripted

The last ten minutes of a first session are the most technically demanding, and the least rehearsed. Here is a script that works, with the reasoning attached.

> **1. PERMISSION.**
> "I would like to tell you what I think I am seeing. Some of it may be wrong. Will you tell me where I am wrong?" Why: it frames the formulation as a hypothesis, and it recruits the family as collaborators rather than recipients.

> **2. WHAT YOU SAW.**
> "I noticed three things. In the first half hour, Ebuka's mother answered two questions that I had asked him. When I asked about the mornings, she described shouting, and then she described him making her tea. And when I asked what happened after the teacher called, she said she felt judged by the school." Why: observations first, in plain language, each of them checkable. No adjectives.

> **3. WHAT YOU MAKE OF IT.**
> "The way I understand it is this: the writing has become the place where several worries meet — Ebuka's, yours, the school's. When the school telephones, you carry the judgement alone, and by the evening the two of you have found a way to repair it. I think the repair is a strength, and I also think it is a lot of work for a nine-year-old." Why: one paragraph, no jargon, one reframe (the repair is a strength), and one gentle observation about cost.

> **4. WHAT YOU ARE UNSURE ABOUT.**
> "What I do not yet know is whether writing itself is hard for him, and whether your husband sees the same things you do. Those are the two things I would want to find out." Why: naming uncertainty is what makes the confident parts credible, and it sets up the next two sessions.

> **5. INVITATION TO CORRECT.**
> "What have I got wrong?" Why: the single most valuable question in the sequence. Families correct the therapist far more often than trainees expect, and the corrections are data.

> **6. THE NEXT STEP, SPECIFIC.**
> "I would like to see the three of you next time, and I would like to ask the school for their written record of what they have observed. Before then, I would like one thing to be different: when the school telephones, you and your husband talk first, before either of you speaks to Ebuka." Why: one task, observable, structural (it creates a parental subsystem), and achievable in a week.

> **7. THE SAFETY NET.**
> "If anything happens before we meet — if anyone is hurt, or you feel you cannot cope — here is how to reach me, and here is what to do if you cannot." Why: families in distress need to know the route out of the room; the therapist who omits it is assuming a stability the family may not have.

What to avoid in the feedback. Four sentences that destroy the sequence: "It is very common" (which erases the family's particularity); "You must not blame yourself" (which introduces blame); "The real problem is your marriage" (which is a verdict, not a hypothesis, and is rarely true in the simple form it is said); and "I think he may be on the spectrum" (a diagnostic opinion offered without assessment, in a first session, with no follow-up plan — an error that can take months to undo).

6.10b Two opening scripts you can memorise

Trainees freeze in the first two minutes, and rehearsed language dissolves the freeze. Two scripts, to be learned and then abandoned once they are no longer needed.

Script A — first session, voluntary family.

> "Thank you for coming — all of you. My name is …, and I am a family therapist. I work with couples and families rather than with one person at a time, because most difficulties that bring people here involve relationships rather than one person's mind. Before I ask you anything, I want to tell you four things. First, what you say here is confidential, with three exceptions: if someone is at risk of serious harm, if a child is being hurt, or if a court orders me to disclose. Second, sessions last fifty minutes. Third, I will not be able to solve this in one hour, and I do not think you expect that. Fourth, if I ask something you do not want to answer, say so and I will move on. Does that make sense? Anything you want to ask me before we start?"

Script B — first session, family referred by an institution and suspicious.

> "I know that you did not choose to be here, and I know that schools — or courts, or doctors — can make a family feel accused. I want to be clear about my position: I am not here to judge whether you are good parents, and I have no authority over what the school decides. What I can do is help this family work out what is happening and what you want to do about it. If at the end of this hour you decide this is not useful, I will tell you honestly whether I think that is a mistake. Shall we start with what you were told, and what you think of it?"

Why these work. Both state the frame before the content; both pre-empt the family's most likely fear; both offer control ("say so and I will move on"; "if you decide this is not useful"); and neither asks a clinical question in the first minute. Script A is 170 words and takes about fifty seconds to say. Script B is 130 words. That brevity matters: a first-session monologue longer than ninety seconds makes a family anxious.

What to do when the script goes wrong. It will. A father will interrupt; a mother will cry before the contract is finished; an adolescent will ask why they are there. The rule is: the contract can be delivered in pieces, but it cannot be skipped. If you are interrupted, return to it before the clinical work: "Before we go on, I need to finish one thing, because it protects all of us."

6.11 Six errors in the first session

1. **The interrogation.** Twenty questions in a row with no reflection, no acknowledgement and no silence. Fix: one question, one answer, one brief acknowledgement; slow to two-thirds of your natural pace.
2. **The premature formulation.** A confident account of the family's dynamics in minute fifteen. Fix: no formulation before you can describe one full sequence in the family's own words.
3. **The missing safety screen.** No questions about harm to self, harm to others, violence or abuse. Fix: build the screen into the first session as a standard section, with a scripted opening that makes disclosure possible.
4. **The informal contract.** Confidentiality mentioned vaguely or not at all. Fix: a concrete script, checked for understanding, in every first session.

5. **The over-identification.** You like one member, and your questions begin building their case. Fix: count your questions by addressee; ask the quietest member first next time.
6. **The unbooked next step.** The session ends warmly and nobody agrees what happens next. Fix: end with a specific appointment, a specific task, and one sentence about what to do if things get worse before then.

6.12 Culture, faith and the first ten minutes

The joining techniques above are culturally saturated, and students should adjust them deliberately rather than applying them uniformly.

Greeting and seniority. In many contexts, the first minutes are governed by protocol: who is greeted first, whether an elder is acknowledged before the presenting problem, whether shoes are removed, whether a visitor is offered water. A therapist who does not observe these courtesies will be experienced as a stranger who does not know how to be in a home, and the clinical work will not recover from it. Ask, or observe, or ask a colleague from the community — and do not treat protocol as an obstacle to the "real" work. The protocol is the work of the first ten minutes.

Language and code-switching. Families often discuss emotion in one language and practicalities in another, or switch when they are uncomfortable. Do not force a language; follow the family. If you do not speak the language they use for the most intimate material, you need an interpreter and a plan for how their words will reach you.

Naming the problem. In some families, naming a difficulty too directly in the first session is experienced as bad manners or as a curse invited. Indirect openings — asking about a typical day, about what has been tried, about what the elders think — will produce the information more reliably than a direct question. The rule is to take the family's route to the problem rather than yours, while making sure you arrive.

Hospitality and fees. Offer of food or drink; who pays for the session; whether a gift is appropriate. These are not trivia: they define the relationship. The professional answers are to accept hospitality graciously and briefly, to be transparent about fees before the work begins, and to have a stated policy about gifts that respects the gesture without creating obligation.

> **CAUTION**
> Do not use cultural respect as a reason to skip the safety screen. A question about violence asked with courtesy, in private, in the family's own idiom, is not a cultural insult; failing to ask it is a professional failure. Where a family's norms make a private conversation difficult, create the privacy structurally: a separate appointment, a home visit, a walk to the car.

6.12a The joined room: what good looks like from outside

Trainees rarely see a good first session from the outside, because they are always inside one. Here is a description of a session that went well, written by an observer, with the specific behaviours that made it work.

The room at minute five. The therapist has moved her chair to the corner of the family's seating rather than opposite them. The father, who arrived last and sat furthest away, has been asked where he would like to sit and has moved closer without being asked twice. The nine-year-old is drawing on paper the therapist provided, and nobody has told him to stop.

The talk. The therapist has spoken for perhaps 40% of the first ten minutes, and about 25% after that. She uses the family's words — "the shouting", "the tea", "the letter" — rather than clinical abstractions. Twice she has said "tell me more about that", which produced the two most informative passages of the session. In the first twenty minutes she has asked the mother six questions, the father five and the child four, each of them at least twice, and the father was asked about his work before he was asked about his son.

The moment of difficulty. At minute twenty-two the mother says, sharply, that the father does not care. The observer expects the therapist to smooth it over. Instead she slows it: "Say that to him rather than to me. Then I would like to hear what happens in you when she says it." The mother repeats it; the father is silent; the therapist waits eleven seconds, visibly, without filling the silence, and the father then says something he had not planned to say. That silence is the most skilled thing in the session.

The correction. At minute forty the therapist offers a hypothesis. The father disagrees with part of it. She writes his correction down, reads it back to him and says "that changes what I think". The observer notes the family's posture change at this point: they are working with the therapist rather than being assessed by her.

The ending. The therapist summarises in five sentences, names one task, books the next appointment while everyone is still in the room, and asks the closing question. The child's drawing is left on the table between them and the therapist asks whether she may keep it; he says yes. Nobody leaves in silence.

What the observer would teach from this session. Three things. The seating and the drawing were not decoration; they were the physical form of the message "you are welcome here and you are not on trial". The eleven-second silence is a technique of a high order, teachable only by practice. And the visible acceptance of correction turned an assessment into a collaboration, which is what the family will remember when deciding whether to come back.

> **DRILL**
> Watch any recorded first session — a training video, a demonstration, or your own role-play — with a stopwatch. Time the therapist's longest silence and count the times they said "tell me more". Then do the same with your own recording. The comparison is humbling and always useful.

> **TASK**
> Write the observer's account of your own last role-play in the same form: room at minute five, talk, moment of difficulty, correction, ending. Write it in the third person, as though about a stranger. Then mark the three things an observer would teach from it, and the one thing they would want changed.

6.13 Evidence note: what predicts a second session

The literature on engagement and dropout in family therapy is unusually practical, and four findings are worth knowing precisely.

The first session is the strongest single predictor. Attendance at a second session is predicted by what happened in the first — alliance, clarity of the plan, and the family's sense that the therapist understood the problem. Families who leave the first session without a clear next step are much less likely to return.

Practical barriers are underestimated. Distance, transport cost, work schedules, childcare, and the timing of appointments predict attendance at least as strongly as motivation. Interventions that address logistics — evening clinics, reminder calls, transport support, co-locating with schools — have larger effects on attendance than most clinical techniques, and they are the least fashionable thing a service can do.

Alliance matters, and it is measurable. Session-rating measures completed by each family member after each session identify alliance ruptures early enough to repair them. The evidence supports routine use, and the practical obstacle is not the measure but the habit.

Motivation is not a fixed trait. Families described as "unmotivated" in referral letters frequently engage well when the therapist's explanation fits their own understanding of the problem. The construct that predicts engagement is not motivation in general but fit: does this explanation, in this room, make sense of what this family is living with?

The implication for the gate. The skills assessed this week are not the most sophisticated in the course; they are the ones with the best evidence for keeping families in treatment long enough for anything else to matter.

6.14 Key terms

Term	Definition
Joining	The therapist's deliberate accommodation to the family in order to work from inside the system
Maintenance	Supporting the family's existing structure and rules while working
Tracking	Following the family's content, language and imagery rather than steering to your agenda
Mimesis	Adopting the family's style, pace and vocabulary enough to be experienced as familiar
Accommodation	The therapist's adjustment to the family's system (Minuchin)

Term	Definition
Alliance (individual)	Each member's relationship with the therapist
Alliance (subsystem)	The alliance with a dyad or subsystem as a unit
Alliance (whole system)	The family's shared alliance with the therapy
Episode question	A question about a specific, recent instance of the problem
Opinion question	A question about general patterns, traits or feelings — less reliable
Rupture and repair	A break in the alliance and the process of mending it
Dropout	Premature termination without agreement

6.15 Self-test

1. Name Minuchin's three joining operations and give one example of each.
2. What are the only two reliable signs that joining succeeded?
3. What are the three alliance dimensions in Pinsof's model?
4. A father says "I don't know why I'm here — she's the one with the problem." What are your first two moves?
5. Why is a polite family often harder than a furious one?
6. Give the six steps of the opening architecture in order.
7. What is an episode question, and why does it outperform an opinion question?
8. What should you record after a first session that students usually omit?

Answers. 1. Maintenance (affirming the family's structure), tracking (following their content), mimesis (matching their style). 2. The family discloses something unplanned, and they book again. 3. Individual, subsystem and whole-system alliances, across content, goals and tasks. 4. Acknowledge the cost of attending and assert his necessity, then ask about his own week before asking about the child. 5. Because politeness is often a rule that forbids disagreement, so the conflict the family needs to address never appears. 6. Orient; contract; agenda; account from each person; slow the sequence; feedback and next step; closing question. 7. A question about a specific recent instance; it produces behaviour rather than characterisations and is harder to answer with a defence. 8. Risk screening, who answered for whom, and whose formulation you adopted — plus what you did not ask and need to ask next time.

6.16 Reflective task

Record a twelve-minute role-play of a first session. Watch it twice: once with the sound off, noting seating, gesture, who leans forward and who looks at whom; once with sound, counting questions by addressee. Then write 300 words answering three questions. Which member did you join most easily, and why? Which member did you avoid, and what did you avoid? And what does the answer to the second question remind you of in your own family?

This is the exercise students most often skip and most need. The pattern of avoidance you find here will follow you into practice, and the only reliable remedy is to have named it early.

6.17 Readings for this week

- **Minuchin (1974)**, Families and Family Therapy, chapter 6 on joining and accommodation. Read it twice: once for the technique, once for the tone.
- **Pinsof (1995)**, Integrative Problem-Centered Therapy, on the alliance in couple and family work. The measurement framework.
- **Friedlander, Escudero & Heatherington (2008)**, Therapeutic Alliances in Couple and Family Therapy. The best single treatment of ruptures and repairs.
- **Alexander, Holtzworth-Munroe & Jameson (1994)**, on engaging resistant families. Practical and evidence-based.
- **Boyd-Franklin (2003)**, Black Families in Therapy, chapters on engagement. On the first session in families whose previous experience of services has been hostile.
- **A recorded first session with a transcript.** Any good training archive will do; read the transcript rather than watching, because reading reveals exactly what the eye edits out: the question you asked twice, the member you never addressed, the moment you filled a silence that was doing useful work.

Next: Week 7 — The first session II: defining the problem

Week 7 — The first session II: defining the problem

7.1 The lecture in ninety minutes

Week 6 was about joining: getting into the system in a position from which change is possible. This week is about the second half of the same hour — turning a complaint into a definition of the problem that the whole family can work with. It is the most cognitively demanding skill in the first month, because it requires you to hold three things at once: the family's language, your clinical hypothesis, and the criteria by which the definition will be judged usable.

The week's central claim is simple and contested: **the way a problem is defined determines what counts as a solution.** A family that defines the problem as "he is stubborn" will accept only that he becomes compliant. A family that defines it as "the evenings have become a battle none of us knows how to end" can work on something. The therapist's job in the first session is not to impose a definition but to negotiate one that is specific, interactional, owned by the family, and changeable.

Minutes	Segment	What the lecturer does	What students do
0–10	Why definitions matter	Two definitions of one problem, and the plans that follow	Predict the plan from the definition

Minutes	Segment	What the lecturer does	What students do
10–25	The four criteria	Specific, interactional, owned, changeable — tested on live examples	Score six definitions
25–45	From complaint to request	The negotiation, demonstrated twice	Negotiate with a scripted parent
45–60	Reframing the definition	From trait to sequence to function	Rewrite five complaints
60–75	Presenting problem versus underlying problem	What to leave alone in session one	Diagnose a formulation for over-reach
75–90	The agreed statement	Write it, read it back, get it agreed	Draft and test in pairs

7.2 Why the definition is the intervention

A definition of the problem is not a description; it is a set of instructions. It tells the family what to notice, who is responsible, what would count as improvement, and who has to change. Four consequences follow, and students should be able to state them.

First, a definition selects the data. A family that defines the problem as "Amara refuses school" will report on school attendance and will not report the mornings, the mother's mood or the father's drinking. The definition is a filter, and the therapist who accepts it unexamined will work with a filtered family.

Second, a definition distributes responsibility. "He is stubborn" places the problem inside a nine-year-old. "The mornings have become a battle" places it between people. The second is not kinder by accident; it is kinder because it is more accurate, and it is more useful because more people can act on it.

Third, a definition sets the criterion of success. If the problem is "he does not obey", success is obedience, and the family will be dissatisfied with anything less. If the problem is "we cannot get through a morning without shouting", success is a calm morning, which is achievable within a fortnight and which changes the family's sense of itself.

Fourth, a definition is a hypothesis about cause smuggled into a description. "She is manipulative" contains an account of intention; "she asks for her mother whenever her father enters the room" contains a sequence. The second can be tested; the first cannot. The therapist's discipline is to convert the first into the second, without correcting the family and without agreeing with them.

> **CAUTION**
> There is a difference between negotiating a definition and rewriting the family's experience. The family's account is data, and the version they bring is the version they can bear to bring. If you replace it too quickly with your own, you have not defined the problem; you have taken it.

7.3 The four criteria, tested

A usable definition of the problem meets four criteria. Teach them as a checklist and use them as a test in the room.

1. Specific. It names an occasion, a behaviour and a time frame. Not: "we have no communication." But: "on weekday evenings, after the children are in bed, we sit in different rooms and neither of us starts a conversation."

2. Interactional. It involves at least two people and at least one sequence. Not: "she is cold." But: "when I ask about her day she answers in one word, and then I stop asking, and then she says I never ask."

3. Owned. The family recognises it as theirs, in their language, without having to be persuaded. The test is simple: read it back and ask, "is that right?" If they correct you twice, the definition is still yours.

4. Changeable. It is the kind of thing that could plausibly be different within the timeframe of the work. Not: "he has always been like this." But: "the last three months, since the new job, the arguments have moved from the weekend to every night."

Scoring exercise. Six definitions, from real referral letters. Score each 0–2 on the four criteria, then rewrite the weakest.

Definition (as presented)	Specific	Interactional	Owned	Changeable	Total
"Family communication problems"	0	1	1	1	3
"Mother is over-involved with daughter"	1	0	0	1	2
"Since the father's redundancy, the parents argue about money every weekend and the daughter refuses school on Mondays"	2	2	2	2	8
"Adolescent behavioural difficulties"	0	0	1	1	2
"He will not talk to me since his brother was born"	2	2	2	2	8
"The family is enmeshed"	0	0	0	0	0

The exercise produces two lessons. Definitions from institutions score badly because they are written for categories, not families; and the best definitions sound like ordinary speech, because they are the family's own words arranged more carefully.

7.4 From complaint to request: the negotiation

Families arrive with a **complaint** (what is wrong) and often without a **request** (what they want). The therapist's task is to convert one into the other without dismissing either. Four moves, in sequence.

Move 1 — get the complaint in full, in the complainant's language. Do not translate yet. "Tell me what it is like. Not the summary — the last time it happened."

Move 2 — find the exception or the contrast. "When was the last time it did not happen?" or "What is different on the days it is better?" This does two things: it

establishes that the pattern is not total, and it gives the family a piece of evidence about their own competence.

Move 3 — ask for the wished-for change, in behaviour. "If this were better, what would I see you doing differently?" Not "how would you feel" but "what would you see". Vague answers are normal; the therapist's job is to keep asking until something observable appears.

Move 4 — test the request for feasibility and ownership. "Is that something you could do this week, or is it too big?" A request that fails this test is not a request; it is a wish, and the therapist should say so gently and shrink it.

The negotiation, worked.

> **MOTHER:**
> He will not listen to me. I have to say everything five times. **Therapist:** Give me the last time. Yesterday morning? **Mother:** Yesterday. I asked him to get ready. Five times. Then I shouted. **Therapist:** And the fifth time — did he get ready? **Mother:** Yes. After I shouted. **Therapist:** So shouting works. It gets him ready and it costs you the morning, and by the evening you are both tired. What would you want instead — what would I see happening in that kitchen if it were better? **Mother:** I would ask once and he would move. **Therapist:** Once. And if it took twice, would that be acceptable? **Mother:** (pause) Twice would be fine.

The negotiation has produced a definition ("in this kitchen, getting ready takes five requests and a shout") and a request ("asking once or twice and he moves") that is specific, interactional, owned and changeable. Note what the therapist did not do: she did not tell the mother that shouting is bad, she did not diagnose the child, and she did not propose a technique. She found the sequence, found its cost, and let the mother name the alternative.

7.5 Reframing the definition

Reframing was introduced in Week 2 as an intervention for meaning. Here it is a tool for the definition itself. Three routes, each converting a trait into something workable.

Route 1 — from trait to sequence. "He is lazy" becomes "when you ask him to start his homework, he says 'nothing', and you then do it yourself". The rewrite is not a correction; it is a request for the episode that produced the trait. Ask: "What does he do, exactly, that you would call lazy?" The answer is always a sequence.

Route 2 — from sequence to function. "When you take over his homework, what happens?" — "It gets done, and we stop arguing." The function is now visible: taking over ends the argument and costs the child the experience of finishing something. Naming the function must be done without blame: "the takeover is doing a job for the household."

Route 3 — from function to a both-and. The mature definition holds the cost and the function: "the two of you have found a way to stop the evening becoming a fight, and it means he never has to finish anything on his own. What would let you keep the peace and let him finish?"

Five complaints, rewritten. From the case bank, with the rewrite a competent therapist would negotiate rather than impose.

Complaint	Usable definition
"My husband is controlling"	"He asks where I am three or four times an evening, and I have stopped telling him things in advance"
"My daughter is rude"	"She answers in one word and leaves the room when I ask about school, which she did not do before the exam period"
"My mother interferes"	"She telephones every morning with advice about the children, and I have not told her that it makes me feel judged"
"He drinks too much"	"He drinks four to six bottles on five nights, and on those nights he does not speak to me or the children"
"We have grown apart"	"We have not had a conversation longer than five minutes, without the children, in about a year"

The rule behind the table. Every usable definition contains a behaviour, a time frame, and at least two people. Every unusable one contains an adjective, a diagnosis or a verdict.

7.6 Presenting problem, underlying problem, and what to leave alone

The commonest error of enthusiastic trainees is to address the underlying problem in the first session. Three categories, and the discipline each requires.

The presenting problem is what the family brings. It must be taken seriously, addressed concretely, and, if possible, improved. A therapist who ignores it in favour of a richer formulation teaches the family that therapy is a place where their actual trouble is not discussed.

The underlying problem is what the therapist believes maintains the presenting one — a disengaged father, an unmourned loss, a marital conflict, a family myth. It is a hypothesis, and it should be held, tested and, in most cases, not named in the first session.

The unnamed problem is the material that is not available yet: a secret, a trauma, a violence that has not been disclosed. It must be suspected carefully and not pursued beyond the safety screen. The therapist who pushes for a disclosure in the first hour often gets one that the family is not ready to have made.

A sequencing rule that serves students well. In the first session: address the presenting problem concretely, test the underlying hypothesis with two or three questions, and make one small move. Name the underlying problem only when (a) the family raises it, (b) the alliance is sufficient that a name will not feel like an accusation, and (c) you have something to offer once it is named.

CASE

A couple presents with "our son will not sleep in his own bed" (presenting). The underlying hypothesis is that the mother has not recovered from a miscarriage two years ago and the child's presence in the bed is the couple's way of not being alone together. The therapist says nothing about the miscarriage in session one. She takes a sleep history, identifies the sequence (child wakes, mother goes to him, father sleeps in the spare room), and offers a graded plan. In session four, the mother mentions the miscarriage; by then the couple have succeeded at something together and can bear the conversation. That sequencing is not avoidance; it is clinical judgement about dose.

7.7 The agreed statement

The first session ends with a negotiated statement of the problem and a first step. It should be short enough to be remembered, and it should be written down — on paper, in front of the family, and given to them.

The template, in five parts.

> What happens — the sequence, in the family's words, with a time frame. What it costs — the consequence, stated by the family. What has been tried — evidence of effort, named so it is not repeated or dismissed. What would be different — the request, behavioural and small. What happens next — who does what, by when.

Worked example, for the Okonkwos.

> "Over the last two months, getting ready for school has taken five requests and usually a shout, and it leaves both of you tired and upset for the day (what happens, what it costs). You have tried rewards, taking things away and talking to the school (what has been tried). What you want is to ask once, or twice, and for him to move (what would be different). Next week I would like to see all three of you, and before then, when the school telephones, I would like the two of you to talk to each other before either of you speaks to Ebuka (what happens next)."

Reading it back. The statement is read aloud and offered for correction. Two corrections are typical: the family will want to add something they consider important, and they will want to soften a word that sounds like an accusation. Accept both; the statement is better with them.

Giving it to them. A written copy on a single sheet — the statement, the task, the next appointment — is one of the cheapest and most effective interventions available. Families keep these; they show them to relatives; they remind themselves of what they agreed when the week gets difficult.

7.7a The Adeyemi definition, negotiated in real time

The general method becomes concrete in a single case. Here is the negotiation, as it happened, with the therapist's reasoning.

The complaint as offered. "Amara refuses to go to school. She has missed six weeks. The school is threatening to report us." Three complaints in two sentences: a behaviour, a duration and an institutional threat. The therapist records all three and returns to the first.

The first attempt at specificity. Therapist: "Tell me about this morning. Not the whole six weeks — this morning, from waking up." Mother: "I woke her at six. She said she was not going. I said she had to. She pulled the cover over her head. I shouted. She cried. I left her." Therapist: "And then what did you do?" Mother: "I telephoned the school and said she was sick."

What the therapist now has. A sequence with five links, a lie told to an institution, and a mother who is managing alone. Note that the sequence is not "Amara refuses school"; it is "waking, refusal, escalation, tears, and a telephone call that converts a family difficulty into an absence". The definition is already more useful than the complaint.

The second attempt — the cost. Therapist: "What does that morning do to the rest of your day?" Mother: "I am angry all day. And then I feel guilty. And then I wait for her to come out of her room." The cost is now specific and is the mother's own, not the institution's.

The third attempt — the request. Therapist: "If I could change one thing about that morning, what would you want it to be?" Mother: "That I did not have to lie." This is the pivot of the session, and it is not what the therapist expected. The mother does not ask for attendance; she asks to stop deceiving the school. The therapist follows it: "So the lie is the worst part." Mother: "Because they will find out and then they will think I am a bad mother."

The reframe. Therapist: "So you have been protecting your daughter from the school's judgement, and yourself from being seen as a bad mother, at the cost of being alone with something you cannot manage." Mother: (cries) "Yes."

The negotiated statement, written on paper and read back.

> "For six weeks, most mornings have gone: you wake Amara, she refuses, you insist, she cries, you telephone the school and say she is ill. The cost is that you spend the day angry and guilty, and you are carrying something alone that you believe will make people think you are a bad mother. You have tried insisting, talking, and threatening. What you want first is to stop having to lie. What I suggest is that we tell the school the truth together — a letter from me, and a conversation with you present — and that we do not make attendance the first goal. The first goal is that you are not alone with this."

What makes this statement good. It is specific (six weeks, most mornings, five links); interactional (mother and daughter, then mother and school); owned (the mother's words: "I did not have to lie", "a bad mother"); and changeable (a letter and a conversation can happen this week). It also does something that a formulation based on the referral could never have done: it identifies the institutional loop as part of the problem, and it makes

the therapist an ally in the mother's relationship with the school rather than an agent of the school's demand.

The lesson for trainees. The definition you would have written from the referral letter ("school refusal in a 16-year-old, six weeks") is accurate and unusable. The definition the family negotiated is accurate and actionable, and it took fourteen minutes of questions and one reframe.

7.8 Annotated transcript: negotiating with a family that does not agree

In many first sessions the family does not have one problem; it has three. The negotiation of a shared definition is the clinical work. Below is a twelve-turn excerpt from a family in which the parents' definitions are incompatible.

> **T1 Mother:** The problem is his temper. He shouts at the children and then they come to me.
> **T2 Father:** The problem is that she undermines me. I say no, she says yes, they learn that I mean nothing.
> **T3 Therapist:** So there are two accounts. Let me hold both for a moment. Mother, when he shouts, what do the children do?
> **T4 Mother:** They cry and come to me. Then I tell him he is frightening them.
> **T5 Therapist:** And then, Father?
> **T6 Father:** Then I leave the room. There is no point.
> **T7 Therapist:** So: he shouts, they go to her, she tells him he is frightening them, he leaves. Is that what happens most weekend evenings?
> **T8 Both:** (nods) Yes.
> **T9 Therapist:** Which part of that would each of you most want to be different? Not the whole thing — one part.
> **T10 Mother:** I would want him not to shout.
> **T11 Father:** I would want her to back me up in front of them, even if she disagrees with me.
> **T12 Therapist:** Those are different requests, and they might be connected. Let me see whether I can put them together in a way that is honest for both of you.

Annotations. T3 is the pivot, and it is a refusal to arbitrate. The therapist does not decide whose definition is correct; she says she is holding both, which is both truthful and the only position from which the loop can be described. Students who arbitrate — even silently, by asking more questions of one partner — lose the other.

T3–T8 build a sequence that both partners confirm. Note the technique: the therapist asks each partner about the other's behaviour, not their own, which is easier to answer honestly and produces the loop rather than two monologues. The confirmation at T8 ("is that what happens most weekend evenings?") matters: a definition the family has confirmed is a definition they cannot easily disown later.

T9 is the request-elicitation move, and the constraint ("not the whole thing — one part") is doing important work. It prevents the session from becoming a negotiation of everything, and it produces two requests that can be compared. T10 and T11 are incompatible as

stated — he must not shout; she must back him up when he shouts — which is precisely the material for the next several sessions.

T12 names the incompatibility honestly and promises an integrative move without pretending there is none. The therapist's next sentences, not shown, construct a both-and: "What would let him speak firmly without shouting, and let her support the decision without supporting the shouting?" That question converts two competing requests into one joint task.

The most common trainee error in this transcript. Asking "how do you feel when he shouts?" That question is sympathetic, and it makes the loop harder to see, because it invites each partner to describe their own inner state rather than their joint behaviour. Feelings become clinically useful after the sequence is visible, not before.

7.9 Laboratory: the definition negotiation

Ninety minutes. Triads, with a scripted family played by two peers from a case card, and an observer scoring against the rubric. Each student takes the therapist's chair once, for twenty minutes.

The card. Both parents present, one adolescent present and silent, and a referral letter containing a diagnosis. The parents' definitions conflict. The adolescent has not been asked.

The task. Produce, in twenty minutes: a sequence the family confirms; one behavioural request from each parent; a both-and formulation that does not require either parent to concede; and a written statement of five lines, read back and agreed.

Marking rubric.

Criterion	Weight	Full marks
Sequence elicited and confirmed	25%	A behavioural loop, stated in the family's words, confirmed by both partners
Requests elicited	20%	One behavioural request per parent; vague requests converted into observable ones
Non-arbitration	20%	Both definitions held; no silent siding; questions balanced
Integrative formulation	20%	A both-and that addresses both requests without collapsing them into one
Written statement	15%	Five lines; specific, interactional, owned, changeable; read back and corrected

The pass standard. Four of five criteria at 3/5 or better, with sequence and non-arbitration at 4/5. Students who fail most often fail on non-arbitration, and the usual cause is a sympathetic identification with the parent who arrived first or who is more articulate.

7.9a The goals grid: four columns, one page

Families rarely have one goal; they have several, held by different members, of different sizes, with different time horizons. A single page, completed in the first or second session, prevents most of the confusion that follows.

Person	What they want (behavioural)	By when	What they would accept as a first step
Mother	To stop lying to the school; to have one calm morning a week	Six weeks	A letter to the school this week
Father (when present)	To know what is happening in the house; to stop being the last to hear	Six weeks	A fortnightly conversation with his wife before the session
Amara	To be left alone in the mornings; not to be asked about school every day	Two weeks	Nobody mentions school before 9 a.m.
Therapist	A shared definition the family owns; one structural change in the parental subsystem	Four sessions	Both parents in the room next week

How to use the grid. Complete it in front of the family, reading each entry back. Where a row is blank, say so: "Amara, I do not have anything from you yet." Where two entries conflict, name the conflict as the work: "Your mother wants one calm morning and you want no conversation before nine. Those might be the same thing."

Why a grid rather than a conversation. Because the conversation produces the illusion of agreement, and the grid produces evidence of disagreement. The families who benefit most from therapy are often those whose members want incompatible things, and the grid makes that incompatibility discussable without making anybody the villain.

> **TASK**
> Complete a goals grid for a family you know from placement, role-play or the case bank. Bring it to the seminar. Then answer: whose goal did you find hardest to write in behavioural terms, and why? The answer is usually the goal you understand least, which is the goal you will accidentally neglect in the work.

7.10 Six errors in defining the problem

1. **Accepting the referral's definition.** The letter says "oppositional behaviour" and the session proceeds as though that were established. Fix: ask the family what they would call it, and note the difference.
2. **The premature insight.** Naming the underlying problem in the first session, before the family has the resources to hear it. Fix: test the hypothesis by questions, name it only when the family raises it.
3. **The vague request.** Accepting "I want us to communicate better" as a goal. Fix: ask what you would see, on a Tuesday, that would tell you it had improved.
4. **The single request.** Agreeing only the more articulate partner's goal. Fix: ask each person separately and write both down, even if they conflict — especially if they conflict.

5. **The definition that blames.** A statement the family experiences as an accusation, however carefully worded. Fix: read it back and ask for corrections; if a member goes quiet, the definition is not agreed.
6. **The missing safety clause.** A definition that omits risk — violence, self-harm, safeguarding — because the family did not raise it. Fix: the screen comes before the definition, always.

7.10a Rewriting six institutional definitions

Referral letters are written for institutions. This week's most practical skill is translating them without losing the information they contain.

1. "Poor engagement; family disengaged from services." Translation: the family has attended four services, each of which asked them to do something they could not do, and the fourth asked them to come to a clinic two buses away on a weekday morning. What to do: ask what each previous service asked of them, and change the practical conditions before interpreting the motivation.

2. "Mother is over-involved; enmeshed relationship with daughter." Translation: the mother is the only adult who has been managing a chronic difficulty, and the daughter has become her confidante because the marriage is disengaged. What to do: widen the support, not detach the daughter; interview the father.

3. "Non-compliant with treatment plan." Translation: the plan required something the family could not sustain — daily homework, weekly attendance, a medication schedule that conflicts with shift work. What to do: ask what happened at home when they tried; redesign the plan around the family's actual week.

4. "Adolescent is manipulative; splits parents." Translation: the adolescent has learned that approaching one parent produces a different answer from the other, and uses the difference to get what most adolescents want, which is to be allowed out. What to do: work with the parents on a consistent position; do not diagnose the adolescent.

5. "Child has behavioural difficulties at school only." Translation: the difficulty is a property of the school–child fit, and possibly of the classroom, not of the family. What to do: obtain the school's observations before assuming a family pattern; involve the school in the plan.

6. "Family in denial." Translation: the family does not accept the institution's definition of the problem, and has reasons. What to do: ask what they think the problem is, and what they make of the institution's account. "Denial" is usually a description of a disagreement, written from the side that has more power.

The general rule. Every institutional definition contains information about the family and information about the institution. Your job is to separate the two, and to notice that the second is often more revealing than the first.

7.10b The definition audit: five questions after every first session

A checklist to complete in the ten minutes after a first session, before the notes. It takes five minutes and it improves the second session more than any technique.

1. Whose words are in my definition? Count the phrases in your agreed statement that came from the family, and the phrases that came from you or from the referral. A statement with more of your language than theirs is a formulation, not an agreement, and it will not survive a difficult week.

2. Who did not contribute a goal? Every family has a member who says nothing when goals are requested. Name them in your notes, and plan how to hear from them next time — usually alone, usually first, and usually with a question about what they would change if they had a magic wand.

3. Did I define the problem in a way that requires someone to become a different person? "He needs to be less aggressive" is a definition that requires a personality change. "When he raises his voice, his wife leaves the room and takes an hour to return" is a definition that describes a pattern two people can alter.

4. What would count as evidence, next week, that the definition was wrong? If nothing could count, you have a belief rather than a hypothesis. Write the evidence condition in your note.

5. Did I leave the session with a statement I could read aloud to the family without flinching? If not, the statement contains something you would not defend in their presence, which means it contains something you have not yet said to them. That is the material for session two.

7.11 Culture, faith and the definition of a problem

The definition of the problem is one of the most culturally loaded moments in the session, because it determines what kind of difficulty this is: medical, moral, spiritual, relational or social. Four adjustments.

Whose problem is it? In many families, the person who telephones is not the person with authority to define the problem. A mother may describe the difficulty, but the father, the grandmother or an uncle may hold the right to name it. A definition agreed with the wrong person will not hold. Ask, early: who else would need to agree that this is the problem?

The spiritual definition. "This is a spiritual attack" is a complete definition, with its own aetiology, treatment and criterion of success. It competes with yours and should not be dismissed. The usable move is to establish what the family would count as evidence of improvement, and to negotiate a definition that allows both accounts: "The family believes this is spiritual, and we are also going to look at what happens in the household, because both matter for a child." Note that where a spiritual definition leads to a child being denied treatment, the professional obligation overrides the negotiation.

The moral definition. "He is disobedient" is a moral claim, and in many families it is a claim about the parents' standing as much as about the child. Challenge it directly and you challenge the parents. Instead, keep the moral frame and widen it: "Disobedience is

serious in this family. I want to understand what makes it possible in this house, because that is what we can change." The parents' values are preserved; the target moves.

The social definition. Poverty, migration status, housing and unemployment are definitions of the problem that therapists often translate into psychological language to their cost. A family whose problem is that the father has no work does not need a formulation about boundaries. Ask the practical question first, and let the family tell you whether the difficulty is material, relational or both.

> **CAUTION**
> Do not use "we need to understand the family's meaning system" as an excuse for not stating the obvious. If a child is not attending school, or a woman is being hit, the professional's definition of the problem has to include it, in plain language, whatever the family's preferred account.

7.11a Working with a family whose definition is a diagnosis

Increasingly, families arrive with a diagnosis already in hand — from a school, a paediatrician, a previous clinician, or increasingly from the internet. The diagnosis is both a definition and a resource, and it must be handled with respect rather than either deference or dismissal.

What a diagnosis gives a family. A name, which reduces chaos; an explanation, which reduces self-blame; an entitlement to services; a community of other families; and, often, a moral status ("this is an illness, not bad behaviour"). These are substantial benefits, and a therapist who attacks the diagnosis attacks the family's coping.

What a diagnosis costs. It can locate the problem entirely inside one person, which removes the family's leverage; it can freeze a developing picture into a label; it can become the family's only way of talking about the child; and it can license a fatalism ("this is how he is") that forecloses change.

The systemic move. Do not dispute the diagnosis; widen the definition around it. Four sentences that work:

> "The diagnosis explains a lot, and it does not explain everything. In this house, the difficult hours are between six and eight in the evening — is that right? Let us look at what happens in those hours, because those are the hours we can change, whatever the diagnosis says about the rest."

The family keeps its explanation and acquires a target. The therapist does not have to win an argument about aetiology in order to do useful work.

A caution about diagnostic enthusiasm. The opposite error is equally common: a family arrives with a relational story, and the clinician supplies a diagnosis that reorganises the family's understanding of their child. The responsibility that comes with naming a condition is significant, and it belongs to an assessment process, not to a first session (see MFT 707).

7.12 Evidence note: goal consensus and outcome

Three findings from the literature on goals and definition of the problem, all practically useful.

Goal consensus predicts outcome. Across psychotherapy research, agreement on goals and tasks between client and therapist is one of the more robust predictors of outcome, and disagreement about goals is a common precursor to dropout. In family work the finding is stronger and more complicated, because there are several clients: consensus must be built with each member, and where members' goals conflict, the therapy must accommodate the conflict explicitly rather than resolve it by fiat.

Behavioural specificity improves engagement. Families given vague goals report less progress and discontinue more often than families given specific, observable goals. This is not merely a measurement convenience; a specific goal allows a family to notice change, and noticing change is what sustains hope in the early weeks.

The first-session formulation predicts attendance at session two. Families who leave the first session with a formulation they recognise — in their own words, offered as a hypothesis — are significantly more likely to return. The mechanism is probably the experience of being understood, which is the same construct the alliance literature measures.

The implication. The negotiating skills in this week are not a preliminary to treatment; they are among its active ingredients. A therapist who spends the first session chasing information and leaves without an agreed statement has spent the hour on the least evidence-based part of the work.

A fourth finding, on measurement. When families and therapists independently rate the goals of therapy, their agreement is moderate at best, and the discrepancy is not random: therapists tend to rate goals as more shared than families do. The practical lesson is to ask rather than to assume, and to ask in a form that can be answered concretely — "which of these three would you put first?" rather than "do we agree on the goals?" The question that invites a yes tells you almost nothing, and the question that offers a choice tells you which of the family's three priorities they will actually act on in the following week.

7.12a Two definitions, two plans: the same family, a year apart

A comparison that makes the week's argument vivid. The same household, defined two ways, six months apart, by two different clinicians.

Definition A (a community clinic, first appointment after the referral). "Amara, 16, school refusal for six weeks, with anxiety features; mother reports exhaustion; father works away." The plan that follows: an anxiety assessment for Amara, a graded return-to-school plan negotiated with the school, individual CBT, and a review in six weeks. What happened: Amara attended two sessions, the graded plan collapsed in the second week, and the family did not return.

Definition B (a family therapist, six months later). "The mornings have become a battle in which the mother asks five times, shouts, and then telephones the school with an

explanation she knows is not true; the cost is a day of anger and guilt for her, and a daughter who is at home and cannot leave; the father is outside this sequence and has not been asked what he thinks." The plan that follows: a letter to the school, written with the mother; both parents in the room; a small morning task; a conversation about the marriage when the parents are ready; review in two weeks. What happened: the family attended eleven sessions; the school return was staged over two months; the parents' arguments became discussable; the father did not stop drinking, and the household still improved.

What the comparison is not saying. It is not saying that CBT is wrong, that anxiety was not present, or that the first clinician was incompetent. It is saying that the first definition located the problem where nothing in the household could be changed, and the second located it where several things could. Two accurate descriptions, one usable.

The question for the examination. "What is the relationship between the accuracy of a formulation and its usefulness?" A strong answer says that they are different virtues: a formulation can be accurate and inert, or actionable and incomplete, and the professional's task is to be accurate and to choose, deliberately, the level of description at which change is possible.

7.13 Key terms

Term	Definition
Presenting problem	The difficulty the family brings and wants addressed
Underlying problem	The therapist's hypothesis about what maintains the presenting problem
Complaint	What is wrong, as described by the complainant
Request	The behavioural change the family wants
Goal consensus	Agreement between therapist and clients on goals (and on tasks)
Reframing	Offering a new frame for the same facts that supports a different response
Both-and formulation	A statement holding two apparently incompatible truths without collapsing them
Agreed statement	A short written summary of the problem, its cost, the request and the next step
Exception	An occasion when the problem did not occur, or occurred less
Scaling question	A question asking the family to place the problem or their coping on a numbered scale

7.14 Self-test

1. What are the four criteria of a usable definition of the problem?
2. Why is "he is stubborn" a poor definition? Give two reasons.
3. What is the difference between a complaint and a request?
4. Give the four moves of the negotiation from complaint to request.
5. What are the three routes of reframing a definition?
6. Why should the underlying problem usually not be named in the first session?

7. What are the five parts of the agreed statement?
8. What does the research say about goal consensus?

Answers. 1. Specific, interactional, owned, changeable. 2. It names a trait rather than a behaviour, so it cannot be observed or tested; and it places the problem inside one person, so only that person can change. 3. A complaint describes what is wrong; a request describes the behaviour the family wants instead. 4. Take the complaint in full; find the exception or contrast; ask for the wished-for change in behaviour; test for feasibility and ownership. 5. Trait to sequence; sequence to function; function to a both-and. 6. Because it is a hypothesis, and naming it before the family has the resources to hear it produces defence or compliance rather than collaboration. 7. What happens; what it costs; what has been tried; what would be different; what happens next. 8. Goal consensus predicts outcome, and disagreement about goals precedes dropout.

7.15 Reflective task

Take a difficulty you have described about your own family, in the words you actually use — "my mother is controlling", "my brother is irresponsible", "we are not a demonstrative family". Now run it through the three reframing routes: the sequence it hides, the function it performs, and the both-and that holds both truths. Write 300 words on what you had to give up to write the both-and. Most students find they had to give up a satisfying certainty.

7.16 Readings for this week

- **Minuchin (1974)**, Families and Family Therapy, chapter 7, on problem definition and restructuring.
- **de Shazer (1985)**, Keys to Solution in Brief Therapy, on well-formed goals. Read it alongside the family therapy literature, because it is the clearest writing on specificity.
- **Friedlander, Escudero & Heatherington (2008)**, chapters on goals and on ruptures — the empirical case for this week's skills.
- **Weakland, Fisch, Watzlawick & Bodin (1974)**, "Brief therapy: focused problem resolution", Family Process. The classic statement on negotiating a workable definition.
- **Anderson & Goolishian (1988)**, "Human systems as linguistic systems", Family Process. The opposite view — problem-dissolving rather than problem-solving — worth arguing with.
- **Any current clinical guideline** for the presenting problem in your placement, to see how institutions define problems and what that definition does to the family's own account. Read one guideline as if you were the family receiving it: what does it say the problem is, who does it say is responsible, and what would it count as failure? That exercise takes twenty minutes and changes how you write a formulation for the rest of your career.

Next: Week 8 — The genogram as a hypothesis machine

7.10b The definition audit: five questions after every first session

A checklist to complete in the ten minutes after a first session, before the notes. It takes five minutes and it improves the second session more than any technique.

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Week 8 — The genogram as a hypothesis machine

8.1 The lecture in ninety minutes

The genogram is taught badly almost everywhere, as a data-collection instrument: symbols to memorise, information to gather, a diagram to file. That version of the genogram is tedious for the family and useless for the clinician. The version in this week is a hypothesis machine — a device for making visible what nobody in the family has said aloud, and for generating questions that could not otherwise be asked.

The technique is associated with Murray Bowen, who used family diagrams in the 1960s as part of his multigenerational approach, and with Monica McGoldrick and Randy Gerson, whose Genograms in Family Assessment (1985) standardised the symbols and, more importantly, the interviewing. The standardisation is a convenience; the interviewing is the method.

Minutes	Segment	What the lecturer does	What students do
0–12	What a genogram is for	Three functions: structure, repetition, relationship	Predict what each function reveals in a case
12–25	Symbols and conventions	Draw live, deliberately including errors	Copy, then criticise the drawing
25–45	The interview	Demonstrate the standard sequence on a volunteer	Interview a peer about their family of origin
45–60	Reading the map	Six patterns that recur across generations	Identify patterns in two maps
60–72	The date-line and the year	Why dates are the most powerful questions	Calculate two family dates
72–85	Genogram with the family present	Demonstrate the shared drawing	Draw with a role-played family
85–90	Set the assignment	The three-generation map and its commentary	Plan their own genogram

8.2 What a genogram is for

Three functions, and a student should be able to say which one they are using in any given session.

Function 1 — structure. The genogram makes visible who is in the family, who is not, who is close, who is cut off, who has died, who has left, who is spoken about and who is never mentioned. A family that describes itself as "just the four of us" will produce, in twenty minutes of genogram work, an absent father, two deceased grandparents, a stillborn child and a cousin who lived with them for three years. The structure is not merely background; it is the field in which the presenting problem is located.

Function 2 — repetition. The genogram makes visible patterns across generations: the age at which people leave home, marry, die, or become ill; the recurrence of a particular kind of loss; the repetition of a role (the caretaker, the black sheep, the one who never

married). Repetition is the most clinically powerful thing a genogram reveals, because it converts a personal difficulty into a family pattern, which is both less shaming and more actionable.

Function 3 — relationship. The genogram records the quality of relationships: close, distant, conflicted, cut off, abusive, fused. These are the family's own descriptions, drawn as they are described, and the drawing makes the emotional geography available for comment in a way that speech does not. "You have drawn your mother very close and your father at the edge; is that how it feels?" is a question a diagram makes askable.

What the genogram is not. It is not an assessment instrument with a scoring system (whatever the commercial packages suggest); it is not a substitute for asking about the present; and it is not a test the family can fail. Its output is not a diagram but a set of hypotheses and a set of questions.

8.2a What the map cannot show

A technique is only understood when its limits are. Five things a genogram cannot tell you.

It cannot show the family's emotional life. A double line means "close" as the family describes it; it does not tell you whether the closeness is warm, anxious, controlling or comforting. The map records the family's description, not the family's experience, and the two diverge exactly where the clinical interest lies.

It cannot show what is happening now. A genogram is a history. The date-line brings it as far as the present, but the sequence in the kitchen on Tuesday morning is not on the map, and that sequence is where the work is done.

It cannot show strength of evidence. A line drawn from one member's account is drawn in the same ink as a line confirmed by four. Mark your sources, or you will forget which is which when you reread the file in six months.

It cannot show the family's own theory. Families have explanations — spiritual, moral, medical, relational — and the genogram has no notation for them. Record them in words beside the diagram: "Family's account: this began after the grandmother's burial, and the family believes the burial was not done properly."

It cannot show the network. Schools, clinics, churches, mosques, employers, immigration authorities and courts are all part of the field in which the family lives. A genogram that stops at the household boundary will systematically miss the institution that is actually keeping the problem going (see Week 9).

8.3 Symbols, conventions and the discipline of drawing

The standard conventions are simple and worth learning precisely, because imprecision is read by the family as carelessness.

Element	Convention
Male	Square

Element	Convention
Female	Circle
Gender unknown or not disclosed	Diamond
Deceased	Symbol with a cross, with date of death
Marriage / partnership	Horizontal line between the symbols, with the date above
Divorce	Two slashes across the marriage line, with the date
Separation	One slash
Cohabitation	Dashed horizontal line
Children	Vertical line from the marriage line, then siblings in birth order left to right
Twins	Line from a single point, then branching
Pregnancy loss / stillbirth	Small filled circle at the end of a short line
Adoption	Dashed vertical line to the child
Foster child	Dashed line with a small "F"
Household membership	Dotted rectangle around those living together
Close relationship	Double line
Conflict	Zigzag line
Distant	Dashed line
Cutoff	Line with a single slash through it
Abuse	Double line with a zigzag, or a labelled arrow (use with care)

Five drawing disciplines.

1. **Draw big.** A genogram on a postcard is a diagram of the therapist's impatience. Use a full sheet.
2. **Three generations minimum.** Two is a family tree; three is a pattern.
3. **Dates on everything.** Births, deaths, marriages, divorces, migrations, the onset of the problem. The dates are where the clinical gold is.
4. **Label what you infer.** If you draw a conflict line because of something the family said rather than explicitly, mark it with a question mark. The family will correct you, which is the point.
5. **Let the family hold the pen.** At least once in the work, ask a family member to draw or correct the map. The corrections are the assessment.

8.4 The interview: a standard sequence

The genogram interview has a shape, and following it prevents the two common failures: gathering endless facts and never asking about feelings, or asking about feelings and never establishing the facts.

Step 1 — the household, now. Who lives here? Ages, occupations, health, who sleeps where, who is at school or working. Five minutes. Draw as you go, and say what you are drawing.

Step 2 — the parents' families of origin. For each parent: their parents (names, ages, occupations, health, deaths), their siblings, and the quality of those relationships. This is where most of the material is, and it is where trainees spend the least time.

Step 3 — the events timeline. Deaths, separations, migrations, losses, illnesses, job changes, moves — with dates, placed against the family's history. Then the crucial question: what was happening in the family when each of these occurred?

Step 4 — the presenting problem on the timeline. When did it start? What else was happening then? The answer frequently identifies the event that the family has not connected to the problem, and sometimes the event that nobody has mentioned at all.

Step 5 — the relationship questions. Who was closest to whom? Who did not speak? Who was the one everyone worried about? Who was the one nobody worried about? Who is most like whom?

Step 6 — the questions the map makes possible. "Your father left home at seventeen; your son is seventeen." "Your mother died in March; the school refusal began in April." "You are the eldest of six and your daughter is the eldest of two." Ask the question, then wait. The family's answer — including "that is a coincidence" — is the clinical material.

Timing. A full three-generation genogram takes 40–60 minutes with a cooperative family, and it is rarely possible in a first session. Two approaches work: a partial genogram in session one (household and the parents' families, drawn quickly and deliberately unfinished), completed over the next two sessions; or the genogram as the agenda for session two, announced at the end of session one. Never rush it; a rushed genogram is a form.

8.4a When a member will not take part

Somebody always refuses the genogram. Usually it is the father, sometimes the adolescent, occasionally the grandmother who "does not see what the past has to do with it". Refusal is information, and it is manageable.

The three refusals, and what they mean.

The unprepared refusal. "I do not see the point." This is the most common and the least meaningful. It is a comment about the therapist's explanation, not about the technique. Fix: explain the purpose in one sentence and give a small, low-threat task: "I am going to ask your wife about her family for ten minutes. If you would correct me when I get something wrong, that would help."

The protective refusal. The member does not want certain material discussed in front of certain people — a death, a previous marriage, a child given away, a prison sentence. This is almost always a request for a different format, not a refusal of the work. Fix: offer the genogram in a different configuration. "Would you rather we did this with just the two of

us first, and then I bring it back to the family?" Almost every protective refusal becomes cooperation once the audience changes.

The defensive refusal. The member has understood that the genogram will make their own history visible, and does not want it examined. This is the refusal to respect and to work with slowly. Fix: do not insist, do not interpret the refusal in the room, and note it: a family member who cannot bear to look at three generations is often the person carrying the most.

What not to do. Do not proceed as though the refuser were absent, drawing their side with a shrug and a comment about "the gaps". Families hear these comments. Do not make the refusal the topic of the session either — the therapist who spends twenty minutes processing why the father will not discuss his mother has made the father the problem.

The long game. The genogram can be returned to at any point in the therapy, and the member who refused in session two often volunteers the crucial piece of information in session eight, when something else has been resolved.

8.5 Reading the map: six patterns that recur

The genogram's value is in pattern recognition, and patterns are learned by looking at many maps. Six recur constantly in clinical work.

1. The repetition of an age. A father whose own father died when he was eleven becomes anxious when his son turns eleven. A woman who left home at seventeen becomes distressed when her daughter turns seventeen. The age is the trigger, and nobody has connected the two events because they are forty years apart.

2. The repetition of a loss at the same point in the life cycle. Miscarriage, stillbirth, the death of a child, a parent's death during the respondent's adolescence. Families often organise themselves around an unmourned loss, and the organisation is visible in the map: a gap in the birth order, a name that is never mentioned, a child who is described as "the one who came after".

3. The replacement child. A child conceived or named after a dead sibling, or treated as though they were the dead person. The pattern is visible in the map when a birth follows a death within a year, and in the interview when the child is described in terms that belong to someone else.

4. The cut-off. A branch of the family with whom contact has ceased, often after a dispute whose original subject nobody remembers. Cut-offs are transmitted: children of cut-off parents often have no relationship with half their kin, and the map makes the amputation visible.

5. The role that recurs. The caretaker, the black sheep, the one who never married, the one who "held the family together". These roles are frequently occupied by one person per generation, and the map shows who is next in line.

6. The migration pattern. Who left, who stayed, who was left behind, and at what cost. In families with significant migration, the map carries an emotional geography as

important as the relational one: the grandmother who raised the children while the parents worked abroad, the sibling who was left and never forgave, the money that flowed one way and the obligation that flowed back.

> **PRACTICAL**
> Take any published genogram from a textbook and read it in ninety seconds, writing one hypothesis and one question. Then read the case discussion. Comparing your hypothesis with the author's is the fastest way to learn pattern recognition, and it takes ten minutes per case.

8.6 The date-line: the most powerful question in the technique

If a student learns one thing from this week, it should be the use of dates.

Why dates work. Families live in time, and they rarely place events against each other. A mother who says "my father died when I was young" and "my son has always been difficult" has not noticed that "always" began in the March her father died. The date-line does the noticing for her, and it does it visibly, on paper, in front of everyone.

How to use it. Draw a horizontal line beneath the genogram, mark the years, and place the events the family has mentioned. Then place the onset of the presenting problem. Then simply point at the proximity and ask an open question: "What was happening in the family in that year?" Nothing more. The family will do the work.

What to place on the line. Births, deaths, marriages, divorces, migrations, job losses, house moves, illnesses, accidents, imprisonments, the onset of the presenting problem, and the date the family first sought help. The last one is often revealing: a family that seeks help five years after the onset has usually had a reason to wait.

Three examples of what the date-line produces.

A couple present with sexual difficulties that began "about two years ago". The date-line shows that the wife's mother died two years and one month ago, and that the couple moved into the mother's house six weeks after the death. Nobody had connected the two.

A father presents with anger at his 14-year-old son. The date-line shows that the father was 14 when his own father left the family, and that the son's difficulties began in the month the father was promoted to a job requiring travel. Two repetitions at once.

A woman presents with panic attacks. The date-line shows a stillbirth eleven years earlier, never discussed, and the panic began in the month her eldest daughter — the child born after the stillbirth — turned eleven.

The rule. Use dates, draw them, and ask the open question. Do not interpret the coincidence for the family; point at it and let them speak.

8.6a Three date-line cases from the case bank

The technique is best learned by applying it. Three short cases, each with the events as the family first gave them and the placement that changed the question.

Case 1 — Folake and Dele. Presented problem: "we argue about everything, mostly about money, for about a year". Events as first given: married 2016; first child 2018; Dele's business incorporated 2021; Folake returned to full-time work 2023; a car accident in 2023; the arguments began "about a year ago". On the date-line the therapist placed the accident and the return to work in the same month, and asked what happened after the accident. Folake had been unable to drive for four months; Dele had driven her everywhere; the arguments began when she resumed driving and no longer needed him. The question "what changed when you got the car back?" produced the session's central material, and it had nothing to do with money.

Case 2 — Mrs Okonkwo and Ebuka. Presented problem: "he will not listen; the school says he is defiant". Events: Ebuka born 2016; father posted to another state 2022; grandmother died 2023; a new baby 2024; the school's first letter 2024. The date-line showed that the defiance began in the term after the grandmother's death, and that the grandmother had been Ebuka's daytime carer from 2017 to 2022. The therapist's question was not about discipline but about who looked after him when his father left; the mother wept, and the "defiance" acquired a bereavement.

Case 3 — Aisha and Sam. Presented problem: "we cannot talk without it becoming a fight — since we married". Events: met 2019; married 2021; Aisha's father died 2022; Sam's job loss 2023; a miscarriage 2024, never mentioned by either partner in the session until the date-line. Placed on the line, the miscarriage sat three months before the couple's first appointment and eleven months after the job loss. The therapist said only: "There is a date here that we have not talked about." That sentence, and the twenty minutes that followed, was the treatment.

The pattern across the three cases. In each, the family's "about a year ago" or "always" concealed a specific date, and the specific date was connected to a loss that had not been spoken about. This is not because loss explains everything; it is because families often present the argument and not the bereavement, and the date-line is the cheapest way to invite the second into the room.

8.7 Drawing with the family present

The genogram is most powerful when the family watches it being drawn, because the drawing itself becomes an intervention.

The mechanics. Large sheet, on a table the family can see, or on a flip chart. Draw slowly. Say what you are drawing as you draw it, and use the family's words. Ask for confirmation at each step. Leave gaps deliberately where you do not know something, and mark them with a question mark.

What happens in the room. Three things, reliably. First, members correct each other, and the corrections reveal disagreements about family history that nobody knew existed ("your father did not leave in 1998, he left in 1996"). Second, members contribute information that the identified patient has never heard, because the conversation is addressed to the map rather than to the child. Third, the family's emotional temperature often drops, because the task is concrete and shared, which makes the genogram useful with families that cannot tolerate direct emotional work early on.

The question that does the most work. After the map is drawn, ask: **"Who is missing from this picture?"** Families produce, in response, the stillborn child, the sibling who died in infancy, the father who left, the aunt who is never mentioned, the child given up for adoption. The answer is the most clinically valuable sentence of the session.

The second question. "What would change on this drawing if things got better?" This converts the map from a history into a plan, and it gives the therapist the family's own criteria of change.

> **CAUTION**
> Drawing a genogram in front of a family can surface material that is painful and unexpected: a death that some members did not know about, or a secret that slips out. Be prepared to stop, to acknowledge, and to ask whether the family wants to continue. The technique must never override the family's pace.

8.7a Mapping three household structures

The standard genogram assumes a nuclear household. Three common Nigerian structures require deliberate adaptation, and each is best taught with a worked description.

Structure 1 — the extended household with an absent parent.

A household of seven: a grandmother (68), her daughter (34, the mother), three children (14, 11, 6), a niece (19) and a nephew (16) whose own mother lives in Abuja. The father works in Port Harcourt and returns monthly.

Standard drawing. Father and mother with a marriage line, three children below, and the niece and nephew attached to nobody, floating in the corner of the page.

The drawing that matches the family. The father is drawn at the top right, in a dotted rectangle labelled "Port Harcourt, monthly"; a cohabitation-style dashed line to the mother, labelled "monthly"; the grandmother's square drawn close to the mother's circle with a double line; a dotted rectangle around grandmother, mother, the three children, the niece and the nephew, labelled "same compound"; the niece and nephew attached to their own mother in Abuja by a dashed line, labelled "fostered since 2019".

Why the second drawing is better. It shows at a glance what the first hides: that the meaningful parental structure in this household has three adults in it, one of whom is physically absent; that the grandmother is the daily authority; and that two children in the house are there because of an arrangement that has a date. The first drawing produces the hypothesis "father is disengaged". The second produces the hypothesis "the parental subsystem is a grandmother–mother coalition, with a father who participates monthly and two children whose placement has never been discussed". The second is more useful and more accurate.

Structure 2 — two households, one father.

A father (41) with a wife (38) and two children (12, 8) in Ibadan, and a second partner (33) with one child (5) in Abeokuta. All four adults know each other. The second partner's child spends school holidays in Ibadan.

The drawing. Two unions from the father's square, each with its own line — a marriage line to the wife, a partner line to the second partner — the children attached to the correct line, and a dotted rectangle around the Ibadan household, with an arrow from the Abeokuta child into the rectangle labelled "holidays".

What the map makes discussable. The obligations, the money, the school fees, the question of who is introduced to whom, and — frequently — the wife's experience of the arrangement, which she has never been asked about in a clinical setting. Drawing the family accurately is itself an act of respect; drawing it as though the second household did not exist tells the family that the therapist has already decided what counts as a family.

Structure 3 — the transnational family.

Parents migrated to the United Kingdom in 2015; three children remained in Lagos with the paternal grandmother, and the eldest joined the parents in 2021 at the age of fifteen; the two younger children joined in 2023.

The drawing. The parents' squares and circles in a dotted rectangle labelled "UK since 2015"; the children connected, with a migration arrow and date beside each movement; the grandmother's symbol in a separate dotted rectangle labelled "Lagos", with a double line to each of the children, labelled "raised her".

What it reveals. The sibling group has been separated and reunited in a particular order, at particular ages, and the person who did the raising is not in the household. The eldest child, who joined first at fifteen, spent six years as an only child in a new country; the youngest, who joined at twelve, spent her early adolescence with her grandmother and joined her parents at the point when adolescents typically begin to separate. These are facts about attachment, obligation and belonging, and they are visible in ninety seconds on the right drawing.

The general principle. Draw what is there. Every adaptation to the standard symbol set is justified if it makes the family more recognisable to itself, and every departure from the standard should be explained to the family in one sentence: "I am drawing it this way because this is how you have described it to me."

8.8 Annotated transcript: the genogram that changed the session

A sixth-turn sequence from a second session with the Adeyemi family. The mother has agreed to a genogram; the father is present but has said little; Amara is at school.

> **T1 Therapist:** I would like to draw your two families — your parents, your brothers and sisters, and the dates. It takes about twenty minutes and I will ask you to correct me. Mother, your side first. Your mother's name?
> **T2 Mother:** Mama. She is 71. She lives in Ibadan.
> **T3 Therapist:** (draws) 71. And your father?
> **T4 Mother:** He died. 1994. I was 19.
> **T5 Therapist:** (draws, then pauses) You were 19. And you are the eldest?
> **T6 Mother:** I am the eldest of five. When he died I stopped school and I looked after them. My mother was not… she was not well after it.
> **T7 Therapist:** You looked after four children at 19, while your mother was unwell. (draws) Where were you all?
> **T8 Mother:** In the same house. Until I married. I married at 24.
> **T9 Therapist:** (to Father) And your side?
> **T10 Father:** My father is 68, he is in Lagos. My mother died in 1998. I am the third of six.
> **T11 Therapist:** (draws) How old were you in 1998?
> **T12 Father:** Sixteen. I left for Lagos that year. For work.
> **T13 Therapist:** And your mother had died that year.

(The therapist draws a date-line: 1994, 1998, 2001, 2008, 2019, 2025. She places the deaths, the mother's marriage, the father's departure, the births of the children, and then the onset of the school refusal.)

> **T14 Therapist:** Here is your mother's father's death in 1994. Here is your father's mother's death in 1998, and your leaving in the same year. Here is 2008, when Amara was born, and 2019, when your husband lost the job. And here is this year, when the school refusal began.
> **T15 Mother:** (after a pause) 2019 is when everything changed.
> **T16 Therapist:** What happened in 2019?
> **T17 Mother:** He was at home for two years. Then he found this work, but it is in Lagos. He is away Monday to Friday. (pause) I am alone with them, like I was alone with my sisters.
> **T18 Therapist:** You were 19 and you were looking after four children, and now you are looking after three children alone on weekdays. (pause) Nobody has said that out loud in this room.
> **T19 Father:** She has not said it to me either.
> **T20 Therapist:** (draws a line between the parents, marked "distant, weekdays") Is that the right line?

Annotations. T1 announces the task, its duration, and the correction clause. The correction clause is what makes the technique collaborative rather than extractive, and it produces better data.

T5 is the date-line question asked early, and it does more work than any question about the marriage. T7 repeats the mother's sentence back, which is a simple and under-used technique: repetition without interpretation invites elaboration.

T13 is a one-sentence observation, followed by silence. Note that it is not an interpretation. It is a statement of fact, in sequence, which the mother can take or leave.

T14–T17 is the date-line doing the clinical work. The therapist has not said that the mother is repeating her adolescence, or that the father is repeating his own departure. She has placed the events on a line, and the mother has done the rest.

T18 is the most dangerous sentence in the transcript and it is carefully built: it names a repetition, but it names it as an observation about the mother's life rather than as an accusation about the father. The sentence is delivered with two facts, not one interpretation, and it is followed by a pause long enough for the mother to respond.

T19 is the moment the session changes. The father speaks, and what he says ("She has not said it to me either") is both a defence and an opening. The therapist does not pursue the defence; she draws a line, labels it with the family's own description, and asks for correction. The line is the intervention: the distance is now on the paper, where both of them can see it, rather than in the mother's private resentment.

What the therapist avoided. She did not ask the father why he drinks. She did not connect his mother's death to his absence, although the connection is obvious and may be right. She did not use the word "abandonment". Each of these would have closed the session in a way that this sequence opens it.

8.9 Laboratory: the three-generation genogram

The assessed practical for this week runs over two sessions: one in class, one completed at home.

Part A (in class, 60 minutes). In pairs, take a full three-generation genogram of your partner's family of origin, using the standard sequence. Then swap, and take the genogram of the partner's chosen family — the people they would call family, which is often not the people on the birth certificate. The second version is the harder and more instructive task.

Part B (at home). Draw your own three-generation genogram on a single A3 sheet, with dates, and write a 500-word commentary answering four questions: what pattern repeats; what event coincides with the onset of a difficulty in the family; who is missing from the map; and what the map suggests you would find hardest to ask about.

Marking rubric.

Criterion	Weight	Full marks
Symbolic accuracy	20%	All symbols correct; dates present; household marked
Coverage	20%	Three generations; both sides; losses, cut-offs and migrations included
Interviewing	20%	Standard sequence followed; family's words used; corrections invited
Pattern reading	25%	Two defensible hypotheses, each tied to a specific feature of the map
Commentary	15%	Honest; identifies the student's own blind spot without exhibitionism

Two warnings for the assignment. Students sometimes produce a genogram that contains material a family member would not want shared. The rule is that the sheet stays in the student's possession and only the commentary is submitted, with names replaced.

And students sometimes discover something about their own family in the exercise that they need to talk about. That is not a failure of the assignment; it is the assignment working, and there is supervision available.

8.9a Reading a published genogram: an exercise in hypothesis generation

The skill this week is not drawing; it is generating hypotheses and knowing which ones to test. Here is the exercise that develops it fastest, done in pairs in fifteen minutes.

The map. Take any published genogram with a case discussion — McGoldrick and Gerson contains dozens. Read only the diagram and the three-line referral. Do not read the discussion.

Step 1 — three structural facts. Write the three most striking structural features: an unusual age gap, a death at a particular point, a cut-off, a household composition that does not match the birth order, a cluster of events in one year. Keep to facts on the page.

Step 2 — two hypotheses. Write two competing hypotheses that would explain the referral, each tied to a different structural fact. The discipline of two is important: it prevents premature certainty and it trains the habit of holding alternatives.

Step 3 — the discriminating question. For each hypothesis, write the single question whose answer would most reduce your uncertainty. This is the most important step, and the one students skip.

Step 4 — read the discussion. Compare. The published author will usually have seen one of your hypotheses and missed the other, and will sometimes have seen something you did not. Note which.

Worked example. A referral: "Parents request help with a 9-year-old son, defiant at home." The map shows a father who is the eldest of five, his own father dead at 62 of a stroke when the father was 9; a mother who is the youngest of three, her parents alive; the son is the couple's only child; the father's youngest brother is drawn with a conflict line to the father.

Structural facts: the father was 9 when his own father died; the son is 9; the father is in conflict with the brother who is closest in age to the son; the son is an only child, in a family where the father was one of five.

Hypothesis 1 (the age repetition): the son's ninth year has reactivated the father's own loss, and his anxiety is expressed as strictness, which the son experiences as defiance.

Hypothesis 2 (the role): the father's conflict with his youngest brother has moved into the household, and the son has become the brother he cannot speak to.

Discriminating questions: "What was your father like when you were nine?" (tests H1). "When you argue with your son, who does he remind you of?" (tests H2). Either answer is informative, and both are askable in the first session.

What the exercise teaches. The genogram does not give answers; it gives the shape of the question. A clinician who leaves the map with three structural facts, two hypotheses

and two questions is ready for a second session. A clinician who leaves with a diagram has drawn a picture.

8.10 Six errors in genogram work

1. **The questionnaire genogram.** Twenty minutes of questions with no drawing in front of the family, and no hypothesis. Fix: draw, say what you are drawing, and ask the sequence questions aloud.
2. **Two generations.** A map that stops at the parents cannot reveal repetition. Fix: three generations, always, even if the third is sparse.
3. **No dates.** A genogram without dates is a diagram of names. Fix: dates on births, deaths, marriages, moves and the onset of the problem.
4. **The therapist's interpretation delivered as fact.** "So your mother's death is why you cannot let your daughter go." Fix: place the events, ask the open question, and let the family make the connection — or not.
5. **Ignoring the missing.** A map that reproduces only what the family offers will omit the disowned, the dead and the never-mentioned. Fix: ask "who is missing?" and draw what appears, including the gaps.
6. **The genogram as an end in itself.** Six sessions of history and no present-day work. Fix: the genogram is a hypothesis machine; every hypothesis it produces should generate a question about this week.

8.10a The ten-minute genogram

Most clinical settings do not permit a sixty-minute genogram interview. A workable compressed version takes ten minutes and produces two hypotheses, which is more than most first sessions produce.

Minutes 1–3: the household. Who lives here, ages, and who else is involved in raising these children. Draw as you ask; keep it rough.

Minutes 4–6: two dates. The year the parents married, and the year the presenting problem began. Ask what else was happening in the year the problem began.

Minutes 7–8: one question about the parents' own childhoods. "When you were your son's age, what was your household like?" Ask both parents; draw a rough sketch of each family of origin.

Minutes 9–10: one question about the missing. "Who else is in the family who we have not mentioned?" Draw what appears. Stop.

The output. Two hypotheses and two questions for the next session, in the notes. Nothing more. The compressed version is not a substitute for the full interview, and it should be completed in the second or third session when the material justifies it.

8.11 Culture, faith and the genogram

The genogram looks like a neutral technique and is not. Five adjustments for work in Nigeria and in any context where kinship does not follow the nuclear template.

Kinship is wider than the diagram. In many Nigerian families, the household includes cousins, wards, nieces, apprentices, house-help and elderly relatives, and obligations extend to a wider lineage than the genogram's three generations can show. A genogram that records only the parents and their children will systematically miss the people who are raising the children and the people who hold authority over them. Draw the household as it actually is, with a dotted line around everyone who eats from the same pot, and ask about the wider lineage separately.

Polygyny and multiple households. A genogram based on a single marriage per person misrepresents a family with two households, co-wives, or children raised between homes. The convention is to draw each union honestly, in whatever form it takes, with a cohabitation line, a marriage line or a partner line, and to record where each child was raised. The map must match the family, not the template.

Names and the dead. Some families will not speak the names of the dead, and some will not discuss a death at all. The genogram must be built at the family's pace, and the therapist must be able to leave a symbol undrawn. Ask permission: "Some families do not like to say the names of those who have died. Tell me what is comfortable for you, and I will draw only that."

The spiritual lineage. In families with strong religious or traditional commitments, the significant "ancestors" are not only biological. A map that includes the church, the mosque, the imam, the pastor, the traditional healer or the family's deity as significant figures in the family's structure is often more accurate than a purely biological one. Ask: "Who else is a parent in this family — in the sense of having authority or responsibility for these children?"

Migration and remittance. A genogram of a transnational family should record who migrated, when, and who was left behind with whom. The grandmother who raised four grandchildren while their parents worked abroad is a structural fact of the family, not a footnote, and the guilt, obligation and resentment that accompany the pattern are frequently the material of the presenting problem.

> **DEBATE**
> Is the standard genogram a culturally specific instrument that imposes a Western kinship model on families whose structure it cannot represent? Take a position, and argue it with reference to a family you know.

8.12 Evidence note: what the genogram is and is not

The genogram's evidence base is thinner than its popularity suggests, and honesty about this is part of professional competence.

What is established. Genograms are reliable as descriptive instruments: trained clinicians can produce similar maps from the same family, and inter-rater agreement on structural facts (household composition, deaths, marriages) is generally good. Genogram data have been used in family medicine to identify risk patterns, in genetic counselling to map heritable conditions, and in systemic research as a way of recording family structure.

What is not established. There is little evidence that genogram-derived interpretations — the multigenerational hypothesis, the "repetition compulsion" reading, the assumption that a pattern in the map explains a symptom in the present — are valid. The genogram generates hypotheses; it does not confirm them. A therapist who treats the map as an explanation has confused a heuristic with a finding.

What the research does support. The genogram is a powerful engagement device. Families report the genogram interview as one of the more comfortable and revealing parts of an assessment, and the technique reliably produces information that families have not previously connected. Its value is in the process — the drawing, the corrections, the date-line — rather than in the diagram that results.

The professional implication. Use the genogram to ask better questions, not to reach conclusions. Record the family's own account of the pattern alongside your hypothesis, and mark your hypothesis as such. When you write a report, distinguish what was drawn from what was inferred, and give the family the opportunity to correct the inference before it becomes part of a record that will outlast the therapy.

8.12a Putting the genogram in the record

The genogram will be filed, and the filing requires judgement. Four rules.

Record the family's account and your hypothesis separately. The record might read: "Family's account: father's mother died 1998, when he was 16; he left for Lagos that year. My hypothesis: father's own experience of loss at 16 may be connected to his difficulty in remaining present for his son's adolescence; not yet tested." The distinction matters for supervision, for any subsequent clinician, and for the family if they ever read the file.

Do not record unconfirmed third-party information as fact. If a mother says that her husband's sister was "sent away" at 17, and nobody confirms what happened, the record says "mother reports" — not "aunt was sent away".

Obtain agreement about who sees the map. The genogram is a document about people who are not in the room, and those people have not consented to being mapped. In a shared record, mark the diagram as clinical material with restricted access where the system allows it, and never circulate it beyond the treating team without consent.

Give the family their own copy, if they want one. Some families will want to keep the map, to take it home and correct it. This is a good outcome: the map that the family corrects at home is better than the map the therapist files.

Storage. A genogram contains health information about relatives who are not clients. Follow the same rules you would for any clinical document: secure storage, defined retention period, no photographic copies on personal devices, and no use in teaching without consent and anonymisation thorough enough that the family could not recognise itself.

8.12b Two genograms of one family, and the difference it made

The final worked example for the week, showing how the drawing changes the formulation.

The map produced in session two. Mother, father, two children aged 9 and 4. Father's parents: both alive; father is the second of four. Mother's parents: both alive; mother is an only child. Presented problem: the 9-year-old's tantrums.

The formulation that followed. A firstborn child, displaced by the arrival of a sibling, in a family where the mother has no experience of siblings and the father has three. The tantrums were read as a protest about the dethronement. The plan was a behavioural programme for the tantrums and a discussion of the parents' expectations of a firstborn.

The map produced in session five, after the family had corrected it. The father is the second of four, but the first of those four died at 14 months, before the father was born. The mother is an only child whose own mother had four pregnancies and three losses. Nobody had mentioned either fact, because in both families the losses were "not spoken about".

The formulation that followed. Two parents who had each grown up in a household organised around a child who died, and who had each learned that a child's survival was not to be assumed. The 9-year-old's tantrums were being met not with ordinary irritation but with alarm, and the father's nightly checking of the child's breathing — which the family had not mentioned either — now made sense as a symptom of the parental history rather than of the child's behaviour. The tantrums were addressed, and so, in the following weeks, was the checking.

Why the father's checking was not asked about. Because the therapist had a map that made it unnecessary to ask. The tantrums were the referral, the parents were cooperative, and the programme was working well enough to satisfy the school, the parents and the therapist. A formulation can be adequate to the referral and still miss the thing the family most needs to say, and the only reliable protection against that is to keep asking the structural questions after the crisis has passed, when there is time to hear the answers.

The lesson. The first map was accurate as far as it went. It lacked two symbols and a date, and those two symbols reorganised the case. The therapist's job is not to draw the perfect map in the first session; it is to keep the map open, to keep asking who is missing, and to expect that the answer will change the case a second time — as this one did in the fifth session, when the family said the breathing had never been mentioned because there was nothing to say.

8.13 Key terms

Term	Definition
Genogram	A multigenerational family diagram with dates, relationships and events
Date-line	A horizontal timeline on which family events and the onset of the problem are placed
Cut-off	A relationship in which contact has ceased, often transmitted across generations
Replacement child	A child conceived or treated as a substitute for a deceased sibling
Fusion	A relationship in which the two parties have little separate identity (Bowen)

Term	Definition
Differentiation	The capacity to remain oneself while emotionally close to others (Bowen)
Triangle	A three-person configuration in which tension between two is displaced onto a third
Family of origin	The family in which a person was raised
Family of procreation	The family a person forms as an adult
Horizontal stressor	A difficulty occurring in the present generation
Vertical stressor	A difficulty transmitted from previous generations (McGoldrick)
Household marker	A dotted line around those living together

8.14 Self-test

1. What are the three functions of a genogram?
2. Why is three generations the minimum?
3. What is the date-line, and what question follows it?
4. Name four patterns that recur across generations and are visible on a genogram.
5. What is the most useful question to ask once the map is drawn?
6. What does the evidence say about genogram interpretation?
7. How should a genogram represent a family with two households?
8. Why should the family hold the pen at least once?

Answers. 1. Structure, repetition, relationship. 2. Two generations show a family; three show a pattern, because repetition requires comparison across at least three. 3. A timeline of family events with the onset of the problem placed on it; the question is "what was happening in the family in that year?" 4. Repetition of an age; repetition of a loss at the same life-cycle point; the replacement child; the cut-off; the recurring role; the migration pattern. 5. "Who is missing from this picture?" 6. Descriptive reliability is reasonable; interpretive validity is not established — the genogram generates hypotheses rather than confirming them. 7. Draw each union honestly, with its own line type, and record where each child was raised. 8. Because the corrections are the assessment, and drawing distributes ownership of the map.

8.15 Reflective task

Draw your own genogram before the seminar, with dates, and place on the date-line the year you decided to train as a therapist. Then write 250 words on the proximity of that decision to any other event on the line. Students frequently find that the decision is closer to a family event than they had realised; that discovery is not a diagnosis of your motives, it is an example of how the technique works.

8.16 Readings for this week

- **McGoldrick, Gerson & Petry (2008)**, Genograms: Assessment and Intervention, 3rd edition. The standard text; read the chapters on the genogram interview rather than the symbol list.
- **Bowen (1978)**, Family Therapy in Clinical Practice, especially the chapters on multigenerational transmission and triangles.
- **McGoldrick & Gerson (1985)**, the first edition, for the historical development of the conventions.
- **Kerr & Bowen (1988)**, Family Evaluation, chapters 4–6, for the theory the genogram was designed to serve; read chapter 5 twice if triangles are still abstract, because they will not be after the second reading, and then draw three of your own from memory before you go back to the book.
- **Bennett & Wolin**, any paper on the "resilience genogram", for a strengths-oriented variation.
- **A published case study** that includes a full genogram and a date-line, read to see how an experienced clinician moves from the drawing to a hypothesis.

Next: Week 9 — Formulating relationally

Week 9 — Formulating relationally

9.1 The lecture in ninety minutes

Assessment gathers. Formulation explains. The distinction is not academic: a service can be full of assessments and empty of formulations, and the result is a file that records everything and understands nothing. This week is about the second half of the process — turning the material of Weeks 6 to 8, plus whatever standardised measures the service uses, into a relational explanation that generates a plan and can be tested.

The week's central claim: **a formulation is a hypothesis written in a form that could be wrong.** Everything else follows from that sentence. If your formulation could not be wrong, it is a description or a belief. If it could be wrong but you have not said what would show it to be wrong, it is not yet usable. If it could be wrong and you have said how, you have a professional document.

Minutes	Segment	What the lecturer does	What students do
0–10	Assessment versus formulation	Two files on one case, both complete	Identify which explains
10–25	What a formulation does	The five jobs, with examples of each	Match jobs to sentences from real reports
25–45	A relational template	Build one on the board from a case	Supply the material from the case bank
45–60	Competing formulations	Four lenses on one family; four plans	Choose a lens and defend the plan

Minutes	Segment	What the lecturer does	What students do
60–72	Writing it	The one-page format; the testability clause	Draft one page in pairs
72–85	Presenting it to the family	Demonstration and correction	Rehearse the first three sentences
85–90	The network	Who else is in the system, and who decides	Map one real network

9.2 What a formulation is, and the five jobs it does

A formulation is a provisional, collaborative explanation of why this difficulty, in this family, at this time, and what would change it. Five jobs, each of which a good formulation performs explicitly.

Job 1 — it organises the information. The assessment produces forty facts: ages, diagnoses, dates, scores, incidents, accounts. A formulation selects and arranges them so that some are central and others are background. The selection is a judgement and should be visible as one: "I am treating the father's absence as central, and the school's response as background, because the pattern changes when he is at home."

Job 2 — it explains the maintenance. The most important single sentence in any formulation is the one that answers: what keeps this going now? Not why it started — that is usually unknowable and almost never the lever — but what is holding it in place this month. A formulation without a maintenance account is a history.

Job 3 — it predicts. A formulation should say what will happen if nothing changes, and what should change first if the intervention works. "If the parents continue to disagree about discipline in front of the child, the child's behaviour will not change, whatever programme is used; if they agree on one rule for two weeks, the behaviour should reduce within a fortnight." That sentence is testable, and it commits the therapist.

Job 4 — it generates a plan. The plan should follow from the formulation with a visibility that a colleague could trace. If the formulation says the problem is maintained by the parents' disagreement, the plan must include work on the parents' disagreement; a plan of individual sessions for the child would contradict the formulation, and either the plan or the formulation is wrong.

Job 5 — it can be revised. Formulations are hypotheses, and they change with evidence. A formulation that survives twenty sessions unchanged is either miraculously correct or no longer being used.

What a formulation is not. It is not a diagnosis (a classification of a condition), not a summary (a condensed record), not an interpretation delivered to the family as insight, and not a story that makes the therapist feel clever. The temptation of the elegant formulation is the subject of one of this week's errors.

> **CAUTION**
> Formulation is a professional activity with a real risk of harm: a persuasive formulation can create a self-fulfilling account that the family adopts and cannot challenge. Write formulations in language the family could read, and give them the chance to disagree with it.

9.2a Bringing the numbers into the formulation

Most services now use standardised measures, and trainees often treat them as an audit requirement rather than as clinical material. Three ways to build self-report data into a relational formulation.

Use the discrepancy, not the score. The SCORE-15 and similar family measures give several viewpoints: each member rates the family's communication, the difficulty, and their own coping. The total score is the least interesting number on the page. What matters is the pattern of disagreement: a family where the mother rates the difficulty at 5 and the father at 1, when both are describing the same household, is telling you something about who carries the worry. The formulation should say so: "The parents' ratings of the difficulty differ by four points on a five-point scale; the mother's rating is consistent with her account of being alone with it."

Use the change, not the level. A family that begins at a high level of distress and falls sharply is improving faster than a family that begins at a moderate level and does not move. Plot the scores, show the family the plot, and ask them what they make of it. Families find their own data persuasive in a way that a therapist's interpretation is not, and the plot gives the therapist a shared object of attention that is not a person.

Use the feedback to break the bad-news loop. Where the scores for a young person are far worse than the parents' ratings, the therapist has evidence for something the young person may not be able to say and the parents may not be able to hear. The formulation can hold it: "The two ratings do not match, and both are true — this is how it looks from where each of you is sitting." This is often the most useful intervention of a first session.

What measures cannot do. They cannot tell you the mechanism, they are culturally loaded in ways that are still poorly understood, and they are easy to over-interpret. A score is a sentence the family has said in a constrained format; treat it as one piece of what the family has said, not as the family's verdict.

9.3 The 4 Ps in a systemic key

The most widely used formulation structure in clinical training is the four Ps: predisposing, precipitating, perpetuating, protective. It transfers well to family work if each P is rewritten in relational terms.

Predisposing (relational history). What in the family's history makes this difficulty more likely? Patterns transmitted across generations, attachment experiences, previous losses, models of marriage and parenting, migration history, experiences of

discrimination, and the family's relationship with institutions. Not "genetics" alone, and not a list of traumas: an account of how the family learned to manage the kind of stress it now faces.

Precipitating (the event and its meaning). What happened, and what did the family make of it? The event is usually identifiable — a redundancy, a death, a move, a diagnosis, an adolescence, a birth. The meaning is what converts an event into a crisis: "his redundancy meant that he was no longer the provider, and in this family that is what a man is." Both parts are needed; the event alone is a chronology, the meaning alone is a mood.

Perpetuating (the pattern). What is keeping it going now? The cycle between the members, the family's attempted solutions, the responses of the wider system (school, employer, clinic), and the meanings that have hardened. This is the section that does the clinical work, and in a family formulation it should name at least one loop with three or more elements, and at least one attempted solution that has become part of the problem.

Protective (the resources). What is holding the family together and what can be built on? Not a list of compliments: a specific inventory of capacities — who works, who is affectionate with whom, what the family has survived, which relationships are intact, which professionals are allies, what the family does well on good days.

A fifth P for family work: the pattern of help-seeking. How has this family used help before, and what happened? Families that arrive with a history of failed services have a formulation-relevant history of failed services, and the history predicts how this episode will go unless the therapist changes something (see Week 5).

Worked four-P summary, the Okonkwo family.

P	Content
Predisposing	Mother the eldest of five, raised siblings from 19 after her father's death; father posted away for work since 2022; both parents raised in households where children were disciplined by senior relatives rather than by parents
Precipitating	Paternal grandmother, Ebuka's daytime carer from 2017 to 2022, died in 2023; new baby born 2024; school's first behavioural letter in the same term
Perpetuating	Ebuka's defiance escalates in the mornings; mother responds with threats she does not carry out; father, home at weekends, over-corrects; school telephones the mother, who feels judged and does not tell the father; the parents' only conversations are about Ebuka
Protective	Mother is resourceful and organised; father is affectionate on return and is not distant by choice; a maternal aunt is a trusted ally; the school's head of year is willing to meet
Help-seeking	First contact with services; the mother telephoned after the school's second letter, without telling her husband

What the table does. It shows, in five rows, why a plan of "parenting classes for the mother" would fail (the perpetuating pattern is triadic and institutional) and why a plan involving the father, the school and the bereavement is more likely to work.

9.4 A one-page relational formulation template

Services use many templates. The one below is deliberately short, because a formulation longer than a page is usually two formulations with the contradiction hidden.

1. The presenting pattern (three sentences). Who, what, when, and what has been tried.

2. The central loop (a diagram or four sentences). The cycle that maintains the problem, with the members' behaviour at each step. This should be drawable.

3. The meanings that hold the loop in place. What each member believes about the others, and about themselves, that makes the behaviour seem necessary.

4. The context. Developmental stage, health, work, money, housing, migration, culture, faith, and the institutions involved.

5. History in one paragraph. How the family arrived here, with the dates that matter.

6. What has been tried, and what it did. Including previous therapy and its ending.

7. Strengths and resources. Specific, evidenced, and usable in the plan.

8. Risk. Stated plainly, with a management plan, or stated as "no current indicators" after a screen.

9. The plan, derived. Each element of the plan should name the element of the formulation it addresses.

10. The testability clause. Two or three sentences: what would show this formulation to be wrong, when it will be reviewed, and what would be done differently if it is.

The testability clause, worked. "If the central loop is the parents' disagreement about discipline, then two weeks of a single agreed rule should reduce the morning conflict, even before any work on the child. If the mornings do not change, the loop is elsewhere — most likely in the mother's bereavement, which we have not yet addressed — and I will shift the focus at the review in three weeks."

9.5 Worked formulation: the Adeyemi family, six sessions in

A full formulation, written as it would appear in a clinical record after six sessions. Read it for how the sections connect, not for the content.

Presenting pattern. Amara, 16, has not attended school for four months. Mornings follow a five-link sequence (wake, refuse, insist, cry, telephone the school with an untrue explanation). The mother has contacted the school four times and has told nobody in the extended family. The father is away Monday to Friday and has attended two of the six sessions.

Central loop. Amara refuses; the mother insists and then telephones the school; the school accepts the explanation; Amara stays home; the mother spends the day angry and guilty; the father returns at the weekend and is told only the headline; the mother feels alone with it; the next morning the sequence repeats with less hope. A second loop runs

between the parents: the father's advice at weekends is experienced as criticism, so the mother tells him less, so he knows less, so his advice is less useful.

Meanings. The mother believes that a good mother manages her children alone, and that asking for help is evidence of failure; her own mother managed four children after a death, and this is the standard she measures herself against. Amara believes that her mother cannot bear her distress, and that staying home protects them both from a conversation neither can have. The father believes he has already lost standing in the household and that his contribution will be rejected.

Context. Two incomes, one of them insecure; the father's work is in another state and is not negotiable in the short term; the maternal grandmother is 71 and lives nearby; the family attends a Pentecostal church where the mother is a respected member, which raises the cost of disclosing the difficulty; Amara is in the final two years of secondary school, with examinations in eight months.

History. The mother was 19 when her father died and she raised four siblings. She married at 24 and had Amara at

1. The father's mother died when he was 16, the year he left for Lagos. The redundancy in 2019 began two years of

his being at home; the new post in 2021 restored the income and removed him from the household in the week. The school refusal began nine months after the father's return to work and three months after Amara's closest friend left the school.

What has been tried. Insistence, withdrawal of the telephone, a conversation with the school's counsellor, prayer and a fast, and a three-week period in which the mother took leave from work to supervise the mornings. All were reasonable; each was tried alone; none addressed the loop.

Strengths. The parents agree on the goal and disagree only on method; the mother is resourceful and has kept the household functioning through two difficult years; the father is affectionate and wants to be involved; the grandmother is available and willing; the school's head of year has offered flexibility; Amara has a part-time job she values and wishes to keep.

Risk. No current indicators of self-harm or suicidal ideation; no domestic violence disclosed and no indicators on screen; safeguarding referral not indicated at this stage; the school's threat to report the absences is live and is itself a stressor. Reviewed each session.

Plan, derived from the formulation. (1) Address the institutional loop first: a letter to the school written with the mother, ending the deception, and a meeting at which the therapist is present. This addresses the "meanings" section — the mother's fear of being seen as a bad mother. (2) Both parents in the room for sessions 4, 5 and 7, to interrupt the weekend–weekday information loop. (3) A conversation about the maternal standard of self-sufficiency, framed as history rather than as criticism. (4) A small, staged return to school, negotiated with Amara and the school, beginning with two mornings a week in a

supported room. (5) Bereavement and loss work with the mother only if she raises it, and not before the school situation is stable. (6) Review at session nine.

Testability clause. If the institutional loop is central, then ending the deception should reduce the mother's guilt and the morning escalation within three weeks, before any change in attendance. If the morning sequence does not shift, the central loop is more likely to be the mother–daughter dyad, or the marriage, and the plan will be reordered at the review.

What this formulation does that an assessment would not. It commits to a lever, a sequence of moves, and a condition under which the therapist would be wrong. It could be read by a colleague, disagreed with, and improved — which is the professional standard.

9.5a The network map: one case, drawn

The technique of mapping the professional network is best shown rather than described. Here is the network for the Okonkwo family, drawn in the fifth session and used in the case conference.

The nodes. Mother; father (weekends); Ebuka (9); the new baby; the maternal aunt; the deceased paternal grandmother; the school (head of year, class teacher, SENCO); the general practitioner; the clinic (therapist); the church (a pastor the mother trusts but has not told).

The links, as drawn. A thick line between mother and school, labelled "telephone, escalating"; a dashed line between father and school, labelled "nothing"; a double line between mother and aunt, labelled "support"; a conflicted line between mother and father, labelled "reports on Saturday"; a dotted line from the therapist to the school, labelled "consent given, letter sent"; and a symbol for the grandmother with a cross, positioned close to Ebuka and to the mother.

What the drawing showed that the file did not. Three things: the school had more contact with this family than any professional, and all of it was aversive; the father had no relationship with the institution that was defining his son; and the one person with moral authority in the mother's life — the pastor — was entirely outside the network, which meant the family's principal source of meaning and support was uninformed about the difficulty and therefore unable to help.

What changed as a result. The therapist wrote to the school with consent, naming a single contact and asking for fortnightly review by telephone with both parents; the school agreed, and the head of year spoke to the father for the first time. The father's inclusion in the institutional loop was, on the family's own account at the review, the change that mattered most. The pastor was not contacted, at the mother's request, and that decision was recorded.

The habit to build. On one page, name every institution and every significant person, draw the links, label each link with its quality and its frequency, and then ask: which link is doing the most harm, and which missing link would do the most good? The answer to the second question is usually the first thing the plan should address.

9.6 Four lenses on one family

The same case, formulated four ways. The purpose of this section is not relativism — "any formulation will do" — but discipline: each lens must be stated in a form that could be tested, and the therapist must be able to say why they chose this lens and what would make them change it.

The case. A couple, married eleven years, two children (7 and 4). They present with "constant arguing, no affection, and we do not know whether we should stay together". Both work; the wife earns more; the husband's mother lives with them; the wife says the husband is "never on my side"; the husband says the wife "has no respect for my mother".

Lens 1 — structural. The household contains a three-generational hierarchy in which the grandmother holds executive authority over the children and the parenting, and the couple has no boundary around itself. The wife is in a coalition with the children against the grandmother; the husband is in a coalition with his mother. The maintenance mechanism is the cross-generational alliance and the absence of a clear couple subsystem.

Plan: strengthen the couple boundary (a weekly conversation with a defined agenda; decisions made by the parents before consulting the grandmother); restructure the hierarchy with the grandmother's authority acknowledged but limited to specific domains; then work on the marriage.

Test: if the formulation is right, agreeing even one domain in which the parents decide without the grandmother should reduce the arguing within three weeks.

Lens 2 — emotion-focused. The arguing is a protest against disconnection. Underneath the conflict are primary emotions — the wife's fear of not mattering, the husband's shame at being judged inadequate — which are expressed as secondary anger. The cycle is: she criticises to be heard; he defends or withdraws to avoid shame; she escalates; he withdraws further. The arguments are the couple's way of staying engaged.

Plan: track the cycle; access the primary emotions; enact a new cycle in which she can name her fear and he can stay present; consolidate.

Test: if the formulation is right, the wife's ability to express the fear without criticism should produce a softening in the husband within one or two sessions. If he withdraws further, the formulation needs revision.

Lens 3 — narrative/social-constructionist. The couple are living inside a story about what a marriage and a family should be — the wife's story of partnership and equality, the husband's story of filial duty and respect, the grandmother's story of a household she holds together. The problem is not the people but the collision of stories, each of which is legitimate and neither of which has room for the other.

Plan: externalise the conflict ("the Respect argument", "the Loyalty problem"); map the influence of each story; find unique outcomes where the couple have resisted the conflict; re-author a story that can hold both duties.

Test: if the formulation is right, externalising should reduce defensiveness and allow the couple to describe their positions without accusing each other, within two sessions.

Lens 4 — behavioural/cognitive-behavioural with a family frame. The couple's interaction is maintained by negative reciprocity and by cognitive attributions ("she does not respect me", "he always takes his mother's side") that are confirmed by selective attention. The grandmother's presence increases the frequency of the triggering events. There is also a skills deficit in negotiation and repair.

Plan: behavioural exchange (daily check-in, agreed small positives); attribution retraining; communication and problem-solving training; a structured conversation with the grandmother about domains.

Test: if the formulation is right, a two-week behavioural exchange should raise the rate of positive exchanges measurably and reduce the frequency of arguments.

What the comparison teaches. The four lenses identify overlapping and partly incompatible mechanisms, and each generates a plan that would probably help some couples. Three disciplines follow.

First, choose deliberately and say why. "I am working structurally first because the household hierarchy is the most immediate source of the wife's experience of being unsupported, and because a behavioural programme will not survive the grandmother's daily countermanding."

Second, write the test. Each lens above comes with a condition under which it would be falsified. A therapist who cannot state such a condition is practising a faith rather than a method.

Third, change lenses when the test fails. The most common professional failure is not choosing the wrong lens; it is keeping it after the evidence has gone the other way, because the therapist is more attached to the model than to the family.

> **DEBATE**
> If all four formulations can be defended, in what sense is formulation a professional activity rather than an artistic one? Argue that the discipline lies in the testing, not in the content.

9.7 Annotated transcript: presenting a formulation to a family

A formulation is only useful when the family can hear it. Below is the presentation of the Okonkwo formulation, six turns in, with the therapist's choices annotated.

> **T1 Therapist:** I want to tell you what I think is happening, and then I want you to tell me where I have got it wrong. Is that all right?
> **T2 Mother:** Yes.
> **T3 Therapist:** What I see is this. Ebuka's difficult mornings started in the term after his grandmother died. She had looked after him every day for five years, and nobody has really talked about that in this room.
> **T4 Mother:** (quietly) No.
> **T5 Therapist:** And I think the mornings became the place where the sadness goes. He is difficult, you are tired, the school telephones, and there is no room left for missing her. Does that fit?
> **T6 Mother:** (crying) It fits.
> **T7 Therapist:** There is a second part, and this is about the two of you. When Ebuka is difficult, you handle it alone all week. On Saturday his father comes home and hears about it, and by then it has become a report.
> **T8 Father:** She does not tell me until it is a problem.
> **T9 Therapist:** And when she tells you, what happens?
> **T10 Father:** I say what she should have done. She says I am not there. Then we are arguing.
> **T11 Therapist:** So the boy's difficult mornings have become the way the two of you talk about the week. I am not saying anyone intends that. I am saying that this is how it works at the moment.
> **T12 Mother:** So what do we do?

Annotations. T1 asks permission and sets the correction clause before delivering anything. This is not politeness; it changes the status of what follows from a verdict to a proposal, and it is the single most reliable way to avoid the family experiencing formulation as judgement.

T3 begins with a date and a person, not a concept. It uses no clinical vocabulary. The phrase "nobody has really talked about that in this room" attributes the silence to the room rather than to any individual, which is both true and safe.

T5 is the formulation itself, in three clauses, ending with an invitation: "Does that fit?" The invitation is genuine. T6 is the moment the formulation is accepted, and note that it is accepted because it is about a loss and not because it is about behaviour.

T7–T11 present the second, harder part. The therapist uses "I am not saying anyone intends that" to pre-empt the shame that would follow an account of the parents' loop, and she uses the plainest possible description ("the way the two of you talk about the week"). The formulation is delivered not as an explanation of the family but as a description of a mechanism, which is what makes it acceptable.

T12 is the question the therapist wanted. The family asking "what do we do?" is the sign that the formulation has landed: it has become useful rather than merely true.

What the therapist did not say. She did not say "Ebuka is depressed" or "this is unresolved grief"; she did not use the words "enmeshed", "triangulated" or "parentified"; she did not present the formulation as complete; and she did not offer the plan in the same

breath as the formulation, because a family that is absorbing an explanation cannot also absorb a task. The plan came eight minutes later, after the father had said something about his own mother's death that he had not said before.

9.8 Laboratory: three formulations, one transcript

The week's assessed practical. Students work from a single thirty-minute video or transcript (the case bank provides three), in groups of three, with a strict division of labour.

Stage 1 (20 minutes) — three lenses. Each student writes a one-page formulation using a different lens (choose from structural, emotion-focused, narrative, behavioural, psychodynamic-systemic). All three must use the ten-part template in 9.4 and must include the testability clause.

Stage 2 (20 minutes) — the challenge. Each student presents for five minutes; the other two attack the formulation on three grounds only: is the maintenance account specific? does the plan follow from it? is the testability clause falsifiable? Nothing else may be criticised.

Stage 3 (20 minutes) — synthesis. The group produces one formulation that would satisfy all three critics, which usually means writing a formulation that names a primary mechanism, acknowledges a secondary one, and states what would change the therapist's mind.

Stage 4 (10 minutes) — the family version. Translate the synthesised formulation into 150 words that could be said aloud to the family, with no jargon and no blame.

Marking rubric.

Criterion	Weight	Full marks
Maintenance account	25%	A specific loop with three or more elements, and one attempted solution identified
Fit to evidence	20%	Every claim traceable to something observed or reported; speculation marked as such
Plan derived	20%	Each element of the plan names the formulation element it addresses
Testability	20%	A falsifiable clause with a review point and a stated alternative
Family version	15%	150 words, plain language, no jargon, correctable by the family

The commonest failure. A formulation that reads well and cannot be tested. The marker's question is always: "what would you expect to see in the next three weeks if this is right, and what if it is wrong?" Students who cannot answer have written an essay.

9.9 Six errors in formulation

1. **The elegant formulation.** A formulation so satisfying that the therapist stops collecting evidence. Fix: write the testability clause before the formulation, not after.

2. **The single-cause formulation.** "Everything is about his mother." Families are over-determined; a single cause is usually a single lens mistaken for the whole. Fix: name a primary mechanism and at least one secondary one.
3. **The formulation by adjective.** "The family is enmeshed and the father is peripheral." Adjectives cannot be tested or changed. Fix: convert every adjective into a sequence with at least two people in it.
4. **The plan without a mechanism.** A list of interventions — genogram, communication skills, a referral for the child — with no account of why these and not others. Fix: delete any element of the plan you cannot connect to the formulation.
5. **The formulation withheld from the family.** A rich internal document the family never hears, so the family is working with a different story from the therapist's. Fix: present it in plain language and invite correction.
6. **The formula that never changes.** A formulation written at assessment and unchanged at discharge. Fix: date it, review it, and record what changed and why.

9.10 Culture, faith and power in the formulation

Formulation is where a therapist's assumptions about families become invisible and consequential. Four disciplines.

Name the cultural frame you are using. A formulation that treats a three-generational household as "enmeshment", or a father's involvement of his own father in decisions as "diffuse boundaries", has imported a normative model of the nuclear family and applied it as a standard. The alternative is to ask what the family's own model is, and to formulate the difficulty relative to their model: "In this family, the eldest son's duty is to consult his father before a decision of this size. The difficulty is not the consultation; it is that his wife learned about the decision last."

Take the spiritual account seriously in the formulation. Where a family attributes the difficulty to spiritual causes, the formulation should record the attribution and its consequences — who in the family believes it, what it licenses, what treatment it produces, and what the family would accept as evidence of change. This is not an endorsement; it is accurate formulation. The one exception, as ever, is where the belief is endangering a child.

Formulate power and material conditions, not only relationships. A formulation that names "the couple's communication" as central when the family's actual difficulty is a husband's unemployment, an unpaid rent, or an immigration status that forbids work, has located the problem in the wrong domain — and has implicitly blamed the family for a structural condition. Ask directly: "What in your circumstances, rather than in your relationships, would have to change for this to get better?"

Watch your own position. The therapist is part of the system: a professional with authority, a cultural position, a class position, sometimes a gender and generational position that mirrors or inverts the family's. The formulation should include a line about the therapist's influence where it is relevant: "I noticed that I aligned with the mother early, and that the father's silence may be a response to that alignment."

> **AUDIT**
> Take your last written formulation and count the number of claims about the family that came from an institution rather than from the family: the school's letter, the referring doctor's note, a previous report. If more than a third of the formulation's claims are institutional, your formulation is about the family's file, not the family.

9.10a Formulating across language, and with an interpreter

A formulation built through an interpreter is a formulation at one remove, and the discipline required is greater than most training acknowledges. Five rules.

Brief the interpreter on the formulation, not only on the case. An interpreter who knows that the therapist is testing a hypothesis about a mother's standard of self-sufficiency will render the relevant words with the relevant weight. An interpreter who has been told only "the family is Nigerian" will render the words literally and lose the meaning.

Ask for the family's words, and then ask what they mean. Interpreters routinely smooth idioms into clinical English: a mother's "my heart is not settled" becomes "she reports anxiety", and the therapist loses the phrase that the family could have recognised. Ask the interpreter to give you the image, then the sense, then the clinical gloss.

Never formulate from a summary. If the interpreter summarises three minutes of a grandmother's speech into one sentence, you have the interpreter's formulation, not the family's account. Ask for the long version at least once per session, and note whose sentence you are using when the formulation later goes into the record.

Check the register, not only the content. Whether a father is addressed formally or familiarly, whether a mother is spoken to as a wife or as a daughter-in-law, whether a child is included in the conversation or discussed over — these are the relational facts the formulation needs, and they are usually lost in translation unless the interpreter is asked about them.

Consider who is interpreting. A child interpreting for a parent, a neighbour from the same community, or a member of the family's church all change what can be said. The professional standard is an independent interpreter, and where that is impossible, the limitation should be recorded in the formulation as a limitation of the evidence.

A final note on the therapist's own language. If you are working in a second language or a second dialect, your formulation will be built from fewer, cleaner, more formal exchanges than you would have in your first. Expect to under-hear humour, irony, proverbs and insult, and compensate by asking more directly: "When she said that, was she angry, or was she joking?" Families will tell you, and they will usually be pleased to be asked.

9.11 Evidence note: how strong is the evidence for formulation?

An honest evidence note, because formulation is the most widely taught and least empirically examined activity in the profession.

What is not established. There is no body of evidence showing that a formulation is accurate — that the mechanism a therapist names is the mechanism operating. Studies of clinical judgement consistently show modest reliability and systematic bias: clinicians weight vivid and recent information, they form early hypotheses and then seek confirming evidence, and they agree poorly with each other about the same case. Formulation, being a judgement task, inherits all of these limits.

What is weakly established. There is some evidence that formulations add predictive value beyond diagnosis in some settings, and that formulation-driven treatment selection outperforms unstructured selection in routine care. The evidence is thin and heterogeneous, and it does not support strong claims.

What is better established. Two adjacent findings give formulation its rational basis. First, routine outcome monitoring with feedback improves outcomes, particularly for clients who are not progressing; a formulation with a testability clause is a low-technology form of the same discipline. Second, collaborative goal setting and agreement on tasks predicts outcome; the family-level version of this is the negotiated formulation that the family recognises as its own.

What clinicians should therefore claim. That a formulation is a provisional, testable account which organises treatment, supports shared decision-making, and is revised in the light of feedback. Not that it is a diagnosis of the family's mechanism, and not that the model which generated it is thereby validated.

The professional implication. Write formulations with markers of uncertainty, review dates and alternative explanations, and be able to say in supervision: "I am working on the hypothesis that X maintains this; if the next three weeks do not confirm it, I will move to Y." That sentence is defensible in a court, a case conference or a journal club. "The family is enmeshed" is not.

9.12 The multidisciplinary network: formulating the system around the family

Families do not live in clinics. They live in a network of institutions — school, workplace, general practice, church or mosque, social services, housing, immigration, sometimes the courts — and that network is part of the maintaining system. A formulation that stops at the household boundary will routinely miss the loop that matters most.

Who is in the network. Map it explicitly, in writing, for every family where more than one agency is involved.

Node	What it contributes	What it can do that you cannot
School	Daily observation; attendance data; the child's peer world	Change the classroom, the timetable, the sanctions, the narrative staff hold
General practice	Medical history; medication; the family's route into services	Investigate physical causes; prescribe; write the letter that unlocks funding
Social services	Legal mandate; safeguarding authority; resources	Remove a child, or support the family to keep one; convene a conference
Religious community	Moral authority; practical support; the family's meaning system	Legitimise the family's engagement with therapy; provide respite

Node	What it contributes	What it can do that you cannot
Employer	Shift patterns; income; the father's absence	Change a rota; grant leave
Extended family	Care, money, arbitration	Succeed where a professional's authority fails

The three network loops that recur.

The referral loop. The school refers because the child's behaviour is unmanageable; the clinic assesses the child; the report goes back to the school; the school changes nothing because the report is addressed to the child's deficits; the behaviour continues; the school refers again. The formulation must include the school's response as part of the maintenance pattern, and the plan must include something the school can do differently.

The escalation loop. A mother does not tell the father about a school letter; the school escalates to a formal warning; the mother conceals the warning; the father discovers it in a letter and experiences the concealment as betrayal; the parents' conflict increases; the child's behaviour worsens; the school escalates again. The therapist who treats only the marriage will be beaten by the institution's timetable.

The mandate loop. A family attends because a court or a child protection plan requires it; the family's participation is compliant but not invested; the therapist interprets the lack of investment as resistance; the therapist becomes more directive; the family becomes more compliant and less honest; the report says the family engaged. The formulation must record the mandate as a feature of the therapeutic system, and the therapy must be designed for a mandated family (see Week 12).

Coordinating well: six practices.

1. **Get consent in writing, and be specific about who will be told what.** "I will tell the school about the plan and about your son's strengths. I will not tell them about your marriage."
2. **Hold one case conference rather than six telephone calls.** A single meeting with an agenda, a decision log and named actions is worth more than a month of coordination.
3. **Send the family the minutes, or read them aloud.** Families are routinely discussed by professionals about whom they know nothing, and this is both an ethical and a clinical error.
4. **Agree who holds the risk, and record it.** Confusion about mandate is the single most common cause of serious case failures.
5. **Do not confuse coordination with agreement.** Professionals can disagree productively if the disagreement is recorded and the family is not caught in the middle.
6. **Keep the family's own network in view.** The aunt, the pastor, the coach and the grandmother are often more influential than any agency, and they cost nothing to involve; a plan that ignores them will be quietly undone at home, usually within a fortnight and usually kindly, by people who were never asked and who would have helped gladly.

> **CASE**
> A 13-year-old is referred by his school for "aggressive behaviour". The clinic sees the family, finds a recent bereavement, and offers six sessions. Meanwhile the school has issued two exclusions, the mother has changed her telephone number to avoid the school's calls, and the father has been told nothing. The formulation that matters is not "unresolved grief"; it is "a school and a family escalating each other while a boy grieves", and the plan begins with a meeting at the school, not with a session in the clinic.

9.13 Writing it: the record, the report and the letter

Formulations circulate. They go into the clinical record, into reports for courts and schools, into referral letters and, sometimes, into the hands of the family. Three formats, three disciplines.

The clinical record. Write for three readers: your future self, a colleague covering your leave, and the family if they request their records. That means plain sentences, no abbreviations that only your team understands, and no speculation presented as fact. Use the phrase "my hypothesis" where it is one, and "the family reports" where that is the source. Date every formulation and note the review date.

The report for an external body. Courts, schools and local authorities want three things: what you were asked, what you found, and what you recommend. Put the formulation in one numbered paragraph in the findings, keep the theory out of the recommendations, and make each recommendation concrete, achievable and attributable to a named person. Avoid adjectives about the family ("chaotic", "enmeshed", "manipulative") — they will be quoted back at the family in a meeting you do not attend.

The letter to the family. Increasingly standard, and a useful discipline. A one-page letter after the assessment containing four things: what you understand the difficulty to be, in their words; what you think is keeping it going; what you propose; and what you would want to know if you were them. Write it as you would write to a colleague you respect. If you cannot, you have not understood the case well enough.

A worked paragraph, for a school. "I met with Ebuka and his mother three times between March and May. Ebuka is a nine-year-old who lost his grandmother in 2023; she had cared for him daily for five years. His difficulty in the mornings began in the term after her death. In my assessment the mornings are the place where the loss is expressed, and the school's letters, while understandable, have increased the pressure on his mother at exactly the point where she needs support. I recommend: (1) that a named member of staff meet Ebuka weekly for six weeks; (2) that attendance be reviewed fortnightly with his mother rather than by letter; (3) that I continue to see the family and report to the school with their consent in June."

What that paragraph does. It gives the school a formulation, a rationale, three actions, and a date. It does not use a single clinical term. It treats the school as a partner with a legitimate concern, and it locates the school's own behaviour in the pattern without blaming it.

9.14 Key terms

Term	Definition
Assessment	The systematic gathering of information about a family and its difficulty
Formulation	A provisional, testable explanation of the difficulty and what maintains it
Predisposing factor	A feature of history that makes the difficulty more likely
Precipitating factor	The event, and its meaning, that precedes the onset
Perpetuating factor	What maintains the difficulty now
Protective factor	A resource that reduces risk or supports change
Maintenance loop	The cycle of behaviour and meaning that keeps the problem in place
Testability clause	A statement of what would show the formulation to be wrong
Differential formulation	An alternative account that the evidence does not yet exclude
Network	The set of institutions and people involved with the family
Mandate	The authority under which the work is done, and by whom
Case conference	A multi-agency meeting that coordinates a plan

9.15 Self-test

1. What are the five jobs of a formulation?
2. Why must a formulation include a maintenance account?
3. Rewrite "predisposing factor" in relational terms.
4. What is a testability clause, and why does it matter?
5. Name two of the three network loops described in this week and explain how each can defeat a good formulation.
6. What are three errors in writing a formulation for an external body?
7. What does the evidence say about the accuracy of formulation?
8. What must a formulation contain where a family holds a spiritual explanation?

Answers. 1. Organise information; explain maintenance; predict; generate a plan; be revisable. 2. Because explaining onset does not tell you what to change now, and the maintenance pattern is usually the lever. 3. The family's relational history that makes this kind of difficulty more likely — patterns of managing stress, loss, authority and intimacy transmitted across generations. 4. A statement of what would show the formulation to be wrong, when it will be reviewed, and what would be done differently; it converts a belief into a hypothesis. 5. The referral loop (the institution's response becomes part of the problem and is omitted from the formulation) and the escalation loop (institutional deadlines outpace the therapy); the mandate loop is the third. 6. Adjectives about the family, theory inside the recommendations, and recommendations without named owners or dates. 7. Accuracy is not established; clinical judgement has modest reliability and known biases; the better-supported claim is that testable, monitored formulations support

treatment selection and feedback. 8. The attribution, who holds it, what it licenses, what the family would accept as evidence of change, and the limits imposed by safeguarding.

9.16 Reflective task

Write a one-page formulation of a family you have observed — from placement, a recorded session, or the case bank — and then write the testability clause. Now write a second paragraph answering: if I am wrong, what are the two most likely alternatives? Bring all three to the seminar. The exercise is uncomfortable for most students, because the professional training rewards confident explanation and this task rewards acknowledged uncertainty. That discomfort is the point, and it is worth naming in the seminar: the capacity to hold a hypothesis firmly enough to act on it and loosely enough to drop it is the whole of clinical judgement, and it is learned by practising the discomfort rather than by resolving it.

9.17 Readings for this week

- **Carr (2006)**, Family Therapy: Concepts, Process and Practice, the chapter on assessment and formulation, for the standard structures and their variants.
- **Johnstone & Dallos (2014)**, Formulation in Psychology and Psychotherapy, for the critical and comparative treatment; the chapters on systemic formulation are directly relevant.
- **Dallos & Vetere (2009)**, Systemic Therapy and Attachment Narratives, for an integrative approach to formulation across models.
- **Stratton et al. (2015)**, on the SCORE-15 and on idiographic measures — how to bring the family's own evaluation into the formulation.
- **A safeguarding review or serious case review** published in your jurisdiction, read for how formulations fail at the boundary between agencies.
- **A published court report or expert witness statement** in a family case, read for how formulation is written when it will be tested in public. Note how little theory appears, how much evidence, and how carefully every recommendation is tied to a named person and a date.
- **One serious case review and one successful family intervention report**, read side by side, for the difference between a network that shared information and one that shared only forms.

Next: Week 10 — Culture, class and power

Week 10 — Culture, class and power

Every family you meet is embedded in a culture, and so are you. Culture is not a variable to be "controlled for"; it is the medium in which the problem is defined. The same is true of class and of power, and the three are entangled: a family's account of a difficulty, the institution's account, and your account of both are all shaped by where each party stands.

10.1 The lecture in ninety minutes

Minutes	Segment	What the lecturer does	What students do
0–10	Three mistakes	Culturalism, colour-blindness, essentialism — named and illustrated	Identify each in a real referral
10–25	The cultural genogram	Draw one live, with the class's own material anonymised	Score their own draft genogram
25–40	Class and money in the room	Two cases, same symptom, different class position	Predict what each family will be offered
40–55	Power in the session	Who speaks, who is translated, who is interrupted	Count interruptions in a recorded session
55–70	Faith as the organising frame	Working with a pastor and an imam	Plan a coordination meeting
70–85	The limits of cultural respect	A case where culture is used to defend harm	Write the safety plan and the referral
85–90	Set the assignment	The cultural genogram and commentary	Plan the interview

10.2 Three mistakes

Most failures in cross-cultural clinical work are not failures of knowledge. They are failures of method, and they cluster into three recognisable errors.

Mistake 1 — culturalism. Treating culture as a set of rules that predict individual families: "Nigerian families are hierarchical", "Asian families do not discuss emotion", "in this culture the grandmother decides". Culturalism is attractive because it promises competence without inquiry, and it is harmful because it turns a hypothesis into a fact, relieves the therapist of asking, and makes the family a specimen of a category. It also flattens enormous within-group variation: two Ibadan families can differ more from each other than either differs from a family in Leeds.

Mistake 2 — colour-blindness. The opposite error: treating everybody as the same, refusing to notice that culture, ethnicity, class and religion shape both the difficulty and the encounter. The therapist who says "I do not see race" has usually failed to notice that the family does, and that the family's experience of institutions — schools, hospitals, police, social services — has taught them something about how they will be seen. Pretending not to see difference is a way of asking the family to do the work of managing the therapist's discomfort.

Mistake 3 — essentialism about the family's own account. Once a family has described their culture, the trap is to treat that description as fixed. Families describe their culture strategically, to the person in front of them, and for reasons: to claim authority, to protect a member, to explain a decision, to test whether the therapist will judge them. "In our culture, a wife does not contradict her husband" may be a description, an aspiration, a negotiating position, or a warning — and the therapist who takes it as a law will be unable to work with the wife who contradicts her husband in the second session.

The alternative to all three. A disciplined curiosity: ask, note, check, and revise. The formulation in Week 9 is the place where this discipline lives, because a formulation records both the family's account and the therapist's hypothesis about it, and distinguishes the two.

10.3 The cultural genogram

The genogram of Week 8 maps kinship. A cultural genogram maps the content carried through that kinship: language, religion, migration, education, occupation, class, and the values each generation transmits and contests.

The seven dimensions to map.

1. **Migration history.** Who came from where, when, why, and what was lost. For a family in Nigeria this may be internal migration (rural to city, state to state) as often as international.
2. **Languages.** Which language is spoken to whom, in which room, about what. The language in which a mother scolds is often not the language in which she consoles.
3. **Religion and religious change.** Who practises, who converted, and who disapproved. A grandmother's traditional practice alongside a granddaughter's Pentecostalism is a live cultural fact in many households.
4. **Education.** Who was educated, to what level, and what education means as a family achievement.
5. **Occupation and class.** Including the family's own class story — "we are not rich, but we are respectable" — and the class position of previous generations.
6. **Duty and individuation.** What a child owes a parent, and at what age; what a wife owes a husband's family; what is expected at a funeral, a wedding, a naming ceremony.
7. **Stigma and disclosure.** What may not be spoken about outside the family, and to whom: mental illness, debt, a divorce, a child born outside marriage, a prison sentence.

How to draw it. Use the genogram's structure, with annotations: a language written beside each person, a symbol for religious practice, a note for migration, arrows for conversion, a dotted line for a cut-off caused by a culturally significant event. Then ask three questions: What did each generation hand on that the next generation values? What did it hand on that the next generation rejects? Where is the disagreement now?

Why this matters clinically. Most family presentations that look like "communication problems" are, on closer inspection, disagreements between generations about duty, respect and autonomy — and those disagreements are cultural in the precise sense that they concern which values a family is going to live by. A genogram that maps the values makes the disagreement visible and, often, discussable for the first time.

PRACTICAL

Draw a cultural genogram of your own family across three generations, with the seven dimensions. Then write one sentence for each dimension beginning "In my family, this was never discussed". Those sentences are the material you will find hardest to ask about in a family that is not yours.

10.4 Class, money and material conditions

Therapy is a middle-class activity conducted in middle-class language, and families who are not middle-class are routinely misread by it. Five mechanisms.

The presentation is shaped by scarcity. A family under financial pressure will often present a behaviour problem and not a marital problem, because behaviour can be fixed with a rule and a marriage cannot be fixed without time, privacy and energy that the family does not have. The therapist who insists on the underlying relational issue may be right and may also be privileged.

Attendance is a class issue. A missed appointment is read as resistance; it is frequently a bus fare, a shift change, a childcare problem, or a job interview. Before interpreting a non-attendance, ask what made it hard to come.

Time is not equally available. Homework, daily diaries and "a conversation every evening" assume evenings. For a family with two jobs, shift work, or a crowded household, the intervention must fit the actual week, and the therapist should ask what the week looks like before prescribing anything.

Language registers communicate class. The therapist's vocabulary, the form, the waiting room, and the length of the sessions all tell the family where they are and who they are dealing with. Families respond accordingly, often with politeness that is not agreement.

Aspiration and shame. Many families are managing a gap between the life they were promised and the life they have, and between their own position and that of relatives who have done better. Questions about work, housing and school fees can touch shame very quickly, and a therapist who asks about money as though it were neutral will get answers that are shaped by the shame.

Three practical moves.

Ask about money directly and without embarrassment. "How are things financially at the moment?" is a clinical question, and families answer it more often than trainees expect.

Ask what the week actually looks like. Hours worked, who is at home when, who sleeps where. The intervention that fits the week will be followed; the one that assumes a different week will not.

Locate the problem in the conditions when the conditions are the problem. A father who is absent because he works six days a week in another state is not "disengaged"; he is absent, and the plan must work with the absence rather than interpret it.

10.5 Power in the room

The therapy room is not a democracy. The therapist has the authority of the institution, the language of the profession, and the power to write the record. The family has the knowledge and the choice of whether to return. Making this explicit, at least to yourself, is a precondition of ethical practice.

Five sites of power, and what to do about each.

Who defines the problem. Already discussed in Week 7, and it is a power question as well as a clinical one. The parent who telephones usually defines; the adolescent who is brought usually does not. Ask the person who has not spoken.

Who is interpreted. The member whose speech is summarised by others in the room — usually the youngest, the oldest, the least fluent, or the one whose first language differs from the therapist's. Notice who is spoken about rather than to, and address them directly.

Whose emotions are permitted. In many families, anger is permitted to the father and tears to the mother, and neither is permitted to the child. The therapist who mirrors the family's permissions will reproduce them. Ask the silent member what they would say if they were allowed to be angry.

Whose account is believed. Where two members give incompatible accounts, the therapist's attention will drift towards the more articulate, the more distressed or the more culturally familiar. Record both accounts, and say aloud that there are two.

Who is the expert. The professional's authority is real and should be used, not denied. The question is whether it is used to deliver a verdict or to structure a process in which the family's knowledge can operate. The difference is visible in the sentence "I think we should try X" versus "what would make X possible in your house?"

> **AUDIT**
> Record fifteen minutes of a role-played session. Count: how many questions were addressed to each member, how many times each member was interrupted, and whose words the therapist repeated. The pattern will not be random, and it will tell you more about your own cultural positioning than any reading.

Four practical disciplines.

1. **Ask rather than assume.** "In your family, who usually makes the big decisions? Who is consulted? Is that how you want it to be?"
2. **Locate yourself.** Complete the same cultural genogram for your own family. Your views on hierarchy, individuation, duty and disclosure are not neutral.
3. **Notice the power in the room** — including yours. The family may be polite because of your status, not because they agree.
4. **Interrogate "normal".** Models that prize individuation can pathologise families who prize interdependence. Ask what the family's own picture of a good life is.

The four are the minimum standard for this week, and they are easy to agree with and hard to practise: each requires the therapist to be uncertain in public, and professional training rewards the opposite.

10.5a Structural competency: when the problem is the institution

"Culturally sensitive" practice has a blind spot: it locates difference in the family and neglects the institutions that are shaping the family's problem. Structural competency is the discipline of noticing the second.

What it means. The capacity to recognise how housing, employment, immigration status, school admissions, policing, benefit rules and waiting lists produce the presentations that arrive in the clinic — and to act on that recognition rather than translating it into psychology.

Three questions that reveal structure in an ordinary session.

> "What would have to change outside this family for this to get better?" "Which of the things you have told me are decided by somebody else?" "When you have asked for help before, what happened?"

The answers are frequently decisive. A family whose evenings are unmanageable because four people share two rooms is not a family with a boundary problem. A father who is absent because his leave is refused is not disengaged. A mother who misses appointments because the clinic bus runs twice a day is not resistant.

The clinical consequence. Where the problem is structural, the intervention should include an institutional move: a letter, a telephone call, an advocate, a benefits check, a referral to a housing service, a school meeting. This is not social work instead of therapy; it is therapy that has formulated accurately. And it is often what the family experiences as most respectful, because it treats their account as true.

The limits. The therapist cannot fix a housing system, and promising to do so is a way of failing the family twice. The professional move is to be precise about what can be changed, honest about what cannot, and explicit in the formulation: "We cannot change the accommodation in the next three months. We can change how the evenings are organised within it, and I will write to the housing office with your consent."

10.6 Worked case: when duty and desire collide

A case that requires culture, class and power to be formulated together.

The referral. A 21-year-old woman, Tolu, brought by her parents to a family clinic. The parents' complaint: "she has refused the man we chose for her, and she wants to marry a boy from another tribe whom she met at university." Tolu's complaint, offered when asked directly: "they are selling me." The parents are Yoruba, from a modest household in Ibadan; the father is a retired teacher, the mother a trader. Tolu is the eldest of four and the first to attend university; the family has paid her fees with contributions from two uncles. The proposed husband is Ibo, a fellow student whose family the parents have never met.

What a culturalist reading would produce. "Arranged marriage is normative; the daughter's refusal is an acculturation conflict; she should be helped to respect her parents' values." This reading is respectful in tone and useless in practice: it takes the parents' description of the custom as the whole truth, ignores the daughter, and assumes a homogeneity that the family itself does not have.

What a power-blind reading would produce. "Forced marriage is a human rights violation; the daughter must be supported to leave." This reading is ethically serious and clinically naive: it treats a contested family process as a crime already committed, forecloses the parents' real authority and affection, and hands the family a choice between therapy and their standing.

The formulation that makes work possible. Tolu's refusal is not a rejection of the family but a disagreement about a decision the parents experience as theirs by right. The parents' position is not coercion in their own understanding: it is duty, it is protection (they believe a chosen husband from a known family is safer than a stranger), and it is standing (a marriage into a family they do not know is a public statement about their competence as parents). The uncles' contribution to Tolu's fees is a live obligation that nobody has named. The mother has reservations that she has not said in front of the father. Tolu has not explained what she wants, because she expects to be told she is ungrateful.

What the therapist does, in order.

First, hears all three accounts separately and without adjudicating. The parents, the daughter, and — critically — the mother alone. In the individual conversation the mother says that she was herself married to a man she had met twice, and that she has never said this to her daughters.

Second, names the difference between duty and coercion, in language that does not accuse. "Your parents are telling me what they owe you and what you owe them. You are telling me what you want. At the moment nobody in this room is talking about the same thing. Let us start with what each of you is protecting."

Third, refuses the false choice. The therapist says directly that she will not help to force a marriage and will not help a daughter to leave her family. Both statements are made to everyone. This is the ethical floor, and it is also strategically effective, because it makes the parents' continued attendance a choice rather than a defeat.

Fourth, finds the negotiable territory. Three things emerge: the parents' real concern is that the suitor's family is unknown, not that he is from another tribe; the uncles' obligation can be addressed by repayment over time; Tolu is willing to delay any marriage for a year and to have the suitor's family visit.

Fifth, sets a task that keeps everyone in the room. A meeting between the two families, facilitated by the therapist, with the father's consent and the mother's support; an agreement that no marriage will be arranged during the year; a plan for Tolu's fees.

What made it work, in system terms. The therapist took culture seriously without treating it as a law; she located the parents' position in duty and standing as well as in

custom; she brought in the uncles' obligation, which was material and unspoken; she found the mother's private dissent, which changed the family's internal balance; and she made the ethical limit explicit in a way that did not require anybody to be humiliated.

Where it could have failed. Had the parents been planning a marriage against Tolu's will within weeks, the plan would have required safeguarding and possibly legal advice, and the therapeutic timeline would have had to give way. Had Tolu's suitor been violent, the same. Culture is a legitimate frame for understanding, and it is never a reason to leave someone unsafe.

10.7 Annotated transcript: naming a cultural assumption in the room

An eight-turn excerpt from the Tolu case, session two, with both parents and Tolu present.

> **T1 Father:** In our culture a child does not choose. We chose for her because we know the family. She is bringing shame.
> **T2 Therapist:** When you say our culture, you mean the way it was done for you as well?
> **T3 Father:** Yes. I did not choose my wife. We have been married thirty-one years.
> **T4 Therapist:** Thirty-one years. And it worked.
> **T5 Father:** It worked.
> **T6 Therapist:** I want to understand what "worked" means for you, because I think that is the part your daughter has not heard.
> **T7 Father:** (pause) It means we did not divorce. It means my children grew up in one house with two parents. It means people respect us.
> **T8 Therapist:** (to Tolu) Did you know that was what he meant?

Annotations. T2 is the central move of the segment: it converts "our culture" from a general law into one man's biography, without contradicting him. The father's answer at T3 gives the therapist the material that matters — thirty-one years — and it is offered proudly, which means the therapist can work with it.

T4 is a simple acknowledgement, and it is doing something specific: it treats the father's marriage as a success rather than an arrangement to be critiqued. Trainees often cannot bring themselves to say this, because they disagree with the practice. Saying it costs nothing and buys the right to ask the next question.

T6 names a gap in the family's conversation rather than a gap in the father's reasoning. The phrase "the part your daughter has not heard" is addressed to the relationship, not to the father's position, and it is true: Tolu had never heard her father say what a marriage was for.

T7 is the father's own account of what he is protecting, in three clauses: continuity, respectability, and the avoidance of divorce. Any of these can be worked with. "Because it is our culture" cannot.

T8 turns to the person who has been spoken about rather than to, and it asks a question whose answer the therapist genuinely does not know. When Tolu said "no", the session changed course: she had not known, and the knowledge made her father's position human without making it right.

What the therapist avoided. She did not ask the father whether he had loved his wife; that question would have been intrusive and would have converted a cultural negotiation into a romantic one. She did not point out the inconsistency between "we respect our elders" and the uncles' money. And she did not tell Tolu that her father's position had merit in front of him, which would have made Tolu feel abandoned by the therapist; she said it to Tolu later, alone.

10.7a Working with a community or religious co-worker

Many families are best served by a therapist working alongside somebody from their own community: a pastor, an imam, a community health worker, a respected elder, a cultural broker. The arrangement is powerful and requires care.

What the co-worker brings. Trust, language, moral authority, knowledge of the family's standing, access to networks of practical support, and the ability to say things the therapist cannot say and be heard.

What the therapist brings. Confidentiality, a formulation, a method, the capacity to hold a boundary with an institution, and the professional accountability that the co-worker does not have.

Five conditions for the partnership to work.

1. **A written, shared understanding of confidentiality.** Who will know what, and what will not be repeated. This is the condition most often skipped and most often fatal.
2. **Clarity about roles in the room.** Who leads, who sits where, who speaks to whom. Families read these arrangements instantly, and a muddled arrangement undermines both authorities.
3. **Agreement on the safety floor.** The co-worker must know, before the first session, what the therapist is obliged to do if a child or a partner is unsafe, and must not contradict it in front of the family.
4. **A way to disagree privately.** The therapist and the co-worker will disagree — about a marriage, a discipline, a disclosure — and the disagreement must have a place to go that is not the session.
5. **A review.** Partnerships drift; a fifteen-minute review every four sessions catches the drift before the family does.

The risk to name. A co-worker who is a member of the same congregation or community cannot offer the family the confidentiality that a clinic can, and a family may prefer the clinic for exactly that reason. The therapist should not assume that community embedding is always preferable; ask the family which they want, and record the answer.

10.8 Faith as the organising frame

In much of the world, faith is not an "aspect" of family life; it is the frame in which family life is understood. Four practices for working with it well.

Ask what the faith says about the problem. Not in general, but in this family's version: is the difficulty a test, a punishment, a spiritual attack, a consequence of disobedience, or

an opportunity? The answer determines what treatment the family has already tried and what they will accept from you.

Ask who in the religious community knows, and who they want involved. A pastor or imam may be the family's most trusted authority, and may be entirely unaware of the difficulty, or may be deeply involved. Both are workable; not knowing is not.

Coordinate rather than compete. The failure mode is the therapist who implies that prayer is a substitute for change, or the religious leader who implies that therapy is a substitute for faith. The working position is complementary: "I will work on what happens in your house between Monday and Saturday; your pastor will do what he does; we will not contradict each other, and if we do, we will talk."

Attend to the cost of disclosure. A family that is prominent in a congregation may experience the difficulty as a threat to standing, and may therefore present late, minimise, or refuse a diagnosis. The formulation should record this, and the plan should protect the family's standing where doing so is compatible with safety.

> **CASE**
> A couple is referred after the wife disclosed to her pastor that her husband has been violent twice. She does not want the police involved and does not want her family told. The therapist must hold three things at once: respect for her autonomy and faith, a safety plan that does not depend on him changing, and honesty about the limits of confidentiality. (Full treatment in MFT 712 and 724.)

The principles, stated once. The woman's safety is not negotiable; her choices about disclosure are hers unless a child is at risk; the pastor's involvement is a resource if he is willing to be bound by the same safety principles; and the therapist's honesty about the limits of confidentiality is required before, not after, the disclosure.

10.9 Discrimination, tribe and the family's experience of institutions

Families arrive with a history of how institutions have treated people like them, and that history is part of the clinical picture.

Ask about it, briefly and directly. "Has anything like this happened before, with a school or a clinic? How was it for you?" The answers often explain a presentation that looks like hostility or withholding: a father who is curt with a professional may be managing a lifetime of being treated as a suspect.

Name what you notice, without claiming authority over it. If a family describes being treated badly because of their ethnicity, tribe, religion, accent or class, the therapist's job is to believe the account as an account, to explore its consequences, and not to adjudicate its accuracy. "That sounds as though it happened more than once" is more useful than "are you sure that was the reason?"

Watch your own reactions. Discomfort, over-compensation, curiosity about difference, and the wish to be seen as an ally are all common and all visible to families. The therapist

who works harder with a family because of their background is treating them differently, which is the problem in a different form.

Do not make the family responsible for educating you. Questions about culture are legitimate; a session spent interviewing a family about their traditions is an extraction. Use supervision and reading for general knowledge, and the session for this family.

Tribal and religious difference inside the family. In Nigeria, as elsewhere, the sharpest cultural conflicts are often internal: a marriage across ethnic lines, a conversion, a chieftaincy dispute, a family divided between denominations. The formulation should locate the conflict precisely, because "culture" as a single word conceals which culture, whose version, and against whom.

10.10 Language, naming and address

Language is the most immediate marker of power in a session, and it is where trainees make the most avoidable mistakes.

Names. Ask how each person wishes to be addressed, and use it. "Mrs Okonkwo" and "Chidi" and "Mummy" carry different relationships, and the therapist's choice will be read as a claim about the relationship. Where a family uses kin terms — "Aunty", "Baba", "Mummy" — for people who are not kin, follow the family's usage rather than correcting it; those terms are the map of obligation.

Address and formality. In many families the therapist's use of a first name for a father is a small presumption that will be noticed and not mentioned. Offer the choice: "I am happy to be called by my first name; what would you prefer?" The offer itself signals that the therapist knows the difference exists.

Whose language the session is conducted in. If the family thinks in their first language about emotional matters, the session may need that language or an interpreter. Interpreters are not furniture: brief them, seat them beside you rather than between you and the family, ask them to interpret speech rather than summaries, and never ask them to translate a disclosure you have not prepared for. (The discipline is set out in Week 9; the cultural point here is that language choice is itself an intervention — a family asked to speak about grief in a second language may be protected from the feeling, or prevented from reaching it.)

Proverbs, idioms and images. Many families speak in proverbs, and a proverb is a compressed theory of the problem. "A house that is not swept inside will be swept by the wind" is a statement about privacy, shame and outside interference. Ask what the proverb means, and use it later in the work; a formulation that can be expressed in the family's proverb is a formulation the family will remember.

Silence and indirectness. Some families regard direct questions about feelings, marriage or money as rude, and some regard indirectness as evasion. The therapist's task is to match the family's register while still obtaining the information, which usually means asking the question in a form that protects face: "Many families find this difficult to talk about; is it like that for you?" is a direct question with a face-saving first clause.

10.11 Gender, hierarchy and the limits of cultural respect

This is the hardest section of the week, and the one on which students most often look for a rule that does not exist.

The principle. Culture is a legitimate frame for understanding a family, and it is never a justification for harm. The professional's obligations are set by law and by the code of conduct, not by the family's account of custom. Where the two conflict, the obligation wins, and the task becomes how to act on the obligation while preserving as much of the family's dignity and engagement as possible.

Where the tension appears. Physical chastisement of children; child marriage; female genital cutting; a wife's economic dependence and her lack of authority over her own movement; the discipline of a daughter by senior relatives; the treatment of a child believed to be a witch; the refusal of medical treatment in favour of prayer; the concealment of a relative's mental illness to protect the family's standing.

What to do, in order.

First, know the law and the guidance in your jurisdiction, precisely. Not approximately. The therapist who believes wrongly that a practice is "cultural and therefore permitted" is a liability, and the therapist who reports without knowing the threshold is also a liability. This is examinable knowledge, and it belongs in supervision.

Second, distinguish the practice from the family's meaning. Most parents who hit their children are not cruel; they are frightened, unsupported, and following a model that was used on them. Naming the meaning does not excuse the practice; it makes the practice changeable.

Third, be transparent before acting. Where a report or a referral is required, say so, in advance, in plain language, and say what you are going to do and what will happen next. Families forgive a professional who told them the truth and acted on it; they do not forgive being reported in secret.

Fourth, do not abandon the family after the referral. The referral is one move in the work, not the end of it. Where it is legally possible, continue to see the family, and make the continuation conditional on the practice changing.

Fifth, get support. These cases are the ones that produce moral injury in clinicians. Take them to supervision, and do not carry them alone.

> **CAUTION**
> Do not use "we must respect the culture" as a way of managing your own discomfort about acting. A therapist who leaves a child in a harmful situation because intervention felt culturally intrusive has not been culturally sensitive; they have been silent.

And the mirror image. Do not use safeguarding as a way of avoiding cultural understanding: the family that is reported within twenty minutes of the first appointment, without the therapist having asked what the practice means, how the family understands it, or what alternatives exist, has been failed twice.

10.11a Worked decision: a discipline case at the threshold

The abstract principle becomes concrete in the case that trainees most often get wrong.

The presentation. A couple attends about their 11-year-old son's behaviour at school. In the second session the mother mentions, without emphasis, that the father "flogs him when he is rude, the way we were flogged", and that the school has noticed marks on his legs. The father is calm, affectionate towards the boy, and puzzled by the referral: "In this house we correct our children. That is not abuse."

What the therapist knows. Corporal punishment is lawful in some jurisdictions and unlawful in others, and the school has a safeguarding duty that will be exercised whether or not the therapist acts. The therapist's own code of conduct requires that a child's welfare is paramount, and that concerns are reported where the threshold in local guidance is met. The father's account is congruent with his own upbringing and with the practice of most families in his community. The mother is worried but loyal, and has said more than she intended to.

The wrong moves. The cultural deferral: "This is how the family does things; I will explore the meaning." This ignores the marks, the school's duty, and the law. The immediate report without conversation: defensible in some jurisdictions where the threshold is met by marks, but undertaken without telling the family, which ends the therapy and teaches the family to conceal. The psychological translation: treating the flogging as an expression of the father's own childhood and offering insight, while the child remains at risk this week.

The sequence that is both lawful and clinically sound.

One: screen and document, factually. What happens, with what, how often, what marks, when last. In the child's presence where appropriate, and separately where not. Record what was said, not what it means.

Two: state the position plainly, to both parents, in the first conversation. "I have to tell you two things. The marks on your son's legs mean I have to act under the safeguarding rules; I am going to speak to the school and to the safeguarding lead today, and I will tell you exactly what I say. And I want to keep working with you, because what happens in your house on a Tuesday evening is something we can change together."

Three: act, and tell the family what was said. Transparency before action, and a report that is factual and non-prosecutorial in tone, is both lawful and more likely to preserve the engagement.

Four: offer the alternative now, not later. Most parents who use corporal punishment do so because they have no second method that they believe in. Give one: a specific, culturally acceptable alternative to the specific incident — the removal of a privilege, a written rule agreed with the child, a cooling-off routine, the involvement of a respected relative — and rehearse it in the session.

Five: remain the family's therapist where it is possible. The family that is reported and then abandoned learns that services punish and disappear. The family that is reported,

told the truth, and then helped to change learns something more useful, and so does the child.

The professional's own support. Cases at the threshold generate anxiety, and anxiety generates either over-reporting or avoidance, and both are failures of the same kind: the therapist's discomfort has replaced the child's welfare as the organising concern. Take the decision to supervision before acting where there is time, and record the advice received. The decision is the therapist's, and so is the accountability; being able to show the reasoning, the advice taken and the family's own account of what was said is the difference between a defensible decision and an unexplained one.

The generalisation. Almost every cultural conflict in clinical work has this shape: a practice the family understands as care, a professional obligation, a child or partner who is not in a position to consent, and a therapist who must act without humiliating the people they are trying to help. There is no formula, but there is a sequence, and the sequence is: know the threshold, document the facts, tell the family the truth, act, stay.

10.12 Migration and the acculturation gap

Migration produces a predictable family dynamic, and it is worth teaching explicitly because it appears in the referral as "conflict between parents and adolescent".

The gap. Parents and children acculturate at different rates. Children acquire the new culture quickly, through school and peers, and often become the family's interpreter, negotiator and public face. Parents acquire it slowly, and often deliberately preserve the values of home as an act of continuity and self-respect. The result is a family in which the adolescent knows the new rules better than the parents and is still expected to obey the old ones.

The pattern in the clinic. The young person is referred (or brought) for defiance, low mood, self-harm or school refusal. The parents describe a child who has "become different". The young person describes parents who "do not understand anything". The therapist who takes either side loses the other, and the family's actual difficulty — that the rules are being negotiated in a household where the two generations have different maps — remains unformulated.

What helps.

Make the gap the client. "In this house there are two sets of rules, and nobody has decided which applies to what. That is not a failure in any of you; it is what happens when a family moves." Externalising the conflict this way converts a contest about loyalty into a shared problem, which is the only version that a family can solve.

Separate the domain from the value. Parents are usually willing to negotiate domains — bedtime, dress, money, going out — while defending values — respect, chastity, family duty, education. The therapist's task is to find the domain trade: what the young person may decide, and what remains a family matter. Negotiating values directly usually fails, because it looks like asking the parents to stop being who they are.

Address the parent's own loss. Migration involves losses that are rarely named: status, language competence, friendship networks, the extended family that would have shared the parenting. Parents who are grieving a lost world are less flexible than parents who are not, and the grief is workable when it is named.

Attend to the children left behind and the children who came later. Sibling groups that were separated by migration have their own history: the child who was raised by a grandmother and rejoined the family at eleven has a different relationship to the parents than the sibling born after migration. This is often the buried structure of the presenting problem, and the genogram of Week 8 shows it in ninety seconds.

10.12a Two migration cases, worked briefly

Case 1 — the reunited family. Parents migrated when their son was two; he was raised by his grandmother and joined them at fifteen. The referral is "defiance and stealing". The formulation is not delinquency: it is a family in which the person who raised the child is absent, the parents are strangers with authority, and the child has lost his language, his school standing and the grandmother in a single move. The intervention begins with the boy's account of the grandmother, a telephone arrangement that keeps her in his life, an explicit negotiation of what these two adults are entitled to ask of him and how quickly, and a plan with the school for the first term. The stealing, it emerges, began when he had no money and did not know how to ask.

Case 2 — the second generation. A 17-year-old girl, born in the city to parents who migrated before her birth, referred for self-harm after her parents discovered a relationship. The parents' account is of a daughter who has betrayed the family; the daughter's is of a family that watches her. The formulation holds both: the parents are protecting a daughter's honour in a community where the family's standing is their children's reputation, and the daughter is managing two sets of rules with no adult who can help her hold both. The intervention includes separate sessions, a negotiated agreement about what the parents need to know and what they do not, a risk plan, and — where it can be found — a trusted adult from the family's own community who can hold the parents' anxiety without endorsing surveillance.

What the two cases have in common. In both, the "culture" is only half the story; the other half is the practical conditions of migration — housing, school, work, money, status — and the family's grief for a world it has lost. A formulation that names only values will miss the conditions, and a plan that addresses only conditions will miss the values.

10.13 Evidence note: does cultural adaptation improve outcomes?

A short and useful evidence note, because "culturally adapted" is a phrase used loosely.

What the research shows. Meta-analyses of culturally adapted interventions — mostly in individual therapy for adults — find modest but consistent benefits of adaptation over unadapted interventions, with larger effects where the adaptation is deep (engaging the client's cultural values and world view) rather than superficial (translating materials and matching the therapist's ethnicity). The strongest evidence is for language matching and

for adaptations that address engagement and retention rather than for adaptations of specific techniques.

What is not established. That matching therapist and family ethnicity improves outcomes by itself; the evidence is mixed, and the mechanism appears to be the family's expectation of being understood rather than the match itself. Nor is it established that an unadapted, evidence-based model fails with families from other cultures; the more common finding is that it works less well, for reasons largely to do with engagement.

What this means for practice. Adapt the engagement deliberately — language, explanation, who is in the room, where the sessions happen, how the family's authority structure is respected — and adapt the content cautiously, because a technique that has been changed beyond recognition is no longer the intervention that was tested. Where you adapt substantially, say so in the record and in supervision, and monitor the outcome.

The professional's obligation. Families from minority or marginalised groups are more likely to receive poorer care, to be diagnosed later, to be offered less evidence-based treatment, and to disengage early. This is a documented pattern across health systems, and the therapist's response to it is not guilt but method: measure outcomes, attend to engagement, and use supervision to examine the ways in which the service is failing this family.

10.14 Six errors

1. **The cultural shortcut.** Explaining the family's behaviour by their background before hearing their account. Fix: ask first, and use culture to generate questions rather than to close them.
2. **The neutral therapist.** Refusing to notice difference, and thereby obliging the family to accommodate the therapist's comfort. Fix: name difference when it is relevant, briefly and without ceremony.
3. **The extraction session.** A session spent asking the family to educate the therapist about their traditions. Fix: general knowledge from reading and supervision; the session for this family's particulars.
4. **The cultural defence of harm.** Deferring to custom where a child or a partner is unsafe. Fix: know the law, name the concern, act transparently, and stay involved.
5. **The over-adaptation.** Rewriting an evidence-based model until it no longer resembles the intervention that was tested, and calling the result culturally sensitive. Fix: adapt engagement deliberately; keep the mechanism; record and monitor what you changed.
6. **The forgotten class.** Formulating the family's difficulty in relational terms when the difficulty is money, housing, immigration status or work. Fix: ask about the material conditions first, and let the family tell you which domain the problem belongs to.

10.15 Key terms

Term	Definition
Culturalism	Treating culture as a set of rules that predict individual families

Term	Definition
Colour-blindness	Refusing to notice cultural, ethnic or class difference and its effects
Cultural genogram	A genogram annotated with language, religion, migration, education, class, duty and stigma
Acculturation gap	The difference in rate of cultural adaptation between parents and children
Interpreter (professional)	A trained interpreter bound by confidentiality, distinct from a family member or broker
Cultural broker	A person who mediates between a family and an institution, often informally
Face-saving question	A question phrased so that the answer does not require the respondent to lose standing
Microaggression	A brief, commonplace exchange that communicates a demeaning message to a marginalised person
Structural competency	The capacity to recognise how institutions and material conditions shape clinical presentations
Cultural adaptation	The systematic modification of an intervention's engagement or content for a cultural group

10.16 Self-test

1. Name the three mistakes described in this week and give an example of each.
2. What are the seven dimensions of a cultural genogram?
3. Give three mechanisms by which class shapes what a family presents.
4. Name three sites of power in a therapy session.
5. What is the professional's position when a family's custom conflicts with the law?
6. What does the evidence say about therapist–client ethnic matching?
7. What is the acculturation gap, and why does it matter clinically?
8. What are two ways a therapist can adapt engagement without distorting the model?

Answers. 1. Culturalism ("Nigerian families are hierarchical"), colour-blindness ("I do not see race"), and essentialism about the family's own account ("in our culture a wife does not contradict her husband"). 2. Migration history; languages; religion and religious change; education; occupation and class; duty and individuation; stigma and disclosure. 3. Scarcity shapes the presenting complaint; attendance is affected by cost and shifts; time is not equally available, so homework and evening conversations may be impossible. 4. Who defines the problem; who is interpreted or spoken about; whose emotions are permitted; whose account is believed; who is treated as the expert.

1. The legal and professional obligation prevails; act transparently, tell the family in advance what you are doing,

and remain involved where possible. 6. The evidence for matching alone is mixed; the apparent mechanism is the expectation of being understood, and language matching has stronger support. 7. Parents and children adapt to a new culture at different rates,

producing a family in which the young person knows the new rules better than the parents while still being expected to obey the old ones; it is a common source of presentations labelled defiance. 8. Matching language and explanation; changing who is in the room and where sessions happen; adjusting the framing to the family's values — while keeping the mechanism of the intervention intact.

10.17 Reflective task

Where in your own family would a therapist from a different culture misread the structure? Write 300 words on what they would get wrong and why. Then write 100 words on what you would want them to ask you, and in what form. Bring both to the seminar; the second paragraph is usually the more useful one, because it is the question you will need to ask a family that is not yours, and in the form in which you would want to be asked it yourself, by somebody who had time to listen to the answer.

10.18 Readings for this week

- **McGoldrick, Giordano & Garcia-Preto (2005)**, Ethnicity and Family Therapy, 3rd edition — read one chapter on a group you work with, and one on a group you do not, to see the difference between description and stereotype.
- **Hays (2016)**, Addressing Cultural Complexities in Practice, for the ADDRESSING framework and its clinical use.
- **Falicov (2014)**, Latino Families in Therapy — the best single account of a culturally grounded family therapy method, useful well beyond its stated population.
- **Boyd-Franklin (2003)**, Black Families in Therapy, for the integration of race, class and clinical method.
- **Griner & Smith (2006)** and later meta-analyses of cultural adaptation, read for the size and character of the effect.
- **Your national code of conduct and safeguarding guidance**, read this week rather than next, because this is the week in which the conflicts become concrete. Mark the three paragraphs you would have to quote aloud to a family, and practise saying them.

Next: Week 11 — What the evidence says

Week 11 — What the evidence says

Students often arrive believing either that therapy "just works" or that only manualised treatments do. The evidence is more interesting than either position, and a clinician who cannot read it is dependent on whoever is willing to summarise it for them.

11.1 The lecture in ninety minutes

Minutes	Segment	What the lecturer does	What students do
0–10	Two bad positions	"It just works" and "only manuals work", stated and dismantled	Identify which they hold

Minutes	Segment	What the lecturer does	What students do
10–25	Designs and what they can answer	RCT, cohort, case series, qualitative, synthesis	Match six questions to six designs
25–40	Effect sizes in plain language	d, NNT, percentages, translated into families	Convert four results into "out of ten families"
40–60	The trial map	Eating disorders, conduct, substance use, psychosis, couple distress	Place each finding on a strength-of-evidence grid
60–75	Reading a paper in twenty minutes	Live walkthrough of a real abstract and results table	Apply the six questions in pairs
75–85	Feedback and outcome monitoring	The evidence for measuring	Design a session-by-session measure
85–90	Set the assignment	The critical appraisal	Choose a paper

11.2 Why a clinician needs research literacy

Four reasons, each of which will appear in an examination.

Because the family will ask. "Does this work?" is the commonest question a family asks in the first session, and an answer of "yes, therapy helps" is not an answer. The clinician needs to know what works for what, with what effect, and for whom, in order to give an honest answer that includes uncertainty.

Because services are commissioned on evidence. Funding, waiting lists and staffing follow evidence of effectiveness. A profession that cannot read the evidence cannot argue for its own resources, and family therapy has spent decades arguing from conviction rather than from data at exactly the wrong moments.

Because the alternative is fashion. Without research literacy, practice follows the last conference, the most charismatic trainer, or the model a service happened to invest in. The result is a profession in which whole counties practise one way and whole counties another, with no one able to say which is better.

Because clients are harmed by ineffective treatment. The most important finding in psychotherapy research over the last thirty years is not that therapy works; it is that some clients deteriorate in treatment, that therapists often fail to notice, and that routine feedback reduces the harm. A clinician who does not measure cannot know whether they are helping the family in front of them.

11.2a How evidence gets into a service

A student who understands the evidence but not how services change will spend a career frustrated. Four mechanisms, and the clinician's place in each.

Commissioning. Services are funded against specifications that cite guidelines, waiting times and outcome targets. A clinician who can read a specification and contribute outcome data is in a position to influence what is commissioned; a clinician who cannot is dependent on others' descriptions of what works. This is why routine measurement matters beyond the individual case: it is the profession's evidence in negotiations.

Fidelity and adaptation. Evidence-based models are usually tested with training, supervision and monitoring of fidelity — the degree to which the model is delivered as designed. Services often adopt the name of a model without the training and supervision, and then report that the model does not work. The clinician's question about any new service should be: what training, what supervision, and how will fidelity be monitored?

Scale-up. Programmes that work in a research trial frequently produce smaller effects when delivered at scale: the therapists are less selected, the supervision is thinner, the population is broader, and the organisational support is weaker. Multisystemic therapy's large-scale rollouts are the standard cautionary example. This is not an argument against evidence-based practice; it is an argument for measuring outcomes continuously, in the service as delivered.

The professional's position. The clinician is not a passive recipient of policy. Three things are within reach of almost everyone: measure your own outcomes; bring them to team meetings and supervision; and learn to write a one-page summary that a manager can use. The alternative — waiting for the research to be translated by somebody else — is what has produced the gap between what is known and what is done.

11.3 Designs, and the questions they can answer

A design is not "better" or "worse" in the abstract; it is better or worse for a question. Six questions and the designs that answer them.

Question 1: does it work, compared with what would otherwise happen? Answer: a randomised controlled trial (RCT). Families are allocated at random to the treatment or to a comparison (waiting list, treatment as usual, another treatment). Randomisation is what allows causal claims, because it distributes unknown confounders across groups. The trial answers the question for the population and the conditions studied, and no further.

Question 2: does it work in ordinary services, with ordinary families? Answer: a pragmatic trial, or a large practice-based cohort. Efficacy (can it work under ideal conditions?) and effectiveness (does it work in the clinic on a Tuesday?) are different questions, and the gap between them is one of the recurring disappointments of the field.

Question 3: what happens over years? Answer: a cohort or a long-term follow-up of a trial sample. Attrition is the enemy; a five-year follow-up that retains 40% of the sample tells you about the 40%.

Question 4: how does it work? Answer: a mediation analysis, a process study, or a qualitative study. Mechanisms are examined by measuring the hypothesised mediator and testing whether change in it precedes change in outcome. Most family therapy models have weak mechanism evidence relative to their outcome evidence.

Question 5: what is it like to receive it? Answer: qualitative methods — interviews, focus groups, thematic or narrative analysis. These cannot tell you whether a treatment works, and they are the only way to find out why families disengage, what the experience of being assessed feels like, or how a model is actually delivered.

Question 6: what does the whole body of evidence say? Answer: a systematic review with meta-analysis, or a guideline. The quality of the answer depends on the quality of the included studies, on publication bias, and on whether the reviewers looked for the studies that did not work.

The hierarchy, stated honestly. Systematic reviews of well-conducted RCTs sit at the top for questions of effect; qualitative studies sit at the top for questions of experience; a single case series is the best available evidence for a rare presentation in your clinic. Any hierarchy that ranks designs without reference to the question is a hierarchy of prestige, not of knowledge.

11.4 Effect sizes in plain language

The most useful skill in this week is converting a statistic into a sentence a family could understand.

What d means. Cohen's d expresses the difference between two groups in standard deviation units. Roughly: d = 0.2 is small, d = 0.5 moderate, d = 0.8 large. In practice, for psychotherapy, meta-analytic effects for depression and anxiety cluster around 0.5 to 0.8; couple therapy for relationship distress shows effects in the moderate-to-large range; family-based treatment for adolescent anorexia is associated with remission rates substantially higher than comparison treatments at end of treatment and follow-up.

Converting d to something a family can picture. A d of 0.5 means that the average treated family is better off than about 69% of untreated families. In a group of ten comparable families, roughly seven would do better with treatment than the average family without it. That is a meaningful benefit and it is not a cure, and saying both is the professional's obligation.

What NNT means. The number needed to treat is the number of families who must receive the treatment for one additional good outcome. An NNT of 5 means that for every five families treated, one does better than they would have done otherwise; the other four either would have improved anyway or do not benefit. Families find this framing honest and clinicians find it uncomfortable, which is a sign it is worth using.

What p-values do not tell you. A p-value is the probability of the data, or more extreme data, if the null hypothesis were true. It says nothing about the size of the effect, nothing about whether the effect matters, and nothing about the individual family. A trial with 4,000 families will produce $p < 0.001$ for an effect too small to notice, and a trial with 30 families will produce $p = 0.4$ for an effect that transforms lives. Read the effect size and the confidence interval first.

Confidence intervals, in one sentence. The 95% confidence interval is the range of effects compatible with the data; a wide interval means the study cannot tell you much, and an interval that crosses zero means the study cannot exclude no effect.

> **EXERCISE**
> Take any four results from this week's reading, and write for each: one sentence a family could understand, one sentence a commissioner could understand, and one sentence of honest limitation. The third is the one that marks you as a professional.

11.4a Reading a forest plot

The forest plot is the signature graphic of a meta-analysis and the quickest way to see a body of evidence at once.

What is on the plot. Each study is a horizontal line: the point estimate of its effect is a square (sized by the study's weight), and the line through it is the confidence interval. The vertical line at the centre is "no effect". A pooled diamond at the bottom summarises the overall estimate.

How to read it in four looks.

Look 1 — where do the squares sit? If most squares are on one side of the no-effect line, the body of evidence points one way. If they are scattered across it, the evidence is inconsistent, and the pooled estimate conceals the inconsistency.

Look 2 — how long are the lines? Long lines mean small studies with imprecise estimates. A plot dominated by long lines cannot support a confident conclusion, however impressive the diamond.

Look 3 — how wide is the diamond? The diamond is the pooled confidence interval. Narrow means precise; wide means the meta-analysis cannot tell you much, usually because there are few studies or they disagree.

Look 4 — is there an asymmetry? If small studies cluster on the positive side and large studies sit nearer the no-effect line, suspect publication bias. Funnel-plot asymmetry is the formal version of the same observation.

What the plot cannot show. Whether the included studies measured what you care about; whether the interventions were delivered as designed; whether the participants resemble your clinic; and whether the authors selected which studies to include. All four are in the text, and all four matter more than the diamond.

11.5 The map of the evidence

A brief, honest map of what family and couple therapy has, organised by presentation. Students should learn the shape of this map and the two or three key references for each entry, not memorise numbers.

Relationship distress in couples. The strongest evidence base in the field. Behavioural couple therapy, cognitive-behavioural couple therapy and emotionally focused therapy all show moderate-to-large effects on relationship satisfaction in randomised trials, with a substantial minority of couples deteriorating or not responding, and with relapse a real phenomenon over one to two years. The comparative evidence between bona fide couple therapies is weak; the alliance and the couple's readiness matter a great deal.

Adolescent behaviour problems, including conduct disorder. Family-based approaches — parent management training, functional family therapy, multidimensional family therapy, multisystemic therapy — have good evidence for reducing behaviour problems, with effect sizes in the moderate range, and with the strongest evidence where the intervention is intensive and delivered with fidelity. The evidence is weaker for adolescents with callous-unemotional traits and for those whose difficulties are embedded in severe neighbourhood disadvantage, where effects have been disappointing in large-scale rollouts.

Adolescent substance misuse. Multidimensional family therapy, functional family therapy and brief strategic family therapy have trial evidence for reducing use, with effects that are typically modest and often not maintained without continued support. The comparison with individual treatment is favourable; the comparison with doing nothing is more favourable still.

Eating disorders. Family-based treatment (the Maudsley approach) for adolescent anorexia nervosa is a first-line, guideline-endorsed intervention, with evidence comparable or superior to individual treatment at end of treatment and at follow-up. For bulimia, family involvement is a component of several evidence-based approaches. This is the clearest example in the field of a specific family intervention for a specific presentation, and it earns its place in the curriculum.

Psychosis. Family interventions combining psychoeducation, communication training and problem-solving reduce relapse rates in schizophrenia when added to medication, with a number needed to treat in the range of five to seven families for one relapse prevented over nine to twelve months. The evidence is old, replicated, and under-implemented, which is itself a lesson about services.

Depression. Couple therapy is an effective treatment for depression where the relationship is distressed, with effects comparable to individual therapy in several trials, and with the additional benefit of improving the relationship. Behavioural activation delivered to a couple is a legitimate and well-evidenced option.

Child and adolescent anxiety and depression. Family involvement in cognitive-behavioural treatment improves outcomes modestly; systemic family therapy has a smaller but real evidence base; the strongest evidence in this area concerns parental accommodation of anxiety, which is a family-level mechanism that can be addressed directly.

Physical health and long-term conditions. Family interventions improve adherence and outcomes in diabetes, asthma and some chronic pain conditions, with small-to-moderate effects. This is an area of growing importance in services with integrated care.

What the map teaches. Family therapy is not one treatment with one evidence base; it is a family of methods with strong evidence in some presentations, moderate in others, and thin in the areas where the profession most wants recognition (for example, complex trauma in adults, where individual and phase-based approaches dominate). Honesty about the thin areas is part of competence.

Four findings worth carrying into practice.

1. **Family and couple therapies work.** Meta-analyses consistently find moderate-to-large effects for couple therapy in relationship distress, and for family-based treatments in adolescent behaviour problems, eating disorders (family-based treatment for anorexia is a first-line intervention), substance misuse and psychosis relapse.
2. **Alliance matters, and it is relational.** In couple and family work, the alliance is not one relationship but several, and shared sense of purpose predicts outcome better than any single dyad's alliance with the therapist.
3. **Model differences are smaller than model loyalties suggest.** Common-factors research attributes much of the variance to client factors, alliance and expectancy. Technique is not irrelevant: it organises what the therapist notices and does, and it matters more for specific problems.
4. **Drop-out is the silent failure.** In couple and family work, 20–40% of families end therapy unilaterally. Attendance, engagement and early change are the best predictors of retention. Track them.

11.5a Two studies that changed practice, described properly

Learning the detail of two landmark studies does more for research literacy than learning the names of twenty.

Study 1 — family therapy for adolescent anorexia nervosa (Russell and colleagues, 1987). Eighty patients with anorexia nervosa were randomised to family therapy or to individual supportive therapy after admission and refeeding. Both treatments were delivered over about a year, with family therapy beginning as a single family session and developing into conjoint family work focused on the parents taking charge of the patient's eating. The central finding was an interaction: in patients whose illness had begun before the age of 19 and whose duration was under three years, family therapy produced substantially better outcomes at one year; in patients with a longer illness, individual therapy did at least as well. The study was small, conducted in one centre by a committed team, and used a comparison treatment that critics argue was not designed as an active competitor. It also changed the treatment of adolescent anorexia nervosa permanently, and later trials and replications supported its central finding. It is a model of a study that is simultaneously underpowered by modern standards and enormously influential, because its finding was specific, plausible and clinically decisive.

Study 2 — family intervention for schizophrenia relapse (Leff and colleagues, 1982, and the subsequent replications). Patients with schizophrenia living in households characterised by high expressed emotion — criticism, hostility and emotional over-involvement — have substantially higher relapse rates than patients in low-EE households. Leff's trial randomised patients discharged to high-EE homes to a family intervention (education, a relatives' group, and sessions at home) or to routine care. Relapse over nine months was markedly lower in the intervention group, and the finding was replicated across several countries with a consistent effect on relapse. The number

needed to treat is in the range of five to seven families for one relapse prevented. What makes this study instructive is the mechanism: the intervention did not change the illness, it changed the emotional environment in which the illness was managed. It is also instructive in another way: the intervention was never widely implemented, for reasons that have nothing to do with evidence and everything to do with service design, professional boundaries and the difficulty of working with families in acute psychiatric settings.

What the two studies teach.

First, the interaction matters. The anorexia trial's finding is conditional on age and duration of illness; a clinician who applies the result to a 35-year-old with a twelve-year history is misreading the evidence.

Second, mechanism is not optional. The expressed-emotion work identified a modifiable family process and changed it; that is a stronger scientific story than a model-level claim that family therapy works.

Third, implementation is a separate problem. A treatment can be well evidenced and unused. The gap between evidence and practice is the subject of the rest of a professional training, and it is not closed by more evidence.

11.6 Reading a paper in twenty minutes

The method, taught as a sequence, so that it can be done under time pressure in a clinic.

Minutes 1–3: the abstract, backwards. Read the conclusion first, then the results, then the method. Abstracts are written to persuade; reading backwards makes the persuasion easier to see. Ask: what did they find, and what did they do to find it?

Minutes 3–5: the design paragraph. Who was recruited, from where, with what inclusion and exclusion criteria, and how many dropped out. The exclusion criteria tell you who was not studied, which is how you decide whether the sample resembles your clinic. A trial that excluded families with active substance misuse, no fixed address, or a child under six cannot tell you about the families you most often see.

Minutes 5–8: the comparison. What was the treatment compared with? Waiting list comparisons inflate effects; treatment-as-usual comparisons vary enormously depending on what usual treatment is; head-to-head comparisons between active treatments are the most informative and the rarest.

Minutes 8–12: the outcome measure. Does the measure capture what the family cares about? Who completed it — the parent, the child, the therapist, or an independent assessor? Blinded assessors matter, and in family therapy they are hard to maintain, because families mention their treatment.

Minutes 12–15: the numbers. Effect size, confidence interval, the proportion who improved, the proportion who deteriorated, and the number who dropped out. The last two are routinely omitted from abstracts and are often the most clinically important.

Minutes 15–18: the limitations. Read the authors' own limitations section, then add the two they did not mention: publication bias (are we reading this because it worked?) and allegiance (do the authors have a stake in the model they tested?).

Minutes 18–20: the verdict. Write three sentences: this study shows X, with these limits; I would use it when Y; it would not apply to Z. If you cannot write the third sentence, you have not read the paper — you have read its abstract.

The six questions, condensed. Who? Compared with what? Measured how? How big? How sure? Does it fit my clinic?

11.6a Where to find the evidence quickly

A clinician does not need to read the primary literature every week; they need to know where the good summaries are and how to check them.

Source	What it is good for	Watch for
National guidelines (NICE, SIGN, APA, and local equivalents)	The current standard of care for a defined presentation	Lag; local applicability; the guideline's scope
Cochrane and Campbell reviews	Questions of effect, with explicit methods	Reviews can be out of date; check the date
Professional association guidance (for example, family therapy associations)	Competence frameworks and model-specific summaries	Allegiance
Systematic review databases and evidence maps	Finding what exists before reading anything	Coverage of non-English and non-Western literature
A good textbook, current edition	The shape of a field in a week	Lag of three to five years
A trusted colleague with a special interest	Orientation and a shortlist	The same biases as yours
The family in front of you	Whether any of it applies	Small n, and no comparison group

The habit. For each presentation you meet regularly, keep a one-page summary: the recommended first-line approach, the key trial or review, the main alternative, the main uncertainty, and the reference. Update it once a year. Five of these pages will cover most of a general family therapy caseload, and they are worth more than a shelf of unread journals.

11.7 Annotated abstract: reading between the lines

The following is a composite abstract constructed for teaching, containing the features students most often miss.

> "In a randomised controlled trial, 120 families with a child aged 11–15 referred for behavioural difficulties were allocated to family therapy (n = 60) or a waiting list (n = 60). The primary outcome was the child's behaviour score at 12 weeks, rated by parents. Family therapy was superior (mean difference 4.2 points, 95% CI 1.1 to 7.3, d = 0.42). Secondary outcomes included attendance at school. Twenty-two per cent of the family therapy group did not complete the intervention."

What the abstract tells you. There was a trial; it was randomised; the comparison was a waiting list; the outcome was parent-rated; the effect was modest; a fifth of the treated families did not complete the treatment.

What it does not tell you. Who rated the child (the parents, who are not blind to treatment). What happened to the waiting-list group afterwards (did they get the treatment, and when?). What "family therapy" consisted of (model, dose, fidelity). What happened to the school attendance outcome (it appears in the secondary list and then disappears from the abstract, which usually means it did not reach significance). Whether the improvement was maintained beyond twelve weeks.

What the numbers mean, in plain terms. A d of 0.42 means the average treated child was better off than about 66% of untreated children on the parent-rated measure. The confidence interval excludes zero, so the effect is unlikely to be chance, and it is small enough that six or seven families in ten would not have shown the benefit. The 22% dropout means that one family in five did not get the full dose, which is entirely typical and is the finding a service should act on.

The question to ask the authors. "What happened to the families who dropped out, and how does the effect look if you analyse them as treatment failures?" The answer is often in the paper's supplementary tables, and it is the answer that matters for a clinic.

11.7a Appraising a qualitative study

Qualitative research answers questions that trials cannot, and it has its own quality criteria. Five questions.

1. Is the question appropriate to the method? "How do families experience a first session in which a therapist asks about the father's drinking?" is a qualitative question. "Does asking about drinking improve outcomes?" is not.

2. Is the sampling described and justified? Who was interviewed, how were they recruited, and who declined? Families who agree to be interviewed about therapy are not a random sample of families in therapy, and the paper should acknowledge it.

3. Is the analysis transparent? Were transcripts coded systematically, by more than one person, with the codes and themes shown? Is there a "audit trail" from data to interpretation? Are negative cases discussed?

4. Does the paper stay close to the data? Good qualitative papers quote extensively and distinguish what participants said from what the authors concluded. Look for the place where the authors' interpretation outruns the quotes.

5. Is reflexivity present? Did the authors consider their own position — as clinicians, as researchers, as members of the same or a different culture as the participants — and its effect on what they saw?

What a good qualitative study gives a clinician. The vocabulary of the family's experience, which is otherwise invisible; the reasons for dropout, which outcome data record as a number; the details of what helps, which families describe in ways that no questionnaire anticipates. It will not tell you what to do, and it will change how you do it.

11.8 Routine outcome monitoring and feedback

The most robustly supported innovation in everyday practice is not a model; it is measurement with feedback.

What it is. Families complete a short measure every session or every few sessions — the ORS and SRS, the SCORE-15 or its session-by-session short form, symptom measures where relevant. The scores are plotted, compared with an expected trajectory, and shown to the clinician and the family. Where progress is off-track, the clinician changes something.

What the evidence shows. Feedback to clinicians improves outcomes overall, with the largest benefits for clients who are not progressing or are deteriorating — that is, exactly the clients a clinician's judgement is most likely to miss. Several trials find that feedback alone reduces deterioration rates; the effects on the average client are smaller.

What it looks like in a family session. Two minutes at the start: each member completes the measure; the therapist notes the scores; at the end, one question. "Your ratings went from 26 to 21 this fortnight — what has been different? And yours went from 18 to 20, which suggests something has got harder; is that right?" The measure becomes a third participant, and families often say things to the measure that they would not say to the therapist.

The discipline required. Plot the scores; do not merely file them. Use the scores in supervision. Have an agreed response to deterioration, which should include a review of the formulation rather than a repetition of the same intervention. And never use the measure as a test the family can fail.

The limits. Measures are culturally loaded, they privilege the presenting symptom, and they cannot capture what matters most to many families. They are an instrument, not a verdict — and an instrument in the hands of a clinician who also listens.

11.8a Choosing measures for a family clinic

A small, workable measurement set, with the trade-offs stated.

The session-by-session pair. The Outcome Rating Scale and Session Rating Scale take under two minutes each, are free for individual use in many jurisdictions, and produce a plot the family can see. Their weakness is that they are very short and give no detail about what changed.

The family-functioning measure. The SCORE-15 gives fifteen items and several perspectives, including the family's own account of strengths and difficulties, and it has been used widely in UK family therapy services. It is completed at assessment, mid-point and end, rather than every session.

The symptom measure. Where the presentation is a defined condition — depression, anxiety, eating disorder, conduct problems — use the standard measure for that condition, so that your outcomes are comparable with the literature and with other services.

The family's own measure. Two or three statements written by the family, scored weekly. This is the measure that families find most meaningful, and the one that most often captures the change the standard measures miss.

The rule. Two minutes per session, one fuller measure at assessment and review, one family-defined target, and a plot that is shown to the family. Anything more elaborate will be abandoned within a month; anything less will leave you guessing.

11.9 Common factors, specific factors, and the therapist

Three findings about how therapy works, each contested and each useful.

The common-factors position. Client factors, the alliance, the therapist's empathy and the family's expectancy account for a large share of outcome variance; model differences account for a smaller share. The position is supported by comparative trials that find few differences between bona fide treatments, and by meta-analytic work on alliance.

The specific-factors position. Technique matters, particularly for specific presentations: exposure for phobias, family-based treatment for adolescent anorexia, behavioural methods for conduct problems, communication and problem-solving training for expressed emotion in psychosis. The evidence for specificity is strongest where the disorder is well defined and the mechanism is well understood.

The therapist-effects position. The most uncomfortable finding in the field: therapists differ substantially in their outcomes, and the differences are not explained by training, experience or theoretical orientation. Some therapists reliably produce better outcomes with the same clients and the same model; some reliably produce worse, and appear unaware of it. Deliberate practice with feedback is the current best candidate for improving this.

What a trainee should take from all three. Work on the alliance, because it is the factor you can influence session by session; learn the specific methods that have evidence for the presentations you meet, because they organise your attention and your actions; and measure your own outcomes, because you cannot otherwise know which therapist you are.

> **DEBATE**
> If therapist effects are larger than model effects, what is the point of learning a model? Defend your answer with reference to the evidence.

11.9a The replication problem, and what it means for a trainee

Several of the most celebrated findings in psychology and psychotherapy have proved harder to replicate than their original reports suggested, and the field has responded with pre-registration, larger samples and a harder look at statistical practice.

Why it matters clinically. A trainee who learns a technique from a single impressive study may be learning an artefact. The defences are simple: prefer systematic reviews to single studies; prefer large samples to small; read the effect size and the confidence interval rather than the p-value; and be suspicious of any result that arrives with a marketing campaign attached.

Why it does not license nihilism. Replication problems are concentrated in social psychology and in some individual-difference research. The core findings of psychotherapy outcome research — that therapy works, that the alliance matters, that family interventions help in the presentations described in 11.5 — are supported by multiple independent trials, by different research groups, and by practice-based data. The evidence is uneven, not empty.

The professional's habit. Ask of any striking claim: has it been found by someone other than the people who invented the method? That single question will protect you from most of the field's enthusiasms, and it is the reason this week's readings include critics of the models we teach.

11.9b Two measures of the alliance, and how to use them

The Session Rating Scale and the Revised Helping Alliance Questionnaire both measure the therapeutic relationship; the first is four visual-analogue items, the second is longer and includes goal and task agreement. In family work the therapist should consider three perspectives rather than one.

Who to ask. The adult who organised the referral, the adult who did not, the child over about nine, and — where relevant — the adolescent who has been brought rather than consulted. A family in which the mother rates the alliance at 34/40 and the father at 16/40 is a family in which one member is likely to drop out, and the rating tells you which.

When to ask. Every session for the first three, then every third. Two minutes at the end of the session, with the family completing it in the room.

What to do with a low score. Not to reassure. The useful sequence is: thank the member; ask what would have made the session more useful; ask what the therapist did that made it less useful; and change one thing next time, visibly. Families notice when a therapist acts on a low rating, and the act of doing so frequently raises the rating more than any technique.

What to record. The score and the action. "Father 16/40 at session two; said he felt 'talked over'; agreed that I would ask him first at the start of each session. 29/40 at session three." That sentence, in a supervision file, is worth more than a page of impressions.

The caution. A high alliance score is not evidence of good therapy; families can be warmly engaged in work that is not helping. Measure outcome alongside alliance, and when the two disagree, trust the outcome and interrogate the alliance.

11.10 The family's own evidence

Research evidence is population evidence; it is not evidence about this family. Three ways to build the family's own data into practice.

Single-case designs. A baseline, an intervention, a return or a comparison, and repeated measurement across all three. In a family setting, a two-week baseline of the target behaviour, followed by a specific intervention, with the family rating the target weekly, is a legitimate and under-used method. It answers the question the family is actually asking: "is this working for us?"

Idiographic measures. Ask the family at the outset to write the two or three statements that would tell them things had improved — "we would eat together twice a week", "I would ask once and she would answer". Score them weekly. The measure is specific to the family, which is its point.

Practice-based evidence. A service that routinely collects outcome data can compare its own results with published benchmarks, identify which presentations it serves well and which it does not, and adjust. This is how a clinic becomes a learning organisation rather than a place where fashions arrive from outside.

The professional's stance. "The research tells me that about seven families in ten with this presentation benefit from this kind of work. I do not know yet whether you are one of them; I would like us to measure it together, and review in six weeks." That sentence contains population evidence, uncertainty, and a commitment to the family's own data, and it is the shape of an evidence-informed practice.

11.10a The one-page evidence sheet

The practical output of this week is a personal evidence sheet for one presentation you meet regularly. The format is deliberately short.

Heading: the presentation, and the age group.

What is recommended first-line, and by whom. One or two sentences, with the guideline and its date.

The key trial or review, in three sentences. What was done, what was found, and how strong the finding is.

The main alternative, and when to choose it. For example: individual therapy where the family cannot be engaged; a supported-carer model where the parents cannot take the required role.

The main uncertainty. What the evidence does not settle.

What I will measure, and when I will review. The measure, the interval, and the criterion for changing course.

One reference, and one person I can ask.

Why the sheet rather than a reading list. Because a clinician under pressure will consult a page, not a literature. Because writing the sheet forces the four judgements that matter — what is first-line, what is the alternative, what is uncertain, and what will I measure — and because it is revisable when the guideline changes or when your own data disagree with it.

> **TASK**
> Complete one evidence sheet for a presentation you have seen in placement. Bring it to the seminar and exchange with a peer working with a different presentation. Then answer: which of the two sheets would you trust if you were the family?

11.11 Six errors in using evidence

1. **Overclaiming.** "Family therapy is proven for depression" when the evidence concerns couple therapy for depression in distressed relationships, in adults, in trials of twelve to twenty sessions. Fix: say what the evidence covers, and name the boundary.
2. **Cherry-picking.** Citing the trial that supports the model and omitting the two that did not. Fix: when you cite evidence for a method, name the best study against it and say why you still use the method.
3. **Dismissing evidence because it is inconvenient.** "That trial used a manual, and real families are not like that." Sometimes a legitimate objection, usually a defence. Fix: write down the specific reason the evidence does not apply, and check whether the same reason would excuse a method you dislike.
4. **Confusing the model with the evidence.** Assuming that because family-based treatment works for adolescent anorexia, family therapy works for everything, or that because one trial failed, the approach is worthless. Fix: keep the map of the evidence by presentation, and consult it before making claims.
5. **Ignoring dropout.** Reporting outcomes only for completers, or planning a service on the assumption that families attend. Fix: treat attendance as an outcome, and measure it.
6. **Evidence without measurement.** Reading the literature diligently and never measuring your own results. Fix: the family's own data, session by session, is the minimum standard of an evidence-informed clinician.

11.11a The journal club that works

Most journal clubs fail: one person presents, everybody nods, nobody changes anything. A workable format, used in several training programmes, takes forty-five minutes and changes practice.

Before (10 minutes). Everyone reads the abstract and the results table only. Everyone writes down, in one sentence, what the study found and one question they would ask the authors.

Presentation (10 minutes). One person presents using the six questions of 11.6 and is not allowed to speak for more than ten minutes. The discipline is the point: a paper cannot be presented properly in ten minutes unless it has been understood.

The clinic test (10 minutes). Each participant names one family on their caseload to whom the study would or would not apply, and says why. This step is what converts reading into clinical reasoning, and it is the step most clubs omit.

The disagreement (10 minutes). The presenter must argue the strongest case against the study's conclusion, and somebody else must defend the study. The roles are assigned, not chosen.

The action (5 minutes). One change to practice, however small, with a date: a measure to be added, a referral criterion to be reviewed, a family to be discussed in supervision. The change is recorded, and at the next journal club the first item is whether it happened.

Why this works. Because it is short, because it requires reading, because it forces the clinic test, and because it ends in a change rather than in agreement. A team that runs this monthly will accumulate twelve practice changes a year, which is more than most services achieve in a decade of training days.

11.12 Culture, faith and the evidence

The evidence base is not culturally neutral, and the critique is now well developed.

Who was in the trials. Most psychotherapy trials are conducted in high-income countries, with samples that are disproportionately white, educated, urban and partnered. The extent to which findings generalise to families in Ibadan, to families with strong religious commitments, or to families in multigenerational households is an empirical question that has not been adequately studied. The clinician's obligation is to apply the evidence thoughtfully and to monitor the outcome, not to assume either that it applies or that it does not.

What counts as an outcome. Trials privilege symptom scores and relationship satisfaction, both measured with instruments developed in the same settings. A family may care about whether the household is peaceful, whether the children are respectful, whether the family's standing is intact, or whether a debt has been paid. The movement for culturally grounded and locally defined outcomes is a corrective, and it has not yet changed the main evidence base.

The cultural validity of measures. A questionnaire translated into another language is not necessarily measuring the same construct; a scale developed to assess "family functioning" in one setting may describe a different construct in another. Where measures are used across cultures, they should be validated locally, and the clinician should ask what the item means to this family rather than assuming.

Faith and the evidence. Where families hold spiritual explanations, the evidence base for collaborative approaches — working with religious coping rather than against it, and integrating spiritual and psychological frameworks with consent — is small but encouraging, particularly in depression and substance use. The relevant professional skill

is not to adjudicate between frameworks but to work with the family's own resources while remaining honest about what the evidence supports.

> **AUDIT**
> Take any guideline you use. Count the studies cited from outside North America and Europe. Then ask what that count implies for a clinic in Ibadan, and write 150 words to a colleague explaining how you will use the guideline anyway.

11.13 Evidence note: where the field is genuinely contested

An honest account of four live disagreements, because students should know that the field is not settled and that competent clinicians disagree.

Contest 1: family-based treatment versus individual treatment for adolescent anorexia. Family-based treatment has strong trial support and is recommended first-line in several guidelines. Critics argue that the trials are small, that the comparison treatments were not always optimally delivered, that the model's emphasis on parental control of eating is difficult in some family configurations, and that newer approaches (for example, supported carer models and adolescent-focused therapy) perform comparably in some trials. The practical position is that family-based treatment is a strong first-line option and not the only defensible one.

Contest 2: do model differences matter? Comparative trials frequently find small or no differences between bona fide therapies, and common-factors researchers argue that this reflects the dominance of shared mechanisms. Others point to specific effects in specific conditions, to the poor quality of many comparative trials, and to the fact that "no difference" in a small trial is not evidence of equivalence. The practical position: model matters most for specific presentations; alliance and feedback matter everywhere.

Contest 3: early intervention and prevention. Programmes for couple relationship education and for preventing child behaviour problems show small effects in universal populations and larger effects in targeted, higher-risk groups. Whether the small universal effects justify the cost is a policy question, not a clinical one, and clinicians should be careful not to overstate prevention claims.

Contest 4: the feedback effect. The routine outcome monitoring literature is generally positive, with some large trials finding smaller effects than the early studies and some finding none once clinician training and attention are controlled. The most defensible claim is that feedback helps most for clients who are not progressing, and that the mechanism is the clinician's attention rather than the numbers.

What to do with the contested areas. State the uncertainty when you teach or when a family asks; use the intervention that has the best evidence and fits the family; measure the result; and do not let loyalty to a model substitute for attention to the evidence.

11.14 Key terms

Term	Definition
Efficacy	Whether an intervention works under ideal, controlled conditions
Effectiveness	Whether an intervention works in ordinary services with ordinary clients
Randomisation	Allocation by chance, which distributes confounders across groups
Effect size (d)	The difference between groups in standard deviation units
Number needed to treat	How many clients must be treated for one additional good outcome
Confidence interval	The range of effects compatible with the data at a given probability
Mediator	A variable through which an effect is transmitted
Moderator	A variable that changes the size of an effect for different groups
Publication bias	The tendency for positive findings to be published and negative ones not
Allegiance effect	The tendency for researchers' commitment to a model to inflate its results
Fidelity	The degree to which an intervention is delivered as designed
Routine outcome monitoring	Session-by-session measurement of progress with feedback to the clinician
Practice-based evidence	Evidence generated from routine services rather than controlled trials
Idiographic measure	A measure constructed for, and about, one family

11.15 Self-test

1. What question does an RCT answer, and what question does it not answer?
2. What does d = 0.5 mean in terms of families?
3. Why is the number needed to treat a useful statistic for a clinician?
4. What six questions should you ask of any paper?
5. What does the evidence for routine outcome monitoring show, and for whom is the effect largest?
6. Name the presentation with the strongest evidence for a specific family-based treatment, and name the treatment.
7. What is the therapist-effects finding, and why is it uncomfortable?
8. Name two ways in which the evidence base is culturally limited.

Answers. 1. It answers whether the intervention causes better outcomes than the comparison in the population studied; it does not answer whether it will work in your clinic, with your families, or by what mechanism. 2. The average treated family is better off than about 69% of untreated families — roughly seven in ten. 3. It translates the effect into the number of families who must be treated for one additional benefit, which is what a service and a family need to know. 4. Who was studied? Compared with what? Measured how? How big is the effect? How confident can we be? Does it fit my clinic? 5. Feedback improves outcomes, with the largest benefit for clients who are not progressing or are deteriorating. 6. Adolescent anorexia nervosa; family-based treatment (the Maudsley approach). 7. Therapists differ substantially in outcomes, and the differences are not

explained by training, experience or orientation; it is uncomfortable because it implies that some of us are less effective and do not know it. 8. Samples are drawn mainly from high-income countries and from particular social groups; outcomes and measures reflect the priorities of those settings rather than those of the families many clinicians serve.

11.16 Reflective task

Choose one belief about your own practice that you hold strongly — perhaps that a particular model is superior, or that diagnosis is harmful, or that manualised treatment cannot work with complex families. Find the strongest evidence against your belief, and write 400 words summarising it fairly. Then write 150 words on what would have to be true for you to change your practice. Bring both to the seminar; the second paragraph is the one that matters, and most students find it easier to write than they expected.

11.17 Readings for this week

- **Carr (2019 or latest edition)**, Family Therapy: Concepts, Process and Practice, the chapters on research and on evidence-based practice, for the map.
- **Stratton (2020)**, Formulation in Family Therapy, and his writing on routine outcome measurement, for the practice-based evidence argument.
- **Wampold & Imel (2015)**, The Great Psychotherapy Debate, for the common-factors case, read critically.
- **Norcross & Lambert** on psychotherapy relationships and on feedback, for the alliance and monitoring evidence.
- **The relevant NICE guideline** (or your national equivalent) for at least one presentation you meet, read as a document with a method rather than as a list of recommendations.
- **One Cochrane review** on a family intervention, read with the six questions from 11.6 in front of you.

Next: Week 12 — Ethics, risk and the limits of the systemic frame

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Week 12 — Ethics, risk and the limits of the systemic frame

Ethics is not a chapter at the end of the course; it is the frame that makes the work possible. Every family therapist meets the four issues below in the first month, and meets them again every year thereafter.

12.1 The lecture in ninety minutes

Minutes	Segment	What the lecturer does	What students do
0–10	Why systemic thinking creates ethical problems	Four ways the frame can be used to avoid responsibility	Identify the four in a case
10–25	Confidentiality in a multiperson system	Write a confidentiality policy on the board	Draft the three sentences they will say aloud

Minutes	Segment	What the lecturer does	What students do
25–45	Risk: screening, assessing, managing	The screen, the assessment and the plan, demonstrated	Screen a role-played case in eight minutes
45–60	The limit case: secrets	Work the affair case in groups, then compare	Decide, and defend the decision
60–72	Competence and the duty to refer	What competence means when the model is relational	Audit one's own limits honestly
72–85	Dual relationships in small communities	The test, applied to five scenarios	Score and justify
85–90	Documentation	Read a good note and a bad note	Rewrite the bad one

12.2 Four ways the systemic frame can be misused

Systemic thinking is a powerful set of ideas, and like any powerful set it can be used to avoid responsibility. Four recognisable abuses, each of which a trainee will hear from an experienced colleague at some point.

Abuse 1 — "The problem is in the system, not in the person", used to avoid individual responsibility. Circular causality explains how a pattern is maintained; it does not dissolve a man's responsibility for violence, a parent's responsibility for a child's safety, or a professional's responsibility for a decision. The circular account is a description of a mechanism, not a moral verdict. Where a therapist uses circularity to soften a disclosure of harm, the theory has become a defence.

Abuse 2 — "We must not take sides", used to avoid protecting the weaker party. Neutrality is a technique for keeping both members engaged, not a moral position. A therapist who is neutral between a parent and a child about whether the child is safe has mistaken a stance for a value.

Abuse 3 — "The family did not engage", used to avoid examining the service. Disengagement is described in the literature as a client characteristic and is often a service characteristic: waiting times, location, language, letters, the room, the first ten minutes. The ethical version of the sentence is: "the family did not return, and here is what we did that may have contributed."

Abuse 4 — "It is cultural", used to avoid acting. Discussed in Week 10, and stated here as an ethical principle: culture explains, law and the code of conduct oblige.

The test. For any principle you are applying, ask: who benefits from my applying it this way in this case? If the answer is the therapist's comfort, the principle is being used as a defence.

12.2a Four principles, applied to a family

The four-principles framework — respect for autonomy, non-maleficence, beneficence and justice — is usually taught for individual work and becomes more interesting, and harder, when there are three or four clients in the room.

Autonomy. Whose autonomy? A father's autonomy to refuse a conversation about his drinking; a mother's autonomy to decide what is disclosed to her family; an adolescent's autonomy over their own body and information; a child's developing autonomy, which is not the same as an adult's. The principles conflict directly in family work, and no framework resolves them. What a framework does is force the therapist to name whose autonomy is being weighed, and why.

Non-maleficence. The duty not to harm. In family work the specific harms to watch for are: a disclosure that produces violence at home; a session that unites the parents against a child; a formulation that becomes a family's permanent story about one member; a record that follows a young person into adulthood. Each is a foreseeable harm and each is preventable with method.

Beneficence. The duty to benefit. In family work this includes the duty to notice when the therapy is not benefiting the family, and to change it — which requires measurement, and which is why Week 11 sits beside Week 12.

Justice. The fair distribution of the therapist's time, attention and access. In a family, the therapist will be tempted to give more to the member who is more articulate, more distressed or more likeable. In a service, the therapist will be tempted to give more to the families who are easier to help. Both are justice problems, and both are visible in the record if the record is honest.

The practical use of the framework. When a decision is hard, write the four headings and, under each, the considerations in this case. Then make the decision and record the reasoning. The framework's value is not that it produces the answer; it is that it prevents the therapist from deciding on the basis of the one consideration that happened to occur to them.

12.3 Confidentiality in a multiperson system

There is no single confidentiality in family work, and the failure to say so at the start is the commonest ethical error in the field.

What to say in the first session, in three sentences. "What is said in this room, with all of us here, is shared between us. If one of you tells me something alone, I will ask you to tell the others, and I may need to raise it myself if it affects someone's safety. What you tell me is confidential outside this family, except where the law requires me to act — if someone is at risk of serious harm, or a child is being harmed, I will have to tell someone, and I will tell you first that I am doing it."

Write it down. A one-page policy, given to the family at the first session, containing: who is seen together and who separately; what happens to information from individual sessions; the limits of confidentiality; how records are kept and who can see them; what happens if the family separates, or if one member asks for the record; and the complaints procedure. This page prevents most of the disputes that would otherwise occupy sessions.

The three categories of information. Useful to teach explicitly.

Shared information — said in a joint session, held by the family as a whole. Can be referred to freely.

Individual information — said to the therapist alone. Cannot be kept indefinitely if it bears on the work. The therapist's task is to help the member bring it into the open, with a deadline and support, and to say in advance what will happen if they do not.

Unkeepable information — disclosure of risk to a child, of serious harm to self or others, of an offence that the law requires be reported. Cannot be kept, must be handled transparently, and the member should be told what will happen and when.

Secrets: a decision model. When a member discloses something and asks that it be kept, five questions in order.

1. Is anyone at risk? If yes, the secret cannot be kept, and the process of disclosure is managed, not optional.
2. Does the information bear on the work? A secret that changes the formulation cannot be held without the therapist working dishonestly.
3. What is the likely consequence of disclosure, for each member? Violence, a breakdown of the therapy, a rupture in the family, a lost job. These are weighed, not dismissed.
4. Can the member be supported to disclose? Almost always the best route: a deadline, a rehearsal, the therapist present.
5. What will the therapist do, and by when, if the member does not? This must be stated in advance, in specific terms, and honoured.

> **CASE**
> During an individual session, a husband tells you he has resumed an affair and asks you not to tell his wife. He is not at risk and nobody is being harmed in the sense that requires disclosure. What do you do? Work it in groups, then compare with the decision model above. The defensible answers differ, and each must be able to answer question 2 honestly.

12.4 Risk: the screen, the assessment, the plan

Risk is not a topic; it is a routine. Every new case, every session where something has changed, and every session before a break.

The screen (two minutes). Five questions, asked of the relevant members, in plain language, every new case.

> "In the last month, have you had thoughts of ending your life, or of harming yourself?" "Have you ever tried?" "Is anyone in the house being hurt — hit, pushed, threatened, or made afraid?" "Is there anyone you are afraid of?" "Is there anything happening to the children that worries you, that we have not talked about?"

The assessment (when the screen is positive). Intent, plan, means, timeframe, protective factors, previous attempts, substance use, and the response of the people around the person. For violence: frequency, severity, escalation, weapons, children's

exposure, and the victim's own safety plan. For child protection: the specific incident, the child's account, the marks, and the family's explanation.

The plan (what must be written). Who is at risk, of what, how imminent; what has been done; who has been informed; what the family has agreed to do; what the therapist will do and by when; the next contact; and the review date. A risk assessment without a plan is a note, not an assessment.

The limits of risk prediction. Actuarial tools outperform clinical judgement for some outcomes, and none of them predict individual events well. The professional standard is therefore not accurate prediction but systematic enquiry, documented reasoning, appropriate action and review. A therapist who asks, records, acts and reviews has met the standard; a therapist who relies on a feeling has not.

The systemic dimension. Risk in families is relational: the risk to a woman is affected by who knows, who believes her, and what the family will do; the risk to a child is affected by the parents' capacity to protect, which is affected by their own circumstances. The formulation of Week 9 should include the risk, and the plan should address the relationships that make protection possible or impossible.

> **PRACTICAL**
> In pairs, take a risk history in eight minutes using the five screening questions, then write a risk plan in six lines. Swap and critique: does the plan name who, what, by when, and who else knows?

12.4b Domestic abuse: a screen that changes the work

Family work with domestic abuse is the ethical terrain on which trainees most often come unstuck, because the instincts of the model — bring everyone together, hold both accounts, avoid taking sides — can be exactly wrong.

The screen. Ask each adult separately, at least once, in words that do not require disclosure in front of a partner: "Has anyone in the house ever hurt you, or made you afraid? Has there been pushing, hitting, or threats?" Asking in front of the partner is not a screen; it is a test of the partner's reaction, and it can be dangerous.

The prevalence. In clinical populations, rates of domestic abuse disclosed to services are high, and in couple therapy samples substantial minorities of couples report aggression, sometimes mutual, often not. The therapist who does not ask will work with several such couples without knowing, which is both unsafe and clinically ineffective.

When disclosure occurs, four decisions.

One: is the couple work safe? Couple therapy is contraindicated where there is coercion and control, fear of the partner, escalation, or a risk of serious injury. It may be possible where there is situational aggression without control and both partners want to address it, with explicit agreements and safety planning. The distinction is not the presence of an argument; it is the presence of control and fear.

Two: what does the victim want? Not as a veto over safety, but as essential information. Many women do not want to involve the police, for reasons that are usually rational: money, children, family pressure, immigration status, and the probability of retaliation. The therapist's task is to widen the options, not to override the choice.

Three: what is the safety plan? Practical and specific — a bag, a telephone, a code word, a relative, a refuge number, a copy of documents. Written, and held where the partner cannot find it.

Four: what is the therapist's legal position? In many jurisdictions, disclosure of a risk to a child triggers a safeguarding duty; some jurisdictions have specific domestic abuse reporting duties. Know yours precisely, and say what you know to the person concerned.

The error to avoid. Splitting the couple work into "communication training" while the abuse continues, which treats a safety problem as a skills problem. The formulation must include the abuse as a fact about power, not only as a pattern of interaction.

The systemic nuance worth teaching. Abuse is not symmetrical, and circular accounts that describe "mutual escalation" can obscure who is afraid. It is legitimate to describe the interaction without implying equal responsibility, and the therapist should be able to say both: "you both shout, and only one of you is afraid of what will happen next."

12.5 Competence, and the duty to refer

Taking a case you are not trained for is not courage. It is a risk to the client and to you.

What competence means. The combination of knowledge, skills, supervision and judgement sufficient to work with this presentation, this family, in this setting. It is specific: a competent family therapist may be incompetent with a family in which a child is psychotic, or with a couple where there is a live court case about custody.

Four domains in which trainees most often over-reach. Adult mental illness (where the family work must be alongside, not instead of, psychiatric care); child protection (where the therapist is not the investigator); substance misuse and detoxification (where medical management is required); and legal proceedings (where the therapist may be asked to write a report beyond their competence).

The duty to refer, and the duty not to abandon. Where a case is beyond your competence, the professional response is: discuss in supervision; identify a colleague or service with the competence; make the referral with the family's agreement; and, where appropriate, continue to work alongside. The failure mode is the therapist who refers and disappears, leaving the family to negotiate a new system alone.

The duty to develop competence. The other side of the same coin: a therapist who refers every family with a particular presentation will never become competent with it. A training plan, with supervision, is the professional way to expand the boundary.

12.6 Dual relationships and power in small communities

Small communities, religious networks and family ties make dual relationships likely rather than exceptional. The test is not "is there a second relationship?" but "does the second relationship impair the first, or exploit the first?"

The five scenarios students most often get wrong.

1. The family in your church. You will see them on Sunday. Not automatically disqualifying; requires explicit agreement about what will be said in public, and a plan for what happens if the therapy becomes known.
2. Your cousin's neighbour. A distant social connection, but information will travel. Ask the family whether they know you, and record the answer.
3. A former client's sister. Usually workable with explicit acknowledgement; the previous client's confidentiality is the constraint.
4. A family where you have already given advice informally at a family gathering. The informal relationship has already compromised the clinical one; refer, and say why.
5. A family who ask you to attend a ceremony, give a lecture to their church, or invest in their business. Each converts a clinical relationship into another kind, and each needs to be declined or examined with supervision.

The power analysis to complete. For each potential dual relationship, ask: who benefits; what could the family feel unable to refuse; what would the family's neighbours think; and what would you write in the record if a complaint were made? If the fourth answer is uncomfortable, the answer to the case is probably no.

12.7 Documentation, and the record as an ethical object

Write notes as if a colleague will take over tomorrow and a court may read them in a year: date, attendees, presenting issues, risk screen result, interventions, plan, and who was told what.

Five disciplines.

1. **Separate fact from inference.** "Mother reports two incidents of pushing in March" is a fact. "Mother is frightened of father" may be an inference; say so if you write it.
2. **Record what you did, not only what you thought.** The intervention, the question you asked, the advice you gave, the referral you made, the time you made it.
3. **Record what the family said, in their words, where it matters.** Quotation is more defensible than summary and more useful to a colleague.
4. **Record the decisions and their reasons.** "Risk assessed as low because X, Y, Z; decided not to refer; will review in two weeks." Fourteen words that would answer most of the questions a review would ask.
5. **Never write anything you would not say to the family.** The record can be requested; the phrase "manipulative mother" in a file will be read by the mother one day, and it will be indefensible.

The record and the family. Increasingly, families have a right of access to their records. Write with that in mind: the record should be truthful, respectful, and comprehensible to an intelligent non-clinician. Where you must record something painful, record it in neutral language and record the context that makes sense of it.

> **AUDIT**
> Take the last note you wrote. Underline every sentence that is inference, every adjective about a person, and every claim you could not evidence. Rewrite the note so that nothing is underlined, without losing any clinical information.

12.7b Who owns the record?

A question that arises in every family case and is almost never discussed with the family until it becomes a dispute.

The legal position varies, and you must know yours. In many jurisdictions the record belongs to the organisation that created it, and the client has a right of access, subject to exemptions where access would cause serious harm to the client or another person. In others the client owns the record. In private practice the practitioner is usually the data controller and must comply with data protection law.

The family complexity. When several people are seen, the record contains information about each of them. A request from one member raises the question of the others' rights, and the professional response is to seek the guidance of the organisation's information governance lead rather than to decide alone.

What to tell the family. In the written policy: who holds the record, who has access, for how long, how it is shared with other professionals, and how to request a copy. The families who are told this at the outset are the families who complain least, because the surprise of discovering a shared record is worse than the requirement.

What this means for writing. Write every sentence on the assumption that every member may one day read the whole document. That single discipline improves record quality more than any template, because it removes exactly the kind of sentence that should never be written.

12.8 The limits of the systemic frame

The course teaches a systemic frame because it is powerful, and a professional education owes students an honest account of what the frame cannot do.

Limit 1 — it cannot substitute for individual treatment where individual treatment is indicated. Severe anorexia in an adult, psychosis, acute suicidality, a trauma requiring a phased individual approach: family work may be part of the plan and it is not the plan.

Limit 2 — it cannot adjudicate facts. Whether a child was hit, whether an assault occurred, whether a man accessed an image: these are questions for investigation, and a

family therapist who offers a relational account of a disputed fact may inadvertently obscure it.

Limit 3 — it can silence the individual. A focus on the system can make it difficult for a member to say "this happened to me". The therapist must be able to move between the relational and the individual register, and to say which register they are in.

Limit 4 — it can become a totalising theory. When every difficulty is a system, the therapist loses the capacity to notice biology, trauma, poverty, injustice or chance. The mature position is that the systemic frame is one of several and that its usefulness is conditional on the question.

Limit 5 — it can over-claim its evidence base. Week 11 sets out what is known. The ethical obligation is to practise within the evidence and to say plainly where it is thin.

The integration. A clinician who can say "I am working systemically here because the maintenance pattern is relational, and I am recommending individual trauma work for the mother in parallel" is demonstrating the frame's mature use, which is to know what it is for. The immature use is to treat the frame as a commitment rather than a tool; the mature use is to be able to name, in any given session, which frame is operating, why, and what it costs the family to have that frame rather than another.

12.9 Six ethical errors

1. **The unstated confidentiality policy.** Working for weeks and only then discovering that members assumed different rules. Fix: three sentences in the first session, written down.
2. **The secret kept too long.** A disclosure held alone while the therapy proceeds on a false basis. Fix: a deadline, support to disclose, and a stated consequence.
3. **The omitted risk screen.** Assuming somebody else asked. Fix: ask, record, and ask again after any change.
4. **The competence overreach.** Accepting a case to avoid appearing inadequate. Fix: supervision before the second session, not after the tenth.
5. **The judgmental record.** Adjectives about people, in a document that outlasts the therapy. Fix: write as if the family will read it, because one day they may.
6. **The theory used as a defence.** Circularity, neutrality or culture invoked to avoid acting on harm. Fix: ask who benefits from the principle as applied, and say the answer out loud in supervision, where it can be examined by somebody who is not invested in your comfort.

12.9a Case: the secret that involves an institution

A case that requires the confidentiality policy, the risk framework and the systemic frame to be used together.

The facts. A mother attends alone and discloses that her 15-year-old son has been taking money from her purse and that she believes he has been involved with a group of older boys who are "doing drugs". She has not told the father, who she says "would beat him".

She has not told the school, which has a policy that would lead to exclusion. She asks the therapist to keep all of it.

Why this is not a simple secret. Three of the three categories are in play. The information bears directly on the work (it is the presenting problem). Some of it is unkeepable (a child involved with drugs and at risk of harm, and a foreseeable risk of physical punishment if the father learns abruptly). And the mother's request is reasonable in her own terms: she is protecting her son from two systems she believes will harm him.

The decision model, applied.

Risk: is anyone at risk? Yes — the son, from the drug involvement and from the father's likely response. Is the risk imminent? Needs assessment, not assumption.

Bearing on the work: complete. The therapy cannot proceed as though the therapist did not know.

Consequences of disclosure: to the son, a beating if the father learns without preparation; to the mother, the loss of her only confidential ally; to the family, possibly a school exclusion.

Support to disclose: the mother can be helped to tell the father in a session, with the therapist present, in a conversation that is prepared; the school conversation can be staged, with the mother and the son, focusing on support rather than sanction.

What the therapist will do if she does not: stated in advance, and specific — an assessment of the son's safety with the mother's knowledge, a safeguarding consultation if the risk is confirmed, and a refusal to continue family sessions on a false basis.

The plan that follows. A session with the mother and son to assess; a joint session with both parents in which the mother discloses with the therapist's support and the father's response is managed; a school meeting arranged with the mother and son and framed around support; a safety plan for the son's contact with the older boys; and a confidentiality addendum recording exactly what was agreed and what the therapist said she would do.

What makes this ethical rather than merely competent. The therapist has not overridden the mother; she has refused to be complicit in a silence that puts the son at risk, and she has used her position to make the disclosures happen in the least harmful order. That combination — honesty about the limits of confidentiality, support for the person disclosing, and action in the interest of the person at risk — is the shape of ethical practice in family work.

> **DEBATE**
> The mother says: "If you tell his father, he will beat him, and that will be your fault." How do you answer? Write the three sentences you would actually say, and defend them against a peer who argues that the therapist should respect the mother's judgement.

12.10 Key terms

Term	Definition
Confidentiality (multiperson)	The negotiated rules governing what is shared, what is individual and what cannot be kept
Unkeepable information	Information that must be disclosed because of risk or law
Risk screen	A routine set of questions asked of every new case
Risk formulation	An account of who is at risk, of what, and what makes it more or less likely
Duty to refer	The obligation to involve a more competent professional or service
Dual relationship	A second relationship with a client or family member that may impair or exploit the clinical one
Competence (professional)	Knowledge, skill, supervision and judgement sufficient for this presentation, family and setting
Record access	The client's or family's right to see their clinical record
Mandated work	Therapy required by a court or statutory body
Whistleblowing	Raising a concern about unsafe practice within or outside an organisation

12.11 Self-test

1. Give two ways in which circular thinking can be misused.
2. What three sentences should introduce confidentiality in a first session?
3. What are the three categories of information in family work?
4. What is the difference between a risk screen and a risk assessment?
5. What must a risk plan contain?
6. What is the test for whether a dual relationship is acceptable?
7. Name three things a clinical note should record, and one thing it should not.
8. Name two limits of the systemic frame.

Answers. 1. To soften a disclosure of harm ("so you both escalate"), and to avoid protecting the weaker party in the name of neutrality. 2. What is said together is shared; what is said alone will be brought into the open with support; confidentiality is limited by risk and law, and the family will be told before action is taken. 3. Shared, individual and unkeepable. 4. The screen is five routine questions asked of everyone; the assessment is the structured enquiry that follows a positive screen. 5. Who is at risk, of what, how imminent; what has been done; who has been informed; what the family has agreed; what the therapist will do and by when; next contact and review date.

1. Whether the second relationship impairs or exploits the first — operationally, who benefits, what the family could

not refuse, and what you would write if a complaint were made. 7. Record: the risk screen result, the intervention, the decisions and their reasons; do not record adjectives about people. 8. It cannot substitute for individual treatment where indicated, cannot adjudicate disputed facts, can silence the individual, and can become a totalising theory (any two).

12.12 Reflective task

Write your own confidentiality policy in the three sentences you would actually say aloud, then read it to a peer and ask them to describe, in their own words, what they understood. Most students discover that one of the three sentences is unclear, usually the second, and that the peer heard a promise the therapist had not intended to make. Revise, and bring the final version to the seminar.

A second exercise, done in the same pairs: each student states, in one sentence, the circumstances in which they would break confidentiality without the family's agreement. The sentences are collected anonymously and read aloud. Almost every cohort produces the same two or three situations — a child at immediate risk, a serious threat to a partner, a disclosure of abuse by a person who is not in the room — and the exercise demonstrates that the ethical position does not depend on the individual therapist's temperament but on a small number of defensible limits that can be stated in advance and held under pressure.

12.13 Readings for this week

- **Your professional code of conduct and ethics**, read in full this week rather than consulted when needed.
- **BACP, AFT or your national association's ethical framework**, with particular attention to the sections on multiple clients, confidentiality and competence.
- **Your jurisdiction's safeguarding guidance**, including the threshold for referral and the definition of harm.
- **Bond (2015)**, Standards and Ethics for Counselling in Action, for the practical ethics of multiperson work.
- **A published professional conduct case** or fitness-to-practise hearing, read for how the standards are applied when things go wrong.
- **Chapter 5 of MFT 704** in this library, on decision models, for the detailed treatment of the affair case.

Next: Week 13 — Integration I: three lenses on one family

12.3a Annotated transcript: setting the frame in ninety seconds

The confidentiality statement is usually delivered badly: too long, too legalistic, or as a form to be signed. Here is the opening of a first session, with annotations.

> **T1 Therapist:** Before we start, I want to tell you how I work with information, because we are three people in this room and there is no single rule. Is that all right?
> **T2 Mother:** Yes.
> **T3 Therapist:** Anything said here, with all of us present, stays with all of us. That is the easy part.
> **T4 Father:** And if she comes alone?
> **T5 Therapist:** If one of you comes alone and tells me something, I will encourage you to tell the others, and I will tell you in advance if I think I have to raise it myself. I will not carry a secret about your family and work as though I did not know it.
> **T6 Father:** What if it is not my business to tell her?
> **T7 Therapist:** Then we talk about what to do with it, and you decide, unless it is about someone's safety. If a child is being hurt, or someone is at risk of serious harm, I am not allowed to keep that, and I will tell you before I do anything.
> **T8 Mother:** And the school?
> **T9 Therapist:** Nothing goes to the school without your agreement, except in the safety situation I have described. If I write a letter, you will see it first.
> **T10 Father:** (nods) That is fair.

Annotations. T1 names the structural fact — three people, no single rule — and asks permission. The permission is not merely courteous: it establishes that the frame is negotiated rather than imposed, which is the first demonstration of how the therapy will work.

T4 is the question the therapist wants, and it comes because she left a gap rather than filling it. Trainees tend to deliver all three categories in a monologue, which produces silence; a short statement followed by a pause produces the question that matters to the family.

T5 contains the crucial phrase: "I will not carry a secret about your family and work as though I did not know it." This is the sentence that prevents the commonest ethical trap, and it is more honest than the usual formula ("what you tell me alone stays alone"), which is a promise the therapist cannot keep.

T7 states the exception in plain language, with two examples, and adds the procedural guarantee: "I will tell you before I do anything." The guarantee is what converts a threat into a protection, and it is what families remember when something goes wrong.

T9 pre-empts the question every family with a school involvement will ask, and it makes a promise the therapist can keep. Note the specificity: "if I write a letter, you will see it first," not "your information is confidential."

What is missing, and should be added in the written policy. The record: how it is kept, who can see it, how it is shared, and what happens if the family separates or if a member requests access. A ninety-second statement cannot contain everything, which is why the written page exists.

12.4a Suicide risk in a family session

The commonest serious-risk situation in family work is an adolescent's disclosure of suicidal thoughts, and the commonest failure is a therapist who does not ask because asking feels like introducing the idea.

The evidence on asking. Asking directly about suicide does not increase risk; there is consistent evidence that screening and asking are safe and that they identify people who would otherwise be missed. The reluctance is the therapist's, not the family's.

What to do when an adolescent discloses, in order.

One: thank them, and slow down. "Thank you for telling me. That took courage. I want to ask you some careful questions, and then I want us to decide together what happens next."

Two: assess. Thoughts, frequency, intent, plan, access to means, previous attempts, what has kept them safe so far, who knows, and what they would want their parents to know. Ask in plain language: "Have you thought about how you would do it?" is a legitimate clinical question.

Three: negotiate disclosure with the young person before speaking to the parents. Not a veto — the parents must be told — but an agreement about how, when and by whom. "I have to tell your mother. Would you rather do it with me in the room, or would you rather I did it while you wait outside?"

Four: involve the parents as protectors, not as investigators. The conversation with parents should give them three things: what has been disclosed, what the plan is, and what to do tonight. Parents who are frightened become intrusive; parents who are informed and given a task become protective.

Five: means restriction. The single most effective intervention available to a family therapist is the reduction of access to means — medication, pesticides, a vehicle, a weapon. Ask: "What is in the house that could be used?" and make a specific plan.

Six: a written safety plan. Warning signs, coping strategies, people to contact, professional contacts, and the means restriction agreed. Written with the young person, in their words, and copied to the parents with their agreement.

Seven: escalate if needed, and record. Same-day psychiatric assessment where intent or plan is present; the family's general practitioner informed with consent; and the reasoning recorded, including what was decided and why.

> **CAUTION**
> Do not manage serious suicide risk in a family session as though the family's presence were a substitute for a clinical risk assessment. The family is a resource, not a plan.

12.5a Mandated families: the ethics of court-referred work

A significant proportion of family therapy is mandated: by a court, a child protection plan, a probation order or a school's conditional enrolment. Mandated work has its own ethical architecture.

Who is the client? The first question, and it must be answered explicitly and in writing. Where the referrer is a court or a local authority, there are three possible answers: the family is the client and the referrer receives reports (the usual arrangement); the referrer is the client and the family are participants in an assessment; or the therapist has a dual role that must be managed in the open. Each has different rules about confidentiality, and the family must be told which one applies before the first session.

Consent under mandate. Consent is not absent because the family is required to attend; it is thinner and must be worked for. "You did not choose to be here. You can choose what you tell me, and how you use the sessions. I am obliged to report on attendance and on risk; I am not obliged to report everything you say, and I will tell you before I write anything."

Reporting. Say what will be reported, to whom and when, in advance. A report that surprises the family is a breach even when it is lawful. Where the report will be read by a court, write it with the standards of 12.7 and give the family the opportunity to correct factual errors before it is filed.

Therapeutic work under mandate. Mandated families can engage genuinely, particularly when the therapist is clear about the mandate, offers something the family actually wants, and does not pretend that attendance is voluntary. The failure mode is the therapist who either becomes an agent of the court (losing the family) or colludes with the family against the court (losing the mandate and the professional standing).

The ethical floor. The therapist does not write a report that misrepresents the evidence, does not accept a referral whose purpose is to produce a predetermined conclusion, and does not conceal a conflict of interest. Where the instruction is to produce a particular finding, the professional response is to decline the instruction in writing.

12.5b Three worked competence decisions

Case 1 — the psychotic adolescent. A family is referred for "communication problems"; the sixteen-year-old son has become withdrawn, is not sleeping, and has begun to say that the neighbours are transmitting his thoughts. Competence question: is this family work? Answer: the presentation requires urgent psychiatric assessment, and family work is a component of the care that follows, not a substitute for it. The professional move: telephone the general practitioner and the local mental health service that day, with the family's knowledge; offer the family sessions alongside the assessment; and record the reasoning.

Case 2 — the live custody dispute. Parents of two children, separated, each requesting therapy for the children and each asking the therapist to "note what the other one is like". Competence question: can a therapist hold a clinical role in a contested legal process? Answer: not without clarity. The professional move is to state, in writing, that the

therapist's role is clinical, that no report will be provided for the proceedings without a separate instruction and the agreement of both parties, and that any letter will be sent to both parents. Where the court requires a report, it should be commissioned from an independent expert, not from the treating clinician, and the therapist should say so.

Case 3 — the family that wants a diagnosis for the father. An adult family brings a 62-year-old father who has become irritable and forgetful; the family wants the therapist to "confirm it is stress". Competence question: is this a psychological presentation? Answer: the change in an older adult requires a medical assessment for neurological and other causes before any psychological formulation. The professional move: state plainly that a medical assessment is needed, help the family arrange it, and hold the family work until the results are known.

12.6a Five dual-relationship scenarios, decided

The test from 12.6 applied, with reasoning shown, so that students can see how a decision is made rather than only what it is.

Scenario A — the family who attend your church. Facts: you will see them weekly in a setting where you are a member rather than a therapist. Risks: the family may feel unable to speak about matters that would become known in the congregation; the therapist's own standing may be affected. Decision: usually workable, with three conditions — an explicit conversation about what will be said in public, an agreement that you will not acknowledge the therapy at church unless they do, and a review after four sessions. If the family is prominent in the congregation, or the presentation involves matters the community would find scandalous: refer.

Scenario B — your cousin's neighbour, in a small town. Facts: information travels; you may be seen as connected to the family's affairs. Risks: the family may censor what they say; you may hear accounts of yourself through the network. Decision: ask the family directly whether they know you, and record the answer. If the connection is genuine and acknowledged, work with a stricter boundary than usual. If the family did not know and would have chosen otherwise, offer an alternative.

Scenario C — a former client's sister, referred for couple work. Facts: you hold confidential information about the sister's family of origin. Risks: inadvertent disclosure; the former client may feel betrayed; the sister may ask you questions you cannot answer. Decision: usually workable, provided you (a) obtain the former client's permission where they are identifiable and the information is relevant, (b) tell the new clients what you cannot discuss, and (c) record the arrangement.

Scenario D — you gave advice informally at a family gathering. Facts: a clinical relationship is being proposed after an informal one. Risks: the family will treat you as an adviser with a prior view; their account will be shaped by what you already said; you have already formed a view without a proper assessment. Decision: decline, and explain why in one sentence. Offer a referral to a colleague; the family will usually accept.

Scenario E — an invitation to speak at the family's church, or to invest in the family's business. Facts: the relationship is being converted from clinical to public or

commercial. Risks: the family may feel unable to refuse; the therapeutic relationship is contaminated by an audience or a financial interest. Decision: decline both while the therapy is live. After discharge, the public invitation may be acceptable if the family initiates it, if no clinical material is used, and if the offer is recorded; the commercial invitation is never acceptable.

The general rule. The question is never "is this comfortable for me?" but "would this family be able to refuse, and would they be able to speak freely afterwards?" Where the answer to either is no, the boundary is in the wrong place.

12.7a A note that survives scrutiny

The same session, written twice.

The bad note.

> "Seen mother and J (14). Mother very hostile and manipulative, clearly enmeshed with J. J sullen, probably depressed. Father not present, as usual. Discussed boundaries. Mother became tearful and blamed everyone. Will review. Risk — none apparent."

What is wrong with it. Four adjectives about a person ("hostile", "manipulative", "enmeshed", "sullen"); a diagnosis made in passing ("probably depressed"); a judgement about an absent person ("as usual"); an intervention described in a word that means nothing without context ("discussed boundaries"); no record of what was actually done or agreed; a risk conclusion without a screen; and a tone that the mother would experience as an attack if she ever read it.

The good note.

> "Session 3, 14 October, 60 minutes. Present: mother, J (14). Father absent — mother reports he is working away and has been informed of the appointment by text. Presenting: mother reports two arguments this week, both about J's telephone. J reports that mother 'checks everything'. Risk screen: mother and J both denied thoughts of self-harm or suicide. No disclosure of physical harm in the household. J said 'it is not that bad'. No safeguarding concern identified today; screen to be repeated at review. Work: I asked each of them to describe the last argument in sequence. The sequence each described was: J takes the telephone to her room; mother asks for it; J refuses; mother takes it; J shouts; mother leaves the room; both silent for the evening. Both agreed this was accurate. Intervention: I offered the reframe that both of them are trying to keep the peace, in different ways, and that the telephone has become the place where the argument happens. Mother said 'I am not trying to control her'. I did not argue; I asked what she would want instead. She said she would want J to tell her who she is talking to. J said she would tell her if she did not 'make it a police matter'. Agreed task: J to tell mother who she is messaging, once a day, unprompted; mother to ask once and not to take the telephone for one week. Both agreed to try; J said 'I will try'. Plan: session 4 next week, father invited by telephone call from me with mother's agreement. Risk screen repeated. Supervision: discussed the father's absence and my own irritation with the mother. Signed, time, date."

Why the second note is defensible. It records fact and quotation; it records the risk screen and the reasoning; it records the intervention precisely; it records what was agreed by whom; it records the plan and the supervisory discussion; and it contains not one adjective about a person. It is also more useful: a colleague taking over tomorrow could continue the work from it.

12.8a Referring well: the handover

A referral is a clinical intervention, not an administrative act. Four components.

One: the conversation before the letter. Tell the family what you are recommending, why, and what will happen. Families who are referred without explanation experience it as rejection, and a proportion of them do not attend.

Two: a letter that would help a colleague. The reason for referral; what has been assessed; the formulation in three sentences; what has been tried; the risk status and the safety plan; what the family wants; and what you have told them. Two sides of A4 at most.

Three: the warm handover where possible. A joint meeting, or a telephone call with the receiving clinician present, raises attendance substantially. Time-consuming and worth it.

Four: the boundary of your continuing role. Say what you will do next: remain available for consultation, hold the family's file, close the case. Ambiguity here is the commonest cause of families falling between services.

> **PRACTICAL**
> Write a referral letter for the Adeyemi family to an adult mental health service for the mother. Then have a peer read it as if they were the receiving clinician and say what they would still need to know.

12.10a When a complaint is made

Complaints are inevitable in a professional career and are survivable. Five principles.

Respond promptly and personally. A telephone call within 48 hours prevents most escalations. The complaint is usually about being unheard, and hearing it is half the remedy.

Separate the complaint from your self-image. Being complained about does not make you incompetent, and dismissing the complaint does not make you competent. Read it twice before responding.

Record everything, factually. What was said, when, to whom, and what you did. Do not amend the clinical record in a way that could look like retrospective justification; add a dated addendum instead.

Use the formal process, and get support. Tell your supervisor and your indemnity organisation. Do not handle a serious complaint alone, and do not discuss it with colleagues who are not part of the process.

Learn, and say what you learned. Where the complaint is justified, an apology and a specific change are worth more than a defence. Where it is not, a clear, kind explanation is worth more than silence.

The systemic point. Most complaints in family work arise from the multiperson structure: one member feels the therapist sided with another, or feels that information was shared wrongly. The confidentiality policy of 12.3 and the record discipline of 12.7 are, in practice, the best complaint prevention available.

Week 13 — Integration I: three lenses on one family

This week is a rehearsal for MFT 702. You will apply three models to the same case and notice that each produces a different question, a different focus and a different first move. The purpose is not to choose a winner but to become fluent enough to choose deliberately.

13.1 The lecture in ninety minutes

Minutes	Segment	What the lecturer does	What students do
0–10	What integration is, and three kinds of it	Define assimilative, eclectic and pluralistic integration	Place themselves on the map
10–25	The case, in full	Read the transcript of the first ten minutes twice	Note what they notice on each reading
25–40	The structural lens	Formulate live, with the case on the board	Add the structural diagram
40–52	The Bowen lens	Formulate live; coach the parents on paper	Identify the triangle and the projection
52–64	The solution-focused lens	Formulate live; find three exceptions	Scale the family's confidence
64–78	The comparison	Three plans side by side	Argue for one, then argue against it
78–90	The switching exercise	Demonstrate one therapist changing lens mid-session	Rehearse a switch

13.2 Three kinds of integration

"Integration" is used loosely in the field. Three distinct activities, each with a different logic.

Assimilative integration. The therapist has one primary model and incorporates techniques or concepts from others without abandoning the core. This is how most experienced clinicians actually work: a structural therapist who uses solution-focused questions in the first session, or a systemic therapist who uses an emotion-focused enactment. The advantage is coherence — the therapist knows what they are doing and why. The risk is that borrowed techniques are used without the theory that makes them work.

Eclectic integration. The therapist selects from several models according to the case, with no single organising framework. The advantage is flexibility. The risk, well documented, is incoherence: a session that begins structurally, shifts to narrative questioning, and ends with a behavioural task can leave the family confused about what kind of conversation they are in.

Pluralistic integration. The therapist works with the client's preferences and the evidence, using a meta-framework that explicitly includes multiple methods and negotiates them with the family. The advantage is collaboration and fit; the risk is that the therapist's own expertise dissolves into consumer choice.

Where a trainee should begin. With assimilative integration, because a primary model gives a trainee something to be wrong about. A therapist who has no primary model cannot tell whether a failure belongs to the model, the technique or themselves, and therefore cannot learn from it. The primary model is the instrument; the others are the extensions.

The four tests of a defensible integration. Does it have a stated primary frame? Can the therapist say, in any given session, which frame is operating and why? Does each borrowed technique retain the conditions that make it work? Does the family experience one coherent conversation rather than three?

13.2a Your primary model: a diagnostic

Before the seminar, answer six questions about your own practice. The answers are not a test; they are a baseline against which the rest of the programme will make sense.

1. In your first ten minutes with a new family, what do you do? If the answer is "ask each person what they have noticed", you are working structurally. If it is "ask what they have already tried", you are working solution-focused. If it is "ask each person what they feel", you are working emotion-focused. Most trainees have a default, and most have never named it.

2. What do you look at first in a transcript? Alliances, sequences, feelings, language, or the absent member. The thing your eye goes to is your model, whatever you say it is.

3. What do you do when the session gets difficult? This is the most revealing question. A structural therapist reorganises the room; a Bowenian slows down and asks about the process; a solution-focused therapist asks about exceptions; an emotionally focused therapist deepens the affect; a narrative therapist externalises. The default response under pressure is the primary model, and it is the one you need to know.

4. What kind of family makes you anxious? A family that will not speak, a family that shouts, a family that idealises the therapist, a family that asks for a diagnosis. The anxiety marks the limit of the primary frame.

5. What would you have to give up to change models? Coherence, speed, a sense of competence, the approval of a supervisor. Naming the cost is what makes a deliberate change possible, and the cost is usually higher than trainees expect: a therapist who has

spent three years becoming fluent in one frame has invested in that fluency, and the investment is invisible until a family arrives who needs something else.

6. Which of the five lenses in this week do you use least? That is the lens to practise in MFT 702, and the one you are most likely to need with the families who currently defeat you.

Why this matters now. A trainee who cannot name a primary model cannot integrate, because there is nothing to integrate into. The naming comes before the borrowing.

13.3 The case: the Adeyemis, session one

The material below is the data for the whole week. Read it twice: once for the content, once for what you notice.

Referral. "Amara Adeyemi, 16, out of school for four months. School has made a formal referral to the education welfare service. Mother reports she 'cannot manage her'. Father works in Lagos, attends when he can. Younger brother 11, no difficulties reported."

Who attends. Mother (41, nurse, works shifts), Amara (16, Year 11, wants to be a pharmacist), brother (11, brought because there was nobody at home). Father absent — "he cannot take time off in the week".

What the mother says. Amara refuses to go to school. She has tried everything: talking, shouting, removing the telephone, prayer, fasting, a conversation with the school counsellor. Amara stays in her room. The mother telephones the school each morning with an explanation. She is tired, and she is frightened of the education welfare officer. She mentions, in passing, that her husband "does not understand" and that when he is home at weekends he tells her she is too soft.

What Amara says. She is not going back. The school is "full of people who think they know you". She has one friend who left in the summer. She says her mother "talks at me". She says her father "does not listen, he just decides things". She denies being bullied, denies self-harm, and says she is "fine" when she is in her room.

What the brother says. "It is quiet when she is at home. When she is at school it is not quiet." Asked what he means, he says: "Mummy is always on the phone to the school."

What is not said. Nobody mentions the father's redundancy in 2019, the two years he was at home, or the paternal grandmother's death in 2022. Nobody mentions the mother's own father, who died when she was 19.

13.4 The structural lens

The formulation. The household has a parental subsystem that is functionally single — the mother manages the children alone for five days a week — and a cross-generational coalition in which Amara and her mother are locked in a struggle that has the intensity of a marital conflict. The father, when present, enters as a third authority whose interventions are not coordinated with the mother's. The sibling is peripheral and articulate about the family's emotional temperature. The boundary around the parental subsystem is weak in the week and rigid at the weekend.

The first move. Strengthen the parental subsystem before addressing the school refusal: a session with the two parents only (by telephone if necessary), in which they agree a single position on the school, including what will happen on Monday morning, and who speaks to the school. In the joint session, block the mother–daughter escalation by moving the conversation to the parents and asking Amara to observe rather than participate for ten minutes — a structural move that puts the adults in charge of the decision.

A sample intervention, scripted. "I would like to talk to your parents for a few minutes about Monday, and Amara, I would like you to listen rather than answer, because the decision is theirs to make. Is that all right? — Mrs Adeyemi, Mr Adeyemi, by Sunday evening I want the two of you to have agreed one sentence about Monday morning that you will both say, in the same words. Not two sentences, not one each. What would that sentence be?"

What this lens sees. Hierarchy, alliances, boundaries, the sibling's position, and the immediate, practical leverage in the parental subsystem.

What it risks missing. The mother's grief and exhaustion; the meaning of school for Amara; the possibility that the refusal is protective of something the family has not named; and the institutional loop with the school, which a purely structural frame tends to treat as background.

What would falsify it. If the parents agree a single position and the mornings do not change at all within two weeks, the coalition is not the maintaining mechanism, and the formulation needs the next lens.

13.5 The Bowen lens

The formulation. The family's anxiety is high and is being managed by distance (the father's work), by conflict (the mother–daughter struggle) and by projection onto the adolescent, who is the family's identified symptomatic member. The mother's own history — the eldest of five, her father's death at 19, her care of four siblings while her mother was unwell — has produced a pattern of over-functioning under stress and a low tolerance for needing help. The father's history — his mother's death when he was 16, his departure for Lagos that year — has produced a pattern of managing anxiety by leaving and by being needed at a distance. The marital dyad is emotionally distant, and the distance is stabilised by the focus on Amara. Amara is triangled into her parents' unresolved business, and her refusal is simultaneously a protest and a stabiliser.

The first move. Lower the family's reactivity and increase the parents' differentiation: coach each parent, separately at first, to observe their own part in the cycle without blame, and to manage their own anxiety rather than attempting to manage Amara's. Concretely: a "detriangulation" task in which the mother, when Amara refuses, does not argue, does not telephone the school, and instead says one sentence and leaves the room — reducing the emotional intensity of the triangle rather than winning the contest.

A sample intervention, scripted. "When Amara says she is not going, and you feel the anger rising, I want you to notice it and to say one sentence — something like 'I am not going to argue this morning' — and then to leave the room. Not because it will get her to

school. It will not, at first. Because it takes you out of the triangle and leaves her with her own decision for twenty minutes. Can you do that and tell me what happens?"

What this lens sees. Multigenerational patterns, the transmission of anxiety, triangles, the function of the symptom in managing marital distance, and the therapist's own reactivity as a source of information.

What it risks missing. The school's timetable, which will not wait for differentiation; the practical conditions of the family's week; and the possibility that the problem is better addressed by a direct, pragmatic move than by a slow process of coaching.

What would falsify it. If the mother can lower her reactivity for two weeks and nothing changes in Amara's behaviour or the family's emotional temperature, the triangle is not the central mechanism.

13.5a The same case at session eight: what each lens would call progress

A useful exercise, because it distinguishes the lenses more sharply than their first moves do. The Adeyemis have now had eight sessions. What would each lens regard as the evidence that the work is succeeding?

The structural criterion. The parents have made three decisions together without Amara in the room, and the same sentence has been used in the kitchen on consecutive mornings. The sibling has been asked, once, how things are at home, and has given an answer that was not about Amara. The hierarchy is functioning: decisions are made in the parental subsystem and communicated downwards, rather than negotiated in the triangle.

The Bowen criterion. The mother has described her own reaction in a session without blaming her daughter, and has reported one occasion on which she left the room instead of escalating. The father has described his own experience of being excluded, rather than criticising his wife. The parents have had one conversation about the family's two deaths. The family's reactivity is lower, and it is lower across several domains, not only around school.

The solution-focused criterion. The family can describe, specifically, what a good morning looks like, and can report three occasions on which it happened. The mother's confidence on a nought-to-ten scale has moved from two to four. Amara has described one thing she wants that is not "to be left alone".

The emotion-focused criterion. The mother has expressed the fear and the shame in Amara's presence, and Amara has responded with something other than withdrawal. The father has said what it is like to come home, and has been heard. The cycle has been slowed enough that the family can describe it together.

The narrative criterion. The family has a name for the problem that is not Amara's name. Each member can describe what the problem has cost them. Somebody has produced a unique outcome — a morning that contradicts the story — and the family can say what it means.

What the exercise reveals. The five criteria are not interchangeable, and a family can meet one and fail the others. A family that has made decisions together but in which

nobody has said anything true about the deaths has met the structural criterion and not the Bowenian one; a family that can describe a good morning but cannot describe what the problem has cost them has met the solution-focused criterion and not the narrative one.

The practical consequence. The therapist should decide, at assessment, which criterion will be the primary measure of success for this family, and say so. "For the next eight weeks, I will judge this work by whether the two of you can make one decision together and hold it. We will look at everything else afterwards." That statement is what a formulation is for, and it prevents the demoralisation that comes from judging progress by every criterion at once.

13.6 The solution-focused lens

The formulation. The family is organised around a problem story that has become total: "Amara refuses school, we cannot manage her, the school is threatening us." Within that story there are exceptions, competences and preferences that nobody has described. The task is not to explain the problem but to build a description of the solution the family wants, and to notice the occasions when it already happens.

The first move. Establish what would be different, in observable terms, and find the exceptions. Questions, in order:

> "Suppose tonight, while you are asleep, the problem is solved. What is the first thing you notice tomorrow morning that tells you something is different?" "What would I see you doing in that kitchen that I would not see now?" "When was the last time it was a little bit like that — even for half a morning?" "What did you do that made that possible?" "On a scale of nought to ten, where are you now, and where would one point higher be?"

A sample intervention, scripted. "You said the last good morning was in September, the week of your sister's wedding, when Amara got up without being called twice. What was different about that morning? — You had cooked something she likes. — What would it take to do that on a Monday, not because it will get her to school, but because it is the kind of morning you want more of?"

What this lens sees. The family's own goals, their existing competences, the small differences that make a difference, and the speed at which a family can move when the conversation is about what they want rather than what is wrong.

What it risks missing. The function of the symptom (what it is doing for the family system); the loss and history that may be driving it; the institutional realities; and the possibility that optimism becomes an implicit demand that the family perform improvement for the therapist's benefit.

What would falsify it. If a well-specified preferred future and two weeks of exception-building produce no change in the mornings and no increase in the family's own sense of agency, the problem is not a solution-description problem.

13.7 Two further lenses, briefly

MFT 702 treats each model in depth. Two more are presented here because comparing three lenses is instructive and comparing five is educational.

Emotion-focused lens. The mother's exhaustion is secondary to a primary emotion — the fear of being alone with a child she cannot reach, and the shame of failing at the one thing she believes a mother should manage. The father's weekend interventions are secondary to his own shame at having been absent and his fear of being irrelevant. Amara's refusal is a protest against a bond that has become functional rather than affectionate. The cycle is: mother pursues, Amara distances, mother escalates and then collapses, Amara feels guilty and withdraws further, father enters at the weekend with a directive that confirms both his irrelevance and the mother's resentment. The first move is to slow the cycle down and access the primary emotions, beginning with the mother's, in an enactment in which Amara is asked to listen.

Narrative lens. The family is living inside a dominant story — "Amara is the difficult one" — that has recruited the school, the mother and, in a different way, Amara herself. The task is to externalise the story ("the Not Going to School problem"), map its influence on each member, and find unique outcomes where the family has resisted it. The first move is to separate the person from the problem and to ask each member what the problem has cost them and what it has taken from them.

Why all five lenses produce a plausible plan. Because the case genuinely contains hierarchy problems, triangles, exceptions, unexpressed emotion and a dominant story. A family is not one thing; it is a system rich enough to support several accurate descriptions. The professional question is therefore not "which description is true?" but "which description gives this family, at this moment, the best chance of change, and what will tell me if I am wrong?"

13.8 The three plans side by side

The existing summary of the three lenses, kept because it is the clearest one-page statement of the comparison.

Model	Key question	First move with the Adeyemis	Likely risk
Structural	What is the hierarchy, and where are the boundaries?	Strengthen the parental subsystem; block the mother–daughter coalition	Imposing a structure the family experiences as foreign
Bowen/multigenerational	How does anxiety travel, and who absorbs it?	Coach the parents to manage their own reactivity; map the projection onto Amara	Slow; family may want symptom relief now
Solution-focused	What is already working, and what would "one point better" look like?	Find exceptions to school refusal; scale the parents' confidence	May bypass the function of the symptom

Adding the two further lenses.

Model	Key question	First move	Likely risk
Emotion-focused	What primary emotion is the argument protecting?	Slow the cycle; access the mother's fear and shame in an enactment	Intensity too early; the family may not have the capacity to tolerate it yet
Narrative	What story has recruited this family, and who is it serving?	Externalise the problem; map its influence on each member	May remain at the level of description and never reach the mornings

The comparison in four sentences. Each lens selects a different unit (structure, triangle, exception, emotion, story). Each generates a different first move and therefore a different first session. Each has a characteristic blind spot that the others cover. Each is testable, and the marker of a competent clinician is that they can state what would show their chosen lens to be wrong.

13.8a When the lenses disagree: a decision protocol

A procedure for the moment in the seminar — or the clinic — when two competent clinicians, using different lenses, produce incompatible plans.

Step 1 — establish that the disagreement is real. Often it is not. "You want to strengthen the parents; I want to lower the mother's reactivity" may be the same first move in different language. Ask each clinician to script the first sentence they would actually say. If the sentences are compatible, the disagreement is theoretical.

Step 2 — identify the level of disagreement. There are four possible levels, and they require different responses.

Level 1: language. The same move, described differently. Response: agree the scripted sentence and proceed.

Level 2: sequence. Both moves are needed; the clinicians disagree about which comes first. Response: decide by the family's circumstances — what will be lost if we delay? — and record the decision and the review point.

Level 3: mechanism. The clinicians disagree about what maintains the problem. Response: design an observation that discriminates between them, usually a two-week task with a specific expected result, and agree in advance what each outcome would mean.

Level 4: values. The clinicians disagree about what the family should want, or about what counts as a good outcome — for example, whether a daughter should be supported to leave her family's expectations. Response: name the value disagreement explicitly, take it to supervision, and where it cannot be resolved, refer to the clinician whose values match the family's needs, saying why.

Step 3 — choose and record. Whatever the level, the decision must be made, recorded and given a review point. The commonest failure in integrated teams is an unresolved disagreement that produces a plan with two incompatible parts, which the family experiences as confusion and which nobody reviews.

Step 4 — tell the family what was decided and why. Not the professional disagreement in detail, but the reasoning in plain terms: "We have decided to start with

the two of you, because the mornings are going to be decided by you before they are decided by Amara. In a month we will look at whether that was the right place to start."

The discipline this teaches. Disagreement between models is not an embarrassment to be smoothed over; it is the mechanism by which a team arrives at a better plan, provided it is made explicit and resolved rather than averaged.

13.9 Annotated transcript: switching lens mid-session

A demonstration of a therapist changing frame deliberately, in session three with the Adeyemis. The structural move fails, and the therapist does not repeat it.

> **T1 Therapist:** I would like the two of you to agree one sentence about Monday, in the same words. — Mrs Adeyemi, what would the sentence be?
> **T2 Mother:** "You are going to school on Monday." That is what I would say.
> **T3 Therapist:** And Mr Adeyemi — the same sentence?
> **T4 Father:** (pause) I would say, "Your mother and I have decided, and we will talk about it on Sunday evening."
> **T5 Therapist:** Those are different sentences. They are both firm. One is a command and one is an invitation to negotiate.
> **T6 Mother:** (to father) You always do this. You come home and you make me the bad one.
> **T7 Therapist:** (pause) Let me stop and say what I am seeing. I asked for one sentence, and within a minute we have the argument that I think happens every weekend. Amara, is this what it is like?
> **T8 Amara:** (nods) Every Saturday.
> **T9 Therapist:** Then I want to change what we are doing. I am going to stop asking you to agree a sentence, because agreeing a sentence is not the problem. The problem is that on Saturday you two have a conversation that is really about the last five days, and Amara is in the room for it. So let us do something different. Mrs Adeyemi, when you heard your husband's sentence, what did you feel?
> **T10 Mother:** (crying) That I am doing it alone and nobody sees it.
> **T11 Therapist:** And Mr Adeyemi, when she said "you make me the bad one"?
> **T12 Father:** (long pause) That I am not in my own house.

Annotations. T5 is a structural observation, made neutrally, and it fails: the parents do not hear the distinction between a command and an invitation, they hear an accusation. A therapist committed to the structural frame would repeat the task, perhaps more firmly, and would spend the session managing a conflict that the task has produced.

T7 is the switch, and it is announced. "Let me stop and say what I am seeing" is the sentence that makes the change of frame explicit rather than a loss of nerve. It also recruits Amara as a witness, which serves two purposes: it gives the therapist information and it interrupts the parents' dyad.

T9 moves from structure to emotion, and the move is justified in the therapist's own words: "agreeing a sentence is not the problem." The therapist has used the failure of the structural task as evidence about the mechanism, which is exactly the discipline described in Week 9's testability clause.

T10 and T12 are the payoff. In four turns the session has moved from a disagreement about a sentence to two statements of primary experience — being alone, not being in one's own house — that no amount of structural instruction could have produced. The therapist will return to the sentence next week; it will be easier to agree.

What makes this integration rather than incoherence. The therapist has a primary frame (structural), states the observation from that frame, and then makes a deliberate, announced shift to a second frame (emotion-focused) with a reason. She has not randomly changed technique; she has followed the evidence that the first frame's first move was not working.

What would have made it incoherent. Switching without saying so; switching because the therapist was uncomfortable with conflict rather than because of evidence; and switching back and forth three times in a session, leaving the family unable to tell what kind of conversation they were in.

13.9a Rehearsing the switch: three scripts

Frame shifts are a skill and are learned by rehearsal. Three scripts to practise in pairs, each with a distinct justification.

Script 1 — from structure to emotion, when the structural task produces conflict.

> "I asked the two of you to agree a sentence, and instead we had the argument. That is useful information, so let me stop the task. Before we go back to the sentence, I want to know what happened inside each of you when it started. Mrs Adeyemi, what did you feel when he gave his version?"

Script 2 — from exploration to structure, when the conversation becomes circular and nothing is decided.

> "We have been going round this for twenty minutes, and I notice that each of you has said the same thing three times. I want to change what we are doing. I am going to ask you, as the two adults, to make one decision in this room, now, about Saturday, and I will help you make it. Is that all right?"

Script 3 — from problem talk to exception talk, when the family is demoralised.

> "I want to stop for a moment, because everything you have told me this morning has been about what is not working, and I believe you. I want to ask a different question. In the last month, was there one morning that went better than the others, even slightly? — There was. What was different about it?"

What all three have in common. Each begins by naming what has just happened ("instead we had the argument", "we have been going round this", "everything you have told me has been about what is not working"). Each states the change of direction. Each

gives a reason. And each asks permission or offers a choice. Those four elements are what distinguish a frame shift from a loss of nerve.

13.10 Laboratory: three plans, one decision

The assessed practical, run over two hours with the Adeyemi data set.

Stage 1 (40 minutes) — three one-page plans. Groups of three. Each student takes one lens — structural, Bowen, solution-focused — and writes a one-page plan containing: the formulation in four sentences; the first move, scripted in the words they would use; the second move; what they expect to see within two weeks; and what would show the plan to be wrong. No plan may exceed one page; the constraint forces selection.

Stage 2 (30 minutes) — presentation and challenge. Each student presents for eight minutes. The other two challenge on three grounds only: does the first move follow from the formulation? could the family refuse it without losing face? and what would tell you it is failing?

Stage 3 (30 minutes) — the decision. The group must produce one joint plan, with a stated primary frame, a justification for choosing it over the other two, two elements imported from the other lenses, and a review point at which the choice will be reconsidered. The justification must address the family's circumstances, not only the model's elegance.

Stage 4 (20 minutes) — the family version. Translate the joint plan into the first two minutes of the next session, spoken aloud, in plain language, ending with a question the family could answer with "no".

Marking rubric.

Criterion	Weight	Full marks
Formulation in each lens	20%	Four sentences; the lens's own logic; no contamination from other models
First move, scripted	20%	Specific words; follows from the formulation; the family could refuse it
Falsifiability	20%	A two-week expectation and a stated condition of failure
The decision	25%	A primary frame, justified by this family's circumstances; two principled imports; a review point
The family version	15%	Two minutes, plain language, one answerable question

The commonest failure, and why. Groups produce three good plans and then choose the one they find most comfortable, without justifying it by the family's circumstances. The marker's question is always: "why this family, this week?" A competent answer refers to the school's timetable, the father's availability, the mother's exhaustion, or the risk of losing the family if the first move is too slow. A weak answer refers to the group's preference for the model.

13.10a A fourth lens: the institution's view of the same case

An exercise that changes how students see the whole week. The same family, formulated by the school.

The school's formulation, written by the head of year. "Amara Adeyemi has 61 unauthorised absences since September. Her mother telephones with explanations that are not supported by medical evidence. The family has not attended two meetings. Amara is a capable student whose predicted grades would allow her to study pharmacy. The school's concern is that the pattern has become established, that the family is not engaging with the support offered, and that the education welfare process will be triggered if attendance does not improve by half term. The school's view is that Amara is being supported at home to remain absent."

What the school sees that the therapist may not. The arithmetic of the absences; the timetable of consequences; the predicted grades, which mean that the loss is greater than the family has acknowledged; the fact that the mother has missed two meetings, which the school reads as non-cooperation; and the legal position of the school, which requires it to act.

What the school cannot see. That the mother telephones every morning because she is trying to protect her daughter and herself from the school's judgement; that the two missed meetings were on days when she was on a seven-o'clock shift; that the family has never been asked what they think is happening; and that the letters themselves are part of the loop.

The integration. A therapist who formulates only the family will write a letter to the school asking for understanding, which the school will read as special pleading. A therapist who formulates only the institutional process will pressure the family, which will produce compliance without change. The competent formulation holds both: "the school is escalating on a timetable, the mother is concealing on a timetable, and the two timetables are producing the absences."

The first move that follows. A meeting at the school, with the mother and the therapist, at which three things happen: the mother tells the truth about the mornings; the school names its legal position plainly; and a staged return is agreed with a named member of staff and a fortnightly review. The formulation has become a joint plan because it included the institution as a member of the system.

The lesson for integration. Every lens in this week is a lens on a family. The family in front of you is usually also a case in somebody else's system, and that system has a formulation of its own, a timetable, and a legitimate view. The most useful integration a trainee can learn is not between two therapy models; it is between a therapy model and the institution's account, held at the same time, without either being surrendered.

13.11 Six errors of integration

1. **The incoherent session.** Three techniques from three models in forty minutes, with no frame the family can follow. Fix: name the frame at the start, and announce any change of frame aloud.

2. **The technique without its theory.** Using an enactment without the alliance work that makes it safe, or a miracle question without the solution-building conversation that gives it meaning. Fix: learn the conditions as well as the technique.
3. **Model loyalty disguised as integration.** "I am integrative" meaning "I do what I always do and call it systemic." Fix: audit your last ten sessions for what you actually did, not what you think you do.
4. **Choosing by comfort.** Selecting the lens that suits the therapist's temperament rather than the family's circumstances. Fix: the "why this family, this week?" test.
5. **The unresolved disagreement dressed as both-and.** "We will do structural work and also explore the past," with no decision about which comes first. Fix: state the primary frame and the sequence.
6. **Integration as avoidance of the evidence.** Treating integration as a reason not to follow a protocol where a protocol has strong evidence (family-based treatment for adolescent anorexia, for example). Fix: the model with the strongest evidence for this presentation is the default; integration is a deliberate departure, recorded and reviewed, and a departure that is not reviewed is simply a preference wearing a method's clothes.

13.11a A second data set: Folake and Dele

A second case, shorter, for students who finish early or who want to practise the comparison without the Adeyemi material they have already discussed.

Referral. "Couple, married nine years, two children (6 and 3). Wife requests help because 'we argue about everything and I do not know if I love him any more'. Husband attends reluctantly."

Who attends. Folake (36, bank officer), Dele (39, runs a small transport business). Both children at school and nursery.

What Folake says. "He is never here. When he is here he is on his phone. I do everything — the school run, the homework, the shopping, his mother's hospital appointments. If I ask for anything he says I am nagging. I am tired of being the only adult in this house."

What Dele says. "I work twelve hours a day so that she can have the life she wants. She does not see it. She criticises the one thing I do — my business — and then she says I am not affectionate. When I come home I am finished. I sit down and she starts."

What neither says at first. Folake's mother died eight months ago, and Folake has been managing her mother's affairs and her father's care since. Dele's business lost its biggest contract four months ago, and he has been borrowing to pay wages.

The structural reading. The couple subsystem has collapsed into a work-sharing arrangement with no emotional boundary; the children and Folake's father have been recruited into the mother's support network while Dele is outside it. The first move: a protected weekly conversation between the adults, with the children's and father's needs scheduled rather than urgent, and an agreement about the household tasks that both can hold.

The Bowen reading. Both partners manage anxiety by over-functioning in their own domains — Folake in care, Dele in work — and both experience the other's over-functioning as criticism. Their reactivity is high because both have recently been bereaved or humiliated. The first move: coaching each to observe and lower their own reactivity, and mapping the two family histories of over-functioning.

The solution-focused reading. The couple have several exceptions — a weekend away in June, a fortnight when Dele was at home after the contract was lost, the way they handled the children's illness in the spring — and a clear preferred future that Folake can describe in detail. The first move: build the description of the preferred future and the exceptions into a small, specific experiment.

The emotion-focused reading. Under Folake's criticism is grief and the fear of being alone with her father's decline; under Dele's withdrawal is shame at the loss of the contract and the fear that he is failing as a provider. The cycle is pursue–withdraw, and the losses have intensified it. The first move: slow the cycle, and access the grief and the shame, in that order.

The narrative reading. Two competing stories — "the man who provides" and "the woman who holds everything together" — have become the only stories available, and each requires the other to be inadequate. The first move: externalise the contest between the two stories, and find the occasions when the couple have lived by a third story.

The exercise. Choose one of the five lenses and write the first two minutes of the session, scripted. Then write the sentence you would say if the family asked, "how long will this take?" The answer should reflect the formulation, not the model's marketing.

13.12 Culture, faith and the choice of lens

Model choice is not culturally neutral, and the three lenses differ in what they assume about families.

The structural lens assumes a hierarchy the therapist may need to change. In families where the hierarchy is strongly normative and religiously grounded, "strengthen the parental subsystem" may mean something different from what the therapist intends: it may mean supporting the father's authority in a way that reduces the mother's. The lens must be used with an explicit question: which hierarchy does this family recognise, and does the mother have a place in it that she considers legitimate?

The Bowen lens assumes that differentiation is a good. Families that value interdependence, duty and the subordination of individual preference to family obligation may experience coaching towards differentiation as an invitation to selfishness. The lens can be used, and it must be translated: "managing your own reaction" can be presented as a duty to the family rather than a right of the individual.

The solution-focused lens assumes that the family can describe a preferred future. Families in acute crisis, or families whose difficulties are primarily material, may find the question abstract. The lens works best after the practical questions have been answered.

Faith and the choice of lens. Where a family's account is spiritual, all three lenses are secondary to the family's own framework, and the therapist's task is to work within it rather than to replace it. A structural intervention framed as "the family's covering" and a solution-focused question framed as "what would give evidence that the prayer is being answered" are both legitimate; a lecture on systems is not.

> **DEBATE**
> "Any model, competently delivered, will help most families; the model is the therapist's preoccupation, not the family's." Argue for and against, using this week's case.

13.13 Evidence note: does integration work?

A short note, because students often assume that integrative practice is obviously superior.

What is known. Comparative trials between bona fide family therapies generally find small or no differences in outcome. There is no substantial evidence that integrative practice outperforms single-model practice; nor is there evidence that it is worse. The evidence on therapist effects (Week 11) suggests that who delivers therapy and how well matters more than how many models they use.

What is plausible. Integration may improve fit — the match between what the family needs and what the therapist offers — and fit is associated with engagement and retention. Integration may also help the therapist persist when a single-model first move fails, which is the mechanism illustrated in 13.9.

What is risky. Integration without a primary frame is associated with drift: the therapist changes approach whenever the session becomes difficult, which teaches the family that difficulty is to be avoided. This is a well-recognised failure mode in supervision, and it is why the primary-frame rule exists.

The defensible claim. Integration is a professional skill that improves fit and persistence; it is not a treatment with an evidence base of its own. Claims for integrative superiority should be treated with the same scepticism the field applies to claims for model superiority, and the clinician's own outcome data should be the arbiter.

13.13a Two transcripts of the same moment, in two frames

A comparison to end the week, using one thirty-second fragment of a session with the Okonkwos, and two therapists' responses to it.

The fragment. Ebuka (9) says: "I do not want to talk about my grandmother."

Response A, structural. "All right. Then let us talk about the mornings. Ebuka, when your mother comes into your room at six, what does she do first?" What this does: respects the refusal, does not pursue the affective material, and returns the session to the behavioural sequence that the formulation identified as the lever. What it costs: the moment of grief is left unaddressed, and Ebuka learns that the therapist will not stay with it.

Response B, emotion-focused. "You do not want to talk about her. That is allowed. Can you tell me what it is like when somebody says her name — not about her, about what happens in your body?" What this does: stays with the refusal and makes it speakable; offers Ebuka a way to be in the feeling without being overwhelmed. What it costs: if the alliance is thin or the session is early, Ebuka may shut down, and the therapist will have pushed too soon.

Which is right? It depends on four things, and a trainee should be able to name them: the strength of the alliance; the session number; whether the parent in the room can tolerate the feeling; and whether the family has already had a chance to address the loss. In session two with a mother who has not yet wept, response A is usually correct. In session six, with an alliance and a mother who has described her own grief, response B is usually correct.

The point of the comparison. Neither response is "the integrative one". Integration is not the habit of doing a little of each; it is the capacity to choose, on evidence, and to know what the choice costs. That capacity is developed in supervision, in recorded sessions, and in exercises like this one, and it is the subject of the final week of the course, where the question is not which lens you can use but which kind of clinician you are becoming.

13.14 Key terms

Term	Definition
Assimilative integration	A primary model extended with techniques from others
Eclectic integration	Selection from several models without a single organising frame
Pluralistic practice	A meta-framework that negotiates methods with the client's preferences and the evidence
Primary frame	The model that organises the therapist's formulation and first move
Frame shift	A deliberate, announced change of model within or between sessions
Drift	Changing approach whenever the work becomes difficult, without justification
Fit	The match between what the family needs and what the therapist offers
Lens	A model understood as a way of seeing rather than a set of techniques
Falsification condition	What would show the chosen formulation or plan to be wrong
Sequencing	The decision about which frame or intervention comes first

13.15 Self-test

1. Name the three kinds of integration and one risk of each.
2. Why should a trainee begin with assimilative integration?
3. What are the four tests of a defensible integration?

4. Give the structural first move with the Adeyemis and the condition under which it would be falsified, and say which piece of evidence in the case would most quickly tell you it was the wrong choice.
5. What does the Bowen lens see that the structural lens misses, and vice versa?
6. What is the characteristic blind spot of the solution-focused lens?
7. Why must a frame shift be announced aloud?
8. What does the evidence say about integrative practice, and what is the one defensible claim a clinician can make for it?

Answers. 1. Assimilative (risk: borrowed techniques used without their theory); eclectic (risk: incoherence); pluralistic (risk: the therapist's expertise dissolving into consumer choice). 2. Because a primary model gives the trainee something to be wrong about, and therefore something to learn from. 3. A stated primary frame; the ability to say which frame is operating and why; borrowed techniques retaining their enabling conditions; the family experiencing one coherent conversation. 4. A session with the two parents to agree one sentence about Monday; falsified if the mornings do not change at all within two weeks. 5. Bowen sees multigenerational anxiety and the triangle, and misses the institutional timetable and practical conditions; structural sees hierarchy, alliances and immediate leverage, and misses grief, meaning and the school loop. 6. It may bypass the function of the symptom and the history that sustains it. 7. Because an unannounced shift leaves the family unable to tell what kind of conversation they are in, and it conceals drift from the therapist herself. 8. There is no evidence that integration outperforms single-model practice; it may improve fit and persistence, and it carries a risk of drift.

13.16 Reflective task

Read the Adeyemi data set again, and write down the first three things you noticed. Then compare with what you noticed on the first reading during the lecture. Most students notice different things on the second reading, and the difference is a portrait of their own attentional habits: some notice hierarchy, some notice feeling, some notice what is missing. Write 300 words on what your attention goes to first, and what it costs you.

13.17 Readings for this week

- **Lebow (2014)**, Couple and Family Therapy: An Integrative Map of the Territory, for a systematic comparison of the models on one grid.
- **Nichols & Davis**, the chapters on each of the three models in this week, read side by side rather than sequentially.
- **Minuchin (1974)** on structural technique, and **Bowen (1978)** on differentiation, read for the first moves each author describes.
- **de Shazer (1988)**, Clues: Investigating Solutions in Brief Therapy, for the solution-focused interview in detail.
- **Johnson (2004)**, The Practice of Emotionally Focused Couple Therapy, for the enactment and the cycle.

- **Sprenkle, Davis & Lebow (2009)**, Common Factors in Couple and Family Therapy, for the argument that model allegiance is the least important variable in the room, and for the counter-argument that follows from it.
- **The transcript of your own most recent session**, watched with one lens at a time; there is no better exercise in this week than watching yourself work and asking which model your eye was following, then watching it again with a second lens and counting the moments you missed.

Next: Week 14 — Integration II: your emerging clinical stance

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Week 14 — Integration II: your emerging clinical stance

Write a two-page **position statement**: what you believe about how relational problems develop, what you think therapy does, what you will not do, and what you still do not know. This is not an exam answer; it is the beginning of a professional identity that will be revised for the rest of your career.

14.1 The lecture in ninety minutes

Minutes	Segment	What the lecturer does	What students do
0–10	What a stance is, and what it is not	Distinguish stance from model, from opinion, from slogan	Identify stance statements in three quotations
10–25	Five commitments	Theoretical, epistemological, ethical, cultural, acknowledged limit	Draft one sentence for each
25–40	The annotated statement	Read a strong statement and mark its parts	Mark their own draft against it
40–55	Two statements compared	A structural statement and a narrative statement, defended	Argue for the one they do not hold
55–70	The viva	Demonstrate a challenge to a stance, and a response	Take five minutes of challenge each
70–85	The development plan	Supervision, CPD, log, review	Write the first six months
85–90	Endings and continuation	How to leave a course, and a family, well	Plan the next review date

14.2 What a stance is

A clinical stance is the set of commitments that determine what you notice, what you do, what you refuse, and what you are willing to be wrong about. It is not a model, although it includes one. It is not an opinion, because it is testable against practice. It is not a slogan, because it can be stated in sentences that could be contradicted.

What a stance is not, in four common counterfeits.

The badge. "I am a systemic therapist." A model's name is not a stance; it is an affiliation. The stance question is what you believe the model is true about.

The list. "I am integrative and draw on structural, narrative and solution-focused approaches." This is a catalogue, not a position; it does not say what you would do when the approaches disagree.

The aspiration. "I want to be a warm, non-judgemental therapist." A virtue, not a stance; it does not say what you would do with a family that requires a judgement.

The certainty. "I know that diagnosis is harmful." A belief held without a falsification condition, which Week 11 taught you to distrust, whichever direction it points.

What a stance does. It makes your work predictable to yourself and legible to others. It gives a supervisor something to challenge. It gives a family an account of what they are being offered. And it gives you a way of noticing when you have drifted from what you believe.

14.2a Where a stance comes from

A stance is not invented in the final week; it has been forming since the first. Four sources, which students should be able to identify in their own development.

Source 1 — the model you were taught best. Most trainees adopt, at least initially, the frame of the teacher who was clearest, or the supervisor who was kindest. This is not a failing; it is how professional identity begins. The question for the final week is whether the adoption was examined or merely absorbed.

Source 2 — the families who defeated you. Every trainee has two or three cases that did not work, and those cases shape a stance more powerfully than the successful ones. A therapist whose first family disengaged during a structural enactment may become a therapist who never uses enactment; the mature version of that experience is to learn what the enactment requires and to use it better, not to abandon the technique.

Source 3 — your own family. Nobody arrives neutral about hierarchy, loyalty, caregiving, conflict or disclosure. The therapist who was parentified as a child will have a particular sensitivity to parentified children, and a particular risk of over-identifying with them. The genogram of Week 8 and the reflection of Week 10 exist for this reason; the stance should be written with knowledge of the writer.

Source 4 — the evidence and the arguments. Week 11 is a source of a stance, not only of information. A therapist who has read the common-factors literature and the specific-factors literature and still believes that the model is the main thing has made a choice; a therapist who has read both and concluded that alliance, fit and technique each matter in different proportions has made a different choice. Both are positions; only one is defended by the evidence as it stands.

The exercise. Identify, in one paragraph each, the teacher, the case, the family pattern and the argument that have most shaped your current position. Most students find that the case they remember is the one that failed, and that the paragraph about their own family is the hardest to write.

14.3 The theoretical commitment

The question. How do you think relational problems develop and change?

What a strong answer contains. A claim about causation (circular, developmental, structural, narrative, or an explicit combination); a claim about what maintains difficulties (the mechanism); a claim about what therapy does (what changes, and how); and an acknowledgement of the model's limits.

Two examples, contrasting.

A structural statement. "I think relational difficulties are maintained by the organisation of the family: by hierarchies that are unclear or inverted, by boundaries that are too diffuse or too rigid, and by cross-generational alliances that recruit children into adult conflicts. Therapy works by changing the organisation — by strengthening the parental subsystem, clarifying boundaries and moving decisions to the people who should be making them — and by giving the family a different experience of itself in the room. This account is strong where the difficulty is maintained by structure and weak where the difficulty is maintained by meaning, loss or biology, and I use it with those limits in mind."

A narrative statement. "I think difficulties are maintained by the stories that families and institutions tell about their members, and by the practices that those stories make possible. Therapy works by making the stories visible, separating persons from problems, and thickening the accounts of what families value and have already done. This account is strong where the difficulty is maintained by meaning and by the descriptions that others hold of the family, and weak where the difficulty is maintained by material conditions, by acute risk or by biology."

Why the limits must be in the statement. A commitment without limits is an ideology; a commitment with limits is a professional position. The examiner's question will be: "where does this account fail?" and the answer must be in the document.

14.4 The epistemological commitment

The question. How do you know what you know, and how do you know when you are wrong?

Why it is the hardest of the five. Most trainees can state a theory; fewer can state an epistemology, and the epistemological commitment is what distinguishes a clinician from an enthusiast.

Four positions, and how to choose.

Empiricist. Knowledge comes from observation and measurement; the therapist's job is to test hypotheses against data, including outcome data. Strength: accountability, and a defence against conviction. Weakness: the risk of treating what can be measured as what matters.

Constructivist. Knowledge is constructed in language, and there is no view from nowhere; the therapist attends to meanings and to the family's own account, and holds their own account as one account among several. Strength: humility and attentiveness to power. Weakness: the risk of relativism, and of losing the capacity to say that a child is unsafe.

Critical realist. There is a reality independent of our descriptions, and our descriptions are always partial and positioned. The therapist works with hypotheses that are more or less adequate, tests them, and expects to revise. Strength: it holds measurement and meaning together. Weakness: it is harder to state in a sentence.

Pragmatist. What matters is what works, for this family, in this setting; truth is assessed by consequence. Strength: flexibility and a focus on outcomes. Weakness: "what works" for whom, and by whose criteria.

A workable formulation for a first statement. "I work as a critical realist. I assume there is something real happening in this family that my description can be more or less adequate to; I hold my formulation as a hypothesis, test it against what the family says and does and against measured outcome, and I expect to be wrong at least once in every case."

The operational test. Your epistemology is visible in three behaviours: what you do when the family disagrees with your formulation; what you do when the outcome data say you are failing; and whether you can say "I do not know" in a session without filling the silence.

14.5 The ethical commitment

The question. What will you not do, and what will you do even when it costs you the case?

What a strong answer contains. A statement about the family's welfare taking precedence over the therapist's comfort or the model's coherence; a statement about honesty with families, including about the limits of confidentiality and about the therapist's own uncertainty; a statement about competence, including what you will refuse to take on; and a statement about your own accountability — supervision, records, complaints.

A worked example. "I will not work with a couple where there is coercion and fear, in joint sessions, whatever the model suggests, because the safety of the person who is afraid takes precedence over the technique. I will tell a family the truth about the limits of confidentiality before I act on them, even when telling them may cost me the work. I will not accept a case beyond my competence in order to avoid appearing inadequate, and I will say so to a referrer in writing. I will keep records that the family could read, and I will take every serious decision to supervision."

Why the negatives matter more than the positives. "I will be warm and respectful" is unfalsifiable and costs nothing. "I will not run joint sessions where there is coercion" is specific, costly, and therefore a real commitment. An ethical statement without a refusal in it is a greeting card.

14.5a A supervision transcript: a stance under pressure

A five-minute excerpt from a supervision session, in which a trainee's stated commitment meets a case that tests it. The trainee has said, in her position statement, that she will not run joint sessions where one partner is afraid.

> **T1 Supervisor:** Tell me about the Ogundipe case.
> **T2 Trainee:** Couple, married fourteen years. She says he has pushed her twice, both times when he had been drinking. He says she "goes on and on" until he loses his temper. They want to stay together. The referrer has asked for couple work.
> **T3 Supervisor:** And what does your stance say about that?
> **T4 Trainee:** That I do not see them together while she is afraid. But the referrer has asked specifically, and there is no other service, and she says she does not want to leave him.
> **T5 Supervisor:** So which of those sentences is your commitment and which is the pressure?
> **T6 Trainee:** (pause) The commitment is the first one. The rest is the pressure.
> **T7 Supervisor:** Good. Now — what does your commitment actually leave you able to offer? Not what it forbids.
> **T8 Trainee:** I could see her individually, and him individually, and be clear with both about why. I could write to the referrer explaining the reasoning. I could look for a programme for men who use violence. And I could reassess if the fear went away.
> **T9 Supervisor:** What would have to be true for you to reassess?
> **T10 Trainee:** She would have to tell me, alone, that she is not afraid of him. And I would have to believe her, which is not the same thing.
> **T11 Supervisor:** That last sentence is the most professional thing you have said this morning. Write it down and put it in your statement.

Annotations. T4 is the moment every trainee recognises: the commitment stated in the abstract colliding with a service pressure, a family's preference and a real absence of alternatives. The temptation is to negotiate the commitment away.

T5 is the supervisory question that separates conviction from pressure, and it is worth memorising, because it can be asked of oneself.

T7 is the crucial move, and the one trainees least expect. A commitment that only forbids leaves the therapist with nothing to offer, and the family experiences the ethical position as abandonment. The supervisor's question — "what does it leave you able to offer?" — converts a prohibition into a service.

T10 is the operational criterion, and T11 is the supervisor's recognition that a stance develops through sentences like this one, not through reading.

The general lesson. A commitment is tested by the cases in which honouring it costs something. The trainee who can name what she will offer instead, and what would change her mind, has a stance. The trainee who can only repeat the rule has a slogan.

14.6 The cultural commitment

The question. How will you work across difference, and what will you do about your own position?

What a strong answer contains. A statement of your own cultural location, honestly described, including the advantages it confers; a statement about the method you will use to learn about families who are not like you (asking, not assuming); a statement about the

power in the room and how you will use it; and a statement about the limits of your competence across difference, with a plan.

A worked example. "I am a middle-class, university-educated woman from a Yoruba family, working in a city where most of the families I see are poorer than I am and many are from other ethnic groups. I will ask about the family's own arrangements rather than infer them, I will not use the family to educate me, and I will notice when I am working harder with a family because of their background — which is a form of treating them differently. I will seek supervision from colleagues with knowledge I do not have, and I will record when I have adapted an approach and why."

The trap to avoid. The cultural commitment that consists entirely of good intentions ("I will be culturally sensitive and aware of my biases") is the least useful sentence in the document. The commitment must specify behaviour: what you will ask, what you will not assume, and who you will consult.

14.7 The acknowledged limit

The question. What do you not know, what are you not yet able to do, and how will you know when you have reached the edge of your competence?

Why this section is the one examiners look for. Because a position statement without a limitation is a performance. The limitation section is the place where a trainee demonstrates that they can hold their own practice as an object of scrutiny, which is the whole point of a professional training.

What a strong answer contains. A specific gap (not "I have much to learn"), with its consequences for a family; the mechanism by which you will notice it (supervision, outcome data, a colleague's feedback, a pattern in your caseload); and the plan for closing it.

Four examples of specific limitations.

"I am not yet reliable with high-conflict separated parents. I become anxious when the conflict is in the room and I try to smooth it, which deprives the children of the adult who can hold the difficulty. I will notice it in supervision, where my supervisor has already begun to point it out, and I will take the MFT 719 module and seek co-working on two such cases this year."

"I over-function with mothers. The pattern is visible in my caseload: I give more advice to mothers than to fathers, and my session recordings show me asking mothers three times as many questions. I will count the questions in my next ten recorded sessions and review them with my supervisor."

"I do not work well with families whose first language is not mine without an interpreter, and I have twice proceeded without one in order not to lose the appointment. I will not do that again; I will reschedule, and I will record the reason."

"I am not competent to assess risk of serious violence, and I know that I sometimes reassure myself that a situation is 'probably fine'. My plan is to use the structured screen

every time, to consult the safeguarding lead whenever the screen is positive, and to say in supervision when I have noticed myself wanting to move on."

The test of a limitation statement. Could a colleague observe you and tell whether you had addressed it? If not, it is a confession rather than a plan.

14.8 Writing the statement: structure and a worked model

The structure, in six sections and two pages.

1. **What I believe about how relational problems develop and change** (400–500 words). The theoretical commitment with its limits.
2. **How I know what I know** (250–350 words). The epistemological commitment, with the operational test.
3. **What I will not do** (250–350 words). The ethical commitment, in refusals.
4. **How I will work across difference** (250–350 words). The cultural commitment, in behaviours.
5. **What I do not yet know, and how I will find out** (250–350 words). The limitation with a plan.
6. **What therapy is for** (150–250 words). The sentence that holds the whole document together.

The final section, which students find hardest. "What therapy is for" requires a value claim, not a method claim. Two examples. Example A: "Therapy is for restoring a family's capacity to go on — to face what is happening without splitting into blame, and to make the next decision together." Example B: "Therapy is for widening the range of stories a family can live by, so that a member is not reduced to the problem that brought them."

A worked model statement, shortened. The following is a complete statement at about 900 words; the assignment requires two pages, and the model is provided to show the shape.

> What I believe. I believe that most of the difficulties I will be asked to help with are maintained by patterns that no single person intends: cycles of pursuit and withdrawal, coalitions across generations, stories that have hardened into descriptions of a person, and the practical arrangements — work, housing, money, migration — that make some patterns inevitable. I believe the pattern is the unit of intervention, and the person is the unit of respect. I believe therapy changes patterns by making them visible and by giving the family a different experience of itself, usually before it has a different understanding of itself. I believe this account is strongest where the difficulty is maintained relationally, weaker where it is maintained biologically, and weakest where the family's problem is material and my room is the only place they can talk about it. How I know. I work as a critical realist: I assume something real is happening, that my description of it is partial, and that the family's description is also partial. I test my formulation by its consequences — does the family recognise it, does the first move produce the change I expected, do the measures move — and I expect to revise it. My operational test of this commitment is what I do when I am wrong in the room, which should be to say so and to ask, not to explain. What I will not do. I will not run joint sessions where one partner is afraid of the other. I will not keep a secret that changes the work. I will not tell a family that their problem is cultural when I have not asked. I will not accept a case beyond my competence in order to avoid seeming inexperienced, and I will say so in writing to the referrer. I will not write a sentence in a record that I would not say to the family's face. How I will work across difference. I will ask about the family's arrangements — who decides, who is consulted, who is raising the children, what may not be spoken outside — rather than infer them from their background. I will notice when I am working harder with a family because of who they are, and I will take that to supervision. I will not use a family to educate me. What I do not yet know. I am not yet reliable when a family's anger is directed at me. My recordings show me becoming conciliatory and losing the thread of the session. My plan is to review three such recordings with my supervisor this term, to practise staying in the room with the anger in role-play, and to ask for feedback from a co-worker on two live cases.

Why this model works, and what it lacks. It is specific, it contains refusals, it contains a falsification condition, and it contains a limitation with a plan. It lacks the breadth that a full two pages allows — in particular, a fuller account of the family's own agency and of the therapist's use of self — which is exactly what the assignment requires the student to supply.

14.9 Two statements compared

Two students in the same cohort, both competent, with very different positions. Reading them side by side demonstrates that a stance is a choice, not a template.

Statement A — the pragmatic structuralist. "I believe families get stuck in arrangements that nobody chose: a mother managing alone, a father at the edge, a child carrying the worry. I believe the most useful thing I can do is to change the arrangement — who sits where, who decides what, who speaks to whom — and that insight usually follows change rather than causing it. My evidence is behavioural: does the morning go differently, do the parents agree a sentence and hold it, does the child get to be a child

again. My limits are with families whose difficulty is primarily about meaning or loss, where I tend to move too fast to a task."

Statement B — the meaning-oriented narrative therapist. "I believe people are not the problems they arrive with, and that families are often trapped by descriptions that others — schools, relatives, clinics — have helped to write. I believe the most useful thing I can do is to make those descriptions visible, to separate the person from the problem, and to find the occasions when the family has already lived by something better. My evidence is in what the family can say about itself at the end of the session that it could not say at the beginning. My limits are with acute risk and with families whose difficulties are material, where a conversation about meaning is a luxury and a practical move is required."

What the comparison teaches. Both statements are defensible, both contain limits, and both could be falsified: A's by a family whose morning improves while the meaning does not, B's by a family who can retell their story and still cannot get to school. The examiner's question — "which family would you send to the other clinician?" — is answerable from the limits section, which is why the limits section is the most useful part of the document.

14.9a The statement as a working document

Students treat the position statement as an assignment to be submitted and forgotten. It is more useful than that.

Use it in supervision. Give it to your supervisor in the first month of placement and ask them to hold you to it: "you said you would not run joint sessions where there is coercion; tell me when you see me drifting." A supervisor who has your statement can challenge you with precision, which is far more useful than generic feedback.

Use it with families. Not the whole document, but its operative sentences. A therapist who can say, in the first session, "I work in a particular way: I will want to hear from everyone, I will be direct about what I see, and I will tell you if I think we are not making progress" is giving the family something to consent to. That is a statement of stance in ninety seconds, and it builds the alliance more effectively than warmth alone.

Use it when choosing a job or a training. A service that cannot accommodate your stated commitments — because its model, its waiting list or its management will not tolerate them — will require you to compromise them daily. Reading your own statement against a job description is a professional act, not an indulgence.

Use it when you are stuck. When a case is not moving, the statement is a diagnostic instrument. Which of your commitments is the case testing? Which limit have you reached? Which refusal are you being tempted to abandon? Most impasses in clinical work are a stance problem before they are a technique problem.

Use it when you are challenged. The statement gives you something to defend and something to revise. A therapist without one defends their habits; a therapist with one can say "that is a fair challenge to my epistemology, and I will think about it", which is the most professional sentence in the language.

14.10 The viva: defending a stance under challenge

The position statement is examined in a twenty-minute viva. What is assessed is not whether the examiner agrees with you but whether you can hold a position under pressure without rigidity or collapse.

The seven questions examiners ask, and what each is testing.

1. **"Where does your account fail?"** Tests whether you have a limits section you actually believe.
2. **"Give me a family you have seen where your approach would not have worked."** Tests whether the stance is connected to practice, or is an essay.
3. **"What would you do if the family rejected your formulation?"** Tests the epistemological commitment in action.
4. **"You say you will not do X. What if the family asks for X and there is no one else?"** Tests whether the ethical commitment survives pressure; the correct answer names what you would do instead, and what you would refuse.
5. **"How do you know you are helping?"** Tests whether measurement is part of your practice or a phrase in the document.
6. **"Who are you in the room, and how does that shape the work?"** Tests self-knowledge and the use of self.
7. **"What will you have changed your mind about in five years?"** Tests whether the stance is alive.

An annotated answer to question 4. Challenged: "You say you will not run joint sessions where there is coercion. This couple has been referred by a court and there is no other service. What do you do?"

> "I do not run joint sessions where one partner is afraid, because the risk of an escalation after the session is greater than the benefit of the joint work, and because a person who is afraid cannot speak freely in the same room. What I would offer instead is individual work with each partner, a group programme for the person using violence where one exists, a written report to the court explaining what I am offering and why, and a clear statement to both partners about what I will and will not do. If the court requires joint work as a condition, I would explain in writing why I am not the right clinician for it."

Why this answer is strong. It names the refusal, gives the reason, offers alternatives, and takes the consequence. It does not waver and it does not moralise.

The three failures in a viva.

Rigidity. Answering every challenge with a restatement of the position, which suggests the position has never been tested.

Collapse. Abandoning the position at the first challenge ("you are right, perhaps I should be more flexible"), which suggests the position was borrowed for the occasion.

Deflection. Answering a question about limits with a statement of strengths, which is the commonest and the easiest to spot.

14.10a A worked viva, in full

The examination, condensed to eight exchanges, with the board's assessment at the end.

> **EXAMINER:**
> Your statement says you work structurally and that your model fails where meaning is the problem. Give me a family where it failed. **Candidate:** A couple where the wife had a stillbirth two years ago and both had stopped speaking about it. I reorganised the household tasks, which worked, and the silence remained, and I did not know how to enter it. I kept doing structural work because it was producing change, and I mistook change for progress. **Examiner:** What would you do now? **Candidate:** I would name the silence in the first or second session and ask what the couple had been told about talking about it. I would use the enactment to slow the moment down rather than to solve the tasks. **Examiner:** You say you will not keep a secret that changes the work. A mother tells you her son is using drugs and asks you not to tell the father, whom she says will beat him. What do you do? **Candidate:** I tell her, in that session, that I cannot hold it, and why; I ask her consent for a joint session in which she tells him with me present; I check the risk to the son before that session; and if she refuses, I say what I will do next and by when. I would also consider whether the father's response constitutes a safeguarding risk in itself, which it may. **Examiner:** How do you know you are helping? **Candidate:** Session-by-session measures from each member, a goal set in the family's words, and a review point at which I say out loud whether we are on track. And my supervisor sees one recording a month. **Examiner:** What will you have changed your mind about in five years? **Candidate:** I expect to have changed my mind about how much I use enactment early in the work. My recordings show me using it too soon with families who are not yet safe enough with each other, and I expect that to be the thing I look back on as naive.

The board's assessment. Three criteria, typically. Coherence: do the answers follow from the statement? Specificity: are there cases, words and dates, or only abstractions? Openness: does the candidate hold the position without defending it against evidence?

The candidate's strongest move, and its lesson. In answer two, she said "I mistook change for progress". That sentence demonstrates that the failure was examined rather than survived, and it is the kind of sentence that distinguishes a reflective practitioner from a competent technician. The lesson for the reader is not to memorise it but to find the equivalent sentence in your own practice before the viva, because the examiners will look for it.

14.11 The development plan: the first six months

A stance without a plan is a wish. The final section of the assignment is a development plan with four elements.

Supervision. Who, how often, what for, and what you will bring. A plan that says "I will attend supervision" is not a plan. A plan that says "I will bring one recorded session each

month, chosen for difficulty rather than success, and I will ask my supervisor to challenge my over-functioning with mothers" is.

Continuing professional development. Three specific activities with dates: a course, a reading programme, a conference. Each should address the limitation identified in 14.7, not merely the therapist's interests.

Outcome measurement. What you will measure, with what instrument, at what interval, and what you will do with the data. A clinician who commits to measuring outcomes in the first six months will still be measuring in five years; one who intends to begin later usually does not.

The record of practice. A log of sessions with the eleven fields listed in the course requirements. The log is not an administrative burden; it is the instrument of reflective practice, and the students who keep it well are, in the experience of most programmes, the students who develop fastest.

The fourth element that students forget: rest and support. The first six months of practice involve a volume of emotional exposure that most trainees underestimate. A plan that does not include protected time, peer support, personal therapy where appropriate, and a limit on the number of complex cases carried simultaneously is a plan for burnout. This is not a soft addition; it is a competence, and it belongs in the document. The clinicians who leave the profession in their first five years rarely do so because they lacked technique; they leave because they had no plan for the emotional load, no peer group, and no permission to be finite, which is a lesson the profession teaches too late and too often by losing people it could have kept.

14.11a Where this course leads

The final act of integration is to see the programme as a whole, and MFT 701 as its foundation.

The sequence. MFT 702 takes the models introduced in Week 13 and works through each in depth, with its own formulation, techniques and evidence. MFT 703–706 build the practical competences — assessment, working with couples, with children, with the wider system. MFT 707–708 address the contexts in which families live: health, schools, faith, and the legal system. The second semester's modules (709–715) develop specialisms and research skills; the third (716–721) returns to complex presentations and to integration at an advanced level. The thesis (770) is where a student's own stance is tested against data.

What MFT 701 supplies to each. To 702: the vocabulary and the capacity to compare models. To 703–706: the first session architecture, the negotiated definition, the genogram and the formulation template. To 707–708: the cultural and ethical frame, and the discipline of writing for institutions. To 709–715: the evidence literacy of Week 11 and the habit of measurement. To 716–721: the ability to say what the systemic frame can and cannot do. To 770: the capacity to hold a hypothesis and to test it.

What the student must supply. Three habits, which no module can teach by itself: the habit of recording sessions and reviewing them; the habit of measuring outcomes and reading the data honestly; and the habit of taking the difficult case to supervision rather

than carrying it alone. A graduate with these three habits will outgrow the curriculum. A graduate without them will have learned a vocabulary.

14.12 Ending well: how the course, the placement and the therapy end

The final clinical skill of the course is the one least taught: how to end.

Ending with a family. Endings should be planned, named in advance, and reviewed. Four elements.

Name it early. "We said we would review in eight sessions; this is the eighth, so today we will look at what has changed and decide together what happens next."

Review the work, with the family's own data. The measures, the goals of Week 7, the family's own statements. Ask what helped, what did not, and what they would say to another family in the same position.

Address the ending itself. Families often have strong feelings about ending, especially families for whom endings have been losses. Ask directly: "What is it like to be finishing?" and be prepared for anger, relief, or a new problem that appears in the penultimate session.

Plan the future. What the family will do if the difficulty returns; who to contact; whether a review session in three months would be useful; what the general practitioner will be told.

Ending badly, and the four common forms. Drift, where sessions become less frequent and stop without a conversation; the unplanned termination, where a family stops attending and the therapist does not follow up; the ending in crisis, where a new problem is produced to avoid the ending; and the ending in disappointment, where the therapist communicates that the family failed to use the work.

Ending the course. The same principles apply. Name the ending; review what has been learned; acknowledge what has not; and plan the next stage. A training that ends in exhaustion and relief is a training that has not taught endings. The final week is designed as a review, not a race.

Ending with a supervisor. Supervision ends too, and the last session should include an honest account, from both directions, of what the supervisee has and has not developed. The habit of asking a supervisor "what would you say I still cannot do?" is one to carry through a career.

14.12a The last session with a family, annotated

An ending, as it should be done, in eight turns.

> **T1 Therapist:** We agreed eight sessions, and this is the eighth, so today I want to look at what has changed, what has not, and what happens next. Is that all right with everyone?
> **T2 Mother:** It is. I am nervous.
> **T3 Therapist:** Tell me about the nervousness.
> **T4 Mother:** I am afraid we will go back to how it was.
> **T5 Therapist:** That is a reasonable fear. Let us look at the evidence, because you have some now. Your ratings went from 28 to 17. Amara has been in school for six weeks. The two of you have made three decisions without a shouting match. What would you say made the difference?
> **T6 Father:** We stopped waiting for her to change first.
> **T7 Therapist:** Amara, what would you say?
> **T8 Amara:** (pause) My mum stopped shouting in the morning. So I stopped pretending to be asleep.

[Twelve minutes later, after the family's review.]

> **T9 Therapist:** Before we finish, I want to ask what it is like to be finishing, for each of you.
> **T10 Mother:** I do not want to stop.
> **T11 Therapist:** That is worth saying, and worth taking seriously. We agreed a review in three months; if something happens before then, you can telephone me, and here is what to do if it is urgent. What would you want me to know if you came back?
> **T12 Amara:** That it is not only me.

Annotations. T1 names the ending at the start of the session, which gives the family the whole session to prepare. T5 reviews the work with data and then asks the family to explain the change, which is the single most powerful thing an ending can do: it leaves the family with its own theory of its own improvement.

T8 is the sentence the therapist will remember, and it is also clinical information: the change was systemic and bidirectional, and the family has described it.

T9 addresses the ending directly. The therapist does not reassure the mother at T10; she takes the feeling seriously and converts it into a plan. T11 names the revisit route and the emergency route separately, which is what a family needs in order to end safely.

What the therapist did not do. She did not add a ninth session because the family was anxious; she did not offer a new goal to soften the ending; and she did not tell the family that they would be fine. Endings are managed by naming them, reviewing them and planning after them, not by avoiding them.

14.13 Six errors in developing a stance

1. **The borrowed stance.** Adopting the position of a charismatic trainer without testing it against your own case material. Fix: write the statement from your own sessions, with examples.
2. **The statement without refusals.** Ethics as aspiration, with nothing you would decline to do. Fix: at least three specific refusals, each with a reason.

3. **The unexamined epistemology.** A theory with no account of how you know when you are wrong. Fix: state the operational test — what you do when the family disagrees, when the data fail, and when you do not know.
4. **The universal claim.** A model presented as adequate for everything. Fix: the limits section, with a named presentation where the model fails.
5. **The performance of humility.** A limitation section that is really a strength ("I care too much"), which no colleague could observe and no family could be protected from. Fix: a limitation a supervisor has already named, with a plan and a date.
6. **The stance that never changes.** A statement written once and never revisited. Fix: an annual review, dated, with what changed and why.

14.14 Key terms

Term	Definition
Clinical stance	The set of commitments that determine what a therapist notices, does, refuses and doubts
Theoretical commitment	A claim about how difficulties develop and change, with its limits
Epistemological commitment	An account of how you know, and how you know when you are wrong
Ethical commitment	What you will not do, and what you will do at cost
Cultural commitment	How you will work across difference, specified in behaviours
Critical realism	The position that there is a reality our descriptions are more or less adequate to
Falsification condition	What would show a belief or plan to be wrong
Development plan	Supervisory, educational, measurement and support commitments with dates
Planned ending	An ending named in advance and reviewed with the family
Reflective practice	The disciplined examination of one's own work, usually with records and supervision

14.14a The three-year review: keeping a stance alive

A stance is revised, and the revision is the evidence that it was real. A structure for the review, to be done annually or at the end of each year of practice.

Review 1 — the case that changed your mind. Name one case from the year in which you were wrong about something important: the mechanism, the family's capacity, your own reaction, the pace. Write 300 words on what you now believe differently. If there is no such case, the year has not been examined.

Review 2 — the pattern in your caseload. What kinds of families do you see most, and what kinds do you avoid or lose early? A caseload is a portrait of a therapist's limits. Compare your outcomes for the families you find easy and the families you find difficult; the difference is your development agenda.

Review 3 — the outcomes, honestly. What proportion of your families improved, stayed the same, and deteriorated? Most clinicians never answer this question, and the ones who do become better clinicians because of it.

Review 4 — the statement itself. Read last year's position statement. Which sentences still hold? Which are now embarrassing? Which commitment did you honour, and which did you quietly stop honouring? Rewrite the section that has changed and date it.

Review 5 — the support. Who knows your work well enough to challenge you? If the answer is nobody, that is the most important finding of the review, and the first item in next year's plan.

Why the review is part of this week's work. Because a position statement that is never revised becomes a description of a therapist who no longer exists, and because the habits established in training are the habits of a career. The students who keep the review are, in most programmes, the ones who are still learning at fifty.

14.14b Two endings, and what each teaches

Ending A — the family that left early. A family attended three sessions and then stopped. The therapist wrote a letter offering a further appointment, telephoned once, and recorded: "Family did not return; letter sent; telephone not answered; case closed with the door open. What I would do differently: the father's work pattern was mentioned in session one and I did not ask about it until session three, by which time the mother had decided I did not understand the household. I will ask about the week in the first session, every time."

What this ending teaches. That an ending is data. The therapist has not blamed the family and has not blamed herself; she has identified one specific procedural error and made one specific commitment. That is what reflective practice looks like when it is not a ritual.

Ending B — the family that stayed too long. A family attended twenty-eight sessions. The work was going well, the family was engaged, and the therapist did not raise the question of ending until the family's attendance became intermittent. At the review the mother said: "We kept coming because we thought you would be disappointed if we stopped."

What this ending teaches. That endings must be planned from the start, and that a therapist's pleasure in being needed can extend a therapy beyond its usefulness. The professional question at every review is not only "is this helping?" but "is this still the best use of this family's time and this service's capacity?"

The comparison. The two failures are opposite and equally instructive: one ended too soon for reasons the therapist could have anticipated, the other continued too long for reasons the therapist could have named. Both were discovered by review rather than by intuition, and both produced a specific change in practice.

14.15 Self-test

1. What are the five commitments of a position statement?
2. Why must the theoretical commitment include its limits?

3. Give the three behaviours in which an epistemology becomes visible.
4. What distinguishes an ethical commitment from an aspiration?
5. What is the test of a limitation statement?
6. Name the three failures in a viva.
7. What are the four elements of a planned ending with a family?
8. Why does a development plan include rest and support, and what happens to a plan that omits them?

Answers. 1. Theoretical, epistemological, ethical, cultural, and acknowledged limitation (with a statement of what therapy is for). 2. Because a commitment without limits is an ideology, and because the examiner — and the family — need to know where the model fails. 3. What you do when the family disagrees with your formulation; what you do when outcome data show failure; whether you can say "I do not know" without filling the silence. 4. An ethical commitment contains specific refusals that cost something; an aspiration contains virtues that cost nothing. 5. Whether a colleague could observe you and tell whether you had addressed it. 6. Rigidity, collapse and deflection. 7. Name the ending early; review the work with the family's own data; address the ending itself; plan the future. 8. Because the first six months involve an emotional exposure that trainees underestimate, and a plan that ignores it is a plan for burnout rather than for development.

14.16 Reflective task

Read your Week 1 reflection again. What has changed? What has not? Write 300 words to yourself, dated, to be reread at the end of the programme. Then add one sentence to the end of your position statement that you would not have been able to write in Week 1, and say why you could not have written it then.

14.17 Readings for this week

- **Your own log, notes and Week 1 reflection**, read in sequence, from the beginning of the course to now. This is the primary text for the week.
- **Cecchin (1987)**, "Hypothesizing, circularity and neutrality revisited", Family Process, for the stance of curiosity and its limits.
- **Anderson & Goolishian (1992)**, on the not-knowing position, read alongside Week 12's limits.
- **Schoen (1983)**, The Reflective Practitioner, for the account of professional knowledge in action that underlies the whole idea of a stance.
- **Your professional body's statement of competence**, read as a specification against which to audit your own development plan.
- **Any published first-person account by an experienced family therapist** of how their practice changed over a career, read for the reassurance that a stance is supposed to change, and for the specific sentences in which the author names what they got wrong.

- **Your own recordings from Weeks 1 to 14**, watched in sequence for twenty minutes each; the difference between the first and the last is the most direct evidence you will ever have that the course did something.

Next: MFT 702 — Theories of Family Therapy

Integration: the whole course on two pages

1. The frame. Treat the relationship as the client. Widen the field; include context; notice the observer.

2. Causality. Move from who-started-it to what-maintains-it. Draw the loop. Punctuate differently.

3. History. Five movements: pre-history, research rupture, heroic era, critique and meaning, evidence and integration. Each school answers a limitation of its predecessor.

4. Concepts. System, subsystem, boundary, feedback, homeostasis, rule, triangle. Six concepts, most of the clinical work.

5. Profession. Know your scope, your setting, your team, your limits. Refer well.

6. The first session. Join, contract, hear everyone, screen for risk, map the pattern, summarise, invite correction, agree next step.

7. The problem definition. Specific, interactional, owned, changeable.

8. The genogram. Three generations, structural and temporal hypotheses, cultural completeness.

9. Formulation. 5-P plus "who is involved" and "what function the symptom serves", ending with one intervention sentence.

10. Culture and power. Ask, locate yourself, notice power, interrogate normal.

11. Evidence. Family therapy works; alliance matters; models overlap; drop-out is the silent failure.

12. Ethics. Confidentiality is negotiated, risk is screened, competence is bounded, dual roles are examined, notes are written for strangers.

13. Integration. Multiple lenses; move between frames without losing coherence.

14. Stance. A written, revisable position on what you believe and what you do not know.

Glossary

Boundary — the implicit rules governing participation and closeness between subsystems. **Circular causality** — an account in which each element both causes and is caused by the others. **Coalition** — an alliance of two or more people against a third. **Cybernetics** — the study of control and communication in systems; first-order (observer outside), second-order (observer inside). **Detriangulation** — the process of extricating a third party from a dyadic conflict. **Enmeshment** — diffuse boundaries with excessive emotional sharing and low autonomy. **Disengagement** — rigid boundaries with little emotional contact. **Feedback (negative)** — a process that reduces deviation and restores stability. **Feedback (positive)** — a process that amplifies deviation and produces escalation. **Genogram** — a three-generation diagram of family structure, relationships and events. **Homeostasis** — the system's tendency to maintain its existing organisation. **Hypothesis** — a provisional, testable explanation that guides enquiry. **Joining** — the therapist's deliberate accommodation to a family's style in order to work with it. **Linear causality** — an account in which A causes B in one direction. **Punctuation** — the way each participant marks the beginning of a sequence, thereby assigning blame. **Reframe** — relabelling a behaviour so that it is understood in a new, usually relational, frame. **Subsystem** — a unit within a system organised by boundaries (couple, siblings, parental dyad). **System** — a set of interacting elements such that a change in one changes the others. **Triangle** — a three-person configuration that absorbs and stabilises dyadic tension.

Self-assessment

Multiple choice (answers below)

1. A therapist says, "The real question is not who started the argument but what keeps it going." This statement reflects: (a) linear causality (b) circular causality (c) psychodynamic causality (d) moral causality

2. A father's drinking has the effect of preventing his wife and son from discussing the son's expulsion. In systemic terms the drinking is: (a) random (b) homeostatic (c) an individual symptom only (d) irrelevant to the family

3. "We never discuss money" is best described as: (a) a boundary (b) a family rule (c) a triangle (d) a reframe

4. In second-order cybernetics the therapist is: (a) outside the system (b) part of the observed system (c) a neutral observer (d) irrelevant

5. The best first move with a family that arrives furious at the referrer is: (a) correct the misunderstanding immediately (b) acknowledge their experience of being sent (c) begin the assessment (d) ask who is the identified patient

6. A negotiated problem definition should be specific, interactional, owned and: (a) clinically imposed (b) changeable (c) blame-allocating (d) historical

7. Which is not an element of a minimum first session? (a) confidentiality and limits (b) a risk screen (c) a full genogram (d) a summary and invitation to correct

8. The strongest predictor of drop-out in couple and family work is: (a) diagnosis (b) therapist's model (c) early engagement and early change (d) family size

9. "Multigenerational transmission" belongs to which tradition? (a) structural (b) Bowen (c) Milan (d) solution-focused

10. The statement "expert in process, not in their lives" describes: (a) neutrality about violence (b) the therapist's stance (c) a legal requirement (d) a diagnostic rule

Answer key

1 b · 2 b · 3 b · 4 b · 5 b · 6 b · 7 c · 8 c · 9 b · 10 b

Short answer (150 words each)

1. Explain the difference between negative and positive feedback using a family example.
2. Why is "punctuation" clinically useful? Give a sample question.
3. Name three limits of confidentiality in family work and how you would state each to a family.
4. What is the difference between a genogram and a formulation?
5. Summarise the common-factors critique of model loyalty in four sentences.

Essay questions (choose one, 1,500 words)

1. "Family therapy is a frame, not a technique." Discuss with reference to at least two models and one case from your placement.
2. Critically evaluate the claim that systemic neutrality is impossible — and undesirable — in cases of domestic violence.
3. Using a disguised case, demonstrate how a linear formulation and a circular formulation would lead to different treatment plans. Which would you choose and why?

Practical and clinical log requirements

For each session you observe or conduct, record:

Field	Requirement
Date and duration	Exact
Attendees	Relationship to index client; ages; who was absent
Referral source and question	In the referrer's words
Presenting problem	In each attendee's words
Risk screen	Completed Y/N; result; action
Pattern observed	One loop or sequence, described behaviourally

Field	Requirement
Intervention	What you did, and why
Response	What happened next
Supervision	Who, when, what was discussed
Plan	Next step, with review date

The log is assessed for **specificity and consistency**, not for how impressive the case was. A log of ordinary sessions written precisely will score higher than a dramatic case written vaguely.

Further reading

Core texts

- Nichols, M. P., & Davis, S. (2021). Family Therapy: Concepts and Methods (12th ed.). Pearson.
- Becvar, D. S., & Becvar, R. J. (2018). Family Therapy: A Systemic Integration (9th ed.). Pearson.
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Foundations and history

- Watzlawick, P., Beavin, J., & Jackson, D. (1967). Pragmatics of Human Communication. Norton.
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- Minuchin, S., & Fishman, H. C. (1981). Family Therapy Techniques. Harvard University Press.
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- McGoldrick, M., Gerson, R., & Petry, S. (2020). Genograms: Assessment and Treatment (4th ed.). Norton.

Culture and critique

- McGoldrick, M., Giordano, J., & Garcia-Preto, N. (Eds.). (2005). Ethnicity and Family Therapy (3rd ed.). Guilford.
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- Wampold, B. E., & Imel, Z. E. (2015). The Great Psychotherapy Debate (2nd ed.). Routledge.
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- American Association for Marriage and Family Therapy. (2015). Code of Ethics.
- International Association of Marriage and Family Counselors. Ethical Code.

Next: MFT 702 — Theories of Family Therapy

MSC MARRIAGE AND FAMILY THERAPY

LECTURE LIBRARY · VOLUME 2 OF 28

MFT 702

Theories of Family Therapy

14 teaching weeks · 6 practicals · 5 worked cases · 6 figures

SEMESTER ONE

1.71 units · Prerequisite: MFT 701

> “Nine major schools, each treated as an answer to a specific question.”

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MFT 702 — Theories of Family Therapy

MSc Marriage and Family Therapy · Semester One · 1.71 Units · 14 teaching weeks · Prerequisite: MFT 701

This volume is one of 28 course books that together cover the whole MSc Marriage and Family Therapy taught programme. Each book is written to carry a 14–16 week semester on its own: every week pairs a core idea with the practice that follows from it, a worked case, and a reflective task, and each book closes with an integration chapter, a glossary and a self-assessment paper with model answers.

The books are designed to be read straight through before the semester begins, revisited week by week, and used again in revision. Cross-references point to the other volumes where a topic is treated in more depth.

How to cite this volume

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A note on the case material

All families, couples and individuals in this book are fictional composites. They are written to teach pattern recognition, not to describe any real person. Nothing here replaces supervised clinical practice, professional registration, legal advice or medical care; legal content is orientation only and must be checked against the law of the relevant jurisdiction.

The lecture library

Twenty-eight self-contained course books carry the taught programme: eight in Semester One, seven in Semester Two, seven in Semester Three including the thesis, and six electives. The volume you are holding is marked in the margin.

Code	Course	Semester	Units
MFT 701	Fundamentals of Marriage and Family Therapy	One	1.71
MFT 702	**Theories of Family Therapy**	One	1.71
MFT 703	Human Development Across the Lifespan	One	1.71
MFT 704	Ethical and Legal Issues in Therapy	One	1.71
MFT 705	Research Methods in Marriage and Family Therapy	One	1.71
MFT 706	Clinical Practice and Techniques	One	1.71
MFT 707	Assessment and Diagnosis in Family Therapy	One	1.71
MFT 708	Cultural Competence in Therapy	One	1.71
MFT 709	Group Therapy Techniques	Two	1.71
MFT 710	Couples Therapy	Two	1.71
MFT 711	Child and Adolescent Therapy	Two	1.71
MFT 712	Crisis Intervention Strategies	Two	1.71
MFT 713	Substance Abuse and Family Dynamics	Two	1.71
MFT 714	Trauma-Informed Care	Two	1.71
MFT 715	Family Systems Theory	Two	1.71
MFT 716	Communication Skills in Therapy	Three	1.71
MFT 717	Psychopathology in Families	Three	1.71
MFT 718	Intimacy and Relationship Dynamics	Three	1.71
MFT 719	Family Life Cycle and Transitions	Three	1.71
MFT 720	Supervision and Consultation in Therapy	Three	1.71
MFT 721	Research Methodology	Three	1.71
MFT 722	Professional Development and Self-Care	Electives	1.71
MFT 723	Special Populations in Therapy	Electives	1.71
MFT 724	Family Law and Advocacy	Electives	1.71
MFT 725	Integrative Approaches in Therapy	Electives	1.71
MFT 726	Neurobiology of Relationships	Electives	1.71
MFT 727	Play Therapy Techniques	Electives	1.71
MFT 770	Master Thesis	Three	1.71

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Course leader's summary. Nine major schools, each treated as an answer to a specific question. You will learn each model well enough to do a first session in it, critique it, and integrate it — and you will practise moving between models on the same family until the choice becomes deliberate rather than habitual.

How to use this book

Theory is learned in three passes. **Pass one:** understand the model's core question and its intervention. **Pass two:** apply it to a case you know. **Pass three:** critique it and ask what it cannot see. Each week in this book gives you all three, plus the practical that makes the pass real. Do not memorise lists of techniques; memorise what problem each model believes it is solving.

1. Course description

There is no single "correct" family therapy. There are traditions, each with a theory of how problems develop, a theory of change, a preferred focus of attention, and a characteristic set of techniques. A clinician who knows only one model does not have a theory; they have a habit.

This course therefore pursues three aims: **comparative competence** (you can describe and demonstrate each major model), **critical appraisal** (you can name each model's evidence base and its blind spots), and **integration** (you can combine two models coherently in one case rather than dithering between them).

Organising question. For each model, ask: What does this model believe is the problem, and what does it therefore do first?

2. Learning outcomes

#	Outcome	Assessed
LO1	Describe the theoretical foundations, core concepts and change mechanisms of nine major models	Exam
LO2	Demonstrate a model-consistent first session for at least three models	Skills assessment
LO3	Compare models on focus, technique, role of the therapist and evidence	Comparative matrix
LO4	Identify the blind spots and cultural assumptions of each model	Essay
LO5	Construct a coherent integrative plan for a single case using two models	Case assignment
LO6	Justify model choice with reference to the client, the evidence and your own competence	Viva

3. Assessment

Component	Weight	Due
Comparative matrix (nine models × six dimensions)	20%	Week 7
Observed model-consistent session (20 min)	20%	Weeks 8–10
Integrative case study (2,500 words)	30%	Week 12
Written examination	30%	Week 15

4. The fourteen-week plan

Week	Model / topic	Practical	Key text
1	How to compare theories: six dimensions	Matrix construction	Nichols ch. 1
2	Psychoanalytic and object-relations family therapy	Genogram + interpretation exercise	Scharff & Scharff
3	Bowen's multigenerational theory	Detriangling role-play	Bowen (1978)
4	Structural family therapy	Enactment and boundary-making	Minuchin (1974)
5	Strategic and MRI brief therapy	Directive design	Haley & Madanes
6	Milan systemic therapy	Circular questioning	Palazzoli et al. (1980)
7	Experiential and human validation (Satir, Whitaker)	Sculpture exercise	Satir (1988)
8	Cognitive-behavioural and behavioural couple therapy	Behavioural exchange design	Epstein & Baucom
9	Emotionally focused therapy	EFT enactment	Johnson (2019)
10	Solution-focused brief therapy	Miracle question and scaling	de Shazer (1985)
11	Narrative therapy	Externalising conversation	White (2007)
12	Integration: combine two models without incoherence	Case integration workshop	Lebow (2014)
13	Evidence, culture and critique of each model	Debate	Sprenkle et al.
14	Your theoretical position	Position paper presentation	—

5. The six dimensions of comparison

Every model can be described on the same six axes. Learn to fill this grid from memory; it is the spine of the exam.

Dimension	The question it answers
Theory of problem	How does this model believe problems develop and persist?

Dimension	The question it answers
Unit of focus	Individual, dyad, family structure, meaning system, or context?
Theory of change	What must happen for things to improve?
Therapist position	Director, coach, collaborator, witness, expert?
Signature techniques	What does this model characteristically do?
Evidence base	What is the strength and quality of supporting research?

> **PRACTICAL**
> Build the matrix in a spreadsheet on day one. Add a row each week. By Week 7 you will have a revision document worth more than any summary anyone could give you.

Week 2 — Psychoanalytic and object-relations family therapy

The question it asks. What unconscious processes, carried from earlier relationships, are being re-enacted in this family now?

Core concepts. Object relations (internal working models of self and other formed in early relationships); projection and projective identification (disowned parts of the self are located in another person); splitting (people experienced as all-good or all-bad); repetition compulsion; the family as a shared unconscious system.

Therapy. Longer-term; the therapist attends to transference and countertransference, interprets patterns, and helps the family "hold" difficult feelings rather than evacuate them into one member. Frameworks include group-analytic family therapy and the Tavistock tradition.

Signature technique — interpretation, used sparingly. "I notice that whenever your mother speaks about her own mother, you begin to speak for her. I wonder whether that is how the family has always managed her sadness."

Contribution. It gave the field its respect for depth, for what is not said, and for the therapist's own emotional response as data.

Blind spots. Long, expensive, thin evidence base in its classical form; risks pathologising parents; the language can feel blaming ("the intrusive mother") and can ignore material context.

> **CASE**
> A young man cannot keep a job; his mother calls his employers before he does. Object-relations reading: the mother's own abandonment anxiety is projected into her son's incompetence, and his failure keeps her needed. Interpretation may be less useful than an experiment: agree that she will not call, and predict that both will feel anxious for two weeks. The prediction turns insight into something testable.

Week 3 — Bowen's multigenerational theory

The question. How is anxiety transmitted across generations, and who in the system absorbs it?

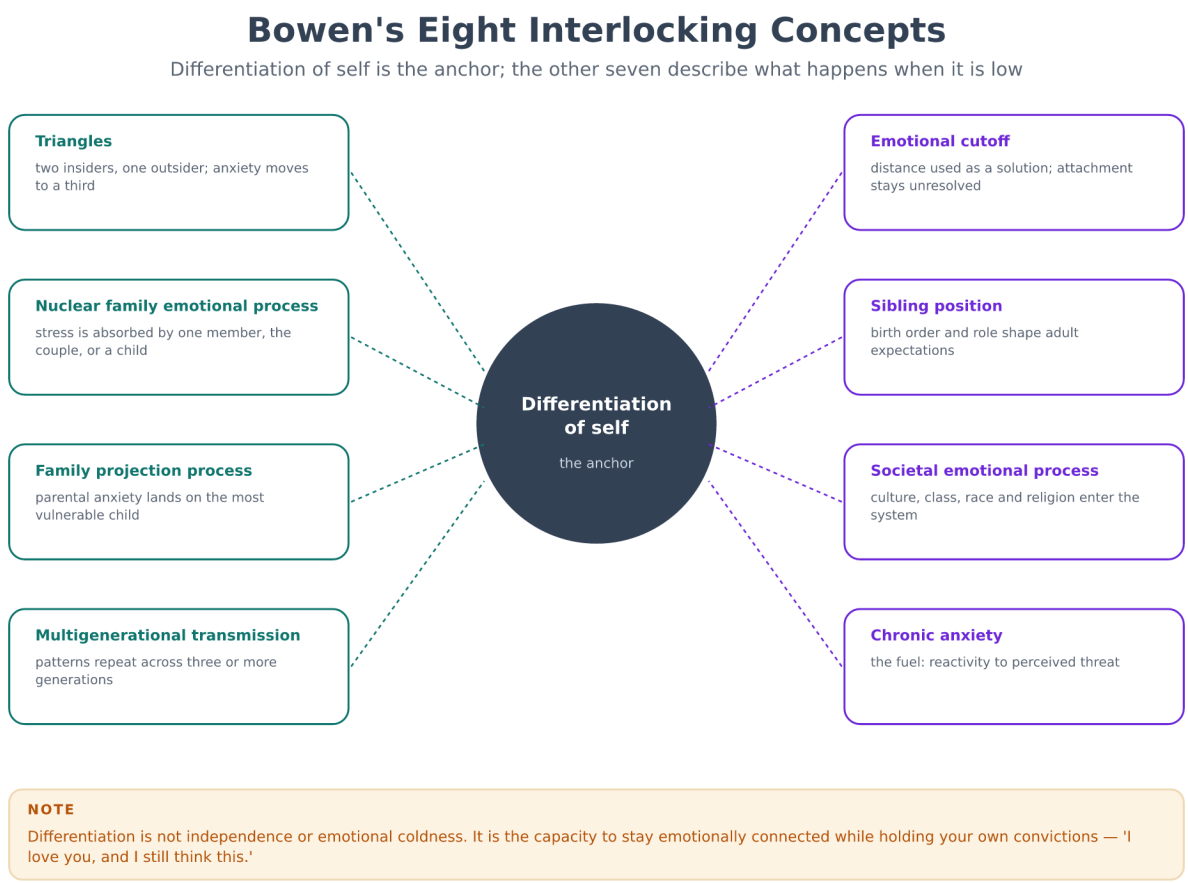


Figure 3.1 — Bowen's eight interlocking concepts

Core concepts. Differentiation of self (the capacity to stay connected while thinking for oneself); triangles; the nuclear family emotional process (marital conflict, dysfunction in a spouse, projection to a child, emotional distance); family projection process; emotional cutoff; multigenerational transmission; sibling position; societal emotional process; chronic anxiety.

Therapy. The therapist is a **coach**, not a fixer. The work is largely with the most motivated person (often "the over-functioner"), who learns to stay in contact with difficult relatives while managing their own reactivity. Genograms are the primary tool; the goal is differentiation, not symptom removal.

Signature technique — detriangling. "When your mother criticises your sister, what happens inside you? What would it look like to stay interested without taking a side and without leaving?"

Contribution. Genuinely systemic over time; explains why patterns repeat; insists the therapist do their own family-of-origin work.

Blind spots. Slow; assumes a level of choice and mobility not everyone has; can under-weight structure, gender and economics; evidence is largely clinical rather than trial-based.

> **PRACTICAL**
> Triads. One person describes a triangle in their family. The therapist practises three moves: name the triangle, ask about the client's own reactivity, and refuse to take a side. Rotate; the "client" rates how neutral (not cold) the therapist was.

Week 4 — Structural family therapy

The question. What is the organisation of this family, and how does its structure maintain the problem?

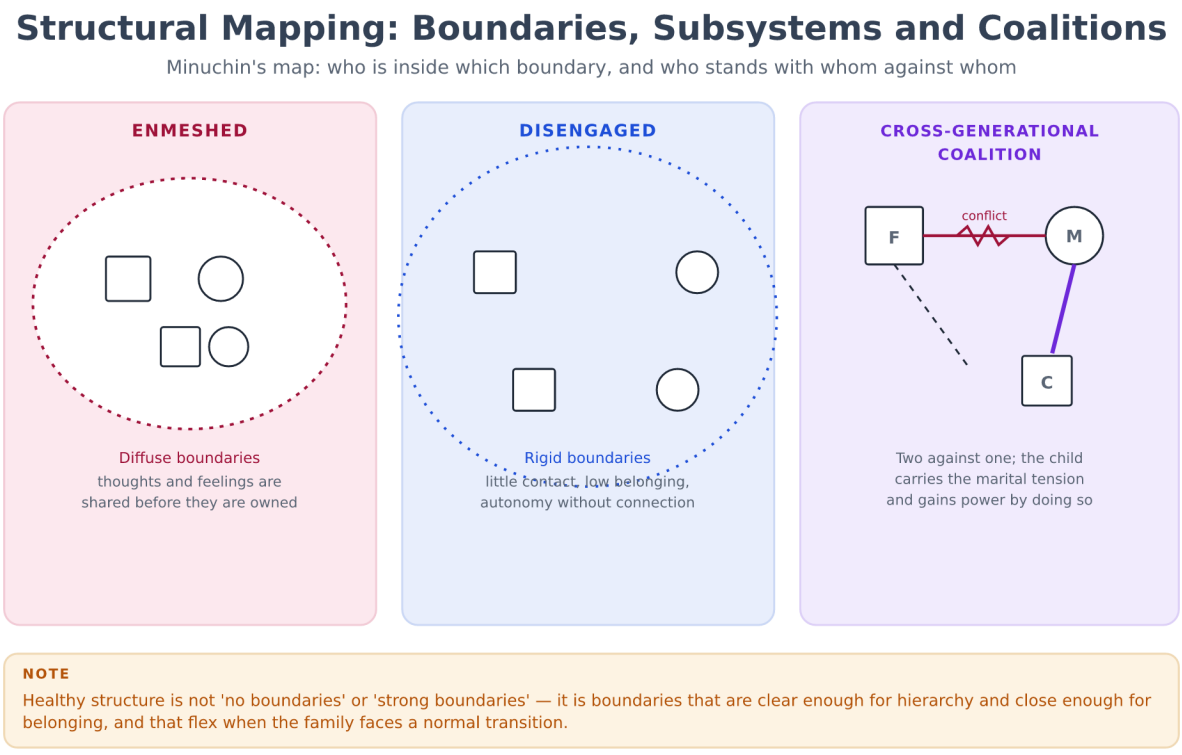


Figure 4.1 — Structural mapping: boundaries, subsystems, coalitions

Core concepts. Structure (the repetitive patterns of interaction that organise the family); subsystems; boundaries (clear, diffuse, rigid); hierarchy; alignment and coalition; enmeshment and disengagement; complementarity; the identified patient as the symptom-bearer of a structural problem.

Therapy. The therapist **joins** and then **challenges** the structure in the room. Change happens through doing, not talking: enactments, boundary-making, unbalancing, raising intensity, and marking hierarchy. Sessions are active and often directive.

Signature techniques.

- **Enactment:** "Say that to her, not to me. I'll listen."
- **Unbalancing:** deliberately supporting one member to shift the system's equilibrium.
- **Boundary-making:** "This is a conversation for the two of you; the children will wait outside."
- **Intensity:** "This is the fourth time you have laughed when your wife described the miscarriage. What is that laugh?"

Contribution. Fast, observable, highly teachable; strong evidence for structural family therapy in child and adolescent problems and in family-based treatment for substance misuse.

Blind spots. Risks imposing a middle-class norm of two-parent hierarchy; can be experienced as controlling; under-attends to inner life and to history.

> **CASE**
> The Adeyemi family. The structural reading: the parental subsystem is weak and the mother–daughter coalition crosses the generational line; the father is structurally peripheral and reacts with criticism when he is pulled in. First move: a session with the parents alone to plan one consistent rule and one shared evening, before any work with Amara. Second move: a joint session in which the parents tell Amara the plan together while the therapist blocks her attempts to negotiate with her mother alone.

Week 5 — Strategic and MRI brief therapy

The question. What is the family doing that keeps the problem going — especially their attempted solutions?

Core concepts. The problem is maintained by current interaction, not by the past; attempted solutions often persist because they "almost" work; the symptom may serve a function in the system; resistance is a response to the therapist's approach, not a property of the family; brief, focused, pragmatic.

Therapy. The therapist is an **active strategist** who designs interventions — tasks, directives, reframes, and sometimes paradoxical instructions. Sessions are few; between-session tasks do the work; termination is early by design.

Signature techniques.

- **Reframing:** "Your daughter's tantrums may be her way of keeping you two in the same room."

- **Ordeals:** a task that must be performed whenever the symptom occurs, making the symptom more costly than the change.
- **Paradoxical directive (with caution):** prescribing the symptom, under close supervision, in carefully selected cases.
- **Pretend technique:** "For one week, pretend to have the symptom on purpose for ten minutes a night."

Contribution. Broke the assumption that therapy must be long; emphasised the therapeutic power of therapist pragmatism and of clear, behavioural tasks.

Blind spots. Manipulation risks; weak evidence for paradoxical techniques; may be superficial in cases with trauma or severe mental illness; the "function of the symptom" can be speculative.

> **PRACTICAL**
> Design a one-week task for a family you know (disguised) that makes the current attempted solution impossible to continue, without instructing them to do the opposite in so many words. Present it; the group judges whether it is clear, doable, and safe.

Week 6 — Milan systemic therapy

The question. How do the family's beliefs and premises organise their behaviour — and how can curiosity disturb them?

Core concepts. The family as a meaning-making system; the **hypothesising, circularity and neutrality** stance; positive connotation (all behaviour is understood as serving the system); rituals; **circular questioning** (questions about differences and relationships rather than facts); the reflecting team and the "irreverent" later Milan style.

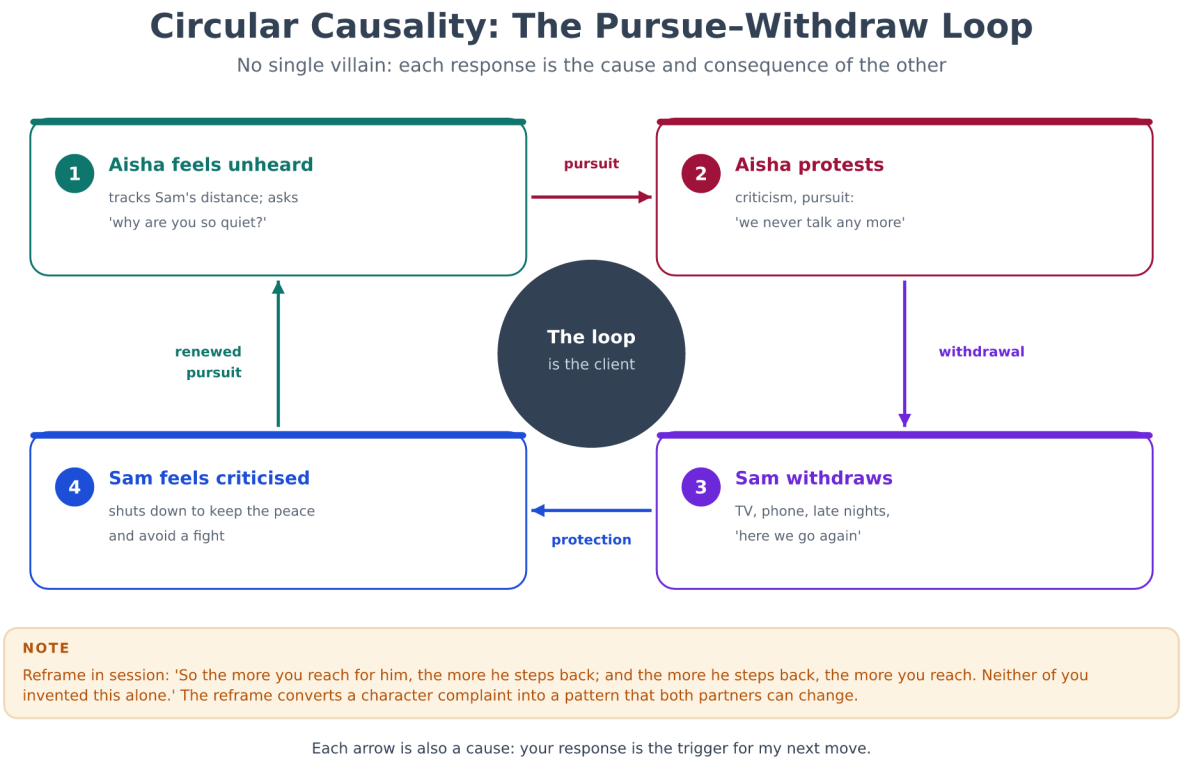


Figure 6.1 — Circular causality in practice

Therapy. The team observes behind a screen; the therapist asks circular questions; a consultation break produces an intervention that positively connotes the family's behaviour, often prescribing a ritual. The modern version is non-directive: the team's reflections are offered as multiple perspectives, not as verdicts.

Signature technique — circular questioning.

- "Who in the family is most worried about your mother's sadness?"
- "If your brother were here, what would he say your father does when you cry?"
- "On a scale from 1 to 10, how close are your parents? And how close would they say they are?"
- "What is the difference between the way your mother treats you and the way she treats your sister?"

Contribution. Gave the field its most transferable skill — the circular question — and its most useful epistemic stance, curiosity.

Blind spots. Can be experienced as evasive in crisis; neutrality is ethically dangerous in violence; team-based format is expensive.

PRACTICAL

Ten-minute interview with a volunteer "family" (two peers). You may ask only circular questions. An observer counts how many were genuinely circular (about difference/relationship) and how many were disguised facts. Aim for 80%.

Week 7 — Experiential and human validation models

The question. What would happen if this family could feel and say the truth to each other, right now?

Key figures. Virginia Satir (family sculpture, communication stances, self-worth, "the five freedoms"); Carl Whitaker (symbolic-experiential therapy, absurdity, the therapist's use of self); Kempler and the Gestalt-influenced tradition.

Core concepts. Growth rather than pathology; the present moment as the site of change; expression of authentic feeling; the therapist's own aliveness as a therapeutic instrument; family sculpture as non-verbal enactment of structure.

Signature techniques.

- **Family sculpture:** family physically arrange themselves to show the emotional map; then rearrange.
- **Communication stances** (Satir): placating, blaming, super-reasonable, irrelevant, and congruent.
- **Here-and-now enactment:** "Tell her now. I want to see what happens to your face when you say it."

Contribution. Humanised the field; insisted on warmth, humour and the person of the therapist. Many of its insights survive inside EFT and other emotion-focused models.

Blind spots. Dependent on the charisma and health of the therapist; thin manualisation; risk of emotional flooding without containment.

CASE

In a family session, the father has not cried in 20 years. Whitaker-influenced response: the therapist does not push the tears, but stays with the father's jaw and breathing, wondering aloud about what it costs to hold it. When the father finally speaks, the therapist keeps the room still long enough for others to respond. The intervention is pace, not technique.

Week 8 — Cognitive-behavioural and behavioural couple therapy

The question. What behaviours and beliefs maintain the distress, and how can they be changed directly?

Core concepts. Behavioural exchange and communication training (TBCT); cognitive restructuring of relationship standards and attributions; the role of selective attention (noticing negatives and missing positives); homework as the engine of change; skills deficit versus skills performance.

Therapy. Structured, time-limited, with agenda and homework. Skills are taught and rehearsed in the room, then practised between sessions. Integrative behavioural couples therapy addresses both behaviour and acceptance, with attention to culture and values.

Signature techniques.

- **Behavioural exchange:** each partner lists small, achievable caring behaviours; each does them daily.
- **Communication training:** speaker–listener with a talking object (see Figure 35, MFT 716).
- **Attribution retraining:** "When he is late, what do you assume? What else could it mean?"
- **Problem-solving:** define one problem, generate options, choose, trial, review.

Evidence. Among the strongest in the field; effective for relationship distress, with good evidence for depression treated in a couple format.

Blind spots. Can feel mechanistic; may under-weight emotion and attachment; assumes a degree of motivation and literacy; "skills" can be performed in session and abandoned at home.

> **PRACTICAL**
> With a peer, design a behavioural exchange contract: three behaviours each, defined observably, cheap, daily, and mutually agreeable. Then critique it: what could make this fail in a real family within two weeks?

Week 9 — Emotionally focused therapy

The question. What attachment need is underneath the anger, and how can the cycle be de-escalated so the bond can be repaired?

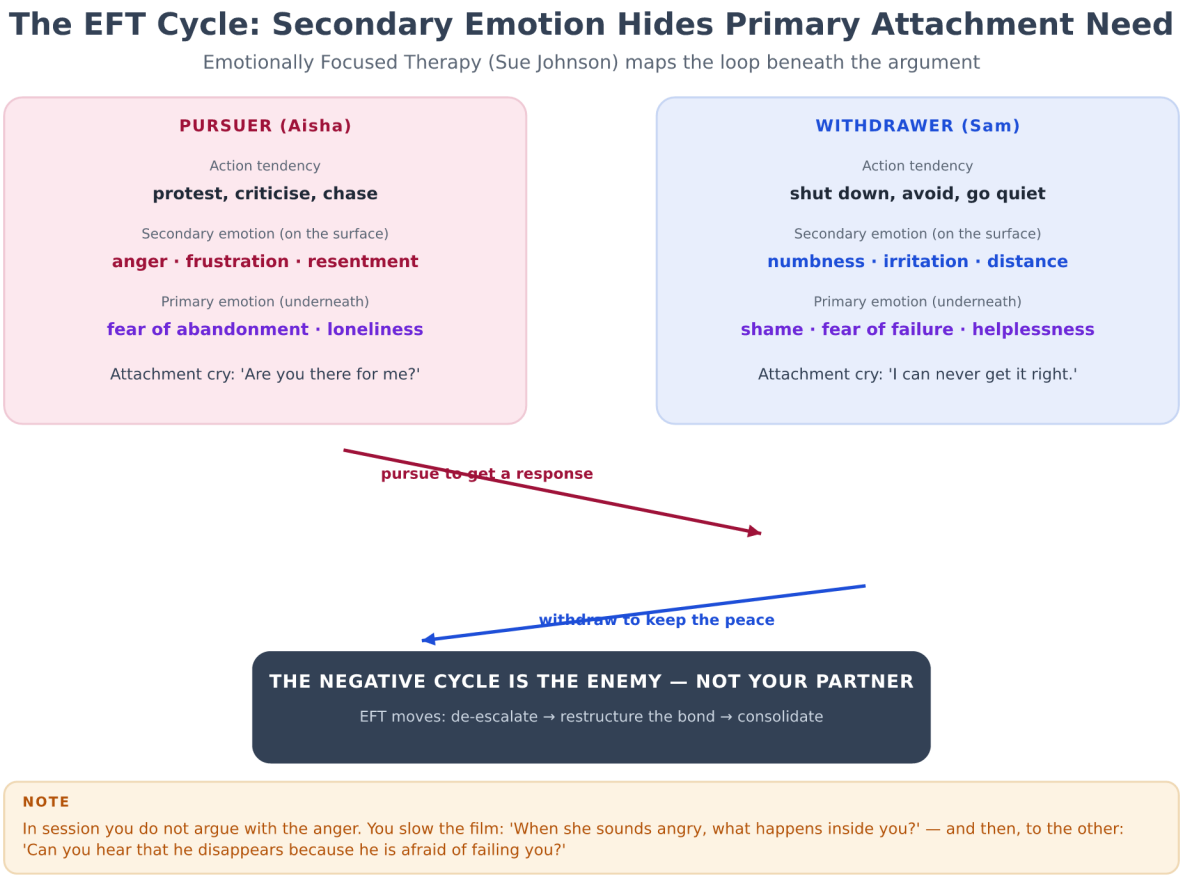


Figure 9.1 — The EFT cycle: secondary emotion and primary attachment need

Core concepts. Attachment theory as the map (see Figure 17); the negative cycle (pursue–withdraw, attack–defend, both-withdraw); primary versus secondary emotion; the "attachment cry"; softening events; the therapist as a process consultant and secure base.

Therapy in three stages. 1) **De-escalation** — identify the cycle, access primary emotion, reframe the problem as the cycle. 2) **Restructuring** — withdrawer re-engagement, pursuer softening, enactment of new responses. 3) **Consolidation** — integrate, problem-solve, prepare for recurrence.

Signature techniques.

- **Tracking the cycle:** "What happens inside you when she says that?"
- **Deepening primary emotion:** "Under the anger — is there something like fear of being alone?"
- **Enactment:** "Can you tell him that now, looking at him?"
- **Softening:** the pursuer's protest turns into an attachment expression, and the withdrawer responds.

Evidence. Among the most robust in couple therapy (over 30 trials, large effect sizes, durability in follow-up). Considered a first-line couple therapy for relational distress.

Blind spots. Requires a specific in-session emotional intensity and skills; less suited to ongoing coercive control; cultural calibration needed (the language of "needs" may feel self-indulgent to some families).

> **CASE**
> Aisha and Sam. Third session: the therapist slows a familiar argument to 20% speed. Aisha's anger is reframed as protest; Sam's silence as fear of failing. The first enactment — Aisha, quietly: "I get loud because I think you are leaving" — is the turning point. The next eight sessions are the work of making that statement safe to repeat at home.

Week 10 — Solution-focused brief therapy

The question. What is already working, and what would one small step look like?



Figure 10.1 — Solution-focused brief therapy in five moves

Core concepts. Solution talk versus problem talk; the miracle question; scaling; exceptions; coping questions; the client as expert; small, observable change; "if it works, do more of it".

Therapy. Conversational, non-normative, often 3–6 sessions. The therapist listens for competence and deliberately amplifies it; homework is usually "notice what works".

Signature techniques.

- **Miracle question:** "Suppose tonight a miracle happens and the problem is solved. What will you notice first tomorrow morning? Who else will notice?"
- **Scaling:** "On 0–10, where are you? What would one point higher look like this week?"
- **Exception questions:** "Tell me about a time the arguing stayed small. What were you doing differently?"
- **Coping question:** "With everything you have faced, how have you managed to keep going?"

Evidence. Good evidence for depression, with brief formats showing effects comparable to longer treatments in some trials; widely used in child protection and school settings.

Blind spots. Can bypass necessary grief or trauma work; risks ignoring risk; may feel invalidating when a family needs to tell the story of what happened.

> **PRACTICAL**
> Full miracle-question sequence in pairs (8 minutes), then a one-point scaling and a concrete task. Observer records: did the therapist ask for concrete behavioural detail, and did the client's posture change?

Week 11 — Narrative therapy

The question. Which story is dominating this family's life, and who wrote it?

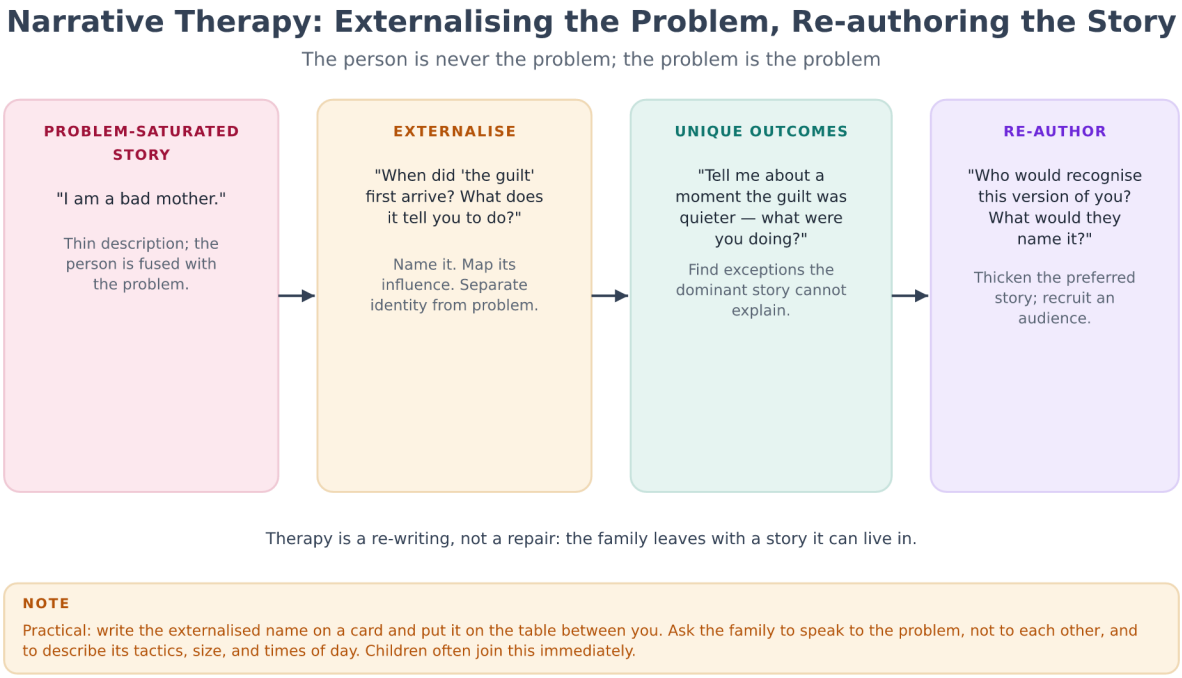


Figure 11.1 — Externalising and re-authoring the preferred story

Core concepts. The person is not the problem; the problem is the problem; dominant and preferred stories; thin versus thick descriptions; unique outcomes; externalising conversations; re-membering; definitional ceremonies; the therapist as decentred but influential.

Signature techniques.

- **Externalising:** "When did 'the guilt' first arrive in your life? What does it whisper to you?"
- **Mapping the influence:** "When does it have the most power? When does it lose power?"
- **Unique outcomes:** "Tell me about a moment when 'the shouting' was quieter."
- **Re-authoring:** "If you were to name this new version of yourself, what would you call it? Who would recognise it?"
- **Letter-writing** and **outsider-witness groups** to thicken the preferred story.

Contribution. The most culturally adaptable of the schools; central in work with children, with communities, and with people who have been defined by a diagnosis.

Blind spots. Can under-attend to material reality and risk; the linguistic sophistication can be inaccessible; evidence base is growing but smaller than EFT or CBT.

> **CASE**
> A mother referred by the school as "negligent". The therapist does not argue with the label but asks when "the exhaustion" took over, what it makes possible, what it prevents. Within thirty minutes the mother has described three occasions when she "fought the exhaustion and won", including a week when she arranged a rota with her sister. The referral, still true, is no longer the whole story.

Week 12 — Integration without incoherence

The question. How do you combine models so the client is not paying for your confusion?

Three legitimate forms of integration:

1. **Assimilative integration.** A home model, enriched by techniques from elsewhere. Most clinicians work this way. The home model keeps the formulation coherent.
2. **Sequential integration.** Different models at different phases — for example structural work to establish safety and hierarchy, then EFT for the couple's bond, then solution-focused work to consolidate.
3. **Theoretically integrated models.** Purpose-built blends (integrative behavioural couples therapy; attachment-based family therapy), coherent because they were designed together.

Rules for staying coherent.

Rule	Why
One formulation, one primary target	Prevents scattergun technique
Name the model out loud in supervision	Makes your reasoning auditable
Change model for a reason you can state	"Because the conflict escalates when we go near feelings"
Do not mix incompatible theories of change in one move	Reframing and behavioural contracting pull in different directions
Review at fixed intervals	Integration fails by drifting, not by decision

Trap to avoid: technique salad. Symptom-tracking from CBT, a miracle question, a family sculpture and an externalising conversation in one session does not make you integrative; it makes you unpredictable, and the family cannot tell what is expected of them.

> **ASSIGNMENT (30%)**
> One disguised case; two models; one formulation each; a reasoned choice of sequence; a predicted risk of the combination; a plan for how you would know it is not working.

Week 13 — Evidence, culture and critique

Model	Strongest evidence	Weakest evidence	Main cultural critique
Structural	Child/adolescent behaviour; substance misuse	Adult mood disorders	Imposes a particular family hierarchy
Bowen	Clinical tradition; modest outcome data	Few RCTs	Assumes mobility and choice; slow
Strategic/MRI	Some RCTs in child problems	Paradoxical techniques	Manipulative; may ignore trauma
Milan	Weak in classical form	Rituals, team format	Neutrality unethical in violence
Experiential	Moderate, largely process research	Manualisation	Charisma-dependent; middle-class
CB couple therapy	Strong	Severe relational trauma	Mechanistic; literacy-dependent
EFT	Strong, replicated	Coercive control	Individualist language of needs
Solution-focused	Moderate (depression, brief formats)	Severe mental illness	Can bypass grief and risk
Narrative	Growing; strong qualitative base	Large trials	Requires verbal fluency

The reflexive question. For each row, ask: whose family is this model describing? Models were developed largely with white, Western, middle-class, two-parent families in clinic settings. When you apply them to a polygynous household, a three-generation

compound, a family separated by migration, or a family whose authority structure is religious rather than developmental, you must translate — and say that you are translating.

Week 14 — Your theoretical position

Write 1,500 words: your home model, why it fits you, what it cannot do, the models you borrow from and under what conditions, and how you will know when to change your mind. Defend it in a ten-minute viva.

Integration: nine models on one page

Model	Problem is…	Change happens by…	Therapist is…	First move
Object relations	Unconscious re-enactment	Making the unconscious usable in the room	Container/interpreter	Notice the transference; interpret gently
Bowen	Undifferentiated anxiety across generations	Differentiation of self	Coach	Genogram + detriangling
Structural	Dysfunctional organisation	Restructuring in the session	Director/joiner	Enactment; strengthen hierarchy
Strategic	Attempted solutions	Interrupting the sequence	Strategist	Task, reframe, directive
Milan	Meaning premises	Curiosity disturbing belief	Curious team member	Circular questions
Experiential	Blocked feeling and authenticity	Present-moment expression	Real person	Sculpture; here-and-now
CB couple	Behaviour and attribution	Skills and rehearsal	Teacher/coach	Behavioural exchange
EFT	Attachment injury and cycle	Accessing primary emotion; enacting new responses	Process consultant	Track and reframe the cycle
Solution-focused	Problem-saturated attention	Amplifying existing solutions	Collaborator	Miracle/scale question
Narrative	Dominant story	Externalising and re-authoring	Decentred, influential	Externalising question

Glossary

Circular questioning — questions about differences and relationships that reveal pattern rather than fact. **Differentiation of self** — the capacity to stay emotionally connected while holding one's own position. **Enactment** — directing family members to speak to each other in the session rather than through the therapist. **Externalising** — locating a problem outside a person, as a thing with tactics and a history. **Negative connotation / positive connotation** — interpreting behaviour as serving the system, rather than as defective. **Paradoxical directive** — prescribing the symptom in order to change its function (used rarely, with supervision). **Reframe** — relabelling behaviour within a new frame that makes change possible. **Restructuring** — changing the family's organisation in the session through enactment and boundary work. **Scaling** — asking a family to rate a state on a numerical scale to make change visible and specific. **Unique outcome** — a moment when the dominant story did not hold, used as a foothold for a preferred story.

Self-assessment

Multiple choice

1. "The problem is the problem; the person is not the problem" is a principle of: (a) structural (b) narrative (c) Bowen (d) strategic

2. A therapist says, "For the next week, I want you to argue for exactly fifteen minutes at 8 p.m., then stop." This is closest to: (a) narrative (b) EFT (c) strategic (d) solution-focused

3. Positive connotation is associated with: (a) Milan (b) CB couple therapy (c) object relations (d) solution-focused

4. Which model places differentiation of self at its centre? (a) structural (b) Bowen (c) EFT (d) Milan

5. "Who would notice first if the problem were solved?" is a technique from: (a) structural (b) solution-focused (c) narrative (d) experiential

6. In EFT, the behaviour that conceals a primary attachment emotion is called: (a) enactment (b) secondary emotion (c) reframe (d) restructuring

7. Which model is most associated with family sculpture? (a) Satir (b) Haley (c) Minuchin (d) White

8. "Unbalancing" is a technique of: (a) structural therapy (b) solution-focused therapy (c) object relations (d) narrative therapy

9. Which model has the strongest replicated randomised evidence in couple therapy? (a) Bowen (b) Milan (c) EFT (d) experiential

10. Assimilative integration means: (a) blending theories equally (b) using a home model enriched by borrowed techniques (c) alternating models by session (d) refusing to commit to a model

Answer key

1 b · 2 c · 3 a · 4 b · 5 b · 6 b · 7 a · 8 a · 9 c · 10 b

Short answer

1. State the theory of change of structural therapy in two sentences, then name one case type where it is a poor fit and say why.
2. Write three circular questions you would ask a family in which a father has just lost his job.
3. What is the difference between positive connotation and reframing?
4. Give two reasons why narrative therapy suits work with children.
5. Name two incompatible assumptions that would make a structural-plus-narrative formulation incoherent.

Essay (choose one, 2,000 words)

1. Compare structural and EFT approaches to the same couple case. Which would you choose first and why?
2. "Neutrality is a luxury the powerful can afford." Discuss with reference to Milan therapy and domestic violence.
3. Critically evaluate the claim that theoretical integration is a sign of maturity rather than confusion.

Further reading

Comparative

- Nichols, M. P., & Davis, S. (2021). Family Therapy: Concepts and Methods (12th ed.). Pearson.
- Lebow, J. L. (2014). Couple and Family Therapy: An Integrative Map of the Territory. APA.
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Primary texts

- Bowen, M. (1978). Family Therapy in Clinical Practice. Aronson.
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- White, M. (2007). Maps of Narrative Practice. Norton.
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The books are designed to be read straight through before the semester begins, revisited week by week, and used again in revision. Cross-references point to the other volumes where a topic is treated in more depth.

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A note on the case material

All families, couples and individuals in this book are fictional composites. They are written to teach pattern recognition, not to describe any real person. Nothing here replaces supervised clinical practice, professional registration, legal advice or medical care; legal content is orientation only and must be checked against the law of the relevant jurisdiction.

The lecture library

Twenty-eight self-contained course books carry the taught programme: eight in Semester One, seven in Semester Two, seven in Semester Three including the thesis, and six electives. The volume you are holding is marked in the margin.

Code	Course	Semester	Units
MFT 701	Fundamentals of Marriage and Family Therapy	One	1.71
MFT 702	Theories of Family Therapy	One	1.71
MFT 703	**Human Development Across the Lifespan**	One	1.71
MFT 704	Ethical and Legal Issues in Therapy	One	1.71
MFT 705	Research Methods in Marriage and Family Therapy	One	1.71
MFT 706	Clinical Practice and Techniques	One	1.71
MFT 707	Assessment and Diagnosis in Family Therapy	One	1.71
MFT 708	Cultural Competence in Therapy	One	1.71
MFT 709	Group Therapy Techniques	Two	1.71
MFT 710	Couples Therapy	Two	1.71
MFT 711	Child and Adolescent Therapy	Two	1.71
MFT 712	Crisis Intervention Strategies	Two	1.71
MFT 713	Substance Abuse and Family Dynamics	Two	1.71
MFT 714	Trauma-Informed Care	Two	1.71
MFT 715	Family Systems Theory	Two	1.71
MFT 716	Communication Skills in Therapy	Three	1.71
MFT 717	Psychopathology in Families	Three	1.71
MFT 718	Intimacy and Relationship Dynamics	Three	1.71
MFT 719	Family Life Cycle and Transitions	Three	1.71
MFT 720	Supervision and Consultation in Therapy	Three	1.71
MFT 721	Research Methodology	Three	1.71
MFT 722	Professional Development and Self-Care	Electives	1.71
MFT 723	Special Populations in Therapy	Electives	1.71
MFT 724	Family Law and Advocacy	Electives	1.71
MFT 725	Integrative Approaches in Therapy	Electives	1.71
MFT 726	Neurobiology of Relationships	Electives	1.71
MFT 727	Play Therapy Techniques	Electives	1.71
MFT 770	Master Thesis	Three	1.71

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Course leader's summary. You cannot assess a family without knowing what is developmentally ordinary. This course covers physical, cognitive, language, social-emotional and moral development from conception to later life, always in family and cultural context — and it trains you to distinguish normal variation, delay, regression and symptom.

How to use this book

Development is not a list of ages; it is a set of **tasks** and a set of **negotiations** between a person and their family. Each week here gives you the developmental task, the family task that accompanies it, what goes wrong, and what it looks like in a consulting room. Use the milestone grid (Figure 23) as a reference you return to, not as a checklist to memorise.

1. Course description

Human development is the backbone of family assessment. A two-year-old's tantrum, a nine-year-old's lying, and a fifteen-year-old's secrecy are not three versions of "defiance": they are three different developmental moments that require three different responses. Clinicians who do not know this either pathologise normal development or miss genuine delay.

The course integrates **neurodevelopmental, cognitive, attachment, systemic and cultural** perspectives. It treats development as bidirectional: children shape parents as much as parents shape children, and families develop too (a theme developed in MFT 719).

2. Learning outcomes

#	Outcome	Assessed
LO1	Describe physical, cognitive, language and social-emotional milestones from 0–18 and in adulthood	Exam
LO2	Distinguish normal variation, delay, regression and symptom in a presenting concern	Case assignment
LO3	Apply attachment theory to family assessment	Case assignment, exam
LO4	Explain developmental tasks of adulthood and later life and their family implications	Exam
LO5	Evaluate developmental theory critically, including its cultural assumptions	Essay
LO6	Communicate developmental information to parents without jargon or alarm	Skills assessment

3. Assessment

Component	Weight	Due
Developmental case analysis (2,000 words)	30%	Week 8
Parent-feedback role-play (10 min, observed)	20%	Week 11
Written examination	50%	Week 16

4. The fifteen-week plan

Week	Topic	Practical
1	Why developmental knowledge is clinical knowledge	Milestone self-audit
2	Prenatal development, temperament and the transition to parenthood	Temperament interview
3	Infancy: attachment, synchrony, regulation	Still-face observation
4	Toddlerhood: autonomy, language, limit-setting	Tantrum consultation role-play
5	Early childhood: play, theory of mind, initiative	Play observation
6	Middle childhood: industry, peers, school, self-concept	School-refusal case
7	Adolescence I: puberty, identity, autonomy	Adolescent interview
8	Adolescence II: risk, peers, digital life, mental health	Risk/development differential
9	Adult development: intimacy, work, generativity	Life-structure interview
10	Couple development across the lifespan	Couple timeline exercise
11	Parenting across stages; communicating with parents	**Observed parent feedback**
12	Later life: cognition, loss, caregiving, legacy	Caregiver family meeting
13	Culture, poverty and development	Critical reading seminar
14	When development goes wrong: delay, disorder, trauma	Differential diagnosis workshop
15	Integration: the developmental lens in assessment	Case presentations

Week 1 — Why developmental knowledge is clinical knowledge

Three clinical errors are prevented by developmental literacy:

1. **Pathologising the ordinary.** A two-year-old who says "no" to everything is doing the developmental task of autonomy, not manipulating the family.
2. **Missing the abnormal.** A five-year-old who has lost previously acquired language is not "being difficult"; regression is a red flag requiring urgent referral.

3. **Mis-timing the intervention.** A parent is told to "reason with" a three-year-old whose prefrontal circuitry cannot yet support the reasoning demanded. The intervention fails and the parent concludes they are failing.

Four categories you must distinguish — this distinction recurs in every case assignment:

Category	Definition	Example
Normal variation	Within the expected range, differing in style and pace	Walking at 11 months vs 15 months
Delay	Functioning below the expected range but progressing	No words at 24 months, but progressing
Regression	Loss of previously acquired skill	A four-year-old stops speaking
Symptom	Behaviour that signals distress rather than development	New soiling at seven after a bereavement



Figure 1.1 — Developmental milestones from 0 to 18: a screening map, not a diagnosis

PRACTICAL

Write your own developmental timeline from memory: first word, first step, a significant illness, a move, a loss. Then ask a parent or sibling to correct it. Note which events you had forgotten and who remembered them.

Week 2 — Prenatal development, temperament and becoming a parent

Prenatal and perinatal. Brain development, maternal stress and nutrition, birth experience, and the "fourth trimester". Clinically relevant: prematurity, birth trauma, postnatal depression (which affects up to 1 in 5 mothers in some settings and is strongly associated with relational and child outcomes), and paternal postnatal depression.

Temperament. Nine dimensions (Thomas & Chess) usually simplified to three profiles: easy, slow-to- warm, and difficult/feisty. Temperament is not destiny but it is **goodness of fit** that matters: a highly sensitive child in a loud, unpredictable household will struggle more than the same child in a predictable one.

The transition to parenthood. The couple must reorganise: time, sleep, money, division of labour, sexuality, relationship with grandparents, and identity. Research consistently finds relationship satisfaction dips after the birth of a child — and that the quality of the co-parenting alliance, more than the amount of conflict, predicts child outcomes.

> **CASE**
> A couple presents with "no time for each other" four months after their first baby. The developmental frame normalises the strain (this is the most common point of relational collapse), then asks the systemic question: which grandparent has been recruited into the parenting triangle, and what did the couple stop negotiating when the grandmother moved in?

Practice. In pairs, conduct a 10-minute temperament interview with a parent (or peer playing one): sleep, feeding, reaction to change, intensity, mood. Then identify the goodness of fit with the household.

Week 3 — Infancy: attachment, synchrony and regulation

The idea. Attachment is not "bonding" and not "love"; it is a behavioural system that organises a child's strategies for seeking safety from a caregiver under stress.

The classic patterns (Ainsworth's Strange Situation; see Figure 17 in MFT 710): secure, insecure- avoidant, insecure-resistant/ambivalent, and disorganised (added by Main & Solomon, associated with frightening or frightened caregiving). In adulthood the same dimensions appear as attachment styles.

Synchrony and rupture–repair. Healthy development does not require perfect attunement — it requires frequent enough attunement and effective repair. Tronick's still-face experiments show how quickly infants respond to a caregiver's unresponsiveness, and how quickly they recover when the caregiver returns.

Co-regulation is the precursor of self-regulation. A baby does not learn to calm alone; they learn to calm with someone, repeatedly, until the pattern is internalised. This has direct clinical implications: telling an overwhelmed parent to "let him cry it out" without attention to the family's capacity and beliefs is a technique imposed on a relationship.

> **PRACTICAL**
> Watch a consented 3-minute video of a caregiver and infant (or a still-face clip). Note: gaze, vocalisation, touch, pace. Then describe one moment of rupture and one of repair in behavioural language. Avoid interpretation for the first five minutes.

Week 4 — Toddlerhood: autonomy, language and limits

Developmental tasks. Autonomy versus shame and doubt (Erikson); language explosion (from ~50 words at 18 months to 300+ and two-word combinations by 24 months); symbolic play; beginning of self-regulation.

The clinical translation of "no". A toddler's "no" is the emergence of a separate self. The parental task is to hold the limit without humiliating the child — "You may not hit. You may be angry." This is the earliest training ground for the emotional vocabulary the family will use for the next fifteen years.

Language and family. Delay in expressive language is the most common developmental concern presented to clinics. Distinguish: late talker (normal variation with good comprehension), bilingual language confusion (normal), hearing loss, global delay, and autism spectrum presentation. Always ask about **hearing** and about **which language is spoken at home and by whom**.

> **CASE**
> A grandmother raises a 2-year-old while both parents work. She reports he "does not listen". Assessment finds: he is spoken to mostly in commands, in two languages, by three adults with different rules. The intervention is not a behaviour chart; it is a family agreement about one set of five rules, one language of instruction, and ten minutes of child-led play a day with each caregiver.

Week 5 — Early childhood: play, theory of mind, initiative

Tasks. Initiative versus guilt; fantasy play; theory of mind (understanding that others have different beliefs — typically emerging around 4); moral development begins as consequence-based reasoning; school readiness.

Play is the work. Play develops language, self-regulation, perspective-taking, and the capacity to tolerate frustration. Children who have experienced adversity often play less

symbolically and more repetitively — a clinical observation that underpins play therapy (MFT 727).

The clinical skill: observing play without hijacking it. Describe what the child does; do not direct it, and do not interpret it prematurely ("that must be your father"). Interpretation before trust produces a performance, not a communication.

> **PRACTICAL**
> Ten minutes of child-led play with a volunteer child or peer. Your only tools: describe, reflect, imitate, and set one limit. Debrief: how many times did you ask a question? (Aim for fewer than three.)

Week 6 — Middle childhood: industry, peers, school

Tasks. Industry versus inferiority; mastery and competence; peer group and friendship rules; self-concept becomes comparative ("I am good at maths, bad at football"); moral reasoning becomes more reciprocal.

What changes in the family. The child's world widens; parents move from direct control to monitoring and scaffolding. School becomes a second system with its own rules, and the family–school interface becomes a clinical site.

School refusal — a developmental and systemic differential. Ask: is this anxiety, separation difficulty, bullying, a learning difficulty, a family crisis, or a bid to protect a parent? The distinction matters: the same behaviour has opposite treatment implications.

> **CASE**
> A nine-year-old refuses school on Mondays only. Developmental enquiry: what happens on Sunday nights? The answer: the parents' weekly argument about money. The child's presence at home on Monday keeps the mother from leaving for work, and the argument stops. The symptom is a Monday-shaped solution to a marital problem — and it is also genuinely distressing to the child.

Week 7 — Adolescence I: puberty, identity, autonomy

Tasks. Identity versus role confusion; pubertal change; separation-individuation without severance; peer belonging; emerging sexuality; moral and ideological commitment.

The neuroscience, briefly. The adolescent brain is not "broken": it is specialised. The reward system matures before the regulatory system, which produces sensation-seeking and heightened sensitivity to peer approval alongside gradually improving impulse control. This is an argument for scaffolding, not for lecturing.

The family task. To renegotiate the relationship: less control, more influence; more privacy, more accountability. Families that cannot renegotiate produce two common

patterns: escalation (control increases as autonomy increases) and abdication (parents withdraw entirely).

> **CASE**
> Amara, 16. The parents describe her as "secretive"; she describes them as "spies". The developmental reframe: privacy is a developmental need, and secrecy is what privacy becomes when there is no negotiated trust. The intervention is a negotiation: what will she share, what will they ask, and what happens when the agreement is broken?

Week 8 — Adolescence II: risk, peers, digital life and mental health

Risk behaviour is normative in moderation and dangerous in combination: substance use, risky driving, unsafe sex, self-harm, disordered eating. Risk is not a moral category; it is a developmental phenomenon shaped by peers, family, school and access.

Digital life. Assess function, not just quantity: what is the phone doing in this adolescent's life — connection, escape, status, self-soothing, harm? Ask about sleep, cyberbullying, sexual content, and the family's rules about devices. Banning without substituting an alternative rarely works.

Mental health. Half of all lifetime mental disorders begin by mid-adolescence. Screen, do not guess. Distinguish transient distress from disorder by **duration, impairment and severity** — and use the same developmental question: is this a normal task being struggled with, or a disorder?

> **PRACTICAL**
> Conduct a 12-minute adolescent interview with a peer who has been briefed to be reticent. You may not ask more than two questions in a row without reflecting something back. Observer records: ratio of questions to reflections; did the adolescent elaborate?

Week 9 — Adult development

Tasks. Intimacy versus isolation; career and work identity; generativity versus stagnation; the "social clock" (the culturally shared timetable for marriage, children, promotion, retirement).

Why this matters clinically. Most couple problems presented in clinics are adult developmental problems wearing a relational mask: a 32-year-old's struggle with intimacy, a 45-year-old's reckoning with a career, a 58-year-old's grief for the life they did not have.

Life structure and the "seasons" of adulthood (Levinson; vaillant's longitudinal work). Treat these as metaphors with empirical support, not as laws. The strongest empirical

finding is **continuity with change**: people change, but in patterned ways that reflect early relationships and current context.

> **CASE**
> A 45-year-old man presents with "irritability". Developmental enquiry finds two things: his father died at 45, and his eldest child is about to leave for university. He is not depressed in the clinical sense; he is renegotiating mortality and role. The intervention is narrative and relational — mourning the father he barely knew, and planning what his own next decade is for.

Week 10 — Couple development

Couples develop too. A workable map (used again in MFT 710 and 718):

Phase	Central task	Common failure
Formation	Commitment and identity as a couple	One partner never really chooses
Early marriage	Building rituals and boundaries with families of origin	In-law triangles
Young children	Dividing labour; protecting the couple bond	Co-parenting replaces partnering
Adolescent children	Re-negotiating autonomy, revisiting one's own adolescence	Escalation or abdication
Launching	Re-intimacy after children leave	The "empty nest" that was already empty
Later life	Facing illness, retirement, loss together	Caretaking replaces relating

> **PRACTICAL**
> With a peer, draw a couple timeline: five high points, five low points, one turning point each. Then ask: which developmental transition explains the lowest point? Present in 5 minutes.

Week 11 — Parenting across stages and talking to parents

Parents do not need lectures; they need **accurate, specific, hopeful information** in plain language. The skill has four parts:

1. **Normalise first.** "What you are describing is very common at this age."
2. **Give one developmental fact.** "At four, the part of the brain that manages impulse is still being built, so he cannot yet do what you are asking consistently."
3. **Give one concrete action.** "Try naming the feeling and offering two acceptable choices."

4. **Set a review.** "If this is still happening in three weeks, or if X or Y appears, come back."

What to avoid: technical vocabulary; predictions ("he will grow out of it"); blame; over-reassurance when the concern is real; and overwhelming a parent with five recommendations.

> **PRACTICAL (OBSERVED)**
> Ten-minute parent-feedback role-play using the four-part structure. Assessor rates: plain language, one fact, one action, review point, and warmth.

Week 12 — Later life

Tasks. Integrity versus despair; adaptation to retirement; bereavement; caregiving; grandparenthood; legacy and meaning.

Clinical realities. Cognitive change must be distinguished from delirium, depression and dementia; hearing loss is a major and under-treated contributor to isolation and apparent confusion. Caregiving burden falls disproportionately on women and on the "responsible" child, and it frequently reopens old sibling hierarchies.

Grief. Use the dual-process model (Figure 22, MFT 712): healthy grieving oscillates between loss-oriented and restoration-oriented activity. In many cultures, mourning is collective and prescribed; the clinician's task is to support the rites rather than to impose a private model of "working through".

> **CASE**
> Three siblings disagree about their mother's care. The eldest, a daughter, has done everything; the youngest, a son abroad, sends money and criticises. The clinical task is not to decide who is right but to make the loyalty ledger explicit: who owes what, to whom, and by what accounting? (This is contextual therapy's territory; see MFT 702 and 715.)

Week 13 — Culture, poverty and development

Development is not culture-free. Three critiques you must be able to state:

1. **The norms are Western.** Attachment classifications, individuation timetables and "school readiness" were normed on particular populations. A child who sleeps in the parental bed until five is not necessarily enmeshed; in many cultures this is ordinary.
2. **Poverty changes development.** Chronic stress, food insecurity, crowding and insecure housing affect attention, sleep, and the capacity for consistent parenting. Diagnosing "inconsistent parenting" without naming material conditions is a category error.

3. **Extended and distributed caregiving is normal.** In much of the world, children are raised by networks — grandparents, aunts, older siblings, co-wives, foster kin. Attachment theory travels better when it is asked "who does this child seek when frightened?" rather than "is the mother attuned?"

> **CRITICAL READING**
> Read one cross-cultural critique of attachment theory and one study of caregiving in a non-Western setting. Write 500 words on what you would change in your own assessment template as a result.

Week 14 — When development goes wrong

Build a differential habit. For any concern, run the four-category check (variation, delay, regression, symptom) and then ask the five systemic questions:

1. **When did it start, and what else changed then?**
2. **Who noticed first, and who disagrees?**
3. **What does the family do when it happens, and what happens next?**
4. **What would improve if it stopped — and what would get worse?**
5. **What has already been tried, and what did that teach us?**

Presentation	Developmental differential	Systemic differential
Aggression at 5	Language delay; impulsivity; trauma	Parental conflict; inconsistent limits; a new baby
Withdrawal at 12	Depression; bullying; neurodivergence	Family secrecy; a parent's illness; migration
Decline at 70	Delirium; dementia; depression; hearing loss	Bereavement; relocation; loss of role
"Not listening" at 3	Hearing; language; attention	Multiple caregivers, conflicting rules

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Week 15 — Integration

The developmental lens does three things for your clinical work: it **calibrates** your expectations, it **dates** the problem (onset relative to transitions), and it **predicts** what the family must renegotiate next. Carry three questions into every first session: What is this person's developmental task right now? What is the family's corresponding task? And what would "on track" look like in six months?

Glossary

Attachment — a behavioural system organising proximity-seeking to a caregiver under threat. **Co-regulation** — the process by which a caregiver's calm and responsiveness regulate the child's arousal. **Developmental task** — the psychosocial challenge characteristic of a life stage. **Disequilibrium** — a period of developmental reorganisation marked by increased conflict or regression. **Goodness of fit** — the match between a child's temperament and the demands of the environment. **Regression** — loss of previously acquired function; a red flag. **Scaffolding** — temporary support that allows a child to do what they cannot yet do alone. **Separation-individuation** — the process of developing autonomy while remaining connected. **Social clock** — culturally shared expectations about the timing of life events. **Theory of mind** — the capacity to attribute mental states to others. **Zone of proximal development** — the gap between what a learner can do alone and with support.

Self-assessment

Multiple choice

1. A four-year-old who stops speaking after a family bereavement is best described as: (a) normal variation (b) delay (c) regression (d) attention-seeking

2. "Goodness of fit" refers to the match between: (a) child and school (b) temperament and environment (c) parents' goals (d) IQ and achievement

3. The most common developmental concern presented to clinics is: (a) motor delay (b) expressive language delay (c) sleep (d) feeding

4. Disorganised attachment is most associated with: (a) daycare (b) frightening or frightened caregiving (c) prematurity (d) bilingualism

5. A toddler's frequent "no" primarily reflects: (a) oppositional disorder (b) emerging autonomy (c) poor parenting (d) language delay

6. Which is not a feature of adolescent brain development? (a) heightened reward sensitivity (b) peer sensitivity (c) mature impulse control (d) later prefrontal maturation

7. A "social clock" refers to: (a) sleep rhythms (b) culturally shared timing of life events (c) school timetable (d) biological ageing

8. In the dual-process model, healthy grieving involves: (a) staying on the loss track (b) avoiding reminders (c) oscillating between loss and restoration (d) suppressing emotion

9. Apparent confusion in an older adult should first prompt assessment of: (a) personality (b) delirium, depression, hearing, dementia (c) family conflict (d) medication refusal only

10. Extended-family caregiving should be assessed by asking: (a) whether the mother is attuned (b) who the child seeks when frightened (c) the parents' marital status (d) the family's income

Answer key

1 c · 2 b · 3 b · 4 b · 5 b · 6 c · 7 b · 8 c · 9 b · 10 b

Short answer

1. Distinguish delay from regression, with one clinical example of each.
2. Name three developmental tasks of adolescence and the corresponding family tasks.
3. Why is "let him cry it out" a relational intervention, not only a behavioural one?
4. Give two cultural critiques of attachment theory and one way to adapt your assessment.
5. What are the four parts of effective parent feedback?

Essay (choose one, 1,800 words)

1. "Normal development is a cultural achievement, not a biological fact." Discuss with reference to attachment and independence.
2. Using a disguised case, show how a developmental formulation changed your understanding of a presenting problem that had been framed as behavioural.
3. Critically evaluate the evidence that couple satisfaction declines after the birth of a child, and discuss the clinical implications.

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Next: MFT 704 — Ethical and Legal Issues in Therapy

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The books are designed to be read straight through before the semester begins, revisited week by week, and used again in revision. Cross-references point to the other volumes where a topic is treated in more depth.

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A note on the case material

All families, couples and individuals in this book are fictional composites. They are written to teach pattern recognition, not to describe any real person. Nothing here replaces supervised clinical practice, professional registration, legal advice or medical care; legal content is orientation only and must be checked against the law of the relevant jurisdiction.

The lecture library

Twenty-eight self-contained course books carry the taught programme: eight in Semester One, seven in Semester Two, seven in Semester Three including the thesis, and six electives. The volume you are holding is marked in the margin.

Code	Course	Semester	Units
MFT 701	Fundamentals of Marriage and Family Therapy	One	1.71
MFT 702	Theories of Family Therapy	One	1.71
MFT 703	Human Development Across the Lifespan	One	1.71
MFT 704	**Ethical and Legal Issues in Therapy**	One	1.71
MFT 705	Research Methods in Marriage and Family Therapy	One	1.71
MFT 706	Clinical Practice and Techniques	One	1.71
MFT 707	Assessment and Diagnosis in Family Therapy	One	1.71
MFT 708	Cultural Competence in Therapy	One	1.71
MFT 709	Group Therapy Techniques	Two	1.71
MFT 710	Couples Therapy	Two	1.71
MFT 711	Child and Adolescent Therapy	Two	1.71
MFT 712	Crisis Intervention Strategies	Two	1.71
MFT 713	Substance Abuse and Family Dynamics	Two	1.71
MFT 714	Trauma-Informed Care	Two	1.71
MFT 715	Family Systems Theory	Two	1.71
MFT 716	Communication Skills in Therapy	Three	1.71
MFT 717	Psychopathology in Families	Three	1.71
MFT 718	Intimacy and Relationship Dynamics	Three	1.71
MFT 719	Family Life Cycle and Transitions	Three	1.71
MFT 720	Supervision and Consultation in Therapy	Three	1.71
MFT 721	Research Methodology	Three	1.71
MFT 722	Professional Development and Self-Care	Electives	1.71
MFT 723	Special Populations in Therapy	Electives	1.71
MFT 724	Family Law and Advocacy	Electives	1.71
MFT 725	Integrative Approaches in Therapy	Electives	1.71
MFT 726	Neurobiology of Relationships	Electives	1.71
MFT 727	Play Therapy Techniques	Electives	1.71
MFT 770	Master Thesis	Three	1.71

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Course leader's summary. This course trains judgement, not just compliance. You will learn the codes, the law, and — more importantly — how to reason when the codes and the law point in different directions, when the family's culture conflicts with the institution's policy, and when your own values are in the room.

How to use this book

Every week presents a **standard**, a **case**, and a **decision**. Ethics is not memorised; it is practised. The weekly incidents in this book are drawn from the ordinary texture of clinical life — the secret, the subpoena, the gift, the neighbour, the unpaid fee — because those are the situations that actually end careers.

1. Course description

Family therapy multiplies ethical complexity. In individual work there are two parties (you and the client). In family work there may be six, with conflicting interests, unequal power, and secrets that each person believes belongs to them. Add a school, a child protection agency, a court, an insurance company, and a faith community, and the question "who is my client?" becomes the central professional question of the course.

The course combines **normative ethics** (codes, duties, principles), **law** (the parts a clinician must know and the parts a clinician must refer), and **professional formation** (the kind of clinician you are becoming).

2. Learning outcomes

#	Outcome	Assessed
LO1	Apply the major ethical codes and principles to relational cases	Exam, case analysis
LO2	Explain confidentiality and its limits in multiperson therapy	Exam
LO3	Conduct and document a risk assessment for suicide, self-harm, violence and abuse	Skills assessment
LO4	Manage secrets, dual relationships, gifts, fees and boundary dilemmas defensibly	Case analyses
LO5	Describe the legal framework for marriage, divorce, custody and child protection in Nigeria	Exam
LO6	Use a structured decision-making model and document the reasoning	Assignment
LO7	Recognise when to refer for legal advice, and how to do so	Viva

3. Assessment

Component	Weight	Due
Ethics incident analyses (4 × 600 words)	30%	Weeks 4, 6, 9, 11
Risk assessment observed skills (15 min)	20%	Week 10
Examination (case-based)	50%	Week 15

4. The fourteen-week plan

Week	Topic	Practical
1	Moral philosophy for clinicians: duties, consequences, virtues, care	Trolley variants in therapy
2	The codes: AAMFT, IAMFC, APA, national associations; informed consent	Consent form drafting
3	Confidentiality in couple and family therapy	Secrecy protocol design
4	Secrets I: the individual disclosure	Scripted incident
5	Secrets II: infidelity, paternity, HIV status	Complex case debate
6	Boundaries and multiple relationships	Boundary audit
7	Competence, supervision, and the duty to refer	Competence self-audit
8	Risk I: suicide and self-harm	**Observed risk assessment**
9	Risk II: intimate partner violence and child protection	Safety planning
10	Legal framework: marriage, divorce, custody, maintenance	Court-report exercise
11	Records, documentation, subpoenas, court testimony	Note-writing lab
12	Money, gifts, barter, and the business of therapy	Fee policy workshop
13	Ethics of research, assessment and diagnosis	Consent and deception cases
14	Integration: your ethical stance and decision model	Viva

Week 1 — Moral philosophy for clinicians

You do not need to become a philosopher, but you need to know which argument you are making when you say "that would be wrong".

Tradition	Central question	Therapy example
Deontology (duty)	What are my duties regardless of outcome?	Keeping confidentiality even when disclosure would be convenient
Consequentialism	What produces the best overall outcome?	Breaking confidentiality to prevent a serious harm
Virtue ethics	What would a person of good character do?	Refusing a gift that would create obligation

Tradition	Central question	Therapy example
Ethics of care	What does this relationship require of me?	Attending to the family's dependence on you
Justice / rights	Are power and resources fairly distributed?	Whose voice is missing from this room?

The four principles (Beauchamp & Childress) remain the most portable framework: **autonomy, beneficence, non-maleficence, justice**. In family work they collide constantly — respect for the father's autonomy may conflict with the mother's safety.

> **PRACTICAL**
> Take three ethical dilemmas from your own life (not clinical). For each, identify which tradition you used, and which you used to justify it afterwards.

Week 2 — The codes and informed consent

What codes do and do not do. Codes set a floor, not a ceiling. They are written to be enforceable, which means they are often silent on the hardest questions. They also vary by jurisdiction and by professional body; know which one governs you.

The core obligations across the major codes:

1. Competence and its limits
2. Informed consent (including consent for recording and for supervision)
3. Confidentiality and its limits
4. Multiple relationships
5. Sexual intimacy with clients and former clients (prohibited; time limits vary)
6. Fees and financial arrangements
7. Advertising and representation
8. Research ethics
9. Cultural competence and non-discrimination
10. Reporting obligations and legal compliance

Informed consent in family therapy is a document and a conversation. It should cover: who the client is; who is seen and in what combination; the confidentiality policy including secrets; recording and supervision; fees and cancellation; the limits of confidentiality (risk, abuse, court order); and the complaint route. Review it aloud in the first session, in the family's language, and record that you did.

> **PRACTICAL**
> Draft a one-page informed-consent document for couple therapy. Exchange with a peer and identify the three sentences most likely to be misunderstood by a family with limited literacy. Rewrite them.

Week 3 — Confidentiality in the multiperson system

There is no single confidentiality in family work; there is a **policy** you negotiate.

Three workable models.

Model	Rule	Advantages	Risks
No-secrets	Anything told individually may be shared with the couple	Simple, transparent	Individual may withhold important material
Full secrecy	Individual disclosures are kept private	Protects individual therapy	Creates a therapeutic secret that distorts the couple work
Selective (negotiated)	The therapist and client agree in advance what will be shared	Flexible and realistic	Requires skill and documentation

Most experienced couple therapists operate a **modified no-secrets policy**: they will not keep a secret that materially affects the couple's work, and they tell each partner this before the first individual session. This protects both the relationship and the therapist.

Documentation. Record the policy in the file, in the family's words if possible, and note the date it was explained. If a secret arises, record what you did and why.

> **CASE**
> During an individual session, a wife discloses that she has been having an affair for two years and has no intention of ending it or telling her husband. No one is at physical risk. Under a no-secrets policy, you tell her at the outset of the individual session that this cannot stay private if the couple work is to continue; you then help her decide how and when to disclose, with a deadline. Under a selective policy, you would negotiate what can be shared. What you must not do is carry the secret silently for months while conducting "couple therapy".

Week 4 — Secrets I: the individual disclosure

Classify before you respond. Three questions: Is anyone at risk? Does this information change the treatment I am providing? Is the client able to disclose it themselves, with support?

Type of secret	Typical response
Harmless (a surprise party)	Keep it; it is not clinically material
Clinically material, no risk (an affair; a debt; a diagnosis)	Negotiate disclosure with a deadline; do not collude
Risk to a third party (HIV status undisclosed; threats)	Assess; follow legal and public-health obligations; document

Type of secret	Typical response
Risk to a child or vulnerable adult	Report; inform; protect
Risk to self	Risk assessment; safety planning; break confidentiality as required

The clinical skill: refusing to be recruited. "I understand you are not ready to tell him. I cannot keep this and also work on your marriage with you, because I would be working with half the truth. Let us plan how you will tell him, and when."

The systemic reality. Secrets are not only information; they are structure. A secret held by one partner and the therapist creates a coalition — and the excluded partner often senses it, which damages the therapy's credibility.

Week 5 — Secrets II: infidelity, paternity, HIV status

Three cases that appear in every couple therapist's career, each with a different legal overlay.

Infidelity. Not a reportable act. The clinical tasks: stop the bleeding, contain the disclosure, assess for sexually transmitted infection risk, address the meaning of the affair (not only its fact), and decide with the couple whether disclosure to a third party (the affair partner's spouse) is required. That last question is a values question with safety implications; consult.

Paternity. A man may discover that a child he is raising is not biologically his. The therapist must not conceal information that affects a child's welfare, must not give legal advice, and must help the family decide who needs to know and in what sequence. Refer for legal advice on paternity, maintenance and custody; the emotional work is yours.

HIV and other communicable conditions. Confidentiality is strong, but public-health law may create duties where a partner is at identifiable risk and the client refuses disclosure. Know your jurisdiction's rule, consult, and document. Never disclose without advice.

> **DEBATE**
> "A therapist who keeps an affair secret for six months is colluding in the harm the affair causes." Argue for and against, then write your own policy in 200 words.

Week 6 — Boundaries and multiple relationships

Definition. A multiple relationship exists when you have another significant relationship with a client or someone closely associated with them. Not all are unethical; all require examination.

The test. (1) Could the second relationship impair your objectivity? (2) Could it exploit the power difference? (3) Would the client experience it as exploitative later? (4) Is it avoidable?

Common in small communities.

Situation	Risk	Defensible approach
Seeing a neighbour's family	Overhearing information; social contact	Discuss openly; agree how to behave at community events
Client is a fellow church member	Role confusion; spiritual authority	Clarify roles; consider referral if you hold a leadership role
A relative of your child's classmate	Dual loyalty	Disclose in supervision; transfer if the risk is unmanageable
Client asks you to be a character witness	Role change; advocacy	Decline; explain; refer to a professional who can assess
Gift-giving after a bereavement	Cultural obligation vs boundary	Accept small cultural gifts; document; discuss meaning
Trading therapy for services	Exploitation risk	Usually decline; if unavoidable, document value and rationale

Touch, self-disclosure, and home visits. Each is sometimes therapeutic and always requires a rationale you can articulate in supervision. The test is never "did it feel right?" but "whose need did it serve?"

> **PRACTICAL**
> Complete a boundary audit of your present life: community roles, family ties, business interests, social media. Identify the three situations most likely to produce a dual relationship in the next two years, and pre-decide your response.

Week 7 — Competence and referral

Competence has four dimensions: knowledge, skills, experience, and appropriateness (personal and cultural fit). You can be knowledgeable and inappropriate: a therapist with unprocessed trauma working intensively with trauma, or a young male therapist seeing a couple in a culture where his authority is contested.

The duty to refer arises when: the problem is outside your training; you lack supervision for it; your personal reactions would compromise care; or the client needs a different modality or medical input.

How to refer well. Name the reason without shaming ("this needs someone with specific training in X"); make a warm handover (a call, not just a name); offer a bridge session if needed; follow up once; and document. A referral is not abandonment.

> **SELF-AUDIT**
> List the ten presenting problems you are most likely to see. For each, rate your competence 1–5 and name the supervisor or service you would refer to. The two lowest ratings are your professional development plan for this year.

Week 8 — Risk I: suicide and self-harm

The standard. Ask directly, ask specifically, ask about means, and document. There is no evidence that asking about suicide increases risk, and substantial evidence that clinicians fail to ask.

A defensible risk assessment includes:

1. **Ideation** — frequency, intensity, duration, controllability.
2. **Plan** — specific method, place, time.
3. **Intent** — "Do you intend to act on this?"
4. **Means** — access to firearms, medication, pesticides, heights.
5. **Previous attempts** — the strongest single predictor.
6. **Protective factors** — reasons for living, responsibility to children, faith, support.
7. **Substance use and impulsivity.**
8. **Family history.**
9. **Current stressors and recent losses.**
10. **Disclosure and support network** — who can be told, who can watch.

Documentation. Record the questions you asked and the answers, your formulation of risk (low/moderate/ high), the plan, who was informed, and the review date. If you decide not to break confidentiality, record why.

In family work. A parent's suicidality, an adolescent's self-harm and a child's disclosure must be handled without turning the family session into an interrogation. Structure: see the person individually first, agree what will be shared, then facilitate the sharing.

> **PRACTICAL (OBSERVED)**
> Fifteen-minute suicide risk assessment role-play with a trained actor. You must ask all ten elements, formulate risk, and agree a safety plan. Pass/fail.

Week 9 — Risk II: intimate partner violence and child protection

Distinguish two patterns.

Situational couple violence	Coercive control
Mutual, low-level, escalates under stress	One partner systematically dominates
Fear is situational	Fear is chronic; surveillance, isolation, threats
Couple therapy may be appropriate after assessment	Couple therapy is contraindicated; risk increases
Safety planning individual	Specialist advocacy and protection

Screening. Screen each partner **separately**, at least once, using direct questions: "Has your partner ever hit, kicked, or pushed you? Has he/she ever made you afraid? Do you feel safe going home today?" Never screen in front of the partner.

Child protection. Know your jurisdiction's reporting duty. In Nigeria, the Child Rights Act (2003, adopted in many but not all states) and state child protection laws create duties and structures; the practical route is usually the state ministry responsible for social welfare, the police family unit, or a designated safeguarding lead. Document the concern, the advice received, and the action taken.

> **CAUTION**
> Conjoint sessions can escalate violence after the session. If there is any indication of coercive control, separate sessions and a safety plan precede any conjoint work. Write it down.

Week 10 — The legal framework a clinician must know

This section is **orientation, not legal advice**. Statutes and state practice vary; verify with a lawyer.

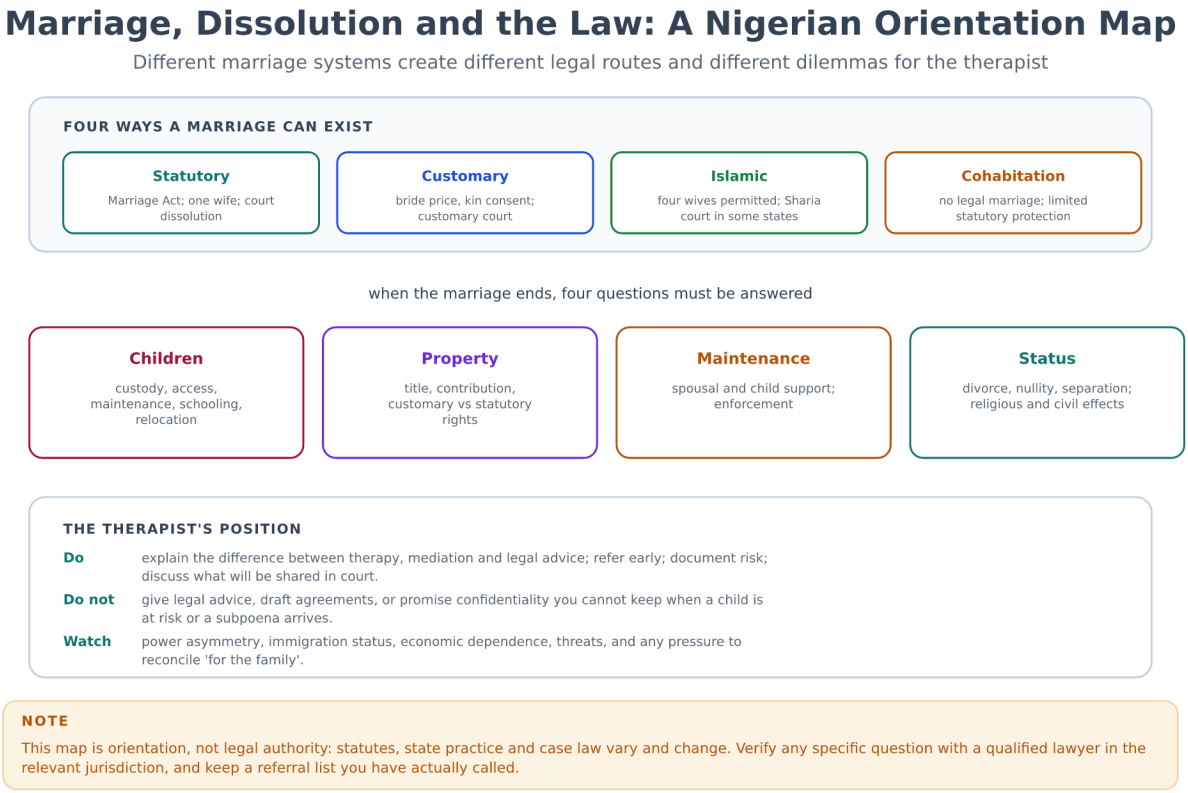


Figure 10.1 — Marriage systems and dissolution questions in Nigeria

Marriage in Nigeria exists in four overlapping forms: statutory (Marriage Act), customary, Islamic, and cohabitation without legal marriage. A family may be married under more than one system.

Dissolution. Statutory divorce requires a court petition (Matrimonial Causes Act, with the two-year rule and recognised grounds); customary and Islamic dissolutions follow their own processes and courts. The therapist's role is not to advise on grounds but to help the family clarify what they are deciding and to refer.

Children. Custody and access decisions are governed by the welfare principle — the child's best interests. Maintenance obligations exist under both statutory and customary law. The therapist may be asked for a report; a treating therapist should generally not conduct a forensic custody evaluation of their own client (role conflict).

Other relevant areas: the Child Rights Act; the Violence Against Persons (Prohibition) Act 2015 (applicable in the FCT and in states that have adopted it); mental health legislation in the states that have adopted the Mental Health Act 2021 framework; data protection obligations regarding client records.

Week 11 — Records, subpoenas and court

Write notes for three readers: your future self, a colleague taking over, and a court. Use factual, behavioural language; avoid speculation and avoid character judgements. Record: date, attendees, risk screen, interventions, plan, and who was told what.

If a subpoena or court order arrives: do not ignore it; do not respond immediately without advice; verify its validity; contact your supervisor, indemnity provider or lawyer; inform the client (unless prohibited); release only what is required, with a privilege log if advised.

Testifying. Your role is to report what you observed, not to advocate for a preferred outcome. Answer the question asked; do not volunteer; do not guess; say "I don't know" when you do not. Distinguish fact, clinical opinion and hypothesis.

> **PRACTICAL**
> Note-writing lab. Write a progress note for a session in which a risk was disclosed and a report to a school was agreed. Exchange; grade each other against the "three readers" standard.

Week 12 — Money, gifts and the business of therapy

Fees are clinical material. They carry meaning about worth, dependency, entitlement and shame. Have a written policy; state it at the start; apply it consistently; discuss non-payment rather than avoiding it.

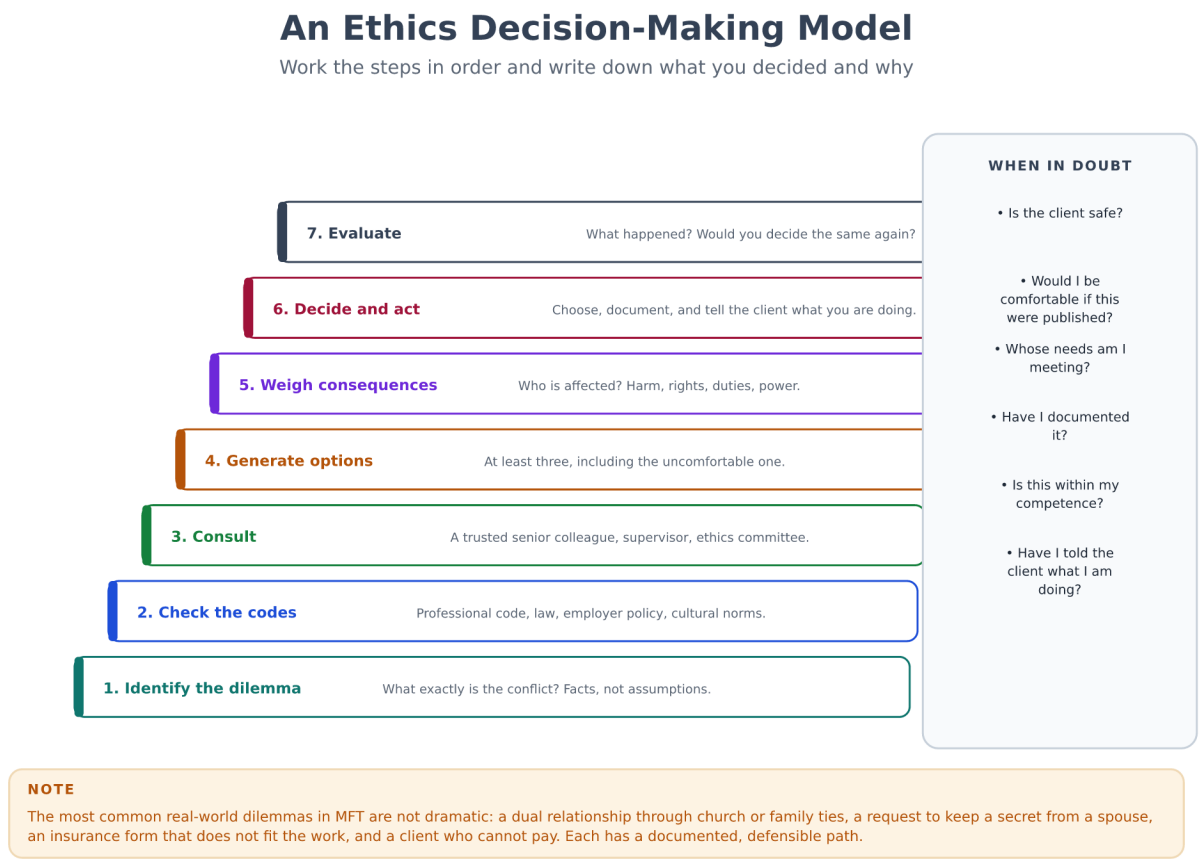
Refusing to reduce a fee sometimes ends therapy; never reducing a fee sometimes ends therapy. Both are true. Decide your sliding scale in advance and document the reason for each exception.

Gifts. Small cultural gifts may be accepted with documentation; expensive gifts should be discussed; gifts with strings should be declined. Ask, "What would it mean to you if I accepted / declined?"

Week 13 — Ethics of research, assessment and diagnosis

- **Informed consent** for research must be voluntary, capacity-appropriate and revocable; families, children and people in crisis require special care.
- **Deception** requires justification, debriefing, and ethics approval.
- **Diagnosis** carries consequences: access to care, stigma, insurance, custody. Diagnose what you can defend and describe what you cannot.
- **Assessment instruments** must be validated for the population and language; using an unvalidated instrument and acting on it is an ethical problem, not just a methodological one.

Week 14 — Integration: the decision model and your stance



NOTE

The most common real-world dilemmas in MFT are not dramatic: a dual relationship through church or family ties, a request to keep a secret from a spouse, an insurance form that does not fit the work, and a client who cannot pay. Each has a documented, defensible path.

Figure 14.1 — A seven-step decision-making model for ethical dilemmas

The seven steps. Identify the dilemma → check codes and law → consult → generate at least three options including the uncomfortable one → weigh consequences for each affected party → decide, act and document → evaluate afterwards.

The documentation habit. A defensible decision is not the same as a correct decision; it is one you can explain. Write the reasoning at the time, not after a complaint.

Your stance. Write 500 words: what you will never do; what you will always consult about; and where your personal values are most likely to distort your clinical judgement.

Glossary

Autonomy — the client's right to self-determination. **Beneficence / non-maleficence** — the duty to benefit and the duty not to harm. **Coercive control** — a pattern of domination, isolation and threat, distinct from situational violence. **Confidentiality** — the duty not to disclose information learned in a professional relationship. **Dual relationship** — a second significant relationship with a client or someone close to them. **Duty to warn / protect** — a legal obligation, in some jurisdictions, to act on a serious threat. **Informed consent** — voluntary agreement based on adequate information and capacity. **Mandated reporting** — a legal duty to report suspected abuse or neglect. **Privilege** — the client's legal right to prevent disclosure in court proceedings. **Subpoena** — a legal order requiring documents or testimony. **Welfare principle** — the rule that a child's best interests govern decisions about them.

Self-assessment

Multiple choice

1. In couple therapy, the most defensible approach to individual secrets is: (a) keep everything private (b) declare a policy in advance and apply it (c) tell the partner immediately (d) refuse individual sessions entirely

2. Asking a client directly about suicide: (a) increases risk (b) does not increase risk and is required practice (c) is only for psychiatrists (d) should be avoided in family sessions

3. Couple therapy with a couple in which there is coercive control is: (a) recommended (b) contraindicated until safety is established (c) harmless (d) the only legal option

4. The four principles of biomedical ethics are: (a) autonomy, beneficence, non-maleficence, justice (b) consent, capacity, confidentiality, competence (c) duty, utility, virtue, care (d) respect, honesty, integrity, courage

5. A treating therapist asked to conduct a custody evaluation for their own client should: (a) accept (b) accept if paid (c) decline because of role conflict (d) accept with supervision

6. The welfare principle means decisions about children are governed by: (a) parental preference (b) the child's best interests (c) the court's convenience (d) financial capacity

7. The strongest single predictor of a suicide attempt is: (a) diagnosis (b) previous attempt (c) gender (d) family income

8. A small cultural gift after a bereavement should usually be: (a) refused (b) accepted, discussed and documented (c) accepted silently (d) reported to the police

9. Screening for intimate partner violence should be done: (a) with the couple present (b) with each partner separately (c) by questionnaire only (d) only if asked

10. A defensible ethical decision is one that: (a) everyone agrees with (b) can be explained and documented (c) produces the best outcome (d) follows the client's preference

Answer key

1 b · 2 b · 3 b · 4 a · 5 c · 6 b · 7 b · 8 b · 9 b · 10 b

Short answer

1. State a confidentiality policy for couple therapy in six sentences a family could understand.
2. List the ten elements of a defensible suicide risk assessment.
3. Distinguish situational couple violence from coercive control, and state the treatment implication.
4. What should you do when you receive a subpoena? Five steps.
5. Why is "I know it is unethical but it was the right thing to do" an inadequate defence?

Essay (choose one, 2,000 words)

1. "The most difficult ethical problems in family therapy are not conflicts between right and wrong but between two rights." Discuss with three cases.
2. A father discloses that his 14-year-old daughter is self-harming and asks you not to tell her mother. Analyse the ethical, legal and clinical dimensions and set out your plan.
3. Critically examine the claim that the professional codes are culturally biased in favour of Western individualism.

Further reading

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Next: MFT 705 — Research Methods in Marriage and Family Therapy

MSC MARRIAGE AND FAMILY THERAPY

LECTURE LIBRARY · VOLUME 5 OF 28

MFT 705

Research Methods in Marriage and Family Therapy

14 teaching weeks · 13 practicals · 2 figures

SEMESTER ONE

1.71 units · Prerequisite: MFT 701

> “You will become a consumer who can read a paper in ten minutes and know what it is worth, and a producer who can design a study you can actually finish.”

One of 28 self-contained course books in the MSc Marriage and Family Therapy lecture library: a full taught semester per volume, with cases, practicals, assessment briefs, glossaries and model answers.

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MFT 705 — Research Methods in Marriage and Family Therapy

MSc Marriage and Family Therapy · Semester One · 1.71 Units · 14 teaching weeks · Prerequisite: MFT 701

This volume is one of 28 course books that together cover the whole MSc Marriage and Family Therapy taught programme. Each book is written to carry a 14–16 week semester on its own: every week pairs a core idea with the practice that follows from it, a worked case, and a reflective task, and each book closes with an integration chapter, a glossary and a self-assessment paper with model answers.

The books are designed to be read straight through before the semester begins, revisited week by week, and used again in revision. Cross-references point to the other volumes where a topic is treated in more depth.

How to cite this volume

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A note on the case material

All families, couples and individuals in this book are fictional composites. They are written to teach pattern recognition, not to describe any real person. Nothing here replaces supervised clinical practice, professional registration, legal advice or medical care; legal content is orientation only and must be checked against the law of the relevant jurisdiction.

The lecture library

Twenty-eight self-contained course books carry the taught programme: eight in Semester One, seven in Semester Two, seven in Semester Three including the thesis, and six electives. The volume you are holding is marked in the margin.

Code	Course	Semester	Units
MFT 701	Fundamentals of Marriage and Family Therapy	One	1.71
MFT 702	Theories of Family Therapy	One	1.71
MFT 703	Human Development Across the Lifespan	One	1.71
MFT 704	Ethical and Legal Issues in Therapy	One	1.71
MFT 705	**Research Methods in Marriage and Family Therapy**	One	1.71
MFT 706	Clinical Practice and Techniques	One	1.71
MFT 707	Assessment and Diagnosis in Family Therapy	One	1.71
MFT 708	Cultural Competence in Therapy	One	1.71
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MFT 721	Research Methodology	Three	1.71
MFT 722	Professional Development and Self-Care	Electives	1.71
MFT 723	Special Populations in Therapy	Electives	1.71
MFT 724	Family Law and Advocacy	Electives	1.71
MFT 725	Integrative Approaches in Therapy	Electives	1.71
MFT 726	Neurobiology of Relationships	Electives	1.71
MFT 727	Play Therapy Techniques	Electives	1.71
MFT 770	Master Thesis	Three	1.71

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Course leader's summary. You will become a consumer who can read a paper in ten minutes and know what it is worth, and a producer who can design a study you can actually finish. The course is deliberately practical: every week you do something small with real or simulated data.

How to use this book

Research methods are learned by doing badly, then better. Each week has a **concept**, a **worked example**, and a **small task** you can complete in an hour. The tasks build towards your thesis proposal in MFT 770, so keep every one of them.

1. Course description

Family therapy research is harder than individual therapy research for a structural reason: **people in a family are not independent observations**. If you treat a couple as two independent people, you violate the central assumption of most statistical tests. This course teaches you to see that problem and to handle it in ways that survive examiners and journal reviewers.

The course covers: framing a good question; quantitative, qualitative and mixed designs; sampling and measurement; basic analysis; appraisal of evidence; ethics; and writing a proposal.

2. Learning outcomes

#	Outcome	Assessed
LO1	Convert a clinical hunch into a focused, researchable question	Assignment 1
LO2	Select and justify a design appropriate to the question	Exam, proposal
LO3	Evaluate the reliability and validity of commonly used family measures	Exam
LO4	Handle non-independence and unit-of-analysis problems	Exam, proposal
LO5	Interpret basic descriptive and inferential statistics and effect sizes	Exam, data exercise
LO6	Appraise quantitative, qualitative and mixed-methods studies	Appraisal assignment
LO7	Write an ethical, feasible research proposal	Proposal

3. Assessment

Component	Weight	Due
Question and design exercise (1,000 words)	15%	Week 4
Systematic appraisal of two studies (1,500 words)	25%	Week 9

Component	Weight	Due
Data analysis exercise (set problems)	20%	Week 12
Research proposal (2,500 words)	40%	Week 15

4. The fourteen-week plan

Week	Topic	Task
1	What research is for; evidence-based practice and its critics	Find one paper you will use in the thesis
2	From hunch to question: PICO/SPIDER; feasibility	Draft three questions
3	Designs overview: quantitative, qualitative, mixed	Map your question to a design
4	Sampling: who is in, who is out, who never comes	Critique a convenience sample
5	Measurement: reliability, validity, responsiveness	Appraise one couple measure
6	The independence problem: unit of analysis in family data	Worked analysis of couple data
7	Quantitative analysis I: descriptive and comparison	Run descriptives in software
8	Quantitative analysis II: association, prediction, effect size	Interpret a regression table
9	Qualitative methods and rigour	Code a transcript extract
10	Mixed methods; participatory approaches	Design a mixed study
11	Systematic review and critical appraisal	Appraise one meta-analysis
12	Data management, security, documentation	Write a data plan
13	Research ethics and with vulnerable families	Complete an ethics application
14	Writing it up; publication and impact	Proposal presentations

Week 1 — What research is for

Three legitimate purposes. Description (what is happening), explanation (why, and under what conditions), and improvement (does this intervention help, for whom). A study that serves none of these is a filing cabinet exercise.

Evidence-based practice is the integration of best available research, clinical expertise, and client values and context. It is not "follow the manual". Two critiques worth knowing:

1. **Efficacy versus effectiveness.** Trials conducted with selected volunteers under ideal conditions may not generalize to your clinic, where families present with debts, migration problems, and court dates.
2. **The evidence hierarchy is question-dependent.** A meta-analysis of trials answers "does it work on average?"; a qualitative study answers "how does it work, and for whom does it fail?". Family therapy needs both.

> **TASK**
> Find one paper relevant to your intended thesis topic. Write its citation and one sentence stating what it establishes and one sentence stating what it leaves open.

Week 2 — From hunch to question

A weak question: "Does family therapy work?"

A workable question: "Among adolescents referred to a Lagos community clinic for school refusal, does a six-session family-based intervention reduce days absent from school at 12 weeks, compared with usual care?"

Frameworks. For quantitative work use **PICO** — Population, Intervention, Comparison, Outcome. For qualitative work use **SPIDER** — Sample, Phenomenon of interest, Design, Evaluation, Research type.

Feasibility screen. Before you fall in love with a design, ask: Do I have access? How many participants can I realistically recruit in six months? Who will approve this? What is the cost? Is the timeline compatible with the semester? Most failed theses are failed at this step, not at analysis.

Common traps.

Trap	Example	Fix
Too broad	"Family functioning in Nigeria"	Specify population, setting, outcome
Unanswerable with your data	"Does family therapy cause lasting change?"	Narrow to what a 12-week design can show
Two questions in one	"Does it work and why?"	One study, one primary question
Question-shaped advocacy	"Why is family therapy better?"	Make it genuinely open ("how does it compare?")

> **TASK**
> Draft three candidate questions. For each, write the PICO or SPIDER elements, the sample you can access, and the main threat to validity.

Week 3 — Designs overview

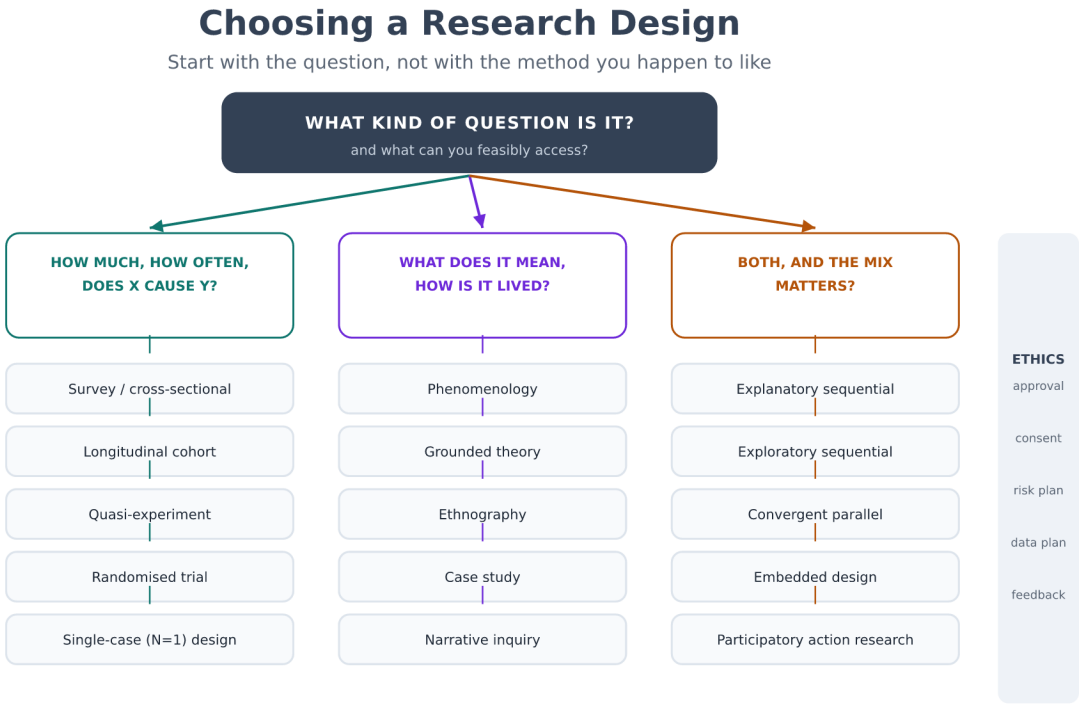


Figure 3.1 — Choosing a research design: start with the question

Quantitative.

Design	Answers	Strengths	Threats
Cross-sectional survey	How common? What is associated?	Cheap, fast	Cannot establish temporality
Longitudinal cohort	What predicts change?	Temporal ordering	Attrition
Quasi-experiment	Does the intervention work without randomisation?	Feasible in clinics	Selection bias
Randomised controlled trial	Does it cause the effect?	Strongest causal inference	Cost, ethics, generalisability
Single-case (N=1) design	Does it work for this family?	Clinically realistic	Limited generalisation

Qualitative.

Approach	Focus	Sample size
Phenomenology	Lived experience of a phenomenon	6–12
Grounded theory	Building a theory from data	15–30, to saturation
Ethnography	Culture and practice in a setting	Prolonged fieldwork
Case study	Depth in one bounded system	1–5 cases

Approach	Focus	Sample size
Narrative inquiry	How people story their lives	3–10
IPA	How individuals make sense of experience	3–6

Mixed methods — see Week 10.

> **TASK**
> Take your strongest question from Week 2 and map it onto two designs. State what each would let you claim, and what it would not.

Week 4 — Sampling

The uncomfortable fact. Clinical samples are almost never representative. Families who attend are different from families who are referred and do not attend, and those are different again from families who are never referred. Your findings describe attenders, not families.

Probability sampling (random, stratified, cluster) permits generalisation. **Non-probability sampling** (convenience, purposive, snowball, quota) does not — and this is fine, if you say so.

In family research specifically:

- **Who is the participant?** Mother, father, child, whole family, or the therapist's rating of the family?
- **Who refuses?** Fathers are the most common absence in family research and in family therapy. A study that reports "family functioning" from mothers' reports is a study of mothers.
- **Sample size.** For quantitative work, do a power calculation before you collect (software: G*Power, or the pwr package in R). For qualitative work, plan for saturation and state how you will judge it.

> **TASK**
> Critique the sample in any published Nigerian family study: who was included, who was excluded, and what does the exclusion do to the conclusions?

Week 5 — Measurement

A measure must be **reliable** (consistent), **valid** (measuring what it claims), and **responsive** (sensitive to change).



Figure 5.1 — Four validity targets and three reliability methods

The four validity targets.

Validity	The question
Internal	Did the intervention, and not something else, produce the change?
External	Can these findings travel to other families, settings and cultures?
Construct	Are you measuring "alliance", or "wanting to please the therapist"?
Statistical	Is the effect large enough, and the test powerful enough, to trust?

Instruments you should know (and check for local validation):

Instrument	Measures	Notes
Dyadic Adjustment Scale (DAS) / DAS-4	Relationship satisfaction	Widely used; short forms available
Revised Dyadic Adjustment Scale (RDAS)	Satisfaction	14 items
FACES-IV	Cohesion and flexibility	Psychometrics debated; use cautiously
Family Assessment Device (FAD)	Family functioning (McMaster)	60 items; subscales
Working Alliance Inventory (WAI-SR)	Therapeutic alliance	Adapt for family work carefully
PHQ-9 / GAD-7	Depression / anxiety	Free, brief, widely validated

Instrument	Measures	Notes
SDQ	Child emotional and behavioural functioning	Parent, teacher, self versions

The cultural validity question. Was this instrument developed and validated with families like the ones you see? If not, report that limitation and consider a local adaptation study (a perfectly good thesis).

> **TASK**
> Take one couple measure. Find two published validations, one in an African sample if it exists. Report: internal consistency, factor structure, and any evidence of measurement invariance.

Week 6 — The independence problem

This is the technical heart of family research.

The problem. In a study of 100 couples, you do not have 100 independent observations; you have 50 couples, and partners' scores are correlated. Standard tests (t-test, ANOVA, OLS regression) assume independence. Violating it inflates Type I error: you will find effects that are not there.

Three solutions.

1. **Analyse at the dyad level.** Compute a couple score (mean, difference, or both) and analyse that. Simple, loses information about within-couple differences.
2. **Treat the dyad as the unit and model non-independence explicitly.** Multilevel models (HLM) with individuals nested in couples nested in therapists. This is the modern standard.
3. **Dyadic models.** The **Actor–Partner Interdependence Model (APIM)** separates actor effects (my predictor on my outcome) from partner effects (my predictor on my partner's outcome). This is often the clinically interesting question.

Worked example. Suppose you measure criticism (predictor) and satisfaction (outcome) in 60 couples. An OLS regression treating all 120 people as independent finds criticism predicts satisfaction, $p = .001$. An APIM re-analysis finds: actor effect significant; partner effect smaller but present; and the standard errors widen. The clinical interpretation changes: criticism damages the person who gives it as well as the person who receives it.

> **TASK**
> Simulate it: in a spreadsheet, create 40 couples with correlated scores. Run a t-test treating all 80 as independent, then compute the couple means and run the test again. Note the difference in p.

Week 7 — Quantitative analysis I

Descriptive statistics. Mean and standard deviation for symmetric data; median and interquartile range for skewed data (income, session counts). Always report n, missingness, and the range.

Comparing groups. t-test (two groups), ANOVA (three or more), chi-square (categorical). Check assumptions: independence, normality (roughly), equal variance.

Clinical significance versus statistical significance. A drop of 2 points on a satisfaction scale can be statistically significant with n = 400 and clinically meaningless. Report **effect sizes** and, where available, the **minimal clinically important difference**.

Common effect sizes.

Statistic	Meaning	Rough benchmark
Cohen's d	Standardised mean difference	0.2 small, 0.5 moderate, 0.8 large
Pearson's r	Association	0.1 small, 0.3 moderate, 0.5 large
Odds ratio	Relative odds	1.5 modest, 2+ substantial
Number needed to treat	How many treated for one extra success	Lower is better

> **TASK**
> Interpret this sentence for a non-researcher in two sentences: "The intervention group showed a greater reduction in distress than controls (d = 0.55, 95% CI [0.21, 0.89], p = .002)."

Week 8 — Quantitative analysis II

Correlation is not causation, and neither is regression. Regression tells you how much of the variance in an outcome is associated with predictors, adjusted for other predictors. It does not tell you the direction, unless the design does.

Reading a regression table. Look at: which predictors are significant; the sign and size of the coefficient (in meaningful units); the confidence intervals; R^2 (variance explained); and whether the model diagnostics were reported.

Moderation and mediation. Moderation: for whom, or under what conditions, does the effect hold? (e.g. does the intervention work better for younger adolescents?) Mediation: through what mechanism? (e.g. does improved communication explain the reduction in distress?). Mediation requires strong assumptions and, ideally, longitudinal data.

> **TASK**
> Take a published regression table from a family study. Write four sentences a clinician could use, including one caveat.

Week 9 — Qualitative methods and rigour

When qualitative methods are the right choice: when you need to know how something is experienced, why a service is not used, what a concept means locally, or how a therapy unfolds. They are not a fallback for people who dislike numbers.

Rigour criteria (Lincoln & Guba; Braun & Clarke for reflexive TA):

Criterion	Practical meaning
Credibility	Do the findings ring true to participants? (member checking, triangulation)
Transferability	Enough context for a reader to judge relevance elsewhere (thick description)
Dependability	An audit trail of decisions
Confirmability	Findings grounded in data, not in the researcher's preferences
Reflexivity	Your own position examined, not hidden

Coding, briefly. Familiarise → code → generate themes → review → define → write. Coding can be inductive (from the data) or deductive (from a framework). Reflexive thematic analysis explicitly treats themes as constructed by the analyst, not excavated from the data.

Sample extracts. Suppose a father says: "They asked me to come, but nobody asked me what I thought was wrong. I sat there like a visitor in my own house." Codes: excluded, not consulted, home as foreign. Candidate theme: "A visitor in my own house": the disengagement of fathers in family therapy. Note how the theme uses the participant's own language.

> **TASK**
> Code the extract above independently, then compare with a peer. Where you disagree, ask whether you are coding the same unit.

Week 10 — Mixed methods and participatory approaches

Why mix. Numbers answer "how much"; words answer "how" and "why". Three designs:

1. **Explanatory sequential** — quantitative first, then qualitative to explain the result.
2. **Exploratory sequential** — qualitative first (to build a culturally valid measure or model), then quantitative.
3. **Convergent parallel** — both at once, integrated in interpretation.

Integration is the hard part. A study that reports a survey and some interviews side by side, without integrating the findings, is two studies in one paper. Use a **joint display** table: finding from strand A

| finding from strand B | convergence / divergence | interpretation.

Participatory action research is particularly well suited to community and family work: families help define the question, collect data, and interpret findings. It strengthens validity and ethics at once.

> **TASK**
> Design a mixed-methods study to understand why families drop out after the first session. Specify the sequence, the integration point, and one joint display row.

Week 11 — Systematic review and critical appraisal

Levels of evidence (roughly, for treatment questions): systematic reviews and meta-analyses > RCTs > quasi-experiments > cohort > case-control > cross-sectional > case series > expert opinion. For questions of meaning, the hierarchy inverts.

Appraising a meta-analysis. Ask: Was the search systematic and pre-registered? Were studies comparable enough to pool? Was heterogeneity (I^2) reported and explained? Was publication bias assessed (funnel plot, Egger's test)? Do the authors' conclusions follow from the data?

A ten-minute paper reading routine.

1. Abstract: design, n, outcome, conclusion.
2. Participants: who, where, how recruited.
3. Comparison: what was the control?
4. Measure: does it measure something you care about?
5. Effect: size and confidence interval, not just p.
6. Limitations: what the authors admit and what they do not.
7. Relevance: your population, your setting, your resources.

> **TASK**
> Appraise one meta-analysis of couple or family therapy. Write 500 words: what it establishes, what it cannot establish, and one clinical implication.

Week 12 — Data management

Principles. Collect the minimum you need; store it encrypted; separate identifiers from data; define retention and destruction; document every decision.

Practical protocol for a master's thesis.

Element	Rule
Identifiers	Separate key file, encrypted, access limited to you and supervisor
Audio/video	Encrypted drive, deletion date agreed with participants

Element	Rule
Transcription	Pseudonymised at the point of transcription, not after
Version control	Date every dataset; never overwrite raw data
Analysis log	One page per week: what you did, what you decided, why
Backup	Two locations, one offline

> **TASK**
> Write a one-page data management plan for your proposed study.

Week 13 — Research ethics

Core requirements. Institutional ethics approval before recruitment; informed consent appropriate to age and capacity; voluntariness (especially for your own clients); protection of confidentiality; a plan for distress and incidental findings; fair selection of participants; and a dissemination plan that returns something to the community.

Specific issues in family research.

- **Consent within families:** if a parent consents, does the adolescent have a genuine choice? Use separate assent and consent processes.
- **Coercion by proxy:** recruiting your own clients creates pressure. Use a third party for recruitment.
- **Group confidentiality:** you cannot guarantee that family members will not repeat what is said.
- **Power and payment:** paying participants can be appropriate compensation or an undue inducement; be transparent about the reasoning.

> **TASK**
> Complete a mock ethics application for your proposal, including consent forms for an adult and for a 14-year-old.

Week 14 — Writing it up

Structure (IMRaD). Introduction (why this question matters; what is known; the gap) → Method (what you did, in enough detail to replicate) → Results (what you found, without interpretation) → Discussion (what it means, limitations, implications) → Conclusion.

Writing rules that examiners reward. One idea per paragraph; active voice where possible; report numbers with units and confidence intervals; state limitations honestly and specifically; do not overclaim in the abstract.

Publication. Target a journal that publishes practitioner research; follow the author guidelines exactly; expect rejection once and revise.

Glossary

Actor–partner interdependence model (APIM) — a dyadic model separating one's own and one's partner's effects. **Attrition** — loss of participants over time. **Confidence interval** — the range within which the true value is likely to fall. **Construct validity** — the degree to which a measure captures the intended concept. **Effect size** — the magnitude of an effect, independent of sample size. **Heterogeneity (I^2)** — the proportion of variation across studies due to real differences rather than chance. **Independence assumption** — the statistical requirement that observations are not correlated. **Measurement invariance** — whether an instrument measures the same construct across groups. **Power** — the probability of detecting an effect that exists. **Saturation** — the point at which new data yield no new themes. **Triangulation** — using multiple methods, sources or analysts to strengthen inference.

Self-assessment

Multiple choice

1. Treating both partners in 50 couples as 100 independent observations primarily threatens: (a) external validity (b) statistical conclusion validity (c) construct validity (d) reliability

2. The APIM distinguishes: (a) pretest and posttest (b) actor and partner effects (c) randomisation and control (d) qualitative and quantitative

3. A qualitative study of lived experience with 8 participants is most likely: (a) grounded theory (b) phenomenology (c) ethnography (d) RCT

4. Cronbach's alpha estimates: (a) test–retest reliability (b) inter-rater reliability (c) internal consistency (d) construct validity

5. The strongest design for causal inference is: (a) cross-sectional survey (b) quasi-experiment (c) randomised controlled trial (d) case series

6. "For whom and under what conditions does the effect hold?" refers to: (a) mediation (b) moderation (c) randomisation (d) sampling

7. Which is not a qualitative rigour criterion? (a) credibility (b) transferability (c) generalisability (d) reflexivity

8. A 95% confidence interval that includes zero suggests: (a) a large effect (b) a non-significant effect at the .05 level (c) a reliable measure (d) a sampling error in the measure

9. The most common absence in family research samples is: (a) mothers (b) fathers (c) adolescents (d) grandparents

10. Reflexivity in qualitative research means: (a) the researcher examines their own position and effect (b) participants review findings (c) the study is replicated (d) data are triangulated

Answer key

1 b · 2 b · 3 b · 4 c · 5 c · 6 b · 7 c · 8 b · 9 b · 10 a

Short answer

1. Convert "does family therapy help adolescents?" into a PICO question.
2. Explain the unit-of-analysis problem in one paragraph a clinician would understand.
3. Name three threats to internal validity in a pre-post clinic study.
4. What is measurement invariance and why does it matter in cross-cultural family research?
5. List five elements of a data management plan.

Essay (choose one, 2,000 words)

1. "The randomised controlled trial is the wrong gold standard for family therapy." Discuss.
2. Design a mixed-methods study of therapy drop-out in a Nigerian family clinic, justifying every methodological choice.
3. Critically appraise the evidence that common factors explain most psychotherapy outcome.

Further reading

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Next: MFT 706 — Clinical Practice and Techniques

MSC MARRIAGE AND FAMILY THERAPY

LECTURE LIBRARY · VOLUME 6 OF 28

MFT 706

Clinical Practice and Techniques

16 teaching weeks · 10 practicals · 4 worked cases · 1 figure

SEMESTER ONE

1.71 units · Prerequisite: MFT 701, 702

> “This is the skills course.”

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MSc Marriage and Family Therapy · Semester One · 1.71 Units · 16 teaching weeks · Prerequisite: MFT 701, 702

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Course leader's summary. This is the skills course. You will leave able to structure a session, ask six kinds of question deliberately, conduct an enactment, reframe without dismissing, assign a task that gets done, and end therapy well. Most of the hours are practice, not lecture.

How to use this book

Technique is a set of **deliberate choices**, not a personality. Each week introduces one family of skills, gives you the language, shows a transcript, and then requires you to perform it. Read the transcript aloud before the lab: your mouth needs to know the sentences before your mind can choose them under pressure.

1. Course description

The gap between knowing about family therapy and doing it is bridged by rehearsal. This course covers the micro-skills (joining, questioning, reflecting, summarising), the meso-skills (structuring a session, managing conflict, conducting an enactment), and the macro-skills (planning treatment, assigning tasks, handling impasse, terminating).

It is assessed almost entirely on **performance**, because a student who can describe an enactment and cannot conduct one is not yet a clinician.

2. Learning outcomes

#	Outcome	Assessed
LO1	Structure a session with a clear opening, middle and close	Observed session
LO2	Use six categories of question deliberately and appropriately	Observed session, exam
LO3	Conduct an enactment and manage its escalation	Observed session
LO4	Reframe, normalise and positively connote without dismissing distress	Skills test
LO5	Design and review between-session tasks	Task portfolio
LO6	Manage conflict, silence and resistance in the room	Observed session
LO7	Plan and conduct a planned ending	Viva, log

3. Assessment

Component	Weight	Due
Skills triad log (weekly)	20%	Fortnightly
Observed session 1 (20 min, mid-semester)	20%	Week 8
Task portfolio (5 designed tasks with rationale)	20%	Week 12

Component	Weight	Due
Observed session 2 (40 min, with feedback)	40%	Week 16

4. The sixteen-week plan

Week	Skill family	Practical
1	The architecture of a session	Structure a 50-minute session
2	Joining and the first five minutes	Opening rounds, three styles
3	Listening: paraphrase, reflect, summarise	Reflection-only interview
4	Questioning I: linear, circular, strategic	Circular questioning drill
5	Questioning II: reflexive, future-oriented, scaling	Solution-focused drill
6	Enactment: making the family talk to each other	Enactment lab
7	Reframing, normalising, positive connotation	Reframe tennis
8	**Observed session 1**	Assessment and feedback
9	Working with conflict and escalation	De-escalation drill
10	Silence, tears, and the therapist's discomfort	Tolerance-of-silence lab
11	Structuring: who sits where, who speaks when, subgroups	Subgroup session design
12	Between-session tasks and homework	Task portfolio workshop
13	Impasses, non-attendance, and drop-out	Difficult-session rehearsal
14	Working with children and adolescents in family sessions	Live supervision
15	Endings: planned, unplanned, and therapeutic	Termination role-play
16	**Observed session 2**	Full assessment

Week 1 — The architecture of a session

A 50-minute family session has five movements. Beginners skip the first and the last, which is why their sessions feel formless.

Movement	Minutes	Purpose
1. Opening / re-joining	3–5	Reconnect, state the frame, ask what has changed
2. Agenda	2–3	Whose concern is most urgent this week?
3. Work	25–30	The core intervention: enactment, questioning, restructuring
4. Consolidation	5–7	Summarise, check, agree the task
5. Close	3–5	Confirm next appointment, name one thing to watch

The "what has changed?" opener. Never start where you left off. Start with the family's own observation: "Before we begin — what has been different this week, even slightly?" This trains the family to notice change and gives you data about what is already working.

> **PRACTICAL**
> Write the five movements on a card. Use it in every triad this semester. By Week 8 you should not need it.

Week 2 — Joining

Three joining styles (choose deliberately):

1. **Tracking** — follow the family's language and pace. "Tell me about 'stubborn' — what does it look like in your house?"
2. **Accommodation** — adopt the family's structure for the moment. With a hierarchical family, address the father first; with a chaotic family, bring structure immediately.
3. **Personal joining** — offer a small, true piece of yourself. "I grew up in a house where nobody raised their voice either. It took me years to learn to argue."

What joining is not. It is not agreement, flattery, or becoming the family's friend. You can join and disagree: "I like the way you two can laugh at this. I am going to say something you may not like."

Three failures of joining.

Failure	Symptom	Repair
Over-joining	You are now part of the family's pattern	Name it: "I think I just took your side. Let me step back."
Under-joining	Family answers in monosyllables	Slow down; ask about something concrete and safe
Premature confrontation	Family defensive from minute five	Return to their account for ten minutes before challenging

> **PRACTICAL**
> Three rounds of five-minute openings, each with a different joining style. The "family" rates: did I feel understood? Did I feel patronised? Did I want to continue?

Week 3 — Listening

Paraphrase — return the content. **Reflect** — return the feeling. **Summarise** — return the pattern across people and time.

Client: "He comes home, eats, and goes to the room. I sit in the parlour with the children and I feel like a widow."

- Paraphrase: "So most evenings he is in the room and you are with the children."
- Reflect: "You feel alone in your own marriage."
- Summarise (later, across people): "So there are two versions of the evening: yours is that he leaves; his is that he arrives already exhausted and afraid of saying the wrong thing. Both of you are alone in the same house."

The 2:1 rule for beginners. Two reflections for every question. It slows the interview, produces richer material, and prevents the interrogation pattern families experience in first sessions.

> **PRACTICAL**
> Ten-minute interview in which you may not ask a single question. Only paraphrase, reflect and summarise. Note how uncomfortable it is, and what the client does when you stop asking.

Week 4 — Questioning I

Six categories, each with a purpose. Learn the label so you can choose under pressure.

Type	Purpose	Example
Linear	Gather facts	"When did the arguing start?"
Circular	Reveal difference and relationship	"Who notices first when your mother goes quiet?"
Strategic	Introduce a direction	"What would happen if you stopped reminding him?"
Reflexive	Invite a new perspective	"If your daughter could hear this conversation, what would she say you both believe?"
Future-oriented	Open possibility	"Suppose this is better in six months — what would be different?"
Scaling	Make change measurable	"On 0–10, how close are you as a couple? What would one point look like?"

The circular question is the field's signature skill. Build it from three elements: a difference (who/what/how much), a relationship (between whom), and a comparison (more than, less than, compared with).

- Weak: "Is your husband supportive?"
- Better: "Who in the family would say your husband is most supportive — and who would disagree?"
- Best: "If your mother were watching the two of you last night, what would she say was different between how he spoke to you and how he speaks to your sister?"

> **PRACTICAL**
> Circular questioning drill: 10 minutes, all questions must be circular. Observer counts and classifies. Target: 80% genuinely circular.

Week 5 — Questioning II

Reflexive questions invite the family to observe itself. They are the safest way to raise something difficult, because the family supplies the content:

- "What do you think your son has learned about marriage from watching the two of you?"
- "If I asked your daughter who is in charge in this house, what would she say?"
- "What is the most useful thing that could happen in this session — and the most uncomfortable?"

Future questions create hope without denying the present:

- "Suppose we meet in a year and this is behind you. What would you want to be able to say you did?"
- "What would your father have wanted for this family?"

The question to avoid: "Why?" in relation to behaviour. "Why did you shout at her?" invites defence or invention. "What happened just before you shouted?" invites sequence.



Figure 5.1 — Solution-focused moves: strengths, miracle, scaling, exceptions, task

PRACTICAL

Convert ten "why" questions into "what/how/who" questions. Compare the answers you get in role-play.

Week 6 — Enactment

Definition. Enactment is the deliberate direction of family members to speak **to each other** in the session, while the therapist moves to the edge of the action.

Why it matters. Most families come to therapy to talk about each other to a third party. Enactment converts description into live interaction, where you can observe and shape it. It is the single most powerful structural technique.

The four-step protocol.

1. **Block the detour:** "Don't tell me — tell her."
2. **Specify the content:** "Tell her what you needed on Saturday, in one sentence."
3. **Direct the process:** "Look at him, not at me." "Say it again, slower."
4. **Intensify or interrupt:** "You just smiled. Say it again with the smile." Or: "Stop. What just happened?"

Transcript (structural enactment with a couple).

THERAPIST:

You have both told me about Saturday. Now I want you to tell each other. Funmi, tell Femi what you needed on Saturday — one sentence, and look at him. **Funmi:** (to therapist) I needed him to — **Therapist:** To him. **Funmi:** I needed you to call and say you would be late. **Femi:** I did call — **Therapist:** Femi, answer her, not me. **Femi:** (pause) I did call. At four. You didn't pick up. **Funmi:** I was with your mother. **Therapist:** Stop there. Funmi — what happened inside you when he said that?

Hazards. Enactment can escalate. Have a stop signal, keep your voice calm and slow, and interrupt before contempt. With coercive control, do not enact conflict; safety first (MFT 704).

PRACTICAL

Enactment lab. Triads with a scripted couple. The therapist may only use the four steps. Observer records: did the therapist stay out of the content? Did the family speak to each other for more than 90 seconds?

Week 7 — Reframing, normalising, positive connotation

Reframing changes the frame, not the facts. It is the most-used intervention in the field and the most easily abused.

Original frame	Reframe	Risk
"He is controlling"	"He is frightened of losing you and tries to hold on"	Can excuse control
"She is manipulative"	"She has learned that tears are the only way to be heard"	Can remove accountability
"You are lazy"	"You are exhausted and have stopped asking for help"	Can be heard as excusing
"Our son is defiant"	"Your son is trying to protect his mother"	Can burden the child further

Three tests before you reframe.

1. **Is it true enough?** A reframe that the family rejects as false damages alliance.
2. **Does it open or close action?** "He is frightened" opens action; "he is a monster" closes it.
3. **Does it remove accountability?** Reframes should increase the family's sense of agency, not reduce it.

Normalising — placing behaviour in a developmental or statistical context: "Arguing about money is the most common argument in the first five years of marriage. It is not a sign that you are failing."

Positive connotation (Milan) — describing all behaviour as serving the family system: "Your arguments may be the family's way of ensuring that nothing important is decided without both of you being fully present." Use sparingly; it can sound like sarcasm.

> **PRACTICAL**
> "Reframe tennis": one student states a complaint, the partner must return three successive reframes, and the first student says which one she would accept and why.

Week 8 — Observed session 1

Twenty minutes with a role-played family. Assessed against five criteria: structure (does the session have a shape?), joining (does each person feel heard?), questioning (at least three circular), one enactment, and a summary with a task.

Feedback is given live by the observing group using the structured sheet, then by the lecturer.

Week 9 — Working with conflict and escalation

The therapist's job during conflict is regulation, not resolution. Five moves:

1. **Slow the pace.** "Stop. Say that again, more slowly."
2. **Name the process.** "I notice that every time she raises her voice, you leave the room. Can we stay with it for two minutes?"

3. **Structural interruption.** "I am going to ask you both to be quiet for one minute and let me speak to your son."
4. **Separate content from process.** "The topic is money. The pattern is that you both start explaining before the other finishes."
5. **Time-out with a return.** "Let's take two minutes. We are not finished."

Escalation signs to interrupt early: raised volume, contempt (eye-rolling, mockery), one person standing, threats, children becoming still or leaving, your own heart rate rising.

> **DE-ESCALATION DRILL**
> Pairs escalate a scripted argument on cue. The therapist must interrupt before the third escalation. Repeat until the interruption happens at the right moment, not the safe one.

Week 10 — Silence, tears, and the therapist's discomfort

Silence is a technique. There are four kinds:

Silence	Meaning	Response
Processing	Someone is thinking	Wait. Count to fifteen.
Emotional	Feeling has arrived	Stay; soften your face; do not rescue
Resistance	Something is not being said	Name it gently: "Something just shifted. What happened?"
Your own	You are lost	Say so: "Let me think for a moment about what I am hearing."

Tears. Do not hand a tissue immediately; it tells the person to stop. Wait three seconds, then offer. Ask: "Is this a sadness that has been waiting a long time?"

Your discomfort is data. If you want to fill the silence, ask what you are avoiding. If you feel irritated with one family member, ask what they represent to you. This is the beginning of using the self as an instrument (developed in MFT 720 and 722).

Week 11 — Structuring the room and the system

Physical structure. Seat families so that coalition patterns are visible and can be interrupted. Put the parents together and the children together; sit between people who never speak; never sit in a line that recreates the hierarchy you are trying to change.

Who attends. Deliberately choose the combination: whole family, parents only, siblings, one parent and one child, individual. State the rationale: "Next week I would like to see the two of you alone, so you can talk as a couple without the children carrying it."

Subgroup sessions. A powerful and under-used technique: see the parents alone to plan a united message, then bring the children in and have the parents deliver it together. The intervention is the difference between the two sessions.

> **PRACTICAL**
> Design a three-session sequence using different attendance combinations for the Adeyemi family (MFT 701). Justify each combination.

Week 12 — Between-session tasks

A task is a test of the system, not homework. It should be small, specific, behavioural, and designed to produce information.

The four-part task formula.

1. **What exactly** ("For the next seven days, at dinner…")
2. **Who does what** ("…each of you names one thing that went well in your day…")
3. **What to notice** ("…and notice what happens to the atmosphere by Thursday.")
4. **What to bring back** ("…we will look at what you noticed next week.")

Five useful task types.

Type	Purpose	Example
Observation	Gather data	"Notice and write down every time the argument stops before it starts."
Ritual	Interrupt a sequence	"One night this week, the parents go out for one hour. The children are told in advance."
Behavioural exchange	Build positives	"Three small caring acts each per day, unannounced."
Communication	Practise a skill	"Two 15-minute speaker–listener conversations, timer on."
Paradoxical (rare, supervised)	Change the symptom's function	"On Tuesday, have the argument on purpose for ten minutes at 8 p.m."

Why tasks fail. Too big, too vague, agreed under pressure in the last two minutes of the session, or the family does not believe the therapist believes they will do it. Write the task down and give them the paper.

> **TASK PORTFOLIO (20%)**
> Five tasks, each with: the presenting problem, the task, the rationale, the predicted failure mode, and the review question you would ask.

Week 13 — Impasses, non-attendance and drop-out

When a session goes flat. Three likely causes: the family is protecting a secret; the therapist has joined too closely to one side; or the work has reached material the family did not consent to. Name the process: "I notice we have been very polite for twenty minutes. What are we all avoiding?"

When someone does not attend. Never pretend it did not happen. "Your father chose not to come today. Let's think about what that means." Explore whether attendance is a form of communication.

When the family drops out. Follow up once, by phone, briefly and without pressure; record the reason if given; and use it as clinical data. In family therapy, drop-out is the most common negative outcome, and it is usually visible early — the first two sessions predict the rest.

Impasses with the therapist. If you are bored, angry, or over-involved, that is information about the system. Take it to supervision before you act on it.

Week 14 — Working with children and adolescents in family sessions

Practical adjustments.

- Shorter sessions for younger children (30–40 minutes), or split sessions.
- Explain the frame to the child directly, not only to the parents: "In this room we talk, and sometimes we draw."
- Give the child a legitimate role: "You are the expert on what happens at school."
- Protect the child from being made the messenger: "That is a question for your mother to answer, not you."
- Never ask a child to choose sides in front of the parents.

The adolescent rule. Speak to the adolescent as the primary person for at least five minutes. Ask their version first. Parents' accounts of adolescents are systematically more negative than adolescents' own reports and than teachers' reports.

> **LIVE SUPERVISION**
> One student conducts a family session with a trained role-play family while the group observes; the supervisor may call in with one suggestion. Debrief with the family's experience.

Week 15 — Endings

Planned ending. Agree it in advance; review what changed and what did not; name the risk of relapse; write a one-page relapse plan with the family; leave the door open.

The four questions of a good ending.

1. What is different now, in your own words?
2. What did each of you do that made the difference?
3. What is most likely to make it come back?
4. What will you do when you notice the first sign?

Unplanned endings. Client-initiated endings are common; acknowledge and, where possible, send one closing message. Therapist-initiated endings (illness, placement change, moving) require a proper handover session — an abrupt ending can replicate an abandonment for the family.

Do not adopt. A warm ending can slip into a friendship. Say what the relationship was: "I have enjoyed working with you. This has been a professional relationship, and it ends here — that is what makes it safe."

> **PRACTICAL**
> Termination role-play: a family that has improved and is anxious about stopping. The therapist must review, name the risk, write the plan and close, in 15 minutes.

Week 16 — Observed session 2

Forty minutes, full assessment, using the criteria below and a self-assessment submitted in advance.

Criterion	Weight
Structure and pacing	20%
Joining and alliance with each member	20%
Questioning (range and timing)	20%
One deliberate intervention well executed	25%
Summary, task and closing	15%

Glossary

Accommodation — adapting one's style to a family's organisation in order to join it. **Circular question** — a question about difference or relationship that reveals pattern. **Enactment** — directing family members to interact with each other in session. **Intensity** — raising the emotional or interactional stakes to produce change. **Positive connotation** — framing all behaviour as serving the family system. **Reflexive question** — a question inviting the family to observe itself. **Reframe** — relabelling behaviour within a new, usually relational, frame. **Subgroup session** — seeing part of the family deliberately, then using the difference. **Unbalancing** — deliberately supporting one member to shift the system's equilibrium.

Self-assessment

Multiple choice

1. The correct response to "she is manipulative" as a first formulation is: (a) agree (b) challenge the parent (c) gather specific behavioural sequences (d) diagnose

2. "Who notices first when your mother goes quiet?" is: (a) linear (b) circular (c) strategic (d) closed

3. The main purpose of enactment is to: (a) gather history (b) observe and shape live interaction (c) calm the family (d) demonstrate expertise

4. A reframe should: (a) excuse the behaviour (b) open new action without removing accountability (c) always be positive (d) replace diagnosis

5. Two reflections for every question is recommended because it: (a) fills time (b) reduces interrogation and enriches material (c) prevents emotion (d) shortens sessions

6. A task is most likely to fail when it is: (a) written down (b) small and specific (c) agreed in the last two minutes (d) behavioural

7. Therapeutic silence after an emotional statement should usually be: (a) filled quickly (b) held for 10–15 seconds (c) ended by offering a tissue (d) followed by a question

8. In a family with coercive control, enactment of conflict is: (a) recommended (b) contraindicated (c) neutral (d) required

9. Asking "why did you shout?" is discouraged because it invites: (a) facts (b) defence or invention (c) circular information (d) emotion

10. A good ending includes: (a) a new diagnosis (b) a relapse plan (c) a friendship (d) a referral to another therapist

Answer key

1 c · 2 b · 3 b · 4 b · 5 b · 6 c · 7 b · 8 b · 9 b · 10 b

Short answer

1. Write one example of each: linear, circular, reflexive and scaling question, on the same topic.
2. Give the four steps of enactment with the exact words you would use for step 1.
3. What are the three tests of a good reframe?
4. Name four reasons between-session tasks fail.
5. How would you structure a session for a family with a 7-year-old and a 15-year-old?

Essay (choose one, 1,800 words)

1. "Technique is the servant of the alliance." Discuss with reference to your own observed sessions.
2. Analyse a transcript of your own session and identify three moments where a different choice was available. Justify each alternative with theory.
3. How should a family therapist adapt technique for a family in which the father is structurally absent?

Further reading

- Minuchin, S., & Fishman, H. C. (1981). Family Therapy Techniques. Harvard University Press.
- Tomm, K. (1988). Interventive interviewing: Part III. Family Process, 27(1), 1–15.
- Hepworth, M., & Larsen, J. (1993). Direct Social Work Practice. Brooks/Cole. (Micro-skills chapters)
- Butler, C., & Pilling, S. (2020). Competence Framework for Systemic Family and Couple Therapy. UCL.
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Next: MFT 707 — Assessment and Diagnosis in Family Therapy

MSC MARRIAGE AND FAMILY THERAPY

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MFT 707

Assessment and Diagnosis in Family Therapy

15 teaching weeks · 6 practicals · 3 figures

SEMESTER ONE

1.71 units · Prerequisite: MFT 701, 703

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MSc Marriage and Family Therapy · Semester One · 1.71 Units · 15 teaching weeks · Prerequisite: MFT 701, 703

This volume is one of 28 course books that together cover the whole MSc Marriage and Family Therapy taught programme. Each book is written to carry a 14–16 week semester on its own: every week pairs a core idea with the practice that follows from it, a worked case, and a reflective task, and each book closes with an integration chapter, a glossary and a self-assessment paper with model answers.

The books are designed to be read straight through before the semester begins, revisited week by week, and used again in revision. Cross-references point to the other volumes where a topic is treated in more depth.

How to cite this volume

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A note on the case material

All families, couples and individuals in this book are fictional composites. They are written to teach pattern recognition, not to describe any real person. Nothing here replaces supervised clinical practice, professional registration, legal advice or medical care; legal content is orientation only and must be checked against the law of the relevant jurisdiction.

The lecture library

Twenty-eight self-contained course books carry the taught programme: eight in Semester One, seven in Semester Two, seven in Semester Three including the thesis, and six electives. The volume you are holding is marked in the margin.

Code	Course	Semester	Units
MFT 701	Fundamentals of Marriage and Family Therapy	One	1.71
MFT 702	Theories of Family Therapy	One	1.71
MFT 703	Human Development Across the Lifespan	One	1.71
MFT 704	Ethical and Legal Issues in Therapy	One	1.71
MFT 705	Research Methods in Marriage and Family Therapy	One	1.71
MFT 706	Clinical Practice and Techniques	One	1.71
MFT 707	**Assessment and Diagnosis in Family Therapy**	One	1.71
MFT 708	Cultural Competence in Therapy	One	1.71
MFT 709	Group Therapy Techniques	Two	1.71
MFT 710	Couples Therapy	Two	1.71
MFT 711	Child and Adolescent Therapy	Two	1.71
MFT 712	Crisis Intervention Strategies	Two	1.71
MFT 713	Substance Abuse and Family Dynamics	Two	1.71
MFT 714	Trauma-Informed Care	Two	1.71
MFT 715	Family Systems Theory	Two	1.71
MFT 716	Communication Skills in Therapy	Three	1.71
MFT 717	Psychopathology in Families	Three	1.71
MFT 718	Intimacy and Relationship Dynamics	Three	1.71
MFT 719	Family Life Cycle and Transitions	Three	1.71
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Course leader's summary. You will learn to gather multi-informant data, use standardised measures properly, formulate relationally, use diagnosis without being used by it, and communicate all of it to a family in words they can use. The course culminates in a full assessment report of publishable quality.

How to use this book

Assessment is the phase of therapy where the most harm and the most help can be done. Work through the weeks in order: the sequence matters, because each week's skill depends on the previous one. The final assignment is a complete assessment report — the same document type you will write for courts, schools and psychiatrists for the rest of your career.

1. Course description

Family assessment answers four questions: What is happening? How does it work? What maintains it? What would help? Diagnosis answers a fifth: Does this presentation meet criteria for a recognised disorder? Both matter, and they are not the same activity.

The course teaches the **multi-informant, multi-method** standard: self-report, collateral report, observation of interaction, standardised measures, records, and the family's own account — integrated in a formulation that is shared, tested and revised.

2. Learning outcomes

#	Outcome	Assessed
LO1	Plan and conduct a multi-informant family assessment	Observed assessment
LO2	Administer, score and interpret at least four standardised measures	Report
LO3	Use DSM-5/ICD-11 criteria appropriately, including their limits	Exam
LO4	Produce a 5-P formulation with relational hypotheses and a risk screen	Report
LO5	Identify medical, substance-related and cultural rule-outs	Exam
LO6	Give assessment feedback to a family in accessible language	Skills assessment
LO7	Write a report that is accurate, defensible and free of jargon	Report

3. Assessment

Component	Weight	Due
Measure administration and scoring (portfolio)	20%	Week 7
Observed assessment interview (30 min)	25%	Week 11

Component	Weight	Due
Full assessment report (3,000 words + appendices)	40%	Week 15
Examination	15%	Week 16

4. The fifteen-week plan

Week	Topic	Practical
1	Purposes of assessment: clinical, legal, institutional	Map the purposes in a case
2	The multi-informant principle; who to see and in what order	Design an assessment plan
3	History taking: development, family, medical, psychiatric, substance	Structured history interview
4	Interaction observation: what to look for and how to record	Live observation coding
5	Standardised measures I: relationship and family functioning	Score and interpret
6	Standardised measures II: symptoms, risk, children	Score and interpret
7	**Measure portfolio due**; psychometrics in practice	Peer review of portfolios
8	Diagnosis I: DSM-5 and ICD-11 — structure, use, limits	Criteria exercise
9	Diagnosis II: differential diagnosis and comorbidity	Case differentials
10	Medical, neurological and substance rule-outs	Referral-writing lab
11	**Observed assessment interview**	Assessment and feedback
12	Relational and systemic diagnosis: beyond the individual	Formulation workshop
13	Cultural formulation and bias in assessment	Cultural formulation exercise
14	Feedback, report-writing and shared decision-making	Report clinic
15	Integration: assessment that changes treatment	Case presentations

Week 1 — Purposes of assessment

Assessment is never neutral. Ask **who wants it and what they will do with it**.

Purpose	Who commissions	What they need
Clinical	The family	Understanding, direction, relief
Institutional	School, employer, agency	A defensible opinion and a recommendation
Legal	Court, solicitor, child protection	Facts, opinion within expertise, documentation
Research	Investigator	Standardised, comparable data
Self-understanding	The family	Narrative coherence

The trap of multiple purposes. A family may come for help and be sent by a court. The therapist must be explicit about which hat they are wearing, because the duties differ — a clinical assessment prioritises the client's interests; a forensic assessment prioritises accuracy for the court and is not confidential in the same way.

> **PRACTICAL**
> Take any referred case. List every party with an interest in the assessment and what each wants. Identify where the interests conflict.

Week 2 — The multi-informant principle

Rule. Never rely on one informant for a relational question. Mothers, fathers, children and teachers systematically disagree, and the disagreement is clinically informative.

A workable order of contact.

1. **Referrer** (5–10 minutes by phone): what is the concern, what has been tried, is there risk?
2. **Whole family** (60–90 minutes): joining, each person's view, interaction observation, contract.
3. **Subgroups** as indicated: parents alone, each child, each partner separately (always screen IPV separately).
4. **Collateral** with consent: school, GP, previous therapist.
5. **Measures** completed between sessions.
6. **Feedback session**: share the formulation, invite correction.

Who is missing? Fathers, paternal relatives, non-resident children, step-parents, grandparents who co-parent, and the person the family has decided not to discuss. Ask: "Who else is part of this family who is not here? Who would disagree with what has been said today?"

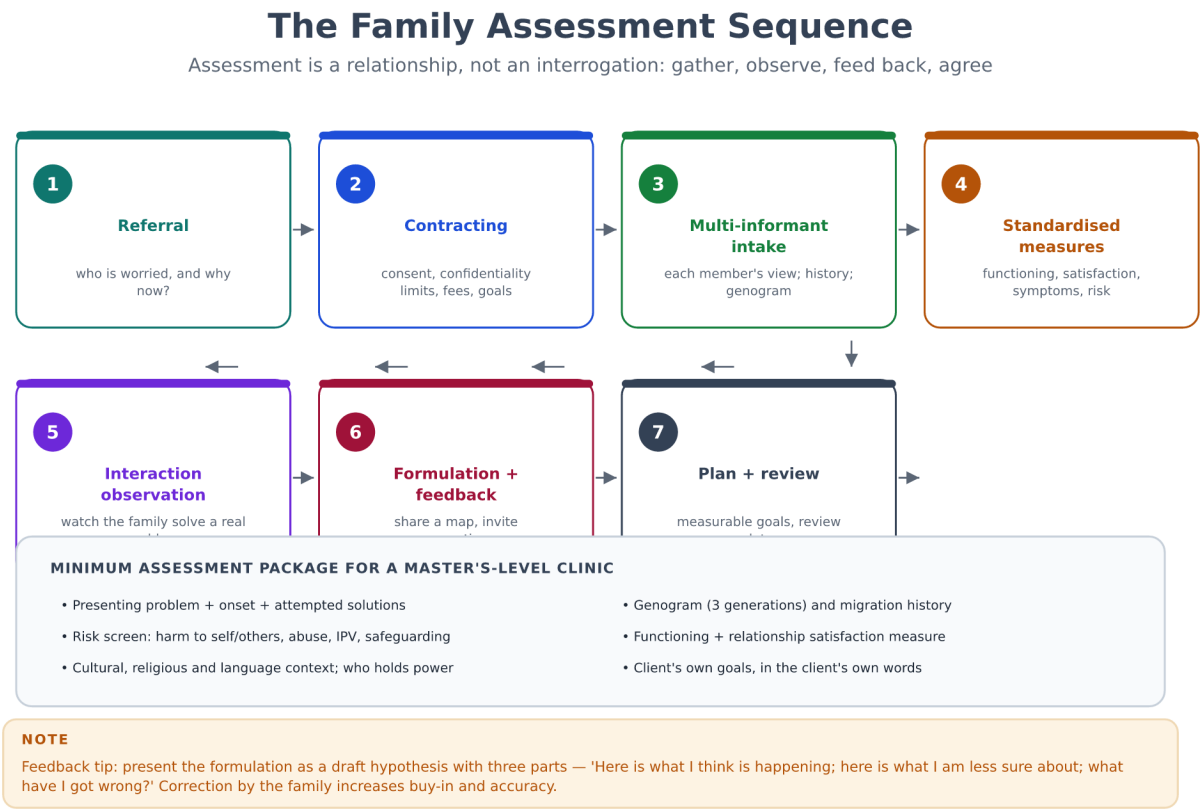


Figure 2.1 — The assessment sequence from referral to review

> **PRACTICAL**
> For a case you know, design the assessment plan: who, in what order, how long, and why. Defend the order.

Week 3 — History taking

A structured history prevents the two classic errors: never asking, and asking everything.

Domains (45 minutes is enough if you are organised).

Domain	Key questions
Presenting problem	Onset, course, severity, triggers, attempted solutions, previous treatment
Developmental	Pregnancy, birth, milestones, illnesses, separations, schooling
Family of origin	Genogram (3 generations), roles, losses, secrets, migration, violence, mental illness
Current family	Household composition, rules, routines, roles, money, division of labour, sexuality
Medical	Chronic illness, medication, sleep, head injury, neurological symptoms
Psychiatric	Previous diagnoses, hospitalisations, self-harm, suicide attempts, treatment response
Substance use	Alcohol, tobacco, cannabis, prescription, other; quantity, frequency, consequences

Domain	Key questions
Social	Work, school, faith community, friends, housing, legal issues, finances
Risk	Harm to self/others, abuse, neglect, IPV, safeguarding
Strengths	What has kept this family going? Who helps? What works?

The strengths column is not decoration. Families referred for assessment have usually been described in deficit terms for years. A careful strengths inventory changes the emotional tone of everything that follows and predicts engagement.

Week 4 — Interaction observation

You do not have to rely on what families say about themselves; you can watch them.

Four standard tasks.

1. **The problem discussion (10 minutes).** "Talk to each other about the problem for ten minutes. I will not interrupt." Observe: who speaks, who is interrupted, who looks at whom, who is silent, how it ends.
2. **The planning task (5 minutes).** "Agree on one change you will make this week." Observe: negotiation, hierarchy, coalition, flexibility.
3. **The positive reminiscence (5 minutes).** "Tell each other about a good day you had together." Observe: affection, repair capacity, humour.
4. **The child-focused task.** "Decide together how to respond to the next tantrum." Observe: who decides, whether the child is consulted, whether the parents support each other.

What to record (behavioural, not judgemental).

Record	Not
"Father interrupted mother 7 times in 10 minutes; she stopped speaking after minute 6."	"Father was dominant."
"Daughter looked at mother before answering each question."	"Daughter is enmeshed."
"Parents agreed on the plan in 90 seconds; the child's objection was not addressed."	"Weak parenting."

The five-minute structural map. Draw the seating, the direction of speech, and the coalitions you observed. It will be more useful than a page of adjectives.

Week 5 — Standardised measures I: relationship and family functioning

Why measure. To obtain a baseline, compare informants, track change, communicate with other professionals, and protect against your own impression management.

Core measures.

Measure	What it measures	Items	Notes
DAS-4 / RDAS	Couple satisfaction	4 / 14	Brief, widely used; cut-offs published
FACES-IV	Cohesion and flexibility	42	Psychometrics debated; use with caution
FAD (McMaster)	Family functioning	60	Six subscales; strong evidence base
Family APGAR	Perceived family support	5	Very brief; useful for screening
WAI-SR	Therapeutic alliance	12	Adapt for family members individually

Scoring discipline. Use the manual's scoring, not a photocopied table. Report raw and standardised scores, the norm group, and the cut-off used. State what a score does not mean: a high DAS score does not mean the couple is safe.

> **PRACTICAL**
> Score an RDAS from a completed (simulated) form: compute the total, compare with the published cut-off, and write three sentences you could say to the couple.

Week 6 — Standardised measures II: symptoms, risk and children

Symptom measures. PHQ-9 (depression; note item 9 is a suicide item and must be followed up), GAD-7 (anxiety), AUDIT (alcohol), DUDIT (drugs), PC-PTSD-5 or PCL-5 (trauma), YSQ or DERS (emotion regulation) as indicated.

Child measures. SDQ (emotional and behavioural, 4–17, parent/teacher/self), RCADS (anxiety and depression), CBCL (broadband; longer), Conners (attention). Always obtain multiple informants and report them separately.

Risk measures. Columbia Suicide Severity Rating Scale (C-SSRS) or a structured clinical interview; do not substitute a questionnaire for a conversation.

Interpretation rules.

1. Measure the same construct with the same instrument at follow-up.
2. Report informant discrepancies rather than averaging them away.
3. Treat screening positives as questions, not diagnoses.
4. Consider literacy, language and cultural fit before interpreting a score.

> **PRACTICAL**
> Administer the SDQ to a volunteer parent and the self-report to a peer playing an adolescent. Compute both. Compare. Write what the discrepancy might mean.

Week 7 — Psychometrics in practice



Figure 7.1 — Validity and reliability: four targets, three methods

Four questions to ask of any instrument.

1. **What is the norm group?** If it was normed on US undergraduates, a Nigerian clinical sample's score needs careful interpretation.
2. **What is the internal consistency?** α ≥ .70 is the usual minimum for research; ≥ .90 is desirable for individual clinical decisions.
3. **Has it been validated locally or in a similar population?** Search before you use.
4. **Is it responsive to change?** A measure that cannot detect improvement is useless for outcome monitoring.

Measurement invariance. The technical question "does this instrument measure the same thing across groups?" is clinically urgent. If a scale behaves differently for men and women, or across languages, then a difference in scores may reflect the instrument, not the people.

Week 8 — Diagnosis I: structure, use and limits

What DSM-5 and ICD-11 are. Classification systems that group patterns of signs and symptoms into categories, using criteria, thresholds and durations. They are reliable vocabularies with imperfect validity: they describe clusters, not known disease entities.

The five DSM-5 axes are gone (they were removed in DSM-5, replaced by a single axial diagnosis plus separate notation of psychosocial stressors, disability and contextual factors). Know the current structure even if you trained on the old one.

Using diagnosis well.

1. Diagnose to communicate, not to explain. "Generalised anxiety disorder" tells a colleague what to expect; it does not tell you why this woman, in this family, at this time.
2. Diagnose only what you can defend with criteria.
3. Record uncertainty explicitly: "provisional", "rule out", "not otherwise specified".
4. Consider the function of a diagnosis in the family: it can unlock care, or become an identity, or a weapon in a custody dispute.

Using diagnosis badly.

- **Premature closure:** the first label constrains all subsequent observation.
- **Diagnostic overshadowing:** the label hides a physical illness, a trauma, or a relational problem.
- **Category fallacy:** assuming that because a category exists, a distinct disease exists.
- **Cultural misreading:** interpreting culturally normative behaviour (spiritual experience, deference, grief expression) as pathology.

> **PRACTICAL**
> Take one case and write two formulations: one purely diagnostic, one purely systemic. Then write the paragraph that integrates them.

Week 9 — Differential diagnosis and comorbidity

The differential habit. For any presenting complaint, generate at least three alternatives before committing. Example: an adolescent's withdrawal.

Possibility	What would support it	What would rule it out
Major depression	Pervasive anhedonia, sleep/appetite change, 2+ weeks	No change in function; mood reactive
Social anxiety	Fear of scrutiny; avoidance specific to social settings	Comfortable with close friends
Trauma response	Intrusion, avoidance, hyperarousal after an event	No identifiable event
Substance use	Changed peer group, money, paraphernalia	Denial plus negative screen
Family crisis	Onset coincides with parental separation	No temporal link
Medical (e.g. anaemia, thyroid, sleep apnoea)	Fatigue, physical signs	Normal investigations

Comorbidity is the rule, not the exception. Depression and anxiety co-occur; substance use and trauma co-occur; child behaviour problems and parental mental illness co-occur. Prioritise: treat what is most dangerous, most disabling, or most treatable first, and treat the family context alongside.

Week 10 — Medical, neurological and substance rule-outs

Refer, do not diagnose. Your job is to notice and to route.

Presentation	Rule out	Route
First episode of psychosis	Organic causes, substance-induced	Urgent psychiatric referral
Sudden confusion in an elder	Delirium, infection, medication	Same-day medical review
Fatigue, weight change, low mood	Anaemia, thyroid, diabetes, sleep apnoea	GP with specific request
Memory decline	Dementia vs depression vs delirium	Cognitive assessment service
Headaches, seizures, personality change	Neurological	Neurology
Erectile dysfunction, low libido	Vascular, hormonal, medication	Medical review before psychologising
Unexplained somatic complaints	Physical illness, somatisation	GP plus psychiatric liaison

Write a referral that works. State what you observed, what you want excluded, why now, and who to contact. "Please assess and advise" produces a delay; "Please exclude anaemia and hypothyroidism in a 34-year-old woman with 3 months of fatigue and low mood, as I am considering a diagnosis of depression and would like to rule out physical causes before starting therapy" produces a response.

Week 11 — Observed assessment interview

Thirty minutes, assessed: contract and confidentiality, each person's view, interaction observation task, risk screen, strengths, and a closing summary with next steps. The family rates their experience of being assessed on three dimensions: heard, not judged, clear about what happens next.

Week 12 — Relational and systemic diagnosis

Individual diagnosis does not describe a relationship. Three ways of adding the relational layer:

1. Relational descriptors. Not a code, but a description: "a couple in a pursue–withdraw cycle with low repair capacity"; "a family with a cross-generational coalition and a disengaged father"; "a family at a stage transition with no renegotiation".

2. Global functioning of the relationship. Instruments such as the GARF (Global Assessment of Relational Functioning) rate the family or couple's functioning along a continuum, with anchors. Useful for outcome monitoring and for communicating severity.

3. The formulation as the relational diagnosis. The 5-P grid plus relational columns is the most clinically useful "diagnosis" a family therapist produces.

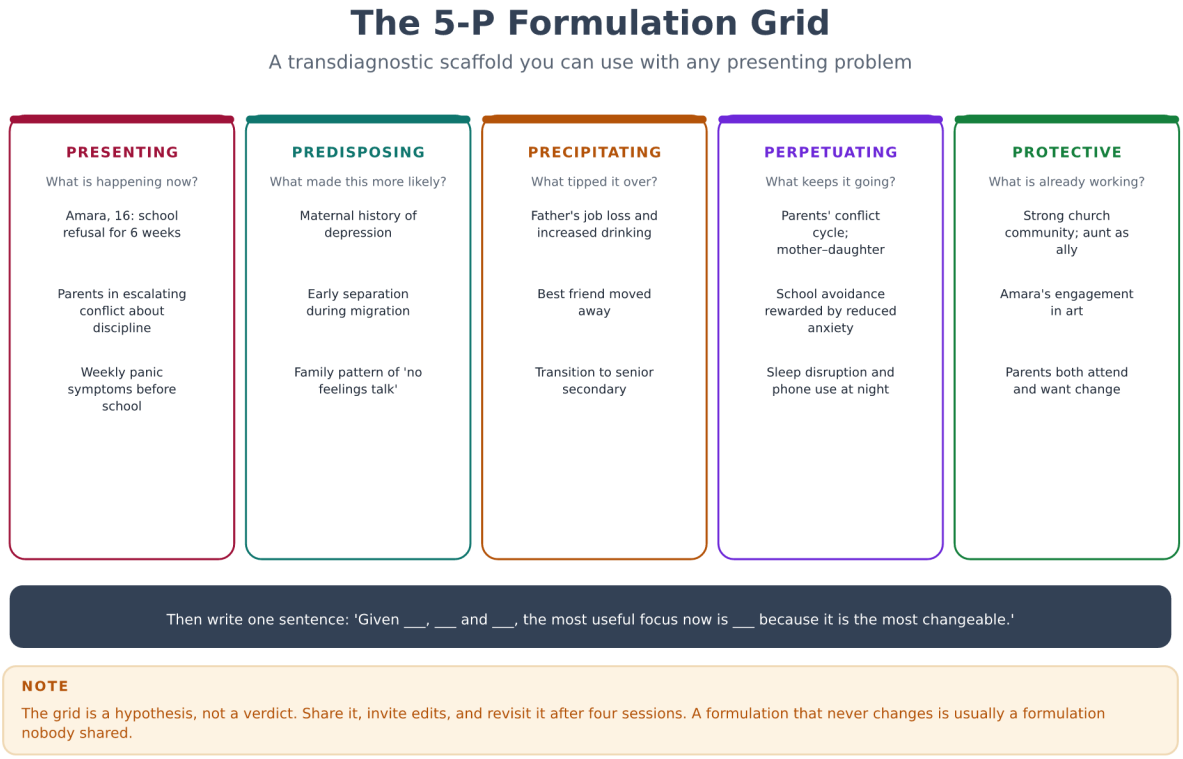


Figure 12.1 — The 5-P formulation grid applied to one family

Worked formulation (Adeyemi family, condensed).

> **PRESENTING.** Amara, 16, has not attended school for six weeks; panic symptoms on school mornings; escalating parental arguments about discipline. **Predisposing.** Maternal history of depression; paternal alcohol misuse; migration removed the extended family's daily support; a family rule that feelings are not discussed. **Precipitating.** Father's redundancy, a best friend's relocation, and the transition to senior secondary school, all within four months. **Perpetuating.** A pursue–withdraw cycle between the parents; a mother–daughter coalition that stabilises the marital distance; the relief Amara experiences when she stays home, which reinforces avoidance; disrupted sleep and night-time phone use. **Protective.** Both parents attend; a trusted aunt; church community; Amara's engagement with drawing; the family's capacity for humour. **Hypothesis (testable).** If the parents can complete one conflict conversation without Amara's involvement, her morning panic will reduce within three weeks. **Focus:** de-escalate the couple cycle while building a graded return-to-school plan that does not depend on Amara's compliance alone.

Week 13 — Cultural formulation and bias

The Cultural Formulation Interview (CFI) — a set of 16 questions from DSM-5 — gives structure to what many clinicians do informally:

1. What brings you here, in your own words?
2. What do you call this problem?
3. What do you think caused it?
4. How severe is it, and what worries you most?
5. What has helped, and what has not?
6. What kind of treatment do you expect?
7. Is there anything about your background or identity I should know?

Sources of bias in assessment.

Bias	Effect	Mitigation
Availability	Over-diagnosing what you saw last week	Use structured criteria
Confirmation	Seeking evidence for your first hypothesis	Actively seek disconfirmation
Cultural	Pathologising difference	Ask about meaning; consult
Gender	Reading assertion in women as pathology; in men as strength	Check your language
Class	Reading material hardship as neglect	Ask about money explicitly
Attribution	Blaming parents for adolescent behaviour	Include context and strengths

---

Week 14 — Feedback and report writing

Feedback is an intervention. Structure it in four parts: what I heard, what I think is happening, what I am unsure about, what I suggest. Then stop talking and invite correction.

The report. Write for three readers: the family, the referring professional, and a future clinician who has never met you. Suggested structure:

1. Identifying information (minimal — no more than necessary)
2. Reason for referral (in the referrer's words)
3. Consent, confidentiality and who was seen
4. Sources of information
5. Presenting concerns, from each informant
6. History (developmental, family, medical, psychiatric, substance, social)
7. Observation of interaction
8. Measures administered, with scores and interpretation
9. Risk assessment and outcome

10. Formulation (5-P plus relational hypothesis)
11. Diagnosis, if any, with criteria met and differentials considered
12. Recommendations, with who will do what
13. Review date and signature

Style rules. No adjectives about people; no speculation without labelling; no jargon the family cannot read; no more than two pages for the family copy.

> **PRACTICAL**
> Write the family-facing summary (one page, no jargon) of your full report, and test it on a non-clinician.

Week 15 — Integration

Assessment is the moment a family learns what kind of problem they have. Get it right and therapy has a spine; get it wrong and every subsequent session is spent working on the wrong thing. The standard is not comprehensiveness; it is **sufficiency** — enough information to make a defensible formulation and a workable plan, gathered in a way that leaves the family feeling respected.

Glossary

Collateral information — data from third parties with consent. **Cultural formulation** — a structured account of the cultural context of a presentation. **Diagnostic overshadowing** — attributing a physical or relational problem to a psychiatric label. **Differential diagnosis** — the systematic consideration of alternative explanations. **GARF** — Global Assessment of Relational Functioning. **Measurement invariance** — whether an instrument behaves equivalently across groups. **Multi-informant assessment** — gathering data from several people who see the person in different contexts. **Rule-out** — a condition that must be excluded before a diagnosis is confirmed. **Screening** — a brief procedure identifying those who need fuller assessment.

Self-assessment

Multiple choice

1. The multi-informant principle exists because: (a) families lie (b) informants see different contexts and disagreement is informative (c) it is legally required (d) it fills the session

2. A high couple-satisfaction score means: (a) the relationship is safe (b) the relationship is satisfactory to the respondent, not that it is safe (c) the couple will stay together (d) no further assessment is needed

3. Diagnostic overshadowing refers to: (a) over-diagnosing (b) a label hiding another problem (c) comorbid diagnoses (d) cultural bias

4. The interaction task "agree on one change this week" is used to observe: (a) memory (b) negotiation, hierarchy and coalition (c) IQ (d) attachment classification

5. Which is the best referral request? (a) "Please assess and advise" (b) "Please see this patient" (c) "Please exclude anaemia and hypothyroidism in a 34-year-old woman with 3 months of fatigue" (d) "Please confirm my diagnosis"

6. The 5-P formulation stands for presenting, predisposing, precipitating, perpetuating and: (a) psychiatric (b) protective (c) prognostic (d) provisional

7. Screening instruments should be treated as: (a) diagnoses (b) questions requiring follow-up (c) evidence of malingering (d) definitive cut-offs

8. The CFI is: (a) a personality test (b) a cultural formulation interview (c) a child functioning index (d) a court report

9. The strengths inventory matters because it: (a) is required by codes (b) changes tone and predicts engagement (c) replaces diagnosis (d) shortens assessment

10. A treating therapist should generally decline to conduct: (a) an intake (b) a custody evaluation of their own client (c) a genogram (d) an outcome measure

Answer key

1 b · 2 b · 3 b · 4 b · 5 c · 6 b · 7 b · 8 b · 9 b · 10 b

Short answer

1. List the five parties who may have an interest in an assessment and what each wants.
2. Give three behavioural (not judgemental) observation statements.
3. What are the four parts of feedback to a family?
4. Name three biases in assessment and one mitigation for each.
5. Why must IPV screening be conducted separately from the partner?

Essay (choose one, 2,000 words)

1. "Diagnosis in family therapy is both necessary and dangerous." Discuss.
2. Using a disguised case, present a full assessment and defend your formulation against two alternatives.
3. Critically evaluate the claim that standardised family measures are culturally biased.

Further reading

- American Psychiatric Association. (2022). DSM-5-TR. APA Publishing.
- World Health Organization. (2019/2022). ICD-11.

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- Snyder, D. K., Heyman, R. E., & Haynes, S. N. (2005). Evidence-based approaches to assessing couple distress. Psychological Assessment, 17(3), 288–307.
- Sperry, L. (2012). Family Assessment: Contemporary and Cutting-Edge Strategies. Routledge.

Next: MFT 708 — Cultural Competence in Therapy

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MFT 708

Cultural Competence in Therapy

14 teaching weeks · 5 practicals · 2 worked cases · 2 figures

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MFT 708 — Cultural Competence in Therapy

MSc Marriage and Family Therapy · Semester One · 1.71 Units · 14 teaching weeks · Prerequisite: MFT 701

This volume is one of 28 course books that together cover the whole MSc Marriage and Family Therapy taught programme. Each book is written to carry a 14–16 week semester on its own: every week pairs a core idea with the practice that follows from it, a worked case, and a reflective task, and each book closes with an integration chapter, a glossary and a self-assessment paper with model answers.

The books are designed to be read straight through before the semester begins, revisited week by week, and used again in revision. Cross-references point to the other volumes where a topic is treated in more depth.

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A note on the case material

All families, couples and individuals in this book are fictional composites. They are written to teach pattern recognition, not to describe any real person. Nothing here replaces supervised clinical practice, professional registration, legal advice or medical care; legal content is orientation only and must be checked against the law of the relevant jurisdiction.

The lecture library

Twenty-eight self-contained course books carry the taught programme: eight in Semester One, seven in Semester Two, seven in Semester Three including the thesis, and six electives. The volume you are holding is marked in the margin.

Code	Course	Semester	Units
MFT 701	Fundamentals of Marriage and Family Therapy	One	1.71
MFT 702	Theories of Family Therapy	One	1.71
MFT 703	Human Development Across the Lifespan	One	1.71
MFT 704	Ethical and Legal Issues in Therapy	One	1.71
MFT 705	Research Methods in Marriage and Family Therapy	One	1.71
MFT 706	Clinical Practice and Techniques	One	1.71
MFT 707	Assessment and Diagnosis in Family Therapy	One	1.71
MFT 708	**Cultural Competence in Therapy**	One	1.71
MFT 709	Group Therapy Techniques	Two	1.71
MFT 710	Couples Therapy	Two	1.71
MFT 711	Child and Adolescent Therapy	Two	1.71
MFT 712	Crisis Intervention Strategies	Two	1.71
MFT 713	Substance Abuse and Family Dynamics	Two	1.71
MFT 714	Trauma-Informed Care	Two	1.71
MFT 715	Family Systems Theory	Two	1.71
MFT 716	Communication Skills in Therapy	Three	1.71
MFT 717	Psychopathology in Families	Three	1.71
MFT 718	Intimacy and Relationship Dynamics	Three	1.71
MFT 719	Family Life Cycle and Transitions	Three	1.71
MFT 720	Supervision and Consultation in Therapy	Three	1.71
MFT 721	Research Methodology	Three	1.71
MFT 722	Professional Development and Self-Care	Electives	1.71
MFT 723	Special Populations in Therapy	Electives	1.71
MFT 724	Family Law and Advocacy	Electives	1.71
MFT 725	Integrative Approaches in Therapy	Electives	1.71
MFT 726	Neurobiology of Relationships	Electives	1.71
MFT 727	Play Therapy Techniques	Electives	1.71
MFT 770	Master Thesis	Three	1.71

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Course leader's summary. Culture is not a topic added to therapy; it is the medium in which therapy happens. This course moves you from "knowing about cultures" to working skilfully with difference, power and meaning — including your own.

How to use this book

You will be uncomfortable at points in this course. That is not a design flaw; it is the learning. Each week pairs a **concept** with a **practice** that requires you to examine your own position. Do the exercises honestly and keep them private — this is formation, not performance.

1. Course description

The field's language has shifted from cultural competence (a set of knowledge and skills, often taught as a list of group traits) to cultural humility (a lifelong process of self-examination, redress of power imbalance, and institutional accountability) and cultural safety (the outcome judged by the person receiving care).

This course teaches all three, and is honest about their limits. It covers race and racism, ethnicity, religion and spirituality, class and poverty, gender and power, sexuality, disability, migration and displacement, language and interpreting, and the intersection of all of these in a single family.

2. Learning outcomes

#	Outcome	Assessed
LO1	Distinguish cultural competence, humility and safety, and use each appropriately	Exam
LO2	Conduct a cultural assessment using a structured framework	Observed interview
LO3	Analyse power, privilege and intersectionality in a clinical relationship	Reflective portfolio
LO4	Work effectively with interpreters and in a second language	Skills assessment
LO5	Adapt assessment and intervention to extended-family, faith-based and polygynous structures	Case analysis
LO6	Recognise and interrupt racism, ethnocentrism and bias in your own practice	Portfolio, viva
LO7	Critically appraise the cultural assumptions of the major therapy models	Essay

3. Assessment

Component	Weight	Due
Reflexive portfolio (6 entries × 400 words)	30%	Fortnightly
Cultural assessment observed interview (25 min)	25%	Week 9
Case analysis with cultural formulation (2,500 words)	45%	Week 14

4. The fourteen-week plan

Week	Topic	Practice
1	What culture is — and is not	Cultural genogram (own family)
2	Competence, humility, safety: history and critique	Concept comparison
3	The ADDRESSING framework	Structured cultural interview
4	Race, racism and the clinical encounter	Microaggression analysis
5	Ethnicity, tribe, language and migration	Interpreter role-play
6	Religion and spirituality as clinical structure	Faith-history interview
7	Class, poverty and material reality	Budget exercise
8	Gender, power and the politics of the family	Power mapping
9	**Cultural assessment observed interview**	Assessed
10	Sexuality, gender identity and family acceptance	Coming-out family session
11	Disability, chronic illness and care	Accessibility audit
12	Polygyny, extended family and communal child-rearing	Structural adaptation
13	Decolonising therapy: models, manuals and evidence	Critical appraisal
14	Integration: your cultural position statement	Viva

Week 1 — What culture is — and is not

Culture is the shared, learned system of meanings, practices, values and identities through which a group makes sense of the world. It is dynamic, internalised, contested and partially unconscious.

Five things culture is not.

1. **Not nationality.** "Nigerian culture" contains hundreds of languages and dozens of marriage systems.
2. **Not a personality.** Culture sets a range of possible behaviours, not a script every member follows.
3. **Not static.** Families migrate, convert, intermarry and change class; their culture changes with them.

4. **Not only "the other".** You have a culture too, and so does your profession.
5. **Not an excuse.** "That is our culture" can describe a practice or justify coercion; the therapist must be able to hold respect and moral clarity at the same time — particularly about violence.

The clinical implication. Never ask "what do people from X do?" Ask "what does this family do, what does it mean to them, who in the family disagrees, and how does it interact with the problem?"

> **PRACTICAL**
> Draw your own cultural genogram: languages, religions, migrations, class position, ethnic identities, and the rules you absorbed about marriage, money, gender and feelings. Note what you would find difficult to say to a family that did the opposite.

Week 2 — Competence, humility, safety

Concept	Core claim	Strength	Limitation
Cultural competence	Knowledge and skills about groups can be taught	Practical, trainable	Risks stereotyping; assumes culture is a trait
Cultural humility	Lifelong self-reflection, redress of power, institutional accountability	Attends to power and to the self	Can become inward-looking; hard to assess
Cultural safety (Ramsden)	The recipient judges whether care is safe	Centres the client's experience	Requires institutional change, not only individual skill

The synthesis used in this programme. Humility as the stance, competence as the skills, safety as the criterion. You reflect on your position (humility), use structured tools and knowledge (competence), and ask the family whether the encounter was safe (safety).

Why "I treat everyone the same" is not an ethical position. Treating everyone the same preserves the advantages of the majority, because the default is not neutral — it was built for someone.

Week 3 — The ADDRESSING framework

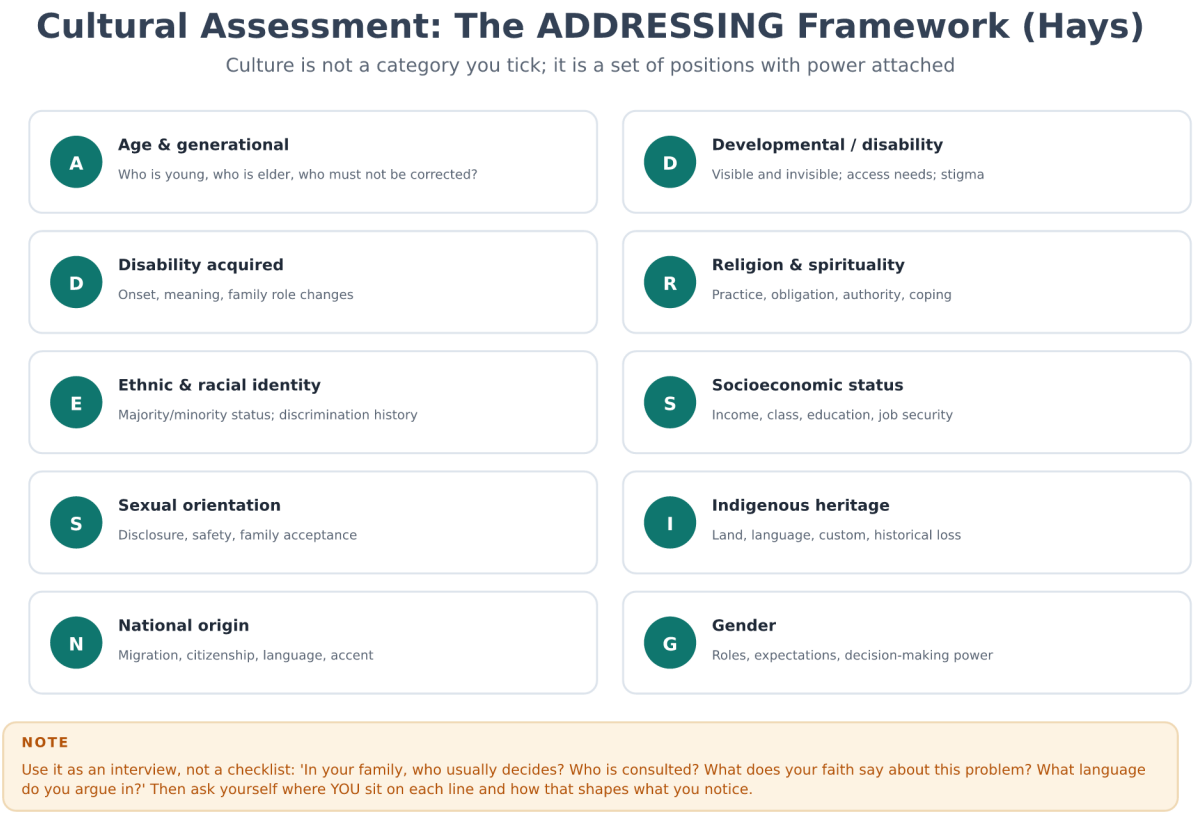


Figure 3.1 — ADDRESSING: ten cultural positions with power attached

Hays's framework asks you to assess ten dimensions, for the family **and for yourself**:

Age and generation · Developmental disability · Disability acquired later · Religion and spirituality · Ethnic and racial identity · Socioeconomic status · Sexual orientation · Indigenous heritage · National origin · Gender.

Interview prompts that work.

- "In your family, who usually decides? Who is consulted? Who is told afterwards?"
- "What does your faith say about this problem? Who in your religious community knows?"
- "Which language do you argue in? Which language do you pray in?"
- "When you were growing up, was money talked about?"
- "What did your family teach you about people from [another group]?"

The power question. For each dimension, ask: is this person in the majority or the minority in this society, in this clinic, and in this room? The same person may be privileged on one axis and marginalised on another — this is intersectionality.

> **PRACTICAL**
> Conduct a 15-minute structured cultural interview with a peer using five ADDRESSING domains. Then reverse roles. Debrief: which question was hardest to ask, and why?

Week 4 — Race, racism and the clinical encounter

Definitions. Race is a social category with no biological basis; its effects are nonetheless real and measurable. Racism is the system of advantage and disadvantage organised around race, operating at interpersonal, institutional and internalised levels.

Three clinical implications.

1. **Racism is a chronic stressor with health consequences** — including hypertension, depression, and adverse birth outcomes. It belongs in the assessment, not in a footnote.
2. **Therapy is not a racism-free space.** Families may test you, protect you, or perform for you. Ask: "Have you had experiences in health settings where you were treated badly because of your background?"
3. **Microaggressions occur in clinical settings** — comments, assumptions, lowered expectations. They are often unintentional and are still harmful.

Four common therapist failures.

Failure	Example	Repair
Colour-blindness	"I don't see race"	Acknowledge difference; ask about its meaning here
Defensiveness	"I'm not racist, but…"	Focus on impact, not intent
Over-attribution	Explaining everything by race	Differentiate race from other factors
Exoticising	Treating the family as a cultural exhibit	Ask about ordinary life too

Talking about race when it has not been mentioned. Wait for a cue, then ask directly: "I wondered whether the fact that I am [X] and you are [Y] matters here. Sometimes it does, sometimes it doesn't." Offering the question is often more important than the answer.

> **PRACTICAL**
> Analyse three microaggression scenarios from a clinical setting: what was said, what was meant, what was heard, what a repair would sound like.

Week 5 — Ethnicity, tribe, language and migration

Ethnicity and tribe. In Nigeria and across Africa, ethnic identity carries histories of majority/minority relations, language, land, and in some cases conflict. Ask about it as a relationship question: how do the two families regard each other's ethnicity? Was the marriage contested? Whose language do the children speak?

Migration. Migration is a family event, not an individual one. Key clinical themes:

- **Split households** — one parent abroad, children with grandparents, "sent" children.

- **Role reversal** — children who acquire the new language become the family's intermediaries.
- **Loss of the extended-family buffer** — the practical support that made parenting possible.
- **Status loss** — a professional who becomes a cleaner; a respected elder who becomes invisible.
- **Legal precarity** — the constant threat of removal, which changes every decision.

Language. A family may think in one language about emotion and another about work. Ask: "In which language do you feel most like yourself? Which language do you dream in?" Offer sessions in the family's preferred language where you can; where you cannot, use an interpreter properly.

Working with interpreters — the standard.

1. Brief before: purpose, confidentiality, first-person interpretation, do not summarise or advise.
2. Seat the interpreter beside you, facing the family.
3. Speak to the family, not to the interpreter ("How do you feel?" not "Ask her how she feels").
4. Allow longer for interpretation; do not fill pauses.
5. Debrief afterwards: what was difficult, what was not said, any cultural information.
6. **Never** use a child as interpreter for anything clinical.

> **PRACTICAL**
> Interpreter role-play: one student interprets, one conducts, one plays a family member who switches languages. Practise briefing, first-person interpretation, and debriefing.

Week 6 — Religion and spirituality as clinical structure

In most of the world, faith is not an "aspect" of family life; it is the organising framework. Asking about it is not optional.

What to ask. What is your faith? How does it shape daily life (prayer, diet, dress, holy days)? What does it say about marriage, divorce, sex, mental illness, healing, and death? Who is your religious authority? Have you consulted them about this problem? What did they say?

Three clinical patterns.

1. **Faith as resource.** Community, meaning, ritual, forgiveness, hope, practical support.
2. **Faith as stressor.** Guilt, prohibition of divorce, deliverance beliefs that attribute mental illness to sin, conflict between religious and medical models.
3. **Faith as site of conflict.** A couple from different traditions; conversion; a teenager leaving the faith; a pastor's authority competing with the therapist's.

Coordination, not competition. With consent, a conversation with the family's pastor, imam or elder can be more powerful than any technique. The aim is a shared plan, not a theological debate. You do not have to share the family's beliefs to take them seriously; you do have to be honest about your own.

> **CASE**
> A couple is referred after the wife disclosed to her pastor that her husband has been violent. She will not go to the police; the pastor has counselled her to be patient. The therapist must respect her faith and autonomy, hold the pastor as an ally where possible, and build a safety plan that does not depend on the husband's repentance. (See MFT 712, 724.)

Week 7 — Class, poverty and material reality

The clinical error. Reading the effects of poverty as individual or family pathology: "inconsistent parenting" when the mother works three jobs; "neglect" when the family has no electricity; "disorganised" when the household has eight people in two rooms.

What to ask, without humiliating.

- "Is money tight at the moment? How tight?"
- "What happens at the end of the month?"
- "Who else depends on your income?"
- "Have you ever had to choose between food and a bill?"

Practical action. Have a short list of referral routes: food support, school fee assistance, legal aid, social welfare, health insurance. A therapist who ignores material reality becomes part of the family's helplessness.

The class position of therapy itself. Sessions cost money, require transport and time off, and are conducted in a register many families do not use. Adapt: flexible times, community venues, shorter paperwork, plain language, a written summary the family can keep.

> **PRACTICAL**
> Complete a household budget for a family with two minimum-wage incomes and two children. Then re-read a "poor parenting" case with that budget in mind.

Week 8 — Gender, power and the politics of the family

Power is not the same as influence. Ask: who decides, who is consulted, who is informed, who is financially dependent, who can leave, who would be believed if they spoke?

Gender patterns you will meet.

Pattern	Presentation	Clinical risk if ignored
Traditional division of labour	Mother "does everything" and is exhausted; father "does nothing"	Therapist aligns with mother; father withdraws further
Head-of-household ideology	Father decides; mother adapts	Mother's distress is invisible; children learn silence
Female economic power	Wife out-earns husband	Status threat becomes the real argument
Male emotional restriction	He cannot say what he feels	Therapist misreads flatness as lack of love

The feminist critique that changed the field (Hare-Mustin, Goldner, Walters): early family therapy described "circular" patterns as though both parties had equal power. In violence, in economic dependence, and in cultural constraints, they do not. A circular description that ignores power becomes a sophisticated form of blame-the-victim.

Clinical practice. Name the asymmetry without moralising; ask the family whether they want it to be this way; work with the person who holds more power to use it differently; never insist a woman "communicate better" as a substitute for a man changing his behaviour.

Week 9 — Cultural assessment observed interview

Twenty-five minutes, assessed against six criteria: use of a structured framework, curiosity rather than assumption, power-attentiveness, no stereotyping, integration of culture with the presenting problem, and a respectful close.

Week 10 — Sexuality, gender identity and family acceptance

The clinical reality. In many settings, disclosure is unsafe, and families are torn between religious conviction, social pressure, and love for their child. Your task is not to resolve that for them but to make the conversation possible and to keep everyone safe.

Principles.

1. **Do not assume or ask for labels prematurely.** Follow the client's language.
2. **Assess safety first.** Is it safe at home, at school, at church? Who knows? What is the risk of disclosure?
3. **Work with the family's ambivalence**, not against it. "You have both said you love your son. You have also said you cannot accept this. I want to understand both, without asking you to give either up today."
4. **Never promise a family you will change the child**, and never promise a child you will change the family. Be honest with both.

5. **Know your legal context and your local referral routes**, including safe medical and mental health services.

When the family rejects. Rejection is a process with stages: shock, denial, negotiation, and — in some families — accommodation. Your job is to keep the door open, reduce harm, and protect the most vulnerable member.

Week 11 — Disability, chronic illness and care

Two models. The medical model locates disability in the body; the social model locates it in barriers — physical, attitudinal, institutional. Both matter clinically.

Assessment questions. What is the diagnosis and functional impact? Who provides care, and at what cost? What has the family had to give up? What is the meaning of the condition in this family's belief system (punishment, test, curse, genetics)? What accommodations are needed?

Avoiding two errors. Minimising ("it's not that bad") and catastrophising ("your life is over"). Ask the person directly: "What do you want me to understand about how this affects you?"

Sibling and parent impact. Caregiving burden falls unequally; siblings of disabled children often develop precocious competence and unspoken resentment. Name it as love and loss together.

Week 12 — Polygyny, extended family and communal child-rearing

Do not treat structure as pathology. Household forms that differ from the Western nuclear norm are not automatically dysfunctional. Assess function, safety and fairness within the form.

Polygynous households — assessment prompts.

- Who lives where, and who decides?
- How are resources and time allocated? Who keeps the accounts?
- What are the relationships between wives — allies, rivals, or both?
- How do children understand their relationships to the other wives?
- Is the marriage legal under a statutory, customary or religious system (the answer affects rights)?
- Who in the household holds authority over the children?

Clinical themes that recur: jealousy and comparison, resource competition, co-wife coalitions and sub-coalitions, children as messengers, unequal treatment, and the husband's role as the only permitted channel of communication.

Communal child-rearing. In many families, a child is raised by a grandmother, an aunt, or a foster kin network. This can be protective (multiple attachment figures) or risky (unclear responsibility, unspoken injury). The question is not "who is the mother?" but "who is responsible for what, and does the child know who to go to?"

> **CASE**
> A husband proposes a second wife. The first wife presents with insomnia and anger; he presents with confusion. The therapist neither endorses nor condemns the practice. The work is: what does each understand the marriage to be? What does the first wife have a right to? What will the children be told? Are there religious and legal structures the family wants involved?

Week 13 — Decolonising therapy

The critique. Most of the therapy models in this programme were developed in North America and Europe, tested on samples that were not representative, and then exported as universal. Three specific problems:

1. **Normative assumptions.** Individuation, self-expression and boundary-setting are treated as health. In many families, interdependence, duty and discretion are the markers of maturity.
2. **Evidence hierarchies.** Trials and manualised models carry more status than community knowledge and qualitative research, which subtly determines what can be funded and taught.
3. **Language.** "Enmeshment", "over-involvement", "dependence" are not neutral words; they encode a picture of the good family.

Responses that are more than rhetoric.

- Learn the local literature; cite African scholars; test instruments locally.
- Adapt models explicitly and say what you changed and why.
- Use the family's language and concepts in formulation.
- Include community and faith resources in treatment plans.
- Publish your adaptations so the field's evidence base changes.

And a caution. Decolonisation is not the replacement of one orthodoxy with another. Families are not monoliths, and "that is our culture" can silence a woman or a child as effectively as a Western manual can.

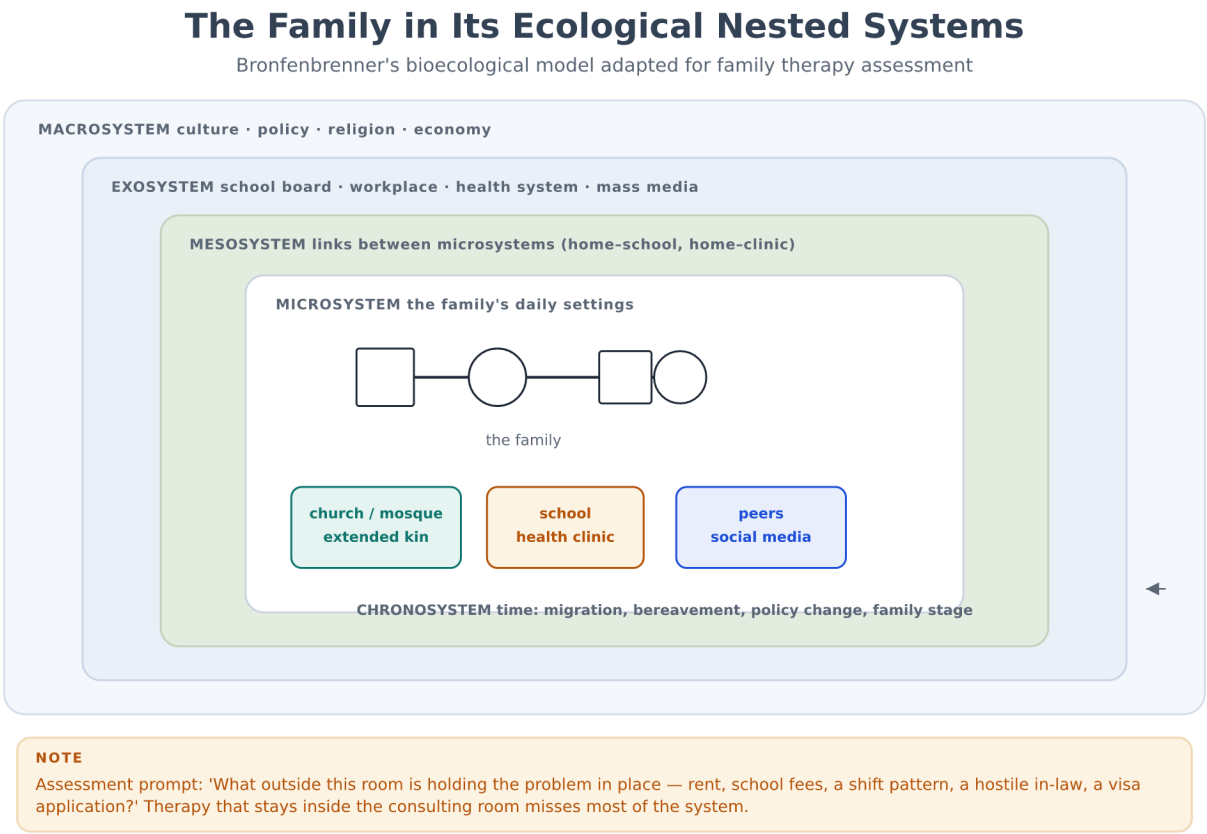


Figure 13.1 — Ecological systems: the family inside institutions, culture and time

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Week 14 — Integration: your cultural position statement

Write 1,000 words: your cultural identities and their advantages and disadvantages; two families with whom you would struggle and why; the adaptations you will make; the assumptions of your preferred model that you will watch; and how you will know when you have got it wrong. Present it in a viva.

Signals that you have got it wrong (learn to recognise them): the family is polite and does not return; you are doing most of the talking; you have explained their culture to them; you feel defensive; you have used the word "they".

Glossary

Cultural humility — a lifelong stance of self-examination and redress of power imbalance. **Cultural safety** — the recipient's judgment that care is safe and free of assault on identity. **Cultural formulation** — a structured account of culture in a presentation (e.g. the DSM-5 CFI). **Ethnocentrism** — judging another culture by the standards of one's own. **Intersectionality** — the interacting effects of multiple social positions. **Microaggression** — a brief, commonplace indignity, often unintentional. **Power** — the capacity to influence decisions, resources and meanings; distinct from influence. **Stereotype threat** — the risk of confirming a negative stereotype about one's group. **Structural competency** — the capacity to recognise and act on upstream social determinants.

Self-assessment

Multiple choice

1. "I treat everyone the same" is inadequate because: (a) it is impossible (b) the default is not neutral (c) clients dislike it (d) it is illegal

2. Cultural safety is judged by: (a) the therapist (b) the institution (c) the recipient of care (d) the code of ethics

3. Asking a family "what does your faith say about this problem?" is: (a) inappropriate (b) essential assessment (c) only for religious therapists (d) a legal requirement

4. An interpreter should: (a) summarise key points (b) interpret in the first person (c) advise the family (d) be the family's child

5. The social model of disability locates disability primarily in: (a) the body (b) barriers in society (c) genetics (d) family dynamics

6. A circular formulation that ignores power risks: (a) complexity (b) blaming the less powerful partner (c) over-diagnosis (d) cultural sensitivity

7. Intersectionality refers to: (a) cultural competence (b) interacting social positions (c) bilingualism (d) family structure

8. A family arrives with a co-wife arrangement. The therapist should first: (a) assess for pathology (b) endorse the arrangement (c) assess function, safety and fairness within the structure (d) refuse to work with them

9. Microaggressions in clinical settings are: (a) always intentional (b) often unintentional and still harmful (c) rare (d) not clinically relevant

10. Decolonising therapy means: (a) rejecting all Western models (b) adapting and testing models, citing local knowledge, changing the evidence base (c) using only local languages (d) avoiding diagnosis

Answer key

1 b · 2 c · 3 b · 4 b · 5 b · 6 b · 7 b · 8 c · 9 b · 10 b

Short answer

1. Distinguish cultural competence, humility and safety in one sentence each.
2. Write four ADDRESSING prompts you would actually use.
3. What are the six steps of working with an interpreter?
4. Give three clinical effects of racism and one assessment question.
5. Name three culturally specific assumptions in EFT or structural therapy and how you would adapt them.

Essay (choose one, 2,000 words)

1. "Cultural humility is a stance, not a technique." Discuss with reference to two clinical encounters.
2. Critically examine the claim that family therapy models pathologise interdependence.
3. Analyse a case in which your own cultural position shaped the assessment, and what you would do differently.

Further reading

- Hays, P. A. (2016). Addressing Cultural Complexities in Practice (3rd ed.). APA.
- Tervalon, M., & Murray-García, J. (1998). Cultural humility versus cultural competence. Journal of Health Care for the Poor and Underserved, 9(2), 117–125.
- McGoldrick, M., Giordano, J., & Garcia-Preto, N. (Eds.). (2005). Ethnicity and Family Therapy (3rd ed.).
- Falicov, C. J. (2014). Latino Families in Therapy (2nd ed.). Guilford.
- Ramsden, I. (2002). Cultural Safety and Nursing Education. Victoria University of Wellington.
- Brady, M. (Ed.). (2012). The Oxford Handbook of the Philosophy of Culture. OUP.

End of Semester One. Next: MFT 709 — Group Therapy Techniques

MSC MARRIAGE AND FAMILY THERAPY

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MFT 709

Group Therapy Techniques

15 teaching weeks · 5 practicals · 1 figure

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This volume is one of 28 course books that together cover the whole MSc Marriage and Family Therapy taught programme. Each book is written to carry a 14–16 week semester on its own: every week pairs a core idea with the practice that follows from it, a worked case, and a reflective task, and each book closes with an integration chapter, a glossary and a self-assessment paper with model answers.

The books are designed to be read straight through before the semester begins, revisited week by week, and used again in revision. Cross-references point to the other volumes where a topic is treated in more depth.

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A note on the case material

All families, couples and individuals in this book are fictional composites. They are written to teach pattern recognition, not to describe any real person. Nothing here replaces supervised clinical practice, professional registration, legal advice or medical care; legal content is orientation only and must be checked against the law of the relevant jurisdiction.

The lecture library

Twenty-eight self-contained course books carry the taught programme: eight in Semester One, seven in Semester Two, seven in Semester Three including the thesis, and six electives. The volume you are holding is marked in the margin.

Code	Course	Semester	Units
MFT 701	Fundamentals of Marriage and Family Therapy	One	1.71
MFT 702	Theories of Family Therapy	One	1.71
MFT 703	Human Development Across the Lifespan	One	1.71
MFT 704	Ethical and Legal Issues in Therapy	One	1.71
MFT 705	Research Methods in Marriage and Family Therapy	One	1.71
MFT 706	Clinical Practice and Techniques	One	1.71
MFT 707	Assessment and Diagnosis in Family Therapy	One	1.71
MFT 708	Cultural Competence in Therapy	One	1.71
MFT 709	**Group Therapy Techniques**	Two	1.71
MFT 710	Couples Therapy	Two	1.71
MFT 711	Child and Adolescent Therapy	Two	1.71
MFT 712	Crisis Intervention Strategies	Two	1.71
MFT 713	Substance Abuse and Family Dynamics	Two	1.71
MFT 714	Trauma-Informed Care	Two	1.71
MFT 715	Family Systems Theory	Two	1.71
MFT 716	Communication Skills in Therapy	Three	1.71
MFT 717	Psychopathology in Families	Three	1.71
MFT 718	Intimacy and Relationship Dynamics	Three	1.71
MFT 719	Family Life Cycle and Transitions	Three	1.71
MFT 720	Supervision and Consultation in Therapy	Three	1.71
MFT 721	Research Methodology	Three	1.71
MFT 722	Professional Development and Self-Care	Electives	1.71
MFT 723	Special Populations in Therapy	Electives	1.71
MFT 724	Family Law and Advocacy	Electives	1.71
MFT 725	Integrative Approaches in Therapy	Electives	1.71
MFT 726	Neurobiology of Relationships	Electives	1.71
MFT 727	Play Therapy Techniques	Electives	1.71
MFT 770	Master Thesis	Three	1.71

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Course leader's summary. Groups are the most efficient and the most unpredictable clinical format. This course teaches you to design a group, build a culture, work with the process rather than only the content, manage the difficult members, and co-facilitate without performing a marriage in front of the members.

How to use this book

A group cannot be learned from a book alone, but it can be ruined without one. Each week gives you the theory, a facilitation script, and a hazard. The practical sessions in this course run as a real group: you will be members before you are leaders, and you will write about both.

1. Course description

Group therapy is a distinct modality with its own theory of change: members help each other, and that mutual aid is the treatment. Family therapists use groups for couples (multiple couples in one group), parents, adolescents, men who have used violence, people in recovery, the bereaved, and children.

This course covers group design, composition, stages of development, therapeutic factors, process observation, difficult members, co-facilitation, ethics and evaluation. It is grounded in Yalom's interpersonal model and in systemic thinking about the group-as-a-system.

2. Learning outcomes

#	Outcome	Assessed
LO1	Design a group: purpose, membership, size, duration, setting	Group proposal
LO2	Explain and use Yalom's therapeutic factors	Exam
LO3	Identify and work with group process (here-and-now)	Observed facilitation
LO4	Manage difficult members and group crises safely	Viva, observed
LO5	Co-facilitate effectively, including managing disagreement	Peer and tutor rating
LO6	Screen and prepare members ethically	Screening role-play
LO7	Evaluate group outcomes	Evaluation plan

3. Assessment

Component	Weight	Due
Group design proposal (2,000 words)	25%	Week 6
Process reflection log (as member, 6 entries)	20%	Fortnightly

Component	Weight	Due
Observed co-facilitation (30 min)	30%	Weeks 12–14
Examination	25%	Week 16

4. The fifteen-week plan

Week	Topic	Practical
1	Why groups work: mutual aid and the social microcosm	First group session (as member)
2	Types of group in family therapy	Observation of a live/recorded group
3	Design: aims, size, duration, open vs closed	Design clinic
4	Screening, consent and preparation	Screening role-play
5	Yalom's therapeutic factors	Factor-spotting exercise
6	**Group proposal due**; group stages I: forming and storming	Process observation
7	Group stages II: norming, performing, adjourning	Leader tasks by stage
8	The here-and-now: working with process	Process commentary drill
9	Difficult members: monopoliser, silent, hostile, help-rejecting	Structured rehearsals
10	Conflict, scapegoating and subgrouping in groups	Live intervention
11	Co-facilitation: division of labour, disagreement, repair	Co-therapy pairs
12	**Observed co-facilitation begins**	Assessed
13	Groups for couples and for parents	Design adaptation
14	Ethics, confidentiality and boundaries in groups	Incident analysis
15	Evaluation and endings	Outcome plan

Week 1 — Why groups work

The core claim. In a group, members are simultaneously helpers and helped. This reciprocal process — Yalom called it mutual aid — produces learning that a one-to-one relationship cannot, because it is horizontal rather than vertical. The leader is not the only source of help; the membership is.

The social microcosm. A group is a small society in which each member eventually behaves as they do outside: the man who must control, the woman who apologises, the adolescent who deflects with humour. The group then becomes a laboratory in which those behaviours are noticed, felt, and changed in vivo.

Systemic framing. A group is a system: it has boundaries (who is in, who is out), roles, norms, a developmental history, and homeostatic mechanisms that resist change.

Everything you learned about families applies. The scapegoat in a group is playing the same structural role as the identified patient in a family.

> **PRACTICAL**
> Begin the experiential group this week. The group runs for the semester with two student co-leaders rotating. Write your first reflection on what it felt like to be a member.

Week 2 — Groups in family therapy practice

Group type	Members	Typical aims
Couples group	4–6 couples	Communication skills, shared normalisation, vicarious learning
Parents group	6–10 parents	Behaviour management, support, reducing isolation
Adolescent group	6–8	Identity, peer feedback, social skills
Men's behaviour change	8–12	Accountability, gender norms, non-violence
Recovery group	8–12	Sobriety support, family reintegration
Bereavement group	6–10	Normalising grief, meaning-making
Psychoeducational	10–20	Information plus discussion
Multi-family group	4–6 families	Family–family learning (strong evidence in psychosis and eating disorders)

Multi-family groups deserve particular attention in this programme: they combine systemic work with mutual aid, are cost-effective, and have good evidence in severe mental illness, eating disorders and adolescent conduct problems.

Week 3 — Design

Eight design decisions. Each should be justified in your proposal.

1. **Aim** — what change, for whom, measured how?
2. **Membership** — who is included, who excluded, and on what criteria?
3. **Size** — 6–10 for interactional groups; larger for psychoeducation.
4. **Duration and frequency** — 8–16 sessions weekly is typical; state the rationale.
5. **Open or closed** — closed builds cohesion; open tolerates attrition.
6. **Setting and materials** — circular seating, privacy, accessibility.
7. **Leadership** — solo or co-facilitated (co-facilitation is strongly preferred for beginners).
8. **Evaluation** — pre/post measures, attendance, satisfaction, and a follow-up.

Composition traps. Mixing victims and perpetrators of violence in one group; mixing adolescents and adults; including two people in a current conflict (e.g. two co-wives, or a

couple in the same parents' group); including a member who knows another member socially without discussing it in advance.

> **PROPOSAL (25%)**
> A 2,000-word design document: population, aims, evidence base, membership criteria, screening protocol, session-by-session outline, facilitation plan, risk protocol, evaluation plan, budget.

Week 4 — Screening, consent and preparation

Screening protects the group as much as the individual. Screen for: fit with the group's aims, current risk, ability to tolerate group feedback, concurrent individual therapy, and practical commitment.

The screening interview (30 minutes).

1. What brings you here, and what do you hope for?
2. What has helped before? What has not?
3. Have you been in a group before? How was it?
4. How do you usually handle conflict and criticism?
5. Is there anyone in the group you know? (Show the member list where appropriate.)
6. Are you currently safe? (Risk screen.)
7. Are you willing to keep confidentiality, knowing it cannot be guaranteed?

Consent must be specific. Explain that confidentiality is a group norm but not a guarantee; that members may be upset by others; that the leaders will break confidentiality only in the circumstances described; and that attendance and punctuality are part of the contract.

Preparation improves outcomes. Tell the member what to expect, what will be asked of them in the first session, and how to introduce themselves. Pre-group preparation reduces drop-out measurably.

> **PRACTICAL**
> Screening role-play with a volunteer who is ambivalent and has been pressured to attend. Practise: exploring ambivalence, explaining the frame, and declining a poor fit without rejecting the person.

Week 5 — Yalom's therapeutic factors

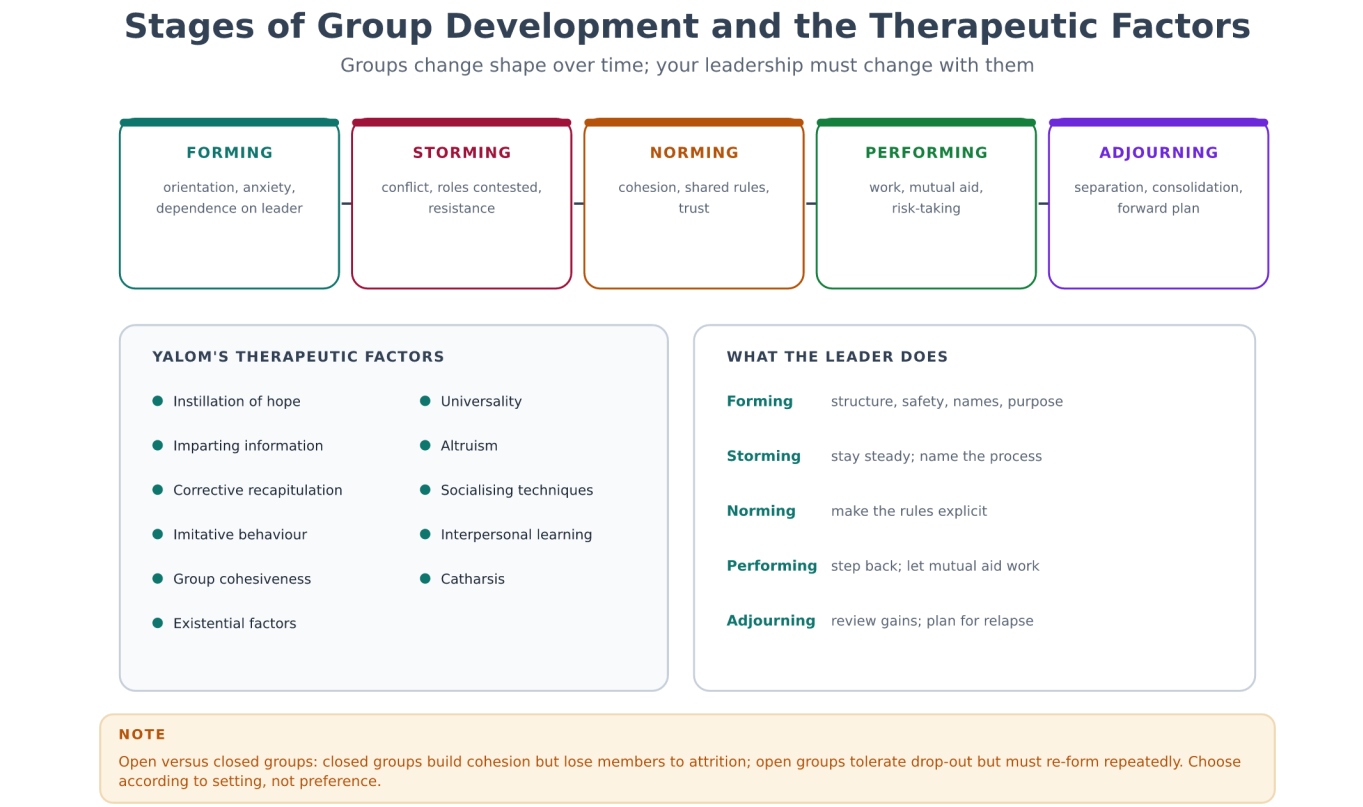


Figure 5.1 — Group stages, therapeutic factors and the leader's changing task

Factor	What it looks like	How the leader strengthens it
Instillation of hope	A member describes recovery	Ask for specifics; keep the member connected to the group
Universality	"I thought I was the only one"	Name the shared experience explicitly
Imparting information	Advice, psychoeducation	Redirect advice-giving into dialogue
Altruism	A member helps another	Point out the help; let the helper be seen
Corrective recapitulation	Old family patterns reappear	Interpret the pattern in the here-and-now
Socialising techniques	Learning social feedback	Model and reinforce it
Imitative behaviour	Modelling the leader or a member	Use yourself deliberately
Interpersonal learning	Feedback changes behaviour	Make the group the site of the experiment
Group cohesiveness	Belonging, attendance, care	Protect the frame; address absences
Catharsis	Emotional release	Contain it; link it to meaning
Existential factors	Responsibility, mortality, isolation	Allow the conversation; do not rush to technique

The beginner's error: chasing catharsis. Tears without integration produce relief for a day and no change. The sequence is: **express → understand → experiment → consolidate**.

Week 6 — Stages I: forming and storming

Forming. Anxiety, politeness, dependence on the leader, search for structure. Members ask: "What are we supposed to do? Is this allowed?"

Leader task: provide structure and safety, model disclosure at an appropriate level, establish norms (punctuality, confidentiality, one person at a time, no advice without asking), and name the anxiety.

Script: "Most groups start quietly. Part of what we will do is notice what we are all avoiding."

Storming. Conflict, role competition, resistance, criticism of the leader, subgrouping. This is not failure; it is the group testing whether it can survive difference.

Leader task: stay steady, do not defend, name the process, and let the group negotiate. Avoid solving it for them.

Script: "There is something happening between the two of you that the rest of us are watching. Can we look at it together rather than settle it?"

> **PRACTICAL**
> Process observation. Two observers code the session for: who speaks, who is interrupted, who is silent, and three moments of process (as opposed to content). Feed back to the group in the last ten minutes.

Week 7 — Stages II: norming, performing, adjourning

Norming. Cohesion, shared rules, mutual trust, humour. Attendance stabilises.

Leader task: consolidate the norms explicitly; use the cohesion to do deeper work.

Performing. The productive stage: mutual aid, confrontation with care, risk-taking, insight.

Leader task: step back. Your best sessions are the ones in which you spoke least and the group worked. Redirect all content to the members: "What does the group think?"

Adjourning. Ending, loss, consolidation, transfer to life outside.

Leader task: review gains, name the loss, have each member say what they gave and received, and make a forward plan. Do not let the group end by attrition.

Termination script: "We have four sessions left. I want us to use them for two things: what each of you has learned, and what each of you will do when the group is not here."

Week 8 — The here-and-now

Two layers. Content is what the group is talking about; process is what is happening between the people while they talk about it. Groups change through process.

The four-step process commentary.

1. **Notice** something concrete: "Every time Ada speaks about her marriage, Femi looks at the clock."
2. **Name it to the group, not about the person:** "I notice something happening here."
3. **Ask the group to look:** "Does anyone else notice it? What do you think it is about?"
4. **Return it to the relationship:** "Femi, what is happening for you when Ada speaks?"

Timing rules. Do not process-comment in the first two sessions (no safety yet). Do not do it to demonstrate skill. Do it when the same pattern has occurred at least twice.

Hazards. Process commentary can feel like exposure. Use the group as the buffer ("does anyone else…"), avoid naming individuals as specimens, and never interpret someone's process before you have checked it with them.

> **DRILL**
> Ten minutes of "process-only": the group may only speak about what is happening here, not about events outside the room. Debrief how uncomfortable it was.

Week 9 — Difficult members

Member type	Behaviour	What it protects	Intervention
Monopoliser	Talks at length; fills silence	Anxiety, fear of invisibility	"I want to bring others in; you may find that hard." Set a turn-taking norm
Silent member	Says nothing; attends	Fear of exposure, cultural deference	Low-demand invitation; do not force; check in privately
Hostile/devaluing	Attacks the leader or a member	Shame, previous betrayal	Name impact; set the boundary; explore what it protects
Help-rejecting complainer	Asks for help then rejects it	Control, fear of change	Move from advice to the function: "What would it cost you to accept help?"
Group mascot	Defuses everything with humour	Avoids grief	"Your jokes are welcome. I also want to know what is underneath this one."
Scapegoat	Becomes the group's problem	Carries disowned material	Protect; interpret the group's function in selecting them
Subgroup	Two or three form a bloc	Safety in numbers	Bring it into the open; do not let it be secret

The principle. Every difficult behaviour is a solution the member has found for a problem. Your task is to understand the problem, not to eliminate the behaviour by force.

Week 10 — Conflict, scapegoating and subgrouping

Escalation in groups is faster than in dyads because of contagion. Interrupt earlier than you would in a family session.

The scapegoat sequence (learn to spot it): a member is different → the group's anxiety attaches to them → the group pressures them → the leader is invited to condemn them → if the leader agrees, the group learns that difference is dangerous.

Leader intervention: name the process, protect the member, and give the group the work: "I notice we have all become very interested in what Femi is doing wrong. What is it about him that makes us comfortable focusing there?"

Subgroups. Two members who meet outside, or a bloc that votes together. Name it without accusation: "I am aware that some of you talk outside. That is allowed, and it affects what happens here. What shall we do about it?"

Week 11 — Co-facilitation

Why co-facilitate. Two sets of eyes, two relational styles, a model of a working relationship, and protection when one of you is triggered.

Division of labour — decide before every session.

- Who opens and closes?
- Who tracks process, who tracks content?
- Who handles risk or a member leaving the room?
- How will you signal a disagreement in the moment?

Managing disagreement in front of the group. Never contradict your co-leader without acknowledging it. Options: (a) defer and discuss after; (b) name the difference openly and model resolution: "I see it differently, and I want the group to hear both." Naming it well is a gift; naming it badly is a rupture.

The classic co-therapy failure: the couple dynamic. Two co-leaders who replicate a parent–child or pursue–withdraw pattern in front of the group. The group will notice and may act it out. Take it to supervision immediately.

> **PRACTICAL**
> Co-therapy pairs plan a session in ten minutes using the division-of-labour checklist, then facilitate with the observed group. The observer records every moment the pair were out of sync.

Week 12 — Observed co-facilitation

Thirty minutes of an experiential group, co-facilitated, assessed against: safety and structure; use of process; inclusion of silent members; containment of difficult behaviour; co-leader coordination; and closing.

Week 13 — Couples and parents groups

Couples group. Four to six couples. Strengths: normalisation ("other couples have this too"), vicarious learning, and reduced shame. Risks: public conflict, competition, and one partner feeling ganged up on.

Structure: 15 minutes check-in (one sentence per person), 20 minutes teaching, 30 minutes exercise (couples work together while leaders circulate), 15 minutes group debrief. Never ask a couple to do a deep vulnerability exercise in front of others before they have done it alone with you.

Parents group. Strengths: reducing isolation, shared problem-solving. Risks: one parent becoming the group's expert, and advice-giving replacing reflection.

Structure: check-in, one skill, one application, one commitment. Keep advice horizontal: "Who has tried something that worked?"

Week 14 — Ethics and boundaries in groups

Issue	Standard
Confidentiality	Explain it is a norm, not a guarantee; revisit when a member leaves
Screening	Never skip it; document it
Dual relationships	Members who know each other; leaders who know members socially
Touch	Discourage between members; use only with explicit norms
Out-of-group contact	Agree a policy; do not become a member's confidant
Recordings	Separate consent from each member; a member may withdraw at any time
Risk disclosure	Leaders follow their duty of care; tell the group if confidentiality must be broken

The most common ethical complaint in groups is not dramatic: it is a member learning that something they said was repeated outside. Prevention: explicit norms, a written agreement, and a leader who models discretion.

Week 15 — Evaluation and endings

Minimum evaluation package. Attendance and drop-out (with reasons), a pre/post measure of the target problem, a session-by-session satisfaction rating, and a follow-up at 8–12 weeks.

Writing up. Report the design, the actual membership, the adaptations you made, the outcome and the limitations. A well-reported small group is a contribution; an unreported one is a private experience.

Glossary

Cohesion — the group's sense of belonging and mutual commitment; a precondition for productive work. **Group-as-a-whole** — the group considered as a single system with its own dynamics. **Here-and-now** — the immediate interpersonal process in the room, as distinct from outside content. **Mutual aid** — the reciprocal helping that occurs between members. **Process** — the relational layer of communication, as distinct from content. **Scapegoating** — the group's displacement of anxiety onto one member. **Social microcosm** — the group as a small society in which members' characteristic patterns appear. **Subgrouping** — the formation of alliances within the group. **Therapeutic factor** — a mechanism of change identified by Yalom (e.g. universality, cohesiveness).

Self-assessment

Multiple choice

1. Mutual aid refers to: (a) the leader helping members (b) members helping each other (c) financial support (d) group cohesion

2. Scapegoating in a group usually functions to: (a) help the scapegoat (b) absorb the group's anxiety (c) improve cohesion (d) test the leader

3. Pre-group preparation: (a) is optional (b) reduces drop-out and improves outcome (c) increases anxiety (d) is only for research

4. The leader's main task in the performing stage is to: (a) direct more (b) step back and let mutual aid work (c) introduce new rules (d) end the group

5. "Here-and-now" work means: (a) discussing childhood (b) working with immediate in-room process (c) using mindfulness (d) homework review

6. Confidentiality in groups is: (a) guaranteed (b) a norm that cannot be guaranteed (c) irrelevant (d) the leader's legal duty only

7. A closed group: (a) admits new members (b) has fixed membership (c) has no rules (d) is always shorter

8. Catharsis without integration typically produces: (a) lasting change (b) temporary relief (c) increased cohesion (d) no effect at all

9. The best composition decision when two members are in an active conflict is to: (a) include both to work it out (b) avoid placing them in the same group (c) seat them together (d) ask them to sign a contract

10. Parallel process in co-facilitation means: (a) the leaders take turns (b) the leaders' relationship replicates a family pattern (c) members copy the leaders (d) two groups run at once

Answer key

1 b · 2 b · 3 b · 4 b · 5 b · 6 b · 7 b · 8 b · 9 b · 10 b

Short answer

1. List five design decisions and the rationale for each.
2. Name five therapeutic factors and one leader behaviour that strengthens each.
3. Give the four steps of process commentary with an example.
4. How would you intervene with a monopoliser without shaming them?
5. What are the four elements of a group evaluation plan?

Essay (choose one, 1,800 words)

1. Design a couples group for eight distressed couples, justifying every design decision.
2. "The group is the treatment, not the leader." Discuss with reference to mutual aid and process.
3. Analyse a difficult moment in your experiential group and set out what you would do differently as leader.

Further reading

- Yalom, I. D., & Leszcz, M. (2020). The Theory and Practice of Group Psychotherapy (6th ed.). Basic Books.
- Bion, W. R. (1961). Experiences in Groups. Tavistock.
- Burlingame, G. M., Strauss, B., & Joyce, A. S. (2013). Change mechanisms and effectiveness of small group treatments. In Bergin and Garfield's Handbook.
- Asen, E., & Scholz, M. (2010). Multi-Family Therapy: Concepts and Techniques. Routledge.

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The books are designed to be read straight through before the semester begins, revisited week by week, and used again in revision. Cross-references point to the other volumes where a topic is treated in more depth.

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A note on the case material

All families, couples and individuals in this book are fictional composites. They are written to teach pattern recognition, not to describe any real person. Nothing here replaces supervised clinical practice, professional registration, legal advice or medical care; legal content is orientation only and must be checked against the law of the relevant jurisdiction.

The lecture library

Twenty-eight self-contained course books carry the taught programme: eight in Semester One, seven in Semester Two, seven in Semester Three including the thesis, and six electives. The volume you are holding is marked in the margin.

Code	Course	Semester	Units
MFT 701	Fundamentals of Marriage and Family Therapy	One	1.71
MFT 702	Theories of Family Therapy	One	1.71
MFT 703	Human Development Across the Lifespan	One	1.71
MFT 704	Ethical and Legal Issues in Therapy	One	1.71
MFT 705	Research Methods in Marriage and Family Therapy	One	1.71
MFT 706	Clinical Practice and Techniques	One	1.71
MFT 707	Assessment and Diagnosis in Family Therapy	One	1.71
MFT 708	Cultural Competence in Therapy	One	1.71
MFT 709	Group Therapy Techniques	Two	1.71
MFT 710	**Couples Therapy**	Two	1.71
MFT 711	Child and Adolescent Therapy	Two	1.71
MFT 712	Crisis Intervention Strategies	Two	1.71
MFT 713	Substance Abuse and Family Dynamics	Two	1.71
MFT 714	Trauma-Informed Care	Two	1.71
MFT 715	Family Systems Theory	Two	1.71
MFT 716	Communication Skills in Therapy	Three	1.71
MFT 717	Psychopathology in Families	Three	1.71
MFT 718	Intimacy and Relationship Dynamics	Three	1.71
MFT 719	Family Life Cycle and Transitions	Three	1.71
MFT 720	Supervision and Consultation in Therapy	Three	1.71
MFT 721	Research Methodology	Three	1.71
MFT 722	Professional Development and Self-Care	Electives	1.71
MFT 723	Special Populations in Therapy	Electives	1.71
MFT 724	Family Law and Advocacy	Electives	1.71
MFT 725	Integrative Approaches in Therapy	Electives	1.71
MFT 726	Neurobiology of Relationships	Electives	1.71
MFT 727	Play Therapy Techniques	Electives	1.71
MFT 770	Master Thesis	Three	1.71

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Course leader's summary. Couple work is the largest single referral stream in most family therapy practices and the most demanding therapeutically: two people, both of whom want you to agree with them. This course integrates attachment-based (EFT), behavioural (TBCT/IBCT) and systemic approaches, and takes infidelity, violence, separation and cultural context seriously.

How to use this book

Couple work is where the therapist's own relationship history is most easily activated. The reflective tasks in this book are not optional extras; they are the difference between a clinician who can hold two perspectives at once and one who quietly takes sides. Do them.

1. Course description

The course covers: assessment of the couple (including individual sessions and violence screening), the major evidence-based models (EFT and behavioural couple therapy), the Gottman research programme, communication training, infidelity and betrayal trauma, sexual problems, separation and divorce work, and cultural and religious context.

Three frameworks are integrated: **attachment** (why the bond matters), **behaviour** (what maintains the pattern), and **system** (how the pattern organises itself).

2. Learning outcomes

#	Outcome	Assessed
LO1	Conduct a couple assessment safely, including IPV screening and individual sessions	Observed assessment
LO2	Formulate the couple's negative cycle in attachment terms	Case assignment
LO3	Deliver core EFT and behavioural interventions under supervision	Observed sessions
LO4	Work with infidelity, including disclosure, meaning and rebuilding	Case study
LO5	Address sexual concerns within the couple frame	Exam
LO6	Manage separation, divorce and stepfamily transitions ethically	Case study
LO7	Adapt couple work to cultural, religious and extended-family context	Viva

3. Assessment

Component	Weight	Due
Couple assessment observed (40 min)	25%	Week 7
Formulation and treatment plan (2,000 words)	25%	Week 10
Recorded or live session with analysis (30 min)	30%	Week 15

Component	Weight	Due
Examination	20%	Week 16

4. The sixteen-week plan

Week	Topic	Practical
1	What couples want and what therapy offers; the evidence	Map a couple's goals
2	Assessment I: structure, history, and the couple narrative	Intake role-play
3	Assessment II: individual sessions, IPV screening, secrets	Screening rehearsal
4	The negative cycle: pursue–withdraw and its variants	Cycle mapping
5	Attachment theory applied to couples	Attachment interview
6	EFT I: tracking and de-escalation	Enactment drill
7	**Observed couple assessment**	Assessed
8	EFT II: restructuring and softening events	Softening rehearsal
9	Behavioural couple therapy: exchange and communication	Speaker–listener
10	**Formulation and plan due**; the Gottman programme	Four horsemen practice
11	Infidelity and betrayal trauma	Disclosure planning
12	Sex and intimacy in couple therapy	Sensate focus design
13	Violence, control and when couple therapy is contraindicated	Safety planning
14	Separation, divorce, co-parenting and stepfamilies	Co-parenting mediation
15	Culture, faith and family-of-origin in couple work	Cultural adaptation
16	Consolidation, relapse prevention and ending	**Session analysis due**

Week 1 — What couples want, and what therapy offers

Couples arrive with a presenting complaint (money, sex, in-laws, phone) and an underlying bond question ("are you there for me?"). The complaint is the doorway; the bond is the room.

The evidence, briefly. Couple therapy is effective for relationship distress with moderate-to-large effects. Behavioural couple therapy, cognitive-behavioural couple therapy, and emotionally focused therapy have the strongest evidence base. Emotionally focused therapy has the most consistent evidence for durability, including in high-distress couples.

What predicts a good outcome. Early change, motivation of both partners, shared definition of the problem, low coercive control, and — consistently — the quality of the shared therapeutic alliance.

What predicts failure. Coercive control, undisclosed affairs that surface mid-treatment, untreated addiction, severe individual psychopathology, and one partner who has already decided to leave but has not said so.

> **REFLECT**
> What did your own family teach you about conflict, repair, and what a marriage is for? Write 400 words. Keep it; you will return to it in Week 13.

Week 2 — Assessment I

Structure of a couple assessment (2–3 sessions).

1. **Joint session:** both partners' accounts, the history of the relationship, the current pattern, and the goals.
2. **Individual sessions:** one with each partner — risk screen, secrets policy, individual history, ambivalence, and "is there anything you would not say in front of your partner?"
3. **Feedback session:** formulation, plan, and the agreement.

What to assess.

Domain	Questions
Commitment	"Is there any part of you that has already decided to leave?"
History	How did you meet? What drew you? What was good in the early years?
Current pattern	"Walk me through last Tuesday's argument, minute by minute."
Attempted solutions	What have you tried? What happened?
Family of origin	Genogram of both families; how each family handled conflict, affection, money
Context	Work, money, migration, in-laws, children, faith, health
Strengths	What still works? What do you still enjoy?
Risk	Violence, coercive control, suicide, substance use, child safety

Ask about the good. Couples in distress systematically under-report positives; a careful strengths inventory is both assessment and intervention.

Week 3 — Individual sessions, IPV screening and secrets

Always see each partner alone at least once. This is where safety is disclosed.

Screening questions (ask directly, in private).

- "Have you ever felt afraid of your partner?"
- "Has your partner ever hit, pushed, slapped, or thrown something at you?"

- "Has your partner ever forced or pressured you sexually?"
- "Does your partner control your money, your phone, or who you see?"
- "If you went home tonight and told him you had said all this, what would happen?"

If coercive control is present: stop conjoint work; safety-plan individually; refer to specialist services; document. Conjoint work can increase danger and can silence the victim in the room.

Secrets policy. State it in the first joint session: "If one of you tells me something individually that affects our work together, I will not keep it as a secret. I will help you decide how and when to tell your partner, and if you cannot, I will ask us all to talk about it."

Ambivalence. One partner may already be planning to leave. Ask: "Where are you on a scale from 'I am fully committed to working on this' to 'I have already decided'?" The answer changes the treatment.

Week 4 — The negative cycle

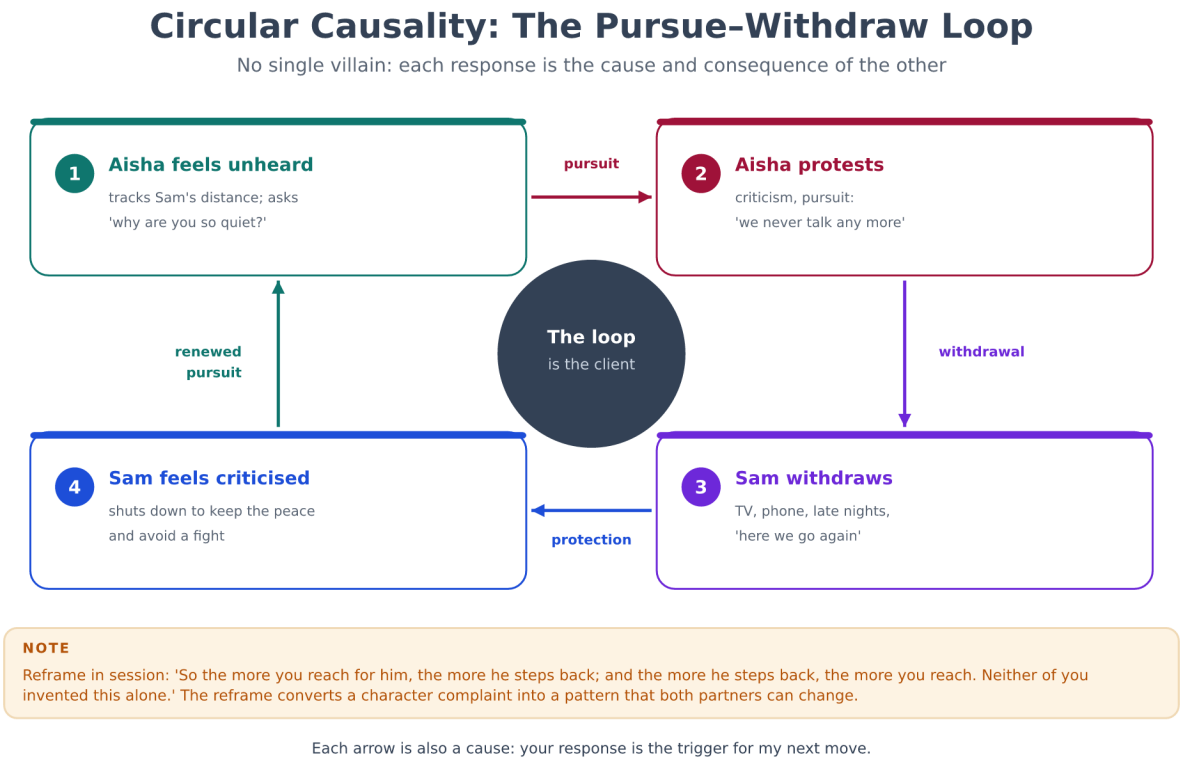


Figure 4.1 — Circular causality: the pursue–withdraw loop

The three common cycles.

1. **Pursue–withdraw.** One escalates to get a response; the other shuts down to keep the peace. The pursuing increases, the withdrawal deepens.

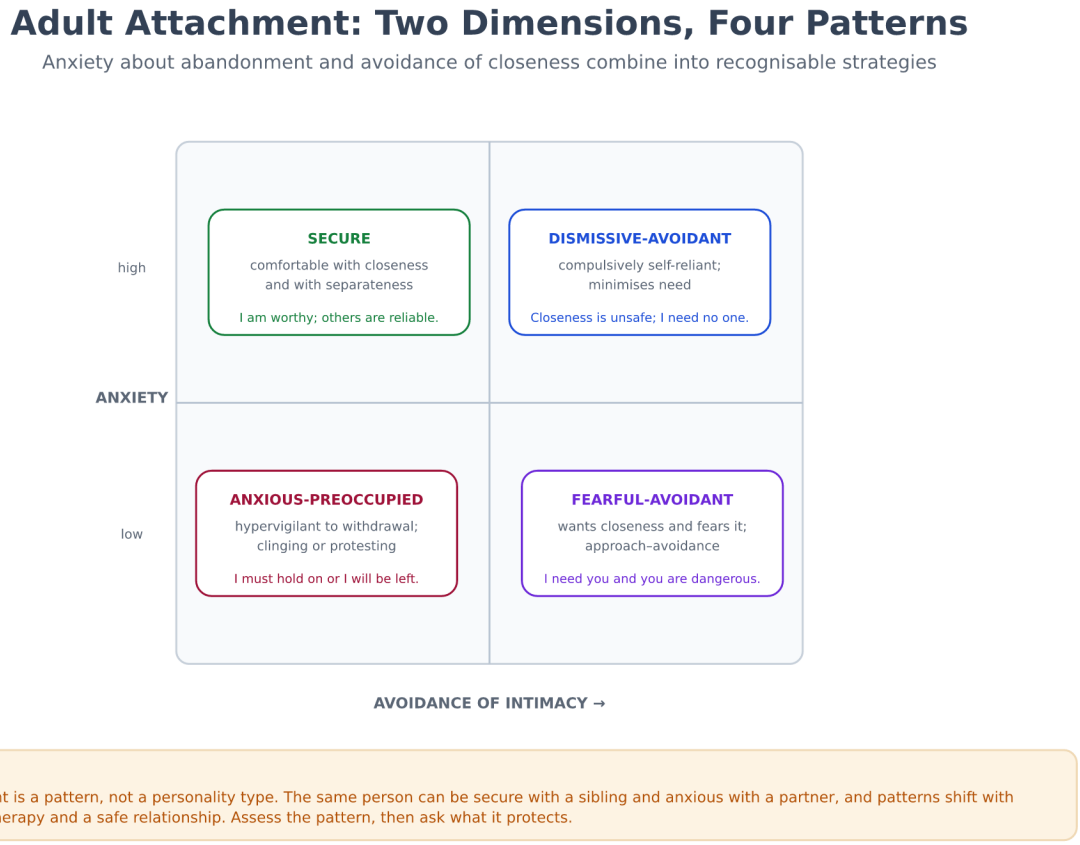
2. **Attack–defend.** Both escalate; complaint is met with counter-complaint; the volume rises and the content becomes irrelevant.
3. **Mutual withdrawal.** Both avoid; the relationship becomes parallel and polite; loneliness without conflict — often the hardest to treat because there is no presenting crisis.

Mapping the cycle in session. Ask each partner for the last three arguments and write the sequence on paper as a loop. Then read it back and ask: "Is this recognisable?" The moment of recognition — often accompanied by a laugh or a sigh — is the therapeutic turn.

The reframe. "The problem is not that you are angry, or that you are cold. The problem is the loop between you, and the loop has a life of its own."

> **PRACTICAL**
> With a role-played couple, map the cycle on a whiteboard in 15 minutes and get both partners to agree with the drawing. The agreement, not the drawing, is the skill.

Week 5 — Attachment applied to couples



> **NOTE**
> Attachment is a pattern, not a personality type. The same person can be secure with a sibling and anxious with a partner, and patterns shift with context, therapy and a safe relationship. Assess the pattern, then ask what it protects.

Figure 5.1 — Adult attachment: two dimensions and four patterns

Core idea. Adult romantic love is an attachment process. Partners monitor accessibility and responsiveness; when the bond feels threatened, protest behaviours appear (criticism, pursuit, withdrawal, numbing), and those protest behaviours are usually the opposite of the need they express.

Two dimensions. Anxiety (about abandonment) and avoidance (of intimacy). Their combination produces four patterns, each with a characteristic strategy and core belief (see figure).

Clinical use.

- Anxious partner: the protest is a request for reassurance. Help them ask for it directly rather than through criticism.
- Avoidant partner: withdrawal is a protection against overwhelm and shame. Help them stay present for longer and return rather than disappear.
- Both: the cycle is an attachment problem, not a character problem.

The attachment question to hold in your head. "What is this person asking for, and what are they doing instead?"

Week 6 — EFT I: tracking and de-escalation

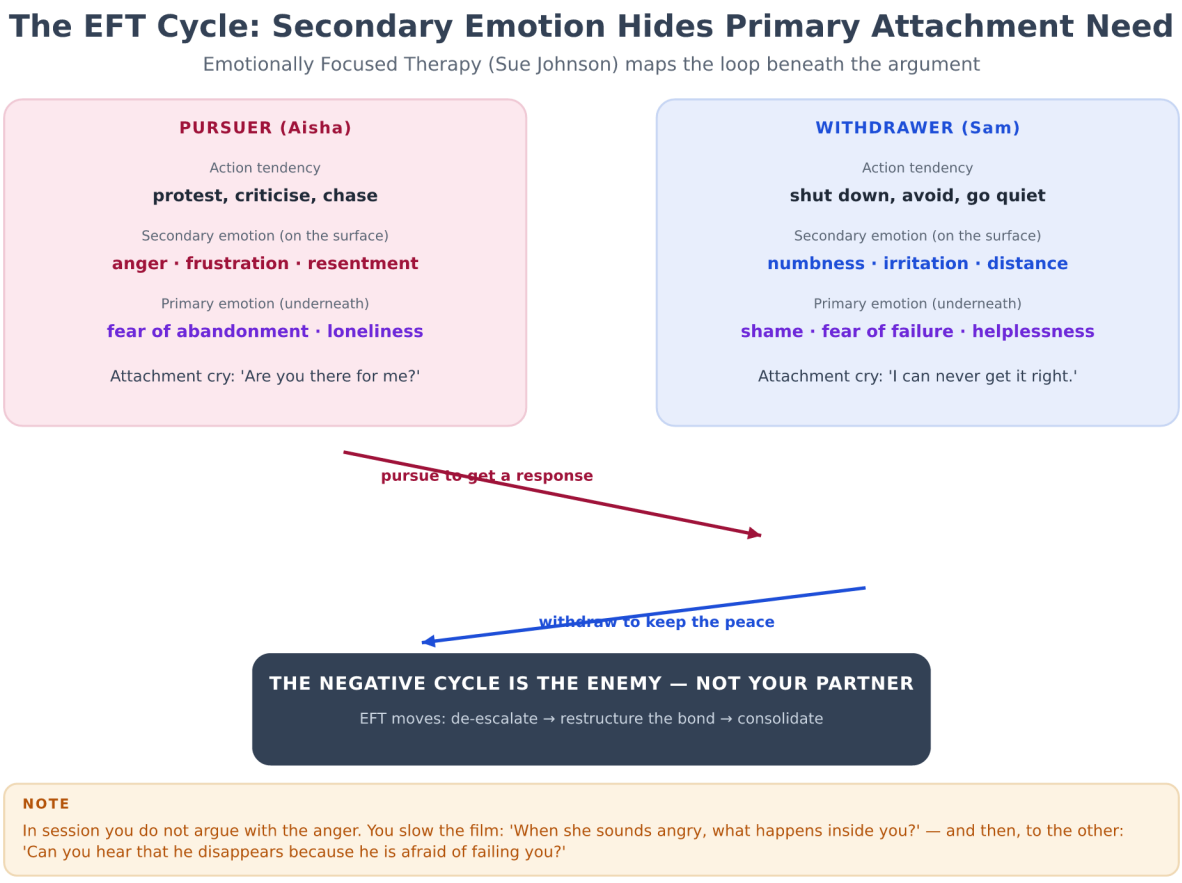


Figure 6.1 — The EFT cycle: secondary emotion and primary attachment need

Stage 1 (de-escalation, sessions 1–8). The goal is that both partners can describe the cycle and see it as the enemy.

Four moves.

1. **Track the cycle** in specific incidents. Slow the film; ask for bodily experience.
2. **Access primary emotion.** "Under the anger — is there something like fear?" Name it tentatively; let them correct you.
3. **Reframe the problem as the cycle.** "Neither of you is the problem. This dance is."
4. **Name the attachment cry.** "So when you say 'we never talk', what you mean is 'I miss you and I am afraid you have gone'."

A worked enactment (Week 6 lab).

> **THERAPIST (TO AISHA):**
> When you get loud, what happens inside your body? **Aisha:** My chest. Like I cannot breathe until he answers. **Therapist:** And if you did not get loud — what would he see? **Aisha:** (pause) That I am frightened. **Therapist:** Can you tell him that? Look at him. "I get loud because…" **Aisha:** I get loud because I think you have already left. **Therapist (to Sam):** Sam, stay with that. What is happening for you hearing it?

Hazard. Primary emotion must be reached, not demanded. If you push a client to feel something in front of you before safety, you will get a performance or a shutdown.

Week 7 — Observed couple assessment

Forty minutes: contract, both accounts, cycle map, risk screen (in the individual sessions if scheduling permits), strengths, and feedback. Assessed on safety, structure, balance (did each partner get equal airtime?), and formulation quality.

Week 8 — EFT II: restructuring

Stage 2 (sessions 9–20). Withdrawal re-engagement; pursuer softening; enactment of new responses.

Softening event — the classic sequence.

1. The pursuer accesses the primary attachment fear and expresses it non-accusatorially.
2. The withdrawer hears it, stays present, and responds with care.
3. The pursuer risks reaching again from a position of vulnerability rather than protest.
4. Both experience a new pattern in the room.

Therapist tasks. Keep the withdrawer engaged ("stay with me, this is the hard part"); block the pursuer's escalation without silencing the need; and slow everything down.

Stage 3 (consolidation). Integrate the new pattern, problem-solve practical issues, and rehearse recovery from relapse: "When the cycle comes back — and it will — what will you do in the first hour?"

Week 9 — Behavioural couple therapy

The behavioural view. Distress is maintained by (a) a low rate of positive exchange, (b) poor communication under conflict, and (c) attributions that make neutral behaviour hostile.

Core interventions.

1. **Behavioural exchange.** Each partner lists 15–20 small caring behaviours; select three each; do them daily without being asked. Not transactional bookkeeping — an increase in the density of positive behaviour.
2. **Communication training.** Speaker–listener with a floor card, then problemsolving.
3. **Attribution retraining.** "When he is late, what do you assume? What else could be true?"
4. **Acceptance work (IBCT).** Some differences will not change; the work is to soften the emotional reactivity to them through empathic joining and tolerance building.

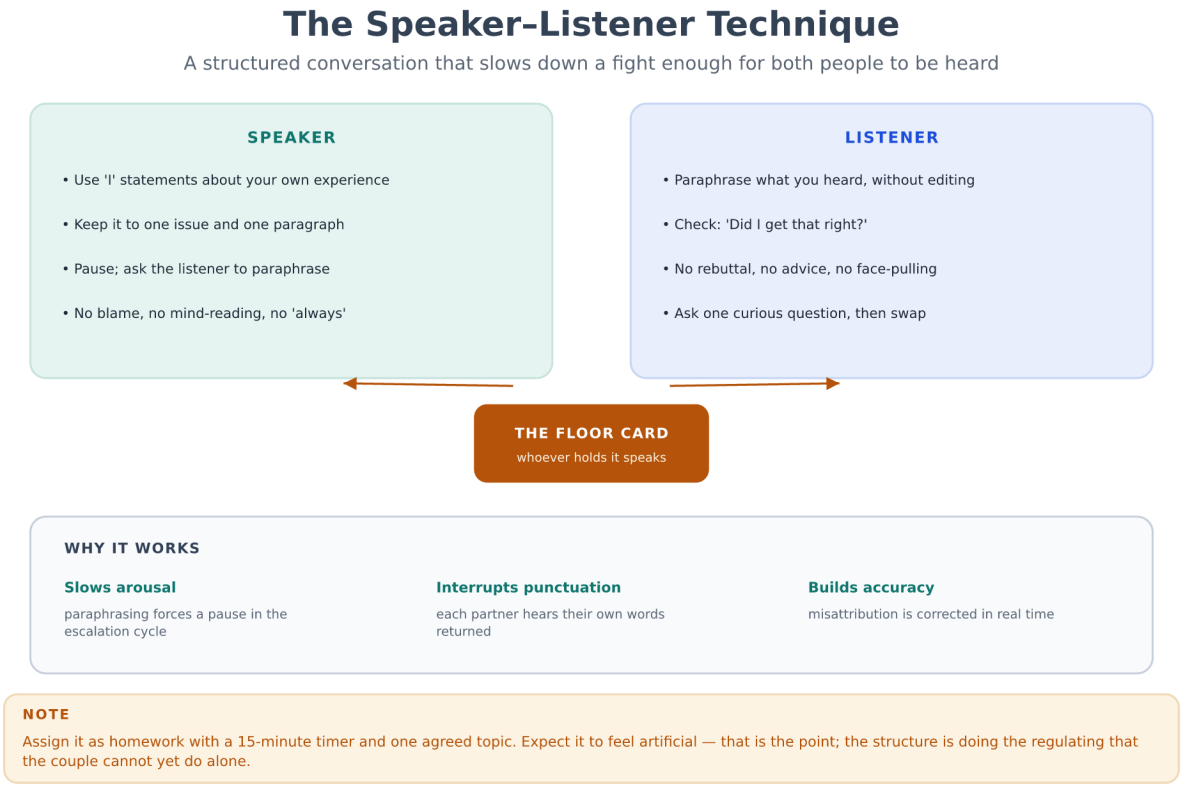
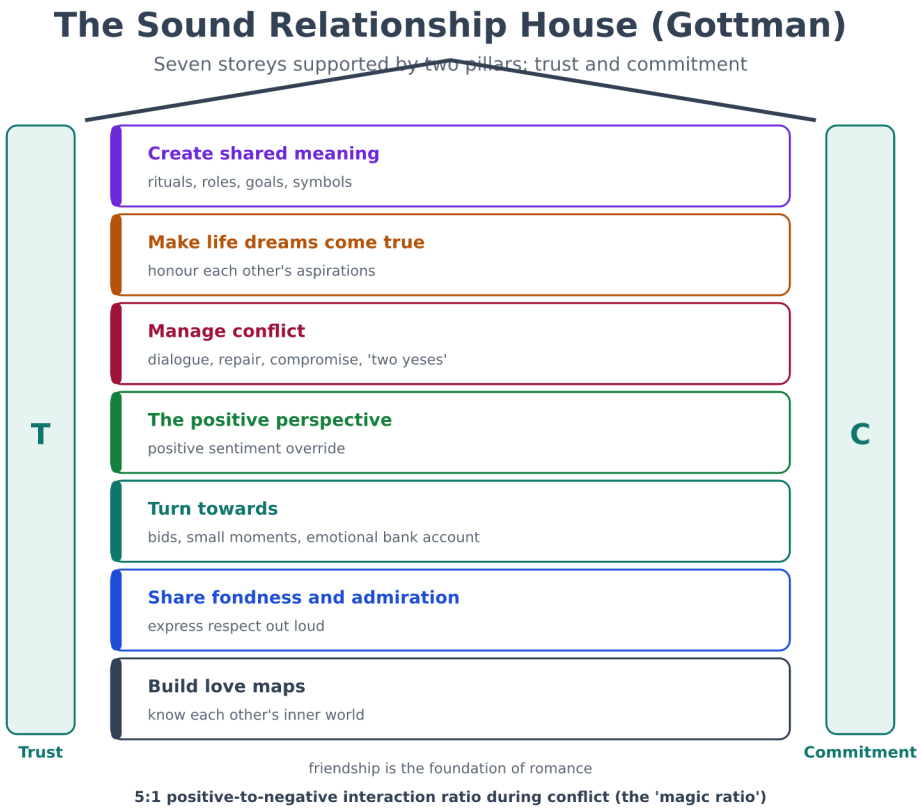


Figure 9.1 — The speaker–listener technique

Sequencing. Positives first (exchange), then communication, then problem-solving. Couples cannot solve problems they cannot discuss.

Week 10 — The Gottman research programme



NOTE

Adaptation caution: Gottman's norms were derived largely from U.S. samples. Use the structure of the house as a clinical map; do not treat the 5:1 ratio as a universal law.

Figure 10.1 — The Sound Relationship House

Findings worth using.

- **The four horsemen** — criticism, contempt, defensiveness, stonewalling — predict dissolution (Figure 14). Contempt is the strongest predictor.
- **The 5:1 ratio** of positive to negative interaction during conflict.
- **Bids for connection** — small moments of "turning towards" build the emotional bank account.
- **Love maps** — detailed knowledge of the partner's inner world.
- **Repair attempts** — the capacity to de-escalate mid-conflict is more predictive than the absence of conflict.
- **Physiological flooding** — above roughly 100 bpm, people cannot listen; a break is a skill, not an escape.

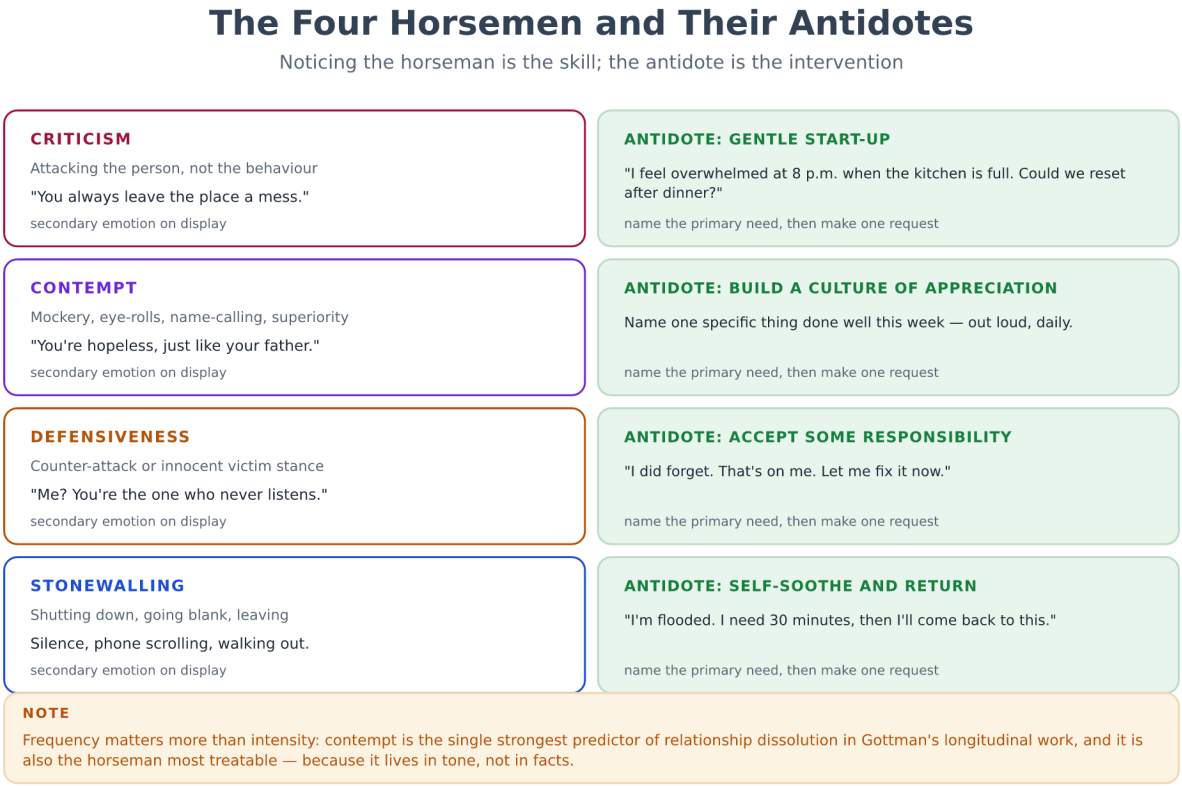


Figure 10.2 — The four horsemen and their antidotes

Adaptation caution. The norms come largely from US samples; treat the structure as a clinical map, not as a universal law. A couple who argue loudly and repair well is not in trouble because of the volume.

Week 11 — Infidelity and betrayal trauma

Two clinical events, often confused.

1. **The disclosure** — an acute crisis requiring containment, safety and structure.
2. **The betrayal trauma** — an injury to the betrayed partner's capacity to trust and to make sense of reality, with symptoms resembling PTSD (intrusion, hypervigilance, dissociation).

Phase 1: stabilise (weeks 1–4).

- Stop the affair (or the contact) — this is usually a precondition, not a negotiation.
- Establish safety: no violence, no coercion, agreed boundaries on phone and whereabouts.
- Contain the disclosure: the betrayed partner has a right to the facts, not to every detail; agree what will be asked and answered.
- Address STI testing and any financial/legal exposure.
- Individual sessions to assess each partner's capacity to continue.

Phase 2: meaning (weeks 4–12).

- What did the affair do for the unfaithful partner? (Not "why did you do it?" but "what was it in service of?")
- What was missing that could have been spoken? (Careful: explanation is not justification.)
- What does the betrayed partner need in order to consider rebuilding — transparency, remorse, time, consistency?

Phase 3: rebuilding or separating. Not every couple should stay together. Your task is to help them decide with full information, not to preserve the marriage at all costs.

> **CAUTION**
> Do not accept a couple for affair work if the unfaithful partner will not end the affair and will not disclose anything; you will be running a therapy that cannot progress and colluding in a lie.

Week 12 — Sex and intimacy

Start with permission, then information, then specific suggestions (Annon's PLISSIT model).

Assessment. Desire, arousal, orgasm, pain; frequency and satisfaction for each partner; the last time it was good; what changed; medical factors (medication, illness, menopause, postpartum); the couple's beliefs; and the meaning of sex in the relationship.

Common presentations and the couple-level formulation.

Presentation	Individual factor	Relational factor
Low desire in one partner	Fatigue, medication, depression, pain	Pursuit and pressure; unresolved resentment
Erectile difficulty	Vascular, hormonal, anxiety	Performance focus; fear of judgement
Painful intercourse	Medical cause must be excluded	Fear, poor communication, coercion
Mismatched desire	Cycles differ normally	Negotiation failure; rejection sensitivity

Intervention basics. Psychoeducation (desire discrepancy is the most common sexual complaint, not a sign of incompatibility); sensate focus (graded, non-goal-oriented touch); scheduling; reducing performance pressure; addressing the relational context; and medical referral when indicated.

Cultural and religious framing. Sexual expectations are among the most culturally loaded topics you will discuss. Ask what each partner was taught, what they believe is permitted, and whether they can say no to each other. Consent within marriage is consent.

Week 13 — Violence, control and contraindications

The distinction that matters (from MFT 704):

Situational couple violence	Coercive control
Mutual, escalating under stress	One-directional domination
No fear outside conflict	Chronic fear, surveillance, isolation
Couple therapy possible after assessment	Couple therapy contraindicated
Safety plan for both	Specialist advocacy for the victim

If a couple presents with violence. See each partner individually first. Ask about fear, injury, sexual coercion, threats, and escalation. Do not use conjoint sessions to "work through" violence; do not ask the victim to say in front of the perpetrator what he did, unless she chooses and it is safe.

REFLECT — Return to your Week 1 reflection. Where in this material are you most likely to lose balance — with the victim, with the perpetrator, with the unfaithful partner, with the one who wants to leave?

Week 14 — Separation, divorce and stepfamilies

Discernment counselling. Some couples are not deciding whether to repair the relationship but whether to end it. A structured, time-limited process (about 4–6 sessions) helps a couple reach a clear decision rather than drifting. Both outcomes — a renewed commitment or a respectful ending — are successes.

When the decision is to separate.

- Help the couple tell the children together, with a simple, blame-free message.
- Refer for legal advice on custody, maintenance, property and any customary or religious processes.
- Address co-parenting: the relationship continues as a parenting partnership.
- Manage the grief: divorce is a loss, even when it is a relief.

Stepfamilies. The central error is treating the stepfamily as a nuclear family that arrived late. It is a different structure with different tasks: the couple bond must be built before stepparent authority is imposed; children need permission to love both households; and the "insider/outsider" split (biological parent and child versus stepparent) must be named.

Week 15 — Culture, faith and family of origin

Questions to ask every couple.

- What did each of your families teach you about marriage, money, sex, and in-laws?
- Who has authority in your household, and who decided that?

- What does your faith require of each of you?
- Which of your families is closer, and what does that closeness cost the other?
- If your parents disagreed with a decision you made, what would happen?

Common cultural configurations. Bride price and its obligations; polygyny; co-residence with parents; gender-role expectations; religious courts and counselling; migration and split households; inter-ethnic and inter-faith marriages.

The rule. Do not treat the cultural pattern as the problem, and do not treat it as beyond discussion. Ask the couple what they want within it.

Week 16 — Consolidation and ending

Write the couple a one-page **relapse prevention plan**: their cycle in their own words, three early warning signs, two repair moves that work for them, and the agreement about when to return. Review the gains explicitly, name the risk of recurrence without frightening them, and end on time.

Glossary

Attachment injury — a betrayal or abandonment that damages trust and requires explicit repair. **Betrayal trauma** — trauma-like symptoms following the discovery of a partner's deception. **Bids for connection** — small attempts to gain a partner's attention or affection. **Coercive control** — a pattern of domination and isolation, distinct from situational conflict. **Negative cycle** — the recurrent pattern of interaction that organises couple distress. **Pursue–withdraw** — a cycle in which one partner escalates while the other distances. **Softening** — in EFT, the pursuer's shift from protest to vulnerable expression. **Sentiment override** — the tendency for the relationship's overall tone to colour the interpretation of specific events. **Sensate focus** — graded, non-goal-oriented touch exercises used in sex therapy.

Self-assessment

Multiple choice

1. The first step in assessing a couple is: (a) communication training (b) assessment including individual sessions and IPV screening (c) homework (d) diagnosis

2. In EFT, criticism is usually understood as: (a) abuse (b) protest at perceived abandonment (c) personality (d) a communication deficit

3. Couple therapy is contraindicated when: (a) the couple argues (b) there is coercive control (c) there is low desire (d) one partner is depressed

4. The strongest predictor of relationship dissolution in Gottman's research is: (a) conflict frequency (b) contempt (c) low income (d) different interests

5. A 5:1 ratio refers to: (a) sessions per month (b) positive to negative interactions during conflict (c) partners to therapist (d) years of marriage

6. In affair work, the precondition for progress is usually: (a) forgiveness (b) ending the affair and establishing transparency (c) a legal separation (d) a new sexual contract

7. "Softening" in EFT refers to: (a) lowering the volume (b) the pursuer expressing primary attachment emotion (c) the therapist's tone (d) reducing homework

8. Physiological flooding is significant because: (a) it indicates anger (b) above a threshold, listening becomes physiologically difficult (c) it predicts divorce (d) it requires medication

9. A key task in stepfamilies is to: (a) impose stepparent authority quickly (b) build the couple bond before stepparent authority (c) treat all children equally in discipline (d) avoid the ex-partner

10. Discernment counselling helps couples to: (a) stay together (b) reach a clear decision about the relationship (c) divorce quickly (d) avoid decisions

Answer key

1 b · 2 b · 3 b · 4 b · 5 b · 6 b · 7 b · 8 b · 9 b · 10 b

Short answer

1. Give the three-session structure of a couple assessment and the purpose of each session.
2. Write your confidentiality policy for secrets in couple therapy, in six sentences.
3. Describe the pursue–withdraw cycle in attachment terms.
4. Name three contraindications to conjoint couple therapy.
5. What are the phases of infidelity work, and what happens in each?

Essay (choose one, 2,000 words)

1. Compare EFT and behavioural couple therapy for a highly distressed couple with a pursue–withdraw cycle.
2. "Infidelity is a symptom and an injury." Discuss the treatment implications.
3. Critically evaluate the cultural assumptions embedded in the concept of "differentiation" in couple therapy.

Further reading

- Johnson, S. M. (2019). Attachment Theory in Practice. Guilford.
- Gottman, J. M., & Gottman, J. S. (2015). 10 Principles for Making Marriage Work. Harmony.

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MSc Marriage and Family Therapy · Semester Two · 1.71 Units · 15 teaching weeks · Prerequisite: MFT 703, 706

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The books are designed to be read straight through before the semester begins, revisited week by week, and used again in revision. Cross-references point to the other volumes where a topic is treated in more depth.

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A note on the case material

All families, couples and individuals in this book are fictional composites. They are written to teach pattern recognition, not to describe any real person. Nothing here replaces supervised clinical practice, professional registration, legal advice or medical care; legal content is orientation only and must be checked against the law of the relevant jurisdiction.

The lecture library

Twenty-eight self-contained course books carry the taught programme: eight in Semester One, seven in Semester Two, seven in Semester Three including the thesis, and six electives. The volume you are holding is marked in the margin.

Code	Course	Semester	Units
MFT 701	Fundamentals of Marriage and Family Therapy	One	1.71
MFT 702	Theories of Family Therapy	One	1.71
MFT 703	Human Development Across the Lifespan	One	1.71
MFT 704	Ethical and Legal Issues in Therapy	One	1.71
MFT 705	Research Methods in Marriage and Family Therapy	One	1.71
MFT 706	Clinical Practice and Techniques	One	1.71
MFT 707	Assessment and Diagnosis in Family Therapy	One	1.71
MFT 708	Cultural Competence in Therapy	One	1.71
MFT 709	Group Therapy Techniques	Two	1.71
MFT 710	Couples Therapy	Two	1.71
MFT 711	**Child and Adolescent Therapy**	Two	1.71
MFT 712	Crisis Intervention Strategies	Two	1.71
MFT 713	Substance Abuse and Family Dynamics	Two	1.71
MFT 714	Trauma-Informed Care	Two	1.71
MFT 715	Family Systems Theory	Two	1.71
MFT 716	Communication Skills in Therapy	Three	1.71
MFT 717	Psychopathology in Families	Three	1.71
MFT 718	Intimacy and Relationship Dynamics	Three	1.71
MFT 719	Family Life Cycle and Transitions	Three	1.71
MFT 720	Supervision and Consultation in Therapy	Three	1.71
MFT 721	Research Methodology	Three	1.71
MFT 722	Professional Development and Self-Care	Electives	1.71
MFT 723	Special Populations in Therapy	Electives	1.71
MFT 724	Family Law and Advocacy	Electives	1.71
MFT 725	Integrative Approaches in Therapy	Electives	1.71
MFT 726	Neurobiology of Relationships	Electives	1.71
MFT 727	Play Therapy Techniques	Electives	1.71
MFT 770	Master Thesis	Three	1.71

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Course leader's summary. Children rarely refer themselves and rarely benefit from an adult conversation. This course teaches you to work with children as agents in a family system: how to speak to them, play with them, assess developmentally, and intervene through the adults who live with them.

How to use this book

Each week has a **developmental frame** (what is normal at this age), a **clinical frame** (what is different when it goes wrong), and a **practice frame** (what you actually do in the room). The play and activity protocols are written so you can use them next week, not after you qualify.

1. Course description

Child and adolescent work in family therapy is not a specialism bolted on to adult work; it is a distinct skill set. Children communicate through play, behaviour and the body; adolescents communicate through testing, silence and peers; and both live inside systems that can either support or undermine change.

The course covers: developmental assessment, engaging children and adolescents, family-based interventions, common presentations (anxiety, behaviour problems, school refusal, trauma, self-harm, eating difficulties), divorce and blended families, safeguarding, and the interface with schools and paediatrics.

2. Learning outcomes

#	Outcome	Assessed
LO1	Conduct a developmental and systemic assessment of a child or adolescent	Observed session
LO2	Engage a child using play, drawing and conversation appropriate to age	Skills assessment
LO3	Formulate a child's presentation relationally and developmentally	Case study
LO4	Deliver a family-based intervention for at least two common presentations	Case study
LO5	Work with schools, paediatricians and child protection systems	Report
LO6	Recognise and respond to safeguarding concerns	Exam
LO7	Manage the ethical complexities of consent, confidentiality and divided loyalty	Viva

3. Assessment

Component	Weight	Due
Child assessment observed (30 min with child + 20 min feedback to carers)	30%	Week 8

Component	Weight	Due
Family-based intervention case study (2,500 words)	40%	Week 13
Examination	30%	Week 16

4. The fifteen-week plan

Week	Topic	Practical
1	Children in context: why family therapy is child therapy	Observation of a child at play
2	Developmental assessment and red flags	Milestone interview
3	Engaging children: language, play, drawing, behaviour	Drawing interview
4	Engaging adolescents: alliance, autonomy, honesty	Adolescent interview
5	Consent, confidentiality and divided loyalty	Ethics scenario work
6	Anxiety and school refusal	Family-based exposure plan
7	Behaviour problems and parenting interventions	Parent session rehearsal
8	**Child assessment observed**	Assessed
9	Trauma and maltreatment in children	Safety and stabilisation
10	Self-harm and suicidality in adolescents	Risk assessment with a young person
11	Divorce, separation and blended families	Telling-the-children protocol
12	Eating difficulties and body image	Family-based approach outline
13	**Case study due**; working with schools and paediatric services	Report writing
14	Neurodivergence in family context	Environmental adaptation
15	Endings with children; integration	Termination play session

Week 1 — Children in context

The paradox of child therapy. A child is referred because of behaviour. The behaviour occurs in a context. If you treat the child alone, you have removed them from the context that maintains the behaviour and asked them to change it on their own. Family-based work is therefore not an ideological preference; it is a practical inference.

Children as agents. Children are not passive recipients of family dynamics. They notice, interpret, protect, sacrifice and resist. A nine-year-old who refuses school may be keeping a depressed mother company; a thirteen-year-old who fights may be absorbing a father's rage to protect a younger sibling.

The evidence. Family-based treatments have strong evidence for: conduct problems (parent management training, functional family therapy, multisystemic therapy),

adolescent substance misuse, eating disorders (family-based treatment for anorexia nervosa is first-line), anxiety in children (parent-involved CBT), and psychosis relapse prevention (family intervention).

> **PRACTICAL**
> Observe a child (with consent) for 15 minutes in free play. Record only behaviour: what they did, in what order, what they returned to. No interpretation for the first ten minutes.

Week 2 — Developmental assessment and red flags



Figure 2.1 — Developmental milestones from 0 to 18

The four-category check (from MFT 703): normal variation, delay, regression, symptom.

Red flags requiring referral, not therapy alone.

Domain	Red flag
Language	Loss of words; no words at 24 months with no comprehension
Motor	Loss of motor skills; persistent asymmetry; regression
Social	Loss of eye contact and social interest; loss of pretend play

Domain	Red flag
Behaviour	Sudden change in personality; new aggression after a specific event
School	Marked decline in a previously competent child
Medical	Seizures, severe headaches, weight loss, unexplained bruising
Mental state	Hearing voices; fixed beliefs; severe withdrawal; self-harm
Safeguarding	Disclosure, injuries, sexualised behaviour, fear of a specific adult

Assessment with children. Use several modes: interview, drawing, play observation, parent report, teacher report, and standardised measures (SDQ, RCADS). Children's self-report and parents' report diverge systematically — collect both, report both.

Week 3 — Engaging children

Rules of engagement.

1. **Explain the frame to the child, not just the adult.** "We will meet four times. We will talk and sometimes draw. You can say 'pass' to any question."
2. **Get on their level.** Sit on the floor, use their name, match their pace.
3. **Use a medium.** Drawing, puppets, sand, figures, or a simple game. The medium is not a distraction; it is the language.
4. **Do not interrogate.** One question, then follow. "Tell me about this house" beats "who lives here?"
5. **Be honest.** Children detect adult pretence and are damaged by it. If you cannot answer, say so.

Three reliable techniques.

- **The three wishes.** "If you had three wishes, what would they be?" (One is usually for the child, one for the family, one impossible.)
- **Draw your family.** Then ask: "Who is this? What are they doing? Who is closest to whom?" Note who is omitted, who is drawn largest, who is at the edge.
- **The magic wand / miracle.** "If you woke up and the problem was gone, what would be different at breakfast?"

Avoid interpretation. "That must be your father" is a projection. Ask instead: "Tell me about this person."

> **PRACTICAL**
> 20-minute drawing interview with a peer playing an 8-year-old. Use the three techniques. Debrief: how many questions did you ask, and what did you learn without asking?

Week 4 — Engaging adolescents

What adolescents need from a therapist. Honesty, respect for autonomy, a genuine alliance with them (not with their parents), and confidentiality that is real except where safety requires otherwise.

The first five minutes. Speak to the adolescent first. Ask their version before the parents'. Explain confidentiality in their language: "What you tell me stays here, unless I think you or someone else is in danger. If that happens, I will tell you before I tell anyone else."

Techniques that work.

- **Third-person questions.** "What do teenagers your age usually say about this?" Lowers the exposure.
- **Scaling.** "On 0–10, how much does this bother you? How much does it bother your mum?"
- **Future questioning.** "What do you want your life to look like in two years? What would have to change first?"
- **Silence tolerance.** Do not fill every pause.
- **Negotiated goals.** "What would make this worth your time?"

What fails. Lectures, moralising, siding with parents, pretending to be young, and asking "why".

Week 5 — Consent, confidentiality and divided loyalty

Consent. For minors, a parent or guardian consents; the young person **assents**. Both matter. Record both. Where a competent adolescent seeks help without parental knowledge, follow your jurisdiction's law and your agency policy — and do not improvise.

Confidentiality with children.

Situation	Approach
Child discloses something minor	Keep it; help them tell the parent if they want
Child discloses self-harm	Assess risk; agree a plan; inform carers with the child's knowledge
Child discloses abuse	Follow safeguarding procedure; tell the child what will happen next
Parent asks what the child said	Give themes, not transcripts; protect the alliance
Parent demands a report for court	Refer; a treating therapist should not provide forensic opinion

Divided loyalty. The most common clinical trap: the child tells you something that would hurt the parent, or the parent asks you to "fix" the child's attitude. Name the dynamic and refuse to be recruited: "I am not going to be the one who tells your mother. I will help you do it."

Week 6 — Anxiety and school refusal

Anxiety in children. Developmentally normal fears (separation, darkness, monsters, social evaluation) become disorders when they are persistent, impairing and out of proportion.

The family-based approach.

1. **Psychoeducation.** Anxiety is a false alarm; avoidance reduces anxiety now and increases it later.
2. **Map the accommodation.** What does the family do to prevent the child's distress? (Answering questions for them, sleeping in their bed, allowing school avoidance, checking repeatedly.) Accommodation is usually maintained by love, and naming it must be done without blame.
3. **Graded exposure.** Together, build a ladder from easiest to hardest; start at the bottom; reward approach, not the absence of anxiety.
4. **Parental coaching.** Parents become coaches, not rescuers: "I know this is hard. I will be here at 3 o'clock."
5. **The school plan.** One named adult, a quiet space, a gradual return timetable, and no negotiation at the school gate.

School refusal — a differential. Anxiety, separation difficulty, bullying, learning difficulty, family crisis or a bid to protect a parent. The treatment differs for each. Assess by asking what happens on Sunday night, and what the child's absence makes possible at home.

> **PRACTICAL**
> Build an exposure ladder with a role-played parent for a 10-year-old with separation anxiety. Ten rungs, specific, graded, with a reward and a review date.

Week 7 — Behaviour problems and parenting

The evidence base. Parent management training (PMT) and its variants have the strongest evidence for child conduct problems. The mechanism is consistent: increase positive attention, give clear instructions, use consistent consequences, reduce harsh discipline, and rebuild the parent–child relationship.

The core skills (teach, model, rehearse, review).

Skill	What it looks like
Special time	10 minutes daily of child-led play, no correction
Praise	Specific, immediate, frequent ("thank you for putting your shoes away")
Instructions	Get attention, get close, one instruction, allow 5 seconds, praise compliance
Planned ignoring	For minor attention-seeking; not for dangerous behaviour

Skill	What it looks like
Consequences	Consistent, proportionate, predictable, delivered calmly
Routine	Predictable structure reduces conflict

Why parenting programmes fail. Too much content per session; the parent is depressed or exhausted; the child has an untreated condition; another adult undermines the plan; or the family's material circumstances make consistency impossible. Address these before adding more technique.

Adaptation. In households where several adults care for the child (grandmother, aunt, house help), bring them all in and agree one plan. A plan implemented differently by four adults is not a plan.

Week 8 — Observed child assessment

Thirty minutes with a child (role-played by a trained peer/actor), then twenty minutes of feedback to the carers. Assessed on: engagement, developmental appropriateness, information gathering without interrogation, risk screen, and the quality of the feedback.

Week 9 — Trauma and maltreatment

Principles (developed in MFT 714). Safety first; stabilise before processing; the caregiver's own regulation is the child's best medicine; and never force a narrative.

What trauma looks like in children.

Age	Presentation
0–2	Feeding and sleep disruption, startled response, withdrawal
3–6	Regression, repetitive play, nightmares, clinging
7–11	Aggression, school problems, somatic complaints, guilt
12–18	Risk-taking, self-harm, dissociation, substance use, precocious sexuality

Family work after maltreatment. The non-offending caregiver's belief and protection is the single strongest predictor of recovery. Work with them first. Where the perpetrator remains in the home, safety planning precedes everything.

Safeguarding. Know your reporting route, document verbatim disclosures without leading questions, and never promise a child secrecy about abuse.

Week 10 — Self-harm and suicidality in adolescents

Ask directly. "Sometimes when people feel this bad, they think about hurting themselves or ending their life. Has that happened for you?" Direct questions do not plant ideas.

Assess: frequency, method, function (relief, punishment, communication, control), intent, plan, means, protective factors, and the family's response.

Manage the family. Parents often respond with panic, punishment or secrecy. Help them respond with calm supervision and connection: remove means, increase monitoring without surveillance, and seek help immediately if risk rises.

Confidentiality. A young person's request for secrecy must yield to safety. Tell them what you will disclose and why, and involve them in how.

Week 11 — Divorce, separation and blended families

Telling the children. Both parents together, a simple message with no blame, permission to love both parents, and reassurance that the adults' decision is not the child's fault. No details about the affair, the money or the courts.

What harms children. Exposure to conflict (more than separation itself), being used as a messenger, being asked to choose, hearing a parent denigrated, and instability.

What protects them. A cooperative co-parenting relationship, structure, a relationship with both parents, and adults who can manage their own distress.

Blended families. Build the couple bond first; stepparents take authority gradually and through the biological parent; children need permission to love both households; and the "insider/outsider" split must be named and worked with.

Week 12 — Eating difficulties

Family-based treatment for anorexia nervosa (FBT, "Maudsley") is a first-line intervention for adolescents. It is agnostic about cause and externalises the illness: the family is recruited to defeat the eating disorder, not blamed for it.

Three phases. 1) Parents take charge of refeeding; 2) control is returned to the adolescent, age-appropriately; 3) identity and developmental issues are addressed.

Medical safety. Eating disorders carry the highest mortality of any mental disorder. Weight, vital signs and bloods are medical matters; do not work in isolation.

Avoid. Explaining the illness as a consequence of family dynamics; power struggles over food in session; and colluding with the idea that the young person can simply "decide to eat".

Week 13 — Schools and paediatric services

The school as a system. Teachers see the child in a different context and hold different data. Ask for specifics, not impressions: attendance, incidents, academic trajectory, peer relationships, and what strategies have been tried.

Writing a useful report for a school. One page: what you observed, what it means, three concrete recommendations, and who to contact. Avoid diagnosis-led recommendations the school cannot implement.

Paediatric liaison. Distinguish functional from organic symptoms without dismissing either. A referral that names what you want excluded is more likely to be answered (see MFT 707).

> **REPORT**
> Write a one-page school communication for a child with school refusal, including a graded return plan with named adults.

Week 14 — Neurodivergence in family context

Reframe. Autism and ADHD are not behavioural choices and not caused by parenting. But the environment determines how disabling they are.

Practical family work.

- Psychoeducation and demystification for siblings and grandparents.
- Environmental adaptation: predictability, visual schedules, sensory accommodations, transition warnings.
- Parental coaching: reduce the demand–avoidance cycle; increase specific praise; build in recovery time.
- Couple work: neurodivergence in a partner or child strains the parental alliance; address it explicitly.
- School liaison: accommodations are usually more effective than consequences.

Avoid. Behavioural contracts for skills the child does not yet have; blaming parents; and treating a diagnosis as an explanation for everything.

Week 15 — Endings with children

Children end differently. Use a session to review: what we did, what changed, what you will remember, and what you will do when you need help again. Use the medium you used (drawing, play) to close the loop. Give the child something concrete (a certificate, a drawing, a letter) and make sure the carers know when and how to return.

Glossary

Accommodation (of anxiety) — family behaviours that reduce a child's distress in the short term and maintain the anxiety. **Assent** — a minor's agreement to treatment, alongside guardian consent. **Externalising (the illness)** — separating the child from the disorder, as in FBT. **Family-based treatment (FBT)** — a manualised family approach to adolescent anorexia nervosa. **Parent management training (PMT)** — behavioural skills training for parents of children with conduct problems. **School refusal** — difficulty attending school owing to emotional distress, distinct from truancy. **Separation anxiety** — developmentally excessive distress about separation from attachment figures.

Self-assessment

Multiple choice

1. The strongest evidence for child conduct problems is for: (a) individual play therapy (b) parent management training (c) family sculpture (d) medication only

2. In anxiety work, family accommodation: (a) is helpful (b) maintains anxiety despite reducing short-term distress (c) is irrelevant (d) should be punished

3. Asking an adolescent directly about suicide: (a) increases risk (b) does not increase risk and is required (c) should be avoided (d) requires parental consent

4. FBT for anorexia: (a) blames parents (b) recruits parents to defeat the illness (c) focuses on insight only (d) is individual therapy

5. School refusal differs from truancy in that it involves: (a) deception without distress (b) emotional distress about attending (c) peer pressure (d) academic failure

6. A child's disclosure of abuse should be: (a) kept confidential (b) documented verbatim and reported (c) discussed with the alleged abuser (d) ignored if vague

7. The strongest predictor of recovery after maltreatment is: (a) the child's temperament (b) a protective, believing caregiver (c) the severity of abuse (d) therapy length

8. Neurodivergence is best addressed by: (a) behavioural contracts (b) environmental adaptation and skills-based support (c) blaming parents (d) exclusion

9. Divided loyalty refers to: (a) the therapist's loyalty to the agency (b) the child's conflict between therapist and parent (c) sibling rivalry (d) custody disputes

10. Telling children about divorce should: (a) include details of the affair (b) be done by both parents together without blame (c) be left to the courts (d) wait until they ask

Answer key

1 b · 2 b · 3 b · 4 b · 5 b · 6 b · 7 b · 8 b · 9 b · 10 b

Short answer

1. List six red flags requiring referral in child assessment.

2. Describe the three drawing/play techniques and what each reveals.
3. How would you explain confidentiality to a 14-year-old?
4. Give the five core skills of parent management training.
5. What are the three phases of FBT and what happens in each?

Essay (choose one, 2,000 words)

1. "There is no such thing as child therapy without the family." Discuss with evidence.
2. Using a case, show how a developmental formulation changed the treatment of a behaviour problem.
3. Analyse the ethical tensions in working with a 15-year-old whose parents disagree with her treatment goals.

Further reading

- Carr, A. (2019). Family Therapy: Concepts, Process and Practice (3rd ed.). Wiley.
- Kazdin, A. E. (2018). Parent Management Training. OUP.
- Lock, J., & Le Grange, D. (2013). Treatment Manual for Anorexia Nervosa: A Family-Based Approach (2nd ed.). Guilford.
- Sroufe, L. A. et al. (2005). The Development of the Person. Guilford.
- Siegel, D. J., & Bryson, T. P. (2011). The Whole-Brain Child. Delacorte.

Next: MFT 712 — Crisis Intervention Strategies

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MFT 712

Crisis Intervention Strategies

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The books are designed to be read straight through before the semester begins, revisited week by week, and used again in revision. Cross-references point to the other volumes where a topic is treated in more depth.

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A note on the case material

All families, couples and individuals in this book are fictional composites. They are written to teach pattern recognition, not to describe any real person. Nothing here replaces supervised clinical practice, professional registration, legal advice or medical care; legal content is orientation only and must be checked against the law of the relevant jurisdiction.

The lecture library

Twenty-eight self-contained course books carry the taught programme: eight in Semester One, seven in Semester Two, seven in Semester Three including the thesis, and six electives. The volume you are holding is marked in the margin.

Code	Course	Semester	Units
MFT 701	Fundamentals of Marriage and Family Therapy	One	1.71
MFT 702	Theories of Family Therapy	One	1.71
MFT 703	Human Development Across the Lifespan	One	1.71
MFT 704	Ethical and Legal Issues in Therapy	One	1.71
MFT 705	Research Methods in Marriage and Family Therapy	One	1.71
MFT 706	Clinical Practice and Techniques	One	1.71
MFT 707	Assessment and Diagnosis in Family Therapy	One	1.71
MFT 708	Cultural Competence in Therapy	One	1.71
MFT 709	Group Therapy Techniques	Two	1.71
MFT 710	Couples Therapy	Two	1.71
MFT 711	Child and Adolescent Therapy	Two	1.71
MFT 712	**Crisis Intervention Strategies**	Two	1.71
MFT 713	Substance Abuse and Family Dynamics	Two	1.71
MFT 714	Trauma-Informed Care	Two	1.71
MFT 715	Family Systems Theory	Two	1.71
MFT 716	Communication Skills in Therapy	Three	1.71
MFT 717	Psychopathology in Families	Three	1.71
MFT 718	Intimacy and Relationship Dynamics	Three	1.71
MFT 719	Family Life Cycle and Transitions	Three	1.71
MFT 720	Supervision and Consultation in Therapy	Three	1.71
MFT 721	Research Methodology	Three	1.71
MFT 722	Professional Development and Self-Care	Electives	1.71
MFT 723	Special Populations in Therapy	Electives	1.71
MFT 724	Family Law and Advocacy	Electives	1.71
MFT 725	Integrative Approaches in Therapy	Electives	1.71
MFT 726	Neurobiology of Relationships	Electives	1.71
MFT 727	Play Therapy Techniques	Electives	1.71
MFT 770	Master Thesis	Three	1.71

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Course leader's summary. A crisis is not a bigger version of an ordinary session. It is a different clinical state with its own rules: safety before insight, structure before exploration, and short contacts with clear follow-up. This course will make you useful in the first hour.

How to use this book

The course is built around the **first hour**: what you do in the ten minutes after a disclosure of suicide, a violent incident, a sudden death, or a family that arrives in chaos. Read the scripts aloud. Under stress, you will use the sentences you have rehearsed, not the theory you remember.

1. Course description

Crises are the point at which families enter services most often and at which therapists feel least prepared. This course covers: crisis theory and the phases of a crisis; assessment of lethality and danger; suicide and self-harm; domestic violence; child protection emergencies; acute grief and bereavement; family crises (separation, disclosure, runaway, arrest); disasters and collective trauma; telephone and remote crisis work; and the organisational context of crisis services.

2. Learning outcomes

#	Outcome	Assessed
LO1	Assess a crisis using a structured model, and prioritise safety	Observed assessment
LO2	Conduct a suicide risk assessment and co-create a safety plan	Skills test
LO3	Manage acute family conflict and violence safely in session	Simulation
LO4	Respond to acute grief, shock and disorganisation	Case study
LO5	Work within the limits of a single session and a crisis team	Viva
LO6	Document crisis work defensibly	Portfolio
LO7	Protect yourself from vicarious trauma in crisis work	Reflective portfolio

3. Assessment

Component	Weight	Due
Simulated crisis assessment (30 min)	30%	Week 6
Safety plan portfolio (3 plans with rationale)	25%	Week 10
Written examination (case-based)	25%	Week 16
Crisis log and reflection (10 entries)	20%	Ongoing

4. The fifteen-week plan

Week	Topic	Practical
1	What makes a crisis a crisis	Map a crisis in phases
2	The first ten minutes: safety, contact, structure	Opening drill
3	Crisis assessment and triage	Triage exercise
4	Suicide I: assessment	Observed risk interview
5	Suicide II: safety planning and means restriction	Safety plan lab
6	**Simulated crisis assessment**	Assessed
7	Self-harm and repeated crisis	Containment planning
8	Domestic violence and danger assessment	Safety planning with a victim
9	Child protection emergencies	Reporting rehearsal
10	**Safety plan portfolio due**; acute grief and bereavement	Death notification role-play
11	Family crises: runaway, arrest, disclosure, eviction	Multi-agency simulation
12	Disasters and collective trauma	PFA demonstration
13	Telephone, text and remote crisis work	Call simulation
14	The crisis team, handover and follow-up	Handover practice
15	Vicarious trauma and sustainable crisis work	Personal plan

Week 1 — What makes a crisis a crisis

Definition. A crisis is a state of psychological disequilibrium in which a person's usual coping methods fail and their functioning is impaired, usually following a precipitating event that is perceived as a threat. It is **time-limited** (typically 4–6 weeks), **dangerous** (risk of harm), and **an opportunity** (people are more open to help than at any other time).

The four phases (Caplan).

1. **Impact / shock** — disorganisation, disbelief, numbness.
2. **Tension and disorganisation** — attempts to cope fail, distress rises, help-seeking begins.
3. **Mobilisation** — active attempts to solve, often chaotic.
4. **Resolution or breakdown** — adaptation, or chronic dysfunction.

The precipitant is not the whole story. A crisis occurs when a stressor meets a vulnerability. Two families can experience the same event and only one present in crisis. Assess both.

Family crisis is systemic. One member's crisis reorganises the household: roles shift, secrets surface, and previously stable patterns break. Your assessment is of the family's equilibrium, not only the identified person's.

> **PRACTICAL**
> Take a crisis you have witnessed (disguised). Map it onto the four phases and identify where intervention would have changed the trajectory.

Week 2 — The first ten minutes

Crisis work has a protocol. In the first ten minutes you do five things, in this order:

1. **Safety.** Is anyone in immediate danger? Are you safe? Is there a weapon, a person at the door, an overdose already taken?
2. **Contact.** Your voice, your pace, your presence. Lower your tone, slow your speech, sit down, say your name and your role.
3. **Structure.** Tell them what is going to happen in the next hour. "I am going to ask you some difficult questions. Then we will make a plan together."
4. **Story.** Let them tell it, but with a boundary: "Start from the beginning; we have twenty minutes."
5. **Mobilise help.** Who is available? Who can come? What has helped before?

Sentences that work.

- "I am glad you told me. You are not in trouble."
- "I need to ask you some direct questions. They may feel intrusive. I am asking because I need to know how to keep you safe."
- "You do not have to solve this tonight. We need to get through the next twenty-four hours."

Sentences that do not work. "Everything will be fine." "You should not feel that way." "At least…" "Do you really mean that?"

Week 3 — Crisis assessment and triage

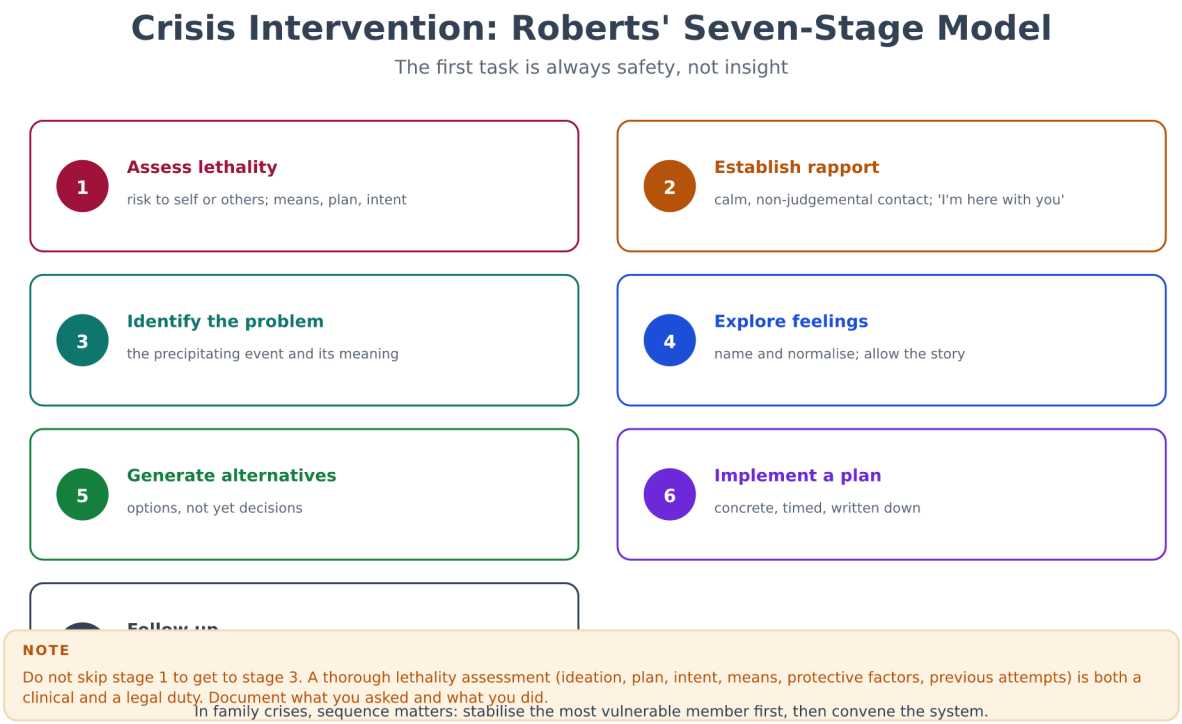


Figure 3.1 — Roberts' seven-stage crisis intervention model

The seven stages (Roberts) provide a structure that survives chaos: assess lethality → establish rapport → identify the problem and precipitant → explore feelings → generate alternatives → implement a plan → follow up.

Triage: three questions.

1. **Is there immediate danger?** (Weapon, overdose, assault in progress, child alone with a dangerous adult.)
2. **What is the appropriate level of care?** (Emergency department, crisis team, same-week appointment, routine.)
3. **Who else needs to know?** (Partner, school, social services, police, GP.)

Do not provide long-term therapy in a crisis. Your task is stabilisation, safety and connection to ongoing care. A crisis service that becomes the client's therapy creates dependence and denies the client the long-term work.

Week 4 — Suicide I: assessment

The epidemiology you need. Suicide is among the leading causes of death in young people; men die more often, women attempt more often; previous attempt is the strongest predictor; access to means matters; most people who die by suicide have communicated something to someone.

The structured assessment (ten domains). Ideation, plan, intent, means, previous attempts, protective factors, substance use, impulsivity, family history, current stressors — plus a direct question about what has kept you alive so far.

Asking.

> "When you say you cannot go on, I need to be clear: are you thinking about ending your life?" "Have you thought about how you would do it?" "Do you have [the means] at home?" "Have you ever tried before?" "What has kept you going until now?"

Formulation and risk level.

Level	Picture	Action
Low	Ideation without plan, strong protective factors, engaged	Safety plan, follow-up within 1 week, means counselling
Moderate	Ideation with plan, limited intent, some protective factors	Same-day mental health review, means restriction, daily contact
High	Plan, intent, means, recent attempt, agitation, intoxication	Do not leave alone; emergency department or crisis team; consider involuntary route

Documentation. Record the questions and answers verbatim where possible, the risk level, the rationale, the plan, who was informed, and the review. If you judged risk lower than others might, state why.

Week 5 — Suicide II: safety planning

A safety plan is not a no-suicide contract. Contracts have no evidence base and can increase guilt. A safety plan is a written, collaborative, stepwise coping plan.

Six components (Stanley & Brown).

1. **Warning signs** — the person's own early signals ("I stop answering my phone").
2. **Internal coping** — things they can do alone ("walk to the mosque", "cold shower", "music").
3. **People and places that distract** — social settings that reduce isolation.
4. **People to ask for help** — names and numbers.
5. **Professionals and agencies** — crisis line, clinic, emergency number.
6. **Making the environment safe** — means restriction: who holds the medication, the keys, the pesticide.

Means restriction is the most effective single intervention. Ask directly: "What is in the house?" Ask the family to take control of it, and be specific about who, what, and where.

With families. The family's response matters: reduce blame, increase supervision without surveillance, and give the person a role in their own plan.

PRACTICAL

Co-create a safety plan with a role-played client. The assessor checks: was it collaborative? Were means addressed specifically? Was a follow-up date agreed?

Week 6 — Simulated crisis assessment

Thirty minutes, unannounced scenario (suicide risk, domestic violence, or acute grief). Assessed on: safety first, contact and pace, structured assessment, formulation, plan, and documentation quality.

Week 7 — Self-harm and repeated crisis

Self-harm is often a regulation strategy, not a suicide attempt. Distinguish: function (relief, control, punishment, communication, feeling something), lethality, frequency, and hidden versus disclosed.

Working with repeated crises. Frequent attenders are often described as "manipulative" — a term that should be banned. The pattern usually reflects: an untreated disorder, chronic trauma, an unstable relationship system, or a service that rewards crisis and does not provide continuity.

The intervention is a plan, not a lecture. A written crisis protocol agreed with the person (what to do, who to call, when to come to hospital) reduces both crisis frequency and staff burnout.

Therapist boundaries. You cannot be available 24 hours. State your availability clearly, give the crisis route for out-of-hours, and do not give your personal number unless your service requires it and you have thought through the consequences.

Week 8 — Domestic violence and danger assessment

Danger assessment questions (adapted from the Danger Assessment).

- Has violence increased in frequency or severity in the last year?
- Has the partner ever used a weapon or threatened one?
- Has the partner ever tried to strangle or choke you?
- Has the partner threatened to kill you or the children?
- Is the partner violently or constantly jealous and controlling?
- Is there a separation in progress (the highest-risk period)?
- Does the partner have access to firearms?
- Has the partner ever forced sex?
- Is the partner using substances heavily?
- Has the partner threatened suicide?

Safety planning with a victim. Identify a safe place, a code word, documents and money, a bag prepared, children's needs, transport, and the number of a specialist service. Do not tell her to leave as though leaving were simple; leaving is the most dangerous period, and she is the expert on her own risk.

Do not conduct conjoint sessions where coercive control is present. Do not inform the abuser of the disclosure. Know your mandatory reporting obligations — and their limits (in many jurisdictions, adult IPV is not mandatorily reportable, but child exposure may be).

Week 9 — Child protection emergencies

Signs requiring immediate action.

- A disclosure of abuse (physical, sexual, emotional, neglect).
- Unexplained injury inconsistent with the explanation.
- A child who is frightened of a specific adult.
- A child left alone or in the care of someone unable to protect them.
- Sexualised behaviour beyond developmental norms.
- A parent who cannot or will not protect the child.

Procedure.

1. Respond calmly; do not interview in detail; use open prompts only ("tell me what happened").
2. Record verbatim, promptly, with the child's words and your questions.
3. Do not promise secrecy.
4. Report to the designated safeguarding lead or the appropriate authority, following your service's procedure and your jurisdiction's law (in Nigeria, the Child Rights Act and state child protection structures; the police family unit; the state ministry responsible for social welfare).
5. Tell the child what will happen next and who will be involved.
6. Document everything, including the advice you received and the time you acted.

> **REHEARSAL**
> Practise the first four minutes of a disclosure, and the phone call to report it.

Week 10 — Acute grief and bereavement

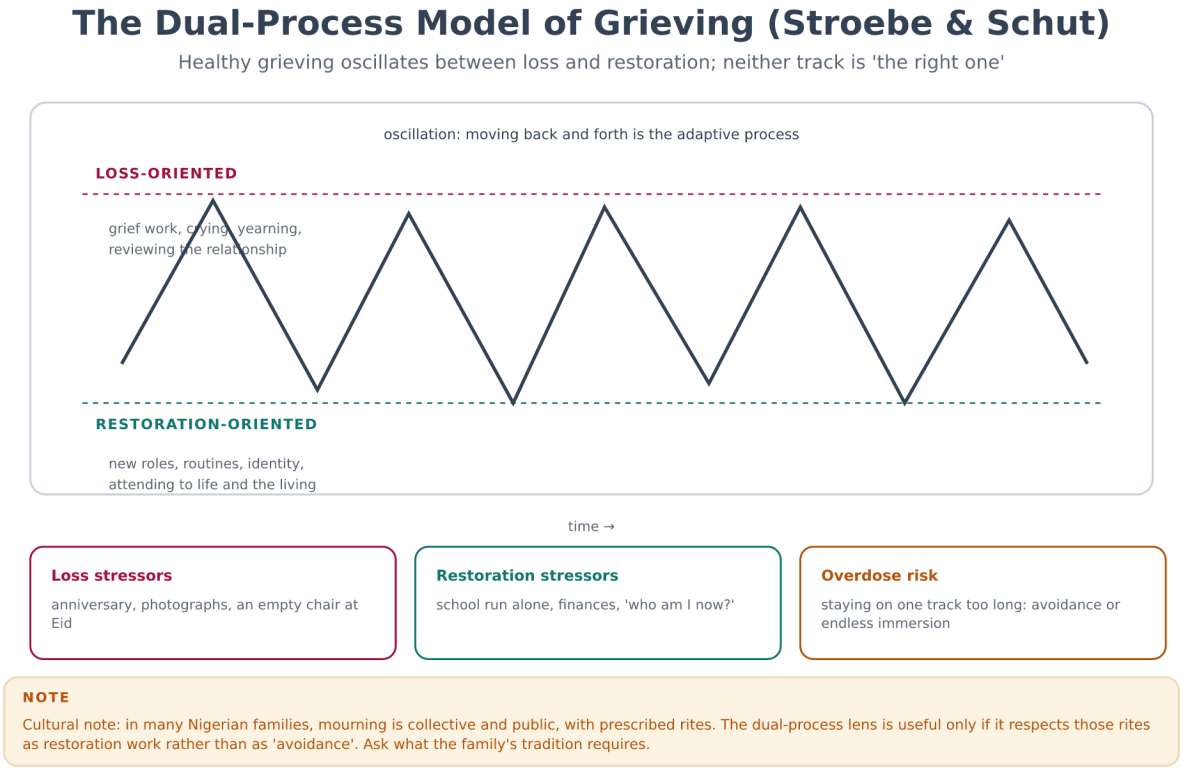


Figure 10.1 — The dual-process model of grieving

Normal acute grief includes waves of yearning, disbelief, anger, guilt, intrusive images, and a sense of unreality. It is not a disorder. It becomes complicated when it is prolonged, severe, and impairing (prolonged grief disorder in ICD-11/DSM-5-TR).

In the first weeks: accompany, do not treat. Practical help, information, permission to grieve, and continuity. Do not rush to meaning-making; do not say "he is in a better place" unless you know the family's faith.

Sudden and traumatic deaths (accident, homicide, suicide) add shock, anger, stigma and, for suicide, a family burden of blame and secrecy. Address the stigma explicitly.

Children and grief. Children grieve in bursts, return to play, and ask direct questions. Be honest, use clear language ("died", not "lost"), and reassure them that they are not to blame.

The dual-process frame. Healthy grieving oscillates between loss-oriented and restoration-oriented activity. Two errors: staying only on the loss track (rumination, avoidance of life) and staying only on the restoration track (avoidance of grief). The family's culture prescribes which is expected; work within it.

Week 11 — Family crises

Crisis	Immediate priorities	Common errors
Adolescent runaway	Safety, whereabouts, who they trust, why they left	Interrogating the parents first; treating the adolescent as the problem
Arrest or imprisonment	Practical information, children's needs, shame, finances	Moralising; ignoring the children
Disclosure of a secret (affair, paternity, HIV, debt)	Containment, safety, who needs to know and when	Forcing immediate disclosure; colluding in concealment
Eviction or financial collapse	Practical referral; safety; children's schooling	Psychologising material reality
Sudden separation	Safety, children's stability, legal referral	Assuming reconciliation is the goal
Death of a child	Presence, not technique; couple's survival	Interpreting; taking sides in blame

Week 12 — Disasters and collective trauma

Psychological first aid (PFA) — five elements: safety, stabilisation, connection, self-efficacy, hope. It is not debriefing. Mandatory psychological debriefing has been shown to be ineffective and possibly harmful; do not do it.

Principles in collective events.

- Attend to basic needs first (shelter, food, information, family reunification).
- Do not pathologise normal distress.
- Reconnect people to their own networks and rituals; community and faith resources are usually more powerful than clinical intervention in the acute phase.
- Identify the vulnerable: unaccompanied children, the injured, the bereaved, those with prior trauma, and responders.
- Plan for the medium term: most distress resolves; a minority need treatment at 4–12 weeks.

Week 13 — Telephone, text and remote crisis work

Advantages. Access, immediacy, anonymity, reach into areas with no services.

Risks. Loss of non-verbal information, difficulty verifying location, inability to intervene physically, and boundary erosion.

Protocol for a remote crisis contact.

1. Verify identity and location at the start ("where are you right now?").
2. Establish whether anyone is with them.
3. Assess risk verbally, more slowly than you would in person.

4. Agree a concrete plan for the next hour.
5. Record the number they called from and a safe contact.
6. If risk is high, keep them on the line while arranging help.

Week 14 — The crisis team and continuity

Handover. A good handover is structured: presenting problem, risk level and basis, what has been done, what is planned, who is responsible, and when the next contact is. Use a standard format (e.g. SBAR: Situation, Background, Assessment, Recommendation).

Follow-up is the intervention. Crisis care that does not follow up fails. Agree a date before the person leaves, and make the contact proactive.

Working with other agencies. You will share information; know the legal basis, share the minimum necessary, and record what you shared and why.

Week 15 — Vicarious trauma and sustainable practice

Crisis work takes a physiological toll. Warning signs: intrusive images, sleep disruption, cynicism, numbness, dread before shifts, irritability at home, and loss of pleasure in work.

Protective practices. Debrief structurally (not emotionally) after critical incidents; use supervision; keep caseload diversity; maintain boundaries on availability; sleep; exercise; and have your own therapist.

The organisational dimension. Individual self-care cannot compensate for an unsafe service. If you are carrying 40 crisis cases with no supervision, that is a management problem, and saying so is part of professional practice.

Glossary

Containment — the provision of structure and safety so that overwhelming material can be held. **Crisis** — a time-limited state of disequilibrium in which usual coping fails. **Danger assessment** — structured assessment of lethality risk in domestic violence. **Means restriction** — reducing access to methods of self-harm; the most effective suicide prevention measure. **Psychological first aid (PFA)** — a five-element supportive intervention after a critical event. **Safety plan** — a written, collaborative, stepwise plan for managing a suicidal crisis. **Triage** — the prioritisation of care according to urgency and risk.

Self-assessment

Multiple choice

1. The first task in a crisis is: (a) insight (b) safety (c) diagnosis (d) history

2. Roberts' model begins with: (a) rapport (b) lethality assessment (c) plan (d) follow-up

3. A no-suicide contract is: (a) evidence-based (b) not evidence-based; a safety plan is preferred (c) legally required (d) sufficient alone

4. The highest-risk period in domestic violence is: (a) during pregnancy (b) separation (c) the first year (d) after counselling

5. Psychological debriefing after disasters is: (a) recommended for all (b) not recommended and potentially harmful (c) legally required (d) equivalent to PFA

6. Means restriction is important because: (a) it is easy (b) it reduces the probability of a lethal attempt (c) it is legally required (d) it satisfies families

7. Conjoint therapy is contraindicated when: (a) there is conflict (b) there is coercive control (c) there is grief (d) there is low income

8. A child's disclosure of abuse should first be: (a) investigated by you (b) recorded verbatim and reported (c) discussed with the parents (d) kept confidential

9. Prolonged grief disorder is distinguished from normal grief by: (a) crying (b) duration, severity and impairment (c) cultural expression (d) family conflict

10. A good crisis handover includes: (a) only the diagnosis (b) situation, background, assessment, recommendation, and next contact (c) your opinion of the family (d) the full case notes

Answer key

1 b · 2 b · 3 b · 4 b · 5 b · 6 b · 7 b · 8 b · 9 b · 10 b

Short answer

1. List the five things you do in the first ten minutes of a crisis.
2. Give six components of a safety plan.
3. Name three questions from a danger assessment.
4. What are the five elements of psychological first aid?
5. Why is follow-up the intervention in crisis work?

Essay (choose one, 2,000 words)

1. "Crisis is both danger and opportunity." Discuss with a case, including what you did and what you would change.
2. Analyse the ethical tensions in responding to a suicidal adolescent whose parents refuse hospital admission.
3. Critically evaluate the evidence on psychological debriefing and its implications for crisis services.

Further reading

- Roberts, A. R. (2005). Crisis Intervention Handbook (3rd ed.). Oxford University Press.
- James, R. K., & Gilliland, B. E. (2017). Crisis Intervention Strategies (8th ed.). Cengage.
- Stanley, B., & Brown, G. K. (2012). Safety planning intervention. Cognitive and Behavioral Practice, 19(2), 256–264.
- Hobfoll, S. E. et al. (2007). Five essential elements of immediate and mid-term mass trauma intervention. Psychiatry, 70(4), 283–315.
- Campbell, J. C. (2002). Danger Assessment.

Next: MFT 713 — Substance Abuse and Family Dynamics

MSC MARRIAGE AND FAMILY THERAPY

LECTURE LIBRARY · VOLUME 13 OF 28

MFT 713

Substance Abuse and Family Dynamics

15 teaching weeks · 4 practicals · 1 worked case · 2 figures

SEMESTER TWO

1.71 units · Prerequisite: MFT 710

> “Addiction is a family-organising process.”

One of 28 self-contained course books in the MSc Marriage and Family Therapy lecture library: a full taught semester per volume, with cases, practicals, assessment briefs, glossaries and model answers.

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MFT 713 — Substance Abuse and Family Dynamics

MSc Marriage and Family Therapy · Semester Two · 1.71 Units · 15 teaching weeks · Prerequisite: MFT 710

This volume is one of 28 course books that together cover the whole MSc Marriage and Family Therapy taught programme. Each book is written to carry a 14–16 week semester on its own: every week pairs a core idea with the practice that follows from it, a worked case, and a reflective task, and each book closes with an integration chapter, a glossary and a self-assessment paper with model answers.

The books are designed to be read straight through before the semester begins, revisited week by week, and used again in revision. Cross-references point to the other volumes where a topic is treated in more depth.

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A note on the case material

All families, couples and individuals in this book are fictional composites. They are written to teach pattern recognition, not to describe any real person. Nothing here replaces supervised clinical practice, professional registration, legal advice or medical care; legal content is orientation only and must be checked against the law of the relevant jurisdiction.

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Twenty-eight self-contained course books carry the taught programme: eight in Semester One, seven in Semester Two, seven in Semester Three including the thesis, and six electives. The volume you are holding is marked in the margin.

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MFT 701	Fundamentals of Marriage and Family Therapy	One	1.71
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MFT 703	Human Development Across the Lifespan	One	1.71
MFT 704	Ethical and Legal Issues in Therapy	One	1.71
MFT 705	Research Methods in Marriage and Family Therapy	One	1.71
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MFT 707	Assessment and Diagnosis in Family Therapy	One	1.71
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MFT 709	Group Therapy Techniques	Two	1.71
MFT 710	Couples Therapy	Two	1.71
MFT 711	Child and Adolescent Therapy	Two	1.71
MFT 712	Crisis Intervention Strategies	Two	1.71
MFT 713	**Substance Abuse and Family Dynamics**	Two	1.71
MFT 714	Trauma-Informed Care	Two	1.71
MFT 715	Family Systems Theory	Two	1.71
MFT 716	Communication Skills in Therapy	Three	1.71
MFT 717	Psychopathology in Families	Three	1.71
MFT 718	Intimacy and Relationship Dynamics	Three	1.71
MFT 719	Family Life Cycle and Transitions	Three	1.71
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MFT 723	Special Populations in Therapy	Electives	1.71
MFT 724	Family Law and Advocacy	Electives	1.71
MFT 725	Integrative Approaches in Therapy	Electives	1.71
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MFT 770	Master Thesis	Three	1.71

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Course leader's summary. Addiction is a family-organising process. This course teaches you to work with the user, the family, and the household system — using motivational, behavioural, systemic and recovery-oriented approaches — without moralising, colluding or abandoning hope.

How to use this book

Read the case at the start of each week before the theory. Addiction work punishes abstraction: families are exhausted, professionals are frustrated, and the evidence is often misrepresented by both. Hold two principles throughout: **the person is not the substance**, and **the family is not the cause**.

1. Course description

The course covers: models of addiction (disease, learning, biopsychosocial, systemic); assessment; the family cycle of use, secrecy and shame; motivational interviewing; relapse prevention; behavioural couples therapy for substance use; CRAFT (for concerned relatives); the interface with mental illness ("dual diagnosis"); recovery as a family process; medication-assisted treatment; and policy and cultural context.

2. Learning outcomes

#	Outcome	Assessed
LO1	Explain and compare models of addiction, including their clinical implications	Exam
LO2	Conduct a substance-use assessment, including readiness to change	Observed assessment
LO3	Use motivational interviewing skills with fidelity	Skills test
LO4	Formulate the family's role in maintaining and resolving use	Case study
LO5	Deliver CRAFT or behavioural couples work for substance use	Case study
LO6	Work with dual diagnosis and coordinate with medical and recovery services	Viva
LO7	Address stigma, culture and shame in your own practice	Reflective portfolio

3. Assessment

Component	Weight	Due
Motivational interviewing observed (25 min)	25%	Week 5
Family formulation and intervention plan (2,500 words)	35%	Week 11
Examination	25%	Week 16

Component	Weight	Due
Reflective portfolio (6 entries)	15%	Ongoing

4. The fifteen-week plan

Week	Topic	Practical
1	Models of addiction and their consequences	Model comparison debate
2	The family cycle: use, secrecy, enabling, crisis, shame, promise	Cycle mapping
3	Assessment: quantity, function, consequences, readiness	Structured interview
4	Motivational interviewing I: OARS and the spirit	OARS drill
5	**MI observed**; MI II: change talk and discord	Change-talk practice
6	The stages of change and the family's stage	Stage-matched planning
7	Working with the user: CBT and relapse prevention	Relapse prevention plan
8	Working with the couple: behavioural couples therapy	Daily recovery plan
9	Working with relatives: CRAFT and boundaries	CRAFT role-play
10	Co-dependency, enabling and the limits of the concept	Boundary-setting lab
11	**Case plan due**; dual diagnosis	Integrated plan
12	Medication-assisted treatment and the medical interface	Referral exercise
13	Recovery, relapse and the family's long haul	Relapse response plan
14	Culture, faith, stigma and the community	Community resource map
15	Integration: what works, for whom	Case presentations

Week 1 — Models of addiction

Model	Core claim	Clinical implication	Critique
Moral	Use is a choice; user is weak or bad	Punishment, exclusion	Shame increases use; ignores biology
Disease / medical	Addiction is a chronic brain disease	Treatment, medication, compassion	Can reduce agency; "once an addict always" fatalism
Learning / behavioural	Use is reinforced behaviour	Skills, contingency management	Can ignore neurobiology and context
Biopsychosocial	Biology × psychology × social context	Multimodal treatment	Descriptive rather than prescriptive
Systemic / family	Use is embedded in and organises relationships	Family-inclusive treatment	Risk of blaming the family

Model	Core claim	Clinical implication	Critique
Recovery / social	Recovery is a social process, not only clinical	Peer support, community, identity	Needs availability of recovery capital

The position used here. Addiction is a **chronic, relapsing condition** shaped by biology, learning, trauma, relationship, culture and economics. The person retains agency but not unlimited agency. Families are neither causes nor cures; they are the environment in which recovery either becomes possible or does not.

> **DEBATE**
> "Addiction is a disease" versus "addiction is a choice". Take a side, then argue the other side, then write the practical position you will hold with clients.

Week 2 — The family cycle

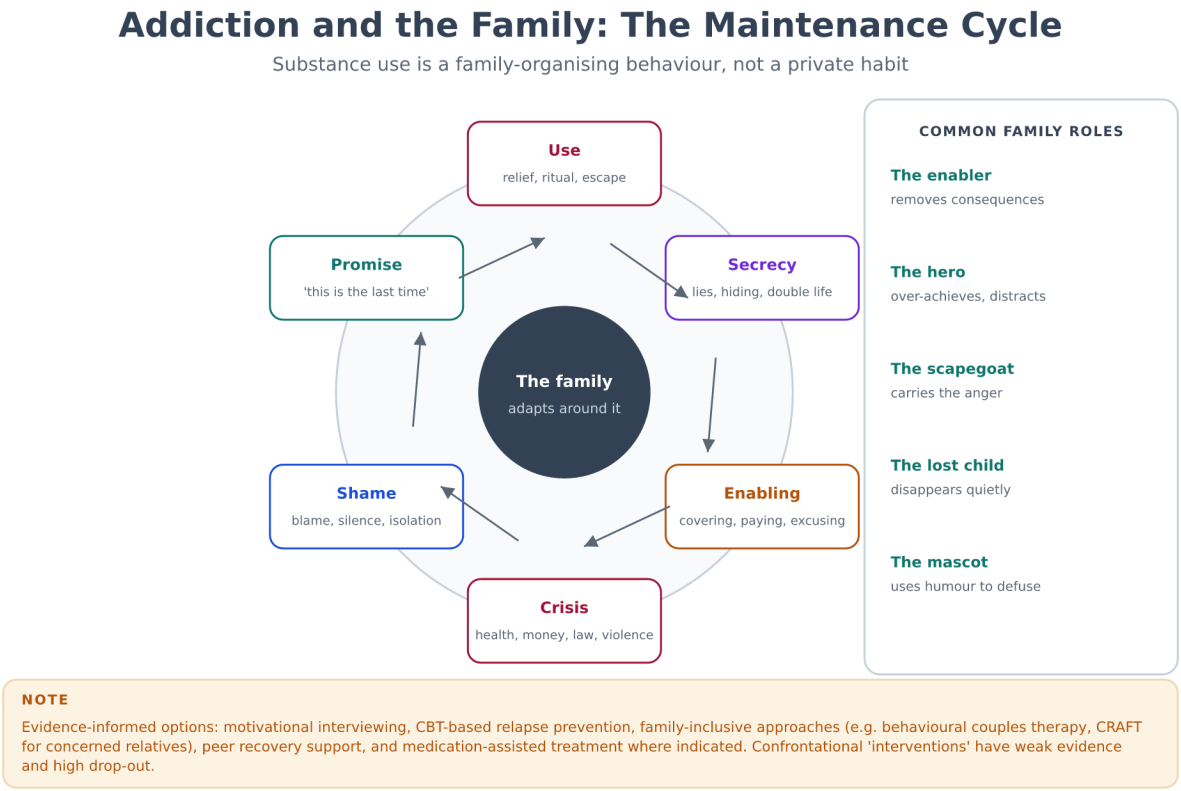


Figure 2.1 — The addiction and family maintenance cycle

Six stations that repeat: use → secrecy → enabling → crisis → shame → promise → use.

Enabling is not weakness. It is a set of behaviours that reduce immediate consequences because the alternative (the person losing their job, being arrested, or dying) feels unbearable: paying a debt, calling in sick, hiding the bottles, giving money "for food".

Family roles (an imperfect but useful map): the enabler, the hero, the scapegoat, the lost child, the mascot. Use them as questions, never as labels: "Who in the family keeps the peace? Who takes the blame? Who disappears?"

The systemic point. The cycle is maintained by both the using and the accommodating. This is not a statement of blame; it is a statement of leverage. Change the accommodating behaviour and the cycle must change.

> **PRACTICAL**
> Map a family cycle from a disguised case. At each station, write: who does what, what they feel, and what they fear. Then identify two points of leverage.

Week 3 — Assessment

Substance history.

Domain	Questions
Substances	What, how much, how often, by what route, for how long
Pattern	Time of day, alone or social, binge or daily
Function	What does it do for you? (sleep, courage, relief, numbness, belonging)
Consequences	Health, money, work, relationships, legal, children
Control attempts	Have you tried to stop? What happened? How long?
Withdrawal	Symptoms, previous seizures, previous medically assisted withdrawal
Dependence	Tolerance, withdrawal, compulsion, time spent, continued despite harm
Readiness	Do you want to change? Why now? What would it take?
Risk	Suicide, violence, driving, overdose, child safety

Validated tools. AUDIT (alcohol), DUDIT (drugs), ASSIST (multiple substances), CRAFFT (adolescents). Use them as a structure for conversation, not a verdict.

Assess the family separately. With the user absent: what has been tried, what has been covered up, what each member fears, and who is at risk.

Week 4 — Motivational interviewing I

The spirit (Miller & Rollnick): partnership, acceptance, compassion, evocation. You are not the expert on what they should do; you are the expert on a conversation that helps them say it themselves.

The four core skills (OARS).

Skill	What it is	Example
Open question	Invites elaboration	"What is a typical evening like?"
Affirmation	Names a strength, specifically	"You kept going to work through all this."
Reflection	Returns content or feeling	"You are worn out by the effort it takes to keep this hidden."
Summary	Collects and connects	"So on one hand it helps you sleep; on the other it is costing you your marriage."

What to avoid. The righting reflex (the urge to fix), confronting, warning, advising without permission, and arguing. When you argue for change, the client argues against it — and you have made change less likely.

Reflection is the active ingredient. Aim for two reflections for every question, and for complex reflections (which add meaning) over simple ones (which repeat content).

> **PRACTICAL**
> OARS drill: 8 minutes, no advice, no questions after the first one. Observer counts reflections, and rates how many were complex.

Week 5 — Motivational interviewing II

Change talk — client statements in favour of change. Four types: desire ("I want to"), ability ("I could"), reasons ("because…"), need ("I have to"). Also mobilising talk: commitment ("I will"), activation ("I am ready"), taking steps ("I bought the medication").

Discord is not resistance; it is a signal about the relationship. It appears as arguing, interrupting, discounting, ignoring. The response is not confrontation but **rolling with it**: "You are not sure this is worth it. Tell me more about that."

The MI-consistent techniques.

- **Elicit–provide–elicit:** "What do you already know about what alcohol does to the liver?" → brief, neutral information → "What does that mean for you?"
- **The decisional balance:** "What are the good things about using? And the less good things?" (Ask in that order, always.)
- **Looking forward:** "If nothing changes, where do you imagine you will be in five years?"
- **Readiness ruler:** "On 0–10, how important is it to change? How confident are you that you could? What would move you from 4 to 6?"

Avoid the readiness trap: a score of 3 is an argument for a smaller intervention, not for discharge.

ASSESSED

25-minute MI session with a trained role-player. Rated on: spirit, OARS fidelity, ratio of reflections to questions, and handling of discord.

Week 6 — Stages of change and the family

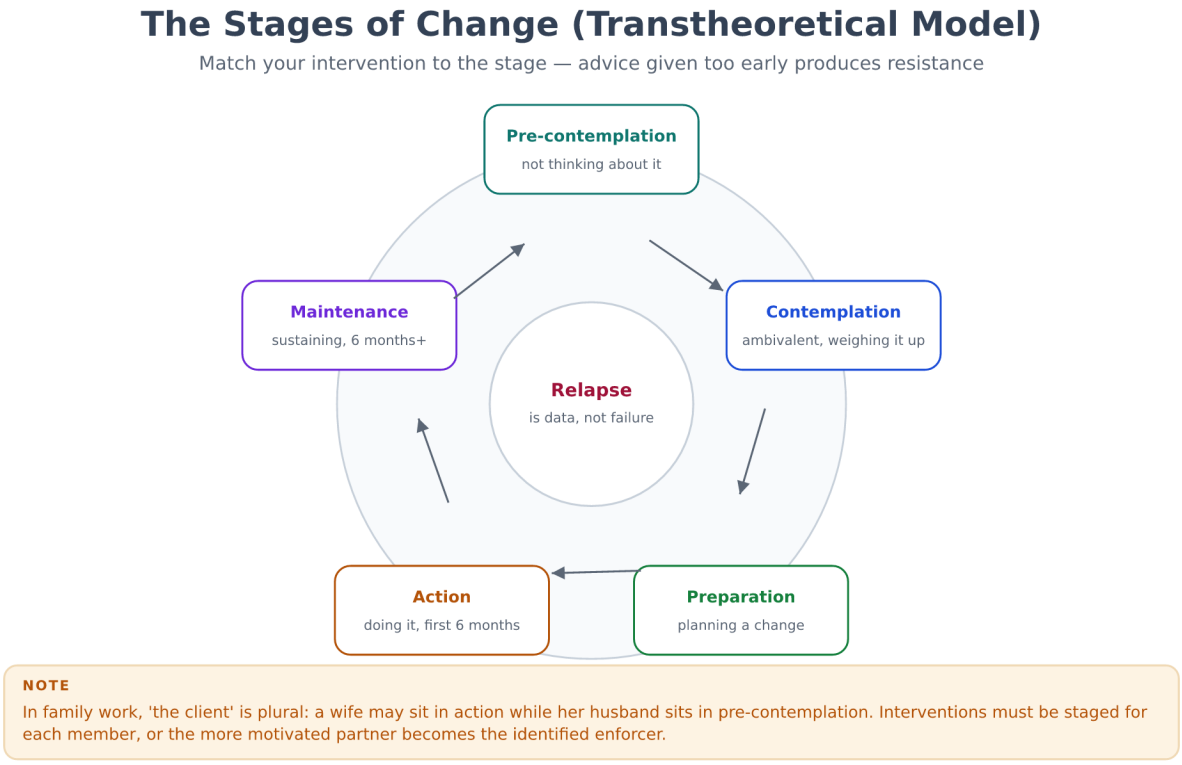


Figure 6.1 — The stages of change, including relapse

Stage	The person	The family member	Your task
Pre-contemplation	Not thinking about change	Frustrated, angry, exhausted	Raise doubt gently; do not push
Contemplation	Ambivalent	Hopeful and suspicious	Tip the balance; build confidence
Preparation	Planning	Mobilising	Concrete plan; remove barriers
Action	Changing	Adjusting, testing trust	Support, skills, relapse prevention
Maintenance	Sustaining	Rebuilding	Protect gains; plan for risk
Relapse	Using again	Devastated, "I told you"	Reframe as data; re-plan fast

The family's stage often differs from the user's. A wife in action married to a husband in pre-contemplation becomes the "enforcer", which damages the relationship and does not work. Stage-matched intervention means working separately where necessary.

Week 7 — Relapse prevention

The model (Marlatt & Gordon). Relapse is a process, not an event: high-risk situations → poor coping → lapse → the abstinence violation effect ("I've blown it, so I may as well continue") → relapse.

The plan.

1. **High-risk situations** — people, places, times, feelings (loneliness, anger, celebration, payday).
2. **Coping strategies** for each.
3. **A lapse protocol** — what to do in the first 24 hours, and whom to call.
4. **Lifestyle balance** — the "shoulds" versus the "wants"; boredom and resentment are relapse risks.
5. **The family's role** — agreed, non-punitive responses to a lapse, decided in advance.

With the family. A relapse response agreed in advance prevents the cycle of crisis, shame and secrecy from restarting. Write it down: "If I use again, we will…"

Week 8 — Behavioural couples therapy for substance use

The evidence. Behavioural couples therapy (BCT) for alcohol and drug problems has good evidence, particularly where the partner is non-using and the relationship is not violent.

The "daily recovery plan" (O'Farrell). Each day: a shared trust discussion in which the abstinent partner states their intention and the other expresses support, then a specified rewarding activity together. It is behavioural, daily, and builds the relationship as a recovery resource.

Why couples work helps. Partners are often the strongest motivator and the most reliable monitoring system — but only if the relationship becomes a source of reward rather than a site of surveillance.

Contraindications. Current violence, coercive control, a partner with active untreated addiction, or a relationship the client is being coerced to remain in.

Week 9 — Working with relatives: CRAFT

CRAFT (Community Reinforcement and Family Training) works with the concerned significant other to change the contingencies around the user's behaviour, without confrontation. It has better evidence for treatment entry than traditional "interventions" or Al-Anon-style support alone.

Three goals. 1) Improve the relative's own wellbeing; 2) reduce the user's substance use; 3) increase the chance the user enters treatment.

Core skills taught to the relative.

- **Functional analysis:** what triggers use, what consequences follow, what the relative does.
- **Positive communication:** brief, specific, warm.
- **Allow natural consequences:** stop removing consequences where it is safe.
- **Reward non-use:** notice and reinforce sober days.
- **Domestic safety:** never ask a relative to accept violence.
- **Self-care and social support:** the relative's life must not be entirely about the user.
- **Invitation to treatment:** a script, rehearsed.

> **PRACTICAL**
> CRAFT role-play: a mother of a 24-year-old cannabis user. Teach positive communication and one natural consequence. Assessor rates warmth, specificity and safety.

Week 10 — Enabling, co-dependency and boundaries

"Co-dependency" is a contested term. Used carefully it describes patterns of over-functioning and self-neglect in relationships with an addicted person. Used carelessly it blames the relative — usually a woman — for the addiction.

Better language: accommodation and boundary. Ask what the relative does, what it costs them, and what they would like to do differently.

Boundary setting that works. Specific, behavioural, enforceable, and about the relative's own actions: "I will not lend money. If you come home intoxicated, I will sleep in the other room and we will talk in the morning." Not: "You must stop drinking or I will leave." (An unenforceable ultimatum teaches the family that threats are empty.)

Week 11 — Dual diagnosis

The overlap is the rule. Anxiety, depression, PTSD, ADHD and psychosis commonly co-occur with substance use. Direction of causality varies; the treatment must be integrated rather than sequential.

Principles.

1. Treat both, at the same time, by the same team where possible.
2. Do not require abstinence before treating mental illness, and do not require remission of mental illness before treating addiction.
3. Be cautious with medication (benzodiazepines, stimulants) and coordinate with the prescriber.
4. Distinguish substance-induced symptoms from independent disorders (timing and persistence).
5. Trauma is common; assume it until assessed.

CASE PLAN (35%)
A family case with dual diagnosis: assessment, formulation, integrated plan, coordination with medical and recovery services, risk plan, and outcome measures.

Week 12 — Medication-assisted treatment

Opioid agonist therapy (methadone, buprenorphine) reduces mortality and improves retention; it is evidence-based treatment, not "substituting one drug for another". **Acamprosate** and **naltrexone** reduce alcohol relapse; **disulfiram** works only with supervision. Withdrawal management alone is not treatment.

The family therapist's role. Support adherence, address stigma within the family, challenge the belief that medication is cheating, and coordinate with the prescriber. Do not advise on doses or discontinuation.

Week 13 — Recovery as a family process

Recovery capital (Cloud & Granfield): social (relationships, support), physical (health, housing), human (skills, education, health literacy), cultural (values, belonging). Treatment that builds recovery capital predicts better outcomes than treatment that focuses only on symptoms.

The family's long haul. Early recovery is often called "the pink cloud"; the harder work is at six to eighteen months, when the family must renegotiate roles that were organised around the addiction. A partner who has run everything may not want to hand back control; a child who has been the carer may not want to be a child. Name this, and plan for it.

Relapse is data. The question is not "why did you fail?" but "what happened, and what does the plan need?"

Week 14 — Culture, faith, stigma and community

- **Substance use in context:** alcohol is culturally embedded in many Nigerian settings (burial rites, festivals, business); kola, cannabis, tramadol and codeine have specific local patterns.
- **Faith:** deliverance and prayer can be powerful resources and can also delay medical care. Coordinate, do not compete.
- **Stigma:** "a disgrace to the family" keeps use hidden and prevents early help. Your language matters: "person with a substance use disorder", not "addict".
- **Community resources:** mutual aid (AA/NA and their local equivalents), faith-based recovery groups, community health workers — map what actually exists near your clinic.

Week 15 — Integration

What works: screening and brief intervention; MI; CBT-based relapse prevention; BCT and CRAFT; medication where indicated; integrated treatment for dual diagnosis; recovery support; and continuity over time. What does not: confrontation, "tough love" as a single strategy, sequential treatment for dual diagnosis, and asking a family to do the work alone.

Glossary

Abstinence violation effect — the spiral of guilt and continued use after a lapse. **CRAFT** — Community Reinforcement and Family Training, for concerned significant others. **Change talk** — client statements favouring change. **Dual diagnosis** — co-occurring substance use and mental disorder. **Enabling** — behaviours that remove the natural consequences of use. **Harm reduction** — interventions that reduce harm without requiring abstinence. **Motivational interviewing** — a collaborative, directive method for strengthening intrinsic motivation. **Recovery capital** — the resources a person can draw on to sustain recovery. **Relapse** — return to problematic use after a period of change.

Self-assessment

Multiple choice

1. The righting reflex refers to: (a) the client's resistance (b) the clinician's urge to fix (c) a CBT technique (d) a legal duty

2. CRAFT is aimed primarily at: (a) the user (b) concerned relatives (c) employers (d) courts

3. Change talk includes: (a) sustain talk (b) desire, ability, reasons, need (c) only commitment (d) only questions

4. Medication-assisted treatment is: (a) substituting one drug for another (b) evidence-based treatment that reduces mortality (c) only for the wealthy (d) discouraged

5. The "abstinence violation effect" describes: (a) legal consequences (b) the spiral after a lapse (c) withdrawal (d) family conflict

6. A well-formed boundary is: (a) an unenforceable ultimatum (b) specific, behavioural and about the relative's own actions (c) a threat (d) a diagnosis

7. In dual diagnosis, treatment should be: (a) sequential (b) integrated (c) only psychiatric (d) only addiction-focused

8. Enabling is best understood as: (a) malice (b) reducing immediate consequences out of care (c) a diagnosis (d) irrelevant

9. Relapse should be treated as: (a) failure (b) data for re-planning (c) grounds for discharge (d) proof of character

10. Recovery capital includes: (a) only abstinence (b) social, physical, human and cultural resources (c) only income (d) only family support

Answer key

1 b · 2 b · 3 b · 4 b · 5 b · 6 b · 7 b · 8 b · 9 b · 10 b

Short answer

1. Compare the disease and systemic models of addiction and name one clinical implication of each.
2. Map the six stations of the family cycle and give one intervention at each.
3. What are OARS, and what is the recommended ratio of reflections to questions?
4. Describe the daily recovery plan in behavioural couples therapy.
5. Name the three goals of CRAFT.

Essay (choose one, 2,000 words)

1. "The family is neither the cause nor the cure, but it is the context." Discuss with a case.
2. Critically evaluate the concept of co-dependency and its gendered assumptions.
3. Design a family-inclusive service for adolescents with substance use problems in a Nigerian city.

Further reading

- Miller, W. R., & Rollnick, S. (2013). Motivational Interviewing (3rd ed.). Guilford.
- O'Farrell, T. J., & Fals-Stewart, W. (2006). Behavioral Couples Therapy for Alcoholism and Drug Abuse. Guilford.
- Smith, J. E., & Meyers, R. J. (2004). Motivating Substance Abusers to Enter Treatment: Working with Family Members. Guilford.
- Marlatt, G. A., & Donovan, D. M. (2005). Relapse Prevention (2nd ed.). Guilford.
- Cloud, W., & Granfield, R. (2008). Conceptualizing recovery capital. Substance Use & Misuse, 43(12–13).

Next: MFT 714 — Trauma-Informed Care

MSC MARRIAGE AND FAMILY THERAPY

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MFT 714

Trauma-Informed Care

15 teaching weeks · 1 practical · 5 figures

SEMESTER TWO

1.71 units · Prerequisite: MFT 712

> “Trauma changes the body, the memory, the attachment system and the family.”

One of 28 self-contained course books in the MSc Marriage and Family Therapy lecture library: a full taught semester per volume, with cases, practicals, assessment briefs, glossaries and model answers.

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MFT 714 — Trauma-Informed Care

MSc Marriage and Family Therapy · Semester Two · 1.71 Units · 15 teaching weeks · Prerequisite: MFT 712

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Course leader's summary. Trauma changes the body, the memory, the attachment system and the family. This course teaches the physiology, the phased treatment models, and the organisational stance — and it teaches you to avoid the two commonest errors: doing too much too soon, and doing nothing at all.

How to use this book

This is the most personally demanding book in the programme. Announce it in advance to your peers; know your own history; use supervision; and do not read the trauma chapters late at night. The material is presented in the order treatment unfolds: safety, then processing, then integration.

1. Course description

Trauma-informed care means organising practice around the prevalence and impact of trauma; **trauma- specific** care means delivering interventions designed to treat it. You need both.

The course covers: definitions and types of trauma; the Adverse Childhood Experiences evidence; the neurobiology of threat and regulation; dissociation; attachment and developmental trauma; the phased model; specific modalities (TF-CBT, EMDR, narrative exposure, somatic approaches); family and couple work with trauma; intergenerational transmission; re-traumatisation in services; and practitioner self-protection.

2. Learning outcomes

#	Outcome	Assessed
LO1	Define trauma and distinguish types, including complex and developmental trauma	Exam
LO2	Explain the neurobiology of threat, regulation and memory	Exam
LO3	Conduct a trauma-informed assessment without re-traumatising	Observed assessment
LO4	Apply a phased treatment model and justify the sequencing	Case study
LO5	Use at least two trauma-specific interventions under supervision	Skills test
LO6	Work systemically with trauma in couples and families	Case study
LO7	Design trauma-informed organisational practices	Report

3. Assessment

Component	Weight	Due
Trauma-informed assessment observed (40 min)	25%	Week 7

Component	Weight	Due
Phased treatment plan (2,500 words)	35%	Week 12
Trauma-informed service audit report (2,000 words)	20%	Week 14
Examination	20%	Week 16

4. The fifteen-week plan

Week	Topic	Practical
1	What trauma is: single, complex, developmental, collective	Typology exercise
2	The ACE evidence and its limits	ACE discussion (with care)
3	Neurobiology I: threat, arousal and the window of tolerance	Regulation skills lab
4	Neurobiology II: memory, dissociation and the body	Grounding practice
5	Attachment and developmental trauma	Attachment-informed formulation
6	Trauma-informed assessment	Structured interview
7	**Observed assessment**	Assessed
8	The phased model: safety, processing, integration	Phase planning
9	Trauma-focused CBT with children and families	TF-CBT component practice
10	EMDR, narrative exposure and somatic approaches	Comparison and critique
11	Couples and trauma: betrayal, violence, and the repair of trust	Couple session design
12	**Treatment plan due**; intergenerational transmission	Genogram of trauma
13	Re-traumatisation in services; the four R's	Service audit
14	**Service audit due**; vicarious trauma and supervision	Personal resilience plan
15	Integration: what trauma-informed care is not	Case presentations

Week 1 — What trauma is

Definition (DSM-5/ICD-11, paraphrased). Exposure to actual or threatened death, serious injury or sexual violence, whether experienced directly, witnessed, learned about in relation to a close person, or through repeated occupational exposure.

Types.

Type	Description	Example
Single-incident	One event, adult onset	Road traffic collision, armed robbery
Complex	Repeated, prolonged, interpersonal, inescapable	Domestic abuse, childhood abuse

Type	Description	Example
Developmental	Occurs during maturation; shapes personality and attachment	Chronic neglect, chaotic caregiving
Collective	Shared by a community	War, displacement, massacre, epidemic
Vicarious / secondary	Exposure through another's trauma	Therapist, first responder, family carer

What makes an event traumatic is not only the event. It is the event plus the meaning, the interpersonal context (was it caused by someone who was supposed to protect you?), the aftermath (was the person believed? supported?), and prior vulnerability.

Complex PTSD (ICD-11) comprises three additional clusters beyond classic PTSD: affect dysregulation, negative self-concept, and disturbances in relationships. This matters clinically: classic PTSD responds well to exposure-based treatment; complex PTSD usually requires a longer stabilisation phase.

> **PRACTICAL**
> Classify five disguised cases by type, and note which additional features each has.

Week 2 — The ACE evidence

The original study. Adverse Childhood Experiences (abuse, neglect, household dysfunction) were found to have a graded relationship with later health outcomes, including depression, addiction, suicide attempts, heart disease and early death.

What it established. Early adversity is common, cumulative, and biologically embedded. It shifted the field from asking "what is wrong with you?" to "what happened to you?"

What it does not establish. That ACE scores are destiny; that any individual's future is determined; or that a high score is a diagnosis. Roughly a third of people with high ACE scores have no significant impairment — protective relationships matter enormously.

The clinical use. ACE enquiry is a conversation, not a score: "Was there anything difficult in your childhood that we should know about? You do not have to give details."

> **CAUTION**
> Asking about adversity can activate distress. Ask when you have time to respond, offer a choice, and have a grounding plan ready.

Week 3 — Neurobiology I: the window of tolerance

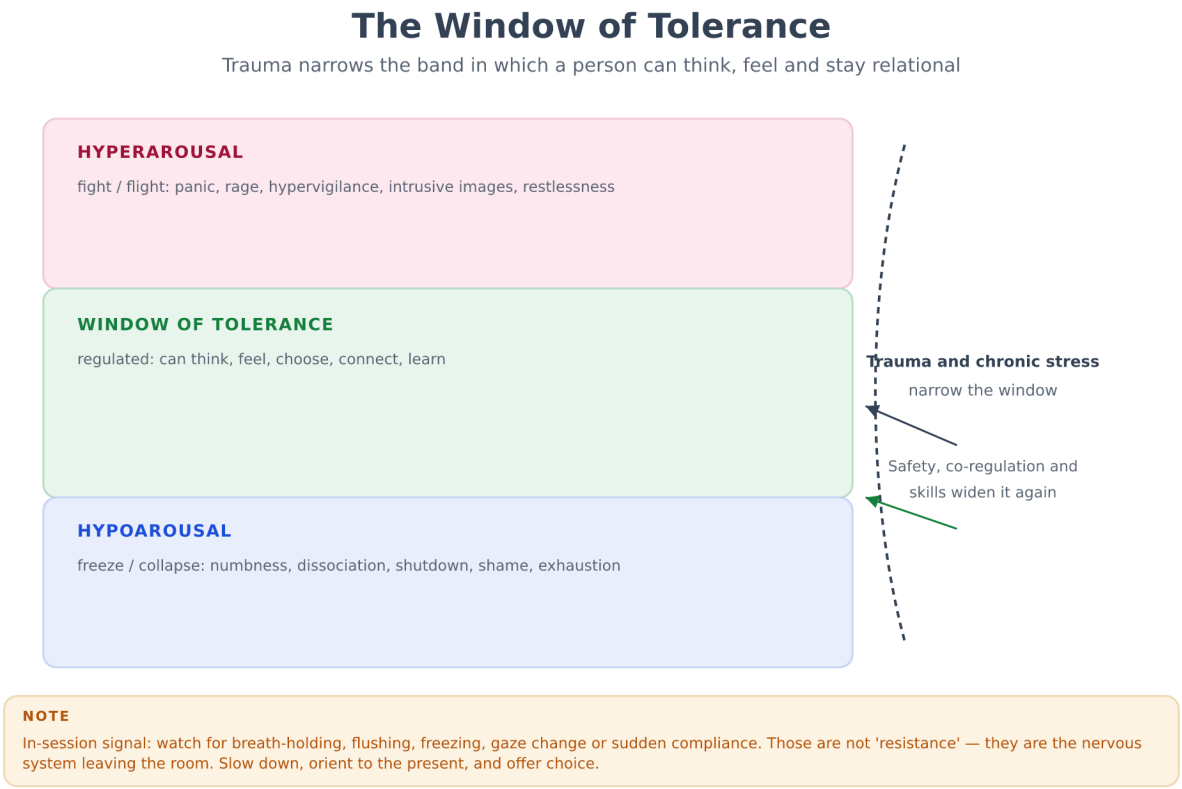


Figure 3.1 — The window of tolerance

The three states.

- **Within the window:** the person can think, feel, choose, connect and learn.
- **Hyperarousal:** fight/flight — panic, rage, hypervigilance, intrusive images, restlessness.
- **Hypoarousal:** freeze/collapse — numbness, dissociation, shutdown, shame, exhaustion.

Trauma narrows the window. Small triggers produce large reactions; the person's capacity to stay relational shrinks. This is not "over-reacting"; it is a nervous system doing what it learned.

Regulation skills to teach (and use in session).

1. **Paced breathing** — exhale longer than inhale (4 in, 6 out) for 90 seconds.
2. **Orienting** — look around the room and name five things you can see.
3. **Grounding** — feet on the floor, weight in the chair, describe the texture of an object.
4. **Movement** — stand, walk, stretch; discharge the mobilised energy.
5. **Temperature and touch** — cold water, a heavy blanket, pressure.
6. **Naming** — "I am having a memory, not the event."

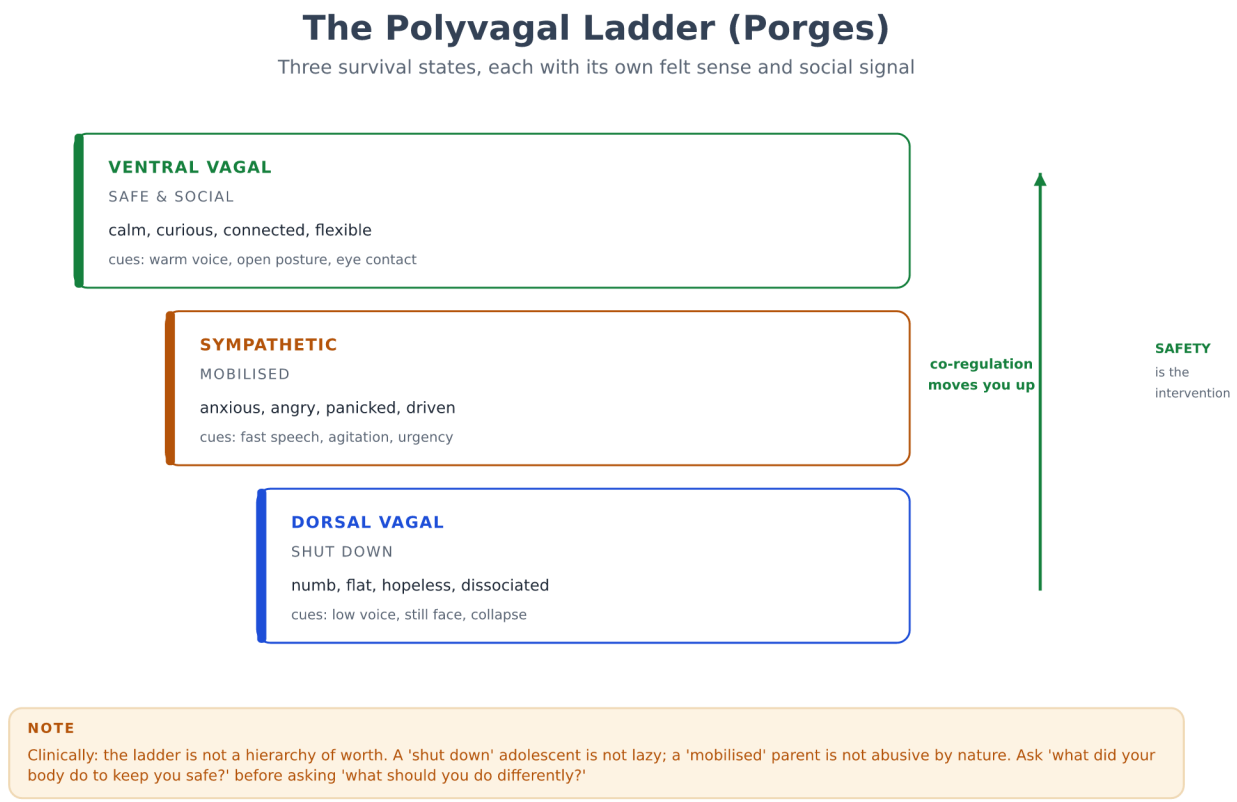


Figure 3.2 — The polyvagal ladder

In family sessions. If one member leaves the window, the session has ended for them. Pause, regulate, and return. Teaching a couple to recognise each other's early signals is itself an intervention.

Week 4 — Neurobiology II: memory, dissociation, the body

Traumatic memory is different. It is often implicit, sensory and fragmented: smells, images, body sensations, without a coherent narrative or a sense of "pastness". This is why "just talk about it" can flood a person and why containment matters.

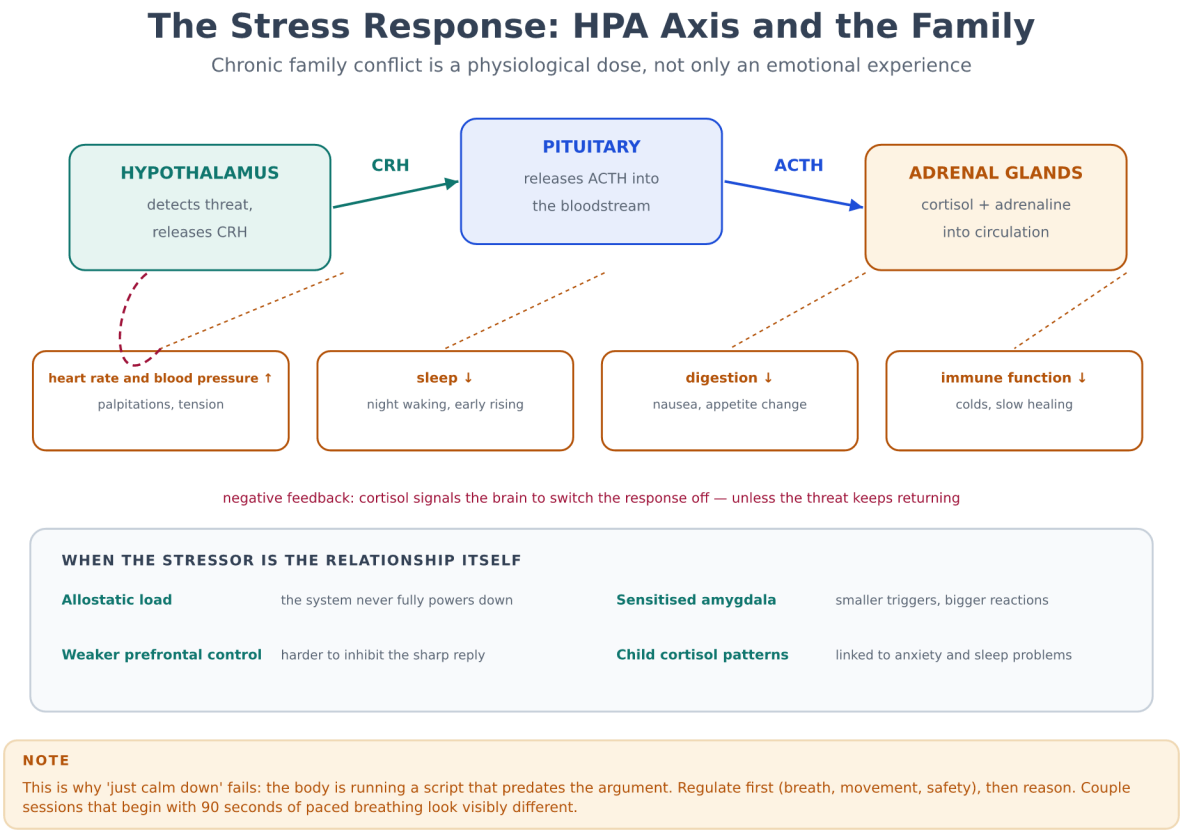


Figure 4.1 — The HPA axis and chronic stress

Dissociation — a disruption of consciousness, memory, identity or perception. Mild forms are common (going blank, feeling unreal); severe forms include depersonalisation, derealisation and dissociative amnesia. Assess gently: "Do you ever feel like you are watching yourself from outside? Do you lose time?"

The clinical rule. Do not interpret a dissociative response as resistance. If a client's eyes glaze, stop the exercise, orient them to the room, and slow down. Then shorten the next segment.

Week 5 — Attachment and developmental trauma

Developmental trauma (sometimes "complex relational trauma") occurs when the person who should provide safety is the source of danger. The child adapts with strategies that later look like personality: vigilance, people-pleasing, aggression, emotional shutdown, compulsive caregiving.

The internal working model. "I am unlovable; others are dangerous or unreliable; I must manage alone (or never let go)." These beliefs are not irrational; they were accurate in the original environment.

Clinical implications.

1. Expect the therapeutic relationship to be tested — idealisation, devaluation, testing, sudden withdrawal.
2. Predictability and reliability matter more than technique.
3. Repair after rupture is the treatment, not a distraction from it.
4. Do not interpret the transference early; be the relationship first.

Family application. Parents with their own unresolved trauma often misread their children's ordinary behaviour as threat. Psychoeducation about trauma and parenting can reduce harsh discipline dramatically.

Week 6 — Trauma-informed assessment

The four R's (SAMHSA).

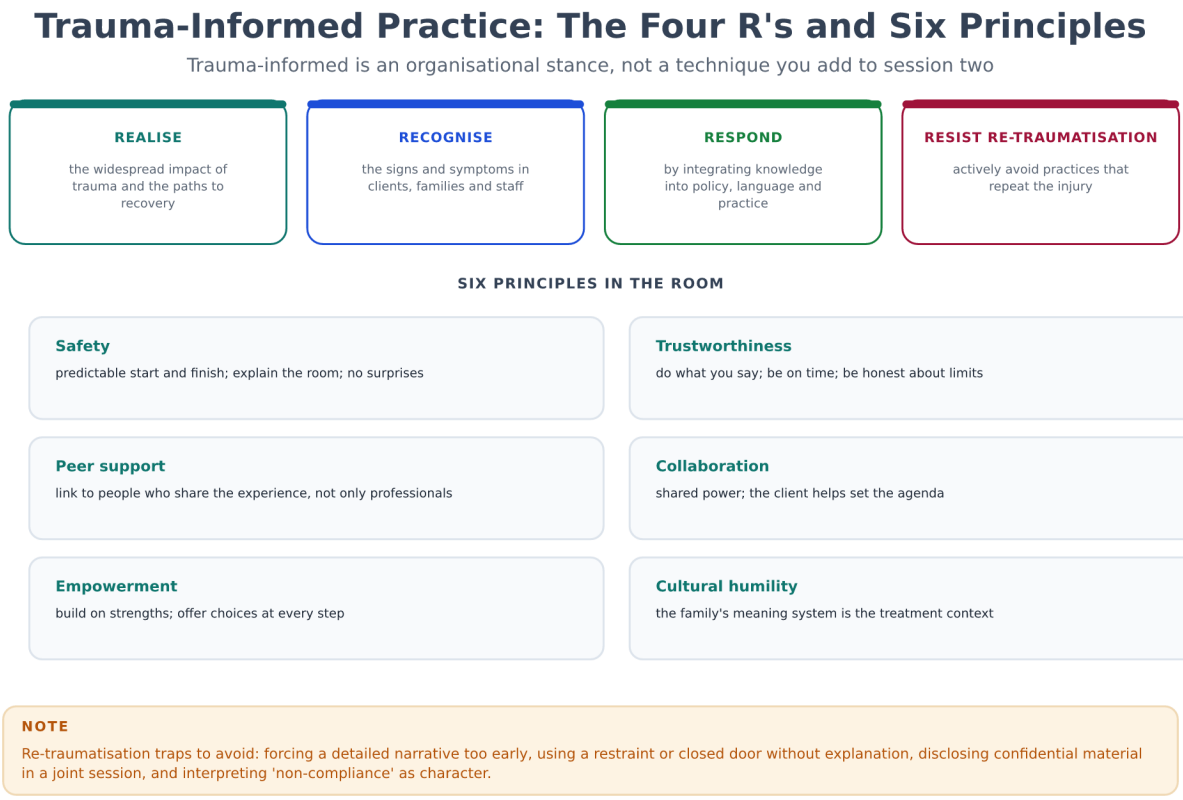


Figure 6.1 — The four R's and six principles of trauma-informed practice

1. **Realise** the widespread impact of trauma.
2. **Recognise** the signs and symptoms.
3. **Respond** by integrating knowledge into practice and policy.
4. **Resist re-traumatisation.**

Assessment principles.

- Explain why you are asking, and offer choices ("you can skip any question").

- Ask about the event only at the level of detail needed for formulation; detail belongs in processing.
- Ask about **strengths, survival strategies and meaning**, not only symptoms.
- Screen for dissociation before any trauma-focused work.
- Screen for current safety (violence, self-harm, substance use).

Instruments. PCL-5 (PTSD symptoms), IES-R, CTQ (childhood trauma; use with care), DES-II (dissociation), PHQ-9, AUDIT. Always follow up a positive screen with a conversation.

Week 7 — Observed trauma-informed assessment

Forty minutes. Assessed on: safety and choice throughout, pacing, grounding as needed, formulation without premature interpretation, and accurate risk screening.

Week 8 — The phased model

Phase 1: Safety and stabilisation. Establish safety, build the alliance, teach regulation, address practical dangers (housing, violence, substance use), and develop what Herman called "the capacity to contain".

Phase 2: Processing. Work through the traumatic material using a validated method (TF-CBT, EMDR, narrative exposure, prolonged exposure) — only when the person can tolerate it and has stabilisation skills.

Phase 3: Integration and reconnection. Rebuild identity, relationships, work, and meaning. This phase is often the longest and the most neglected.

Why sequence matters. Processing without stabilisation produces flooding, drop-out, and sometimes retraumatisation. Stabilisation without processing can produce a stable but constrained life. The judgment is clinical, not clerical: some people need two sessions of stabilisation; some need twenty.

Family application. The family's safety and stability is Phase 1 at the system level. You cannot do trauma processing in a household where the perpetrator is still present.

Week 9 — Trauma-focused CBT with children

TF-CBT (Cohen, Mannarino & Deblinger) is the best-evidenced treatment for childhood trauma. Its PRACTICE components:

Component	Content
Psychoeducation and parenting skills	Trauma responses; consistent, non-punitive discipline
Relaxation	Breathing, muscle relaxation, grounding
Affect expression and modulation	Feelings identification, coping

Component	Content
Cognitive processing	Correcting trauma-related beliefs ("it was my fault")
Trauma narrative	Gradual, structured account in the child's medium (book, drawing)
In vivo mastery	Facing avoided, safe situations
Conjoint sessions	Child shares the narrative with the caregiver
Enhancing safety	Safety skills and planning for the future

The caregiver's role is essential. The non-offending caregiver's belief, protection and support is the strongest predictor of recovery. Work with them in parallel.

Week 10 — Other trauma-specific modalities

Modality	Mechanism	Evidence	Cautions
EMDR	Bilateral stimulation during memory recall	Strong for PTSD	Requires training; not a magic technique
Narrative exposure therapy	Chronological testimony of the whole life	Strong for multiple/collective trauma	Can be destabilising without stabilisation
Prolonged exposure	Repeated imaginal and in vivo exposure	Strong	Drop-out; requires strong alliance
Somatic approaches	Body-based regulation and discharge	Emerging evidence	Beware of unfounded claims
Yoga / mindful movement	Regulation, interoception	Good adjunctive evidence	Not a substitute for trauma processing

The professional duty. Use what you are trained and supervised to use. A half-learned trauma protocol is more dangerous than a well-delivered stabilisation phase.

Week 11 — Couples and trauma

Three presentations.

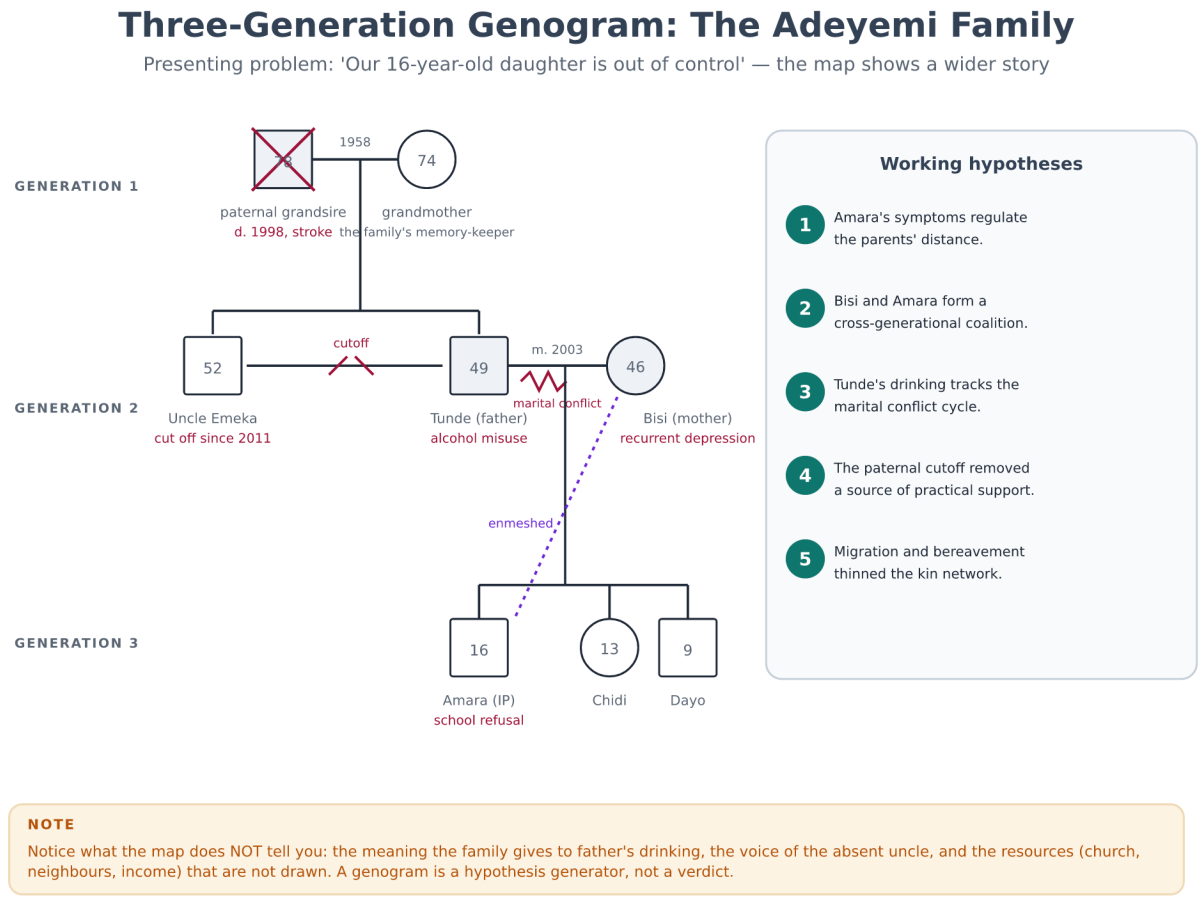
1. **One partner has a trauma history.** The other becomes a de facto carer; intimacy becomes a trigger; the couple's sexual relationship collapses. Work with both: psychoeducation for the partner, boundaries for the survivor, and a paced intimacy plan.
2. **The trauma happened inside the relationship** (betrayal, violence, coercion). This is betrayal trauma. Safety and accountability come first; joint work only where the offending partner acknowledges the harm and the victim chooses it.
3. **Both partners are trauma survivors.** Two dysregulated nervous systems in one room. The pace must be slower, the structure tighter, and the regulation explicit.

The couple-level formulation. "When she freezes, he experiences rejection and pursues; when he pursues, she freezes harder. The loop is a collision of two survival strategies, not a failure of love."

Week 12 — Intergenerational transmission

Mechanisms. Parenting behaviour (frightening, inconsistent, harsh), attachment patterns, family narrative, epigenetic and stress-physiology effects, and structural factors (poverty, displacement, community violence).

The genogram of trauma. Draw three generations and mark losses, violence, separation, migration, untimely deaths and secrets. Then ask: what was never spoken about? What did each generation have to survive?



NOTE

Notice what the map does NOT tell you: the meaning the family gives to father's drinking, the voice of the absent uncle, and the resources (church, neighbours, income) that are not drawn. A genogram is a hypothesis generator, not a verdict.

Figure 12.1 — A three-generation genogram as a map of transmission

The hopeful finding. Transmission is probabilistic, not deterministic. A consistent, protective relationship at any point breaks the chain. Your therapy may be that relationship.

Week 13 — Re-traumatisation in services

How services re-traumatise, usually without meaning to.

Practice	Why it harms	Alternative
Restraint without explanation	Replicates powerlessness	Explain, offer choices, debrief
Closed doors, no warning	Replicates entrapment	Predictability, warnings, exits
Being asked to repeat the story to every professional	Replicates helplessness	Record once, share with consent
Long waits in public areas	Replicates exposure	Private, quiet space
Disclosing in front of a partner	Replicates danger	Always screen individually
"Non-compliance" language	Replicates blame	"What got in the way?"

The organisational standard. Trauma-informed care is a policy and management issue: staff training, physical environment, restraint policy, complaint routes, and supervision. A trauma-informed clinician in a trauma-organised service will burn out.

Week 14 — Vicarious trauma

Distinguish three concepts. Burnout (exhaustion from workload), compassion fatigue (reduced empathy), and vicarious trauma (changes to the therapist's beliefs and inner world from exposure to others' trauma).

Signs. Intrusion of clients' material, nightmares, cynicism, loss of hope, changed worldview ("the world is unsafe"), avoidance of trauma cases, over-involvement.

Protection. Supervision that addresses the work's emotional impact; caseload diversity; clear boundaries; your own therapy; sleep, exercise and relationships; and acknowledging meaning — many clinicians find that sustained trauma work deepens their sense of purpose when it is properly supported.

Week 15 — What trauma-informed care is not

It is not the avoidance of difficult material. It is not an excuse for never processing trauma. It is not a synonym for "be gentle". It is a disciplined sequencing of safety, processing and integration, delivered in a relationship and an organisation that do not repeat the injury.

Glossary

Adverse childhood experiences (ACEs) — potentially traumatic events in childhood, measured cumulatively. **Betrayal trauma** — trauma caused by someone on whom the person depends. **Complex PTSD** — PTSD plus affect dysregulation, negative self-concept and relational disturbance. **Dissociation** — disruption of consciousness, memory, identity or perception. **Grounding** — techniques that return attention to the present and the body. **Re-traumatisation** — repetition of the dynamics of the original trauma in the therapy or service. **Stabilisation** — the first phase of trauma treatment, focused on safety and regulation. **Window of tolerance** — the arousal range within which a person can think and connect.

Self-assessment

Multiple choice

1. Complex PTSD is distinguished by: (a) one event (b) affect dysregulation, negative self-concept, relational disturbance (c) psychosis (d) amnesia only

2. The first phase of trauma treatment is: (a) processing (b) safety and stabilisation (c) integration (d) exposure

3. Dissociation should first be: (a) interpreted (b) assessed and accommodated (c) ignored (d) challenged

4. TF-CBT's trauma narrative is: (a) forced in session one (b) gradual and structured, in the child's medium (c) written by the therapist (d) optional and never used

5. The strongest predictor of recovery in childhood trauma is: (a) the trauma's severity (b) a believing, protective caregiver (c) IQ (d) medication

6. Vicarious trauma is: (a) the same as burnout (b) changes to the therapist's beliefs and inner world from exposure (c) a diagnosis (d) avoidable by holidays

7. Re-traumatisation in services often occurs through: (a) privacy (b) predictability (c) restraint without explanation (d) choice

8. Processing trauma without stabilisation risks: (a) faster recovery (b) flooding and drop-out (c) improved alliance (d) no effect

9. Intergenerational transmission is: (a) deterministic (b) probabilistic and can be interrupted (c) genetic only (d) irrelevant

10. A trauma-informed organisation is characterised by: (a) only individual therapy (b) policy, environment, training and supervision aligned with trauma knowledge (c) shorter sessions (d) avoiding trauma discussion

Answer key

1 b · 2 b · 3 b · 4 b · 5 b · 6 b · 7 c · 8 b · 9 b · 10 b

Short answer

1. Distinguish complex PTSD from classic PTSD.
2. Describe three regulation skills and when you would use each.
3. Name the four R's of trauma-informed care.
4. Give the three phases of trauma treatment with the task of each.
5. List four ways services re-traumatise clients and one alternative for each.

Essay (choose one, 2,000 words)

1. "Trauma-informed care is a stance, not a technique." Discuss with reference to your own practice.
2. Design a trauma-informed family service: policies, environment, staffing and clinical pathway.
3. Analyse the challenges of trauma work with a couple where the trauma occurred inside the relationship.

Further reading

- Herman, J. L. (1992/2015). Trauma and Recovery. Basic Books.
- van der Kolk, B. (2014). The Body Keeps the Score. Viking.
- Cohen, J. A., Mannarino, A. P., & Deblinger, E. (2017). Treating Trauma and Traumatic Grief in Children and Adolescents (2nd ed.). Guilford.
- Porges, S. W. (2011). The Polyvagal Theory. Norton.
- SAMHSA. (2014). SAMHSA's Concept of Trauma and Guidance for a Trauma-Informed Approach.
- Felitti, V. J. et al. (1998). Relationship of childhood abuse and household dysfunction to many of the leading causes of death in adults. American Journal of Preventive Medicine, 14(4), 245–258.

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This volume is one of 28 course books that together cover the whole MSc Marriage and Family Therapy taught programme. Each book is written to carry a 14–16 week semester on its own: every week pairs a core idea with the practice that follows from it, a worked case, and a reflective task, and each book closes with an integration chapter, a glossary and a self-assessment paper with model answers.

The books are designed to be read straight through before the semester begins, revisited week by week, and used again in revision. Cross-references point to the other volumes where a topic is treated in more depth.

How to cite this volume

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A note on the case material

All families, couples and individuals in this book are fictional composites. They are written to teach pattern recognition, not to describe any real person. Nothing here replaces supervised clinical practice, professional registration, legal advice or medical care; legal content is orientation only and must be checked against the law of the relevant jurisdiction.

The lecture library

Twenty-eight self-contained course books carry the taught programme: eight in Semester One, seven in Semester Two, seven in Semester Three including the thesis, and six electives. The volume you are holding is marked in the margin.

Code	Course	Semester	Units
MFT 701	Fundamentals of Marriage and Family Therapy	One	1.71
MFT 702	Theories of Family Therapy	One	1.71
MFT 703	Human Development Across the Lifespan	One	1.71
MFT 704	Ethical and Legal Issues in Therapy	One	1.71
MFT 705	Research Methods in Marriage and Family Therapy	One	1.71
MFT 706	Clinical Practice and Techniques	One	1.71
MFT 707	Assessment and Diagnosis in Family Therapy	One	1.71
MFT 708	Cultural Competence in Therapy	One	1.71
MFT 709	Group Therapy Techniques	Two	1.71
MFT 710	Couples Therapy	Two	1.71
MFT 711	Child and Adolescent Therapy	Two	1.71
MFT 712	Crisis Intervention Strategies	Two	1.71
MFT 713	Substance Abuse and Family Dynamics	Two	1.71
MFT 714	Trauma-Informed Care	Two	1.71
MFT 715	**Family Systems Theory**	Two	1.71
MFT 716	Communication Skills in Therapy	Three	1.71
MFT 717	Psychopathology in Families	Three	1.71
MFT 718	Intimacy and Relationship Dynamics	Three	1.71
MFT 719	Family Life Cycle and Transitions	Three	1.71
MFT 720	Supervision and Consultation in Therapy	Three	1.71
MFT 721	Research Methodology	Three	1.71
MFT 722	Professional Development and Self-Care	Electives	1.71
MFT 723	Special Populations in Therapy	Electives	1.71
MFT 724	Family Law and Advocacy	Electives	1.71
MFT 725	Integrative Approaches in Therapy	Electives	1.71
MFT 726	Neurobiology of Relationships	Electives	1.71
MFT 727	Play Therapy Techniques	Electives	1.71
MFT 770	Master Thesis	Three	1.71

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Course leader's summary. This is the theoretical anchor of the programme. Where MFT 702 surveys schools, this course interrogates the theory beneath them: cybernetics, general systems thinking, structure, communication, constructionism and their critics. You will finish able to explain precisely what you mean when you say a family is "a system".

How to use this book

The word "system" is used loosely in clinical settings. This course tightens it. Each week takes one theoretical tradition, states its claims precisely, applies it to a case, and then asks what it cannot explain. You are expected to leave with arguments, not slogans.

1. Course description

Family systems theory is not one theory but a family of theories sharing a commitment: **the unit of analysis is the relationship, and the whole is more than the sum of its parts**. The course traces that commitment from von Bertalanffy's general systems theory and the Macy conferences on cybernetics, through the Palo Alto communication school, Minuchin's structuralism, Bowen's multigenerational theory, and the Milan group, to social constructionism, critical and feminist theory, and contemporary complexity and network perspectives.

2. Learning outcomes

#	Outcome	Assessed
LO1	Explain the core propositions of general systems theory and cybernetics	Exam
LO2	Analyse a family using at least four theoretical traditions	Case analysis
LO3	Critique each tradition, including its cultural and political assumptions	Essay
LO4	Distinguish first- and second-order cybernetics and demonstrate reflexivity	Viva
LO5	Apply constructionist ideas to meaning-making in a family	Case analysis
LO6	Evaluate the evidence for systemic propositions	Exam
LO7	Construct a coherent theoretical position of your own	Position paper

3. Assessment

Component	Weight	Due
Theoretical analysis of a case using four traditions (2,500 words)	35%	Week 9
Essay: critique of a systemic concept (2,000 words)	25%	Week 13
Examination	25%	Week 16

Component	Weight	Due
Position paper and viva (1,000 words)	15%	Week 15

4. The fifteen-week plan

Week	Topic	Practical
1	The systemic turn: what it was and what it answered	Compare two accounts of one case
2	General systems theory: wholes, boundaries, equifinality	Systems vocabulary drill
3	Cybernetics I: feedback, control, homeostasis	Feedback mapping
4	Cybernetics II: first and second order; reflexivity	Reflexivity exercise
5	Communication theory: axioms, punctuation, paradox	Transcript analysis
6	Structural theory: organisation, hierarchy, boundaries	Structural mapping
7	Multigenerational theory: anxiety, triangles, differentiation	Triangle mapping
8	Milan and meaning: beliefs, premises, circularity	Circular questioning
9	**Theoretical analysis due**; constructivism and social constructionism	Meaning-making interview
10	Postmodern and narrative theory: knowledge and power	Texts and stories exercise
11	Critical, feminist and anti-racist systems theory	Power audit of a model
12	Attachment and neuroscience meet systems theory	Integration diagram
13	**Essay due**; critiques and limitations of systems thinking	Debate
14	Complexity, networks and the contemporary field	Network map of a family
15	Your theoretical position	Viva

Week 1 — The systemic turn

What changed. Before the systemic turn, the family was understood as the environment of an individual patient. After it, the family became the unit of treatment, and individual symptoms became functions of relationship patterns.

What it answered. Three problems the individual model struggled with: Why does a symptom appear in a particular child? Why does the family resist change (homeostasis)? Why does the same individual behave differently in different relationships?

What it risked. Dissolving the person into the system; treating "the family" as a homogeneous actor; ignoring power, gender, class and history; and producing descriptions that could not be falsified.

PRACTICAL

Take one case. Write it twice: once as an individual formulation (as a CBT or psychodynamic clinician would) and once as a systemic one. Then write a paragraph on what each version makes visible and invisible.

Week 2 — General systems theory

Von Bertalanffy's propositions, translated for clinicians:

Proposition	Clinical meaning
A system is a set of elements in interaction	The family is defined by its interactions, not its members
The whole is more than the sum of its parts	The family has properties no member has alone
Systems are open or closed to their environment	A family that refuses outside contact becomes closed and rigid
Systems have boundaries	Who is inside, who is outside, and how permeable
Systems are hierarchical	Subsystems nested within larger systems
Equifinality	Different starting points can reach the same end state
Equipotentiality	Similar starting points can reach different end states
Negative entropy	Systems import energy to counter disorder (information, support)

Equifinality and equipotentiality are the most useful and most neglected. Equifinality explains why two very different families produce the same symptom; equipotentiality explains why two similar families produce very different outcomes. Together they demolish simple causal stories.

Example. Two children are referred for school refusal: one in a family with high conflict and one in a family with high achievement pressure. Equifinality explains the identical presentation; equipotentiality explains why one family recovers in four sessions and the other takes two years.

Week 3 — Cybernetics I

Feedback. A system regulates itself by monitoring its own output and adjusting its input.

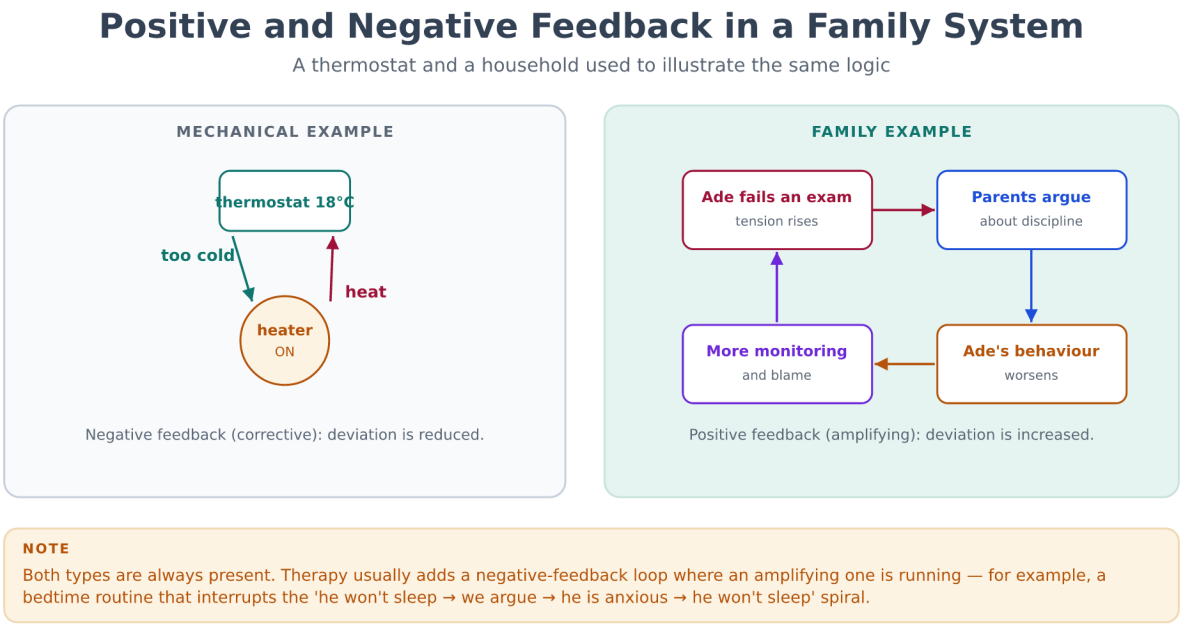


Figure 3.1 — Negative and positive feedback in a machine and in a family

- **Negative feedback** corrects deviation: the thermostat, the bedtime routine, the family that de-escalates when voices rise.
- **Positive feedback** amplifies deviation: the microphone screech, the escalation, the panic spiral.

Homeostasis. The system's tendency to maintain its organisation. **Morphostasis** maintains; **morphogenesis** changes. A family can be stably unhappy: the symptom is one of the mechanisms that holds the system together.

The clinical insight. Attempts to change a system are themselves inputs. If the family's equilibrium depends on the symptom, your intervention will produce a counter-move. Predicting the counter-move — and sometimes prescribing it — is the strategic tradition's contribution.

Calibration example. A family's daughter has "tantrums" that end a 45-minute argument between her parents. The tantrum is negative feedback for the marital system (it restores calm) and positive feedback for the child (it works). Two feedback systems, one event.

> **PRACTICAL**
> Map both feedback loops in a disguised case: one for each "layer" (the couple system and the child's behaviour). Show how the same event serves both.

Week 4 — Cybernetics II: first and second order

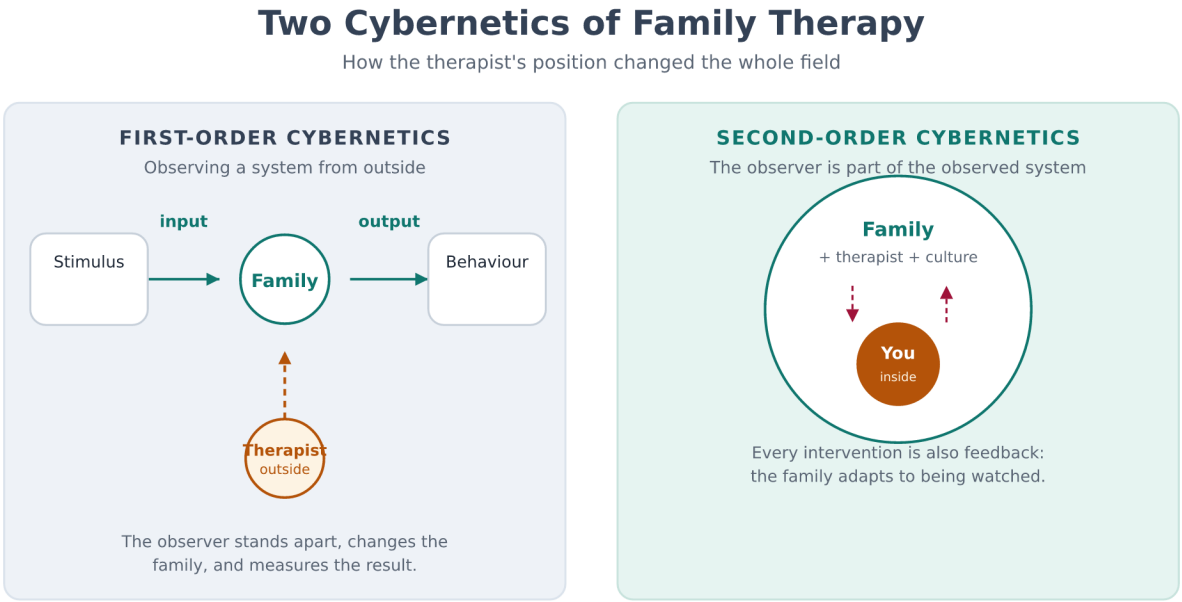


Figure 4.1 — First- and second-order cybernetics: where the therapist stands

First-order cybernetics. The observer is outside the system; interventions are inputs; the aim is control of the system's output. This produced the heroic era: directive, strategic, clever.

Second-order cybernetics. The observer is part of the observed system; distinctions are drawn by an observer; knowledge is provisional. This produced curiosity, the reflecting team, and the therapist's accountability for their own effect.

The clinical consequences.

First order	Second order
"What intervention will fix this family?"	"What am I noticing, and what am I blind to?"
The family is the object of change	The family and the therapist co-construct change
Neutrality is possible	Position is unavoidable; transparency is required
Expertise is the therapist's	Expertise is distributed

Reflexivity is not self-indulgence. It is the discipline of accounting for your own contribution to the clinical picture: which family member you find easier to like, which story you believed first, which data you stopped collecting once you had a hypothesis.

Week 5 — Communication theory



Figure 5.1 — Watzlawick's five axioms of communication

The five axioms (Watzlawick, Beavin & Jackson).

1. One cannot not communicate.
2. Every message has a content and a relationship level.
3. Communication is punctuated into sequences by participants.
4. Messages are digital (words) and analogical (analogue).
5. Relationships are symmetrical or complementary.

Double bind hypothesis (Bateson): a person receives contradictory messages at different levels, cannot comment on the contradiction, and cannot leave the relationship. The hypothesis was weakened as an explanation of schizophrenia, but its clinical descendant — recognising contradictory, un-commentable communications in families — remains useful.

Critique. Communication theory could become mechanistic (families as machines exchanging messages) and could pathologise families of people with psychosis. It also tended to describe "systems" without the power differences that determine who gets to define what is communicated.

> **PRACTICAL**
> Transcript analysis: take ten lines of a couple's argument and identify punctuation, relationship-level messages, and symmetrical/complementary moves.

Week 6 — Structural theory

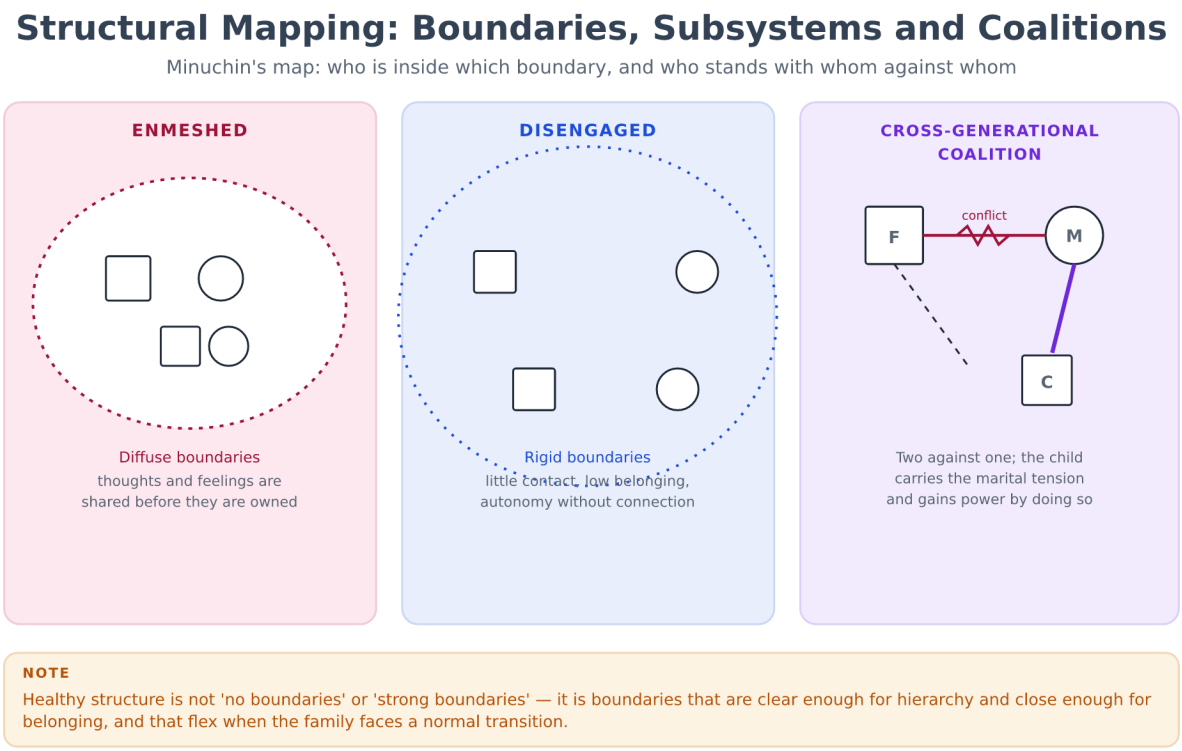


Figure 6.1 — Structural mapping: enmeshed, disengaged and cross-generational coalition

Core propositions (Minuchin).

1. The family's structure is the invisible set of rules that govern transactions.
2. The structure organises the family into subsystems (spousal, parental, sibling) separated by boundaries.
3. Symptoms are best understood as the family's response to its structure, and sometimes as the only way the structure can remain stable.
4. Change in structure produces change in experience.

The evaluation criteria for a structure. Boundaries must be clear (not diffuse, not rigid); hierarchy must be appropriate to stage and culture; and subsystems must have enough autonomy to function and enough connection to belong.

Power and structure. Structural theory was criticised for assuming a normative hierarchy (executive parents, subordinate children) and for not asking how that hierarchy is upheld. Contemporary structural practice inserts the questions: whose authority is legitimate in this family, and who bears the cost of it?

Week 7 — Multigenerational theory

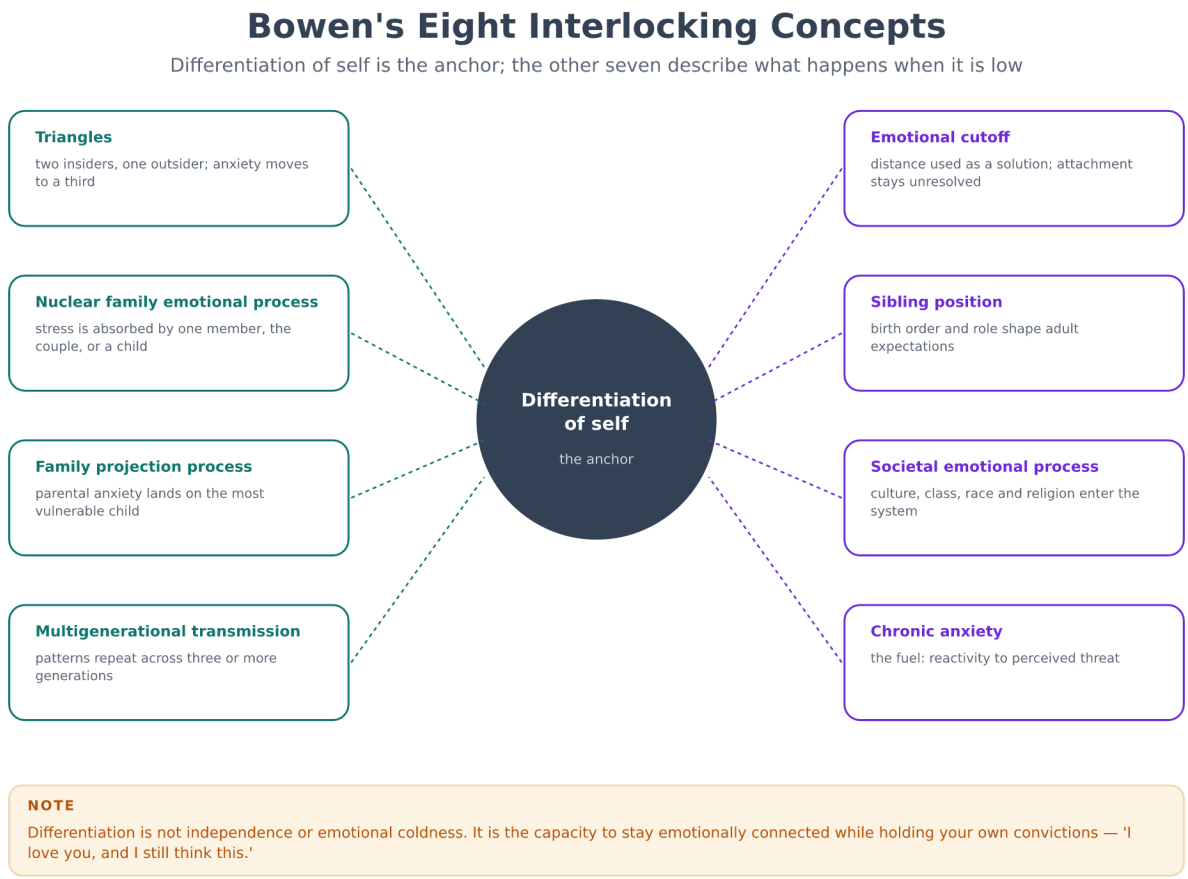


Figure 7.1 — Bowen's eight interlocking concepts

Core propositions (Bowen).

1. The family is an emotional unit; anxiety is its most important variable.
2. Differentiation of self is the capacity to remain connected while thinking independently.
3. Anxiety is managed in one of four ways: conflict, distance, projection to a child, or dysfunction in one member.
4. Triangles distribute anxiety and stabilise dyads.
5. Patterns are transmitted across generations; symptomatic children are often the most triangulated.

The theory's strength is time: it offers an account of why now, by locating the present pattern in several generations of adaptation to war, migration, loss and religion.

The theory's weakness is its insufficient account of power, material constraint and culture. A woman cannot always "differentiate" out of an arranged marriage; a family cannot always "stay out of triangles" when migration has severed the network.

Week 8 — Milan and meaning

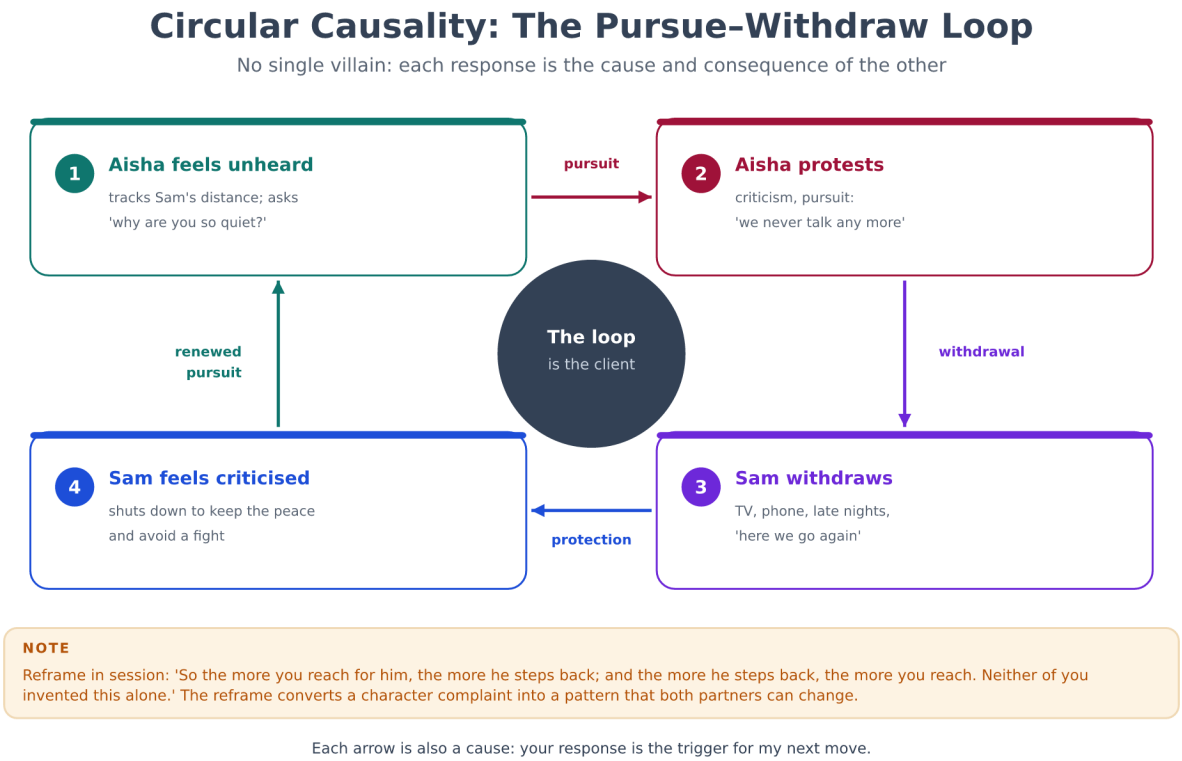


Figure 8.1 — Circular causality in the consulting room

Core propositions (Palazzoli, Boscolo, Cecchin, Prata).

1. The family is a system of meanings (premises) that organise behaviour.
2. The therapist forms hypotheses and tests them with circular questions.
3. Neutrality is a stance of curiosity towards all members, not indifference.
4. Positive connotation frames all behaviour as serving the system's coherence.
5. Interventions disturb the system by changing premises, not by demanding behaviour change.

The evolution. Classical Milan used teams, rituals and directives; later Milan (and the reflecting team) abandoned directive intervention and emphasised multiple perspectives and the therapist's own openness.

Critique. Neutrality became indefensible in cases of violence and abuse; circular questioning can become an elegant evasion; and the intellectual register excluded many families.

Week 9 — Constructivism and social constructionism

Constructivism (Maturana, von Glasersfeld): each person constructs a model of reality from their own experience; we cannot access an objective reality directly. Clinically: take each family member's account seriously, and stop trying to establish "the truth".

Social constructionism (Gergen, Shotter): meanings are produced between people, in language and in culture, and change with context. Clinically: the problem, the roles and the identities are made in talk, and so they can be made differently.

The distinction matters. Constructivism risks relativism and individualism (everything is "just your view"); social constructionism locates meaning in relationships and power, which makes it compatible with the critical and cultural traditions.

Implication for practice. The therapist's formulation is one description among several. Offering it humbly — "here is a story I am beginning to see; how does it look to you?" — is both a constructionist technique and a mark of respect.

Week 10 — Narrative and postmodern theory

Narrative (White & Epston). Stories organise experience; dominant stories make alternatives invisible; the task is to externalise the problem and thicken a preferred story.

Postmodern influences. Foucault on discourse and power-knowledge; Derrida on deconstruction. The clinical translation: question the "official" account, ask who benefits from it, and look for what it silences.

The reflexivity of narrative therapy is unusual: the therapist is "decentred but influential", and the client is the author. This is an ethical stance as much as a technique.

Critique. The linguistic emphasis can under-serve families with limited literacy or in acute crisis; the evidence base, while growing, is smaller than the cognitive-behavioural tradition's.

Week 11 — Critical, feminist and anti-racist systems theory

The feminist critique (Hare-Mustin 1978; Goldner; Walters) made four claims that changed the field:

1. "Circular" descriptions conceal power asymmetries.
2. "Neutrality" in the face of violence colludes with the perpetrator.
3. The gendered division of labour, not "the family system", is often the object of the wife's complaint.
4. Models that prize individuation encode a male norm.

Anti-racist and postcolonial theory extend the critique: whose family structures are treated as healthy? Which knowledge counts as evidence? How do services reproduce the state's relationship to marginalised families?

Clinical translation.

- Ask about power explicitly, in every assessment.

- Never use systemic language to make a victim responsible for the violence she receives.
- Place race, migration, class and religion inside the system, not outside it.

Week 12 — Attachment and neuroscience in systems theory

Two imports that have reshaped the field.

1. **Attachment theory** gives systemic thinking a developmental mechanism: the couple and parent–child systems are organised by attachment strategies, and those strategies are formed in relationships.
2. **Neuroscience** gives systemic thinking a physiology: co-regulation, the window of tolerance, mirroring and stress physiology explain how relationships change bodies.

The integration. "Systems" are not abstract patterns; they are embodied patterns between nervous systems. A couple's pursue–withdraw cycle is simultaneously a communication pattern, an attachment strategy, and a sequence of autonomic states.

Critique of the imports. Neuroscience is often oversold in clinical texts; "mirror neurons" and "the right brain" are frequently invoked without evidence. Use physiology where it is well established (stress, regulation, sleep, trauma) and keep the metaphors labelled as metaphors.

Week 13 — Critiques and limitations

Critique	Substance	Response
The system has no agent	"The family did it" hides who actually did what	Locate action; ask who decided
Power is under-theorised	Systemic descriptions can neutralise abuse	Explicit power analysis
Culture is externalised	"The system" was a Western two-parent family	Cultural and critical turns
Evidence is thin	Many systems concepts are unfalsifiable	Operationalise and test
The therapist disappears	Neutral observer stance	Second-order reflexivity
Complexity obscures action	Endless circular description delays intervention	Use the simplest map that guides action

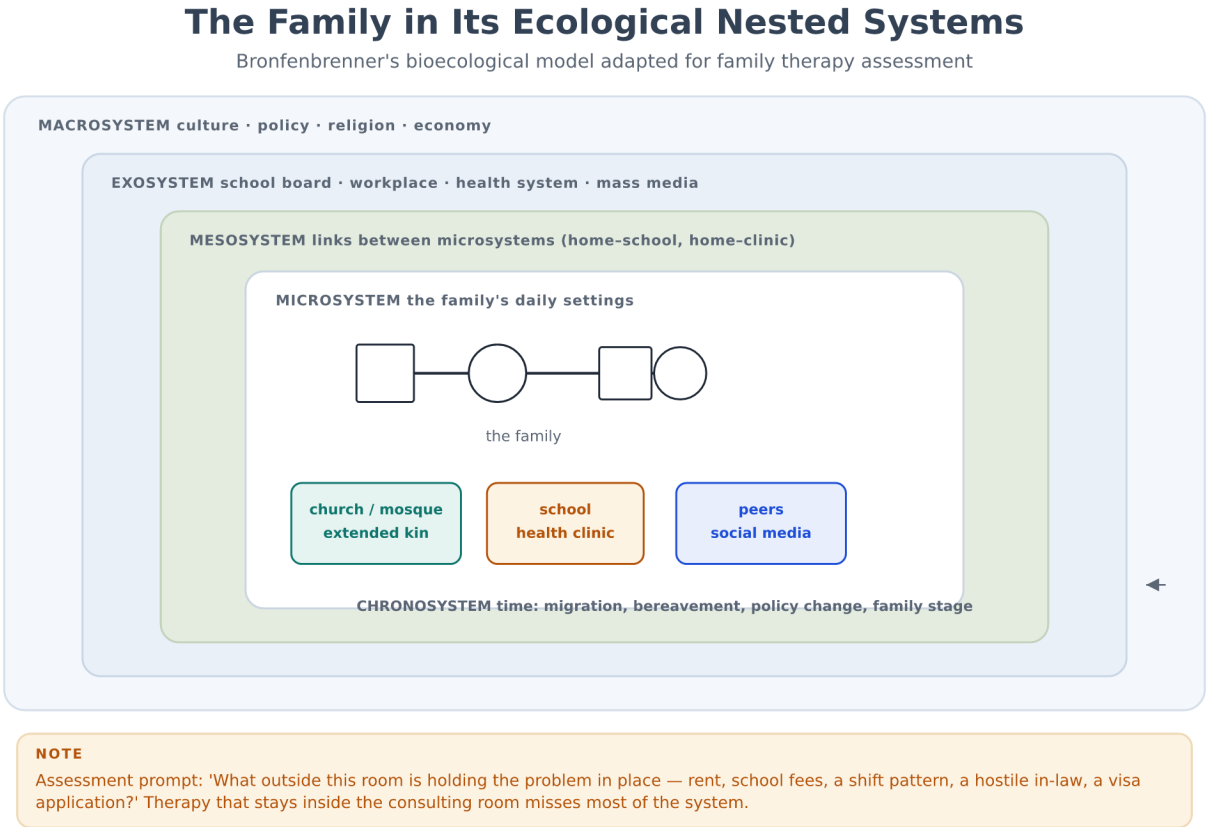
The mature position. Systems theory is a disciplinary sensibility, not a complete theory. It tells you where to look (between people, across generations, in context) and what to resist (single villains and simple causes). It does not, by itself, tell you what to do.

Week 14 — Complexity, networks and the contemporary field

Complexity theory. Families are non-linear, self-organising systems: small differences can produce large divergence; cause and effect are not proportional; and feedback loops are the rule. This fits clinical experience better than linear models, but it resists prediction.

Network thinking. Contemporary research treats families as networks of relationships, with density, centrality and clustering measurable. Who is central? Who is isolated? Who is the only link between two clusters? This is a formalised genogram, and it is improving our understanding of family structure and resilience.

Ecological models. Bronfenbrenner's nested systems (Figure 7 in MFT 701) place the family inside institutions, culture and time — a corrective to the field's tendency to treat the family as self-contained.



NOTE

Assessment prompt: 'What outside this room is holding the problem in place — rent, school fees, a shift pattern, a hostile in-law, a visa application?' Therapy that stays inside the consulting room misses most of the system.

Figure 14.1 — The family inside nested ecological systems

Week 15 — Your theoretical position

Write 1,000 words and defend it in ten minutes:

1. What is the unit of analysis you will work with, and why?
2. What do you believe about causation in families?

3. What is the therapist's position, and how will you manage your own power?
4. Which systemic concept do you find least defensible, and what do you use in its place?
5. What would make you abandon your position?

Glossary

Autopoiesis — the self-producing organisation of living systems. **Boundary** — the rules governing participation and exchange between systems. **Circularity** — reciprocal causation within a system. **Cybernetics** — the study of control and communication in systems. **Differentiation of self** — see Bowen. **Equifinality** — different paths leading to the same end state. **Equipotentiality** — similar starting points leading to different end states. **Homeostasis / morphostasis** — the maintenance of system organisation. **Morphogenesis** — change in system organisation. **Punctuation** — the marking of a sequence's beginning; the source of blame. **Reflexivity** — the therapist's accountability for their own participation. **Second-order cybernetics** — cybernetics of observing systems. **Structure** — the repetitive patterns of transaction that organise a family.

Self-assessment

Multiple choice

1. Equifinality means: (a) same cause, same outcome (b) different paths, same outcome (c) same paths, different outcomes (d) no causation

2. Homeostasis in a family refers to: (a) change (b) the maintenance of existing organisation (c) hierarchy (d) communication

3. Second-order cybernetics holds that the observer: (a) is outside the system (b) is part of the observed system (c) is irrelevant (d) is objective

4. "One cannot not communicate" is: (a) axiom 1 (b) axiom 3 (c) axiom 5 (d) a double bind

5. Punctuation is clinically important because it: (a) describes memory (b) assigns blame for a sequence (c) measures cohesion (d) is a diagnostic criterion

6. The feminist critique of family therapy argued that: (a) systems are too complex (b) circular descriptions conceal power asymmetries (c) hierarchy is unnecessary (d) therapy should be individual

7. Positive connotation is associated with: (a) structural theory (b) Milan (c) Bowen (d) behavioural therapy

8. A closed system in family terms is one that: (a) has clear boundaries (b) has limited exchange with its environment and becomes rigid (c) is disengaged (d) is enmeshed

9. Social constructionism locates meaning in: (a) the individual brain (b) relationships, language and culture (c) genetics (d) behaviour

10. The most defensible use of systems theory is as: (a) a complete predictive model (b) a disciplinary sensibility that shapes where you look (c) a diagnosis (d) a legal framework

Answer key

1 b · 2 b · 3 b · 4 a · 5 b · 6 b · 7 b · 8 b · 9 b · 10 b

Short answer

1. Define equifinality and equipotentiality and give one clinical example of each.
2. Distinguish negative and positive feedback with a family example.
3. What are the three criteria for evaluating a family's structure?
4. State the feminist critique of "circular" formulations in four sentences.
5. What is reflexivity, and why is it a clinical skill?

Essay (choose one, 2,000 words)

1. "Systems theory tells you where to look but not what to do." Discuss.
2. Critically evaluate the concept of homeostasis in families where violence is present.
3. Compare Bowen's and Minuchin's accounts of how problems develop, and assess which is better supported.

Further reading

- von Bertalanffy, L. (1968). General System Theory. Braziller.
- Watzlawick, P., Beavin, J., & Jackson, D. (1967). Pragmatics of Human Communication. Norton.
- Minuchin, S. (1974). Families and Family Therapy. Harvard University Press.
- Bowen, M. (1978). Family Therapy in Clinical Practice. Aronson.
- Becvar, D. S., & Becvar, R. J. (2018). Family Therapy: A Systemic Integration (9th ed.). Pearson.
- Gergen, K. J. (2009). An Invitation to Social Construction (2nd ed.). Sage.
- Hare-Mustin, R. T. (1978). A feminist approach to family therapy. Family Process, 17(2), 181–194.

End of Semester Two. Next: MFT 716 — Communication Skills in Therapy

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MFT 716 — Communication Skills in Therapy

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This volume is one of 28 course books that together cover the whole MSc Marriage and Family Therapy taught programme. Each book is written to carry a 14–16 week semester on its own: every week pairs a core idea with the practice that follows from it, a worked case, and a reflective task, and each book closes with an integration chapter, a glossary and a self-assessment paper with model answers.

The books are designed to be read straight through before the semester begins, revisited week by week, and used again in revision. Cross-references point to the other volumes where a topic is treated in more depth.

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A note on the case material

All families, couples and individuals in this book are fictional composites. They are written to teach pattern recognition, not to describe any real person. Nothing here replaces supervised clinical practice, professional registration, legal advice or medical care; legal content is orientation only and must be checked against the law of the relevant jurisdiction.

The lecture library

Twenty-eight self-contained course books carry the taught programme: eight in Semester One, seven in Semester Two, seven in Semester Three including the thesis, and six electives. The volume you are holding is marked in the margin.

Code	Course	Semester	Units
MFT 701	Fundamentals of Marriage and Family Therapy	One	1.71
MFT 702	Theories of Family Therapy	One	1.71
MFT 703	Human Development Across the Lifespan	One	1.71
MFT 704	Ethical and Legal Issues in Therapy	One	1.71
MFT 705	Research Methods in Marriage and Family Therapy	One	1.71
MFT 706	Clinical Practice and Techniques	One	1.71
MFT 707	Assessment and Diagnosis in Family Therapy	One	1.71
MFT 708	Cultural Competence in Therapy	One	1.71
MFT 709	Group Therapy Techniques	Two	1.71
MFT 710	Couples Therapy	Two	1.71
MFT 711	Child and Adolescent Therapy	Two	1.71
MFT 712	Crisis Intervention Strategies	Two	1.71
MFT 713	Substance Abuse and Family Dynamics	Two	1.71
MFT 714	Trauma-Informed Care	Two	1.71
MFT 715	Family Systems Theory	Two	1.71
MFT 716	**Communication Skills in Therapy**	Three	1.71
MFT 717	Psychopathology in Families	Three	1.71
MFT 718	Intimacy and Relationship Dynamics	Three	1.71
MFT 719	Family Life Cycle and Transitions	Three	1.71
MFT 720	Supervision and Consultation in Therapy	Three	1.71
MFT 721	Research Methodology	Three	1.71
MFT 722	Professional Development and Self-Care	Electives	1.71
MFT 723	Special Populations in Therapy	Electives	1.71
MFT 724	Family Law and Advocacy	Electives	1.71
MFT 725	Integrative Approaches in Therapy	Electives	1.71
MFT 726	Neurobiology of Relationships	Electives	1.71
MFT 727	Play Therapy Techniques	Electives	1.71
MFT 770	Master Thesis	Three	1.71

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Course leader's summary. Every therapeutic effect passes through communication. This course refines the therapist's own instrument — listening, responding, questioning, silence, feedback, and the management of difficult conversations — and teaches the protocols you will hand to families.

How to use this book

Skills are built by repetition under observation. Each week has a **micro-skill** to practise, a **protocol** to teach to families, and a **transcript** to analyse. Record yourself weekly; the gap between what you think you said and what you said is the curriculum.

1. Course description

The course covers therapeutic communication at three levels: the **micro** (attending, reflecting, summarising, questioning), the **meso** (structuring difficult conversations, giving feedback, managing conflict), and the **macro** (written communication, interprofessional communication, and communication across language and culture).

It also teaches the **structured communication protocols** used with couples and families — speaker–listener, the Imago dialogue, negotiation formats, family meetings — and requires you to deliver them with fidelity.

2. Learning outcomes

#	Outcome	Assessed
LO1	Demonstrate micro-skills with fluency and intention	Observed session
LO2	Analyse communication at content, relationship and process levels	Transcript analysis
LO3	Teach two structured communication protocols with fidelity	Skills test
LO4	Conduct a difficult conversation (bad news, disclosure, confrontation)	Simulation
LO5	Adapt communication for children, interpreters and low-literacy contexts	Viva
LO6	Write clinical communication that is accurate and usable	Report portfolio
LO7	Identify and modify your own characteristic communication patterns	Reflective portfolio

3. Assessment

Component	Weight	Due
Transcript analysis (1,500 words)	20%	Week 6
Observed micro-skills session (30 min)	25%	Week 8
Protocol teaching test (speaker–listener or Imago)	25%	Week 11

Component	Weight	Due
Difficult conversation simulation (20 min)	30%	Week 14

4. The fifteen-week plan

Week	Topic	Practical
1	Communication as the medium of therapy	Self-recording baseline
2	Attending and non-verbal communication	Non-verbal drill
3	Listening: paraphrase, reflect, summarise	Reflection-only interview
4	Empathy: affective, cognitive, and its limits	Empathy accuracy exercise
5	Questioning and prompting	Question taxonomy drill
6	**Transcript analysis due**; content, relationship, process	Three-level analysis
7	Feedback: giving it, receiving it, teaching families to give it	Feedback lab
8	**Observed micro-skills session**	Assessed
9	Conflict communication and repair	Repair rehearsal
10	Speaker–listener and Imago dialogue	Protocol fidelity test prep
11	**Protocol teaching test**	Assessed
12	Difficult conversations: bad news, disclosure, confrontation	Simulation
13	Communication with children, interpreters and across literacy	Adaptation lab
14	**Difficult conversation assessment**	Assessed
15	Written and interprofessional communication	Report portfolio

Week 1 — Communication as the medium

Three levels of every message.

1. **Content** — what is said.
2. **Relationship** — what it implies about the relationship ("you are the kind of person who…").
3. **Process** — what is happening between the people while it is said.



Figure 1.1 — Five axioms of communication

The therapist's communication is an intervention. Every reflection selects something from the client's account and returns it amplified. There is no neutral way to summarise.

Baseline recording. Record ten minutes of your own session (consented) or a role-play. Count: questions, reflections, advice statements, interruptions, and seconds of silence you initiated. Most beginners are surprised by the advice count.

Week 2 — Attending and non-verbal communication

The SOLER posture (Egan): **S**it squarely, **O**pen posture, **L**ean slightly forward, **E**ye contact (culturally calibrated), **R**elax.

What families read. Pace, volume, facial expression, seating, who you look at, how you respond to distress, and whether you check your phone. A therapist who says "take your time" while glancing at the clock has said something else.

Cultural calibration. Eye contact signals respect in some cultures and challenge in others; physical distance varies; silence may indicate deference rather than resistance. Ask, or observe what the family does with each other.

Working with more than two people. Your attention is a resource that structures the family. Distribute it deliberately; narrate your choices ("I am going to come back to you, I have not forgotten"). Do not let the most articulate person become the narrator of the family.

> **DRILL**
> Triads. The therapist must convey three different messages using only non-verbal behaviour (welcome, "I need to interrupt", "take your time"). The client guesses each.

Week 3 — Listening

Paraphrase (content), **reflect** (feeling), **summarise** (pattern across people and time).

Reflection levels.

Level	Example	Effect
Simple	"You are angry about the money."	Shows attention; may stall
Complex	"You are angry about the money, and underneath that, frightened that you are the only one thinking about the future."	Adds meaning; deepens
Double-sided	"You want him to be more involved, and you also do not trust him to do it right."	Holds ambivalence

The 2:1 rule. Two reflections for every question, until it becomes automatic.

What blocks listening. Formulating your next question; evaluating; wanting to reassure; your own emotional reaction; diagnostic thinking that closes the account prematurely.

Week 4 — Empathy

Three components.

1. **Affective empathy** — feeling something in response.
2. **Cognitive empathy** — understanding the other's perspective.
3. **Empathic communication** — conveying that understanding so the other feels understood.

The therapist needs all three, and needs to keep the first from driving. Affective empathy without boundaries produces over-involvement; cognitive empathy without warmth produces technique; communication without either produces scripts.

Empathy is not agreement. "I understand why you would want to leave" is empathy. "You are right to want to leave" is agreement. The first opens exploration; the second closes it and creates a coalition.

Empathic accuracy is testable. Watch a recorded session and pause before each therapist response. Write what you think the client felt. Then check with the client (or with the recording's subsequent cues).

> **DRILL**
> In pairs, one person describes a minor grievance. The listener must respond with an empathic statement that contains no advice, no question and no story of their own. Three rounds each.

Week 5 — Questioning and prompting

Six families of question (from MFT 706), refined for communication work:

- **Open** — "What was that like?"
- **Circular** — "Who noticed first?"
- **Clarifying** — "When you say 'disrespectful', what did he actually do?"
- **Reflexive** — "If your daughter were listening, what would she say you both do?"
- **Scaling** — "How close are you, 0–10?"
- **Confronting (gently)** — "You say it does not matter, and you have mentioned it four times."

The prompt instead of the question. "Tell me more about that" often produces more than any question. Use prompts when the client is in flow; questions when you need to change direction.

Week 6 — Content, relationship, process

Transcript analysis exercise. Take fifty lines of a session transcript and mark:

- every **content** statement (topic, fact)
- every **relationship-level** message ("you always…", "you are the kind of…")
- every **process** moment (something audible happening between speakers: interruption, softening, joke, silence)

Then ask: what did the therapist respond to? Beginners respond to content and miss process. Advanced practitioners respond to process and use content as evidence.

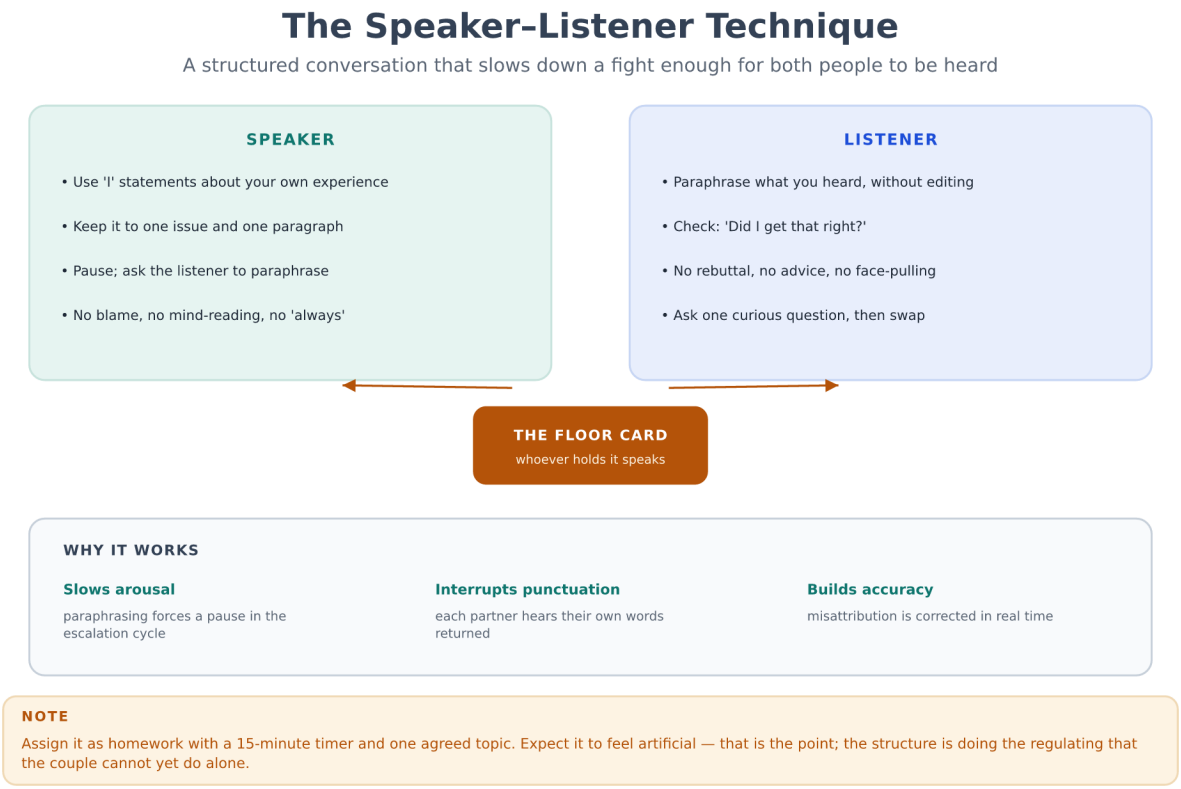


Figure 6.1 — The speaker–listener protocol

Week 7 — Feedback

Feedback to clients should be behavioural, specific, timely and owned: "I noticed your voice dropped when you said that" (not "you are passive-aggressive").

Feedback between family members is often the missing skill. Teach the four-part structure:

1. "When you ___" (behaviour, no adjectives)
2. "I feel ___" (feeling)
3. "Because ___" (meaning)
4. "What I would like is ___" (specific request)

Feedback to colleagues and supervisees: use the same discipline. "What I saw / what it did to me / what to keep / what to try" — never the sandwich, which trains people to hear praise as a warning.

Week 8 — Observed micro-skills session

Thirty minutes. Assessed on: attending; reflection ratio; complex reflections; question variety; one deliberate silence; one summary; and a clean close.

Week 9 — Conflict communication and repair

The escalation sequence: complaint → criticism → contempt → defensiveness → stonewalling.

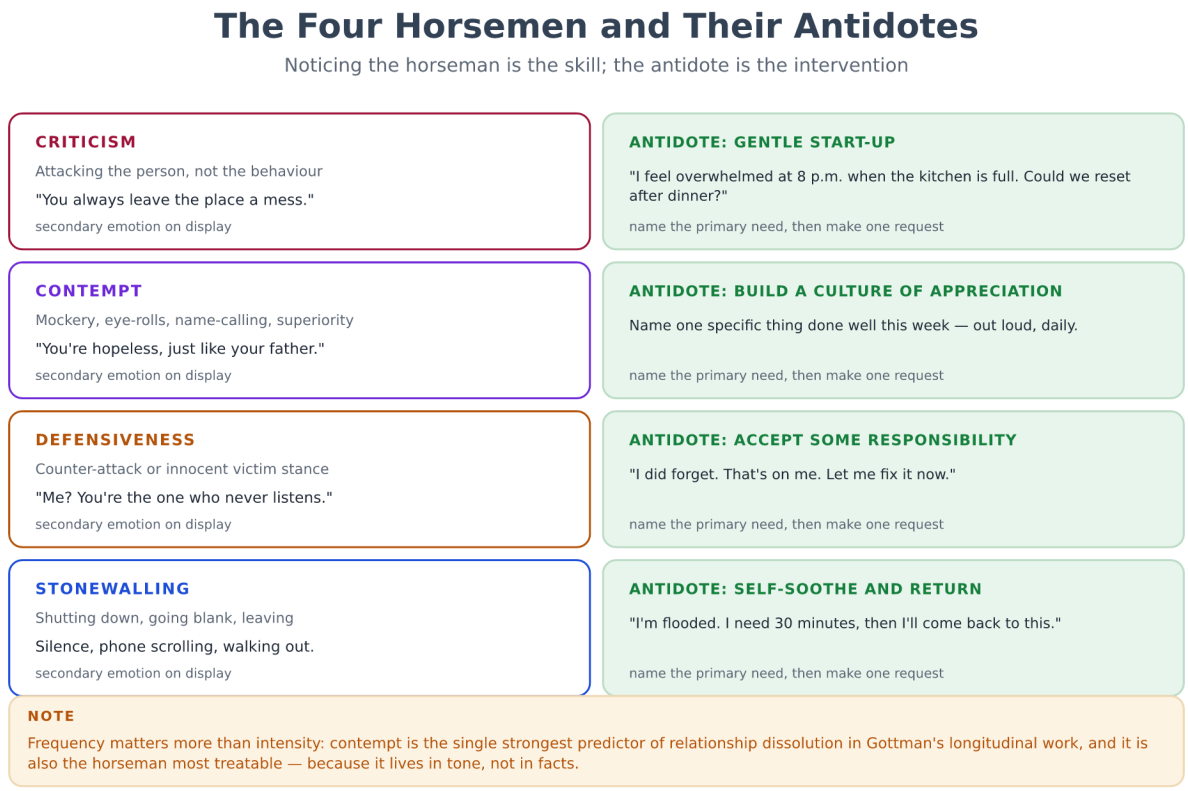


Figure 9.1 — The four horsemen and their antidotes

Repair is the skill that matters. Teach couples to use a repair phrase: "Can we start again?", "I am flooded, give me twenty minutes", "That came out wrong."

The therapist's repair. You will make mistakes: mis-summarise, miss a disclosure, take a side. Repair in the moment: "I got that wrong. Let me try again." Modelling repair is more powerful than describing it.

Week 10 — Protocols

Speaker–listener. A floor card, a timer, paraphrase before responding, then swap. Teach it, rehearse it in session, and set it as homework with one agreed topic.

Imago dialogue. Three steps: mirror ("so what I heard you say is…"), validate ("that makes sense because…"), empathise ("I imagine you feel…"). More demanding than speaker–listener, and more powerful with practised couples.

Negotiation format. Define one issue; each states a position; generate three options; choose one to trial for two weeks; review.

Fidelity matters. A protocol delivered loosely is a protocol that fails, and the couple will conclude that "communication exercises do not work". Practise until you can teach it in ten minutes without notes.

> **ASSESSED**
> Teach speaker–listener to a role-played couple in twelve minutes, then coach them through one round. Assessed on clarity, demonstration, coaching, and management of a partner who will not follow the rules.

Week 11 — Protocol teaching test

As above, assessed formally, with feedback.

Week 12 — Difficult conversations

Four types and their structures.

1. **Bad news** (a diagnosis, a death, a safeguarding outcome). Structure: warning shot ("I have some difficult news") → information in small chunks → check understanding → silence → emotional response → plan → follow-up. Do not fill the silence after the news.
2. **Disclosure** (a secret that must be shared, an affair, a risk). Structure: prepare the person → agree the content → be present → manage the response → plan.
3. **Confrontation** (a boundary violation, a safety concern, a colleague's conduct). Structure: describe the behaviour → its impact → the expectation → the consequence → the support available.
4. **Ending therapy when it is not working** (referral, rupture, incompetence). Structure: name the difficulty → own your part → offer a route → document.

The common failure is rushing to reassurance. "At least…" and "it will be fine" terminate the conversation the client needs to have.

Week 13 — Adaptation

Children. Short sentences, concrete language, check understanding by asking them to explain it back, and use a medium.

Interpreters. Speak to the family, first-person interpretation, brief and debrief (see MFT 708).

Low literacy. Avoid written-only instructions; use drawings, demonstration, and teach-back ("show me how you would do it").

Hearing or cognitive impairment. Reduce background noise, face the person, one idea per sentence, and confirm comprehension.

Second-language sessions. Slow down, avoid idiom, check meaning ("when you say 'respect', what does that include?").

Week 14 — Difficult conversation assessment

Twenty-minute simulation drawn from a bank (bad news, disclosure, confrontation, ending). Assessed on: structure, pacing, emotional attunement, clarity of the message, and a documented follow-up.

Week 15 — Written and interprofessional communication

Clinical notes. Behavioural, dated, factual, and signed. Avoid adjectives about people; avoid speculation.

Referral letters. One page: reason, relevant history, what you have tried, what you are asking for, and who to contact.

Reports for courts and schools. Answer the question asked; state the limits of your expertise; separate fact from opinion.

Interprofessional communication. Use SBAR (Situation, Background, Assessment, Recommendation). It is short, it survives interruption, and it is what other professions are trained to receive.

Glossary

Complex reflection — a reflection that adds meaning or feeling beyond the client's words. **Double-sided reflection** — a reflection that holds both sides of an ambivalence. **Empathic accuracy** — the degree to which a therapist correctly infers a client's experience. **Mirroring** — repeating back the other's words as the first step of a structured dialogue. **Process** — the relational layer of communication, distinct from content. **Repair** — an attempt to de-escalate and reconnect after a rupture. **SBAR** — Situation, Background, Assessment, Recommendation: a handover format. **SOLER** — a posture mnemonic for attending behaviour.

Self-assessment

Multiple choice

1. The recommended ratio of reflections to questions for beginners is: (a) 1:1 (b) 2:1 (c) 1:2 (d) 5:1

2. "You are right to want to leave him" is: (a) empathy (b) agreement (c) a reflection (d) a summary

3. A complex reflection: (a) repeats content (b) adds meaning or feeling (c) asks a question (d) gives advice

4. The main error in delivering bad news is: (a) pausing (b) rushing to reassurance (c) checking understanding (d) using plain language

5. SBAR stands for: (a) Situation, Background, Assessment, Recommendation (b) Summary, Behaviour, Action, Review (c) Safety, Boundaries, Alliance, Repair (d) Session, Block, Ask, Respond

6. In Imago dialogue the three steps are: (a) mirror, validate, empathise (b) ask, tell, repeat (c) listen, advise, conclude (d) reflect, question, summarise

7. Fidelity in teaching a communication protocol matters because: (a) it is required by codes (b) a loosely delivered protocol fails and discredits the method (c) it is faster (d) clients prefer it

8. Eye contact should be: (a) constant (b) culturally calibrated (c) avoided (d) used only with adults

9. The 2:1 rule exists to: (a) reduce interrogation and enrich material (b) fill time (c) avoid emotion (d) demonstrate technique

10. A referral letter should include: (a) the full case file (b) reason, relevant history, what you tried, the request, contact (c) only a diagnosis (d) your opinion of the family

Answer key

1 b · 2 b · 3 b · 4 b · 5 a · 6 a · 7 b · 8 b · 9 a · 10 b

Short answer

1. Describe the three levels of a message with one example of each.
2. Give three examples of a complex reflection on the same client statement.
3. What are the four steps of the feedback structure for family members?
4. Outline the structure for delivering bad news.
5. Name four adaptations for a family with limited literacy.

Essay (choose one, 1,800 words)

1. Analyse a transcript of your own session: what did you respond to, and what did you miss?
2. "Repair, not avoidance, is the mark of a skilled communicator." Discuss with clinical examples.
3. How should communication protocols be adapted for a family in which the couple does not share a first language?

Further reading

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Next: MFT 717 — Psychopathology in Families

MSC MARRIAGE AND FAMILY THERAPY

LECTURE LIBRARY · VOLUME 17 OF 28

MFT 717

Psychopathology in Families

15 teaching weeks · 1 practical · 1 worked case

SEMESTER THREE

1.71 units · Prerequisite: MFT 707, 715

> “Severe mental distress is not caused by families, and it is not treated without them.”

One of 28 self-contained course books in the MSc Marriage and Family Therapy lecture library: a full taught semester per volume, with cases, practicals, assessment briefs, glossaries and model answers.

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MFT 717 — Psychopathology in Families

MSc Marriage and Family Therapy · Semester Three · 1.71 Units · 15 teaching weeks · Prerequisite: MFT 707, 715

This volume is one of 28 course books that together cover the whole MSc Marriage and Family Therapy taught programme. Each book is written to carry a 14–16 week semester on its own: every week pairs a core idea with the practice that follows from it, a worked case, and a reflective task, and each book closes with an integration chapter, a glossary and a self-assessment paper with model answers.

The books are designed to be read straight through before the semester begins, revisited week by week, and used again in revision. Cross-references point to the other volumes where a topic is treated in more depth.

How to cite this volume

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A note on the case material

All families, couples and individuals in this book are fictional composites. They are written to teach pattern recognition, not to describe any real person. Nothing here replaces supervised clinical practice, professional registration, legal advice or medical care; legal content is orientation only and must be checked against the law of the relevant jurisdiction.

The lecture library

Twenty-eight self-contained course books carry the taught programme: eight in Semester One, seven in Semester Two, seven in Semester Three including the thesis, and six electives. The volume you are holding is marked in the margin.

Code	Course	Semester	Units
MFT 701	Fundamentals of Marriage and Family Therapy	One	1.71
MFT 702	Theories of Family Therapy	One	1.71
MFT 703	Human Development Across the Lifespan	One	1.71
MFT 704	Ethical and Legal Issues in Therapy	One	1.71
MFT 705	Research Methods in Marriage and Family Therapy	One	1.71
MFT 706	Clinical Practice and Techniques	One	1.71
MFT 707	Assessment and Diagnosis in Family Therapy	One	1.71
MFT 708	Cultural Competence in Therapy	One	1.71
MFT 709	Group Therapy Techniques	Two	1.71
MFT 710	Couples Therapy	Two	1.71
MFT 711	Child and Adolescent Therapy	Two	1.71
MFT 712	Crisis Intervention Strategies	Two	1.71
MFT 713	Substance Abuse and Family Dynamics	Two	1.71
MFT 714	Trauma-Informed Care	Two	1.71
MFT 715	Family Systems Theory	Two	1.71
MFT 716	Communication Skills in Therapy	Three	1.71
MFT 717	**Psychopathology in Families**	Three	1.71
MFT 718	Intimacy and Relationship Dynamics	Three	1.71
MFT 719	Family Life Cycle and Transitions	Three	1.71
MFT 720	Supervision and Consultation in Therapy	Three	1.71
MFT 721	Research Methodology	Three	1.71
MFT 722	Professional Development and Self-Care	Electives	1.71
MFT 723	Special Populations in Therapy	Electives	1.71
MFT 724	Family Law and Advocacy	Electives	1.71
MFT 725	Integrative Approaches in Therapy	Electives	1.71
MFT 726	Neurobiology of Relationships	Electives	1.71
MFT 727	Play Therapy Techniques	Electives	1.71
MFT 770	Master Thesis	Three	1.71

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Course leader's summary. Severe mental distress is not caused by families, and it is not treated without them. This course builds the knowledge and the nerve to work with psychosis, mood disorders, severe anxiety, personality-level difficulties and eating disorders in a relational frame — alongside medication and psychiatry, not instead of them.

How to use this book

The course deliberately reverses a historical mistake. For decades, family therapy implied that families caused severe mental illness; that legacy still harms families who arrive expecting to be blamed. Each week therefore begins with what is known about aetiology, then asks what a family therapist actually contributes.

1. Course description

The course covers the major categories of severe psychopathology as they present in family life: psychotic disorders, bipolar disorder, major depression, severe anxiety and OCD, personality-level difficulties, eating disorders, and the interface with substance use. It emphasises **family psychoeducation**, **expressed emotion**, **family accommodation**, **relapse prevention**, **caregiver burden**, the **recovery model**, and respectful collaboration with psychiatric services.

2. Learning outcomes

#	Outcome	Assessed
LO1	Describe the aetiology and family impact of major disorders	Exam
LO2	Deliver family psychoeducation for psychosis or bipolar disorder	Skills test
LO3	Assess and reduce expressed emotion and family accommodation	Case study
LO4	Work with caregiver burden and family adaptation	Case study
LO5	Collaborate with psychiatric services and understand medication	Viva
LO6	Avoid both blaming and neglecting the family	Reflective portfolio
LO7	Apply a recovery-oriented frame	Position paper

3. Assessment

Component	Weight	Due
Family psychoeducation session observed (40 min)	30%	Week 8
Case study: family intervention for severe mental illness (2,500 words)	35%	Week 12
Examination	25%	Week 16

Component	Weight	Due
Reflective portfolio	10%	Ongoing

4. The fifteen-week plan

Week	Topic	Practical
1	Historical legacy: from schizophrenogenic mothers to family partnership	Timeline and critique
2	Psychosis I: what it is and what families see	Symptom and experience mapping
3	Psychosis II: family intervention and expressed emotion	EE assessment
4	Family psychoeducation in practice	Session delivery
5	Bipolar disorder and the family	Relapse signature mapping
6	Depression in relational context	Couple-based depression work
7	Severe anxiety, OCD and family accommodation	Accommodation reduction plan
8	**Psychoeducation observed**	Assessed
9	Personality-level difficulties and the family	Boundary and validation work
10	Eating disorders and the family	FBT principles
11	Caregiver burden, siblings and the household	Carer support plan
12	**Case study due**; medication, psychiatry and collaboration	Medication interface
13	Risk, hospitalisation and the family	Admission and discharge planning
14	Recovery, stigma and the family's future	Recovery plan
15	Integration: partnership without blame	Case presentations

Week 1 — The historical legacy

The mistake. Mid-twentieth-century theories blamed mothers for schizophrenia ("schizophrenogenic mother"), attributed psychosis to family communication ("double bind"), and described "families of origin" as pathogenic. These ideas were poorly evidenced, deeply damaging, and are still remembered by families.

The correction. Contemporary research shows that families are neither the cause nor the cure of psychotic disorders, but they are among the strongest predictors of outcome — through support, monitoring, accommodation and emotional climate.

What family therapy offers now:

- psychoeducation (reliable information, in plain language)
- communication and problem-solving training

- reduction of criticism, hostility and over-involvement where they are high
- relapse prevention and early-warning planning
- support for carers
- help with the ordinary relational life that the illness interrupts

> **PRACTICAL**
> Read one historical text and one contemporary family-intervention trial. Write 400 words on what changed, and what harm the earlier framing did.

Week 2 — Psychosis

What it involves. Hallucinations, delusions, disorganised thinking, negative symptoms (flat affect, anhedonia, withdrawal), and cognitive impairment. Onset is typically late adolescence or early adulthood, which means it lands in the middle of a family's launching stage.

What families experience. Fear, grief, confusion, exhaustion, stigma, violence in a minority of cases, and a chronic state of vigilance. Siblings often report invisibility; parents often report marital strain.

What helps (evidence summary). Antipsychotic medication is the foundation. Family intervention (typically 10–12 sessions of psychoeducation plus communication and problem-solving) reduces relapse and improves functioning. Supported employment, cognitive remediation and long-term follow-up matter.

What to avoid. Blaming; interpreting delusions symbolically; colluding with the illness ("he cannot help it, so nothing is required of him"); and treating the family as the patient.

Week 3 — Expressed emotion and family intervention

Expressed emotion (EE) comprises criticism, hostility and emotional over-involvement. High EE predicts relapse in schizophrenia and other conditions, though the effect is smaller than early research suggested and is bidirectional (illness severity also raises EE).

Assessing EE without blaming. Ask about the last month: how many arguments, what triggered them, how the carer responded, how much time is spent managing symptoms, and whether the carer feels able to have a life of their own.

The compassionate reframe. High criticism is usually exhaustion; over-involvement is usually love and fear. Naming that — before teaching any skill — is what makes the intervention acceptable.

Script: "You have been managing something terrifying for four years. Of course you are exhausted and sometimes sharp. The aim of these sessions is not to make you nicer; it is to give you back some room."

Communication training then teaches: making positive requests, active listening, expressing negative feelings safely, and problem-solving in structured steps.

Week 4 — Family psychoeducation

The format. 6–12 sessions, ideally including the person with the diagnosis and their carers, with a combination of information, communication training and problem-solving.

A single-session structure.

Segment	Content
1. What the family already knows	Elicit their model of the illness
2. What is known	Symptoms, course, causes (multi-factorial), treatment, prognosis
3. What helps	Medication adherence, sleep, stress reduction, early warning signs, routines
4. What the family can do	Communication, problem-solving, reducing accommodation, protecting their own life
5. What to do in a crisis	Numbers, triggers, what to say, who to call
6. Questions	Ten minutes of honest answers

Language. Plain, non-technical, and hopeful without being false: "This is a serious condition that can be managed. It is not anyone's fault, and it is not a life sentence."

Week 5 — Bipolar disorder

What families see. Periods of elevated mood, reduced sleep, grandiosity, irritability, risk-taking, and then depression. The family's experience is often of a person they recognise only intermittently.

Family tasks. Recognising the **relapse signature** (early signs of an episode in this person), agreeing a plan in advance, protecting sleep, managing money during highs, and responding to risk.

The relapse signature plan.

1. List the person's own early signs (e.g. sleeping 4 hours and feeling fine; starting three projects).
2. Agree what the family will do (e.g. call the psychiatrist; take over the bank card; reduce stimulation).
3. Agree what the family will not do (argue about whether an episode is real; wait for a crisis).
4. Review after each episode.

Psychoeducation plus family-focused treatment has good evidence for relapse prevention in bipolar disorder.

Week 6 — Depression in relational context

Two directions. Depression is influenced by relationships (criticism, conflict, isolation, caregiving burden), and it influences them (withdrawal, irritability, reduced libido, helplessness).

Couple-based interventions have good evidence for depression, particularly where relationship distress is present. Key mechanisms: reducing criticism and hostility, increasing positive exchange, addressing attributional blame, and supporting behavioural activation with a partner's help.

What not to do. Treat the depressed person as the sole problem; ask the partner to become a therapist; ignore the couple's sexual and practical reality; or reassure prematurely.

> **CASE**
> A husband's depression follows a job loss. The couple's cycle: he withdraws, she takes over the household, he feels useless, she feels abandoned. Intervention: behavioural activation with agreed tasks that restore his role, plus work on her anxiety about relinquishing control.

Week 7 — Severe anxiety, OCD and accommodation

Family accommodation is the family's participation in the anxiety: answering the same question forty times, checking locks, avoiding supermarkets, providing reassurance, taking over tasks.

Why it persists. It reduces distress immediately, which reinforces it for both parties.

Reducing accommodation without cruelty. Build a hierarchy with the family; reduce one item at a time; agree what the person will do instead; expect a temporary increase in distress; and rehearse the script: "I love you, and I am not going to answer that again. I will sit with you for ten minutes instead."

For OCD specifically: psychoeducation about the function of compulsions; family agreement not to participate in rituals; ERP as the treatment of choice, with the family as allies.

Week 8 — Observed psychoeducation session

Forty minutes: eliciting the family's model, delivering information in plain language, teaching one communication skill, agreeing a crisis plan, and answering questions honestly.

Week 9 — Personality-level difficulties

Caution with language. "Personality disorder" is a contested and stigmatising label. Many clinicians describe "personality-level difficulties" or "complex relational patterns" and focus on the pattern rather than the category.

What families experience. Emotional storms, self-harm, idealisation and devaluation, fear of abandonment, and often years of contact with services that have not helped.

What helps.

1. **Validation before change:** "Given what you have survived, this reaction makes sense."
2. **Consistency:** the same rules, the same times, the same responses.
3. **Boundary clarity:** "I will call you back at six" — and doing it.
4. **Family psychoeducation** about emotion regulation and invalidation.
5. **Coordination between services**, because splitting between professionals is part of the pattern.

What harms. Moralising; interpreting the behaviour as manipulation; inconsistency; and treating every crisis as a new emergency rather than part of a known pattern.

Week 10 — Eating disorders

The relational reality. Eating disorders reorganise the household: meals become battlegrounds, siblings become collateral, and parents oscillate between control and helplessness.

Family-based treatment (FBT) for adolescent anorexia is first-line. The stance is **agnostic about cause** and **externalising of the illness**: the family defeats anorexia, and no one is blamed for it.

Phase 1: parents take charge of refeeding, with support, and are absolved of guilt. **Phase 2:** control is returned to the adolescent as developmentally appropriate. **Phase 3:** identity and adolescent issues.

Medical safety is primary. Eating disorders have the highest mortality of any mental disorder. Weight, electrolytes and cardiac status are medical matters; do not work alone.

For adults with longstanding eating disorders, the work is often individual and relational: shame, identity, control, and the couple's accommodation.

Week 11 — Caregiver burden and the household

Assess the carers, not only the patient. Burden is measurable and treatable: use a brief carer measure and ask directly about sleep, work, finances, social life, and their own health.

Siblings. Often invisible, often parentified, often resentful and guilty about the resentment. Give them a session, language for their experience, and permission to have their own life.

The household system. Who is doing the caring? Who has given up what? Is there a "designated carer" whose own life has been suspended? Naming this is often the most useful intervention of the month.

Carer support plan. Respite, information, peer support groups, practical help, and their own GP.

Week 12 — Medication and collaboration

What you need to know (not prescribe).

Class	Common uses	Common side effects to ask about
Antipsychotics	Psychosis, bipolar	Weight gain, sedation, tremor, sexual dysfunction
Antidepressants (SSRIs)	Depression, anxiety, OCD	Nausea, sexual dysfunction, initial agitation
Mood stabilisers (lithium, valproate)	Bipolar	Thirst, tremor, weight, thyroid and renal monitoring
Benzodiazepines	Short-term anxiety	Dependence, sedation, falls

Your role. Support adherence without policing it; explore ambivalence about medication as you would any other ambivalence; report side effects; never advise stopping or changing a dose; and know the prescriber's name.

Collaboration. Write a short letter; attend a case conference; use SBAR on the phone; agree who is responsible for what. Families are often caught between two professionals who have never spoken.

Week 13 — Risk, hospitalisation and the family

Admission is a family event: relief, guilt, fear, and often secrecy. Prepare the family, explain the process, agree visiting, and plan from the first day for discharge.

Discharge is the highest-risk period. Plan: medication supply, follow-up appointment, who is at home, what the early warning signs are, and what to do at night.

When the family is afraid. Take the fear seriously; it is usually based on experience. Build a written crisis plan that the whole household knows.

Week 14 — Recovery, stigma and the future

The recovery model emphasises hope, identity, meaning and self-determination, not only symptom reduction. For families, this means asking: what does a good life look like for this person, and what would support it?

Stigma operates inside the family too: shame, secrecy, "we do not tell anyone", blame of the mother. Addressing internalised stigma is a clinical task.

Planning the future. Housing, work, relationships, and what happens when the parents can no longer care. This conversation is difficult and usually deferred; the therapist's job is to make it possible.

Week 15 — Integration

Four principles.

1. **No blame.** Families are partners, not pathogens.
2. **Information is an intervention.** Accurate, plain-language psychoeducation changes outcomes.
3. **Emotion is contagious.** Reducing criticism and over-involvement helps — and it is achieved by supporting carers, not by instructing them.
4. **Collaboration beats isolation.** Medication, psychiatry, community services and family work together.

Glossary

Accommodation — family behaviours that participate in and maintain symptoms. **Expressed emotion (EE)** — criticism, hostility and emotional over-involvement in carers. **Family-focused treatment (FFT)** — a manualised programme of psychoeducation, communication and problem-solving, with good evidence in bipolar disorder. **Family psychoeducation** — structured information and skills for families living with a disorder. **Prodrome** — the early signs preceding a full episode. **Recovery model** — an orientation emphasising hope, identity, meaning and self-determination. **Relapse signature** — an individual's characteristic early warning signs of an episode.

Self-assessment

Multiple choice

1. Contemporary research on families and psychosis shows that families: (a) cause schizophrenia (b) can significantly influence outcome (c) are irrelevant (d) should be excluded

2. High expressed emotion comprises: (a) criticism, hostility, over-involvement (b) warmth, humour (c) avoidance (d) grief

3. Family accommodation in OCD: (a) reduces long-term symptoms (b) maintains symptoms despite short-term relief (c) is irrelevant (d) should be punished

4. FBT for adolescent anorexia is: (a) blame-based (b) agnostic about cause and externalises the illness (c) individual therapy (d) medication-only

5. A relapse signature is: (a) a diagnosis (b) an individual's early warning signs (c) a blood test (d) a medication

6. The highest-risk period after psychiatric admission is: (a) admission (b) discharge (c) the first year (d) none

7. The therapist's role regarding medication is to: (a) prescribe (b) adjust doses (c) support adherence and report side effects (d) advise stopping

8. Siblings of a person with severe mental illness commonly report: (a) no impact (b) invisibility and guilt about resentment (c) greater support (d) reduced burden

9. The recovery model emphasises: (a) symptom removal only (b) hope, identity, meaning, self-determination (c) institutional care (d) family blame

10. The most damaging historical legacy for families was: (a) too much medication (b) theories blaming families for causing psychosis (c) short admissions (d) family therapy

Answer key

1 b · 2 a · 3 b · 4 b · 5 b · 6 b · 7 c · 8 b · 9 b · 10 b

Short answer

1. State the evidence for family intervention in psychosis in four sentences.
2. What is expressed emotion, and how do you assess it compassionately?
3. Outline a single-session family psychoeducation structure.
4. Give three ways to reduce family accommodation without cruelty.
5. What should a discharge plan contain?

Essay (choose one, 2,000 words)

1. "Families are partners, not pathogens." Trace the history of this claim and its clinical implications.
2. Design a family intervention for a young person with first-episode psychosis, including the first four sessions.
3. Critically discuss the ethics of using the diagnosis "personality disorder" with a family in the room.

Further reading

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Next: MFT 718 — Intimacy and Relationship Dynamics

MSC MARRIAGE AND FAMILY THERAPY

LECTURE LIBRARY · VOLUME 18 OF 28

MFT 718

Intimacy and Relationship Dynamics

15 teaching weeks · 1 practical · 1 worked case · 1 figure

SEMESTER THREE

1.71 units · Prerequisite: MFT 710

> “Intimacy is not a single thing, and sex is not the whole of it.”

One of 28 self-contained course books in the MSc Marriage and Family Therapy lecture library: a full taught semester per volume, with cases, practicals, assessment briefs, glossaries and model answers.

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MFT 718 — Intimacy and Relationship Dynamics

MSc Marriage and Family Therapy · Semester Three · 1.71 Units · 15 teaching weeks · Prerequisite: MFT 710

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The books are designed to be read straight through before the semester begins, revisited week by week, and used again in revision. Cross-references point to the other volumes where a topic is treated in more depth.

How to cite this volume

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A note on the case material

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Twenty-eight self-contained course books carry the taught programme: eight in Semester One, seven in Semester Two, seven in Semester Three including the thesis, and six electives. The volume you are holding is marked in the margin.

Code	Course	Semester	Units
MFT 701	Fundamentals of Marriage and Family Therapy	One	1.71
MFT 702	Theories of Family Therapy	One	1.71
MFT 703	Human Development Across the Lifespan	One	1.71
MFT 704	Ethical and Legal Issues in Therapy	One	1.71
MFT 705	Research Methods in Marriage and Family Therapy	One	1.71
MFT 706	Clinical Practice and Techniques	One	1.71
MFT 707	Assessment and Diagnosis in Family Therapy	One	1.71
MFT 708	Cultural Competence in Therapy	One	1.71
MFT 709	Group Therapy Techniques	Two	1.71
MFT 710	Couples Therapy	Two	1.71
MFT 711	Child and Adolescent Therapy	Two	1.71
MFT 712	Crisis Intervention Strategies	Two	1.71
MFT 713	Substance Abuse and Family Dynamics	Two	1.71
MFT 714	Trauma-Informed Care	Two	1.71
MFT 715	Family Systems Theory	Two	1.71
MFT 716	Communication Skills in Therapy	Three	1.71
MFT 717	Psychopathology in Families	Three	1.71
MFT 718	**Intimacy and Relationship Dynamics**	Three	1.71
MFT 719	Family Life Cycle and Transitions	Three	1.71
MFT 720	Supervision and Consultation in Therapy	Three	1.71
MFT 721	Research Methodology	Three	1.71
MFT 722	Professional Development and Self-Care	Electives	1.71
MFT 723	Special Populations in Therapy	Electives	1.71
MFT 724	Family Law and Advocacy	Electives	1.71
MFT 725	Integrative Approaches in Therapy	Electives	1.71
MFT 726	Neurobiology of Relationships	Electives	1.71
MFT 727	Play Therapy Techniques	Electives	1.71
MFT 770	Master Thesis	Three	1.71

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Course leader's summary. Intimacy is not a single thing, and sex is not the whole of it. This course integrates attachment, desire, communication, culture and the body — and gives you language for the conversations most couples cannot start.

How to use this book

The material is intimate by design. You will be asked to examine your own beliefs about sex, bodies, gender and obligation, because those beliefs will shape what you notice and what you avoid. Supervision is essential for this course; several exercises should be processed there.

1. Course description

The course covers the psychology and physiology of intimacy and desire, the major sexual concerns presenting in couple therapy, the relational dynamics of pursuit and distance, betrayal and repair, the effects of culture and religion, and the practical skills of talking about sex.

It is explicitly **not** a sex therapy certification. It is the level of competence a family therapist needs to assess sexual concerns confidently, intervene relationally, and refer appropriately.

2. Learning outcomes

#	Outcome	Assessed
LO1	Assess intimacy across multiple domains	Observed assessment
LO2	Explain desire, arousal and response, including discrepancy	Exam
LO3	Conduct a respectful sexual history	Skills test
LO4	Formulate sexual concerns relationally and identify medical rule-outs	Case study
LO5	Deliver sensate focus and communication interventions	Skills test
LO6	Address culture, religion and consent	Reflective portfolio
LO7	Recognise the limits of your competence and refer	Viva

3. Assessment

Component	Weight	Due
Sexual history observed (30 min)	30%	Week 6
Case formulation and intervention plan (2,500 words)	40%	Week 11
Examination	20%	Week 16

Component	Weight	Due
Reflective portfolio	10%	Ongoing

4. The fifteen-week plan

Week	Topic	Practical
1	What intimacy is: eight domains	Domain assessment
2	Attachment and intimacy	Attachment-informed formulation
3	Desire: responsive and spontaneous	Desire mapping
4	The sexual response and its variation	Physiology teaching practice
5	Taking a sexual history	Observed practice
6	**Sexual history assessed**; talking about sex with couples	Language lab
7	Desire discrepancy and negotiation	Negotiation rehearsal
8	Common concerns I: erectile and arousal difficulties	Biopsychosocial formulation
9	Common concerns II: pain, orgasm, low desire	Referral decision-making
10	Betrayal, infidelity and the repair of trust	Disclosure planning
11	**Case plan due**; culture, religion and sexual scripts	Cultural genogram of sex
12	Consent, coercion and obligation	Boundary and consent work
13	Sexuality across the lifespan and in special populations	Adaptation planning
14	Sensate focus and intimacy exercises	Exercise design
15	Integration: the therapist's own comfort	Viva

Week 1 — What intimacy is

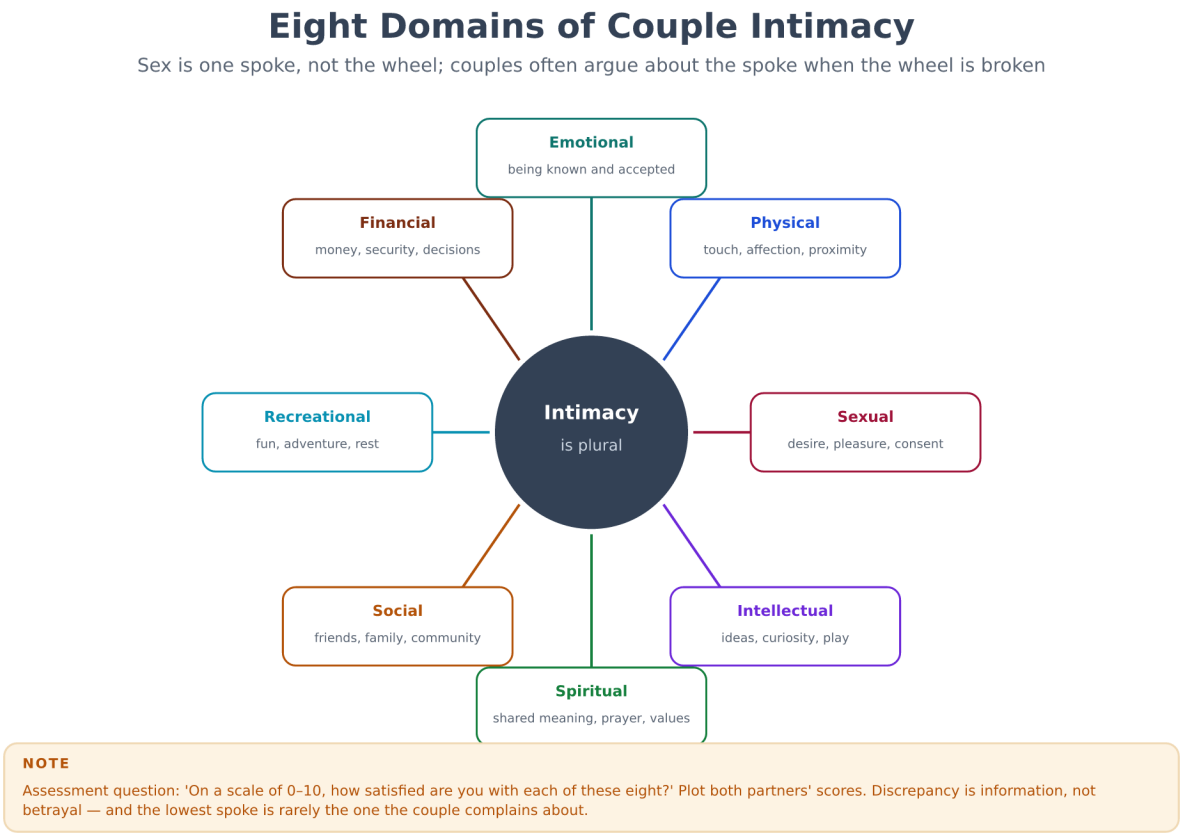


Figure 1.1 — Eight domains of couple intimacy

Couples rarely complain about "intimacy". They complain about sex, or loneliness, or a partner who is always on the phone. Your task is to locate the complaint in the wider system of intimacy domains: emotional, physical, sexual, intellectual, spiritual, social, recreational, financial.

The common pattern. A couple presents with a sexual problem, and assessment reveals that the emotional domain has been starved for years. Treating the sexual symptom alone usually fails; rebuilding the other domains usually helps the sexual one.

Assessment question. "On a scale of 0 to 10, how satisfied are you with each of these eight areas?" Plot both partners' answers. The gap between them, and the gap between the couple's stated problem and their lowest domain, is the clinical map.

Week 2 — Attachment and intimacy

Secure attachment allows both closeness and separateness: the person can ask for comfort and can tolerate a partner's independence.

Anxious attachment produces hypervigilance: sex may be used to secure reassurance, and any distance feels like rejection.

Avoidant attachment produces discomfort with dependency: sex may be easier than emotional closeness, and post-coital distance is common.

Disorganised attachment produces approach–avoidance: desire and fear in the same moment.

Clinical application. "What does sex mean to each of you?" often reveals the attachment function: proof of love, proof of adequacy, a way to avoid conversation, a way to end a conflict, a rare moment of safety.

Week 3 — Desire

Two models.

- **Spontaneous desire**: desire appears before arousal, seemingly unbidden. More commonly reported by men in many samples, and by people in new relationships.
- **Responsive desire**: desire emerges after arousal, in response to context, closeness, relaxation or stimulation. More commonly reported by women, by parents of young children, and by people under stress.

The clinical error. Treating responsive desire as "low desire" or "dysfunction". If a person's desire is responsive, the intervention is context — time, privacy, relaxation, non-sexual affection — not pressure to "want it more".

The dual control model (Bancroft & Janssen): sexual response depends on both excitation and inhibition. Stress, shame, performance anxiety, religious guilt and fear of pregnancy all raise inhibition. Lowering inhibition is often more effective than increasing stimulation.

Week 4 — The sexual response

The classic model (Masters & Johnson; Kaplan): desire → excitement → plateau → orgasm → resolution. **The circular model** (Basson) is more accurate for many people, especially women: intimacy, closeness, context and stimulation interact repeatedly, without a fixed sequence.

What clinicians need to know.

- Arousal is not desire, and lubrication is not consent or interest.
- Arousal non-concordance is normal: the body can show arousal without desire, and desire without arousal.
- Orgasm is not the goal; pleasure and connection are.
- Pain is always a medical question first (vaginismus, vulvodynia, endometriosis, infection, scarring, menopause).
- Medication (especially SSRIs) commonly affects desire, arousal and orgasm.
- Ageing changes speed and reliability, not the capacity for pleasure.

Week 5 — Taking a sexual history

Principles. Ask permission; explain why; use the person's language; be matter-of-fact; do not react with surprise, approval or disapproval; and take a history from each partner separately at least once.

A workable sequence.

1. Permission: "I ask everyone about this because it is often part of what brings couples here. Is that all right?"
2. Current: "How is intimacy between you at the moment? How often, and how satisfying, from 0 to 10?"
3. History: "Was there a time it was good? What was different then?"
4. Meaning: "What does sex mean to you — closeness, relief, duty, play, reassurance?"
5. Body: "Any pain, any difficulty with arousal or orgasm, any medical condition or medication?"
6. History of harm: "Has anyone ever pressured or forced you sexually?" (Ask individually.)
7. Culture and faith: "What were you taught about sex growing up? What do you believe is allowed?"
8. Expectations: "What would you like to be different — and what would your partner say you want?"

Language. Avoid euphemism and avoid clinical jargon. Use the words the couple uses, and offer a plain alternative if they have none.

> **ASSESSED**
> A 30-minute sexual history with a role-played client. Assessed on permission, pacing, breadth, non-judgemental tone, and the identification of medical or safety issues requiring referral.

Week 6 — Talking about sex

Why couples cannot. Shame, fear of hurting, fear of being judged, previous rejection, cultural prohibition, and the belief that "if you loved me you would know".

The therapist's job is to make the conversation normal, structured and safe. Techniques:

- **The third object.** Talk about "the sex thing" as an object between you, rather than about the people.
- **Scaling.** "How satisfied are you, 0–10?" Numbers are less exposing than adjectives.
- **Written disclosure.** Each writes three sentences about what they want more of; you read them aloud.
- **The permission sentence.** "I am going to ask something direct. You can say pass."

Scripts to give couples.

> "I want to talk about us, and I am nervous. Can we take twenty minutes on Saturday?" "I miss being close to you. I am not asking for sex tonight; I am asking whether we can talk about it this week." "When you turn away, I tell myself you do not want me. Is that what you mean?"

Week 7 — Desire discrepancy

The most common sexual complaint in couple therapy. It is a normal difference, not a disorder, until it produces distress.

The pattern. The higher-desire partner pursues; the lower-desire partner avoids; the pursuit increases pressure; the avoidance increases; both feel rejected, and the couple stops touching altogether.

Intervention.

1. **Normalise.** "Desire differences are the norm, not the exception. The problem is not the difference; it is the cycle around it."
2. **De-pressurise touch.** Reintroduce non-sexual affection with an explicit ban on sex for a period.
3. **Negotiate.** What is acceptable to both? Frequency is a poor target; quality, initiation and affection are better ones.
4. **Address the context.** Sleep, children, work, resentment, body image, medication.
5. **Reject the scoreboard.** Sex is not a debt; "I did it last time" is a symptom of the problem, not a solution.

Week 8 — Erectile and arousal difficulties

Always rule out medical causes: vascular disease, diabetes, hypertension, medication, hormonal factors, alcohol and smoking, neurological conditions. Refer to a physician; do not treat as purely psychological.

The psychological pattern. Performance anxiety → monitoring → arousal drops → confirmation of the fear → increased anxiety. The partner's disappointment (real or imagined) intensifies it.

Relational intervention. Reduce performance focus; reintroduce sensate focus; involve the partner in non-demanding intimacy; address the couple's broader pattern; and refer for medical assessment and, where appropriate, medication.

Arousal difficulties in women follow a similar logic, with a stronger contribution from context, pain, relationship quality and hormonal change.

Week 9 — Pain, orgasm and low desire

Presentation	First question	Relational work
Painful intercourse	Has a medical cause been excluded?	Pace, consent, fear, communication
Difficulty reaching orgasm	Is this new? Is it situational?	Reduce goal focus; explore what works alone
Absent or low desire	Is it global or partner-specific?	Context, resentment, responsive desire model
Aversion	Is there a history of coercion or trauma?	Safety, trauma-informed pacing, referral

Referral is not failure. Persistent pain, unexplained physical symptoms, severe trauma responses and psychiatric complexity belong with specialists. The relational work continues alongside.

Week 10 — Betrayal and repair

Betrayal trauma (developed in MFT 710). The betrayed partner's symptoms mimic PTSD: intrusion, hypervigilance, intrusive images, and a shattered assumption that the partner is safe.

Repair requires four things, in order.

1. **Safety** — the behaviour has stopped; boundaries are in place; no violence.
2. **Disclosure** — enough truth to end the lying, not so much detail that it becomes a weapon. Agree what will be asked and answered.
3. **Remorse that is not self-pity** — "I did this, it was my choice, and I see what it cost you."
4. **Consistency over time** — transparency, reliability, and patience with the betrayed partner's cycling.

The cycling is normal. Recovery is not linear: the betrayed partner may be calm for two weeks and then devastated by a song. Warn both partners, so the cycle does not become a second betrayal.

Sex after betrayal. Often the couple either avoids sex entirely or uses it to short-circuit the conversation. Both are understandable and neither is repair. Rebuild slowly, with explicit consent and no expectation.

Week 11 — Culture, religion and sexual scripts

Sexual scripts are culturally learned instructions about who should want what, when, and with whom.

Questions for the couple (each individually first).

- What were you taught about sex before marriage? By whom?
- What did you learn about your body?
- What is permitted, and what is forbidden? What happens if you break the rule?
- Is sex a duty, a gift, a right, a sin, a reward?
- Who initiates? Who is allowed to refuse? What happens when someone refuses?

Clinical themes in Nigerian and similar contexts.

- Pre-marital abstinence teaching that produces guilt after marriage.
- The expectation that sex is a marital obligation, which can silence refusal.
- Female sexual pleasure as unspoken or stigmatised.
- Bride price and the sense of "ownership" it can create.
- Family planning, fertility pressure, and the stigma of childlessness.
- Religious counselling that frames desire as sin.
- Polygyny, and the sexual dynamics between co-wives.

The clinical stance. You are not there to adjudicate the script; you are there to make it speakable, and to ensure that consent is real for both people.

> **CASE PLAN (40%)**
> A couple with a desire discrepancy and a history of religious guilt. Includes sexual history, relational formulation, medical rule-outs, intervention plan, and referral criteria.

Week 12 — Consent, coercion and obligation

Consent is ongoing, specific and freely given. It can be withdrawn at any point, including during sex, and including in marriage.

Coercion includes pressure, sulking, threats, guilt, economic withholding, and the use of religion. It does not have to be physical to be harmful.

Assess individually: "Can you say no to your partner without fear of what will happen? What does happen when you say no?"

When there is coercion, the work is with the person experiencing it first: safety, validation, and options. Couple work is contraindicated until safety is established.

Week 13 — Lifespan and special populations

Group	Considerations
Parents of young children	Exhaustion, touch saturation, interrupted privacy; plan rather than wait for spontaneity
Midlife	Hormonal change, body image, teenagers in the house, career strain

Group	Considerations
Later life	Assumption of asexuality by families and clinicians; health, medication, bereavement, new partners
Chronic illness and disability	Access, pain, fatigue, body image, and the partner's carer role
LGBTQ+ couples	Minority stress, disclosure, family rejection, and the absence of affirming services
Asexual people	Absence of sexual attraction is not pathology; negotiate intimacy on other terms
Post-cancer	Body change, fertility, fear, and the couple's avoidance

The clinician's assumption to avoid: that everyone wants sex, that everyone should, and that a relationship without sex is deficient. Ask what this couple wants.

Week 14 — Sensate focus

Purpose. To reduce performance anxiety and rebuild non-demanding physical intimacy.

Progression.

1. **Non-genital touch, no sex.** Twenty minutes, one partner touches, the other receives; then swap. Feedback only on what felt pleasant, not on what was missing.
2. **Genital touch, still no intercourse.** Continue the same structure.
3. **Intercourse permitted but not required.** Often the couple is advised to continue the exercises first.
4. **Generalisation.** Gradually return to the couple's own sexual repertoire.

Rules. No goal, no intercourse during the first stages, no negative feedback, and a clear opt-out at any time. Couples who "fail" at sensate focus are usually still performing.

Week 15 — The therapist's comfort

Your own sexual history, beliefs and discomfort will shape this work. Write 500 words on: what you find hardest to ask; what judgements you notice; and what you will do when you are out of your depth. Then defend it in a viva, and take it to supervision.

Glossary

Arousal non-concordance — mismatch between physiological arousal and subjective desire. **Desire discrepancy** — a difference between partners in the frequency or intensity of sexual desire. **Dual control model** — sexual response as the balance of excitation and inhibition. **Responsive desire** — desire that emerges in response to arousal or context. **Sensate focus** — graded, non-demanding touch exercises. **Sexual script** — culturally learned expectations about sexual behaviour and meaning. **Spontaneous desire** — desire experienced before arousal, without an obvious trigger.

Self-assessment

Multiple choice

1. Lubrication indicates: (a) consent (b) physiological arousal, which may not match desire (c) desire (d) orgasm

2. Responsive desire is best supported by: (a) pressure to initiate (b) context, relaxation and non-sexual affection (c) performance goals (d) frequency targets

3. The most common sexual complaint in couple therapy is: (a) infidelity (b) desire discrepancy (c) pain (d) infertility

4. Sensate focus requires: (a) intercourse from stage one (b) no goal and no intercourse initially (c) feedback on what is missing (d) daily practice

5. Erectile difficulties should first prompt: (a) psychodynamic exploration (b) medical rule-out (c) couple blame (d) medication advice

6. Consent in marriage is: (a) automatic (b) ongoing, specific and revocable (c) implied by the ceremony (d) not a clinical matter

7. Betrayal trauma symptoms resemble: (a) depression only (b) PTSD (c) psychosis (d) nothing

8. Asexual identity: (a) is a disorder (b) is not pathology; intimacy may be negotiated otherwise (c) requires treatment (d) means no relationships

9. Painful intercourse is first: (a) a relational problem (b) a medical question (c) a sign of avoidance (d) untreatable

10. The therapist should refer when: (a) the couple mentions sex (b) persistent pain, trauma complexity or issues beyond competence arise (c) the couple is young (d) culture differs

Answer key

1 b · 2 b · 3 b · 4 b · 5 b · 6 b · 7 b · 8 b · 9 b · 10 b

Short answer

1. Name the eight domains of intimacy and explain why the lowest is not always the complaint.
2. Distinguish spontaneous and responsive desire and give the clinical implication.
3. Write six questions from a sexual history.
4. What are the four stages of sensate focus?
5. How do you assess for coercion without leading the client?

Essay (choose one, 2,000 words)

1. "Desire discrepancy is a relationship problem, not a disorder." Discuss with a case.

2. Analyse the influence of religious teaching on sexual intimacy in couple therapy.
3. Discuss the ethical and clinical issues in working with sex in a couple where coercion is suspected.

Further reading

- Basson, R. (2001). Human sex response cycles. Journal of Sex and Marital Therapy, 27(1), 33–43.
- Bancroft, J., & Janssen, E. (2000). The dual control model of male sexual response. Neuroscience & Biobehavioral Reviews, 24(5), 571–579.
- Leiblum, S. R. (Ed.). (2007). Principles and Practice of Sex Therapy (4th ed.). Guilford.
- Johnson, S. M. (2019). Attachment Theory in Practice. Guilford.
- Perel, E. (2006). Mating in Captivity. Harper.
- Nagoski, E. (2019). Come as You Are (rev. ed.). Simon & Schuster.

Next: MFT 719 — Family Life Cycle and Transitions

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LECTURE LIBRARY · VOLUME 19 OF 28

MFT 719

Family Life Cycle and Transitions

15 teaching weeks · 3 practicals · 2 figures

SEMESTER THREE

1.71 units · Prerequisite: MFT 703, 715

> “Most families come to therapy during a transition they did not choose, or one they chose but did not anticipate.”

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MFT 719 — Family Life Cycle and Transitions

MSc Marriage and Family Therapy · Semester Three · 1.71 Units · 15 teaching weeks · Prerequisite: MFT 703, 715

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Course leader's summary. Most families come to therapy during a transition they did not choose, or one they chose but did not anticipate. This course teaches you to locate the symptom in time: at which stage is this family, which transition is it facing, and what must be renegotiated.

How to use this book

The life-cycle framework is a diagnostic instrument, not a moral timetable. Each week applies it to a different stage and a different kind of transition, including the ones that do not fit the classic sequence — migration, polygyny, foster care, sudden death, disability. Culture is not an exception to the model; it is the material from which the timetable is made.

1. Course description

The course covers the Carter–McGoldrick stage model; the distinction between predictable (normative) and unpredictable (non-normative) transitions; horizontal and vertical stressors; the family's need to renegotiate roles, rules and boundaries at each transition; rituals; intergenerational legacies; and the transitions particular to contemporary African and diasporic family life.

2. Learning outcomes

#	Outcome	Assessed
LO1	Place a family within the life-cycle model and identify its transition	Case analysis
LO2	Distinguish normative from non-normative transitions and their clinical implications	Exam
LO3	Analyse horizontal and vertical stressors in a family	Case analysis
LO4	Design a transition plan with a family	Observed session
LO5	Use rituals and marking to consolidate change	Skills test
LO6	Adapt the model for migration, polygyny, fosterage and non-linear trajectories	Essay
LO7	Work with loss and unfinished transitions	Reflective portfolio

3. Assessment

Component	Weight	Due
Life-cycle case analysis (2,000 words)	30%	Week 7
Transition plan observed session (40 min)	30%	Week 11

Component	Weight	Due
Essay on cultural adaptation (1,500 words)	20%	Week 13
Examination	20%	Week 16

4. The fifteen-week plan

Week	Topic	Practical
1	Why transitions produce symptoms	Transition timeline
2	The six stages and their tasks	Stage identification
3	Normative and non-normative transitions	Stressor mapping
4	Horizontal and vertical stressors	Genogram + timeline
5	Stage 1–2: leaving home and joining	Case analysis
6	Stage 3: young children and the couple's reorganisation	Case analysis
7	**Case analysis due**; Stage 4: adolescents	Family session practice
8	Stage 5: launching and the empty nest	Couple re-negotiation
9	Stage 6: later life, caregiving and loss	Caregiver family meeting
10	Migration and displacement as family transitions	Migration genogram
11	**Transition plan observed**	Assessed
12	Divorce, remarriage and blended families	Structural adaptation
13	**Essay due**; births, deaths and unfinished transitions	Ritual design
14	Rituals, anniversaries and marking	Ritual design lab
15	Integration: the family in time	Case presentations

Week 1 — Why transitions produce symptoms

The thesis. Symptoms appear at points of transition because the family's existing organisation is no longer adequate to the new demands, and the renegotiation has not yet happened.

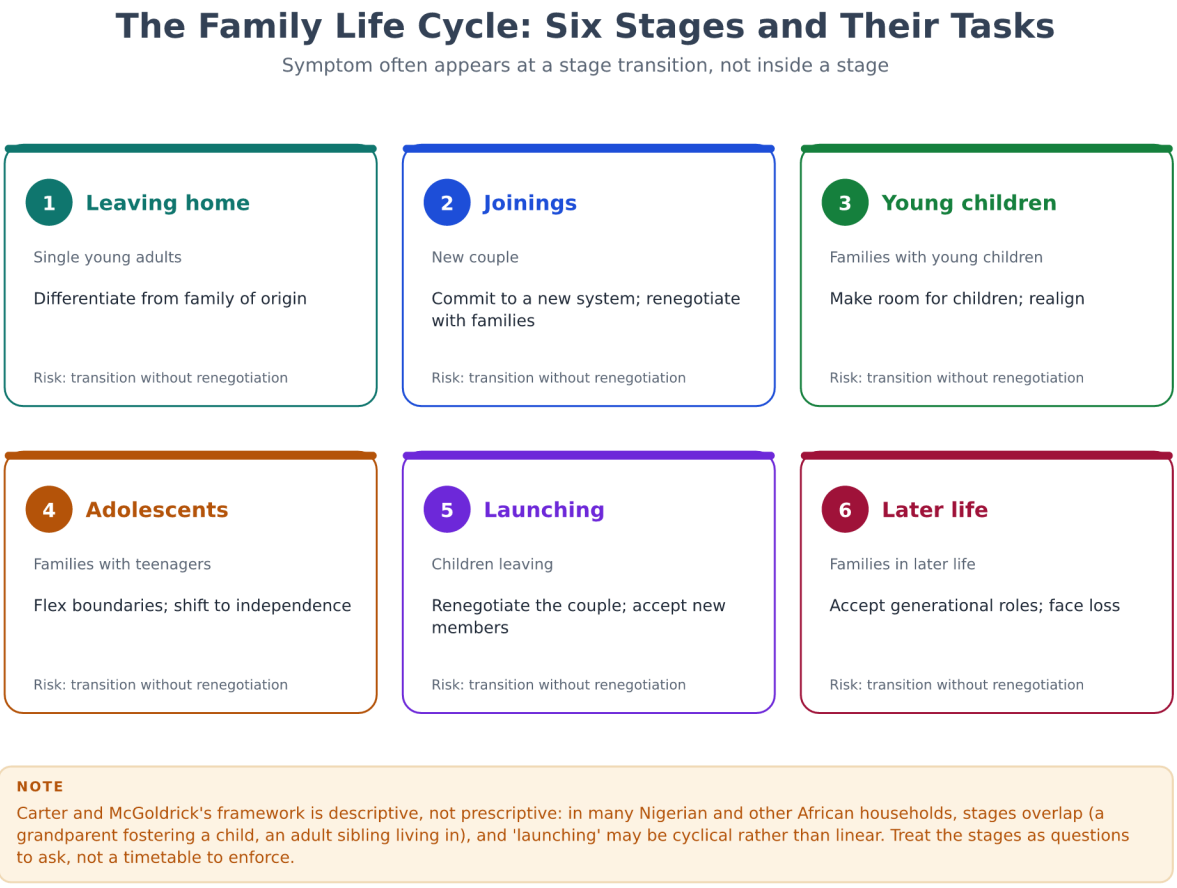


Figure 1.1 — Six stages of the family life cycle and their tasks

Three mechanisms.

1. **Role vacuum.** A role disappears (the working mother, the eldest child at home) and no one has been assigned the new one.
2. **Boundary mismatch.** A boundary that suited the previous stage is now too rigid or too diffuse (parents who cannot let an adolescent have privacy; an adolescent who has become the mother's confidante).
3. **Unfinished business.** The previous transition was never completed (a father who never really left home; a family that never mourned a stillbirth), so the new one cannot be navigated.

Clinical question. "What changed in the last 6–18 months?" almost always produces the precipitant.

> **PRACTICAL**
> Draw a timeline of your family of origin marking moves, births, deaths, marriages, migrations, and job changes. Mark where symptoms or conflicts appeared. Note the pattern.

Week 2 — The six stages

Stage	Emotional task	Second-order change required
1. Leaving home	Accept emotional and financial responsibility	Differentiate from family of origin; develop intimate peer relationships
2. Joining	Commit to a new system	Form a couple; realign relationships with extended families
3. Young children	Accept new members	Adjust the couple system; co-parent; realign with grandparents
4. Adolescents	Increase flexibility	Shift parent–child boundaries; refocus on midlife marriage and careers
5. Launching	Accept many entries and exits	Renegotiate the marriage as a dyad; include in-laws and grandchildren
6. Later life	Accept shifting roles	Deal with loss, disability, and the next generation's centrality

Two warnings. The model is **descriptive of a particular Western, middle-class trajectory**. And the stages overlap: in many families, launching and caregiving occur simultaneously, children return home, and grandparents raise grandchildren.

Week 3 — Normative and non-normative transitions

Normative transitions are expected: marriage, first child, school entry, adolescence, launching, retirement, death of elderly parents.

Non-normative transitions are unexpected or off-time: divorce, a child's disability, a death of a young parent, migration, redundancy, imprisonment, a pandemic, infertility, a child born outside marriage.

Why the distinction matters. Normative transitions have cultural scripts — rituals, advice, role models. Non-normative transitions usually do not, which leaves families improvising under stress, often with shame.

Clinical implication. For non-normative transitions, part of the work is script-making: helping the family construct a way to do something no one they know has done.

> **PRACTICAL**
> Take a non-normative transition (e.g. a child's diagnosis). List the scripts that exist, the scripts that do not, and what the family has had to invent.

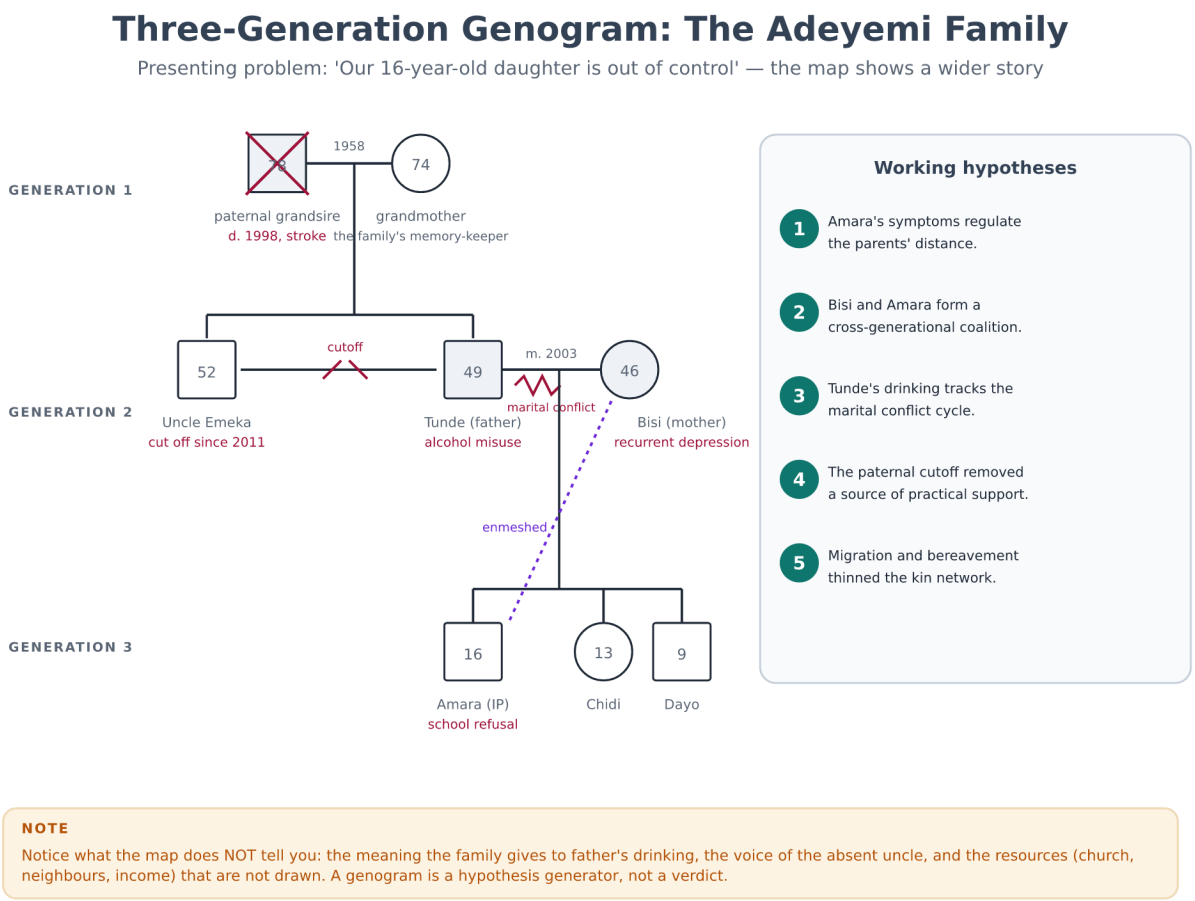
Week 4 — Horizontal and vertical stressors

Horizontal stressors — events moving through time: life-cycle transitions, illness, accidents, migration, poverty, discrimination.

Vertical stressors — legacies handed down: family myths, expectations, secrets, taboos, ethnic and religious beliefs, scripts about gender and success.

The clinical insight. A family can usually manage horizontal stressors or vertical ones; the crisis comes when both arrive at once.

Example: A family facing redundancy (horizontal) in which the father's family has a myth that "a man who cannot provide is worthless" (vertical). The financial problem is manageable; the collision is not.



NOTE

Notice what the map does NOT tell you: the meaning the family gives to father's drinking, the voice of the absent uncle, and the resources (church, neighbours, income) that are not drawn. A genogram is a hypothesis generator, not a verdict.

Figure 4.1 — A genogram as a map of vertical stressors

Week 5 — Leaving home and joining

Leaving home. The task is differentiation, not departure. A young adult may live at home and be differentiated, or live abroad and be fused. The clinical markers: can they disagree with a parent without catastrophe? Can they ask for help without regression?

Joining. The couple must build a "we" while remaining connected to two families of origin. The most common failure is a boundary problem: a spouse who remains more loyal to a parent than to the partner.

Assessment questions.

- "When you disagree with your mother, what happens?"
- "Who did you consult first about this decision?"
- "How often do you see each family? Who decided that?"
- "Whose opinion matters most in this marriage?"

Week 6 — Young children

The reorganisation. Time, sleep, money, division of labour, sexuality, identity, and the relationship with grandparents all change at once. Research consistently shows a dip in relationship satisfaction after the first child, and that the quality of the co-parenting alliance predicts child outcomes more strongly than conflict.

The clinical presentations.

1. **The couple disappears into parents.** No couple time, no sexuality, no conversation that is not about the child.
2. **The grandmother becomes a third parent.** A cross-generational coalition forms; the father becomes peripheral.
3. **The child becomes the regulator.** A toddler's behaviour organises the household's emotional life.

Intervention. Protect the couple subsystem (time, privacy, an agreement about the division of labour); clarify the parental subsystem (a united front, agreed rules); and negotiate explicitly with grandparents about their role.

Week 7 — Adolescents

The family task is to increase flexibility: more autonomy with more accountability; less control and more influence; boundaries that permit privacy without permitting secrecy.

Two failure patterns.

Pattern	What it looks like	Consequence
Escalation	Parents tighten as the adolescent pushes	Conflict, running away, secrecy, risk
Abdication	Parents withdraw to avoid conflict	Unsupervised risk, no structure, anxiety

The middle path is negotiation with limits: clear non-negotiables (safety), negotiable domains (time, phone, clothes), and consequences agreed in advance.

Also at this stage: the parents' own midlife renegotiation, which often appears as an "adolescent problem" in the referral.

Week 8 — Launching and the empty nest

The task is to renegotiate the marriage as a dyad, and to accept new members and exits (partners, grandchildren, returns home).

The common presentations.

- **The couple who have nothing left to say.** They organised everything around the children; now there is silence.
- **The parent who cannot launch.** A mother who sabotages the child's independence because her role depends on being needed.
- **The child who will not leave.** Economic dependency, anxiety, or a parent's covert invitation.

Intervention. Rebuild the couple relationship deliberately; address the parent's identity and loss; and set a graduated plan for independence that neither humiliates the young adult nor abandons them.

Week 9 — Later life

The tasks. Accept generational role changes; deal with loss (parents, siblings, spouse, friends); support the next generation without taking over; prepare for disability and death; and make meaning of a life.

Clinical presentations. Caregiving disputes among siblings; a widowed parent's new relationship; the "responsible child" who carries everything; inheritance conflict; a grandparent raising grandchildren.

Intervention. Make the loyalty ledger explicit (who has given what, and what is owed); negotiate caregiving tasks concretely; and address the unspoken grief that usually sits under the conflict.

The dual-process frame for grief (Figure 22 in MFT 712) helps here: restoration-oriented activity (tasks, roles, legal arrangements) is not avoidance; it is part of grieving.

Week 10 — Migration and displacement

Migration is a family transition with all the features of a life-cycle stage and none of the rituals.

Phase	Family task	Common difficulty
Decision	Who goes, who stays, who pays	Coercion, debt, secrecy
Separation	Maintain attachment across distance	Children left with relatives; marital drift
Arrival	Establish a household	Status loss, housing, language

Phase	Family task	Common difficulty
Reunification	Re-form as a family	Children who do not know the parent; new authority conflicts
Settlement	Integrate without dissolving identity	Second-generation conflict; "you are not like us"

The "astronaut" and "parachute" patterns. A parent working abroad while the family remains; a child sent abroad for education. Both produce high achievement and high loneliness, and often a "satellite child" who is expected to justify the family's sacrifice.

Clinical questions. Who decided? Who was left? What was lost (status, language, community, identity)? What does the family owe the ones who stayed?

Week 11 — Observed transition plan session

Forty minutes with a role-played family at a transition (launching, caregiving, migration reunification). Assessed on: accurate location of the transition; renegotiation of roles and boundaries; inclusion of all members; and a concrete, reviewable plan.

Week 12 — Divorce, remarriage and blended families

Divorce as a transition, not an ending. The spousal relationship ends; the parental relationship continues in a different form. The family's task is to reorganise across two households while protecting the children from conflict.

The stages. Decision → telling the children → practical reorganisation → legal process → emotional adjustment → new equilibrium (often 2–4 years).

Remarriage and stepfamilies. A new structure with different tasks:

- Build the couple bond before the stepparent assumes authority.
- Negotiate household rules across two systems.
- Allow children to love both households without disloyalty.
- Expect 4–7 years to integrate, not months.
- Address the "insider/outsider" split explicitly.

Clinical error. Treating the stepfamily as a nuclear family that started late, and applying a nuclear family timetable.

Week 13 — Births, deaths and unfinished transitions

Unfinished transitions are those that were never marked, mourned or renegotiated: a stillbirth nobody mentions, a migration that was supposed to be temporary, a divorce that was never explained to the children, a death the family does not discuss.

Signs. Anniversary reactions, a name that cannot be spoken, a child named after the dead, a room kept unchanged, a recurring argument with no present cause.

Intervention. Name the unfinished transition gently; help the family construct a way to mark it; and allow the grief that the avoidance was managing.

Week 14 — Rituals

Rituals consolidate transitions. They mark the boundary between the old state and the new, assign new roles, and connect the family to its wider culture.

Types. Cultural and religious rites (naming, marriage, burial); family rituals (Sunday meals, holidays, anniversaries); and therapeutic rituals (a letter, a ceremony, a shared object, a family meeting).

Designing a therapeutic ritual.

1. **Purpose.** What transition needs marking?
2. **Participants.** Who must be present, and who must be absent?
3. **Symbol.** What object, place, word or action carries the meaning?
4. **Structure.** Beginning, middle, end.
5. **Follow-up.** How will the change be maintained?

Caution. Respect the family's religious and cultural boundaries; do not invent rites that displace their own; and never use a ritual to bypass a necessary conversation.

> **PRACTICAL**
> Design a ritual for a family that has never marked a bereavement. Present it in five minutes.

Week 15 — Integration: the family in time

The life-cycle lens gives you three questions that belong in every assessment:

1. **Where is this family in time?** (Stage, and the transition in progress.)
2. **What must be renegotiated?** (Roles, boundaries, loyalties, rules.)
3. **What has not been finished?** (Losses, transitions, legacies.)

Ask them in the first session and you will rarely be short of a focus.

Glossary

Horizontal stressors — events and transitions moving through time. **Vertical stressors** — legacies, myths and expectations transmitted across generations. **Normative transition** — an expected, culturally scripted life event. **Non-normative transition** — an unexpected or off-time event. **Second-order change** — change in the system's organisation, not only in its behaviour. **Unfinished transition** — a transition that was never marked or renegotiated. **Satellite child** — a child sent abroad or away for education, carrying the family's hopes.

Self-assessment

Multiple choice

1. Symptoms most often appear: (a) randomly (b) at transitions requiring renegotiation (c) only in adolescence (d) only after trauma

2. Vertical stressors are: (a) events in time (b) legacies and myths across generations (c) financial problems (d) illnesses

3. The task of "leaving home" is: (a) departure (b) differentiation while remaining connected (c) financial independence only (d) marriage

4. The dip in couple satisfaction after a first child is: (a) rare (b) a well-documented common pattern (c) caused by the child (d) unavoidable

5. The two adolescent failure patterns are: (a) escalation and abdication (b) fusion and cutoff (c) pursuit and withdrawal (d) enmeshment and disengagement

6. Blended family integration typically takes: (a) months (b) 4–7 years (c) weeks (d) one year

7. A "satellite child" is: (a) a child in orbit (b) a child sent away for education carrying family hopes (c) an only child (d) a foster child

8. Unfinished transitions often present as: (a) clear grief (b) anniversary reactions and unspoken names (c) financial problems (d) delinquency

9. Therapeutic rituals should: (a) replace the family's own rites (b) respect and complement them (c) be religious (d) be avoided

10. The life-cycle model is best used as: (a) a timetable to enforce (b) a descriptive diagnostic lens (c) a legal standard (d) a diagnosis

Answer key

1 b · 2 b · 3 b · 4 b · 5 a · 6 b · 7 b · 8 b · 9 b · 10 b

Short answer

1. Distinguish normative from non-normative transitions with two examples each.
2. Explain horizontal and vertical stressors with a case.
3. What must be renegotiated when a first child is born?
4. Name four features of migration as a family transition.
5. Give five elements of a therapeutic ritual.

Essay (choose one, 2,000 words)

1. "The life cycle is a Western timetable." Critically evaluate and adapt the model for Nigerian families.

2. Analyse a family's presenting problem as a transition problem, and show what changed in your treatment plan.
3. Discuss the role of rituals in consolidating therapeutic change.

Further reading

- Carter, B., & McGoldrick, M. (Eds.). (2005). The Expanded Family Life Cycle (3rd ed.). Allyn & Bacon.
- McGoldrick, M., Garcia-Preto, N., & Carter, B. (2016). The Multicultural Family Life Cycle. In The Expanding Family Life Cycle (5th ed.).
- Falicov, C. J. (2007). Working with transnational immigrants. Family Process, 46(2), 157–171.
- Walsh, F. (2016). Strengthening Family Resilience (3rd ed.). Guilford.

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The books are designed to be read straight through before the semester begins, revisited week by week, and used again in revision. Cross-references point to the other volumes where a topic is treated in more depth.

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A note on the case material

All families, couples and individuals in this book are fictional composites. They are written to teach pattern recognition, not to describe any real person. Nothing here replaces supervised clinical practice, professional registration, legal advice or medical care; legal content is orientation only and must be checked against the law of the relevant jurisdiction.

The lecture library

Twenty-eight self-contained course books carry the taught programme: eight in Semester One, seven in Semester Two, seven in Semester Three including the thesis, and six electives. The volume you are holding is marked in the margin.

Code	Course	Semester	Units
MFT 701	Fundamentals of Marriage and Family Therapy	One	1.71
MFT 702	Theories of Family Therapy	One	1.71
MFT 703	Human Development Across the Lifespan	One	1.71
MFT 704	Ethical and Legal Issues in Therapy	One	1.71
MFT 705	Research Methods in Marriage and Family Therapy	One	1.71
MFT 706	Clinical Practice and Techniques	One	1.71
MFT 707	Assessment and Diagnosis in Family Therapy	One	1.71
MFT 708	Cultural Competence in Therapy	One	1.71
MFT 709	Group Therapy Techniques	Two	1.71
MFT 710	Couples Therapy	Two	1.71
MFT 711	Child and Adolescent Therapy	Two	1.71
MFT 712	Crisis Intervention Strategies	Two	1.71
MFT 713	Substance Abuse and Family Dynamics	Two	1.71
MFT 714	Trauma-Informed Care	Two	1.71
MFT 715	Family Systems Theory	Two	1.71
MFT 716	Communication Skills in Therapy	Three	1.71
MFT 717	Psychopathology in Families	Three	1.71
MFT 718	Intimacy and Relationship Dynamics	Three	1.71
MFT 719	Family Life Cycle and Transitions	Three	1.71
MFT 720	**Supervision and Consultation in Therapy**	Three	1.71
MFT 721	Research Methodology	Three	1.71
MFT 722	Professional Development and Self-Care	Electives	1.71
MFT 723	Special Populations in Therapy	Electives	1.71
MFT 724	Family Law and Advocacy	Electives	1.71
MFT 725	Integrative Approaches in Therapy	Electives	1.71
MFT 726	Neurobiology of Relationships	Electives	1.71
MFT 727	Play Therapy Techniques	Electives	1.71
MFT 770	Master Thesis	Three	1.71

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Course leader's summary. Supervision is the profession's quality-control mechanism and its main method of training. This course prepares you to receive supervision well and to give it competently — with models, structure, feedback skills, attention to power, and the discipline of parallel process.

How to use this book

You are simultaneously a supervisee and a developing supervisor in this course. Each week you will analyse your own supervision (what worked, what did not, what you did not say) and practise supervising a peer. The two roles teach each other.

1. Course description

The course covers models of supervision (developmental, systemic, competency-based, reflective), the supervisory alliance, contracting and boundaries, live and recorded supervision, feedback, evaluation and gatekeeping, power and culture in supervision, parallel process, ethical and legal responsibilities, and consultation to organisations.

2. Learning outcomes

#	Outcome	Assessed
LO1	Describe and apply at least three supervision models	Exam
LO2	Contract clearly with a supervisee, including evaluation criteria	Contract document
LO3	Give specific, behavioural feedback that changes practice	Observed supervision
LO4	Identify and use parallel process	Case analysis
LO5	Manage power, culture and conflict in supervision	Reflective portfolio
LO6	Fulfil gatekeeping responsibilities ethically	Viva
LO7	Consult to a team or organisation	Consultation report

3. Assessment

Component	Weight	Due
Supervision contract and learning plan	15%	Week 4
Observed supervision session (40 min)	30%	Week 10
Case analysis of parallel process (1,500 words)	20%	Week 12
Consultation report (2,000 words)	25%	Week 14
Reflective portfolio	10%	Ongoing

4. The fifteen-week plan

Week	Topic	Practical
1	What supervision is for; supervision versus therapy versus mentoring	Role clarification
2	Models I: developmental and competency-based	Model application
3	Models II: systemic and reflective	Reflecting team in supervision
4	**Contract due**; contracting and boundaries	Contract negotiation
5	The supervisory alliance and rupture repair	Alliance repair rehearsal
6	Structure: agenda, case presentation formats, time	Structured case presentation
7	Feedback that changes practice	Feedback lab
8	Live supervision and recorded review	Live observation
9	Parallel process and the supervision triangle	Parallel process analysis
10	**Observed supervision session**	Assessed
11	Power, culture, gender and race in supervision	Power audit
12	**Parallel process analysis due**; conflict and incompetence	Difficult conversations
13	Evaluation, gatekeeping and fitness to practise	Evaluation interview
14	**Consultation report due**; consultation to organisations	Consultation simulation
15	Integration: the supervisor you want to be	Viva

Week 1 — What supervision is for

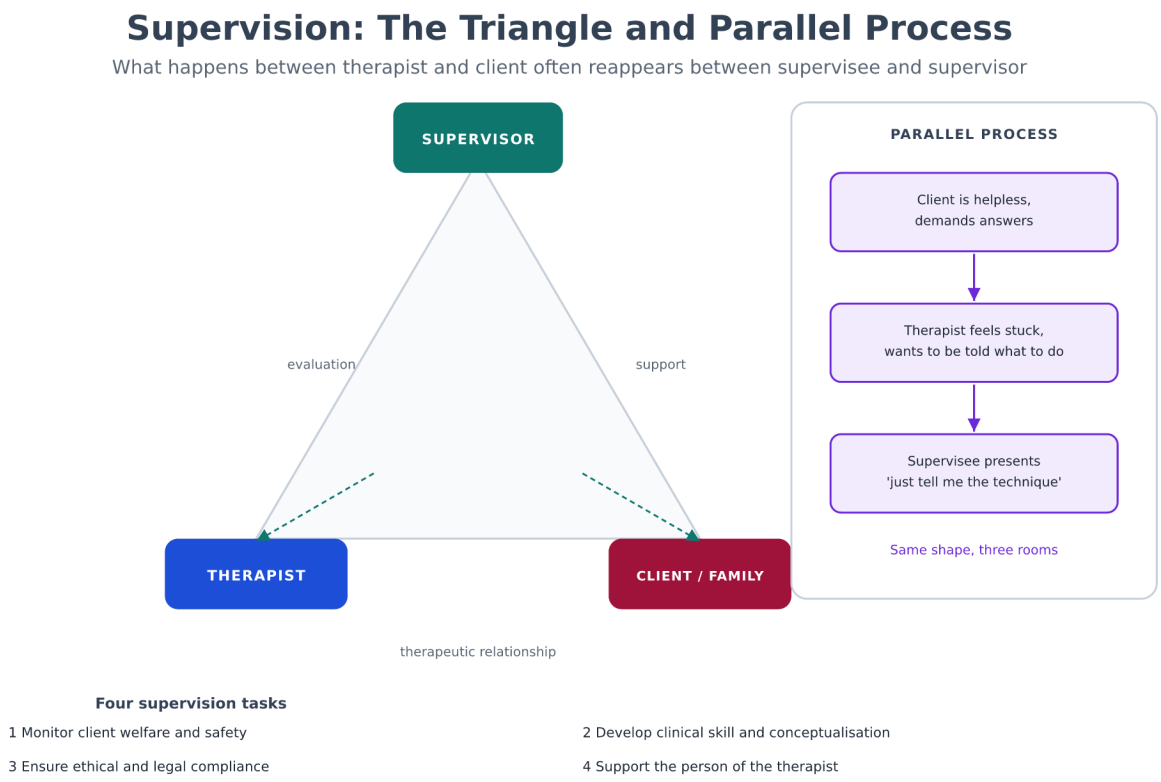


Figure 1.1 — The supervision triangle and parallel process

Four functions (Kadushin, adapted).

1. **Clinical** — ensuring the client receives good, safe care.
2. **Developmental** — building the supervisee's competence.
3. **Supportive** — sustaining the person doing difficult work.
4. **Gatekeeping** — protecting the public and the profession.

What supervision is not. It is not therapy, although it may be therapeutic. It is not mentoring, which is about career. It is not line management, although it overlaps. Confusing these roles is the most common source of boundary difficulty.

The client is the third party in the room. Whatever the supervisee needs, the question that governs the supervision is: is this client safe and well served?

Week 2 — Developmental and competency models

Developmental models (e.g. Stoltenberg) describe stages: the supervisee moves from dependence and anxiety, through dependence–autonomy conflict, to conditional autonomy, to mastery. The supervisor's style must change with the stage: more structure early, more consultation later.

Stage	Supervisee needs	Supervisor stance
Novice	Structure, direct guidance, safety	Teaching, modelling, clear expectations
Advanced beginner	Autonomy with support	Coaching, prompting, joint formulation
Competent	Challenge, complexity	Consulting, debating, opening alternatives
Proficient	Peer-level dialogue	Collegial, focusing on the system and self

Competency-based models specify observable skills and rate them, using frameworks such as the UCL competence frameworks. Their strength is transparency and defensibility; their risk is reducing supervision to a checklist.

The best use of both. Use developmental stage to decide how much structure, and competencies to decide what to look for.

Week 3 — Systemic and reflective models

Systemic supervision attends to the system of client–therapist–supervisor, and to the wider systems (agency, family, culture). The supervisor asks: what is the function of this presentation in this system, at this time?

Reflective supervision uses the supervisor's and supervisee's emotional responses as data, and models curiosity rather than expertise. Techniques include the reflecting team (in supervision), genograms of the case, and structured self-reflection.

Live supervision (behind a mirror, or in-room) gives immediate data. Formats: the phone-in, the "whispering" supervisor, the consultant who joins for ten minutes, and the reflecting team.

The advantage of live work is that it reveals what the therapist does, not what they remember doing. The disadvantage is anxiety, which must be managed explicitly in the contract.

Week 4 — Contracting

A supervision contract covers:

1. Purpose and scope; who else is involved (agency, university).
2. Frequency, duration, location, cancellation.
3. Case presentation format and preparation expectations.
4. Recording and observation policy, including consent from clients.
5. Confidentiality and its limits (client safety, fitness to practise, legal).
6. Evaluation: criteria, timing, who sees it, and consequences.
7. Boundaries: contact between sessions, personal disclosure, dual relationships.
8. Procedures for disagreement, complaint and termination.

Negotiate, do not impose. The contract should include the supervisee's goals: "By the end of this placement, I want to be able to…"

> **DOCUMENT (15%)**
> A supervision contract and learning plan for a first-placement supervisee.

Week 5 — The supervisory alliance

Three components (parallel to the therapeutic alliance): goals, tasks, and bond.

Ruptures are inevitable: a supervisee feels judged; a supervisor feels ignored; a disagreement about a case becomes personal. The repair is the model.

Repair script.

> "I noticed you went quiet when I suggested you had missed the risk. Did I get that wrong, or did it land badly?" "What I intended was a question, not a verdict. Let me say it again differently."

The unspoken curriculum. Supervisees learn more from how you behave than from what you teach. If you avoid difficult feedback, they will avoid it with clients. If you are curious about your own mistakes, they will be curious about theirs.

Week 6 — Structure

A 50-minute supervision session.

Minutes	Content
0–5	Check-in; agenda setting
5–15	Case 1: presentation, formulation, question
15–30	Case 1: exploration, recording review, options
30–40	Case 2 or a theme (risk, ethics, culture)
40–47	Learning points; tasks for the week
47–50	Administrative and evaluation notes

A case presentation format (five minutes, no notes).

1. Who is the family, and who is in the room?
2. What is the presenting problem, in their words?
3. What is my formulation and my uncertainty?
4. What did I do, and what happened?
5. What is my question for supervision?

The discipline matters: a supervisee who cannot present in five minutes is not yet thinking systemically.

Week 7 — Feedback

Principles. Behavioural, specific, timely, owned, and limited to what can be acted on.

The four-step structure.

1. **What I observed** ("you asked three questions in a row after she cried").
2. **What it did** (to the client, or to me as observer) ("she stopped elaborating").
3. **What to keep** ("your opening was excellent — you named the risk clearly").
4. **What to try** ("next time, try a reflection before the next question").

The banned practices. The sandwich (praise–criticism–praise), "you need to be more confident" (not behavioural), public criticism, and feedback given weeks after the event.

Receiving feedback is a skill too: listen, ask for an example, resist explaining, and decide what to use.

Week 8 — Live supervision and recording

Consent. Families must consent to recording and to observation, in writing, with a clear explanation of who will watch and how long the recording is kept.

In-session formats.

- **Phone-in:** the supervisor calls in with one or two suggestions. Risk: the therapist becomes a puppet.
- **Reflecting team:** the team talks in front of the family (or behind a mirror). The therapist chooses what to bring back. Strong evidence and strong engagement effects.
- **Consultant joins:** a senior clinician joins for ten minutes and then leaves.

Debriefing. Always debrief the supervisee's experience first, then the clinical content. The order matters: the person before the problem.

Week 9 — Parallel process

Definition. The pattern in the therapy reappears in the supervision: a helpless family produces a helpless therapist who produces a supervisor who takes over; a controlling family produces a therapist who complies and a supervisor who feels ignored.

How to use it.

1. Notice your own experience in supervision (boredom, irritation, the urge to solve).
2. Ask: is this mine, the supervisee's, or the system's?

3. Name it tentatively: "I notice I am doing a lot of the thinking here. I wonder whether that is also how it feels in the room."
4. Use it to generate a hypothesis about the family.

Caution. Parallel process is a hypothesis, not a law. Do not use it to explain away a supervisee's difficulty or your own poor supervision.

> **ANALYSIS (20%)**
> A 1,500-word analysis of a parallel process from your own supervision, with evidence and alternatives considered.

Week 10 — Observed supervision session

Forty minutes, supervising a peer with a real (anonymised) case. Assessed on: contract and structure; case presentation discipline; formulation support; feedback quality; attention to the supervisee's experience; and clear learning tasks.

Week 11 — Power, culture, gender and race

Power is structural. The supervisor evaluates, gatekeeps and holds institutional authority. It cannot be wished away; it can be made transparent.

Cultural dynamics. Differences in language, ethnicity, religion, gender, class and age shape what is said and what is withheld. A supervisee from a culture that does not openly disagree with elders may appear "unreflective" when they are being respectful.

Practical disciplines.

- Discuss difference explicitly at the start: "What should I know about how you prefer to receive feedback?"
- Invite disagreement in a form that is culturally possible: "What would you say if you disagreed?"
- Notice who speaks first, who is interrupted, and whose cases get the most time.
- Examine your own assumptions about competence.

Week 12 — Conflict, incompetence and impairment

Conflict. Address early, privately, behaviourally. Distinguish disagreement about a case (healthy) from disagreement about values (harder) from a relationship rupture (needs repair).

Incompetence. Define it against the competencies, in writing, with a development plan and a review date. Most incompetence is developmental and remediable.

Impairment. When the supervisee's own difficulties affect client care (substance use, untreated trauma, exhaustion), the response is support plus boundaries, not silence. Silence is the most common failure of supervisors and the most damaging to clients.

The conversation. "I am concerned about you, and I am also concerned about your clients. Let us talk about both."

Week 13 — Evaluation and gatekeeping

Evaluation criteria should be known in advance and behavioural.

Domain	Example indicator
Clinical competence	Can conduct a structured assessment and formulation
Professional conduct	Punctuality, record-keeping, boundaries
Reflective capacity	Uses feedback; identifies own contribution
Ethical practice	Recognises and escalates risk
Relational capacity	Builds alliances with all family members

Gatekeeping. If a supervisee is not safe, the supervisor must act, even when it is painful: extend supervision, restrict caseload, require therapy, or fail the placement with a documented rationale.

Failing well. Give clear reasons, a written plan, support, and a route to appeal. Document throughout, not only at the end.

Week 14 — Consultation to organisations

The shift. Supervision develops a person; consultation helps a system. Your client is the team, the school, the clinic or the agency.

A consultation cycle.

1. **Contract:** who is the client, what is the question, who holds authority, what will be shared?
2. **Assess the system:** structure, communication, roles, alliances, and the presenting problem's function.
3. **Feedback:** present the systemic picture, with hypotheses rather than verdicts.
4. **Intervention:** a workshop, a reflecting team, a policy change, a case conference structure.
5. **Exit and evaluate:** what changed, and how will it be sustained?

Common organisational dynamics. The identified "difficult team member" who carries the team's anxiety; the "impossible case" that holds a conflict between services; the over-worked manager who mirrors the over-worked family therapist.

> **REPORT (25%)**
> A consultation report for a team or organisation, with a systemic assessment and a proposal.

Week 15 — The supervisor you want to be

Write 700 words: your model of supervision; how you will handle power; what you will never do; the feedback you find hardest to give; and how you will know you are doing harm. Defend it in a viva.

Glossary

Competency-based supervision — supervision organised around observable, rated skills. **Gatekeeping** — the supervisor's responsibility to protect the public by preventing unsafe practice. **Live supervision** — observation of therapy as it happens, with in-session input. **Parallel process** — the reproduction of the therapy dynamic in the supervision. **Reflecting team** — a group that discusses the case in front of the family, with the therapist choosing what to bring back. **Supervisory alliance** — the goals, tasks and bond of the supervision relationship.

Self-assessment

Multiple choice

1. The client in supervision is: (a) the supervisee (b) the family receiving therapy (c) the agency (d) the university

2. Gatekeeping exists to: (a) protect the profession's reputation only (b) protect the public and the profession (c) increase pass rates (d) satisfy administrators

3. Parallel process is: (a) a proven law (b) a hypothesis about the reproduction of dynamics in supervision (c) transference (d) supervision of supervision

4. A good feedback statement is: (a) general and encouraging (b) behavioural, specific and owned (c) delayed (d) public

5. Live supervision requires: (a) no consent (b) written client consent and clear explanation (c) only supervisor consent (d) audio only

6. The sandwich technique is discouraged because: (a) it is too positive (b) it trains people to hear praise as a warning (c) it is fast (d) it is illegal

7. Developmental models suggest that novices need: (a) less structure (b) more structure and direct guidance (c) peer consultation only (d) no evaluation

8. Impairment in a supervisee should be met with: (a) silence (b) support plus clear boundaries (c) immediate dismissal (d) transfer without discussion

9. Consultation differs from supervision in that it: (a) develops a person (b) helps a system (c) evaluates students (d) is always clinical

10. A supervision contract should include: (a) only the times (b) purpose, frequency, confidentiality, evaluation and boundaries (c) the supervisor's fees only (d) the client's diagnosis

Answer key

1 b · 2 b · 3 b · 4 b · 5 b · 6 b · 7 b · 8 b · 9 b · 10 b

Short answer

1. Name the four functions of supervision.
2. Describe a five-minute case presentation format.
3. What are the four steps of behavioural feedback?
4. Give three ways power operates in supervision and one way to make it transparent.
5. What should a supervisor do when a supervisee is unsafe?

Essay (choose one, 1,800 words)

1. "Supervision is the profession's most important ethical practice." Discuss.
2. Analyse a parallel process from your own experience and what it revealed about the case.
3. Discuss the management of cultural difference in supervision, with examples.

Further reading

- Hawkins, P., & Shohet, R. (2012). Supervision in the Helping Professions (4th ed.). Open University Press.
- Stoltenberg, C. D., & McNeill, B. (2010). IDM Supervision (3rd ed.). Routledge.
- Bernard, J. M., & Goodyear, R. K. (2019). Fundamentals of Clinical Supervision (6th ed.). Pearson.
- Andersen, T. (1991). The Reflecting Team. Norton.
- Pilling, S., & Fornells-Ambrojo, M. (2015). Supervision and Consultation Competence Frameworks. UCL.

Next: MFT 721 — Research Methodology

MSC MARRIAGE AND FAMILY THERAPY

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MFT 721

Research Methodology

14 teaching weeks · 2 practicals · 2 figures

SEMESTER THREE

1.71 units · Prerequisite: MFT 705

> “This is the advanced methods course that turns a proposal into a thesis.”

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MFT 721 — Research Methodology

MSc Marriage and Family Therapy · Semester Three · 1.71 Units · 14 teaching weeks · Prerequisite: MFT 705

This volume is one of 28 course books that together cover the whole MSc Marriage and Family Therapy taught programme. Each book is written to carry a 14–16 week semester on its own: every week pairs a core idea with the practice that follows from it, a worked case, and a reflective task, and each book closes with an integration chapter, a glossary and a self-assessment paper with model answers.

The books are designed to be read straight through before the semester begins, revisited week by week, and used again in revision. Cross-references point to the other volumes where a topic is treated in more depth.

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A note on the case material

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The lecture library

Twenty-eight self-contained course books carry the taught programme: eight in Semester One, seven in Semester Two, seven in Semester Three including the thesis, and six electives. The volume you are holding is marked in the margin.

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MFT 701	Fundamentals of Marriage and Family Therapy	One	1.71
MFT 702	Theories of Family Therapy	One	1.71
MFT 703	Human Development Across the Lifespan	One	1.71
MFT 704	Ethical and Legal Issues in Therapy	One	1.71
MFT 705	Research Methods in Marriage and Family Therapy	One	1.71
MFT 706	Clinical Practice and Techniques	One	1.71
MFT 707	Assessment and Diagnosis in Family Therapy	One	1.71
MFT 708	Cultural Competence in Therapy	One	1.71
MFT 709	Group Therapy Techniques	Two	1.71
MFT 710	Couples Therapy	Two	1.71
MFT 711	Child and Adolescent Therapy	Two	1.71
MFT 712	Crisis Intervention Strategies	Two	1.71
MFT 713	Substance Abuse and Family Dynamics	Two	1.71
MFT 714	Trauma-Informed Care	Two	1.71
MFT 715	Family Systems Theory	Two	1.71
MFT 716	Communication Skills in Therapy	Three	1.71
MFT 717	Psychopathology in Families	Three	1.71
MFT 718	Intimacy and Relationship Dynamics	Three	1.71
MFT 719	Family Life Cycle and Transitions	Three	1.71
MFT 720	Supervision and Consultation in Therapy	Three	1.71
MFT 721	**Research Methodology**	Three	1.71
MFT 722	Professional Development and Self-Care	Electives	1.71
MFT 723	Special Populations in Therapy	Electives	1.71
MFT 724	Family Law and Advocacy	Electives	1.71
MFT 725	Integrative Approaches in Therapy	Electives	1.71
MFT 726	Neurobiology of Relationships	Electives	1.71
MFT 727	Play Therapy Techniques	Electives	1.71
MFT 770	Master Thesis	Three	1.71

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Course leader's summary. This is the advanced methods course that turns a proposal into a thesis. You will work with real data, choose and justify analytic strategies, defend your design against examiners, and learn to write findings that practitioners will actually read.

How to use this book

Every week's task contributes directly to your thesis. Bring your own dataset, or use the teaching dataset supplied with the course. Do not read statistics passively; run them, break them, and interpret them badly at least once.

1. Course description

The course covers advanced design (longitudinal, dyadic, single-case, participatory), analysis (regression, multilevel modelling, dyadic models, thematic and framework analysis, mixed-methods integration), measurement and psychometric evaluation, systematic reviewing, data management and reproducibility, writing for publication, and defence of a study.

2. Learning outcomes

#	Outcome	Assessed
LO1	Select and justify a design for a complex relational question	Thesis proposal
LO2	Conduct and interpret appropriate analyses for relational data	Analysis portfolio
LO3	Evaluate the psychometric quality of a measure in a new population	Psychometric report
LO4	Integrate quantitative and qualitative findings coherently	Mixed-methods report
LO5	Defend design decisions against alternative explanations	Viva
LO6	Write a publishable findings chapter	Thesis chapter
LO7	Manage data ethically, securely and reproducibly	Data plan and audit

3. Assessment

Component	Weight	Due
Analysis portfolio (4 exercises)	30%	Weeks 5, 8, 11, 13
Psychometric evaluation report (1,500 words)	20%	Week 9
Mixed-methods integration report (2,000 words)	20%	Week 12
Thesis methodology chapter + viva	30%	Week 15

4. The fourteen-week plan

Week	Topic	Task
1	From proposal to protocol; pre-registration and its limits	Protocol draft
2	Design for relational questions	Design decision log
3	Sampling, power and feasibility in clinical settings	Power calculation
4	Data preparation and the analysis plan	Data cleaning log
5	**Analysis 1:** regression and its assumptions	Run and interpret
6	Multilevel models for nested family data	Model comparison
7	Dyadic models: APIM in practice	APIM exercise
8	**Analysis 2:** thematic and framework analysis	Coding audit
9	Psychometrics: reliability, factor structure, invariance	Report
10	Systematic review and meta-analysis basics	Search and appraise
11	**Analysis 3:** single-case and small-N designs	SCED exercise
12	Mixed methods and integration	Joint display
13	**Analysis 4:** robustness, missing data, common pitfalls	Sensitivity analysis
14	Writing, publishing and defending	Chapter and viva prep

Week 1 — Protocol and pre-registration

The protocol is the study's constitution: question, design, sample, measures, analysis plan, ethics, and timeline. Write it before you collect data; revise it with dated amendments if you must change course.

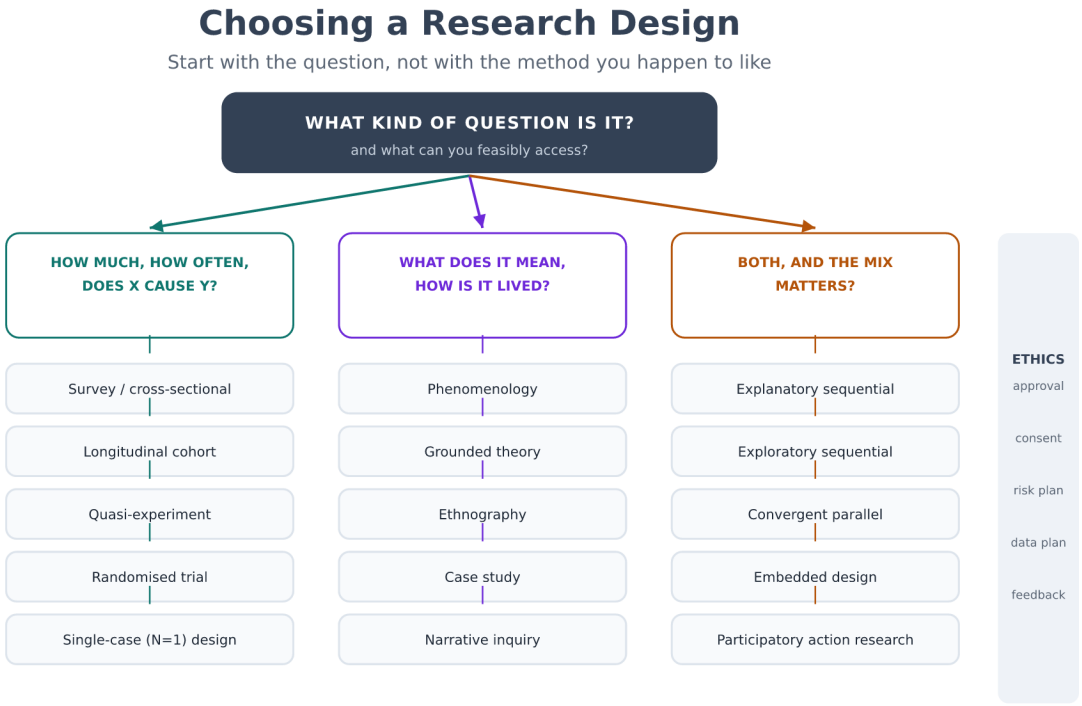
Pre-registration (e.g. on OSF) commits you publicly to a plan, which reduces the risk of unconscious analytical flexibility ("p-hacking"). It is standard in trials and increasingly expected elsewhere.

Honest limits. Exploratory qualitative work and community-based participatory research are often incompatible with rigid pre-registration. Declare your approach and be transparent about decisions made during the study.

A live analysis log — one page per week, dated — is the single most useful document in a thesis. It records what you did, what you decided, and why.

> **TASK**
> Draft a two-page protocol for your thesis. Include one paragraph on what you will do if recruitment is half of what you expect.

Week 2 — Design for relational questions



NOTE

In MFT the unit of analysis is the relationship, not only the individual — which creates real statistical problems: partners are non-independent. Plan for it (dyadic data analysis, actor–partner models, or a design that samples one person per household).

Figure 2.1 — Choosing a design from the question

Four designs especially suited to family questions.

Design	Question	Strength	Weakness
Longitudinal panel	Does X predict change in Y?	Temporal ordering	Attrition, cost
Dyadic (APIM)	How do partners affect each other?	Separates actor and partner effects	Requires both partners
Single-case experimental (SCED)	Does this intervention work for this family?	Clinical realism, causal inference within case	Generalisation
Participatory action research	What would improve this community's experience?	Validity, ethics, impact	Time, power negotiation

The design decision log. For each choice, record: the alternative considered, the reason for rejection, and the threat it leaves unresolved. Examiners reward this; it shows you know what your design cannot do.

Week 3 — Sampling, power and feasibility

Power. Do the calculation before you collect (GPower, R's `pwr`, or simulation). For dyadic and multilevel designs, power depends on the number of clusters* (couples, families), not the number of individuals.

Practical realities in clinical settings.

- Referral rates: how many families does the clinic actually see per week?
- Eligibility: what proportion will meet your criteria?
- Consent: what proportion will agree? (Often 30–60% in clinical samples.)
- Attrition: assume 20–30% at follow-up.
- Seasonality: school terms, holidays, religious calendars.

The feasibility sentence. "To recruit 60 couples in six months, I need 4 eligible referrals per week with a 60% consent rate; the clinic receives 9 per week; two colleagues have agreed to refer."

Week 4 — Data preparation

The unglamorous work that determines everything.

1. **Codebook** — every variable, its label, its values, its missingness convention.
2. **Raw data frozen** — never edit the original file after data entry.
3. **Cleaning log** — every decision (a value of 99 recoded to missing; one duplicate removed).
4. **Missing data** — describe the pattern; distinguish missing at random from missing not at random; consider multiple imputation rather than listwise deletion.
5. **Reverse scoring** — check; a mis-scored subscale invalidates the results.
6. **Outliers** — inspect, do not automatically delete.

> **TASK**
> Produce a codebook and cleaning log for your dataset (or the teaching dataset).

Week 5 — Regression

What regression does. Estimates the association between predictors and an outcome, adjusting for other predictors, and expresses it in the outcome's units.

Assumptions to check. Linearity, independence of errors, homoscedasticity, normality of residuals (for inference), and no severe multicollinearity. Report the checks; do not just assert them.

Reading output. Coefficient (direction, size, units), standard error, confidence interval, p-value, R^2, and model fit. Report effect sizes with confidence intervals, not only p-values.

Clinical translation. "Each additional point on the criticism scale was associated with a 0.3-point lower satisfaction score (95% CI −0.5 to −0.1), holding depression constant." That sentence is worth more than a table of asterisks.

Week 6 — Multilevel models

The problem. Individuals nested in couples, couples nested in therapists, repeated measures nested in people. Standard regression assumes independence and will produce confidence intervals that are too narrow.

The multilevel solution. Model the nesting explicitly:

- **Fixed effects** — the average relationship across the sample.
- **Random effects** — how intercepts (and slopes) vary across clusters.
- **ICC** — the proportion of variance attributable to the cluster. A high ICC means the clustering matters greatly and cannot be ignored.

Model-building sequence. Null model (ICC) → add level-1 predictors → add level-2 predictors → test random slopes → compare with likelihood ratio tests or information criteria (AIC/BIC).

Report. Always report the ICC, the number of clusters, and the estimation method.

Week 7 — Dyadic models

APIM in four numbers.

- **Actor effect (a):** my predictor on my outcome.
- **Partner effect (p):** my predictor on my partner's outcome.
- **Distinguishability:** whether partners are distinguishable (e.g. by gender or role) and whether effects can be constrained equal.
- **K:** the correlation between partners' residuals, which models the non-independence.

Interpretation example. In a study of 80 couples, criticism predicted lower satisfaction for the person giving it (actor effect) more strongly than for the person receiving it (partner effect). Clinical implication: the critic is also being harmed, which is a more useful reframe than "stop criticising your partner".

Software. R (`lme4`, `nlme`, `ape`), Mplus, SPSS MIXED.

Week 8 — Qualitative analysis

Two workable approaches.

Approach	Logic	Output
Reflexive thematic analysis (Braun & Clarke)	Themes constructed from coding, with the analyst's role explicit	Rich thematic account

Approach	Logic	Output
Framework analysis (Ritchie & Spencer)	Matrix of cases × themes, often for applied policy questions	Structured matrix, comparable across cases

Rigour in practice.

- Keep a **coding audit trail**: initial codes, revisions, discarded codes and why.
- Use **analytic memos** after each transcript.
- **Second-coder check** on at least 20% of data with a documented discussion of divergence.
- **Member reflection** where appropriate (not to validate themes, but to deepen them).
- **Reflexive journal** on your own reactions and assumptions.

The common error is presenting themes as "findings that emerged" from data, as though the analyst were not present. Themes are constructed; say so.

Week 9 — Psychometrics



Figure 9.1 — Validity, reliability and their limits

A psychometric evaluation of a measure in your population.

1. **Internal consistency** — Cronbach's α (and ω, which is more appropriate for congeneric items).

2. **Factor structure** — exploratory or confirmatory factor analysis; report fit indices (CFI, TLI, RMSEA, SRMR).
3. **Construct validity** — correlations with related and unrelated measures.
4. **Measurement invariance** — configural, metric, scalar, across groups (gender, language, age).
5. **Responsiveness** — change scores in a treated sample.

A local validation study is a perfectly good thesis, and often more useful to the field than another outcome study using an unvalidated instrument.

> **REPORT (20%)**
> A 1,500-word psychometric evaluation of one instrument, with recommendations.

Week 10 — Systematic review basics

Steps. Register the protocol (PROSPERO) → define the question (PICO) → search systematically (at least three databases; record the strategy) → screen (two reviewers; report agreement) → extract data → assess risk of bias (e.g. Cochrane RoB 2, or CASP for qualitative) → synthesise (narrative or meta-analytic) → assess certainty (GRADE).

Meta-analysis, briefly. Pool effect sizes with a random-effects model unless there is a justification for fixed effects; report heterogeneity (I^2, τ^2); explore it with subgroup analysis or meta-regression if there are enough studies; assess publication bias with a funnel plot and Egger's test, understanding their limitations.

What examiners look for. A search strategy that could be reproduced; a bias assessment that is not perfunctory; and a conclusion proportionate to the evidence.

Week 11 — Single-case and small-N designs

Why they matter in family therapy. Clinical samples are small, families are heterogeneous, and randomised trials are expensive. Single-case designs give causal inference within a case.

Core designs.

Design	Structure	Inference
A–B	Baseline, then intervention	Weak (confounded by time)
A–B–A	Baseline, intervention, withdrawal	Stronger if reversal occurs
Multiple baseline across behaviours	Staggered introduction	Strong; no withdrawal needed
Multiple baseline across participants	Staggered across families	Strong; realistic in clinics

Analysis. Visual analysis (level, trend, variability) plus a quantitative index (e.g. Tau-U) and, ideally, systematic replication across cases.

Reporting standard. Follow the SCED reporting guidelines (e.g. SCRIBE), including baseline stability and treatment fidelity.

Week 12 — Mixed methods integration

The integration question. What does the combination tell you that neither strand could alone?

Joint display format.

Quantitative finding	Qualitative finding	Convergence / divergence	Interpretation
Attendance predicted outcome (β = .32)	Families described "being known" by the therapist as decisive	Divergence in mechanism	Attendance may be a proxy for alliance
Drop-out 34%	Families cited transport and work shifts	Convergence	Structural barriers, not motivation

Avoid the two common failures. Reporting two parallel studies with no integration; and using qualitative data merely to illustrate quantitative findings.

> **REPORT (20%)**
> A 2,000-word integration report with at least three joint display rows.

Week 13 — Robustness and pitfalls

Pitfalls you will meet.

Pitfall	What it looks like	Fix
p-hacking	Many tests, reporting the significant ones	Pre-specify; adjust; report all
HARKing	Hypothesis stated after results known	Register the hypothesis
Overfitting	Model too complex for the sample	Simpler model; cross-validation
Ignoring clustering	Treating partners as independent	Multilevel or dyadic models
Selective reporting	Reporting one measure, ignoring another	Report all pre-specified outcomes
Causal language	"Caused" from cross-sectional data	Use associational language

Sensitivity analysis. Run your main analysis again with a different reasonable decision (e.g. excluding an outlier, using a different missing-data approach) and report whether conclusions change. If they do, say so.

Week 14 — Writing, publishing and defending

The findings chapter. Present, do not interpret (that is the discussion's job). Structure by question. Report descriptive statistics before inferential. Use tables for numbers and prose for meaning.

The discussion. What you found → how it compares with the literature → mechanisms → limitations (specific, not generic) → implications for practice and policy → future research.

Defending. Prepare for four questions: (1) Why this design? (2) What are the threats to validity and how did you address them? (3) Why these analyses? (4) What can and cannot be concluded? Rehearse short answers.

Publishing. Choose a journal that publishes practitioner research; follow the author guidelines exactly; expect a rejection and a revision; respond to reviewers point by point.

Glossary

APIM — Actor–Partner Interdependence Model. **Cluster** — a grouping unit (couple, family, therapist) whose members are not independent. **GRADE** — a system for rating the certainty of evidence. **ICC** — intraclass correlation; the share of variance due to clustering. **Measurement invariance** — equivalence of a measure's functioning across groups. **Pre-registration** — public commitment to a study plan before data collection. **SCED** — single-case experimental design. **Sensitivity analysis** — re-analysis under different assumptions to test robustness.

Self-assessment

Multiple choice

1. Treating partners as independent observations primarily affects: (a) external validity (b) standard errors and Type I error (c) reliability (d) sampling

2. The ICC indicates: (a) internal consistency (b) the proportion of variance due to clustering (c) effect size (d) power

3. A multiple baseline design is strong because: (a) it requires withdrawal (b) staggered introduction controls for time (c) it uses large samples (d) it is qualitative

4. Joint displays are used to: (a) present descriptive statistics (b) integrate quantitative and qualitative findings (c) assess bias (d) calculate power

5. Pre-registration mainly reduces: (a) attrition (b) analytical flexibility and p-hacking (c) cost (d) sample size

6. Measurement invariance concerns: (a) whether a measure behaves equivalently across groups (b) internal consistency only (c) sample size (d) randomisation

7. A sensitivity analysis tests: (a) whether conclusions hold under different assumptions (b) the sample's normality (c) the coding frame (d) the ethics

8. HARKing means: (a) hypothesising after results are known (b) random sampling (c) qualitative coding (d) data entry

9. In framework analysis the output is typically: (a) a case × theme matrix (b) a regression table (c) a factor loading matrix (d) a funnel plot

10. The most useful document in a thesis is: (a) the title page (b) a dated analysis log (c) the abstract (d) the reference list

Answer key

1 b · 2 b · 3 b · 4 b · 5 b · 6 a · 7 a · 8 a · 9 a · 10 b

Short answer

1. Explain why clustering matters for standard errors.
2. Distinguish actor and partner effects with an example.
3. List five elements of a systematic review protocol.
4. What is the difference between a sensitivity analysis and a subgroup analysis?
5. Name four threats to validity in a pre-post clinic study.

Essay (choose one, 2,000 words)

1. Defend (or attack) the claim that randomised trials are the gold standard for family therapy research.
2. Present a design for a dyadic study of criticism and satisfaction, justifying every choice.
3. Critically appraise a systematic review of family interventions, focusing on method and certainty.

Further reading

- Kenny, D. A., Kashy, D. A., & Cook, W. L. (2006). Dyadic Data Analysis. Guilford.
- Hox, J., Moerbeek, M., & van de Schoot, R. (2017). Multilevel Analysis (3rd ed.). Routledge.
- Braun, V., & Clarke, V. (2022). Thematic Analysis: A Practical Guide. Sage.
- Higgins, J. P. T. et al. (Eds.). (2023). Cochrane Handbook for Systematic Reviews of Interventions.
- Kazdin, A. E. (2021). Single-Case Research Designs (3rd ed.). OUP.

Next: MFT 770 — Master Thesis

MSC MARRIAGE AND FAMILY THERAPY

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MFT 722

Professional Development and Self-Care

14 teaching weeks · 1 practical · 1 figure

ELECTIVES

1.71 units · Prerequisite: MFT 706

> “This is not a wellbeing add-on at the end of training.”

One of 28 self-contained course books in the MSc Marriage and Family Therapy lecture library: a full taught semester per volume, with cases, practicals, assessment briefs, glossaries and model answers.

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MSc Marriage and Family Therapy · Elective · 1.71 Units · 14 teaching weeks · Prerequisite: MFT 706

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MFT 705	Research Methods in Marriage and Family Therapy	One	1.71
MFT 706	Clinical Practice and Techniques	One	1.71
MFT 707	Assessment and Diagnosis in Family Therapy	One	1.71
MFT 708	Cultural Competence in Therapy	One	1.71
MFT 709	Group Therapy Techniques	Two	1.71
MFT 710	Couples Therapy	Two	1.71
MFT 711	Child and Adolescent Therapy	Two	1.71
MFT 712	Crisis Intervention Strategies	Two	1.71
MFT 713	Substance Abuse and Family Dynamics	Two	1.71
MFT 714	Trauma-Informed Care	Two	1.71
MFT 715	Family Systems Theory	Two	1.71
MFT 716	Communication Skills in Therapy	Three	1.71
MFT 717	Psychopathology in Families	Three	1.71
MFT 718	Intimacy and Relationship Dynamics	Three	1.71
MFT 719	Family Life Cycle and Transitions	Three	1.71
MFT 720	Supervision and Consultation in Therapy	Three	1.71
MFT 721	Research Methodology	Three	1.71
MFT 722	**Professional Development and Self-Care**	Electives	1.71
MFT 723	Special Populations in Therapy	Electives	1.71
MFT 724	Family Law and Advocacy	Electives	1.71
MFT 725	Integrative Approaches in Therapy	Electives	1.71
MFT 726	Neurobiology of Relationships	Electives	1.71
MFT 727	Play Therapy Techniques	Electives	1.71
MFT 770	Master Thesis	Three	1.71

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Course leader's summary. This is not a wellbeing add-on at the end of training. It is the course that decides whether you are still practising in fifteen years, and whether the practising is any good. It treats the therapist's own life as the primary instrument, and professional development as a discipline rather than an ambition.

How to use this book

Every week has a **professional** task and a **personal** one. Do both. The course is assessed largely on a portfolio, and a portfolio written retrospectively in the final week will be visible and weak. Keep a weekly entry; it takes eight minutes.

1. Course description

The course covers: the person of the therapist; burnout, compassion fatigue and vicarious trauma; boundaries and availability; personal therapy; reflective practice and supervision; continuing professional development; career pathways and private practice fundamentals; money, contracts and pricing; professional identity and belonging; and sustainable practice in resource-limited settings.

2. Learning outcomes

#	Outcome	Assessed
LO1	Assess your own burnout and vicarious trauma risk using structured tools	Portfolio
LO2	Design a personal and professional self-care plan that survives a busy month	Plan
LO3	Set and maintain boundaries with clients, colleagues and institutions	Viva
LO4	Plan continuing professional development against a competency framework	CPD plan
LO5	Explain the fundamentals of ethical private practice (contracts, fees, records)	Exam
LO6	Use reflective practice and personal therapy appropriately	Portfolio
LO7	Articulate a five-year professional trajectory	Presentation

3. Assessment

Component	Weight	Due
Reflective portfolio (10 entries)	35%	Fortnightly
Personal and professional sustainability plan (1,500 words)	25%	Week 10
Practice readiness viva (20 min)	25%	Week 13
CPD and career presentation (10 min)	15%	Week 14

4. The fourteen-week plan

Week	Topic	Task
1	The person of the therapist	Therapy autobiography (private)
2	Burnout: what it is and how it develops	Maslach self-assessment
3	Compassion fatigue and vicarious trauma	Symptom inventory
4	Boundaries: time, contact, disclosure, gifts	Boundary audit
5	Personal therapy and its place in training	Decide and plan
6	Reflective practice: models and habits	Weekly reflection structure
7	Supervision as a career-long practice	Supervision plan
8	CPD and competency frameworks	CPD gap analysis
9	Career pathways in MFT	Pathway map
10	**Sustainability plan due**; money, fees and contracts	Fee and contract design
11	Private practice fundamentals	Practice blueprint
12	Professional identity, community and belonging	Professional network plan
13	**Practice readiness viva**	Assessed
14	**CPD and career presentation**	Assessed

Week 1 — The person of the therapist

The premise. You cannot separate the clinician from the person. Your history shapes what you notice, what you avoid, whom you like, and where you lose your balance.

The "therapy autobiography". Write, privately, about: your family of origin; the rules you absorbed; the losses; the roles you played; what you learned about conflict, intimacy, money and authority; and why you chose this work. Do not submit it. Do keep it, and revisit it in three years.

Common motivations and their risks.

Motivation	Strength	Risk
"I want to help people"	Compassion	Rescuing; over-functioning
"I want to understand my family"	Insight	Using clients to solve your own history
"People tell me I'm easy to talk to"	Alliance skill	Difficulty setting boundaries
"I survived something"	Credibility	Over-identifying; avoidance of certain material
"I want status/expertise"	Drive	Distance; difficulty with not-knowing

The professional requirement. Know your history well enough that you can tell the difference between your client's material and your own reaction. Personal therapy and supervision exist for this, not because something is wrong with you.

Week 2 — Burnout

Definition (Maslach): a syndrome of **emotional exhaustion**, **depersonalisation/cynicism**, and **reduced sense of accomplishment**, arising from chronic occupational stress.

The six drivers (Maslach & Leiter) — all of them organisational:

1. Workload
2. Control (or lack of it)
3. Reward (financial, social, intrinsic)
4. Community
5. Fairness
6. Values (mismatch between what you believe and what the system asks)

The clinically important point. Burnout is not a personal failing and cannot be solved by yoga. If the caseload is impossible, the supervision absent and the values compromised, self-care is a distraction from the real problem.

Self-assessment. Use the Maslach Burnout Inventory or the freely available Copenhagen Burnout Inventory. Score yourself honestly; write one paragraph on which of the six drivers is most active in your current situation.

Week 3 — Compassion fatigue and vicarious trauma

Three distinct concepts.

Concept	Definition	Signature
Burnout	Exhaustion from workload	"I am too tired to care"
Compassion fatigue	Reduced empathy and satisfaction from caring	"I used to feel for people; now I don't"
Vicarious trauma	Changes to beliefs and inner world from exposure to others' trauma	"The world is not safe; people are dangerous"

Signs to monitor in yourself. Intrusion of client material; nightmares; avoidance of certain cases; cynicism; irritability at home; numbness; loss of hope; over-involvement; boundary erosion; sleep and appetite change; increased alcohol use.

Protective factors with evidence. Supervision that addresses the emotional impact; caseload diversity; control over scheduling; peer support; meaning-making; sleep and exercise; personal therapy; and organisational support.

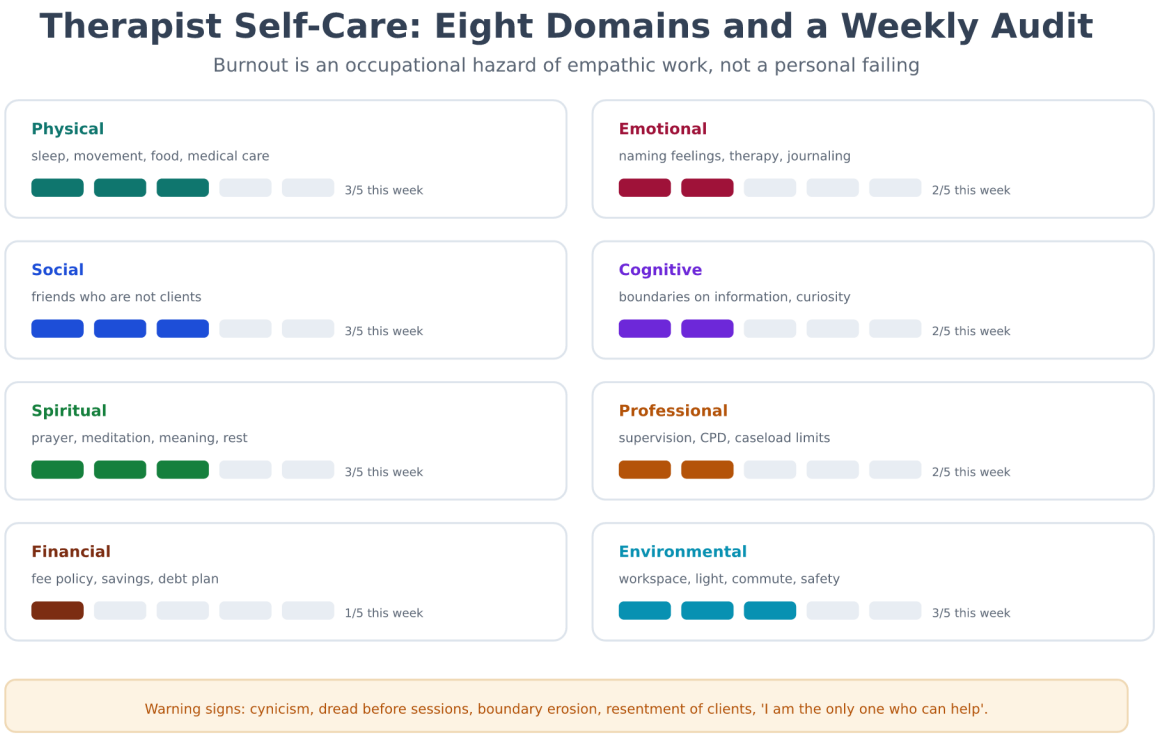


Figure 3.1 — Self-care audit across eight domains

TASK

Complete the eight-domain audit. Rate each domain 0–5 for last week. Identify the lowest two and write one change for each that is small enough to happen on a Tuesday.

Week 4 — Boundaries

The boundaries that matter most in early practice.

Boundary	The trap	A workable standard
Time	Sessions over-run; you arrive early, leave late	Fixed session length; 10 minutes between clients
Contact	Clients text at night; you reply	Written policy; emergency route; no personal number
Disclosure	You share to reassure	Disclose for the client's benefit, rarely, and briefly
Gifts	Refusing feels rude	Accept small cultural gifts; discuss significant ones
Social media	Client requests; you see their posts	Clear policy; never search for clients
Dual roles	Church, community, family ties	Disclose, discuss, refer if unmanageable
Emotional labour	You become the family's friend	Warmth plus frame; endings are explicit

The test. Not "was it kind?" but "whose need did it serve, and would I be comfortable explaining it in supervision?"

Week 5 — Personal therapy

Requirements vary. Many training programmes require a minimum number of personal therapy hours. Treat the requirement as a floor, not a target.

What it is for. Not to become "fixed", but to know your patterns well enough to notice them in the room; to have a place to process work that is not your supervisor's job; and to experience being a client.

What to look for in a therapist. A different orientation from yours (to see another way of working); a recommendation from someone you trust; a clear contract; and someone you can be honest with.

The cost problem. Therapy is expensive, and trainees are often poorly paid. Options: reduced-fee schemes, group therapy, university counselling services, peer consultation groups, and budgeting it as a professional expense rather than a luxury.

Week 6 — Reflective practice

Reflection is a method, not a mood. Use a structure.

The Gibbs cycle, applied to a session: description → feelings → evaluation → analysis → conclusion → action plan.

Six questions to answer weekly.

1. What happened that surprised me?
2. What did I feel, and where in my body?
3. What did I avoid?
4. What did I do that worked?
5. What would I do differently?
6. What do I need to bring to supervision?

Writing. Keep one page per week. Do not write case details in a personal journal that is not secure; use initials or pseudonyms, and never store client-identifying material outside the clinical record.

Week 7 — Supervision as a career-long practice

The norms. One hour of supervision for every five hours of clinical work is the common standard; more for complex cases and for trainees.

How to use supervision well.

- Arrive with an agenda and one clear question.
- Bring the case you least want to bring.
- Say what you felt, not only what you did.
- Ask for observation, not only opinion ("what would you have said?").
- Record what you will do differently, and report back next time.

Peer consultation groups. A complement, not a substitute: 3–5 clinicians, monthly, structured, with a confidentiality agreement and a rotating facilitator.

Week 8 — Continuing professional development

Frameworks. Most professional bodies require annual CPD across domains: clinical practice, supervision, ethics, research, teaching, management. Map your year against the domains rather than collecting certificates.

A CPD plan that works.

1. Identify three gaps from your competency self-assessment.
2. For each, specify an activity, a date, and an evidence of completion.
3. Include one activity that stretches you (a new modality, a research skill, a leadership role).
4. Review every six months; adjust.

Beware CPD theatre. Ten webinars on topics you already know count for nothing. One supervised course in an area of genuine weakness counts for a great deal.

Week 9 — Career pathways

Pathway	What it needs	What it gives	What it costs
Clinical specialist	Depth in a modality; supervision; certification	Expertise, referrals	Narrowing
Public sector clinician	Systems navigation; resilience	Security, breadth	Bureaucracy, heavy caseloads
Private practice	Business skills, capital, network	Autonomy, income potential	Isolation, income volatility
Academic/teaching	Research, publication, teaching	Influence, stability	Bureaucracy, distance from clinic
Supervision/consultation	Experience, accreditation	Leverage, variety	Responsibility for others' practice
NGO/humanitarian	Flexibility, languages, resilience	Meaning, breadth	Insecurity, exposure to trauma
Policy/advocacy	Writing, networks, patience	Scale of impact	Slowness, compromise

The honest question. Which of these fits the life you actually want — including your family, your health, and your finances — not only the career you imagined at 22?

Week 10 — Money, fees and contracts

Fees are clinical decisions. Set them deliberately; publish them; apply them consistently; review annually.

A fee policy covers: the fee; session length; payment method and timing; late cancellation; non-attendance; sliding scale criteria (decided in advance); annual review; and what happens to outstanding balances.

Contracts. Even a one-page agreement prevents most disputes: parties, purpose, confidentiality and limits, fees, cancellation, contact policy, complaints route, and signatures.

The uncomfortable truths.

- Charging is not exploitation; under-charging until you resent clients is.
- A sliding scale with no criteria becomes arbitrary and unfair.
- Chasing unpaid fees is a clinical conversation, not an administrative one.
- You will need to budget for tax, insurance, supervision, CPD and sick leave.

Week 11 — Private practice fundamentals

The blueprint. Location and room (privacy, sound, waiting area, accessibility); legal structure and registration; professional indemnity insurance; record-keeping and data protection; booking and payment systems; referral network; risk and emergency protocols; supervision arrangement; and a marketing approach that is ethical and honest.

Starting safely. Keep a part-time employed post while you build; take a business course or find a mentor; decide your niche only after 50 cases, not before; and never practise without supervision, however experienced you are.

Isolation is the occupational hazard. Build a peer group before you need one.

Week 12 — Professional identity and belonging

Belonging matters. Clinicians who are connected to a professional community report less burnout, better decision-making, and greater persistence.

How to build it. Join a professional association; attend one conference a year; contribute to a peer group; teach one workshop; write one short piece; mentor one junior colleague.

Identity in a contested field. Family therapy sits between psychology, psychiatry, social work and counselling. Be clear about who you are and what you offer, without disparaging

others: "I am a systemic therapist; my expertise is in relationships. I work alongside your psychiatrist, not instead of them."

Week 13 — Practice readiness viva

Twenty minutes. Questions drawn from: boundary scenarios, burnout scenarios, ethical dilemmas, fee conversations, supervision difficulties, and career planning. Assessed on judgement, honesty, and the ability to name your own limits.

Week 14 — CPD and career presentation

Ten minutes on where you are, where you are going, what you need, and what you will stop doing. Include one commitment to your own sustainability that is specific, dated, and verifiable.

Glossary

Burnout — exhaustion, cynicism and reduced efficacy from chronic occupational stress. **Compassion fatigue** — reduced empathy and satisfaction resulting from prolonged caring. **Compassion satisfaction** — the positive counterpart: meaning and pleasure derived from helping. **CPD** — continuing professional development. **Reflective practice** — structured examination of one's own work and reactions. **Vicarious trauma** — transformation of the therapist's inner world through exposure to others' trauma.

Self-assessment

Multiple choice

1. Burnout is primarily caused by: (a) personal weakness (b) chronic occupational stressors (c) too much supervision (d) client resistance

2. Vicarious trauma is characterised by: (a) tiredness (b) changes to beliefs and inner world (c) low fees (d) poor timekeeping

3. A workable contact boundary is: (a) giving your personal number (b) a written policy with an emergency route (c) replying at night (d) no policy

4. Personal therapy in training is best understood as: (a) a punishment (b) a professional requirement and an investment (c) unnecessary (d) a substitute for supervision

5. The recommended supervision ratio is roughly: (a) 1 hour per 20 client hours (b) 1 hour per 5 client hours (c) 1 hour per year (d) no standard

6. A useful CPD plan starts from: (a) available webinars (b) a competency gap analysis (c) cheap courses (d) colleagues' choices

7. Fee policies should be: (a) improvised (b) written, published and applied consistently (c) secret (d) identical everywhere

8. Peer consultation groups: (a) replace supervision (b) complement supervision (c) are unnecessary (d) are illegal

9. Reflection is best done: (a) once a year (b) weekly with a structure (c) only in crisis (d) verbally only

10. Private practice should ideally begin: (a) with no supervision (b) alongside employed work with supervision in place (c) with maximum debt (d) without insurance

Answer key

1 b · 2 b · 3 b · 4 b · 5 b · 6 b · 7 b · 8 b · 9 b · 10 b

Short answer

1. Name the three dimensions of burnout and the six organisational drivers.
2. Distinguish burnout, compassion fatigue and vicarious trauma.
3. List five boundaries and the standard you will use for each.
4. What are the six questions of a weekly reflection?
5. Outline the components of a fee policy.

Essay (choose one, 1,500 words)

1. "Self-care is a professional obligation, not a luxury." Discuss with reference to the evidence on burnout.
2. Design a sustainable practice model for a newly qualified family therapist in a low-resource setting.
3. Analyse your own family-of-origin patterns and their likely effect on your clinical work.

Further reading

- Norcross, J. C., & Guy, J. D. (2019). Leaving It at the Office (2nd ed.). Guilford.
- Figley, C. R. (Ed.). (1995). Compassion Fatigue. Brunner/Mazel.
- Maslach, C., & Leiter, M. P. (2016). Understanding the burnout experience. World Psychiatry, 15(2), 103–111.
- Skovholt, T. M., & Trotter-Mathison, M. (2014). The Resilient Practitioner (3rd ed.). Routledge.
- Bolton, G. (2018). Reflective Practice (5th ed.). Sage.

Next: MFT 723 — Special Populations in Therapy

MSC MARRIAGE AND FAMILY THERAPY

LECTURE LIBRARY · VOLUME 24 OF 28

MFT 723

Special Populations in Therapy

ELECTIVES

1.71 units · Prerequisite: MFT 708, 714

> “"Special populations" is a compromise term.”

One of 28 self-contained course books in the MSc Marriage and Family Therapy lecture library: a full taught semester per volume, with cases, practicals, assessment briefs, glossaries and model answers.

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MFT 723 — Special Populations in Therapy

MSc Marriage and Family Therapy · Elective · 1.71 Units · 14 teaching weeks · Prerequisite: MFT 708, 714

This volume is one of 28 course books that together cover the whole MSc Marriage and Family Therapy taught programme. Each book is written to carry a 14–16 week semester on its own: every week pairs a core idea with the practice that follows from it, a worked case, and a reflective task, and each book closes with an integration chapter, a glossary and a self-assessment paper with model answers.

The books are designed to be read straight through before the semester begins, revisited week by week, and used again in revision. Cross-references point to the other volumes where a topic is treated in more depth.

How to cite this volume

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A note on the case material

All families, couples and individuals in this book are fictional composites. They are written to teach pattern recognition, not to describe any real person. Nothing here replaces supervised clinical practice, professional registration, legal advice or medical care; legal content is orientation only and must be checked against the law of the relevant jurisdiction.

The lecture library

Twenty-eight self-contained course books carry the taught programme: eight in Semester One, seven in Semester Two, seven in Semester Three including the thesis, and six electives. The volume you are holding is marked in the margin.

Code	Course	Semester	Units
MFT 701	Fundamentals of Marriage and Family Therapy	One	1.71
MFT 702	Theories of Family Therapy	One	1.71
MFT 703	Human Development Across the Lifespan	One	1.71
MFT 704	Ethical and Legal Issues in Therapy	One	1.71
MFT 705	Research Methods in Marriage and Family Therapy	One	1.71
MFT 706	Clinical Practice and Techniques	One	1.71
MFT 707	Assessment and Diagnosis in Family Therapy	One	1.71
MFT 708	Cultural Competence in Therapy	One	1.71
MFT 709	Group Therapy Techniques	Two	1.71
MFT 710	Couples Therapy	Two	1.71
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MFT 720	Supervision and Consultation in Therapy	Three	1.71
MFT 721	Research Methodology	Three	1.71
MFT 722	Professional Development and Self-Care	Electives	1.71
MFT 723	**Special Populations in Therapy**	Electives	1.71
MFT 724	Family Law and Advocacy	Electives	1.71
MFT 725	Integrative Approaches in Therapy	Electives	1.71
MFT 726	Neurobiology of Relationships	Electives	1.71
MFT 727	Play Therapy Techniques	Electives	1.71
MFT 770	Master Thesis	Three	1.71

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Course leader's summary. "Special populations" is a compromise term. It names groups whom services tend to miss, misread or harm — and it trains the clinician to adapt the frame without lowering the standard of care.

How to use this book

Each week takes one population and works through four questions: what is the specific stressor, what does the evidence say helps, what must be adapted, and what should I not do? The aim is competence in adaptation, not expertise in every group.

1. Course description

The course covers families affected by forced displacement; LGBTQ+ clients and their families; people with disabilities and chronic illness; older adults; families involved with the criminal justice system; veterans and uniformed services; single-parent and grandparent-headed households; childless and infertile couples; families affected by HIV; and street-connected children.

Each carries a specific combination of stigma, legal precarity, service exclusion and clinical complexity that requires a specific adaptation of the systemic frame.

2. Learning outcomes

#	Outcome	Assessed
LO1	Identify the specific stressors and strengths of each population	Exam
LO2	Adapt assessment and intervention without diluting standards	Case study
LO3	Recognise and interrupt the ways services exclude these groups	Reflective portfolio
LO4	Work with interpreters, advocates and community organisations	Skills test
LO5	Address legal, safeguarding and ethical complexities	Viva
LO6	Evaluate the evidence for culturally adapted interventions	Essay
LO7	Design an accessible service pathway	Proposal

3. Assessment

Component	Weight	Due
Case study with adaptation plan (2,500 words)	40%	Week 9
Service pathway proposal (1,500 words)	25%	Week 12
Viva on legal and ethical complexity (20 min)	15%	Week 13
Examination	20%	Week 14

4. The fourteen-week plan

Week	Population	Adaptation focus
1	Families affected by forced displacement	Safety, loss, limbo
2	Refugees and asylum-seeking families	Legal precarity and the "second trauma"
3	LGBTQ+ clients and their families	Risk, disclosure, acceptance
4	Families with a member with a disability	Access and the social model
5	Chronic illness and palliative care families	Illness as a family member
6	Older adults and their families	Ageing, loss, elder abuse
7	Criminal justice involvement and imprisonment	Absence, stigma, re-entry
8	Families of uniformed services	Deployment, injury, silence
9	**Case study due**; single-parent and grandparent-headed households	Structure and load
10	Infertility, childlessness and reproductive loss	Stigma, grief, gender
11	Families affected by HIV	Stigma, secrecy, transmission anxiety
12	**Pathway proposal due**; street-connected children and youth	Safety, trust, systems
13	**Viva**; intersectionality and service design	Integrated planning
14	Integration and examination	Review

Week 1 — Forced displacement

The specific stressors. Loss (home, status, language, network, identity); prolonged uncertainty; trauma before, during and after flight; family separation; role reversal; and the "limbo" of waiting for legal status.

The clinical tasks. Safety and basic needs first; normalising distress without pathologising; restoring roles; supporting the family's own rituals and faith; and addressing the grief of the life left behind — often unmourned because the family is still in survival mode.

Adaptation.

- Do not require a full trauma narrative in early sessions.
- Use practical problem-solving alongside emotional work; both are therapeutic.
- Involve community and faith organisations.
- Ask about the family members who are missing — literally and emotionally.

Week 2 — Asylum-seeking families

The "second trauma" is the legal process: interviews, credibility assessments, detention, dispersal, uncertainty, and the requirement to tell the worst story repeatedly to strangers.

Clinical realities.

- Status anxiety dominates every decision: whether to report violence, whether to seek medical care, whether to disclose sexuality.
- Children may be the family's cultural and linguistic bridge.
- Court and immigration reports may be requested; the report and the therapy must be kept distinct.

What helps. Concrete assistance with documentation and access; clear, honest explanation of what you will write and to whom; liaison with legal representatives (with consent); and a therapeutic space where the person is not required to justify their deserving.

Week 3 — LGBTQ+ clients and their families

Minority stress. Prejudice, expectation of rejection, internalised stigma and concealment produce chronic stress and elevated rates of anxiety, depression, self-harm and suicide.

Family work. Families often present in crisis after disclosure, with religious and cultural conflict. The work is to: create safety for the young person; hear the parents' fear without endorsing rejection; slow the timetable (rejection, like acceptance, is a process); and set limits on harm — verbal abuse, expulsion, forced "corrective" interventions.

What not to do.

- Do not promise to change the young person, or to change the family.
- Do not require the client to come out as a condition of care.
- Do not collude with secrecy that is unsafe, or with disclosure that is premature.
- Do not refer to "the lifestyle"; use the client's language.

The clinical stance. You are not there to adjudicate theology or politics. You are there to reduce harm, improve relationships where possible, and support the person's dignity.

Week 4 — Disability

The two models. Medical (the body is the problem) and social (society's barriers are the problem). Clinically, both matter.

What families need.

- Accurate information about the condition and its trajectory.
- Help with the burden of care and the sibling experience.
- Support for the parental relationship, which is often absorbed by the caregiving.

- Advocacy with schools, employers and health services.
- Attention to the disabled person's own voice and autonomy, which families often override in the name of protection.

Adaptation. Accessible rooms; longer sessions; communication support; written materials in accessible formats; and a willingness to conduct sessions in the home when necessary.

Week 5 — Chronic illness and palliative care

The illness as a third member of the family. It takes a role, demands attention, reorganises routines and relationships.

Clinical tasks. Rebalance the couple relationship when one partner becomes a carer; address the children's understanding and fears; manage the family's beliefs about illness and fate; support decision-making about treatment; and prepare for death without abandoning hope.

Palliative and end-of-life work. Advance care planning; legacies; unfinished business; the family's conflicts about treatment; and bereavement support afterwards.

The clinician's own mortality. This work touches your own fears. Supervision is essential.

Week 6 — Older adults

Specific stressors. Retirement, bereavement, physical decline, loss of role, relocation, dependence, and the assumption by others that life is over.

Clinical presentations. Depression misdiagnosed as ageing; anxiety; cognitive change (which must be differentiated from delirium, depression, hearing loss and dementia); family conflict about care; elder abuse; and grief.

Adaptation. Slower pace; larger print; hearing support; sessions at home; involving the family in practical decisions; and taking the older person's goals as primary, not the family's convenience.

Elder abuse. Ask directly and separately: "Has anyone hurt you, taken your money, or stopped you seeing people?" Know your safeguarding route.

Week 7 — Criminal justice involvement

The absence. Imprisonment removes a person from the household while keeping them psychologically present. Children are often told a story; partners carry shame and practical burden.

Clinical tasks. Support the remaining household; address the children's understanding and stigma; prepare for release and re-entry; manage the couple's renegotiation; and address the family's history (which may include the behaviour that led to imprisonment).

Re-entry. The returning member often expects to resume an old role, while the family has reorganised without them. Naming this mismatch early prevents a second rupture.

Adaptation. Sessions in prison settings; rules about contact; careful confidentiality (the prison is not a confidential setting); and coordination with probation or social services.

Week 8 — Uniformed services

The specific culture. Stoicism, mission focus, hierarchy, and a strong norm against help-seeking.

Clinical presentations. Deployment and reintegration; moral injury; PTSD; substance use; domestic violence; and the family's secondary traumatisation.

Adaptation. Frame treatment in terms the culture respects (performance, mission readiness, the family's stability); involve the unit where appropriate; and address the couple's shift in roles during absence and return.

Week 9 — Single-parent and grandparent-headed households

The specific stressors. Role overload; financial strain; the absent parent's shadow; stigma; and children's questions.

Strengths. Many single-parent households function very well; treat the structure as a structure, not a deficit.

Grandparent-headed households. Grief (for the parent, often due to death or incapacity); the generation gap; legal status (custody, maintenance); health limitations; and the children's loyalty conflicts.

Adaptation. Practical support first; a clear parenting plan; support for the child's contact with the absent parent where safe; and attention to the carer's own health.

Week 10 — Infertility and childlessness

The cultural weight. In many settings, childlessness carries severe social stigma, blamed disproportionately on women, with consequences for marriage stability and family standing.

Clinical themes. Grief (often disenfranchised); shame; blame and the couple's conflict; pressure from extended family; religious meaning; treatment decision-making and its cost; and the couple's sexual relationship, which becomes medicalised.

The clinical stance. Do not assume that parenthood is the goal for every couple. Ask what a good life looks like for them. Name the social pressure as external, not personal failure.

Week 11 — Families affected by HIV

The clinical realities. Stigma and secrecy; disclosure decisions (to partner, children, family, church); medication adherence; transmission anxiety; bereavement and orphanhood; and the intersection with poverty and gender inequality.

Disclosure work. Help the person decide who, when and how; rehearse; prepare for different responses; and ensure the children's questions are answered at their developmental level.

Adaptation. Confidentiality is paramount in small communities; coordinate with HIV services carefully; address the family's beliefs about illness and blame; and support the "worried well" partner as well.

Week 12 — Street-connected children and youth

The specific realities. Survival needs first (food, safety, shelter); mistrust of adults and institutions; peer networks that function as family; substance use; exposure to violence and exploitation; and legal vulnerability.

What helps. Outreach and continuity; a trusted adult who does not disappear; harm reduction; family reunification work only where it is safe; and collaboration with community organisations, not only clinics.

What harms. Coercive removal; moralising; short-term projects that end abruptly; and treating the peer group as pathology when it is often the only protection available.

Week 13 — Intersectionality and service design

The design questions.

1. Who does not come to our service, and why?
2. What are the access barriers — cost, transport, language, hours, stigma, documentation, disability?
3. Who is the first point of contact, and are they trained?
4. What does the physical environment communicate?
5. What happens when a client needs an interpreter, a wheelchair, a child-friendly space, or a private entrance?
6. How do we measure whether these groups are actually being served?

A pathway that works typically includes: community outreach; a named link worker; flexible appointments; interpreters; transport support; plain-language materials; and a feedback loop with the communities served.

> **PROPOSAL (25%)**
> A service pathway for one under-served population, with access analysis, staffing, and outcome measures.

Week 14 — Integration

The generalizable skills. Ask rather than assume; adapt the frame, not the standard; work with community organisations; attend to legal precarity; protect confidentiality in small communities; and examine your own discomfort.

The generalizable caution. "Special population" can become a way of othering. Every family is specific; the skill is in the specificity, not in the label.

Glossary

Asylum seeker — a person seeking protection who has not yet been granted refugee status. **Disenfranchised grief** — grief that is not socially recognised or supported. **Minority stress** — chronic stress from stigma, prejudice and concealment. **Moral injury** — psychological distress from acting against one's values in service. **Social model of disability** — disability located in societal barriers rather than the body.

Self-assessment

Multiple choice

1. The "second trauma" for asylum seekers refers to: (a) the original event (b) the legal process (c) the journey (d) therapy

2. Family work after a young person's coming out should first: (a) change the young person (b) create safety and hear the parents' fear (c) enforce acceptance (d) require celibacy

3. The social model of disability locates disability in: (a) the body (b) societal barriers (c) genetics (d) the family

4. Elder abuse should be: (a) assumed unlikely (b) asked about directly and separately (c) ignored (d) reported without consent always

5. On release from prison, families commonly: (a) resume old roles easily (b) have reorganised without the returning member (c) refuse contact (d) need no support

6. Childlessness in many settings is: (a) neutral (b) severely stigmatised and blamed on women (c) rare (d) a legal matter

7. For street-connected youth, the first priority is: (a) family therapy (b) survival needs and a trusted adult (c) diagnosis (d) schooling only

8. Minority stress explains elevated rates of: (a) income (b) anxiety, depression and self-harm (c) physical fitness (d) employment

9. Effective pathways for under-served groups begin with: (a) more clinics (b) access analysis and community outreach (c) longer waiting lists (d) more diagnosis

10. The risk of the term "special populations" is: (a) inaccuracy (b) othering and stereotyping (c) legality (d) cost

Answer key

1 b · 2 b · 3 b · 4 b · 5 b · 6 b · 7 b · 8 b · 9 b · 10 b

Short answer

1. Name four specific stressors of forced displacement and one clinical task for each.
2. What are three "do nots" in family work after a coming-out disclosure?
3. How does the social model of disability change your assessment?
4. What must a service pathway include for under-served populations?
5. Why is confidentiality harder in small communities, and what do you do about it?

Essay (choose one, 2,000 words)

1. "Adaptation without dilution." Discuss with reference to one special population.
2. Critically evaluate the evidence for culturally adapted family interventions.
3. Design an accessible family therapy service for one excluded group in your context.

Further reading

- Falicov, C. J. (2014). Latino Families in Therapy (2nd ed.). Guilford.
- Papadopoulos, R. K. (Ed.). (2002). Therapeutic Care for Refugees. Karnac.
- Laing, L., & Topping, K. (2019). Family Therapy with Older Adults. Routledge.
- Lev, A. I., & Sennott, S. L. (2019). Gender and Sexuality in Family Therapy. Routledge.
- Miller, G. (2011). Management of Alcohol and Drug Problems. (For harm-reduction principles in outreach work.)

Next: MFT 724 — Family Law and Advocacy

MSC MARRIAGE AND FAMILY THERAPY

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MFT 724

Family Law and Advocacy

14 teaching weeks · 2 figures

ELECTIVES

1.71 units · Prerequisite: MFT 704

> “Family therapists work at the edge of the legal system: custody disputes, protection proceedings, divorce, maintenance, guardianship.”

One of 28 self-contained course books in the MSc Marriage and Family Therapy lecture library: a full taught semester per volume, with cases, practicals, assessment briefs, glossaries and model answers.

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MFT 724 — Family Law and Advocacy

MSc Marriage and Family Therapy · Elective · 1.71 Units · 14 teaching weeks · Prerequisite: MFT 704

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A note on the case material

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The lecture library

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Code	Course	Semester	Units
MFT 701	Fundamentals of Marriage and Family Therapy	One	1.71
MFT 702	Theories of Family Therapy	One	1.71
MFT 703	Human Development Across the Lifespan	One	1.71
MFT 704	Ethical and Legal Issues in Therapy	One	1.71
MFT 705	Research Methods in Marriage and Family Therapy	One	1.71
MFT 706	Clinical Practice and Techniques	One	1.71
MFT 707	Assessment and Diagnosis in Family Therapy	One	1.71
MFT 708	Cultural Competence in Therapy	One	1.71
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MFT 713	Substance Abuse and Family Dynamics	Two	1.71
MFT 714	Trauma-Informed Care	Two	1.71
MFT 715	Family Systems Theory	Two	1.71
MFT 716	Communication Skills in Therapy	Three	1.71
MFT 717	Psychopathology in Families	Three	1.71
MFT 718	Intimacy and Relationship Dynamics	Three	1.71
MFT 719	Family Life Cycle and Transitions	Three	1.71
MFT 720	Supervision and Consultation in Therapy	Three	1.71
MFT 721	Research Methodology	Three	1.71
MFT 722	Professional Development and Self-Care	Electives	1.71
MFT 723	Special Populations in Therapy	Electives	1.71
MFT 724	**Family Law and Advocacy**	Electives	1.71
MFT 725	Integrative Approaches in Therapy	Electives	1.71
MFT 726	Neurobiology of Relationships	Electives	1.71
MFT 727	Play Therapy Techniques	Electives	1.71
MFT 770	Master Thesis	Three	1.71

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Course leader's summary. Family therapists work at the edge of the legal system: custody disputes, protection proceedings, divorce, maintenance, guardianship. This course gives you the legal literacy to be useful, the boundaries to stay in your lane, and the advocacy skills to change the systems your clients pass through.

How to use this book

This is orientation, not legal advice. Statutes, case law and state practice vary and change. Every week names the questions you should ask a lawyer, not the answers you should give a client. The final weeks turn from individual cases to systemic advocacy.

1. Course description

The course covers: the Nigerian marriage and family law framework (statutory, customary, Islamic and cohabitation); divorce and dissolution; custody, access and maintenance; the welfare principle; child rights and protection law; domestic violence legislation; mediation and alternative dispute resolution; report writing and expert testimony; the therapist's role in court; and advocacy for policy and service change.

2. Learning outcomes

#	Outcome	Assessed
LO1	Explain the four marriage systems and their dissolution routes	Exam
LO2	Describe the legal framework for custody, maintenance and child protection	Exam
LO3	Distinguish therapy, mediation, expert assessment and legal advice	Viva
LO4	Write a defensible court-facing report within your competence	Report
LO5	Prepare a client for legal processes without becoming an advocate for a version of events	Case study
LO6	Conduct safe mediation or refer appropriately	Skills test
LO7	Design an advocacy campaign or policy proposal	Proposal

3. Assessment

Component	Weight	Due
Legal framework examination	25%	Week 7
Court-facing report (2,000 words)	30%	Week 10
Mediation skills assessment (30 min)	20%	Week 12
Advocacy proposal (1,500 words + presentation)	25%	Week 14

4. The fourteen-week plan

Week	Topic	Practical
1	Why family therapists need legal literacy	Role boundary mapping
2	Four marriage systems	Case classification
3	Divorce and dissolution	Route mapping
4	Custody, access and the welfare principle	Custody scenario
5	Maintenance and financial provision	Calculation exercise
6	Child rights and protection law	Reporting scenario
7	**Examination**; domestic violence law	Protection order pathway
8	Mediation and ADR	Mediation demonstration
9	The therapist in court: reports and testimony	Report drafting
10	**Report due**; expert versus treating roles	Role conflict analysis
11	Working with lawyers and courts	Interprofessional simulation
12	**Mediation assessment**	Assessed
13	Advocacy: from case to policy	Campaign design
14	**Advocacy proposal**; integration	Presentations

Week 1 — Why legal literacy matters

Three reasons.

1. **Your clients are inside legal processes.** Divorce, custody, maintenance, protection orders, immigration, inheritance. If you do not know the terrain, you cannot help them navigate it.
2. **You will be asked for opinions you should not give.** "Will I get custody?" is a legal question with a clinical component. Confusing them harms clients.
3. **You may be a witness or a report-writer.** The quality of your documentation determines whether your evidence is useful or damaging.

The boundary map.

Activity	Who does it	Your role
Legal advice	Lawyer	Refer; explain the difference
Mediation	Trained mediator	Conduct only if trained and if safe; otherwise refer
Therapeutic work	You	Your core role
Expert assessment (custody)	Independent expert	Generally not your own client

Activity	Who does it	Your role
Court testimony	Witness	Fact witness; opinion only within expertise
Advocacy	You, as a professional	Legitimate, within your competence and evidence

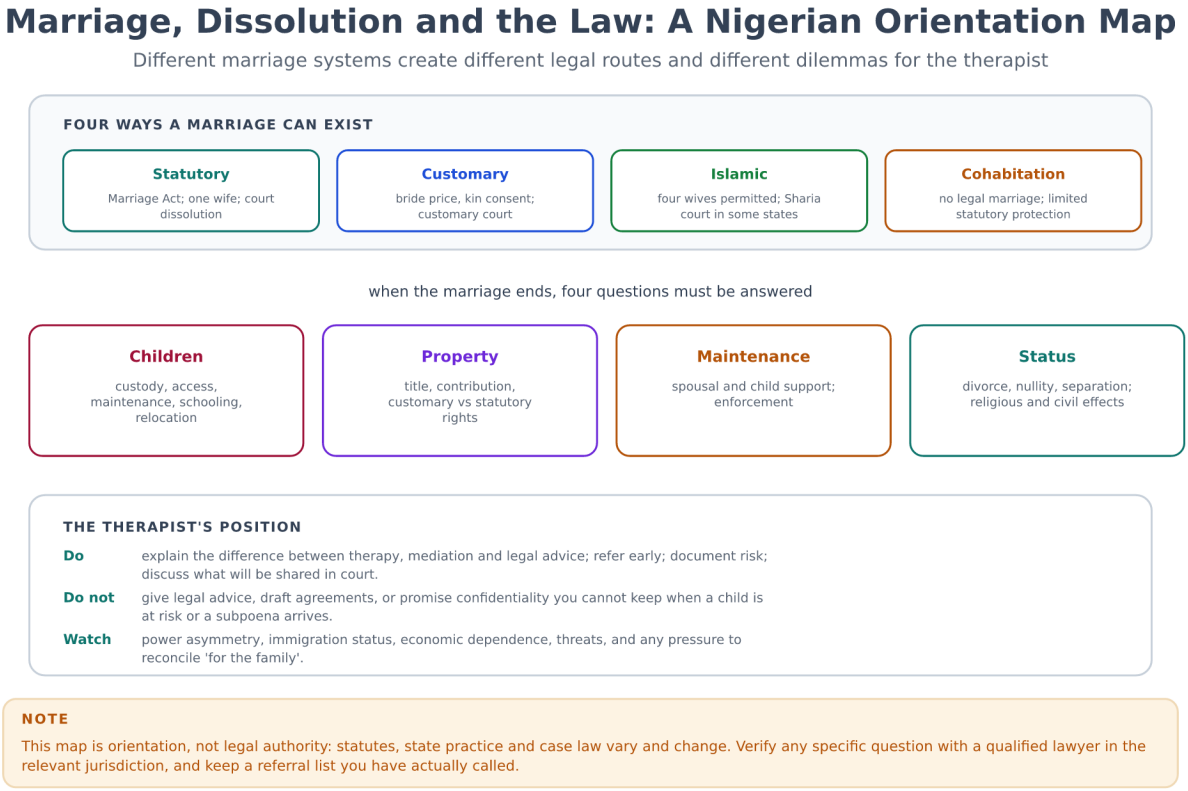


Figure 1.1 — Marriage systems, dissolution questions and the therapist's position

Week 2 — Four marriage systems

System	Legal basis	Form	Dissolution route
Statutory	Marriage Act	Monogamous; registered	High Court petition under the Matrimonial Causes Act
Customary	Customary law; state customary courts	Bride price, kin consent; potentially polygynous	Customary court; customary arbitration
Islamic	Islamic law; Sharia courts in relevant states	Up to four wives	Sharia court; talaq, khul', or judicial dissolution
Cohabitation	No marriage	—	No dissolution; limited statutory protection

Clinical implications of the overlap. A couple may be married under two systems; the customary marriage may not be registered; the religious ceremony may have legal consequences; and children's status may depend on which system applies. Do not assume.

Ask: "How was your marriage formalised? Was it registered? Is there a certificate? Which court would you go to if you needed to?" Then refer to a lawyer for the answer's consequences.

Week 3 — Divorce and dissolution

Statutory divorce (orientation). Governed by the Matrimonial Causes Act: a petition to the High Court; a two-year minimum duration (with exceptions); a recognised ground (adultery, unreasonable behaviour, desertion, separation, or living apart); and provision for custody, maintenance, property and costs.

Customary and Islamic dissolution. Different processes, different courts, different evidentiary rules, different outcomes for women in practice. Know where to refer.

The therapist's role.

- Help the couple/clients clarify what they are deciding (discernment).
- Explain that therapy is not a legal process and does not produce evidence for court.
- Support the emotional work alongside the legal process.
- Refer for legal advice early, not at the point of crisis.
- Warn clients that therapy records may be requested in proceedings.

Week 4 — Custody, access and the welfare principle

The welfare principle. Decisions about children are governed by the child's best interests, not the parents' preferences or the mother's automatic entitlement.

What courts consider (broadly). The child's wishes (weighted by age and understanding); continuity of care; the capacity of each parent; the child's relationship with each parent; siblings; education and health; and any risk of harm.

The therapist's contribution. You can describe the child's relationships, needs and views (carefully, and with appropriate methods); you can support the parents to reduce conflict; and you can help them construct a parenting plan.

What you should not do. Offer a custody recommendation for your own client. That is an expert assessment requiring independence, a defined methodology and often a court appointment. Role conflict invalidates the opinion and harms the family.

The parenting plan. Residence, contact, holidays, schooling, health decisions, communication, transitions, handovers, new partners, and a dispute-resolution clause. Concrete plans reduce conflict.

Week 5 — Maintenance and financial provision

Child maintenance. Both parents have an obligation to maintain their children, under statutory and customary law. Amounts are determined by need, capacity and standard of living; enforcement is a practical problem.

Spousal maintenance. Available in some statutory proceedings; less so in customary and religious frameworks and highly variable in practice.

The therapist's role. Money is clinical material: it carries meaning about fairness, power and abandonment. Help the couple separate the amount from the meaning; encourage full financial disclosure; and refer for legal advice on entitlements.

Practical support. Referral to legal aid, mediation services, and women's rights organisations where appropriate.

Week 6 — Child rights and protection

The Child Rights Act (2003) — adopted in many but not all Nigerian states — sets out the rights of the child, including protection from abuse and neglect, the right to be heard, and provisions on child marriage, child labour, and alternative care. State child protection laws and structures operationalise it.

Your duties.

1. Recognise signs of abuse and neglect.
2. Respond without leading or investigating.
3. Record verbatim.
4. Report through the designated route (state ministry responsible for social welfare; police family unit; designated safeguarding lead).
5. Document the advice received and the action taken.
6. Support the child through the process.

What not to do. Investigate yourself; promise secrecy; confront the alleged abuser; or delay the report while you "gather more information".

Week 7 — Domestic violence law

The Violence Against Persons (Prohibition) Act 2015 applies in the FCT and in states that have adopted it; other states have their own laws or rely on criminal provisions. Protection orders, duties of the police and support for victims vary substantially by jurisdiction.

The clinician's knowledge. Where the nearest police family unit is; what a protection order requires; which shelters and legal aid organisations exist; and what happens procedurally to a woman who reports.

The clinician's limits. You are not the police, the lawyer, or the judge. Your role is safety planning, documentation, referral and support — and not making the client's decisions for her.

Week 8 — Mediation and alternative dispute resolution

What mediation is. A structured, voluntary process in which a neutral third party helps the parties reach their own agreement. It is not therapy and not arbitration.

When it is suitable. Both parties voluntary and safe; no coercive control; reasonably balanced power; an issue that can be negotiated; and willingness to disclose information.

When it is not. Violence or coercion; a severe power imbalance (including literacy, language or economic dependency); untreated substance use or mental illness that prevents participation; and safeguarding concerns.

Structure of a mediation session. Opening and ground rules → each party's account → identification of issues → exploration of interests → generation of options → negotiation → agreement documented → review.

The therapist's temptation. To slip into therapy mid-mediation (exploring feelings) or into advice. Explain the frame at the start and hold it.

Week 9 — Reports and court

The report structure.

1. Your role, qualifications and the basis of your opinion.
2. Who you saw, when, and with whose consent.
3. Methodology (interviews, observation, measures).
4. Findings, described behaviourally.
5. Opinion, clearly separated and limited to your expertise.
6. Limitations, explicitly stated.
7. Recommendations, if requested and within competence.

Language. "It is my opinion that…" not "the court should". No adjectives about character. No speculation about events you did not observe.

Testimony. Answer the question asked; do not volunteer; distinguish fact from opinion from hypothesis; say "I don't know" when you don't; and remain courteous under challenge.

Records. Assume that anything you write may be read in court. Write accordingly.

Week 10 — Expert versus treating roles

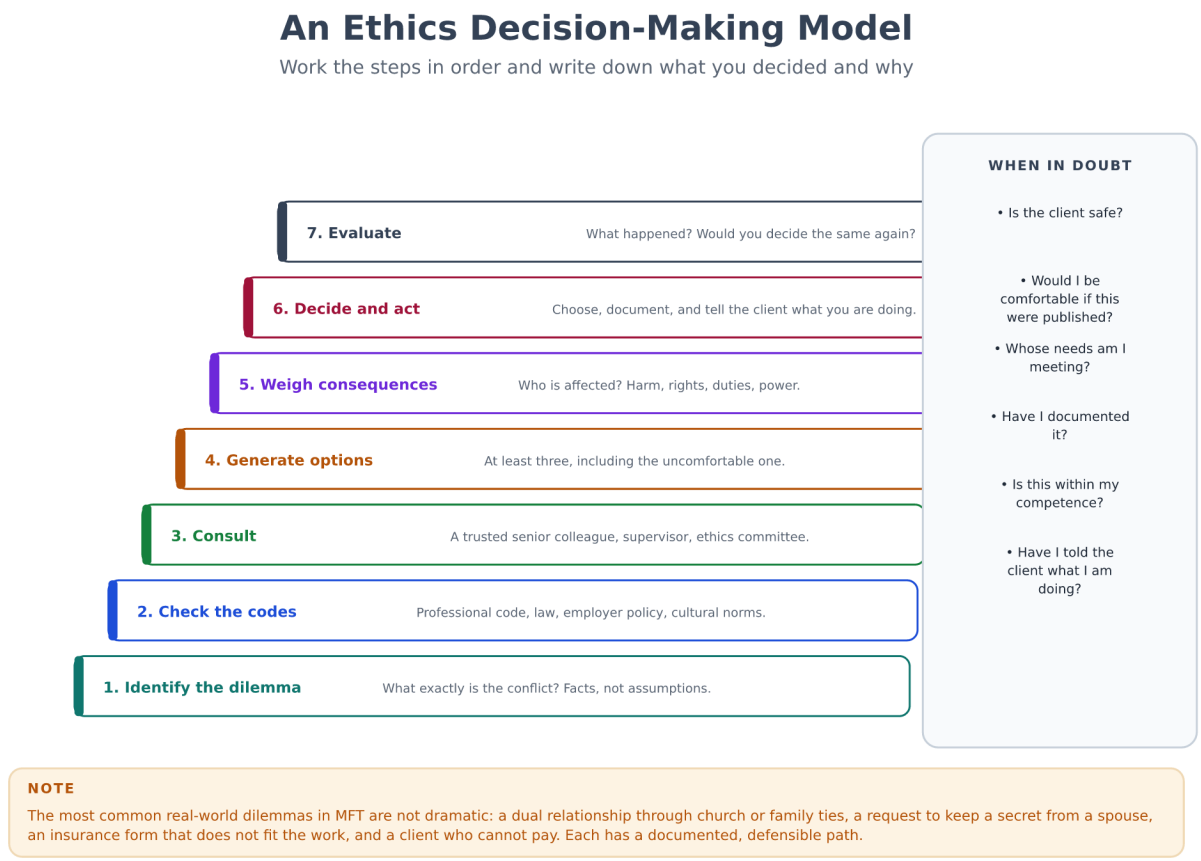


Figure 10.1 — An ethics decision model for role conflicts

Treating clinician	Expert assessor
Paid by/for the client; therapeutic duty	Paid by the court; duty to the court
Confidential	Limited confidentiality; report disclosed
Advocates for the client's wellbeing	Neutral; reports what the evidence shows
Continuous relationship	Time-limited, defined assessment
Cannot be objective about custody	Can be objective if independent

The conflict is structural, not a matter of willpower. Accepting both roles compromises the therapy and the evidence. If asked, explain and refer.

Where you can help. Provide factual records with consent and a court order as required; support the client through the process; and help parents reduce conflict.

Week 11 — Working with lawyers and courts

What lawyers need from you. Clear, dated, factual information; a timely response; an opinion you can defend; and an understanding of your limits.

What you need from them. Clarity about the purpose of the report; the legal questions to address; the deadline and format; and confirmation of consent arrangements.

Interprofessional etiquette. Do not tell a lawyer their job; do not let a lawyer tell you your clinical findings. Negotiate the boundary explicitly and in writing.

Week 12 — Mediation skills assessment

Thirty minutes, role-played. Assessed on: framing and ground rules; neutrality; screening for safety; interest-based exploration; option generation; documentation; and referral decisions.

Week 13 — Advocacy

From case to policy. Individual work reveals systemic failures: no shelter, no legal aid, no interpreter, no protection order enforcement, no follow-up. Advocacy is the professional response.

Levels.

1. **Case advocacy** — with one client: writing a letter, making a call, accompanying to a service (with care).
2. **Service advocacy** — improving your own organisation's pathway.
3. **Policy advocacy** — submissions, research briefs, media, coalitions.
4. **Community advocacy** — working with community organisations and faith groups.

The evidence base for advocacy. Documented case patterns, service data, and research. Anecdote persuades sympathetic listeners; data persuades sceptical ones.

Ethical guardrails. Consent before using a client's story; never identify a client; separate your professional opinion from your personal politics; and do not compromise clinical relationships for a campaign.

Week 14 — Advocacy proposal and integration

Present a 1,500-word proposal: the problem, the evidence, the target, the ask, the allies, the timeline, and how you will know whether it worked. Then, in the final discussion, return to the boundary map from Week 1 and revise it with what you have learned.

Glossary

ADR — alternative dispute resolution. **Coercive control** — a pattern of domination and isolation that makes mediation unsafe. **Guardian ad litem** — a person appointed to represent a child's interests in proceedings. **Maintenance** — financial support owed to a spouse or child. **Matrimonial Causes Act** — the Nigerian statute governing statutory divorce. **Protection order** — a court order restraining a person from specified conduct. **Welfare principle** — the rule that the child's best interests govern decisions about them.

Self-assessment

Multiple choice

1. A therapist asked by a client "will I get custody?" should: (a) predict the outcome (b) explain the welfare principle and refer for legal advice (c) refuse to speak (d) advise on strategy

2. Conducting a custody evaluation of your own client is: (a) acceptable with consent (b) a role conflict (c) required (d) ideal

3. Mediation is contraindicated when: (a) parties disagree (b) there is coercive control (c) there are children (d) money is involved

4. The welfare principle concerns: (a) the parents' rights (b) the child's best interests (c) the court's convenience (d) the family's income

5. A child protection concern should first be: (a) investigated by you (b) recorded and reported through the designated route (c) discussed with the family (d) held until you are certain

6. In mediation the therapist should: (a) explore childhood (b) hold the mediation frame (c) give legal advice (d) take sides

7. An expert report should separate: (a) facts from opinions (b) parents from children (c) fees from findings (d) nothing

8. Advocacy with a client's story requires: (a) no consent (b) informed consent and anonymisation (c) media involvement (d) the client's name

9. The four marriage systems include all except: (a) statutory (b) customary (c) Islamic (d) corporate

10. Therapy records in litigation should be treated as: (a) permanently confidential (b) potentially disclosable (c) irrelevant (d) destroyed immediately

Answer key

1 b · 2 b · 3 b · 4 b · 5 b · 6 b · 7 a · 8 b · 9 d · 10 b

Short answer

1. Distinguish therapy, mediation, expert assessment and legal advice.
2. What are the grounds for refusing to mediate?
3. Outline a court-facing report structure.
4. List the steps when you suspect child abuse.
5. Give three examples of legitimate professional advocacy.

Essay (choose one, 2,000 words)

1. "The therapist's duty is to the family, the expert's duty is to the court." Analyse the consequences.

2. Critically examine the protection available to victims of domestic violence in your jurisdiction.
3. Design an advocacy campaign to improve one aspect of family justice in your context.

Further reading

- Matrimonial Causes Act (Nigeria), Cap M7 LFN 2004.
- Child Rights Act (Nigeria), 2003.
- Violence Against Persons (Prohibition) Act, 2015.
- Stahl, P. M. (2011). Conducting Child Custody Evaluations (2nd ed.). Sage.
- Emery, R. E. (2012). Renegotiating Family Relationships (2nd ed.). Guilford.
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Next: MFT 725 — Integrative Approaches in Therapy

MSC MARRIAGE AND FAMILY THERAPY

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MFT 725

Integrative Approaches in Therapy

14 teaching weeks · 1 figure

ELECTIVES

1.71 units · Prerequisite: MFT 702, 715

> “Most experienced clinicians are integrative, whether or not they say so.”

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MSc Marriage and Family Therapy · Elective · 1.71 Units · 14 teaching weeks · Prerequisite: MFT 702, 715

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MFT 714	Trauma-Informed Care	Two	1.71
MFT 715	Family Systems Theory	Two	1.71
MFT 716	Communication Skills in Therapy	Three	1.71
MFT 717	Psychopathology in Families	Three	1.71
MFT 718	Intimacy and Relationship Dynamics	Three	1.71
MFT 719	Family Life Cycle and Transitions	Three	1.71
MFT 720	Supervision and Consultation in Therapy	Three	1.71
MFT 721	Research Methodology	Three	1.71
MFT 722	Professional Development and Self-Care	Electives	1.71
MFT 723	Special Populations in Therapy	Electives	1.71
MFT 724	Family Law and Advocacy	Electives	1.71
MFT 725	**Integrative Approaches in Therapy**	Electives	1.71
MFT 726	Neurobiology of Relationships	Electives	1.71
MFT 727	Play Therapy Techniques	Electives	1.71
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Course leader's summary. Most experienced clinicians are integrative, whether or not they say so. The question is whether the integration is disciplined or accidental. This course builds a principled framework for combining models without producing incoherence, and teaches you to justify every combination.

How to use this book

Integration is a design problem, not a mood. Each week adds a dimension to a framework you will use for the rest of your career. By Week 14 you will have a written integrative framework, tested on three cases, and defended in a viva.

1. Course description

The course covers: the varieties of integration (assimilative, theoretical, sequential, pluralistic); common factors and their implications; the major dimensions of family therapy (systemic, attachment, behavioural, experiential, cognitive, narrative, biological); decision frameworks for matching model to client and phase; feedback-informed treatment; staged and stepped care; and the risks of eclecticism.

2. Learning outcomes

#	Outcome	Assessed
LO1	Distinguish the forms of integration and their assumptions	Exam
LO2	Explain common factors and use them to justify technique selection	Exam
LO3	Construct a coherent integrative formulation for a complex case	Case study
LO4	Sequence models across phases with an explicit rationale	Treatment plan
LO5	Use routine outcome monitoring and feedback to guide integration	Portfolio
LO6	Avoid incoherence and demonstrate coherence to a supervisor	Viva
LO7	Articulate a personal integrative framework	Framework paper

3. Assessment

Component	Weight	Due
Integrative case study (2,500 words)	35%	Week 8
Treatment plan with sequenced models (1,500 words)	25%	Week 11
Outcome monitoring portfolio	15%	Week 13
Framework paper and viva (2,000 words)	25%	Week 14

4. The fourteen-week plan

Week	Topic	Practical
1	Why integrate: evidence, clients, and clinicians	Integration audit of your own practice
2	Forms of integration	Mapping four forms onto a case
3	Common factors and their clinical use	Common-factors observation
4	The systemic dimension	Systemic formulation
5	The attachment dimension	Attachment-informed plan
6	The behavioural and cognitive dimensions	Skills module design
7	The experiential and narrative dimensions	Emotion and meaning work
8	**Case study due**; the biological dimension	Medication-aware plan
9	Matching, sequencing and staging	Sequence design
10	Outcome monitoring and feedback-informed treatment	ORS/SRS practice
11	**Treatment plan due**; comorbidity and complexity	Complex case lab
12	Culture, values and the ethics of integration	Values audit
13	**Portfolio due**; coherence and supervision	Coherence defence
14	**Framework paper and viva**	Assessed

Week 1 — Why integrate

Three pressures.

1. **Evidence.** No single model is superior for everything. Different models have stronger evidence for different problems (FBT for adolescent anorexia, EFT for couple distress, PMT for conduct problems, family psychoeducation for psychosis).
2. **Clients.** Real families present with multiple, interacting problems: trauma plus addiction plus couple distress plus a child's school refusal.
3. **Clinicians.** Most experienced therapists drift towards integration as their caseload diversifies.

The danger. Integration without discipline becomes eclecticism: a technique for every mood, no coherent theory of change, and a client who cannot tell what is expected of them.

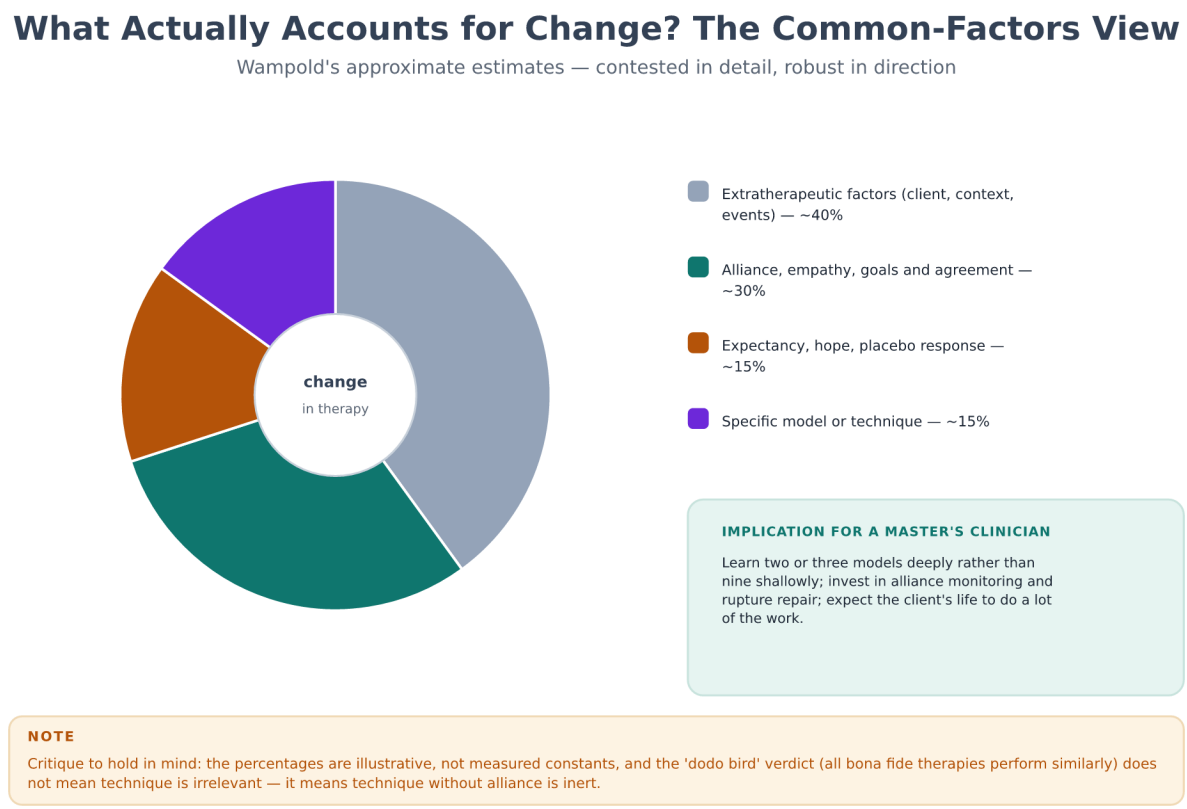


Figure 1.1 — What accounts for change: a common-factors view

> **AUDIT**
> List the models you actually use. For each, name the client problem you use it for and the theory of change you rely on. Identify any technique you use without being able to explain why.

Week 2 — Forms of integration

Form	Definition	Example	Risk
Assimilative	A home model, enriched by techniques from elsewhere	EFT therapist borrowing a behavioural task	Drift from the core theory
Theoretical	A new model synthesising several	Integrative behavioural couples therapy	Complexity, training burden
Sequential	Different models at different phases	Structural stabilisation, then EFT	Discontinuity for the family
Parallel	Different models for different subsystems	Couple work + individual trauma work	Splitting between clinicians
Pluralistic	Collaborating with the client on method	"Which approach fits you?"	Requires an informed client

Choosing a form. Assimilative integration is the most realistic for a newly qualified clinician: it keeps coherence while allowing flexibility. Sequential and parallel integration are appropriate for complexity. Pluralistic practice is an ethical stance as much as a technique.

Week 3 — Common factors

The claim. Much of psychotherapy's effect comes from factors shared across models: the alliance, agreement on goals, therapist empathy, client expectancy, and extratherapeutic factors.

The clinical use — without relativism.

1. **Monitor the alliance** in every session, with every family member.
2. **Repair ruptures** quickly; the repair is often more powerful than the original intervention.
3. **Agree goals explicitly**; disagreement about goals predicts drop-out.
4. **Engender hope** honestly (not false reassurance).
5. **Then use techniques** — which matter most for specific problems and for structuring the work.

The critique. Percentages attributed to common factors are illustrative, not measured; "all models are equal" is over-stated; and technique does matter for specific disorders. The defensible position: the alliance is necessary but not sufficient; technique without alliance is inert; alliance without technique is often insufficient.

Week 4 — The systemic dimension

What it contributes. Attention to pattern, structure, triangles, context and power. It tells you where to look and what maintains the problem.

What it lacks alone. Specific techniques for symptom change; guidance for individual emotion and trauma; and a sufficient account of biology.

Integration in practice. Systemic formulation + any intervention. You can use a behavioural task, an EFT enactment, or a narrative externalising conversation, provided the systemic formulation justifies it.

Week 5 — The attachment dimension

What it contributes. A developmental account of why bonds matter; a map of strategies under threat; a mechanism for change (corrective emotional experience, enacted in the room).

Integration with systemic work. Systemic thinking tells you the loop; attachment tells you why the loop matters so much and what each person is asking for.

Integration with behavioural work. Behavioural tasks build positive exchange; attachment work deepens the meaning of the exchange. Alone, either can be shallow.

Week 6 — The behavioural and cognitive dimensions

What they contribute. Specific, testable, teachable skills; measurable change; homework; structure; and a strong evidence base for many problems.

Integration cautions. Behavioural work can feel mechanical when the emotional bond is damaged; cognitive restructuring can be premature in acute trauma; and skills practice without meaning can become performance.

A practical sequence. Stabilise and structure (behavioural) → access emotion and meaning (experiential, attachment) → consolidate with skills and plans (behavioural, cognitive).

Week 7 — The experiential and narrative dimensions

What they contribute. Access to primary emotion; meaning-making; identity; and the language of the family's own story.

Integration. Experiential work deepens the emotional field; narrative work organises it into a story the family can hold. Together they answer "what do we feel?" and "what does it mean?".

Caution. Emotional work without containment can flood; narrative work without emotion can become intellectual. The integration is in the pacing.

Week 8 — The biological dimension

What it contributes. Sleep, medication, substances, hormonal change, illness, and the physiology of stress and trauma.

Integration. A formulation that ignores biology will fail in predictable ways: the depressed client who does not respond to therapy because of untreated hypothyroidism; the couple whose sexual problem is a medication side effect; the adolescent whose sleep deprivation mimics ADHD.

Your role. Screen, refer, coordinate, and never advise on medication.

Week 9 — Matching, sequencing and staging

Decision framework — five questions.

1. **What is the most dangerous thing here?** (Risk governs sequence.)
2. **What is the most changeable thing here?** (Start where change is possible; hope is built on early wins.)

3. **What must be in place before other work can happen?** (Safety, stabilisation, attendance, sobriety.)
4. **Which model has the best evidence for the primary problem?** (Use it, or refer.)
5. **What does this family want and believe in?** (Engagement is a precondition, not a luxury.)

Staging example. A couple with active violence, alcohol misuse, and a child's school refusal: Phase 1 — safety and sobriety (individual, specialist referral); Phase 2 — couple de-escalation and parenting plan; Phase 3 — child's return to school and family consolidation.

Week 10 — Feedback-informed treatment

The instruments. The Outcome Rating Scale (ORS) and Session Rating Scale (SRS) are brief, free, and practical: four visual analogue items each, completed every session.

How to use them.

1. Introduce them as a way of making sure the therapy is working.
2. Look at the scores before you speak.
3. Ask about a drop: "Your score fell by three points. What happened?"
4. Ask about the alliance: "Is there anything about how we work that is not fitting?"
5. Act on the answer in the session, not next month.

The evidence. Routine outcome monitoring with feedback improves outcomes and reduces deterioration, particularly for clients who are not progressing. It is one of the most robust findings in the field.

> **PORTFOLIO (15%)**
> Ten sessions with ORS/SRS data, your interpretation, and the changes you made.

Week 11 — Comorbidity and complexity

Complexity is the norm. Typical community caseloads include trauma plus substance use plus couple distress plus child behaviour plus financial strain.

Principles.

1. Formulate the whole system, not a list of diagnoses.
2. Identify the keystone: the one change that would make others possible.
3. Sequence: safety → stabilisation → skills → processing → consolidation.
4. Share the load: refer where specialist input is needed; coordinate rather than compete.
5. Review: complexity requires more frequent review, not longer sessions.

Week 12 — Culture, values and the ethics of integration

Culture shapes which models fit. An approach that prizes emotional expressiveness may feel exposing; one that prizes autonomy may feel cold; one that requires homework may be impractical.

Values shape technique choice. Your beliefs about gender, family duty, forgiveness and individual autonomy will show up in what you emphasise. Audit them.

Ethical integration. Do not use a technique you are not trained in; do not combine models to appear sophisticated; and do not use integration as a reason to avoid referring.

Week 13 — Coherence

The coherence test.

Question	If the answer is no…
Can I state the formulation in three sentences?	You do not have one yet
Does every intervention follow from the formulation?	You are improvising
Can I name the model I am using at this moment?	You are drifting
Can I predict what will happen next, and be wrong productively?	Your formulation is too vague
Would a supervisor understand my reasoning without me explaining it?	Document it

The coherence defence. Present a case to peers and answer: what are you doing, why, and how will you know whether it worked?

> **COHERENCE DEFENCE**
> Ten minutes with a case, followed by hostile questioning from peers.

Week 14 — Your integrative framework

Write 2,000 words:

1. Your home model and its theory of change.
2. The dimensions you integrate (systemic, attachment, behavioural, experiential, narrative, biological).
3. Your decision rules for matching and sequencing.
4. Your use of feedback and outcome data.
5. Your limits: what you do not do, and to whom you refer.
6. How you will know you are drifting.

Defend it in a viva.

Glossary

Assimilative integration — a home model enriched by techniques from elsewhere. **Common factors** — mechanisms shared across therapies (alliance, empathy, expectancy, goals). **Feedback-informed treatment** — using session-by-session outcome and alliance data to adjust therapy. **Keystone change** — a change that makes other changes possible. **Pluralistic practice** — collaborating with the client to choose methods. **Sequential integration** — using different models at different phases of treatment. **Stepped care** — matching intensity of intervention to need.

Self-assessment

Multiple choice

1. The main risk of undisciplined integration is: (a) cost (b) incoherence for the family (c) slow progress (d) paperwork

2. Assimilative integration means: (a) equal blending (b) a home model enriched by borrowed techniques (c) alternating models by session (d) refusing a model

3. Common factors research suggests: (a) technique is irrelevant (b) alliance matters greatly and technique matters for specific problems (c) all models are identical (d) diagnosis is useless

4. The first question in sequencing is: (a) what is most interesting (b) what is most dangerous (c) what is cheapest (d) what the therapist prefers

5. Feedback-informed treatment uses: (a) long questionnaires (b) brief session-by-session measures such as ORS/SRS (c) only diagnosis (d) supervisor ratings

6. A keystone change is: (a) the first session (b) a change that makes others possible (c) a diagnosis (d) a referral

7. The dimensional approach includes all except: (a) systemic (b) attachment (c) behavioural (d) astrological

8. Integration is unethical when it involves: (a) borrowing a technique (b) using techniques outside your competence (c) sequencing models (d) monitoring outcomes

9. Coherence is demonstrated by: (a) using many techniques (b) a formulation from which interventions follow (c) longer sessions (d) confidence

10. Complexity should lead to: (a) longer sessions (b) more frequent review and coordination (c) more diagnoses (d) abandoning formulation

Answer key

1 b · 2 b · 3 b · 4 b · 5 b · 6 b · 7 d · 8 b · 9 b · 10 b

Short answer

1. Distinguish four forms of integration with an example of each.

2. Give three clinical uses of common-factors research.
3. State the five sequencing questions.
4. How do you use ORS/SRS data in a session?
5. What are three ethical limits on integration?

Essay (choose one, 2,000 words)

1. "Integration without a theory of change is improvisation." Discuss with a case.
2. Design an integrative treatment plan for a family with trauma, addiction and couple distress.
3. Critically evaluate feedback-informed treatment as a professional standard.

Further reading

- Lebow, J. L. (2014). Couple and Family Therapy: An Integrative Map of the Territory. APA.
- Sprenkle, D. H., Davis, S. D., & Lebow, J. L. (2009). Common Factors in Couple and Family Therapy. Guilford.
- Duncan, B. L., Miller, S. D., & Sparks, J. A. (2004). The Heroic Client. Jossey-Bass.
- Norcross, J. C., & Goldfried, M. R. (Eds.). (2019). Handbook of Psychotherapy Integration (3rd ed.). OUP.

Next: MFT 726 — Neurobiology of Relationships

MSC MARRIAGE AND FAMILY THERAPY

LECTURE LIBRARY · VOLUME 27 OF 28

MFT 726

Neurobiology of Relationships

14 teaching weeks · 4 figures

ELECTIVES

1.71 units · Prerequisite: MFT 714

> “Neuroscience does not replace systemic thinking; it supplies the mechanism.”

One of 28 self-contained course books in the MSc Marriage and Family Therapy lecture library: a full taught semester per volume, with cases, practicals, assessment briefs, glossaries and model answers.

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MFT 726 — Neurobiology of Relationships

MSc Marriage and Family Therapy · Elective · 1.71 Units · 14 teaching weeks · Prerequisite: MFT 714

This volume is one of 28 course books that together cover the whole MSc Marriage and Family Therapy taught programme. Each book is written to carry a 14–16 week semester on its own: every week pairs a core idea with the practice that follows from it, a worked case, and a reflective task, and each book closes with an integration chapter, a glossary and a self-assessment paper with model answers.

The books are designed to be read straight through before the semester begins, revisited week by week, and used again in revision. Cross-references point to the other volumes where a topic is treated in more depth.

How to cite this volume

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A note on the case material

All families, couples and individuals in this book are fictional composites. They are written to teach pattern recognition, not to describe any real person. Nothing here replaces supervised clinical practice, professional registration, legal advice or medical care; legal content is orientation only and must be checked against the law of the relevant jurisdiction.

The lecture library

Twenty-eight self-contained course books carry the taught programme: eight in Semester One, seven in Semester Two, seven in Semester Three including the thesis, and six electives. The volume you are holding is marked in the margin.

Code	Course	Semester	Units
MFT 701	Fundamentals of Marriage and Family Therapy	One	1.71
MFT 702	Theories of Family Therapy	One	1.71
MFT 703	Human Development Across the Lifespan	One	1.71
MFT 704	Ethical and Legal Issues in Therapy	One	1.71
MFT 705	Research Methods in Marriage and Family Therapy	One	1.71
MFT 706	Clinical Practice and Techniques	One	1.71
MFT 707	Assessment and Diagnosis in Family Therapy	One	1.71
MFT 708	Cultural Competence in Therapy	One	1.71
MFT 709	Group Therapy Techniques	Two	1.71
MFT 710	Couples Therapy	Two	1.71
MFT 711	Child and Adolescent Therapy	Two	1.71
MFT 712	Crisis Intervention Strategies	Two	1.71
MFT 713	Substance Abuse and Family Dynamics	Two	1.71
MFT 714	Trauma-Informed Care	Two	1.71
MFT 715	Family Systems Theory	Two	1.71
MFT 716	Communication Skills in Therapy	Three	1.71
MFT 717	Psychopathology in Families	Three	1.71
MFT 718	Intimacy and Relationship Dynamics	Three	1.71
MFT 719	Family Life Cycle and Transitions	Three	1.71
MFT 720	Supervision and Consultation in Therapy	Three	1.71
MFT 721	Research Methodology	Three	1.71
MFT 722	Professional Development and Self-Care	Electives	1.71
MFT 723	Special Populations in Therapy	Electives	1.71
MFT 724	Family Law and Advocacy	Electives	1.71
MFT 725	Integrative Approaches in Therapy	Electives	1.71
MFT 726	**Neurobiology of Relationships**	Electives	1.71
MFT 727	Play Therapy Techniques	Electives	1.71
MFT 770	Master Thesis	Three	1.71

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Course leader's summary. Neuroscience does not replace systemic thinking; it supplies the mechanism. This course teaches what is well established, what is contested, and what is oversold — so that you can use physiology to help families regulate, without pretending to be a neuroscientist.

How to use this book

The field is full of confident claims that outrun the evidence. Each week distinguishes three levels: **robust** (well replicated), **plausible** (reasonable but uncertain), and **popular** (widely repeated, weakly supported). You are expected to be able to say which is which.

1. Course description

The course covers the autonomic nervous system and co-regulation; the social brain; stress physiology and the HPA axis; attachment as a biological system; memory and trauma; the physiology of couple conflict; sleep, exercise and substances; adolescent and ageing brains; neuroplasticity; and the responsible use of neuroscience in clinical communication.

2. Learning outcomes

#	Outcome	Assessed
LO1	Explain the autonomic, endocrine and central mechanisms relevant to relationships	Exam
LO2	Use physiological knowledge to pace and structure sessions	Observed session
LO3	Teach regulation skills accurately and without over-claiming	Skills test
LO4	Explain the physiology of couple conflict and repair	Case study
LO5	Distinguish robust from popular claims in the literature	Critical appraisal
LO6	Integrate physiological and systemic formulations	Case study
LO7	Communicate neuroscience to families in accessible language	Viva

3. Assessment

Component	Weight	Due
Critical appraisal of a "neuro" claim (1,500 words)	20%	Week 5
Regulation skills teaching (20 min)	25%	Week 8
Integrative case study (2,500 words)	35%	Week 12
Examination	20%	Week 14

4. The fourteen-week plan

Week	Topic	Practical
1	Why physiology belongs in family therapy	Physiology in a case
2	The autonomic nervous system	Vagal braking demonstration
3	Co-regulation and the dyad	Co-regulation observation
4	Polyvagal theory: use and critique	Ladder mapping
5	**Appraisal due**; stress and the HPA axis	Stress mapping
6	Attachment as biology	Attachment formulation
7	The social brain	Neuroanatomy for clinicians
8	**Regulation skills teaching**	Assessed
9	Memory, trauma and the body	Trauma-informed pacing
10	Couple conflict physiology	Flooding and repair
11	Sleep, exercise, substances and mood	Lifestyle screening
12	**Case study due**; adolescent and ageing brains	Developmental adaptation
13	Neuroplasticity and the limits of the claim	Critical reading
14	Integration and examination	Review

Week 1 — Why physiology belongs in family therapy

Four clinical facts that physiology explains.

1. Why a person cannot "just calm down" mid-argument (autonomic arousal; prefrontal resources offline).
2. Why a parent's calm voice regulates a child (co-regulation before self-regulation).
3. Why the same conversation is possible at 10 a.m. and impossible at 11 p.m. (fatigue, cortisol, blood sugar).
4. Why trauma changes memory and reaction (implicit memory, sensitised threat detection).

What physiology does not explain. Meaning, culture, power, loyalty and choice. A physiological account is a mechanism, not a formulation. It tells you how the system works; it does not tell you what the family's behaviour means.

The rule of use. Use physiology to (a) pace the session, (b) teach regulation, (c) reduce shame ("your body is doing its job"), and (d) justify referral. Do not use it to explain away relational responsibility.

Week 2 — The autonomic nervous system

Two branches, three states (simplified).

- **Sympathetic** — mobilisation: heart rate up, pupils dilate, digestion down, muscles tense. Fight or flight.
- **Parasympathetic** — rest and digestion, plus (via the ventral vagal pathway) social engagement.
- **Dorsal vagal shutdown** — immobilisation: numbness, collapse, dissociation.

Robust findings. The autonomic system responds rapidly and largely outside awareness; arousal affects cognition; chronic stress alters baseline functioning; breathing and movement can shift arousal.

Plausible but contested. The precise mapping of "ventral vagal" states onto social behaviour is a theoretical proposal (Porges) with partial empirical support; the popular version often overstates what is known.

Clinical translation. Teach simple, reliable regulation: slower exhale, orienting, movement, grounding, cool water. These work regardless of which theory explains them.

Week 3 — Co-regulation

The concept. Regulation develops in relationships: a calm adult's voice, face, breathing and touch regulate an infant's arousal before the infant can regulate alone.

Robust. Infants synchronise with caregivers on multiple channels; caregiver depression and unresponsiveness affect infant physiology and behaviour; parental calm reduces child distress in experimental settings.

Clinical use.

1. In session, your own regulated state is an intervention: slow your speech, lower your volume, breathe out slowly.
2. Teach parents to co-regulate before reasoning: "First calm the body, then talk about the behaviour."
3. In couple work, teach partners to be a regulating presence rather than a stimulus.

The asymmetry to remember. Co-regulation is not always available. A dysregulated parent cannot lend a calm nervous system. Support the adult first.

Week 4 — Polyvagal theory: use and critique

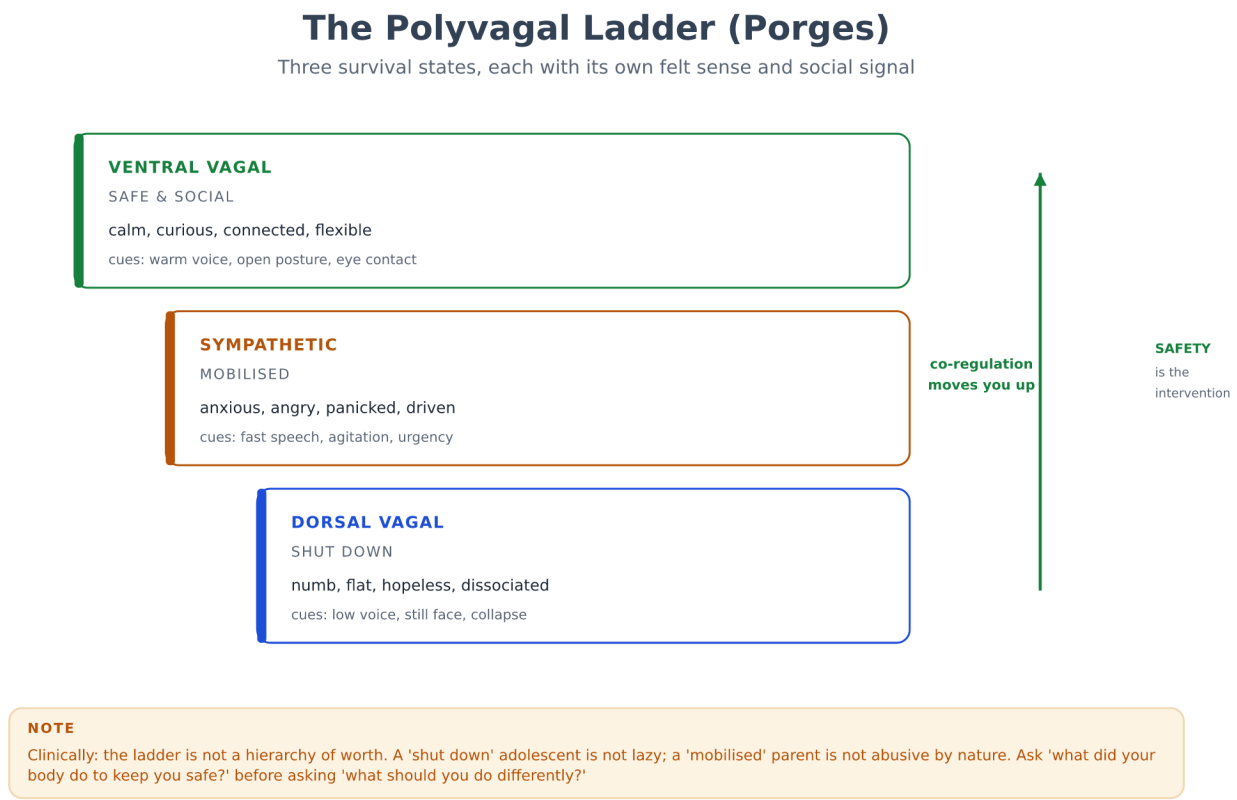


Figure 4.1 — The polyvagal ladder

What the theory claims. Three phylogenetically ordered states — ventral vagal (safe/social), sympathetic (mobilised), dorsal vagal (shutdown) — with the ventral vagal system enabling social engagement via myelinated pathways.

What is useful clinically. The ladder metaphor helps clients recognise state; "neuroception" (unconscious threat detection) explains reactions that seem disproportionate; and the emphasis on safety and co-regulation is good practice.

What is contested. The evolutionary claims, the strict correspondence between anatomy and behaviour, and the tendency to treat the three states as discrete and universal. Critics note the evidence is thinner than the popularity suggests.

The professional stance. Use the metaphor, label it as a metaphor, and do not present contested claims as established fact. Clients benefit from the map, not from a lecture on vagal anatomy.

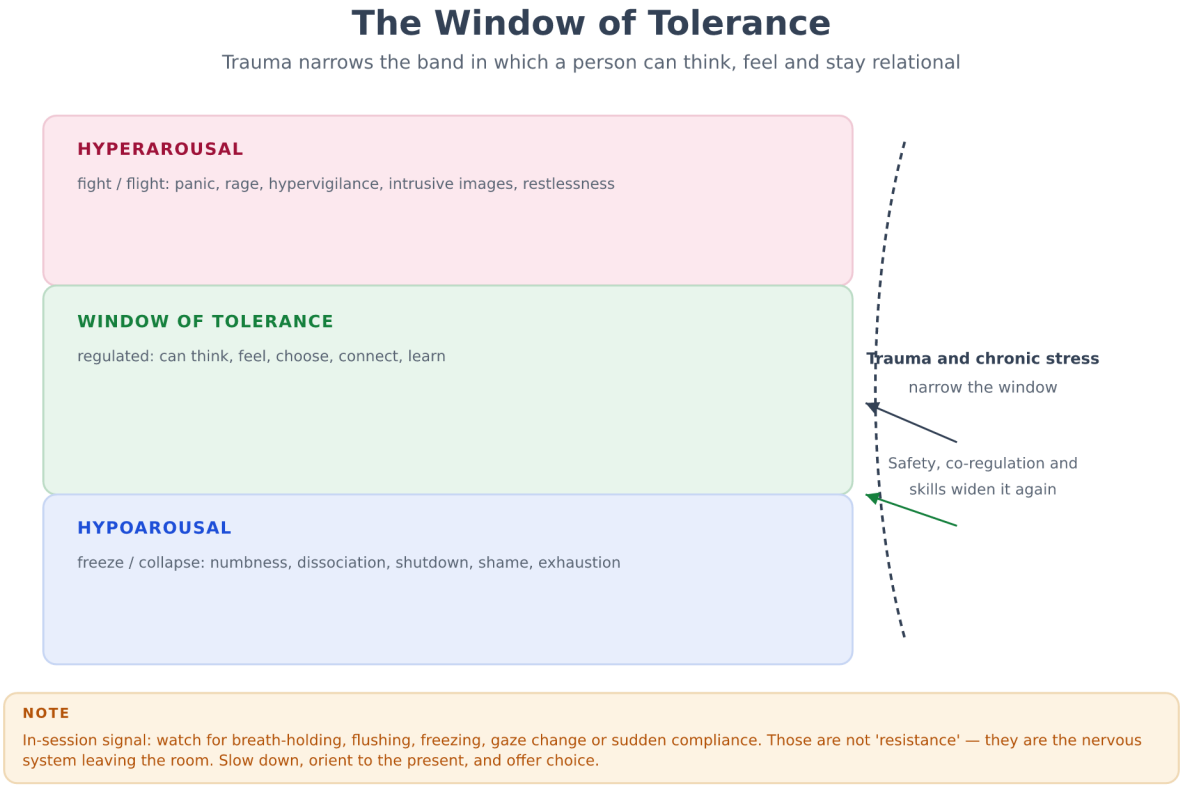


Figure 4.2 — The window of tolerance

Week 5 — Stress and the HPA axis

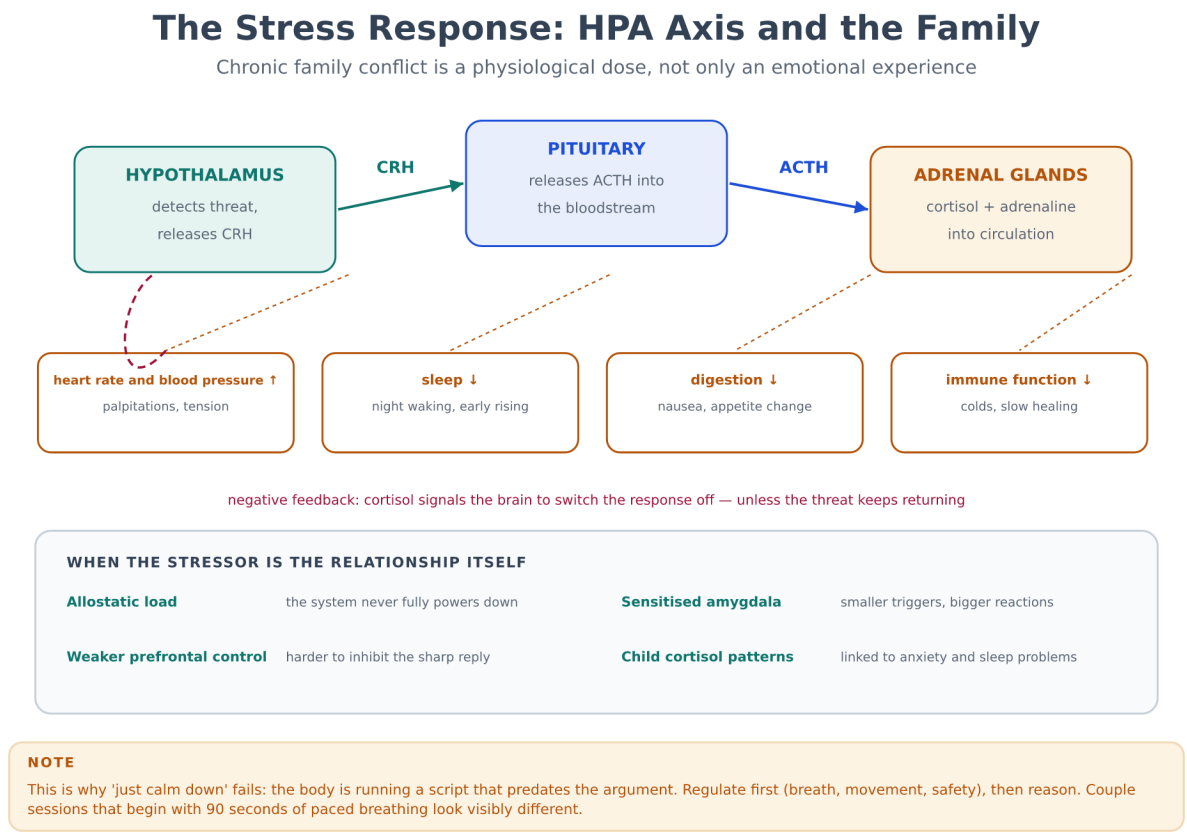


Figure 5.1 — The HPA axis and chronic family stress

The pathway. Hypothalamus (CRH) → pituitary (ACTH) → adrenal glands (cortisol) → feedback to the brain. Acute stress is adaptive; chronic stress produces allostatic load.

Robust findings. Chronic stress is associated with altered cortisol patterns, impaired immune function, sleep disruption, cardiovascular risk, and (in children) altered stress reactivity.

Plausible. Chronic relational conflict functions as a chronic stressor with measurable physiological effects. The size of the effect, and the direction of causality, remain debated.

Clinical translation.

- Screen for sleep, exercise, caffeine, alcohol, and medical conditions.
- Teach that physiological state affects the conversation: "You are not bad at communicating; you are tired at 9 p.m."
- Justify referral: persistent fatigue, weight change, palpitations, unexplained pain.

Week 6 — Attachment as biology

Robust. Attachment behaviours are observable and cross-culturally recognisable; caregiver responsiveness predicts later social and emotional outcomes; severe early deprivation has lasting effects (Romanian orphan studies).

Plausible. Attachment patterns are encoded as expectations and physiological set-points, and can be revised by later relationships.

Popular but overstated. Claims that specific brain regions "store" attachment, or that a single relationship "rewires" the brain in a defined number of weeks.

Clinical use. The attachment frame explains why a partner's tone matters more than their argument, and why therapy must provide a predictable relational experience before it can provide insight.

Week 7 — The social brain

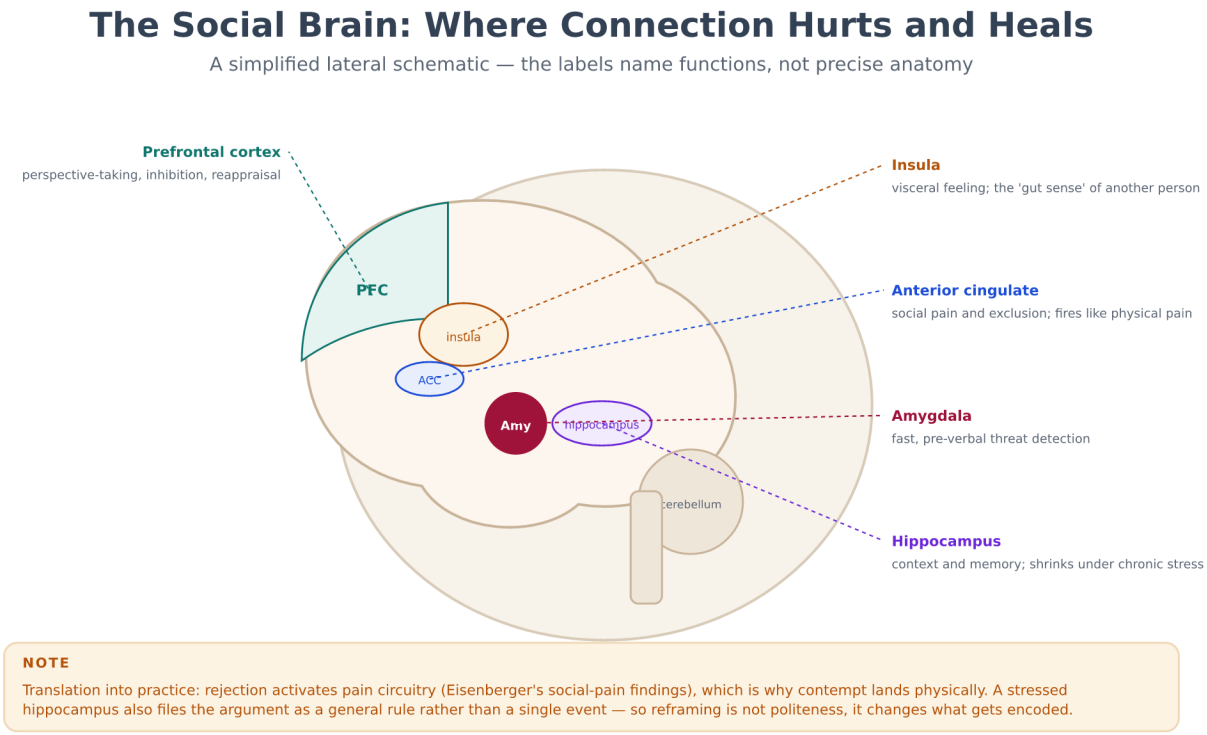


Figure 7.1 — A simplified social brain map

Regions and functions (functionally, not precisely).

Region	Function relevant to therapy
Prefrontal cortex	Perspective-taking, inhibition, reappraisal — offline under stress
Amygdala	Rapid threat detection, pre-verbal alarm
Insula	Interoception; the "gut sense" of another person
Anterior cingulate	Social pain, exclusion, conflict monitoring
Hippocampus	Context and memory; vulnerable to chronic stress
Mirror system (contested)	Imitation and resonance; overstated in popular accounts

Robust. Social exclusion activates pain-related circuitry (Eisenberger); reappraisal changes emotional response; stress impairs prefrontal function.

Contested. "Mirror neurons" as an explanation of empathy; the "right brain/left brain" attachment story; "the amygdala hijack" as a precise mechanism.

Clinical translation. Rejection hurts physically — say so, because it reduces shame. Reappraisal works but requires a regulated brain — so regulate first.

Week 8 — Teaching regulation

Assessed: teach a regulation skill in 20 minutes so that a family can use it at home.

A teaching sequence.

1. **Explain briefly** why the body matters (30 seconds, no lecture).
2. **Demonstrate** the skill yourself.
3. **Practise together** in the session.
4. **Troubleshoot** ("what would make this hard at home?").
5. **Anchor it** to a specific moment ("when you feel your jaw tighten").
6. **Write it down** and review next session.

Four reliable skills.

Skill	How	When
Extended exhale breathing	In for 4, out for 6–8, for 90 seconds	Before a difficult conversation
Orienting	Slowly look around, name five things	Dissociation, panic
Grounding	Feet, weight, texture, temperature	Trauma intrusion
Movement	Stand, walk, stretch, shake out	After conflict; restlessness

---

Week 9 — Memory, trauma and the body

Robust. Traumatic memories are often fragmented and sensory; stress hormones affect memory consolidation; avoidance maintains fear; exposure-based treatments work.

Plausible. Implicit memory contributes to reactions without a narrative; the body "remembers" in the sense that physiological responses are conditioned.

Popular but false. That traumatic memories are stored "in the muscles" and released by massage; that the body has a literal memory independent of the brain; that a single technique can "release" trauma.

Clinical translation.

- Do not require a narrative before stabilisation.
- Expect sensory fragments and treat them as memory, not madness.
- Use grounding to establish "now" versus "then".
- Refer for trauma-specific therapy when indicated; do not improvise.

Week 10 — Couple conflict physiology

Robust. Physiological arousal during conflict predicts poor problem-solving; high arousal narrows attention and reduces empathy; recovery after conflict predicts relationship outcomes.

Gottman's flooding threshold (roughly 100 bpm) is a clinical heuristic rather than a precise cut-off, but the principle is sound: above a certain arousal, listening is physiologically difficult.

Clinical use.

1. Teach a **break protocol**: agree a signal, separate for 20–30 minutes, self-regulate, return.
2. Explain that the break is not abandonment; it is a condition for a useful conversation.
3. Do not hold difficult conversations late at night, when tired, hungry, or after alcohol.
4. In session, watch for signs of flooding and pause before continuing.

Week 11 — Sleep, exercise, substances and mood

Sleep. Robustly linked to mood, irritability, attention and emotion regulation. Ask about it in every assessment, and treat it as a clinical target, not a lifestyle issue.

Exercise. Modest but reliable effects on mood and anxiety; a useful adjunct.

Alcohol and cannabis. Disrupt sleep architecture, increase impulsivity, and reduce inhibition — all relevant to couple conflict and parenting.

Caffeine and screens. Contribute to sleep disruption; worth asking about without moralising.

The clinical frame. "Let us look at what your body is doing before we ask it to have a better conversation."

Week 12 — Adolescent and ageing brains

Adolescence. Robust: increased reward sensitivity, heightened peer influence, gradual maturation of regulatory systems. Popularly overstated: "the adolescent brain is not fully formed until 25" is a simplification that can infantilise young people.

Ageing. Robust: processing speed and some memory functions decline; crystallised knowledge and emotional regulation often improve; social selectivity increases. Popular: "the brain stops changing" — neuroplasticity continues.

Clinical use. Match expectations to capacity without lowering them; scaffold rather than lecture; and design sessions around the person's best time of day.

Week 13 — Neuroplasticity and its limits

Robust. Experience changes the brain; learning involves synaptic change; rehabilitation after injury exploits plasticity.

Oversold. "You can rewire your brain in 21 days"; "neuroplasticity means anything is possible"; specific protocols claiming to rebuild neural pathways in defined sessions.

The clinical use. Neuroplasticity is the reason therapy can work and the reason it takes time. Say that; it is both hopeful and honest.

Week 14 — Integration

A physiological formulation answers: what is the person's current state, what regulates them, what dysregulates them, and what does the family do that helps or harms. Add it to the systemic formulation, do not replace it.

The communication standard. If you cannot explain it to a 12-year-old and to a sceptical colleague, you do not understand it well enough to use it.

Glossary

Allostatic load — cumulative physiological cost of chronic adaptation to stress. **Co-regulation** — regulation of one person's arousal by another's presence and behaviour. **Cortisol** — the primary glucocorticoid stress hormone. **HPA axis** — hypothalamic–pituitary–adrenal axis, the endocrine stress pathway. **Interoception** — perception of internal bodily states. **Neuroception** — unconscious evaluation of safety and threat (Porges). **Polyvagal theory** — a theory linking autonomic states to social behaviour. **Window of tolerance** — the arousal range within which a person can think and connect.

Self-assessment

Multiple choice

1. Above a certain level of physiological arousal, a person: (a) listens better (b) finds listening and empathy physiologically difficult (c) becomes more rational (d) is unaffected

2. Co-regulation means: (a) self-soothing alone (b) one person's calm regulating another's arousal (c) medication (d) breathing only

3. The HPA axis ends with: (a) the amygdala (b) cortisol release from the adrenal glands (c) the hippocampus (d) the vagus nerve

4. Social exclusion: (a) is emotionally neutral (b) activates pain-related circuitry (c) improves resilience (d) has no physiological correlate

5. "The brain is fully formed at 25" is: (a) a precise finding (b) a simplification that can infantilise young people (c) false in every sense (d) irrelevant

6. Polyvagal theory is best described as: (a) fully established (b) clinically useful and partly contested (c) disproven (d) unrelated to therapy

7. Neuroplasticity means that: (a) change is instant (b) experience shapes the brain, over time (c) therapy is unnecessary (d) the brain stops changing at 25

8. A break protocol in couple conflict works because: (a) it avoids the issue (b) it allows arousal to fall before the conversation continues (c) it punishes the partner (d) it is recommended by law

9. Sleep should be assessed because it: (a) is not clinical (b) is robustly linked to mood and regulation (c) only matters for children (d) is a lifestyle preference

10. Physiological explanations should be used to: (a) replace formulations (b) pace sessions, teach regulation and reduce shame (c) assign blame (d) impress families

Answer key

1 b · 2 b · 3 b · 4 b · 5 b · 6 b · 7 b · 8 b · 9 b · 10 b

Short answer

1. Name three robust and one contested claim in the neuroscience of relationships.
2. Describe the HPA axis in five steps.
3. Give a four-step teaching sequence for a regulation skill.
4. Why is "regulate before reason" clinically important in couple conflict?
5. What is the difference between a physiological account and a formulation?

Essay (choose one, 2,000 words)

1. "Neuroscience has been oversold to therapists." Critically evaluate with three examples.
2. Integrate a physiological and a systemic formulation for a couple with escalating conflict.
3. How should a family therapist talk about the brain to a sceptical 15-year-old and a religious grandmother?

Further reading

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Next: MFT 727 — Play Therapy Techniques

MSC MARRIAGE AND FAMILY THERAPY

LECTURE LIBRARY · VOLUME 28 OF 28

MFT 727

Play Therapy Techniques

14 teaching weeks · 3 practicals · 1 figure

ELECTIVES

1.71 units · Prerequisite: MFT 711

> “Play is the child's language and the therapist's instrument.”

One of 28 self-contained course books in the MSc Marriage and Family Therapy lecture library: a full taught semester per volume, with cases, practicals, assessment briefs, glossaries and model answers.

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MFT 727 — Play Therapy Techniques

MSc Marriage and Family Therapy · Elective · 1.71 Units · 14 teaching weeks · Prerequisite: MFT 711

This volume is one of 28 course books that together cover the whole MSc Marriage and Family Therapy taught programme. Each book is written to carry a 14–16 week semester on its own: every week pairs a core idea with the practice that follows from it, a worked case, and a reflective task, and each book closes with an integration chapter, a glossary and a self-assessment paper with model answers.

The books are designed to be read straight through before the semester begins, revisited week by week, and used again in revision. Cross-references point to the other volumes where a topic is treated in more depth.

How to cite this volume

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A note on the case material

All families, couples and individuals in this book are fictional composites. They are written to teach pattern recognition, not to describe any real person. Nothing here replaces supervised clinical practice, professional registration, legal advice or medical care; legal content is orientation only and must be checked against the law of the relevant jurisdiction.

The lecture library

Twenty-eight self-contained course books carry the taught programme: eight in Semester One, seven in Semester Two, seven in Semester Three including the thesis, and six electives. The volume you are holding is marked in the margin.

Code	Course	Semester	Units
MFT 701	Fundamentals of Marriage and Family Therapy	One	1.71
MFT 702	Theories of Family Therapy	One	1.71
MFT 703	Human Development Across the Lifespan	One	1.71
MFT 704	Ethical and Legal Issues in Therapy	One	1.71
MFT 705	Research Methods in Marriage and Family Therapy	One	1.71
MFT 706	Clinical Practice and Techniques	One	1.71
MFT 707	Assessment and Diagnosis in Family Therapy	One	1.71
MFT 708	Cultural Competence in Therapy	One	1.71
MFT 709	Group Therapy Techniques	Two	1.71
MFT 710	Couples Therapy	Two	1.71
MFT 711	Child and Adolescent Therapy	Two	1.71
MFT 712	Crisis Intervention Strategies	Two	1.71
MFT 713	Substance Abuse and Family Dynamics	Two	1.71
MFT 714	Trauma-Informed Care	Two	1.71
MFT 715	Family Systems Theory	Two	1.71
MFT 716	Communication Skills in Therapy	Three	1.71
MFT 717	Psychopathology in Families	Three	1.71
MFT 718	Intimacy and Relationship Dynamics	Three	1.71
MFT 719	Family Life Cycle and Transitions	Three	1.71
MFT 720	Supervision and Consultation in Therapy	Three	1.71
MFT 721	Research Methodology	Three	1.71
MFT 722	Professional Development and Self-Care	Electives	1.71
MFT 723	Special Populations in Therapy	Electives	1.71
MFT 724	Family Law and Advocacy	Electives	1.71
MFT 725	Integrative Approaches in Therapy	Electives	1.71
MFT 726	Neurobiology of Relationships	Electives	1.71
MFT 727	**Play Therapy Techniques**	Electives	1.71
MFT 770	Master Thesis	Three	1.71

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Course leader's summary. Play is the child's language and the therapist's instrument. This course trains you to set up a room, hold a session, use directed and non-directed methods, involve parents as partners, and recognise when play is not enough.

How to use this book

Play therapy is learned in a room, not in a book — but a badly equipped room produces bad therapy. Work through the weeks in order: room, stance, skills, methods, family application, limits. Practise every technique on a peer before you use it with a child.

1. Course description

The course covers the history and theory of play therapy; the child-centred (non-directive) model and its core skills; directive and cognitive-behavioural play; sandtray; art and puppetry; filial therapy (teaching parents to be therapeutic agents); family play therapy; assessment through play; trauma and play; and the evidence base.

2. Learning outcomes

#	Outcome	Assessed
LO1	Explain the theory and evidence for play therapy	Exam
LO2	Set up a safe, well-equipped playroom	Room plan and rationale
LO3	Conduct a non-directive session using core skills	Observed session
LO4	Use at least three directive techniques appropriately	Skills portfolio
LO5	Teach parents filial skills	Parent session observed
LO6	Assess through play without over-interpreting	Case study
LO7	Recognise limits and refer	Viva

3. Assessment

Component	Weight	Due
Playroom plan with item rationale (1,200 words)	15%	Week 4
Observed non-directive session (30 min)	30%	Week 7
Techniques portfolio (five techniques, with reflection)	20%	Week 10
Case study including filial work (2,500 words)	25%	Week 13
Examination	10%	Week 14

4. The fourteen-week plan

Week	Topic	Practical
1	What play is for; history and theory	Free play observation
2	The therapeutic powers of play	Session structure mapping
3	The room and the toys	Room setup
4	**Room plan due**; limit setting	Limit-setting rehearsal
5	Non-directive skills I: tracking and reflecting	Tracking drill
6	Non-directive skills II: returning responsibility, esteem-building	Skills drill
7	**Observed non-directive session**	Assessed
8	Directive techniques: art, puppets, role-play, stories	Technique practice
9	Sandtray	Sandtray practice
10	**Portfolio due**; assessment through play	Assessment exercise
11	Trauma and play; dissociation in the playroom	Pacing and grounding
12	Filial therapy: parents as agents	Parent coaching
13	**Case study due**; family play therapy	Family session design
14	Limits, evidence and integration	Viva

Week 1 — What play is for

Theory. Play is children's natural medium for expression, mastery, and meaning-making. Axline's eight principles (1947) remain the most quoted statement of the non-directive stance: the therapist develops a warm relationship; accepts the child as they are; establishes permissiveness; recognises and reflects feelings; respects the child's capacity to solve problems; does not direct; does not hurry; and sets only those limits that are necessary.

Why it works (proposed mechanisms). Play allows symbolic expression of experience that cannot be verbalised; it provides a sense of control and mastery; it allows rehearsal of difficult situations; and the relationship with the therapist provides a corrective relational experience.

Evidence. Play therapy has moderate support for a range of child problems, with stronger effects when parents are involved (filial therapy) and when the approach is matched to the problem (e.g. CBPT for anxiety). It is not a panacea, and it is not a substitute for trauma-specific treatment where indicated.

> **PRACTICAL**
> Observe a child in free play for 15 minutes. Record behaviours only: what they chose, what they returned to, what they avoided, how they ended.

Week 2 — The therapeutic powers of play

Power	What it looks like	How the therapist supports it
Communication	The child enacts the problem	Track and reflect
Emotional regulation	Repetitive calming play	Allow repetition; contain
Mastery	Replaying a frightening event with a different ending	Do not interpret; note the shift
Symbolic distance	The "monster" rather than the father	Stay in the metaphor
Relationship	The child tests and then trusts	Consistency, warmth, limits
Imagination	New possibilities	Follow, do not lead

Session shape for a child. Opening ritual (same each time), free play, a focused activity, a closing ritual. Predictability is the intervention.

Week 3 — The room and the toys

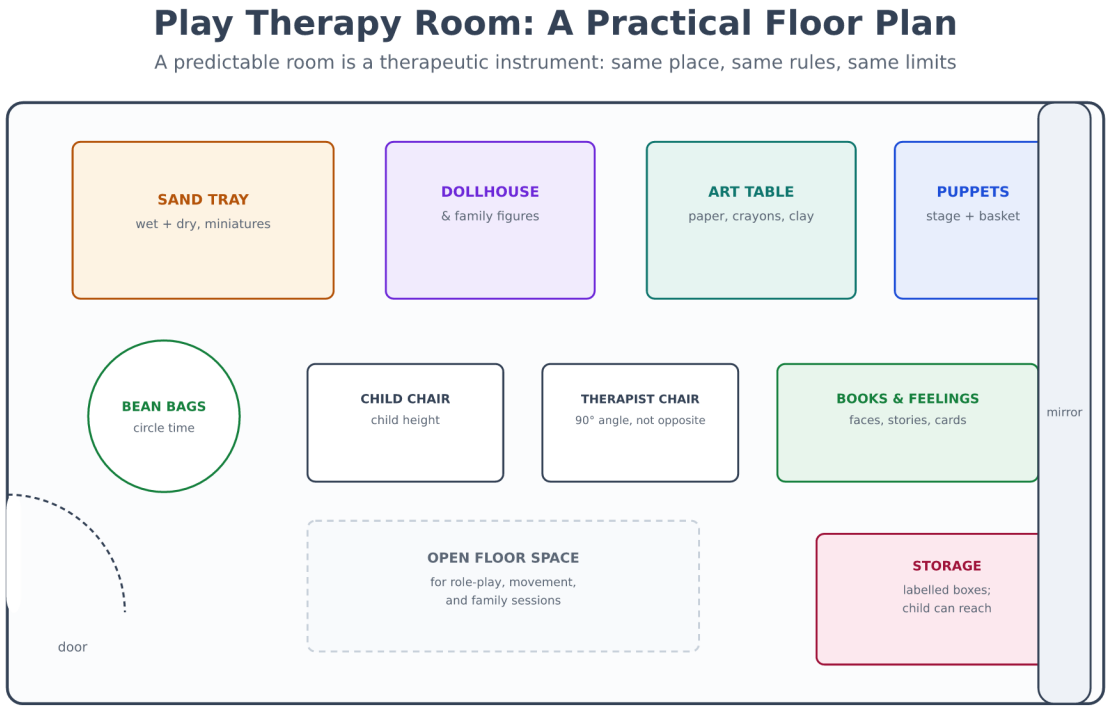


Figure 3.1 — A play therapy room: layout, furniture and sightlines

Principles. Safety, predictability, accessibility, and privacy. The child should be able to reach what they need; the therapist should be able to see everything.

Toy categories (Landreth's classification).

1. **Real-life toys** — dolls, family figures, cars, a cash register, a phone, cooking utensils.
2. **Acting-out/aggressive toys** — toy soldiers, a punch bag, wild animals, a toy gun, dinosaurs.
3. **Creative/expressive toys** — crayons, paint, clay, sand, blocks, musical instruments.

What to include. Sand tray, water, dolls and family figures, animal families, puppets, art materials, blocks, a dollhouse, a medical kit, a small house, blankets, and a soft toy.

What to avoid. Toys that require the therapist's help to operate; toys with one "right" answer; electronic toys that dominate; and anything that could be used to hurt without a limit.

Room rules posted at child height. "We stay in the room. We do not hurt ourselves. We do not hurt each other. We do not hurt the room. We put things away when we finish."

Week 4 — Limit setting

Why limits matter. Limits create safety, teach self-control, and protect the relationship. Permissiveness without limits is not acceptance; it is abandonment.

The ACT model (Landreth).

- **A — Acknowledge the feeling:** "You are really angry with me."
- **C — Communicate the limit:** "But I am not for hitting."
- **T — Target an acceptable alternative:** "You can hit the cushion, or you can tell me with words."

Sequence. State the limit calmly, once; repeat with the alternative; if it continues, offer a choice ("you can choose to put the toy down or I will put it away"); then follow through without anger.

What not to do. Negotiate; explain at length; threaten consequences you will not enforce; or shame the child.

> **REHEARSAL**
> Role-play a child who throws a toy at you, three times. The third attempt should be delivered with the same calm as the first.

Week 5 — Non-directive skills I

Tracking — narrating behaviour without evaluation: "You are putting the blue block on top. It wobbles."

Reflecting content — "You want the mother doll to go away."

Reflecting feeling — "You look pleased with that." / "That made you angry."

Rules of thumb.

- Describe, do not question.
- Name feelings tentatively ("you seem…"), allowing correction.
- Follow, do not lead.
- Stay in the metaphor; do not translate ("so the monster is your father" is an interpretation, not a reflection).

> **DRILL**
> Ten minutes with a peer playing a 6-year-old. You may only track and reflect. No questions. The observer counts interpretations (target: zero).

Week 6 — Non-directive skills II

Returning responsibility — handing the problem back: "That is something you can decide." / "You can try it."

Esteem-building — attributing effort and choice, not talent: "You kept trying until it stood up."

Facilitating self-direction — "You know what you want to do next."

Enlarging the meaning — offering a slightly wider frame: "You are the one who decides what happens in this story."

The therapist's own regulation. Children notice your body. Breathe slowly, sit at their level, keep your face open, and speak more slowly than you think you should.

Week 7 — Observed non-directive session

Thirty minutes with a trained child actor (or a filmed role-play marked against criteria). Assessed on: room and structure; tracking and reflection; returning responsibility; one limit set well; warmth; and the closing ritual.

Week 8 — Directive techniques

Directive play is not instruction. It is the deliberate use of a medium to address a specific issue, always following the child's lead within the structure.

Technique	Use	How
Drawing (family, house, fears, "worst day")	Assessment and expression	"Draw your family doing something." Then ask them to tell you about it

Technique	Use	How
Puppets	Safe displacement	Let the child choose a puppet; interview the puppet, not the child
Role-play	Rehearsal and mastery	"Show me what happens at the school gate"
Bibliotherapy	Normalising and skills	Read a story about a similar problem; discuss the character
Feelings cards / thermometer	Emotional vocabulary	Sort into "big/little", "now/then"
CBPT	Anxiety, trauma	Psychoeducation, coping skills, graded exposure in play

Sequence for a directive activity. Explain briefly → demonstrate → let the child lead → observe → reflect → close.

Week 9 — Sandtray

What it is. A tray of dry and wet sand with miniature figures, used to create a world.

Two traditions. Non-directive (the child builds, the therapist witnesses and reflects) and directive (therapist suggests a theme, e.g. "make a world where you feel safe").

Practicalities. Trays with a water-resistant base; dry sand and water; a wide range of miniatures (people, animals, buildings, vehicles, natural objects, fantasy, symbols of spirituality); a camera to record the scene (with consent); and a clear policy about dismantling.

Interpretation discipline. Do not impose symbolic meanings. Ask: "Tell me about this world." "What happens next?" "Who is this?" The child is the authority on their world.

With older children and adults, sandtray can be used as a non-verbal way of representing a family system — who is placed near whom, what is buried, what is outside the tray.

Week 10 — Assessment through play

What you can learn. Developmental level (language, symbolic play, attention, motor control); emotional expression; themes (abandonment, violence, rescue, repetition); relational capacity (can they play with you?); and the impact of specific events.

What you cannot learn reliably. Specific causation; the identity of an abuser; or a diagnosis from a single drawing. Any assessment claim must be cautious and triangulated with other data.

Signs warranting attention. Play that is repetitive, narrowed and joyless; sexualised play beyond developmental norms; play involving harm to self or others; a sudden change in play after an event; and a child who cannot play at all.

Documentation. Describe behaviour and the child's own words; avoid interpretation in the record.

Week 11 — Trauma and play; dissociation

Principles (from MFT 714). Safety and stabilisation first; the caregiver's regulation is the child's best medicine; do not force the narrative; and expect repetition.

In the playroom. Watch for signs of dissociation (stillness, glazed eyes, going blank, sudden compliance). Slow down, orient to the room, offer a grounding object, and return to safe play.

Post-traumatic play is repetitive and lacks resolution. The therapeutic task is not to stop it but to help it move — by reflecting feeling, introducing a small element of mastery, and involving the caregiver.

When play is not enough. Severe trauma symptoms, self-harm, dissociation, and complex abuse histories need trauma-specific treatment (e.g. TF-CBT) alongside or after stabilisation.

Week 12 — Filial therapy

What it is. Training parents to conduct child-centred play sessions at home, with supervision. Developed by Guerney; extensively researched; strong evidence for improving parent–child relationships and reducing child problems.

Why it works. It increases parental empathy and reflective function, changes the parent's view of the child, and provides a structured positive interaction that most distressed families lack.

The structure (typically 10 sessions).

1. Teach the principles and skills (tracking, reflecting, limit setting).
2. Demonstrate a play session with the child while the parent observes.
3. The parent conducts a session while the therapist coaches.
4. The parent conducts sessions at home (30 minutes weekly) and reports back.
5. Review and generalise.

Practical adaptations. Limited space at home; other children; a parent who works nights; a parent with their own trauma history (support first).

> **PARENT SESSION**
> Coach a parent through a 15-minute play session using tracking, reflection and one limit.

Week 13 — Family play therapy

Why involve the family. The child's problem exists in a system; play can make the system visible.

Techniques.

- **Family drawing and sculpture** — who sits where, who is close, who is left out.
- **Co-constructed play** — the family builds one world together; observe negotiation, hierarchy and exclusion.
- **Role reversal** — the child directs the parents in a scene; useful for revealing the child's experience.
- **Structured family games** — cooperative tasks that require negotiation.

Caution. Family play can expose conflict. Have a structure for containing it, and be ready to end the activity and debrief.

Week 14 — Limits, evidence and integration

The limits of play therapy. Not a substitute for medical assessment, safeguarding action, trauma-specific treatment or family work where the family system is the problem. Play therapy with a child in a family that continues to harm them is not treatment; it is containment.

The evidence summary. Moderate support across problems; stronger when parents are involved; strongest when the approach is matched to the problem and delivered with fidelity.

Integration. Play therapy sits comfortably within a systemic frame when it is used as the child's channel of communication within a family assessment, and when parents are brought into the work.

Glossary

ACT model — Acknowledge, Communicate the limit, Target an alternative. **Child-centred play therapy** — non-directive approach following the child's lead (Axline, Landreth). **CBPT** — cognitive-behavioural play therapy. **Filial therapy** — training parents to conduct child-centred play sessions. **Post-traumatic play** — repetitive, unresolving play following trauma. **Sandtray** — a tray of sand with miniatures used for symbolic expression. **Therapeutic powers of play** — the mechanisms through which play produces change.

Self-assessment

Multiple choice

1. The ACT model stands for: (a) Acknowledge, Communicate, Target (b) Ask, Calm, Tell (c) Assess, Contain, Treat (d) Attend, Connect, Teach

2. In non-directive play therapy the therapist: (a) leads the play (b) follows the child's lead (c) interprets symbols (d) teaches skills

3. Telling a child "the monster is your father" is: (a) a reflection (b) an interpretation and generally to be avoided (c) tracking (d) limit setting

4. Filial therapy involves: (a) the child alone (b) training parents to conduct child-centred sessions (c) medication (d) group therapy

5. Post-traumatic play is typically: (a) joyful (b) repetitive and without resolution (c) absent (d) directed by the therapist

6. Sandtray interpretation should be: (a) imposed by the therapist (b) led by the child's own account (c) avoided entirely (d) symbolic decoding

7. Limits in play therapy exist to: (a) control the child (b) create safety and teach self-control (c) save toys (d) satisfy parents

8. A child's drawing alone: (a) diagnoses abuse (b) cannot diagnose or identify an abuser (c) is irrelevant (d) replaces assessment

9. Play therapy is contraindicated as a substitute for: (a) warmth (b) safeguarding action and trauma-specific treatment (c) structure (d) toys

10. The strongest play therapy effects are found when: (a) the child is seen alone for a year (b) parents are involved and the approach matches the problem (c) the therapist interprets (d) sessions are longer

Answer key

1 a · 2 b · 3 b · 4 b · 5 b · 6 b · 7 b · 8 b · 9 b · 10 b

Short answer

1. State Axline's principles in your own words (five of the eight).
2. List the three categories of toys with two examples of each.
3. Give the ACT sequence with a scripted example.
4. What are the five steps of a child-centred session structure?
5. When is play therapy not enough, and what do you do?

Essay (choose one, 2,000 words)

1. "Play is the child's language." Discuss the implications for assessment and treatment.
2. Design a play therapy service for children in a Nigerian primary school, including the room, the toys, the referral pathway and the evidence.

3. Analyse a play session (yours or a recorded one) using the core skills, and identify what you would do differently.

Further reading

- Axline, V. M. (1947/1969). Play Therapy. Ballantine.
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- Bratton, S. C., Ray, D., Rhine, T., & Jones, L. (2005). The efficacy of play therapy with children: A meta-analytic review. Professional Psychology: Research and Practice, 36(4), 376–390.
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End of the elective sequence. Return to the Programme Handbook.

MSC MARRIAGE AND FAMILY THERAPY

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MFT 770

Master Thesis

14 teaching weeks · 2 practicals

SEMESTER THREE

1.71 units · Prerequisite: MFT 705, 721

> “The thesis is not a bigger essay.”

One of 28 self-contained course books in the MSc Marriage and Family Therapy lecture library: a full taught semester per volume, with cases, practicals, assessment briefs, glossaries and model answers.

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MFT 770 — Master Thesis

MSc Marriage and Family Therapy · Semester Three (continuing) · 1.71 Units · 14 structured weeks + submission and defence · Prerequisite: MFT 705, 721

This volume is one of 28 course books that together cover the whole MSc Marriage and Family Therapy taught programme. Each book is written to carry a 14–16 week semester on its own: every week pairs a core idea with the practice that follows from it, a worked case, and a reflective task, and each book closes with an integration chapter, a glossary and a self-assessment paper with model answers.

The books are designed to be read straight through before the semester begins, revisited week by week, and used again in revision. Cross-references point to the other volumes where a topic is treated in more depth.

How to cite this volume

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A note on the case material

All families, couples and individuals in this book are fictional composites. They are written to teach pattern recognition, not to describe any real person. Nothing here replaces supervised clinical practice, professional registration, legal advice or medical care; legal content is orientation only and must be checked against the law of the relevant jurisdiction.

The lecture library

Twenty-eight self-contained course books carry the taught programme: eight in Semester One, seven in Semester Two, seven in Semester Three including the thesis, and six electives. The volume you are holding is marked in the margin.

Code	Course	Semester	Units
MFT 701	Fundamentals of Marriage and Family Therapy	One	1.71
MFT 702	Theories of Family Therapy	One	1.71
MFT 703	Human Development Across the Lifespan	One	1.71
MFT 704	Ethical and Legal Issues in Therapy	One	1.71
MFT 705	Research Methods in Marriage and Family Therapy	One	1.71
MFT 706	Clinical Practice and Techniques	One	1.71
MFT 707	Assessment and Diagnosis in Family Therapy	One	1.71
MFT 708	Cultural Competence in Therapy	One	1.71
MFT 709	Group Therapy Techniques	Two	1.71
MFT 710	Couples Therapy	Two	1.71
MFT 711	Child and Adolescent Therapy	Two	1.71
MFT 712	Crisis Intervention Strategies	Two	1.71
MFT 713	Substance Abuse and Family Dynamics	Two	1.71
MFT 714	Trauma-Informed Care	Two	1.71
MFT 715	Family Systems Theory	Two	1.71
MFT 716	Communication Skills in Therapy	Three	1.71
MFT 717	Psychopathology in Families	Three	1.71
MFT 718	Intimacy and Relationship Dynamics	Three	1.71
MFT 719	Family Life Cycle and Transitions	Three	1.71
MFT 720	Supervision and Consultation in Therapy	Three	1.71
MFT 721	Research Methodology	Three	1.71
MFT 722	Professional Development and Self-Care	Electives	1.71
MFT 723	Special Populations in Therapy	Electives	1.71
MFT 724	Family Law and Advocacy	Electives	1.71
MFT 725	Integrative Approaches in Therapy	Electives	1.71
MFT 726	Neurobiology of Relationships	Electives	1.71
MFT 727	Play Therapy Techniques	Electives	1.71
MFT 770	**Master Thesis**	Three	1.71

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Course leader's summary. The thesis is not a bigger essay. It is a supervised, ethical, original inquiry that a professional body would accept as evidence that you can generate knowledge, not only consume it. This book is the project management manual plus the academic guide.

How to use this book

Work the milestones in order; they are dated to protect you from the two classic disasters — a brilliant plan that cannot be completed, and a completed study that answers nothing. Bring the weekly artefact to supervision even if it is imperfect. Progress beats polish.

1. What the thesis is

Element	Requirement
Length	15,000–20,000 words (excluding appendices), or the institutional equivalent
Contribution	Original, however modest: new data, new analysis, a local validation, a qualitative account, a service evaluation
Methods	Quantitative, qualitative or mixed; must be defensible and feasible
Ethics	Institutional approval before any data collection
Supervision	Minimum fortnightly contact with a named supervisor
Output	Bound thesis, oral defence, and (encouraged) a publishable manuscript

What it is not. A literature review alone (that is a different degree component). A clinical case series without a systematic method. A replication of a manual with no question. A project that requires resources you do not have.

2. The fourteen structured weeks

Week	Milestone	Artefact due in supervision
1	Topic refinement and scope	One-page problem statement
2	Literature search strategy	Search log and 20 key references
3	Critical literature review draft	2,000-word draft
4	Research question and aims	PICO/SPIDER and aims table
5	Design selection	Design decision log
6	Measurement and materials	Instrument pack
7	Ethics application	Completed ethics form
8	Sampling and recruitment plan	Recruitment protocol and letters
9	Analysis plan	Pre-specified analysis plan

Week	Milestone	Artefact due in supervision
10	Pilot or feasibility work	Pilot findings and amendments
11	Data collection checkpoint	Recruitment and data quality report
12	Analysis progress	First results tables/coding frame
13	Findings and discussion drafting	Draft chapters 4–5
14	Full draft and defence preparation	Complete draft, viva questions

Timeline discipline. A part-time student should assume 12–18 months from registration to defence. Full-time students commonly need 9–12 months. If your plan assumes everything goes right, add 30%.

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3. Choosing a question you can finish

The three-circle test. A viable thesis sits where (1) you are curious, (2) the field needs it, and (3) you have access. If any circle is missing, choose a different question, not a bigger effort.

Good thesis questions in this field (examples).

1. What is the reliability and factor structure of the RDAS in a Nigerian clinical sample?
2. How do fathers describe their experience of being excluded from family therapy?
3. Does a four-session psychoeducation group reduce caregiver burden among families of people with psychosis?
4. What do couples report about the process of rebuilding trust after infidelity?
5. What predicts drop-out after a first family therapy appointment in a community clinic?
6. How do therapists in training describe the influence of their own family of origin on their clinical decisions?
7. Does a culturally adapted version of the speaker–listener technique improve communication in a small clinical series?

Questions to avoid. "The effect of family therapy on family functioning" (too broad); "Why is family therapy better than individual therapy?" (advocacy, not inquiry); anything requiring a sample you cannot access or an instrument you cannot obtain.

> **ARTEFACT**
> A one-page problem statement: what is unknown, why it matters, who it affects, and what you will produce.

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4. The literature review

Purpose. Not to summarise everything, but to establish (a) what is known, (b) what is contested, (c) what is missing, and (d) why your question follows.

Search discipline.

- At least three databases (PubMed, PsycINFO, Scopus, AJOL, Google Scholar as a supplement).
- Document the strategy: terms, limits, date, number of results, number retained.
- Use a reference manager from day one.
- Read systematic reviews first; they map the field faster than primary papers.

Structure of the chapter. Context → key constructs and definitions → what the evidence shows → what is contested → what is missing (the gap) → your question and aims.

The gap must be specific. "Few studies have examined X in Y population using Z method" — not "more research is needed."

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5. Design and methods

The rule. The method chapter must be detailed enough that a competent reader could replicate the study, and honest enough that they could see its limits.

Chapter contents.

1. Design and rationale.
2. Setting and context.
3. Participants: inclusion, exclusion, recruitment, consent, sample size justification.
4. Measures: source, psychometrics, scoring, adaptation and translation procedures.
5. Procedure: what happened, in what order, for how long.
6. Analysis: software, models, assumptions, missing data, coding approach, rigour procedures.
7. Ethical considerations: approval reference, consent, confidentiality, risk, data storage.
8. Researcher position and reflexivity.

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6. Ethics

Required before any data collection. A retrospective application is a serious breach.

Standard components. Purpose and design; participants and recruitment; consent process and documents; risks and benefits; confidentiality and data management; payment; dissemination; and procedures for distress or incidental findings.

Family-specific issues. Consent from guardians plus assent from minors; separate consent for each family member; managing the possibility that one member withdraws; protecting group confidentiality; and a protocol for disclosures of risk during data collection.

ARTEFACT

A complete ethics application with consent forms for an adult and a minor, and a risk protocol for a participant who discloses harm during an interview.

7. Data collection

Pilot first. Five to ten participants will reveal problems that a hundred will not forgive: unclear questions, unworkable recruitment, unusable audio, unrealistic time estimates.

Data quality. Check recordings the same week; transcribe promptly; keep a fieldwork log; record refusals and reasons (they are data).

Recruitment tactics that work in clinical settings. A named contact who can introduce you; a one-page information sheet; flexible times; a clear statement of what participation involves; and a follow-up call at 48 hours. Do not rely on posters.

8. Analysis

Quantitative. Follow the pre-specified plan; document deviations; check assumptions; report descriptives before inference; report effect sizes with confidence intervals; run sensitivity analyses; keep the analysis log current.

Qualitative. Transcribe verbatim; code systematically; keep memos; audit your coding; discuss divergence with a second coder; move from codes to themes with an explicit, documented logic; and present themes with data extracts (anonymised).

Mixed methods. Decide in advance how the strands will be integrated, and produce at least one joint display.

9. Writing the thesis

Standard structure.

Chapter	Content	Typical length
1	Introduction and background	2,000
2	Literature review	5,000
3	Methodology	3,500
4	Findings	4,000
5	Discussion	3,500
6	Conclusion and recommendations	1,000
—	References, appendices	—

Writing rules.

- Write the method first; it is the easiest chapter and it clarifies everything.
- Draft findings as soon as you have data; do not wait for "enough".
- Keep a "parking lot" file for ideas that belong elsewhere.
- Write 500 words a day rather than 5,000 in a panic.
- Read your work aloud; if you run out of breath, the sentence is too long.

Voice. Academic but human. First person is acceptable in reflective and qualitative sections; follow your institution's guidance. Avoid both inflated jargon and chatty informality.

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10. The supervision relationship

What your supervisor owes you. Regular contact; timely feedback; methodological and ethical guidance; honesty about the standard of your work; and reasonable availability.

What you owe your supervisor. Prepared meetings with an agenda and an artefact; drafts submitted on time; decisions recorded; and honesty about difficulties — especially delays.

Managing difficulty. If supervision is not working, raise it early with the programme director. A change of supervisor is legitimate and common; drifting for six months is not.

The meeting agenda template.

1. Progress since last meeting (three bullets).
2. Decisions needed today (the most important item first).
3. Problems I am stuck on.
4. What I will do before the next meeting.
5. Date of next meeting.

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11. The viva

What examiners test. Whether you understand your own study: why you chose it, what you did, what it means, and what it cannot support.

Preparation.

1. Re-read your thesis and mark every claim you could not defend.
2. Prepare a two-minute summary.
3. Prepare answers to the four standard questions: design, validity, analysis, conclusion.
4. Rehearse with a peer who is instructed to be difficult.
5. Bring a copy of your thesis, your analysis log and a pen.

In the room. Answer the question asked. Say "I don't know" when you don't. Distinguish what the data show from what you hope they show. Correct errors gracefully.

Common reasons for referral or failure. Design cannot answer the question; analysis misapplied; ethics breach; unacknowledged limitations; and evidence of work not being the candidate's own.

12. After the defence

Corrections. Make them promptly, with a tracking table mapping each examiner comment to the change made.

Dissemination. Submit a manuscript to a journal; present at a conference; give a plain-language summary to the participating service. Dissemination is part of the ethical contract with your participants.

Your data. Retain as required by your institution's policy, then destroy securely.

13. Project management

Risk	Early warning	Mitigation
Recruitment failure	Fewer than 50% of target by mid-point	Widen eligibility; add sites; adjust design (with supervisor and ethics approval)
Analysis paralysis	Repeated re-coding without writing	Fixed decision date; write the methods and limitations
Supervisor unavailability	Missed meetings	Written agenda; escalate early
Personal crisis	Missed deadlines	Tell the programme early; request extension with evidence
Scope creep	New questions each week	Freeze the question after Week 4; park ideas

The one-page dashboard. Question, milestones, current position, next action, date, supervisor appointment. Update weekly; it will save your degree.

14. Assessment

Component	Weight
Proposal (approved by panel)	10%
Ethics approval	Pass/fail gate
Thesis	70%
Oral defence	20%

Marking criteria (typical). Originality and significance; command of the literature; appropriateness and rigour of method; quality of analysis; critical discussion; ethical practice; presentation and referencing.

Appendix: thesis checklist

- ■ Question is specific, answerable, and stated in one sentence
- ■ Literature search documented and reproducible
- ■ Gap statement is specific, not "more research is needed"
- ■ Design justified against at least one alternative
- ■ Sample size justification provided
- ■ Ethics reference number included
- ■ Measures described with psychometric properties and any adaptation
- ■ Analysis plan pre-specified; deviations recorded and explained
- ■ Assumptions checked and reported
- ■ Effect sizes with confidence intervals
- ■ Findings presented before interpretation
- ■ Limitations specific and honest
- ■ Implications for practice are actionable
- ■ Referencing consistent and complete
- ■ Plain-language summary prepared for participants

Further reading

- Wisker, G. (2012). The Good Supervisor. Palgrave.
- Booth, W., Colomb, G., & Williams, J. (2016). The Craft of Research (4th ed.). Chicago.
- Swetnam, D., & Swetnam, R. (2009). Writing Your Dissertation. How To Books.
- Braun, V., & Clarke, V. (2022). Thematic Analysis: A Practical Guide. Sage.
- Field, A. (2018). Discovering Statistics Using IBM SPSS Statistics (5th ed.). Sage.

End of Semester Three. Continue to the electives: MFT 722 — Professional Development and Self-Care